

PROFESSOR
AHMED TIJJANI MORA

A PILLAR OF PHARMACY REGULATION IN NIGERIA

PROFESSOR AHMED TIJJANI MORA

A PILLAR OF PHARMACY REGULATION IN NIGERIA

Authors:

Aliyu Jibrin, Ibrahim Yakasai, Umar Pateh, Ibrahim Ahmed,
Ibrahim Amidu, Kabiru Gulma

Ahmed Tijjani Mora: A Pillar of Pharmacy Regulation in Nigeria

© 2026 by *The Book Committee on Ahmed Tijjani Mora: A Pillar of Pharmacy Regulation in Nigeria*

All rights reserved.

No part of this publication may be reproduced, stored in a retrieval system, or transmitted in any form or by any means—electronic, mechanical, photocopying, recording, or otherwise—without prior written permission of the copyright holder, except for brief quotations used in scholarly or critical works.

For permissions and inquiries, contact:

The Book Committee on “Ahmed Tijjani Mora: A Pillar of Pharmacy Regulation in Nigeria”

First Edition, 2026

Printed and bound in Nigeria

ISBN: 1114-2108-1379.

Typeset in Times New Roman

This publication was developed under the auspices of the **Book Committee on “Ahmed Tijjani Mora: A Pillar of Pharmacy Regulation in Nigeria,”** established to document the professional journey and enduring impact of Professor Ahmed Tijjani Mora on pharmacy regulation and leadership.

All reasonable efforts have been made to ensure the accuracy of the information contained in this work. However, the publisher and editors assume no responsibility for any errors or omissions, or for the consequences arising from the use of the information herein.

Dedication

“To all who have dedicated their lives to strengthening the pharmacy profession.”

Table of Contents

Dedication.....	v
Table of Contents.....	vi
Acronyms	xxv
Foreword.....	xxix
Preface.....	xxxi
Acknowledgments.....	xxxv
 Chapter 1: Environment and Brief on Professor Ahmed Tijjani Mora.....	 1
Origins and Early Development of Ancient Zaria.....	1
The Name “Zaria”	1
The Era of Powerful Queens	1
Economic and Cultural Importance	1
The Fulani Jihad and the Sokoto Caliphate Era	2
Colonial and Modern Period	2
Tudun Wada Zaria	3
His Father’s Legacy.....	14
1. University of London Institute of Education Certificate Course (1945–1946).....	15
2. United States Department of State Fellowship Programme (1952– 1953).....	16
Ahmed’s Family Background.....	17
Childhood Years (1956–1971).....	18
Travels and Family Bonding	19
Formative Impact.....	22
Schools Attended	22

2.1 Qur’anic Schools in Zaria	22
2.2 LEA Primary School, Tudun Wada, Zaria.....	23
2.3 Government College, Zaria (Now Barewa College, Zaria)	25
2.4 School of Basic Studies (SBS), ABU, Zaria.....	26
2.5 Department of Pharmacy and Pharmacology, ABU, Zaria	26
2.6 Internship Training, ABUTH Kaduna	26
2.7 MBA Programme, ABU, Zaria	27
2.8 Fellowship of WAPCP	27
2.9 Ph.D. Programme, Usmanu Danfodiyo University, Sokoto (UDUS), Sokoto.....	27
2.10 Senior Executive Course, NIPSS Kuru	27
3.0 University Academic Career.....	28
3.1 Ahmadu Bello University (ABU), Zaria	28
3.2 Kaduna State University (KASU), Kaduna	29
3.3 Igbinedion University, Okada (IUO), Edo State	30
4.0 Career Employment in the Practice of Pharmacy.....	31
4.1 NYSC Pharmacist, General Hospital Potiskum, Borno State	31
4.2 Messrs. Grünenthal GmbH C/O Pharco (Nig.) Ltd., Kano	32
4.3 Kaduna State Distribution Agency (KSDA) Ltd., Kaduna	32
4.4 Health Management Board (HMB), Kaduna State	33
4.5 Ministry of Health (MoH), Kaduna State	33
4.6 Project Implementation Unit (PIU) – AFDB/AFDF Kaduna Health Rehabilitation Project (KHRP), Ministry of Health Headquarters, Kaduna.....	34
4.7 Disposable Syringes Manufacturing Plant Project, Dakace, Zaria	34

4.8 Zaria Pharmaceutical Company (ZPC) Ltd., Kaduna State	35
4.9 Zazzau Pharmaceutical Industries Ltd. (ZPIL), Zaria	35
4.10 Pharmacists Council of Nigeria (PCN), Abuja	35
4.11 Governing Council of the Pharmacist's Council of Nigeria (PCN)/Pharmacy Council of Nigeria (PCN)	36

5.0 Involvement in the Formation and Management of Learned Organizations

5.1 Zaria Educational Development Association (ZEDA)	36
5.2 Ahmadu Bello University Pharmacy Alumni Association (ABUPAA)	37
5.3 Barewa Old Boys Association (BOBA) – (1970 Intake Class Group)	38
5.4 Barewa Old Boys' Association (BOBA) – (National)	40

6.0 West African Pharmaceutical Manufacturers Association (WAPMA)

7.0 Ahmadu Bello University (ABU) Alumni Association

7.1 Nationwide Tours & Branch Revitalization	43
7.2 Infrastructure Development	43
7.3 Student Support Initiatives	43
7.4 Hostel Development	44
7.5 Capacity Building	44
7.6 Water Supply Project	44
7.7 International Collaboration	44
7.8 High-Level Engagements	44

8.0 The Conference of Alumni Associations of Nigerian Universities (CAANU) (Now Association of Nigerian Universities Alumni – (ANUA).....	45
8.1 Ahmed’s First Association with CAANU.....	46
8.2 The 2016 Election	46
8.3 Tenure and Re-Election	46
8.4 Key Achievements as National Chairman of CAANU (2016–2022).....	47
8.5 End of Tenure.....	50
9.0 The Nigerian Association of Pharmacists and Pharmaceutical Scientists in the Americas (NAPPSA).....	50
Participation in NAPPSA Conferences.....	50
Appointment to the Board of NAPPSA.....	51
Missed Conferences	51
Professional Standing and Collaboration.....	51
10.0 Member of the Nigerian Medical Team for the Care of Nigerian Muslims in Saudi Arabia During the Hajj Exercise.. ..	52
11.0 Honors, Awards, and Fellowships.....	53
12.0 Family of Prof. Ahmed T. Mora.....	63
13.0 Board and Council Appointments.....	64
14.0 Travels and Papers Presented	66
15.0 Chairman, D.S. Wali Book Project Committee.....	66
16.0 Who is the Most Renowned Pharmacist in Nigeria?	68
<i>Chapter 2: Pharmacists Board of Nigeria (PBN), As Chief Pharmacists/DPS, MoH, Kaduna) (1989-1992).....</i>	<i>71</i>

PHARMACISTS' BOARD OF NIGERIA (PBN): (1989 – 1992).	81
Membership of the PBN and PCN Boards.....	82
1. Disciplinary Tribunal	85
2. Investigating Panel.....	85
3. Appointment, Promotion, and Disciplinary Committee	85
4. Finance and General-Purpose Committee	85
5. Law and Ethics Committee.....	86
6. Education Committee	86
From Member to Registrar and Chief Executive Officer of the Governing Council	86
<i>Chapter 3: (Pharmacists Council of Nigeria (PCN): (i) DPS (1992/1993) and (ii) Rep of PSN (1999 - 2002).....</i>	<i>88</i>
Setting the National Scene — Pharmacy in Transition.....	95
The Young Reformer from Zaria	96
The First Governing Council (1992–1993) — Building on Empty Ground	96
Milestones of the First Council.....	97
(i) Appointment of the First Registrar	97
(ii) Introduction of Clinical Pharmacy and Pharmacy Practice in Nigerian Universities	97
(iii) Standardization and Inspection Initiatives	98
A Council Interrupted.....	98
Between Silence and Rebirth (1993–1999)	98
The Second Governing Council (1999–2002) — From Renewal to Reform.....	99
Achievements of the Second Council.....	99
(i) The Abuja Headquarters Project	99
(ii) Strengthening the Inspectorate and Logistics	100

(iii) Central Control of PPMV Licensing	100
(iv) Phased Relocation and Staff Welfare	100
Mora’s Influence and Leadership Style	100
The End of the Second Council and Transition to New Leadership.....	101
The Adenika Memorial and the Culture of Excellence.....	101
The Broader Significance of His DPS and PSN-Representative Years	103
Reflection — The Architect Before the Blueprint	104
Epilogue — The Seeds that Became Structures.....	104
<i>Chapter 4: Pharmacists Council of Nigeria (PCN) (Registrar and CEO) (2003-2012)</i>	<i>106</i>
Appointment as the Registrar and Chief Executive of PCN	115
1. Unparalleled and Diverse Professional Experience	115
2. Proven Leadership and Managerial Prowess	116
3. Deep-Rooted Commitment to Education and Profession Building.....	117
4. Pre-existing Relationship and Understanding of PCN	117
5. National and International Reputation and Network	117
Roles and Responsibilities as Registrar.....	118
1..... Administrativ e and Chief Executive Role	118
2..... Regulator y and Licensing Functions.....	119
3..... E nforcement and Compliance.....	119
4..... Educational and Professional Development.....	120

**The State of the Pharmacists Council of Nigeria (PCN) Pre-2003:
Challenges and Global Context..... 120**

1.....Regulatory
and Enforcement Capabilities 120

2.....Administrative
and Structural Deficiencies..... 121

3..... Educational
and Professional Standards 121

4.....Inter-
professional and Political Challenges 122

5..... Comparison with Global Pharmacy
Regulatory Bodies (Pre-2003)..... 122

Meetings Held 123

Key Council’s Decisions at its 15th Regular Meeting of May 20,
2004 125

Key Council’s Decisions at its 16th Regular Meeting of August 19,
2004 126

Key Council’s Decisions at its 17th Regular Meeting of December 9th,
2004 127

Key Council’s Decisions at its 18th Regular Meeting of February 17,
2005 128

Key Council’s Decisions at its 19th Regular Meeting of May 19,
2005 128

Key Council’s Decisions at its 20th Regular Meeting of August 16,
2005 128

Key Council’s Decisions at its 21st Regular Meeting of December 8,
2005 129

Key Council’s Decisions at its 22nd Regular Meeting of March 30,
2006 130

Key Council’s Decisions at its 23rd Regular Meeting of July 13,
2006 131

Key Council’s Decisions at its 24 th Regular Meeting of December 7, 2006	131
Key Council’s Decisions at its 25 th Regular Meeting of April 5, 2007	132
Key Council’s Decisions at its 26 th Regular Meeting of August 2, 2007	132
Major Achievements and Challenges.....	133
Addressing Pre-2003 Challenges: Mora's Strategic Reforms	133
1. Weak Enforcement & Proliferation of Illegal Outlets	133
2. Administrative & Structural Deficiencies	134
3. Outdated Educational Standards.....	136
4. Erosion of Professional Boundaries & Low Political Will	137
5. Ethical Re-orientation and Professional Development:	139
6. Improvement of Hospital Pharmacy Practice	139
Key Contributions to the PCN	140
Moving the PCN Forward: A Comparison with Global Agencies Post-Reforms.....	141
Concrete Actions PCN Took (2003–2012).....	142
Contributions of Pharm. A. T. Mora to the Pharmaceutical Society of Nigeria (PSN).....	144
Challenges Faced by Pharm A. T. Mora as Registrar of PCN ...	146
1. The Clash of Mandates: NAFDAC vs. PCN (2004).....	147
2. Ambiguity and Overlap in Enabling Laws.....	147
3. The Catalyst: NAFDAC's Aggressive Anti-Counterfeiting Campaign	148
4. The Immediate Trigger: The "Port Harcourt Raid"	148
5. Professional Jurisdiction vs. Public Health Mandate.....	149

6. Government Intervention and the "MoU"	149
7. The Clash with PSN	150
8. Registrar's Achievement from 2003 – 2012	150
<i>Chapter 5: Pharmacists Council of Nigeria: (Chairman of the Governing Council) (2020 - 2022).....</i>	156
Achievements and Initiatives Under the Leadership of Professor A. T. Mora	160
1. Regulatory Reforms and Policy Strengthening	160
2. Institutional Strengthening and Capacity Development.....	161
3. Digital Transformation and Process Automation	161
4. Infrastructural Development and Resource Optimization.....	161
5. Stakeholder Collaboration and Professional Engagement	162
6. Promotion of Professionalism, Ethics, and Continuing Education	162
7. Legacy and Impact	163
<i>Chapter 6: Pharmacy Council of Nigeria (PCN) (Chairman of the Governing Council (2022-2023)</i>	166
Custodian of a New Regulatory Era	169
6.1 From Transition to Transformation.....	169
6.2 Announcing the Rebirth.....	170
6.3 Strengthening Enforcement and Ethics.....	170
6.4 Consolidating Professional Reforms.....	171
6.5 Advancing Education and Practice Standards	171
6.6 Institutional Development and Resource Mobilization.....	172
6.7 Regulatory Diplomacy and Stakeholder Engagement	172
6.8 Challenges and Strategic Response.....	173
6.9 Closing a Chapter, Opening a Legacy	173
6.10 Reflection - The First Chairman.....	174

Chapter 7: Tributes.....	176
---------------------------------	------------

The list of those who sent tributes (In Alphabetical Order)

(i)	Adamu Mamman Kontagora “ Prof. Ahmed Tijjani Mora, OON, mni ”	177
(ii)	Air Commodore EK Jekada <i>Ph.D,mni</i> (Rtd) “ Prof. AhmedTijjani Mora, OON, mni ”	180
(iii)	Alh. Abdulbasir Abubakar Jargab’a “ Celebrating Prof. Ahmed Tijjani Mora @70 ”	183
(iv)	Alh. Awwalu M. Makarfi “ Prof. Ahmed T. Mora, OON, mni: A Pharmacist Par Excellence ”...	185
(v)	Alh. Sani Ibrahim Mohammed“ Prof. Ahmed TijjaniMora, OON, mni, ”	187
(vi)	Alh. Shehu Garba (<i>Dokajen Zazzau</i>) “ A Pillar of Pharmacy Regulation in Nigeria ”	189
(vii)	Alh. Surajuddeen B. Yakubu (<i>Santuraki Babba Zazzau</i>) “ A Tribute to Professor Ahmed Tijjani Mora, OON, mni, ”	191
(viii)	Alhaji (Pharm) Musibau Ade Mustapha (<i>Ph.D</i>) “ Priceless Encounter – A Tribute to a Pharmacy Regulator: Prof. A. T. Mora, OON, mni ”	194
(ix)	Dr Haruna Muhammed Jumare (<i>Maradin Zazzau</i>) “ A Visionary Leader, Distinguished Scholar, and Mentor — Celebrating Seven Decades of Excellence in Service to Humanity ”	196
(x)	Dr Ibraheem Mu’azzam Tunau “ Prof. Ahmed Tijjani Mora, OON, mni ”	199
(xi)	Dr. Abdul-Alimi Bello “ Prof. Ahmed Tijjani Mora, OON, mni ”	202
(xii)	Dr. Lawal Ladan “ A Tribute in Honor of Prof. A. T.Mora, OON, mni, Wakilin Magani Zazzau Emirate ”	203
(xiii)	Dr. Rahmatu Hassan “ Prof. Ahmed Tijjani Mora, OON, mni ”	205

(xiv)	Dr. Salamatu Hussaini “ Prof. Ahmed Tijjani Mora, OON, mni ”	
(xv)	Dr. Tajudeen Olusegun Uthman “ Prof. Ahmed Tijjani Mora, OON, mni ”	209
(xvi)	Echeazu Ogu PhD “ Prof. Ahmed Tijjani Mora, OON, mni ”	213
(xvii)	Hajiya Halima Inuwa Mora, <i>Ph.D</i> “ A Tribute to Professor A. T. Mora, OON, mni ”	215
(xviii)	Hon. Yusuf Hamisu Abubakar	217
(xix)	HRH. Alh. Ya’u Shehu Usman (<i>Sarkin Kauru</i>) “ Prof. Ahmed Tijjani Mora, OON, mni ”	220
(xx)	HRM Jonathan Phraguwa Zamuna (<i>Ukarma Pirigal</i>) “ Prof. Ahmed Tijjani Mora, OON, mni ”	222
(xxi)	Lt.Gen Abdurrahman Bello Dambazau “ Prof. Ahmed Tijjani Mora, OON, mni ”	225
(xxii)	Moqhtari Mohammed Danmallam, Malumfash I “ To My Big Brother, Prof. A.T. Mora, Who Sees The Best In Everyone ”	228
(xxiii)	Mrs. Olusola Orimoloye “ Prof. Ahmed Tijjani Mora, OON, mni ”	229
(xxiv)	Pharm Aliyu Jibrin FPSN “ Prof. Ahmed Tijjani Mora, OON, mni ”	231
(xxv)	Pharm Ibrahim Jatau Abubakar, PhD “ Prof. Ahmed Tijjani Mora, OON, mni ”	243
(xxvi)	Pharm Nuhu Isah Abdullahi (<i>Dankaden Gumel</i>) “ Prof. Ahmed T. Mora, OON, mni, FPSN: A Bosom Friend and Classmate ”	245
(xxvii)	Pharm. Ibrahim Baba-Shehu Ahmed “ Prof. A.T. Mora: Lessons from the Career of an Unmatched and Accomplished Pharmacist ”	247

(xxviii)	Pharm. Salisu Bashir Bawa “ Prof. Ahmed Tijjani Mora, OON, mni ”.....	254
(xxix)	Pharm. Yakubu Akawu “ Prof. Ahmed Tijjani Mora, OON, mni ”.....	256
(xxx)	Pharm. Yoosu Tersoo Jonathan (<i>FPCPharm</i>) “ Prof. Ahmed Tijjani Mora, OON, mni ”.....	258
(xxxi)	Prof. Ahmed Awaisu, Ph.D, “ Prof. Ahmed Tijjani Mora, OON, mni ”.....	260
(xxxiii)	Prof. Aminu Salihu Mikailu “ Foundations of Excellence: The Formative Years of Professor Ahmed Tijjani Mora During the 1960S, Tudun Wada, Zaria ”.....	263
(xxxiv)	Prof. Isa Odidi “ Prof. Ahmed Tijjani Mora, OON, mni ”.....	266
(xxxv)	Prof. Martins Emeje “ 70th Birthday Tribute to a Beloved Teacher ”.....	269
(xxxvi)	Prof. Mohammed Garba Magaji “ A Man Whose Life and Career Embody the Seamless Blend of Knowledge, Innovation, Service, and Generosity ” “ Prof. Ahmed Tijjani Mora, OON, mni ”.....	271
(xxxvii)	Professor Abdulmumini Ibrahim “ Dedicated Mentor who Ensures his Mentees Succeed ”.....	174
(xxxviii)	Professor Charles Okechukwu Esimone: “ Professor Ahmed T. Mora is a Renowned Authority in Pharmaceutical Practice, not only in Nigeria but also Across the West African Sub-Region. ”.....	281
(xxxix)	Professor Ibrahim M. Jumare “ A. T. Mora: An Icon of Scholarship, Administration, and Leadership ”..	283

(xl)	Rabi Abdallah (Health Broadcast Journalist) “ A Tribute a Pillar of Pharmacy Education and Regulation”...	285
(xli)	Salahudeen Na'ibi Sulaiman Wali “ A Pillar of Integrity, Vision, and Unwavering Purpose”.....	289
(xlii)	Steve Ozioko “ Prof. Ahmed Tijjani Mora, OON, mnr ”.....	290
(xliii)	The Family of <i>Sardauna</i> of Sokoto, Sir Alh. Abubakar Alhaji KBE “ Tribute from Sardaunan Sokoto ”...	291

Chapter 8: Some of the Papers Presented by Professor Ahmed Tijjani Mora at Conferences and Seminars (1988-date)..... 293

The List of the Papers Presented:

(i)	Mandatory Continuing Professional Development (MCPD): Successes and Challenges. A Lecture by Pharm. Ahmed T. Mora, at the One-Day MCPD Train-the-Trainer Workshop Fred Adenika Auditorium Hall, PCN, Idu, Abuja, Wednesday, July 27, 2005.....	295
(ii)	Pharmacists Council of Nigeria (PCN), PCN Standards for Monitoring Registered Premises. A Paper by Pharm. Ahmed T. Mora, FPSN, FPCPharm, Registrar and Secretary to the Council, Pharmacists Council of Nigeria (PCN), Presented at the National Pharmaceutical Inspectors Workshop. Organized by the Pharmacists Council of Nigeria, Merit House, Abuja, Tuesday 2nd – Wednesday 3rd August, 2005.....	300
(iii)	Presentation by the Registrar, Pharmacists Council of Nigeria (PCN), Pharm. Ahmed T. Mora, <i>FPSN, FPCPharm</i> , on the Laws and Regulations Governing Pharmacy Practice in Nigeria and Reciprocity. At the First Annual Convention of the Nigerian Association of Pharmacists and Pharmaceutical Scientists in the Americas (NAPPSA), Houston, Texas, USA, September 14–16, 2007.....	300
(iv)	Presentation by the Registrar, Pharmacists Council of Nigeria (PCN), Pharm. Ahmed T. Mora, <i>FPSN, FPCPharm</i> , on the Laws and Regulations Governing Pharmacy Practice in Nigeria and Reciprocity. At the First Annual Convention of the Nigerian	

	Association of Pharmacists and Pharmaceutical Scientists in the Americas (NAPPSA), Houston, Texas, USA, September 14–16, 07.....	303
(v)	ext of Paper Presented by Pharm. Ahmed T. Mora, <i>FPSN</i> , <i>FPCPharm</i> . The registrar, Pharmacists Council of Nigeria (PCN) on the Occasion of the Formal Inauguration of the Drug Distribution Inspection Committee (DDIC) of the PCN at the Fred Adenika Auditorium on Wednesday, July 02, 2008.....	326
(vi)	The Nigerian Pharmacist, Medication Errors and the Regulation and Control of Pharmacy Practice by the Pharmacists Council of Nigeria (PCN). Delivered as the Registrar and Chief Executive Officer (CEO) of the PCN at the Second Annual Convention of the Nigerian Association of Pharmacists and Pharmaceutical Scientists in the Americas (NAPPSA), held at Atlanta, Georgia, USA, from September 18–20, 2008.....	334
(vii)	Inter-Country Pharmacy Practice: The Responsibility of National Pharmacy Councils. Presented by Pharm. Ahmed T. Mora, Registrar and CEO of the Pharmacists Council of Nigeria (PCN), at a Workshop on the Harmonisation of Pharmacy Curriculum Across the West African Sub-Region, organised by the West African Health Organisation (WAHO), at the West African Health Community Secretariat, Yaba, Lagos, on October 23, 2008...	339
(viii)	Speech of the Registrar, Pharmacists Council of Nigeria (PCN), Pharm. Ahmed T. Mora, <i>FPSN</i> , <i>FPCPharm</i> , ON VISION 20-2020. The Pharmacist as an Indispensable Professional in Healthcare Delivery, July 07, 2009 ...	343
(ix)	Welcome Address, By Pharm. Ahmed T. Mora, <i>FPSN</i> , <i>FPCPharm</i> , Registrar/CEO, Pharmacists Council of Nigeria (PCN). At the Opening Ceremony of a Two-Day	

	Workshop for Superintendent and Production Pharmacists of Pharmaceutical Manufacturing Industries in Nigeria (Lagos & South-West States). Theme: “Achieving Excellence in Current Good Manufacturing Practice (cGMP) for the Assurance of Quality Pharmaceutical Products in Nigeria”, Venue: All Seasons Multipurpose Plaza, Agidingbi, Ikeja, Lagos. Date: Tuesday, July 12, 2009.....	347
(x)	Regulatory Challenges Facing Pharmacy Practice in Nigeria. Delivered as Registrar and CEO of the PCN at the Third Annual Convention of NAPPSA, held in Bethesda, Maryland, USA, from September 17–20, 2009...	349
(xi)	Remarks by the Chairman of the Committee of Registrars of Health Professions (CRHP). Presented by Pharm. Ahmed Tijjani Mora at the Retreat of Members of the CRHP held at the Yankari Game Reserve, Bauchi State, from January 18–19, 2011.....	363
(xii)	The Health Regulatory Agencies as Instruments for Professional Harmony in Nigeria. Presented by Pharm. Ahmed Tijjani Mora at a Workshop organised by the Committee of Registrars of Health Professions (CRHP) at the Sheraton Hotel, Abuja, on Thursday, March 31, 2011.....	366
(xiii)	An Overview of the Regulatory Functions of the Pharmacists Council of Nigeria: Inspectorate Activities. Presented by Pharm. Ahmed T. Mora, Registrar and CEO of the PCN, at the Pharmaceutical Inspectors’ Workshop organised by the PCN at the Raw Materials Research and Development Agency, Abuja, from July 5–6, 2011.....	373
(xiv)	A Review of the Guidelines and Code of Ethics for Pharmaceutical Inspectors. Presented by Pharm. Ahmed T. Mora, <i>mni</i> , Registrar and CEO of the PCN, at the Pharmaceutical Inspectors’ Workshop organised by the PCN at the Raw Materials Research and Development Council, Abuja, from July 5–6, 2011	384
(xv)	Innovative Trends in Pharmacy Practice in Nigeria: A Regulatory Perspective. Presented as Registrar and CEO of the PCN at the 2011 Annual Scientific Conference and Exposition of NAPPSA, held at the	

	Sheraton Gateway Hotel, Los Angeles, USA, on September 5, 2011.....	388
(xvi)	The Challenges of Health Regulation in Nigeria: The Big Picture. A Paper by Dr. Ahmed T. Mora, <i>Ph.D., FPSN, FPCPharm, mni</i> , Dean, Faculty of Pharmaceutical Sciences, Kaduna State University (KASU), Kaduna, at National Conference on Healthcare Management/ Membership Induction Hosted by the Institute of Healthcare Management, Nigeria Agura Hotel, Abuja, November 30, 2012.....	402
(xvii)	A Workshop to Develop Curriculum-Based Accreditation Criteria for Pharmacy Technicians' Training Programmes in the ECOWAS Region. Presented by Dr. Ahmed T. Mora at a Workshop organised by the West African Health Organisation (WAHO), Bobo-Dioulasso, Burkina Faso, from July 2–3, 2013.....	413
(xviii)	Ensuring Access to Registered Pharmaceutical Products in Licensed Pharmaceutical Premises: The Role of Regulatory Agencies. Presented by Dr. Ahmed Tijjani Mora, <i>mni</i> , Dean, Faculty of Pharmaceutical Sciences, Kaduna State University (KASU), at the First International Conference of the Nanomedicine Association of Nigeria, held at the African University of Science and Technology (AUST), Galadimawa, Abuja, on November 27, 2013.....	421
(xix)	The Challenges of Health Regulation in Nigeria: The Big Picture. Published by the Institute of Healthcare Management, Nigeria, by Dr. Ahmed T. Mora, in December 2015.....	428
(xx)	Interfacing Practitioners with Students to Achieve an Apex Healthcare Learning Environment. Presented by Dr. Ahmed Tijjani Mora, Dean, Faculty of Pharmaceutical Sciences, KASU, at a Workshop for Staff of Barau Dikko Teaching Hospital (a Tertiary Healthcare Provider) held at Hotel Seventeen, Lafia Road, Kaduna, on Wednesday, April 13, 2016.....	444

(xxi)	he Role of Health Regulatory Agencies in Enhancing Effective Healthcare Delivery Services in Nigeria. Presented by Dr. Ahmed Tijjani Mora on the occasion of the 2016 Joint Scientific Conference and Exposition of NAPPSA and ANPA, held in Las Vegas, Nevada, USA, from June 23–26, 2016.....	456
(xxii)	Text of Remarks on the Opening Ceremony of a Sensitisation Workshop to Kick-Start the Implementation of the Pharmacist Consultant Cadre within the Healthcare System in Nigeria. Presented by Prof. Ahmed Tijjani Mora at the Banquet Hall, NAF Conference Centre and Suites, Abuja, on May 18, 2021.....	483
(xxiii)	The Implementation of the Regulatory Functions of the Pharmacists Council of Nigeria (PCN) in Respect of Education and Continuing Professional Development (CPD) Programmes. Presented by Prof. Ahmed Tijjani Mora, Chairman, Pharmacists Council of Nigeria (PCN), at the 2021 Scientific Conference and Exposition of NAPPSA, held in Baltimore, Maryland, USA, from September 23–26, 2021.....	487
(xxiv)	Policy Reforms and Regulatory Challenges in Nigeria’s Pharmaceutical Sector. Presented by Prof. Ahmed Tijjani Mora as the Keynote Speaker at the Pharmacy Week (Academic Pharmacy Summit) organised by the Pharmaceutical Association of Nigerian Students (PANS), Sa’adu Zungur University, Gadau, Bauchi State, on Friday, July 25, 2025.....	495

Appendices

<i>S/NO</i>	<i>APENDIX</i>	<i>TITLE</i>	<i>PAGE</i>
1.	<i>I</i>	<i>The Members of BOBA (1970 intake) Class Group and their contributions to the Development of Tertiary Education in Nigeria.</i>	280
2.	<i>II</i>	<i>The Various Pharmacy Regulatory Bodies that Prof. Ahmed Tijjani Mora a Member/Appointed into and his carier ascension in the profession.</i>	282
3.	<i>III</i>	<i>1. Pharmacists Council of Nigeria</i> <i>2. Alhaji Ahmed Tijjani Mora</i> <i>Vs.</i> <i>1. Pharmaceutical Society of Nigeria</i> <i>2. National Council, Pharmaceutical Society of Nigeria</i> <i>Court of Appeal, Lagos Division.</i>	
4.	<i>IV</i>	<i>ICPC: Petition Against Alhaji Tijjani Mora, Registrar and Secretary of Pharmacists Council of Nigeria (PCN) for Financial Misappropriation Malfeasance and Fraud.</i>	
	<i>V</i>	<i>Invitation to the Inauguration of the PCN Governing Council and the Inaugural Meeting of the Council.</i>	
	<i>VI</i>	<i>Pharmacists Reject Mora as PCN Chairman, Threaten Nationwide Strike (Thisday Newspaper)</i>	
	<i>VII</i>	<i>FG Shuns Pharmacists, Inaugurates PCN Council: Speech by the Honourable Minister of Health, Dr. Osagie Ehanire during the Inauguration of the PCN Council with Prof. Mora as Chairman.</i>	

	<i>VIII</i>	<i>Response by Prof. Ahmed Tijjani Mora, mni, after the Hon. Minister of Health, Dr. Osagie Ehanire has Inaugurated the Governing Council of the Pharmacists Council of Nigeria (PCN) on 15th June, 2020.</i>	
	<i>IX</i>	<i>Commendation Letters from the Leadership of the PSN</i>	
	<i>X</i>	<i>Prof. Ahmed Tijjani Mora Appreciation to Nigerian Pharmacists for support.</i>	

About the Authors554

I. Pharm Aliyu Jibrin

II. Prof. Ibrahim Adamu Yakasai

III. Prof. Umar Usman Pateh

IV. Pharm. Ahmed Ibrahim Umar

V. Prof. Kabiru Abubakar Gulma

VI. Pharm. Ibrahim Muhammad Amidu

Bibliography

Acronyms

A

- **ABU** – Ahmadu Bello University
- **ABUTH** – Ahmadu Bello University Teaching Hospital
- **AFDB** – African Development Bank
- **AFDF** – African Development Fund
- **A-Levels** – Advanced Level (qualification)
- **ASCON** – Administrative Staff College of Nigeria
- **ATM** – Ahmed Tijjani Mora (used as initials)

B

- **B.Sc.** – Bachelor of Science
- **BoT** – Board of Trustees

C

- CAANU** – Conference of the Alumni Associations of Nigerian Universities
- **CEO** – Chief Executive Officer
- **CFR** – Commander of the Federal Republic

D

- **DPS** – Director of Pharmaceutical Services

E

- **EdT** – Education and Training (PCN category)

F

G

- **GCON** – Grand Commander of the Order of the Niger
- **GmbH** – Gesellschaft mit beschränkter Haftung (German for “Company with Limited Liability”)

H

- **H.M.B. / HMB** – Health Management Board
- **H.E.** – His Excellency / Her Excellency
- **HH** – His Highness
- **HRM** – His/Her Royal Majesty

I

- **ISSN** – International Standard Serial Number
- **ISBN** – International Standard Book Number
- **IUO** – Igbinedion University, Okada

K

- **KASU** – Kaduna State University
- **KHRP** – Kaduna Health Rehabilitation Project
- **KSDA** – Kaduna State Distribution Agency

L

- **LEA** – Local Education Authority
- **Ltd.** – Limited

M

- **MBA** – Master of Business Administration
- **Mni / mni** – Member of the National Institute
- **MoH** – Ministry of Health
- **MoR** – Monitoring and Regulation (PCN category)
- **MD** – Managing Director
- **MIT** – Massachusetts Institute of Technology

N

- **NAPPSA** – Nigerian Association of Pharmacists and Pharmaceutical Scientists in the Americas
- **NEC** – National Executive Committee
- **NPA** – Nigerian Ports Authority
- **NYSC** – National Youth Service Corps
- **NIPSS** – National Institute for Policy and Strategic Studies
- **NPA** – Nigerian Ports Authority
- **NTI** – National Teachers' Institute

O

- **OON** – Officer of the Order of the Niger

P

- **PANS** – Pharmaceutical Association of Nigerian Students
- **PCN** – Pharmacy Council of Nigeria / Pharmacists Council of Nigeria
- **PCN. Rec** – PCN Records (Publication Category)
- **PCN. MoR** – PCN Monitoring & Regulation Publications
- **PCN. EdT** – PCN Education & Training Publications
- **Ph.D.** – Doctor of Philosophy
- **PIU** – Project Implementation Unit
- **PBN** – Pharmacists Board of Nigeria
- **PIC** – Pharmaceutical Inspection Committee
- **Plc** – Public Limited Company

S

- **SBS** – School of Basic Studies
- **SGF** – Secretary to the Government of the Federation

U

- **UDUS** – Usmanu Danfodiyo University, Sokoto
- **UN** – United Nations
- **USA** – United States of America

W

- **WAPCP** – West African Postgraduate College of Pharmacists

Z

- **ZPIL** – Zazzau Pharmaceutical Industries Ltd
- **ZPC** – Zaria Pharmaceutical Company

Foreword

I am glad to see that Prof. Ahmed Tijjani Mora has a team of trusted people, who found the time to put together his achievements into a book. Memoirs are a vital part of historiography; not only do they capture and preserve history, but they also form a repository of experiences and lessons that future generations will benefit from. Professionals and public servants who have acquired experience over time will serve their society best by preserving and presenting it in ways that will serve posterity. I congratulate him on this achievement.

In addition to holding the title of *Wakilin Magani of Zazzau*, Prof. Ahmed Mora is also a holder of the title of the *K'ayaye* of the Sultan in the Sokoto Sultanate Council. During our college days at Barewa, Ahmed was diligent, hardworking and responsible. It was not surprising that he excelled in his chosen career as a pharmacist. He not only rose to the enviable position of Professor of Clinical Pharmacy but also became a Registrar of the Pharmacists Council of Nigeria (PCN), a regulator of pharmacy education, training and practice. He has had a distinguished career in pharmacy spanning over forty years during which he has pursued advanced knowledge in pharmacy education and practice, as well as disseminated scientific research findings. He also presented pharmacy position papers to national and global audiences at conferences, seminars and workshops.

I hope this is only the beginning. Because now that he holds a title in Sokoto, he needs to turn his attention to the writings on medicine and pharmacy by the leading scholars of Sokoto. As it is becoming increasingly clear, Shehu Usman dan Fodio, his brother, Shehu Abdullahi, and Sultan Muhammad Bello wrote books on various

aspects of medicine. Those who came immediately after them had maintained the tradition. These writings need to be taught in the history of pharmacy in our universities, especially those in the domain of the caliphate. I also understand that one of the Emirs of Zazzau, Aliyu dan Sidi, has made an ingenious commentary on a compendium of pharmaceutical preparations done by a Turkish Scholar. I hope Prof. Ahmed Tijani Mora will devote time to unearth this manuscript and subject it to pharmaceutical analysis for the benefit of posterity.

I therefore strongly recommend this book to pharmacy students and practitioners, members of the wider healthcare professions and the general public, who seek to broaden their knowledge horizons and learn from his rich experiences.

His Eminence Alhaji Muhammad Sa'ad Abubakar, CFR, mni
The Sultan of Sokoto
August 2025

Preface

The book, **Ahmed Tijjani Mora: A Pillar of Pharmacy Regulation in Nigeria** chronicles the remarkable life and career of Professor Ahmed Tijjani Mora, *OON, mni*, a distinguished Nigerian pharmacist, who has served his country with excellence across all the four major technical areas of the pharmacy profession—academic, community, hospital and industrial practice—before ultimately rising to the pinnacle as regulator of the profession itself.

Spanning a period of thirty-seven years (1986–2023), Professor Mora contributed immensely to Nigeria’s pharmaceutical and healthcare landscape. He excelled in every sphere of practice, attaining the highest positions in each: Professor and Foundation Dean of the Faculty of Pharmaceutical Sciences, Kaduna State University; Pharmaceutical Divisional Manager, Kaduna State Distribution Agency; Director of Pharmaceutical Services, Ministry of Health, Kaduna State; and Managing Director/CEO of the Kaduna State-owned Zaria Pharmaceutical Company (manufacturers of *ZARINJECT* syringes and needles) as well as Zazzau Pharmaceutical Industries Ltd. (The manufacturers of solid and liquid dosage forms). Perhaps no Nigerian pharmacist has attained such breadth and depth of professional achievement to date. From practitioner to regulator, Professor Mora as distinguished himself in shaping both pharmacy practice and the regulatory framework of the profession.

This book documents his services in the now-defunct Pharmacists Board of Nigeria (PBN), the former Pharmacists Council of Nigeria (PCN), and the present Pharmacy Council of Nigeria over a cumulative period of nineteen years. His contributions spanned roles as Board Member, Registrar/CEO and ultimately the first Chairman of the Governing Council of the Pharmacy Council of Nigeria—a position to which he was appointed by President Muhammadu Buhari, GCFR,

serving from June 2020 to June 2023. Notably, he remains the only Nigerian pharmacist to have served both as Registrar/CEO and later Chairman of the PCN.

Professor Mora's professional journey was made possible through opportunities provided by the Kaduna State Government in institutions, such as the Kaduna State Distribution Agency Ltd. (KSDA Ltd), the Health Management Board (HMB), the Ministry of Health, the Zaria Pharmaceutical Company Ltd. (ZPC) and Kaduna State University (KASU), where he consistently demonstrated exceptional competence, leadership and innovation. At the national level, the PBN, the Pharmacists Council and the Pharmacy Council provided platforms where he applied his expertise in regulatory affairs to strengthen the profession.

His academic engagements were equally impressive, spanning three universities, representing the federal, state and private sectors: Ahmadu Bello University (ABU), Zaria; Kaduna State University (KASU) and Igbinedion University, Okada (IUO). Collectively, he dedicated over fifty-one years to teaching and research in pharmaceutical sciences. He currently serves as the Pro-Chancellor and Chairman of the Governing Council of IUO.

The essence of this book is to showcase how, through resilience and perseverance, one Nigerian pharmacist who elevated pharmacy to a position of prominence among healthcare regulatory agencies in the country. A testament to his leadership was his unprecedented service as the Chairman of the Committee of Registrars of Health Professions (CRHP) for six years (2006–2012) during his tenure as Registrar and CEO of the Pharmacists Council of Nigeria.

The book opens with a Foreword written by His Eminence, Alhaji Muhammad Sa'ad Abubakar, CFR, mni, the Sultan of Sokoto and President-General of the Nigerian Supreme Council of Islamic Affairs (NSCIA). The association between His Eminence and Professor Mora

dates back to 1970 in Barewa College, Zaria. Over the past fifty-five years, a strong bond has been established between them. In 2021, the Sultan appointed Professor Mora as ***K'ayayen Sarkin Musulmi (Chief of the Sultan)*** within the 220-year-old Sokoto Caliphate. Earlier in 2013, he was turbaned as ***Wakilin Maganin Zazzau*** by the Zazzau Emirate Council in recognition of his dynamic contributions as a pharmacist and son of the Emirate.

The book is organized into eight chapter and represents:

- A background to Professor Mora's upbringing, education and professional development followed by a detailed account of his engagements with pharmacy's regulatory institutions. It highlights his roles as Board Member, Registrar/CEO and Chairman of the Governing Council along with key regulatory milestones and institutional reforms achieved during his service.
- Tributes from teachers, mentors, colleagues, friends, associates, students and family members. These reflections illuminate his professional legacy as well as his personal values and social impact.
- Compilation of some of his scholarly and professional contributions at national and international conference papers and presentations from 1988 to date. To enrich its utility as a reference text, the section reproduces fifteen selected papers on pharmacy regulation.

There is also a list of Appendices referred to in the eight (8) chapters of the book.

The book concludes with a comprehensive index, making it a practical resource for scholars, regulators, students and practitioners, who wish to understand the evolution of pharmacy regulation in Nigeria through the lens of one of its most accomplished flag-bearers.

Pharm. Aliyu Jibrin

Chairman, Prof. Ahmed Tijjani Mora Book Project Committee.

January, 2026

Acknowledgments

The Book Committee on *Professor Ahmed Tijjani Mora: A Pillar of Pharmacy Regulation in Nigeria* expresses its profound gratitude to **Professor Ahmed Tijjani Mora**, whose vision, humility and generosity of spirit made this project possible. His willingness to entrust the Committee with the sacred task of documenting his life's journey as a distinguished pharmacy regulator, educator and leader is a mark of deep confidence. Beyond sharing invaluable materials, documents and personal reflections, Professor Mora graciously opened doors to those whose voices and recollections have enriched these pages with authenticity and warmth.

Our heartfelt appreciation goes to **Pharm. Aliyu Jibrin**, the indefatigable Chairman of the Book Committee, whose commitment, tact and leadership have been the compass guiding this project from its conception to completion. His meticulous coordination and steady oversight have ensured that every part of this monumental work found its rightful place.

Special appreciation goes to **Professor Umar Usman Pateh**, whose scholarly discipline and organizational flair were instrumental in coordinating the collation of tributes—each one a heartfelt testament to Professor Mora's towering influence across generations. We also acknowledge **Professor Ibrahim Adamu Yakasai**, whose insights and contributions added depth to the narrative and strengthened the intellectual tone of the book.

We warmly recognize **Pharm. Ahmed Ibrahim Umar**, whose full engagement in the writing process, consistent participation in all Committee meetings and unwavering commitment to the project significantly enriched the quality and coherence of this work. His

intellectual contributions and collaborative spirit were evident at every stage of the book's development.

Our colleague, **Professor Kabiru Abubakar Gulma**, deserves equal commendation for his thoughtful coordination of the compilation and harmonization of the various chapters. His editorial guidance and literary finesse helped to weave the different contributions into a single, coherent story befitting the subject's stature.

With solemn hearts, we pay tribute to the **late Pharm. Haruna Aliyu**, the pioneer Chairman of this Book Committee, whose early leadership laid the foundation for all that followed. Though he is no longer with us, his dedication and passion for this cause remain etched in the spirit of this book. His memory lives on in every page.

We are deeply honored and profoundly grateful to **His Eminence, Alhaji Muhammad Sa'ad Abubakar III, the Sultan of Sokoto**, for graciously penning the Foreword to this volume. His words not only lend prestige to this work but also serve as a national affirmation of Professor Mora's legacy of service and leadership.

Finally, our gratitude extends to **every individual whose name may not have been mentioned here**—friends, colleagues, mentees and associates, who contributed their voices, memories and tributes. Your reflections have given this book its heart and humanity.

To everyone who played a part, in ways large or small, we say *thank you*. This work stands as a collective labor of respect, affection and remembrance.

Pharm. Ibrahim Mohammed Amidu
Secretary, Prof. Ahmed Tijjani Mora Book Project Committee
January, 2026

Chapter 1

Environment and Brief on Professor Ahmed Tijjani Mora

The Origins and Early Development of Ancient Zaria

Zaria, one of the original Hausa city-states (known as the **Hausa Bakwai**), traces its origins to the 11th and 12th centuries. It began as the kingdom of **Zazzau** inhabited mainly by the Hausa people. According to tradition, Zazzau was ruled by the descendants of **Bayajidda**, the legendary hero credited with founding the Hausa states.

The Name “Zaria”

Originally called **Zazzau**, the city was later renamed **Zaria** in honor of **Queen Zaria (Zariya)**, one of the famous daughters of **Queen Bakwa Turunku**, who founded the town around 1536. Queen Zaria’s reign marked a period of expansion and consolidation.

The Era of Powerful Queens

Zazzau holds a unique place in Hausa history for its succession of female rulers. The most notable among them was:

- **Queen Amina (Amina of Zazzau)** — who reigned in the late 16th century. She was renowned for her military conquests and the expansion of Zazzau’s territory as far as Nupe and the Middle Niger. She fortified cities with defensive walls (known as “*Ganuwar Amina*”) and strengthened trade routes.
- Her reign is remembered as a golden age of military might, political influence and commercial prosperity.

Economic and Cultural Importance

Zazzau flourished as a hub of the **trans-Saharan trade**, dealing in commodities, such as kola-nuts, leather, horses and textiles. Its strategic location made it a key meeting point between northern traders from the Sahara and southern forest kingdoms.

The Fulani Jihad and the Sokoto Caliphate Era

In the early 19th century, the **Fulani Jihad** led by **Usman dan Fodio** brought Zazzau under the control of the **Sokoto Caliphate** (around 1804). It was reorganized into an emirate ruled by Fulani Emirs, yet it retained its importance as a cultural, religious and commercial center. Since 1804, nineteen Emirs have ruled Zazzau (see below)

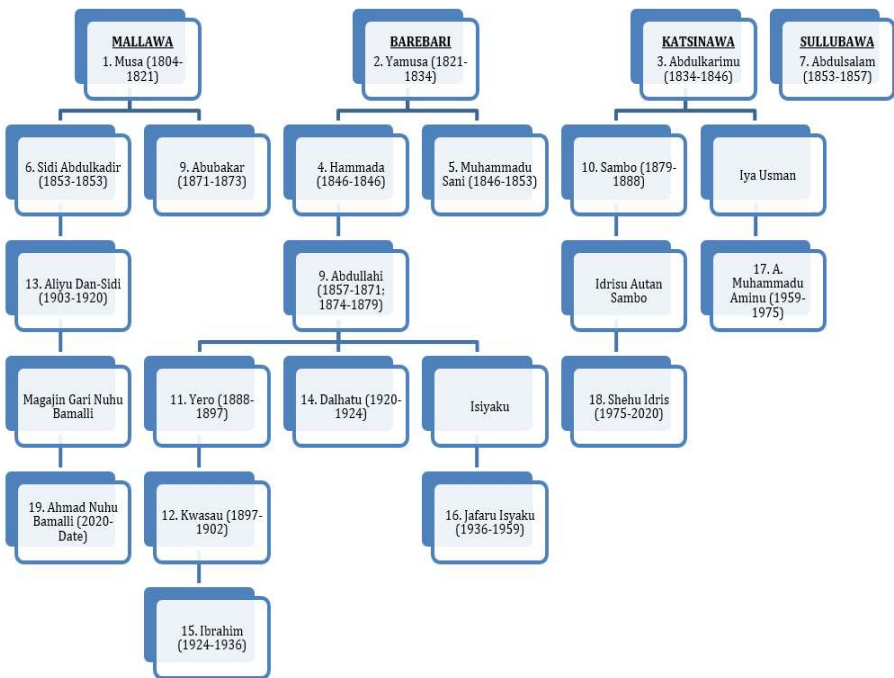


Figure 1: The Emirs of Zazzau and their Ruling Houses

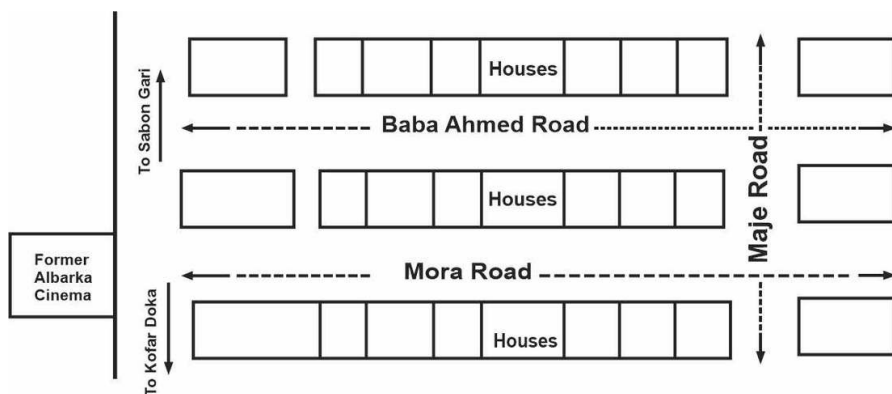
Colonial and Modern Periods

By the late 19th century, British colonial forces had incorporated Zaria into the Protectorate of Northern Nigeria (1903). Under Indirect Rule, the emirate system was preserved, with the Emir of Zaria serving as a traditional leader.

Today, Zaria is part of Kaduna State and home to major institutions, most notably Ahmadu Bello University (ABU), one of Africa’s leading centers of higher learning.

Tudun Wada, Zaria

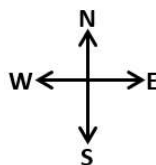
The rectangular area is characterized by a high concentration of political, civil servants, public servants, professionals, military personnel, paramilitary personnel, CEOs and their family homes.



The above area within Tudun Wada, Zaria is renowned for having one of the highest concentration of parents and their children who have served the nation in significant capacities—within Tudun Wada, across Zaria and Kaduna State, throughout Northern Nigeria and in some cases, at the national level.

This area is defined as follows:

- **North** – *Baba Ahmed Road*
- **South** – *Mora Road*
- **East** – *Maje Road*
- **West** – The major road linking *Sabon Gari* (to the north) and *Kofar Doka* (to the south)



As shown in the diagram above, the enclave includes houses on both sides of Baba Ahmed Road and Mora Road, which are the family houses for the distinguished sons and daughters of the area of Zaria LGA, as the case may be. From Nigeria's Independence in 1960 to the present day, the distinguished personalities and their children, who hail from these two roads, have made lasting contributions, positively impacting the lives of countless Nigerians through their service.

S/N	DISCIPLINE/ APPOINTMENT	NAME OF PERSONALITY	POSITION HELD	FAMILY HOUSE	HOUSE WITHIN THE “RECTANG LE”
1	ELECTIVE POLITICAL OFFICES	Arc. Muhammad Namadi Sambo <i>GCON</i>	(a) Vice President of Nigeria (2010 – 2015) (b) Governor of Kaduna State (2007-2010)	Alh. Sambo	Baba Ahmed Road
		Hon. Yusuf Datti Baba Ahmed	(a) Member House of Representatives (Zaria Constituency) (2003 – 2007)	Alh. Baba Ahmed	Baba Ahmed Road
			(b) Distinguished Senator (Kaduna North Senatorial District) (2011- 2012)		
			(c) Contested for the position of Vice-President (Labour Party) (2023)		
		Hon. Mahmud Sani Sha’aban	(a) Member House of Representatives (Sabon Gari Constituency) (2003 – 2007) (b) Contested for the position of Governor of Kaduna State, a number of times	Baba Sha’aban	Mora Road
2	APPOINTED POLITICAL OFFICER HOLDERS (MILITARY REGIMES)	(1) Arc. Muhammad Namadi Sambo	Honourable Commissioner, Ministry of Works, Kaduna State (1987- 1991).	Alh. Sambo	Baba Ahmed
		(2) Alh. Usman Khalifa Baba Ahmed	Honourable Commissioner, Ministry of Information and	Alh. Baba Ahmed	Baba Ahmed

			Culture, Kaduna State.		
		(3) Dr. Hakeem Baba Ahmed	Secretary to the Kaduna State Government (SSG), (1996-1998)	Alh. Baba Ahmed	Baba Ahmed
		(4) Col. Garba Ahmed	Member of the Plateau State Executive Council as a military officer posted to Plateau State (1997-1999)	Alh. Ahmadu Mohammed	Mora Road
		(5) Alh. Abdussalam Baba Ahmed	Honourable Commissioner, Kaduna State Ministry of Lands and Survey (2013-2015) (Civilian Regime)	Alh. Baba Ahmed	Baba Ahmed
3	FEDERAL CIVIL SERVICE	(1) Dr. Abdurrahman Mora	(i) Federal Permanent Secretary, Ministry of Lagos Affairs (1962-1966)	Dr. Abdurrahman Mora	Mora Road
			(ii) Federal Permanent Secretary, Ministry of Internal Affairs (1966-1967)	Dr. Abdurrahman Mora	Mora Road
		(2) Dr. Hakeem Baba Ahmed	(i) Federal Permanent Secretary, Office of the Head of Civil Service of the Federation.	Alh. Baba Ahmed	Baba Ahmed
			(ii) Federal Permanent Secretary, INEC	Alh. Baba Ahmed	Baba Ahmed
			(iii) Federal Permanent Secretary, Ministry of Foreign Affairs	Alh. Baba Ahmed	Baba Ahmed

4	CHAIRMANSHIP/ CEO OF FEDERAL PARASTATALS	(1) Alh. Muhammadu Inuwa Mora	Chairman, Governing Council of Yaba College of Technology (1984-1993)	Dr. Abdurahman Mora	Mora Road
		(2) Prof. Ahmed Tijjani Mora	(i) Chairman, Governing Council of the Pharmacists/ Pharmacy Council of Nigeria (PCN), (2020-2023)	Dr. Abdurahman Mora	Mora Road
			(ii) Registrar and CEO, Pharmacists of Council of Nigeria (PCN), (2003-2012)	Dr. Abdurahman Mora	Mora Road
		(3) Alh. Mouftah Baba Ahmed	Executive Director, Power Holding Company of Nigeria (PHCN)	Alh. Baba Ahmed	Baba Ahmed
5	CHAIRMANSHIP/ CEO OF STATE PARASTATALS /NGOs	(1) Dr. Abdurrahman Mora	(i) Chairman, Governing Council of College of Advanced Studies (CAS) (now Nuhu Bamalli Polytechnic), Zaria (1983-1987)	Dr. Abdurrahman Mora	Mora Road
			(ii) Chairman, Board of Trustees (BoT) of the Zaria Education Development Association (ZEDA) (1992-1997)	Dr. Abdurrahman Mora	Mora Road
		(2) Alh. Muhammadu Inuwa Mora	(i) Government Printer, Government Printing Press, Kaduna (1974-1979)	Dr. Abdurahman Mora	Mora Road
			(ii) Chairman and MD/CEO, Gaskiya	Dr. Abdurahman Mora	Mora Road

			Corporation Ltd, Zaria (1979-1981)		
			(iii) Chairman, Northern Nigeria Publishing Company (NNPC), Zaria, (1979-1981)	Dr. Abdurahman Mora	Mora Road
		(3) Prof. Ahmed Ahmed Tijjani Mora	(i) Project Manager (PM), Project Implementation Unit (PIU), at the AfDB/AfDF-Assisted Kaduna Health Rehabilitation Project (KHRP), MOH Headquarters, Kaduna (1992-1998)	Dr. Abdurahman Mora	Mora Road
			(ii) Managing Director and CEO, Zaria Pharmaceutical Company (ZPC) Ltd, Zaria (1997-2000)	Dr. Abdurahman Mora	Mora Road
		(4) Alh. Mahmoon Baba Ahmed	(i) GM/CEO, Kano Radio, Kano (1979-1983)	Alh. Baba Ahmed	Baba Ahmed
			(ii) MD and CEO, Kaduna State Media Corporation (KSMC)	Alh. Baba Ahmed	Baba Ahmed
6	MILITARY AND AVIATION	(1) Capt. Sabo Sambo	Airline Captain	Alh. Sambo	Baba Ahmed
		(2) Colonel Garba Ahmed	Military Officer, Nigerian Army	Alh. Ahmadu Muhammad	Mora Road
		(3) Bello Shaban	Nigerian Air Force (NAF)	Baba Shaban	Mora Road
		(4) Sule Shaban	Nigerian Police Force (NPF)	Baba Shaban	Mora Road
7	HOLDERS OF Ph.D DEGREES	(1) Dr. Hakeem B. Ahmed	Ph.D	Alh. Baba Ahmed	Baba Ahmed

		(2) Dr. Yusuf Datti B. Ahmed	Ph.D	Alh. Baba Ahmed	Baba Ahmed
		(3) Dr. Ahmed T. Mora	Ph.D	Dr. Abdurrahman Mora	Mora Road
		(4) Dr. Hadiza Ahmed	Ph.D	Alh. Ahmadu Muhammad	Mora Road
8	PROFESSORS	(1) Prof. Ahmed T. Mora	Prof. of Pharmacy	Dr. Abdurrahman Mora	Mora Road
		(2) Prof. Hadiza Ahmed	Prof. of Education	Alh. Ahmadu Muhammad	Mora Road
9	AMBASSADORS/HIGH COMMISSIONERS	(i) Dr. Abdurrahman Mora	Nigeria's High Commissioner to the Islamic Republic of Pakistan (1967-1971)	Dr. Abdurrahman Mora	Mora Road
10	PROFESSIONALS	(1) Arc. Muhammad Namadi Sambo	Architecture (ABU)	Alh. Sambo	Baba Ahmed
		(2) Barr. Nafi'u B. Ahmed	Law (ABU)	Alh. Baba Ahmed	Baba Ahmed
		(3) Dr. Muhammad Saifuddeen Mora	Medicine (ABU)	Dr. Abdurrahman Mora	Mora Road
		(4) Pharm. Ahmed T. Mora	Pharmacy (ABU)	Dr. Abdurrahman Mora	Mora Road
		(5) Alh. Abbas A. Mora	Accounting (ANAN)	Dr. Abdulrahman Mora	Mora Road
		(6) RN Aishatu Ahmed	Nursing	Alh. Ahmadu Mohammed	Mora Road
		(7) Dr. Saratu Mora	Medicine (UDUS)	Dr. Abdurrahman Mora	Mora Road
10	ENTREPRENEURSHIP	(1) Alh. Ahmadu Mohammad	Idon Ganin Alheri Ltd	Ahmadu Mohammad	Mora Road
		(2) Arc. Muahmmad Namadi Sambo	Founder, NALADO Ltd and other Companies	Alh. Sambo	Baba Ahmed
		(3) Alh. Usman Khalifa Baba Ahmed	Founder, DITV Television, Kaduna	Alh. Baba Ahmed	Baba Ahmed

Environment and Brief on Professor Ahmed Tijjani Mora

		(4) Dr. Muhammad Saifuddeen Mora	Co-Founder and Registered Zaria Clinic, Tudun Wada, Zaria	Dr. Abdurrahman Mora	Mora Road
		(5) Prof. Ahmed T. Mora	Lease of Zazzau Pharmaceutical Industries Ltd (ZPIL), Zaria from ABUTH (2000 - 2010)	Dr. Abdurrahman Mora	Mora Road
		(6) Sen. Yusuf Datti Baba Ahmed	Founder of Baze University, Abuja and Baba Ahmed University, Kano	Alh. Baba Ahmed	Baba Ahmed
		(7) Hon. Mahmud Sani Sha'aban	Founder of Bakaries and Metal Nails (for building) Companies in Zaria (MAZARI)	Baba Shaban	Mora Road
		(8) Mall. Kabiru Ahmed	Founder of Private Security Outfit in Zaria	Alh. Ahmadu Mohammed	Mora Road
11	LECTURERS/TEACHERS	(1) Dr. Hakeem Baba Ahmed	University of Sokoto (now UDUS) and Baze, Abuja	Alh. Baba Ahmed	Baba Ahmed
		(2) Prof. Ahmed T. Mora	*ABU, Zaria *KASU, Kaduna and *IUO, Okada	Dr. Abdurrahman Mora	Mora Road
		(3) Prof. Hadiza Ahmed	ABU, Zaria	Alh. Ahmadu Mohammed	Mora Road
		(4) Alh. Umar F. Mora	MOE Kad State (Secondary School)	Dr. Abdurrahman Mora	Mora Road
		(5) Sheikh Ja'afar A. Mora	MOE Kad State (Secondary) and Private Islamic School	Dr. Abdurrahman Mora	Mora Road
		(6) Haj. Hafsatu A. Mora	Lecturer, Kadpoly, Kaduna	Dr. Abdurrahman Mora	Mora Road
12	NATIONAL HONOURS	(1) Arc. Muhammad Namadi Sambo	<i>GCON</i>	Alh. Sambo	Baba Ahmed
		(2) Dr. Hakeem Baba Ahmed	<i>OON</i>	Alh. Baba Ahmed	Baba Ahmed

		(3) Prof. Ahmed Tijjani Mora	<i>OON</i>	Dr. Abdurrahman Mora	Baba Ahmed
13	TRADITIONAL TITLES	(1) Arc. Muhammad Namadi Sambo	<i>“Sardaunan Zazzau”</i>	Alh. Sambo	Baba Ahmed
		(2) Alh. Lawal Iro	<i>“Magajin Malam Zazzau”</i>	Alh. Iro Kaya	Maje Road
		(3) Dr. Hakeem Baba Ahmed	<i>“Yariman Jama’a”</i>	Dr. Hakeem Baba Ahmed	Baba Ahmed
		(4) Prof. Ahmed Tijjani Mora	• <i>“Kayayen Sarkin Musulimi”</i> • <i>Wakilin Maganin Zazzau</i> • <i>“Iyan Piriga”</i>	Dr. Abdurrahman Mora	Mora Road
		(5) Hon. Mahmud Sani Shaban	<i>“Danburan Zazzau”</i>	Baba Shaban	Mora Road
		(6) Alh. Abdullahi Lawal Iro	<i>“Wazirin Tudun Wada ”</i>	Alh. Iro K’aya	Maje Road
		(7) Alh. Abdussalam Baba Ahmed	<i>“Wazirin Tudun Wada”</i>	Alh. Baba Ahmed	Baba Ahmed
14	PRO-CHANCELLORS AND CHAIRMEN OF UNIVERSITY COUNCILS	(a) Sen. Dr. Yusuf Datti Baba Ahmed	(i) Baze University, Abuja (ii) Baba-Ahmed University, Kano	Alh. Baba-Ahmed	Baba Ahmed
		(b) Prof. Ahmed Tijjani, Mora, <i>OON, mni</i>	Igbinedion University, Okada (IUO), Okada, Edo State	Dr. Abdurahaman Mora	Mora Road

BRIEF LIFE HISTORY OF PROFESSOR AHMED TIJJANI MORA

Professor Ahmed Tijjani Mora in Summary

1. Name

- Ahmed Tijjani Mora

2. Qualifications, Fellowships & National Honour

- *B.Sc (Pharmacy), MBA, FPC Pharm, PhD, FPSN, FICEN, FIHMN, FNIM, OON, mni*

3. Key Positions Held

- Pharmaceutical Representative (North), Messrs Grünenthal GmbH, Pharco (Nig.) Ltd, Kano
- Pharmaceutical Divisional Manager, Kaduna State Distribution Agency (KSDA) Ltd, Kaduna
- Chief Pharmacist (CPh) & Head, Pharmacy Department, Health Management Board (HMB), Kaduna State
- Chief Pharmacist (CPh), Ministry of Health Headquarters, Kaduna State
- Director of Pharmaceutical Services (DPS), Ministry of Health Headquarters, Kaduna State
- Project Manager (PM), Project Implementation Unit (PIU), AfDB/AfDF-Assisted Kaduna Health Rehabilitation Project (KHRP), Ministry of Health Headquarters, Kaduna
- Project Coordinator (PC), Disposable Syringes Manufacturing Plant, Dakache, Zaria, Ministry of Health, Kaduna State
- Managing Director/CEO, Zaria Pharmaceutical Company (ZPC) Ltd (Manufacturers of *Zarinject* disposable syringes & needles) Zaria, Kaduna State
- Managing Director/CEO, Zazzau Pharmaceutical Industries Ltd (ZPIL) (Manufacturers of solid & liquid pharmaceutical products) Kofar Doka, Zaria, Kaduna State
- Registrar & CEO, Pharmacists Council of Nigeria (PCN), Head Office, Idu Industrial Layout, Abuja, FCT
- Part-Time Lecturer, *Pharmacy Management*, Undergraduate & Postgraduate courses, Faculty of Pharmaceutical Sciences, ABU Zaria

- xii. Foundation Dean & Pioneer Head, Department of Clinical Pharmacy & Pharmacy Management, Kaduna State University (KASU), Kaduna
- xiii. Professor, Department of Clinical Pharmacy & Pharmacy Practice, College of Pharmacy, Igbinedion University, Okada (IUO), Edo State
- xiv. Chairman, Governing Council, Pharmacists/Pharmacy Council of Nigeria (PCN), Abuja, FCT
- xv. Pro-Chancellor & Chairman, Governing Council, Igbinedion University, Okada (IUO), Edo State

4. Learned Societies & Alumni Associations

- i. Chairman, Pharmaceutical Society of Nigeria (PSN), Kaduna State Branch
- ii. National Chairman, Ahmadu Bello University Pharmacy Alumni Association (ABUPAA)
- iii. National Chairman, Barewa Old Boys Association (BOBA), (1970 - Intake) Class Group
- iv. Chairman, Committee of Registrars of the Health Professions (CRHP)
- v. Secretary - General, Barewa Old Boys Association (BOBA)
- vi. Worldwide President, Ahmadu Bello University (ABU) Alumni Association
- vii. National Chairman, Conference of the Alumni Associations of Nigerian Universities (CAANU) (now Association of Nigerian Universities Alumni) (ANUA)

5. Traditional Titles

- i. *Wakilin Maganin Zazzau* of Zazzau Emirate
- ii. *Iyan Piriga* of Piriga Kingdom, Lere LGA, Kaduna State
- iii. *K'ayayyén Sarkin Musulmi* of the Sokoto Sultanate Council

6. Literary Works:

- Authored/Co-authored eighteen (18) Books
- Edited 27 Pamphlets and books;
- Attended more than 400 conferences, workshops and seminars, nationally and internationally from 1988 to date.

- Delivered about 140 lectures as a guest lecturer, session participant or scientific research presenter, pharmacy position papers on regulations at conferences/workshops out of the 400 attended.
- 25 Peer-reviewed papers and research findings and published in reputable journals.
- Supervised many MSc students in Clinical Pharmacy and Pharmacy practice and numerous Fellows of the West African Postgraduate College of Pharmacists (WAPCP).
- Lectured in three (3) prominent Nigerian universities for a cumulative period of fifty-one (51) years (1986 – date).
- Received almost one hundred (100) awards and recognitions by renowned organizations in the last thirty (30) years.
- Involved in no fewer than eighteen (18) Boards and Councils as Chairman, Chairman of Board of Trustees (BOT), or Member.

7. Married Life

- Married to Hajiya Aisha Uwa and Hajiya Asiya Ladi and blessed with children and grandchildren.

8. Travels

- Has travelled extensively across all the **36 states of Nigeria**, their capitals and the **Federal Capital Territory, Abuja**
- Internationally, he has visited **88 cities in 33 countries** around the world.

Table 1: The Various Pharmacy Regulatory Bodies that Prof. Ahmed Tijjani Mora was Involved/Appointed

S/N	PHARMACY REGULATORY BODY	REGULATOR AS	PERIOD
1.	Pharmacists Board of Nigeria (PBN)	Chief Pharmacist MoH/DPS, Kaduna	1989 – 1992
2.	Pharmacists Council of Nigeria (PCN)	DPS, Kaduna State	1992 – 1993

3.	Pharmacists Council of Nigeria (PCN)	Member of the Council (Representing PSN)	1999 – 2002
4.	Pharmacists Council of Nigeria (PCN)	Registrar and CEO	2003 - 2012
5.	National Agency for Food and Drug Administration and Control (NAFDAC)	Member of the Governing Council	2020 –2023
6.	Pharmacists Council of Nigeria (PCN)	Chairman of the Governing Council (Last Chairman)	2020 – 2022
7.	Pharmacy Council of Nigeria (PCN)	Chairman of the Governing Council (First Chairman)	2022 - 2023

Ahmed Tijjani Mora was born on **13th May, 1956 (2nd Shawwal 1375 AH)** into the distinguished family of the late **Mallam (Dr.) Abdurrahman Mora (1916–1997)** and the late **Hajiya Aminatu Dikko (1926–1996)**.

His Father’s Legacy

Mallam Abdurrahman Mora was a teacher at the Zaria Middle School before joining the Northern Nigerian Civil Service. He later rose to become a Federal Permanent Secretary in the now-defunct **Ministry of Lagos Affairs** and subsequently the **Federal Ministry of Internal Affairs**.

He also served Nigeria as **High Commissioner (Ambassador) to the Islamic Republic of Pakistan**, first in Karachi and later in Islamabad.

At the time of Ahmed’s birth in 1956, his father had just completed an important assignment as one of the ten **Adult Education Officers (AEOs)** appointed by the Northern Regional Government in 1954 to spearhead the fight against illiteracy (*Yak’i da Jahilci*). He served for one year as AEO for the defunct Zaria Province. Earlier, from **1950 to 1954**, he had been **Headmaster of the Zaria Middle School**.

The Legacy of Mallam Abdurrahman Mora: Pioneer Contributions to Education in Northern Nigeria

The late Mallam Abdurrahman Mora, the father of Professor Ahmed Tijjani Mora, left behind a remarkable legacy as one of the pioneer educators in British Northern Nigeria and indeed Nigeria. Two of his most notable achievements were his participation in two groundbreaking international educational programmes.

1. University of London Institute of Education Certificate Course (1945–1946)

In 1945, the Central Government of Nigeria offered ten (10) places for a special one-year Certificate Course at the University of London's Institute of Education. These places were distributed among the three regions of Nigeria as follows:

- Northern Region – four (4) slots, being the most populous,
- Eastern Region – three (3) slots, and
- Western Region – three (3) slots.

The selection process was rigorous, competitive and strictly merit-based, with no room for lobbying. After the interviews, the four candidates selected from the Northern Region were:

- Mallam Abubakar Tafawa Balewa,
- Mallam Yahaya Gusau,
- Mallam Bello Kano, and
- Mallam Abdurrahman Mora (then a classroom teacher at Zaria Middle School).

By this achievement, Mallam Abdurrahman Mora became one of the first Northern Nigerian teachers to attend the University of London Institute of Education, studying under a Central Government scholarship from 1945 to 1946.

2. United States Department of State Fellowship Programme (1952–1953)

In 1952, while serving as Headmaster of Zaria Middle School, Mallam Abdurrahman Mora was nominated for another pioneering opportunity. That year, the United States Department of State invited three hundred (300) educators from forty-nine (49) countries to participate in a six-month Fellowship Programme in America. Nigeria was allotted two slots and the responsibility for selection was given to the Nigerian Union of Teachers (NUT). Mr. E. E. Esua, then Secretary-General of the NUT, nominated Mallam Mora based on his distinguished record. Mallam Mora had earlier served as Assistant Secretary and later Secretary-General of the Northern Teachers Association (NTA) before its merger with the Southern counterpart to form the NUT. Thus, Mallam Abdurrahman Mora (representing the North) and Mr. S. I. Amifili (representing the East) were selected to attend the programme.

On arrival in the United States, Mallam Mora's group was placed at the Iowa State Teachers College, Cedar Falls, Iowa (now University of Iowa) (UNI), Iowa, USA, alongside Fellows from Liberia, Indonesia, the Philippines, Cambodia, Pakistan, Portugal, Brazil, Nicaragua, Mexico and other nations.

- After three months of intensive academic activities, the participants were granted freedom to travel across the U.S. Mallam Mora visited Chicago and New York.
- After the Christmas break, the group reconvened at Concord, New Hampshire, near the Atlantic coast.

This made Mallam Abdurrahman Mora the first Northern Nigerian to be sponsored by the U.S. Department of State for an international fellowship.

Upon his return, he resumed his position as the Headmaster of Zaria Middle School, further cementing his reputation as a pioneer in advancing education in Northern Nigeria.

In essence, through these landmark experiences in London (1945–46) and the United States (1952–53), Mallam Abdurrahman Mora blazed a trail in international academic exposure for Northern Nigerian educators. His pioneering role opened doors for future generations, leaving behind a proud legacy of leadership, excellence and service to education. From 1955, just before Ahmed’s birth, Mallam Mora became Chief Adult Education Officer for the entire Northern Region, a role he held until 1958. His career then advanced further, leading to senior administrative, diplomatic and ambassadorial appointments.

In appreciating the pioneering roles of Mallam Abdurrahman Mora, the Zaria Local Government Council (ZLGC) and some community groups named some schools, roads and centers after Mallam Mora as a form of immortalization. They include:

- **Mora Road:** This road (No. 23/25/27) was named after him. He moved out of Zaria City in 1955 to Tudun Wada and his personal house is located on this road.
- **Abdurrahman Mora Road:** Named after him in a new residential layout in Gaskiya Area of Tudun Wada, Zaria.
- **Abdurrahman Mora LGEA Primary School:** A Primary School at “Pampo” Tudun Wada, Zaria.
- **Abdurrahman Mora Educational and Vocational Centre:** The National Secretariat of ZEDA was constructed and named after Mallam Mora. It is opposite Barewa College on Gaskiya Road, Zaria.
- **Abdurrahman Mora College of Islamic Studies:** Located at Tudun Wada, Zaria, named after him by the proprietor, Sheikh Nuhu Tahir Tajuddeen.

Ahmed’s Family Background

Ahmed is the fourth child of Mallam Mora, the third son overall and the second son of his mother, Hajiya Aminatu Dikko.

- The first child of the family was the late Alhaji Muhammadu Inuwa Mora (1940–2001), who was also the first son of Hajiya Aminatu.
- His siblings at the time of his birth included Dr. Muhammad Saifuddeen (b. 1951) and Hajiya Aishatu (b. 1954), children of his stepmother, Hajiya Binta Atti (b. 1935).
- In February 1957, Hajiya Binta Atti gave birth to Muhammad Aminu Mora, just nine months after Ahmed.

Ahmed and Aminu grew up as close companions, sharing the same room, attending the same primary and secondary schools and only parting briefly when they enrolled at different campuses of Ahmadu Bello University (ABU) Zaria in the mid-1970s.

Childhood Years (1955–1971)

From 1955 to 1971, a span of sixteen years, Mallam Mora was seldom continuously resident in Zaria due to his national assignments. During this period, Ahmed and his siblings grew up largely under the care and supervision of their mothers, extended family and guardians.

- 1955–1961: Mallam Mora was in Kaduna, serving as Chief Adult Education Officer and later as one of the first four Senior Assistant Secretaries in the North and posted to the Ministry of Health, Kaduna.
- 1961: He was posted to Jeddah, Saudi Arabia, as Consul and Pilgrims Officer, later becoming Chargé d’Affaires at the Nigerian Embassy, when diplomatic relations was opened with Saudi Arabia.
- December 1961: He was recalled to Nigeria and appointed Deputy Permanent Secretary, Federal Ministry of Mines and Power, Lagos.
- 1962–1966: Appointed Permanent Secretary, Federal Ministry of Lagos Affairs.

- 1966–1967: Served as Permanent Secretary, Federal Ministry of Internal Affairs.
- 1967–1971: Appointed Nigeria’s High Commissioner to Pakistan by General Yakubu Gowon. He retired from public service in 1971 at the age of 55 and returned to Zaria.

Thus, it was only when Ahmed was 15 years old and already in secondary school at Government College Zaria (now Barewa College) that his father finally settled at home in Zaria with both wives.

Travels and Family Bonding

Despite long absences, Mallam Mora ensured his family bond remained strong:

- Ahmed and Aminu frequently visited their father in Kaduna during early childhood.
- Together with siblings Dr. Saifuddeen and Aishatu, they travelled by train to Lagos annually to spend time with him.
- They visited Pakistan in 1968 (Karachi) and again in 1970 (Islamabad) during his diplomatic posting.
- Every year, during the holy month of *Ramadhan*, Mallam Mora returned home to Zaria for his annual leave, leading the family in nightly *Taraweeh* prayers.

The following people were close to the late (Dr.) Abdurrahman Mora (1916-1997). Their closeness, love and mutual respect for one another have been a source of inspiration and have had a profoundly positive impact on Ahmed. They includes the following (in no particular order):

1. Sir Ahmadu Bello (*Sardaunan Sokoto*)
2. Sir Abubakar Tafawa Balewa
3. Alh. Ahmed Mohammed (*Ahmadu Ido*)
4. Alh. Ahmed Talib
5. Alh. Muhammadu Bello Kagara (*Walin Katsina*)

6. Alh. (Dr.) Abubakar Imam (Mallam Mora's father –in-law)
7. Alh. Suleiman *Dangaladiman* Kaiama
8. Alh. Aliyu Maiborno
9. Alh. Baba Ahmed
10. Alh. Abubakar Tunau Mafara (*K'ayayen Sardauna*)
11. His Eminence Alh. Haliru Binji
12. Alh. Hassan Rafindadi (*Sarkin Yakin Katsina*)
13. Alh. Hussaini Mashi (*Sarkin Daji*)
14. Alh. Ibrahim H. Musa (Pharmacist),
15. Alh. Inuwa Mai Gidan Sauro
16. Alh. Isa Koto
17. Alh. Jimada Pategi (*Nakordi Pategi*)
18. Alh. Muhammadu Inuwa Jibo
19. Alh. Muhammadu Inuwa Musa (Dispenser)
20. Alh. Muhammadu Sani (*Limamin Kamfawul*) (*Unguwan Juma, Zaria*)
21. Alh. Musa Jam'iyya
22. Alh. Musa S. Abubakar (*Madawakin Kaiama*)
23. Alh. Musa Yar'adua (*Mutawallen Katsina*)
24. Alh. Nuhu Bamalli (*Magajin Garin Zazzau*)
25. Alh. Nuhu Bayero (*D'an Iyan Zazzau*)
26. Alh. Shehu Diriver
27. Alh. Tanimu Kubau
28. Alh. Yahaya Gusau (*Shettiman Sokoto*)
29. Hajiya Dada Sare
30. His Highness Alh. Muhammadu Aminu (17th Emir of Zazzau)
31. His Highness Alh. Shehu Idris (18th Emir of Zazzau)
32. Liman Muhammad Sani Gummi (*Limamin Kongo*)
33. Mallam Ladan Na Mazadu (*Unguwan K'arfe, Zaria*)
34. Sheikh Abubakar Mahmood Gummi
35. Sheikh Nuhu Tahir Tajuddeen
36. Gen. Yakubu Gowon, *GCFR*
37. Alh. Muhammadu Lawan

38. Alh. Abba Habib
39. Brig. Abba Kyari
40. Prof. Ango Abdullahi
41. Sultan Ibrahim Dasuki
42. Amb. Nuhu Mohammed
43. Alh. Hamidu Yahaya
44. Alh. Umaru Idris (*Dangaladiman Zazzau*)
45. Alh. Yusufu Ladan (*Dan Iyan Zazzau*)
46. Alh. Abdullahi (*Shettiman Zazzau*)
47. Alh. Ishaku Suleiman (*Katukan Zazzau*)
48. Dr. Dalhatu Sarki Tafida (*Tafidan Zazzau*)
49. Alh. Umaru Abdul Mutallab
50. Alh. Aliko Mohammed (*Dan Iyan Misau*)
51. Alh. Hamidu Ladan Zuru
52. Alh. Abdurrahman Okene
53. Alh. Ibrahim Namaska Kagara
54. Alh. Bala (*Tafarkin Birnin Gwari*)
55. Alh. Nasiru (Mokwa)
56. Alh. Muhammadu Lawal Tudun Wada
57. Alh. Idris Morrow
58. Alh. Hamidu Shafi'i
59. Alh. Salisu Ahmed (Chairman)
60. Alh. Tijjani Manumfashi
61. Alh. Abubakar Kaiama (*Wazirin Kaiama*)
62. Alh. Haruna Soba.
63. Alh. Sha'aban (Baba Sha'aban)
64. Alh. Ja'afaru Dogo
65. Alh. Bashir Ahmed Banadeen
66. Amb. John Garba
67. Alh. Abu Musawa
68. Alh. Bala Bello Dambazau
69. Alh. Abdullahi Makarfi (*Barden Zazzau*)
70. Sheilk Muntak'a Kumasi (*Unguan Lemo, Zaria*)

The Influence of Family and Guardians

During their father's long absences, several key figures influenced Ahmed's upbringing:

- **The late Alhaji (Dr.) Abubakar Imam (1910–1981)**, renowned Hausa novelist and step-grandfather, oversaw the household, particularly as his own home in Tudun Wada was nearby.
- **The late Alhaji Muhammadu Inuwa Mora (1940–2001)**, Ahmed's elder brother, who, after returning from studies in the UK, worked with the Government Printing Press in Kaduna, often supervised the family during weekends.
- **The late Alhaji Muhammadu Inuwa Musa ("Dispenser", d. 2024)**, a relative and co-founder of United Arewa Chemists in Zaria, regularly checked on the family in the evenings and cared for their health needs.

Formative Impact

The combined influence of his father, his mother's (Hajiya Aminatu Dikko and Hajiya Binta Atti), his step-grandfather Baba Imam, his elder brother, Inuwa and other close relatives deeply shaped Ahmed's upbringing. Even after his father's retirement and return to Zaria in 1971, these relationships continued to guide him. It was under this nurturing and intellectually vibrant environment in Tudun Wada, Zaria that Professor Ahmed Tijjani Mora grew up and began the journey that would define his later life and career.

Schools Attended

2.1 Qur'anic Schools in Zaria

As is traditional in Northern Nigeria and parts of the Southwest where Islam is dominant, Ahmed and his siblings all attended Qur'anic schools. In his case, however, his late father engaged a learned scholar, Mallam Danlami Attah (of blessed memory), in Tudun Wada, Zaria to

teach the children from his house and from Baba Imam's household the recitation of the Glorious Qur'an. Thus, Ahmed Tijjani, Muhammad Aminu, Surajuddeen, Yusuf Adamu (Danlami) and Abubakar (Dogara) were all in the same Qur'anic class. During his annual leave, Mallam Mora would complement these teachings in the evenings, especially for his own children.

During holidays in Lagos, Karachi or Islamabad, Ahmed Tijjani and Muhammad Aminu were required to recite the Qur'an before their father after the *Isha* (late-night) prayers. Around 1968, Ahmed completed the full recitation of the Qur'an after which a large ceremony was held at the Qur'anic School of Mallam Dan Ammu, Imam Road, Tudun Wada, Zaria. Immediately afterwards, he began studying *Ahlari*, *Ishmawiyi* and *K'awa'id al-Tauhid*, advanced texts in Islamic Theology.

2.2 LEA Primary School, Tudun Wada, Zaria

Ahmed was enrolled into L.E.A. Primary School, Tudun Wada, Zaria in 1963 for his formal primary education. Before this, he and his brother Aminu attended what would now be regarded as kindergarten, then called "*Aji Gutsure*" (half-class), at the location of the present Nurul-Huda Primary School, (now Abubakar Imam LGEA Primary School) Tudun Wada, Zaria.

The school was co-educational, with boys and girls mainly from Tudun Wada and Zaria City (now Zaria Local Government Area, Kaduna State). A few pupils also came from Sabon Gari. At that time, the school only offered classes I–IV (Junior Primary). Pupils had to proceed to Soba (about 40 km away on Jos Road) or Giwa (25 km away on the Funtua road) for Senior Primary. Ahmed's elder brother, Saifuddeen, was sent to Soba. However, by 1966, when Ahmed reached Class IV, the school was upgraded to include Senior Primary (Classes V–VII).

Among his 1963 classmates were the late Lawal Ahmed; Aminu Salihu (now Prof. Aminu Salihu Mika'ilu, the former Vice-Chancellor of UDUS, KASU, and NSU Keffi); Saidu Shafi'i; Awwalu Aliyu (class monitor) and Ibrahim Balarabe (later a Permanent Secretary in Kaduna State). By Class VII, more pupils had joined, including Idris Hayatu (now Arc. Idris Hayatudeen, a District Head in Borno State), Mohammed Dalhatu Zurmi, Harisu Ibrahim and Abdulkadir Ajuji.

In Class VI, Ahmed and three others (Saidu Shafi'i, Tijjani Mohammed and the late Sani Ahmed Aliyu) formed a group called "*The Four Wonders*," engaging in healthy rivalry with their classmates Ismaila Datti (later Capt. Ismaila Datti, (Rtd), the late Ibrahim Sambo and the late Dalhatu Yusuf, who called themselves "*The Three Musketeers*." Notable teachers included Mallama Mairamu, Mallam Yahaya Barau, Mallam Halidu Tanko (later Justice H. Tanko Soba), Mallam Tukura and Mallam Abubakar. The headmasters during his time were Mallam Ibrahim Sarki Lukman, Mrs. Clara Wayo and Mallam Sambo Kusallo.

Ahmed enjoyed football as a goalkeeper. In 1969, he sat for the Common Entrance Examination and was one of only eight pupils (out of 40) who passed, obtaining the second-best result. Among the girls in his class were Amina Ammani, Aishatu Jibrin, Shatu Jibrin, Maimuna Mohammed, Amina Mohammed, and Jidda Baba-Ahmed (now Hajiya Jidda Umaru Abdulmutallab). Hajiya Jidda, daughter of the late Alhaji Baba Ahmed of Tudun Wada, left in Class V (1967) to marry Dr. Umaru Abdulmutallab, the renowned banker.

The Baba Ahmed family had a close bond with Ahmed's family (the Moras), with their houses close by in Tudun Wada. This bond has lasted nearly a century. In recognition of Baba Ahmed's legacy, the L.E.A. Primary School Tudun Wada has since been renamed Baba Ahmed Primary School.

2.3 Government College, Zaria (Now Barewa College, Zaria)

After excelling in the Common Entrance Examination, Ahmed was admitted to Government College, Zaria (now Barewa College, Zaria) after having been interviewed by the Principal, Mr. S.V. Baker, a British World War II veteran. He was admitted alongside his brother, Muhammad Aminu. They reported on January 17, 1970: Ahmed was placed in Nagwamatse House (Form IB, Admission No. 2511) and Aminu in Mallam Smith House (Form IC, Admission No. 2547).

Life in Barewa was exciting. Ahmed quickly adapted, though he became a “fag” (junior assigned to serve a senior) to his elder brother, Saifuddeen, then in Form V. In Form II, he was assigned to Kolo Ibrahim (now Professor in IBB University), who, however, never exploited him. His roommates included Bello Ahmed (a close friend who died tragically in 1981), Irmiya Yawuk (Jermaine Ya’u) and Pama Nagaba. Their room head, Aminu Umar Bambale, was a popular athlete and a good mentor. Other notable seniors included Prof. Jacob Kwaga, the late Peter Adamu (later GM/CEO of KSPC Ltd) and Dr. Usman Bugaje, Balarabe B. Ibrahim, the late Sani Isah Hadeja, etc.

By 1972, Ahmed was appointed class monitor, a position he held until graduation in June 1974. He also became Prefect of Nagwamatse House in 1973. He excelled academically, obtaining Division One with Distinction in the WASC Mock Exams (aggregate 12, later adjusted to 13 by Kaduna Ministry of Education) and emerged as the best all-round graduating student of Form VB.

Barewa College, founded in 1921 (originally Katsina Training College) has produced five Nigerian Heads of State more than any other institution in West Africa. Ahmed’s family is closely tied to the school: his father (Mallam Abdurrahman Mora, Admission No. 233) and 10 other family members attended. Collectively, three (3) members of the Mora family taught at Barewa for 35 years, the longest

cumulative service by any family in the 104-year history of the college.

2.4 School of Basic Studies (SBS), ABU, Zaria

In July 1974, Ahmed gained admission to the School of Basic Studies at Ahmadu Bello University (ABU), Zaria as part of the pioneer set for the experimental one-year A-Level programme. He studied Mathematics, with Physics, Chemistry and Biology and passed the IJMB exams in 1975. His classmates included Sa'idu Shafi'i, Ibrahim Sambo, Sani Ahmed and others who later became pharmacists, engineers, doctors and academics. Ahmed resided in Alexander Hall and Ribadu Hall during this period, forming lifelong friendships. He was room mate with Abdulmumini Hassan, Mamu Mukhtar and others. He also maintained strong ties with classmates, who joined the Nigerian Defence Academy (NDA), Kaduna and rose to senior military ranks, including Sultan of Sokoto, Alh. Sa'ad Abubakar, Lt. Gen. Abdulrahman Bello Dambazau (Rtd) and Air Commodore Mohammed Abdullahi Yahaya (deceased).

2.5 Department of Pharmacy and Pharmacology, ABU, Zaria

Ahmed was admitted to the Department of Pharmacy and Pharmacology at ABU (now the Faculty of Pharmaceutical Sciences) in 1975 and graduated with a B.Sc. in 1978. Pharmacy degree. His project was titled, *"The Applications and Technology of Aerosols."*

He was elected Public Relations Officer of the Pharmaceutical Association of Nigerian Students (PANS, ABU Chapter), where he organized the first Pharmacy Week in 1977. Today, he serves as the Life Patron and Grand Patron of PANS (ABU) and PANS (KASU), respectively.

2.6 Internship Training, ABUTH Kaduna

From June 1978 to June 1979, Ahmed completed his mandatory internship at the Department of Pharmacy, Ahmadu Bello University Teaching Hospital (ABUTH), Kaduna. He trained under Alhaji

Abdulyakini Daromosu (Deputy Chief Pharmacist) and worked with Chief Pharmacist G.O. Obiaga, the Chief Pharmacist who resided in the ABUTH headquarters in Zaria.

2.7 MBA Programme, ABU, Zaria

In October 1983, Ahmed enrolled in a two-year full-time MBA programme at ABU, graduating in 1985. His project, supervised by Dr. J.G. Donli, was titled: *“Marketing Problems and Strategies of the Pharmaceutical Industry in Nigeria: A Study of Astra Arewa Ltd, Kaduna.”*

2.8 Fellowship of WAPCP

In 2002, Ahmed became a Fellow of the West African Postgraduate College of Pharmacists (WAPCP) in Lagos. The certificate was formally presented in March 2002, in Banjul, Gambia during the graduation ceremony.

2.9 Ph.D. Programme, Usmanu Danfodiyo University, Sokoto

In April 2012, Ahmed successfully defended his PhD thesis in Management, titled: *“Distribution Strategies in the Pharmaceutical Industry: A Case Study of Selected Pharmaceutical Companies in Nigeria.”* He was supervised by Prof. Abdullahi Muhammad Bashir, Prof. Aminu Diyo Sheidu and Dr. Yahaya Zakari.

2.10 Senior Executive Course (SEC), NIPSS Kuru, near Jos.

In February 2010, Ahmed attended a ten-month Senior Executive Course (SEC) at the prestigious National Institute for Policy and Strategic Studies (NIPSS), Kuru, near Jos. He served as Chairman of Group 6, worked with senior officers from the Army, NAF, the Navy, Immigration, Police, Customs, DSS, Civil Service, etc. and with Prof. Ibrahim Muhammad Jumare as the Study Group Director. As the syndicate Group (6) Chairman, he led his Study Group to Kogi and

Bayelsa States in Nigeria and Germany and Burundi for World and Africa study tours respectively.

His final project, supervised by Dr. Bulus Kyauta Tanyigna, was titled: *“The Problem of Fake Drugs in Nigeria: A Case Study of the National Agency for Food and Drug Administration and Control (NAFDAC).”* He graduated with excellent grades and was awarded the prestigious Membership of the National Institute (*mni*). Ahmed attended many other courses in Nigeria and abroad. Those lasting for more than one week include the following:

- The Advanced Management Course (AMC) at the Administrative Staff College of Nigeria (ASCON), Badagry, Lagos, Nigeria, in 1990.
- A course on *“Making Decentralization Work: Tools for Policy Makers and Managers”*, Harvard University School of Public Health, Boston, MA, USA, in March 2009.
- A course at Liverpool Hope University, Liverpool, UK, on *Strategic Policy Development* for the “Members of Committee of Registrars of the Health Profession (CRHP)” in March 2011.

3.0 University Academic Career:

3.1 Ahmadu Bello University (ABU), Zaria

Ahmed Tijjani began his academic career in 1986 as a Part-Time Lecturer in the Department of Pharmaceutics and Pharmaceutical Microbiology, Faculty of Pharmaceutical Sciences, ABU Main Campus, Samaru, Zaria. He taught Part III (now 400-Level) undergraduate courses in *Pharmacy Management*. In 2001, he was promoted to Part-Time Senior Lecturer. By then, the Department of Clinical Pharmacy and Pharmacy Practice had been created, and he, along with other lecturers, was reassigned to the new department. From 2016, he began delivering lectures to M.Sc. (Clinical Pharmacy) students at ABU, in addition to supervising both M.Sc. and Ph.D.

theses in Clinical Pharmacy. Now in his thirty-ninth (39th) year of service, Ahmed continues to lecture at ABU on a *pro bono* basis.

During his period of teaching in the Faculty of Pharmaceutical Sciences, ABU Zaria, the following served as Deans: Prof. C.O.N. Wambebe, Prof. Abdullahi Mustapha, Prof. P.F. Olurinola, Dr. Amos Bangudu, Prof. J.A. Onaolapo, Prof. E.M. Abdurrahman (twice), Prof. H.A. Kaita, Prof. M.I. Sule, Prof. U.U. Patch, Prof. M. Garba, Prof. N.M. Danjuma, Prof. Aliyu Musa Mohammed, and the current Dean, Prof. Mohammed Magaji. Remarkably, four (4) of these Deans had been Ahmed Tijjani's students at the undergraduate level.

3.2 Kaduna State University (KASU), Kaduna

In October 2012, following a competitive interview process, Ahmed was appointed as a tenured staff member of Kaduna State University (KASU). He became the Foundation Dean of the Faculty of Pharmaceutical Sciences and the Pioneer Head of the Department of Clinical Pharmacy and Pharmacy Management. Earlier, he had been invited by the University Management to chair a committee on the viability of establishing a degree - awarding Faculty of Pharmaceutical Sciences at KASU. Members of the committee included Prof. M. Garba (later Vice Chancellor, Federal University Gusau), Prof. (Mrs.) Suzanne Allagh (Pharmaceutics, ABU Zaria), Prof. M.S. Abubakar (Pharmacognosy, ABU Zaria), Pharm. Yohana Gaiya (Director of Pharmaceutical Services, Kaduna State Ministry of Health) and Barr. Abdullahi Zubairu Abdullahi (Secretary). The committee worked for three months (February – May 2012) and submitted its report, which was fully adopted by the University. Subsequently, the National Universities Commission (NUC) team on Resource Verification Team visited and inspected facilities in the faculty and approved the proposed B. Pharm programme with high commendation. The programme formally commenced in January 2013, with Ahmed delivering the very first lecture to the pioneer set

of 38 of the B. Pharm 100-level students. He oversaw the admission exercises for the 2012/2013, 2013/2014, 2014/2015, and 2016/2017 academic sessions.

In May 2016, Ahmed voluntarily resigned from KASU. Sadly, earlier on 2nd May 2016, his mother, Hajiya Aminatu Dikko, passed away in Zaria at the age of 92 after a prolonged illness. She was laid to rest the same day at the Dambo Muslim Burial Ground along Jos Road beside her late husband and first son.

During Ahmed's tenure, the Vice-Chancellor of KASU was Prof. William Barnabas Quirix (currently Vice-Chancellor, Federal University of Allied Sciences, Kachia, Kaduna State). His Deputy Dean at the Faculty, Prof. Ben A. Chindo, was one of Ahmed's former undergraduate students at ABU.

3.3 Igbinedion University, Okada (IUO), Edo State

In August 2016, Ahmed was appointed **Visiting Professor** in the Department of Clinical Pharmacy and Pharmacy Practice, College of Pharmacy, Igbinedion University, Okada (IUO). He taught 300L, 400L and 500L B. Pharm students, and mentored young lecturers of the College. He eventually became the first professor of Clinical Pharmacy and Pharmacy Practice in IUO

Despite the distance from his base, Ahmed dedicated his time and energy to ensure that the students of IUO received the best possible training in Clinical Pharmacy and Pharmacy Practice. His colleagues — Dr. Oseji, Dr. Isobighe, Dr. Bishop and Dr. Nnamani and others were supportive and collaborative. Ahmed also enjoyed immense encouragement from the then Vice-Chancellor, Prof. Eghosa Osaghae, an internationally renowned political scientist, who is now the DG/CEO of the Nigerian Institute of International Affairs (NIIA),

Lagos. His successor, Prof. Lawrence Ikechukwu Ezemonye, further supported Ahmed's work, described as both "multi-tasking and genial." Perhaps his greatest motivation at IUO came from His Excellency Sir Gabriel O. Igbinedion, *CFR*, the *Esama* of Benin Kingdom and Founder of IUO (the first private university in Nigeria) and his son, His Excellency Chief Lucky Igbinedion, the former two-term Governor of Edo State and Deputy Chancellor of the University. Over time, Ahmed rose to serve not only as a Member of the Igbinedion University Teaching Hospital (IUTH) Board and later the Governing Council of the University, but was also appointed Pro-Chancellor and Chairman of Council of Igbinedion University, Okada, in 2025.

4.0 Career Employment in the Practice of Pharmacy

4.1 NYSC Pharmacist, General Hospital Potiskum, Borno State

In the area of hospital pharmacy practice, Ahmed began his career as the Pharmacist-in-Charge of the Pharmacy Department, General Hospital, Potiskum (then in Borno State, now Yobe State) during his National Youth Service Corps (NYSC) programme from June 1979 to June 1980. The Chief Pharmacist, Ministry of Health, Borno State, at the time was Alhaji Garba Abubakar Daya (deceased), (later *Madakin Fika*). Ahmed initially worked at the General Hospital in Maiduguri alongside Pharmacist Isa Marte Hussaini (now Prof. Isa Marte Hussaini), an outstanding pharmacist and scientist, who later obtained his M.Sc. and Ph.D. at the University of London, UK. Prof. Hussaini, currently Chief of Staff to the Governor of Borno State, Prof. Babagana Umara Zulum, graduated in 1979 from ABU Zaria, a year junior to Ahmed. At that time, Isa was undergoing his internship at General Hospital, Maiduguri. Later in life, he worked as a brain tumor researcher at the University of Virginia Teaching Hospital, Charlottesville, USA. Ahmed frequently visited Isa in Charlottesville and often stayed at his home. While Ahmed was at General Hospital, Potiskum, the Medical Officer in Charge was Dr. Audu Dauda, and the

Chief Nursing Officer was Mr. Gadzama. Ahmed worked with auxiliary pharmacy staff, including Mallam Bah and Mallama Laraba, with whom he still maintains occasional contact.

4.2 Messrs. Grünenthal GmbH under Pharco (Nig.) Ltd., Kano

After NYSC, Ahmed was appointed as the Northern Area Pharmaceutical Representative for products manufactured by Messrs Grunenthal GmbH, marketed in Nigeria through Pharco (Nig.) Ltd along Beirut Road, Kano. He was stationed in Kano and oversaw the northern region. The Branch Manager, Mr. Shoge (a pharmacist), took special interest in Ahmed and worked closely with him. Grunenthal GmbH, based in Stolberg, Germany manufactured products, such as *Acabel*, *Enterosediv*, *H-115 cream*, *Grunovits multivitamins*, *Megacillin injections* and *Nevin*, all of which were marketed in Nigeria. At the Lagos headquarters, the General Manager was Mr. Peter Ewers (a German) and the Marketing Manager was the late Pharm. S.A. Adeniji, while the Managing Director/CEO of Pharco (Nig) Ltd was Mr. Bola Alaka.

Within a year under Ahmed's leadership, marketing and sales performance grew significantly. Consequently, Mr. Ewers approved the opening of a new branch in Jos to serve the North-Eastern states as well as Plateau, Nasarawa and Benue States. Ahmed was tasked with nominating a suitable pharmacist. He recommended his friend and former classmate, Pharm. Nuhu Namallam Ibrahim, who understudied Ahmed in Kano for six months before resuming in Jos. Ahmed and Pharm Ibrahim shared a flat in Kaduna when they were with ABUTH, Kaduna, as Intern Pharmacists. Unfortunately, Pharmacist Ibrahim died in April, 2025 aged 72.

4.3 Kaduna State Distribution Agency (KSDA) Ltd., Kaduna

In January 1986, after a successful interview, Ahmed was appointed as Pharmaceutical Divisional Manager of the Kaduna State Distribution Agency Ltd (KSDA), becoming the first pharmacist ever

employed by the agency. KSDA planned to diversify into pharmaceutical wholesales and importation.

Ahmed established the agency's pharmaceutical division, setting up an office showroom along Alkali Road, Kaduna. Products were procured from Lagos-based manufacturers and, at one time, Rima Pharmaceuticals Ltd in Ireland. These were distributed to community pharmacies and hospitals across Kaduna and Katsina States. The General Manager/CEO of KSDA was the late Alhaji Yuguda Usman.

4.4 Health Management Board (HMB), Kaduna State

In May 1988, Ahmed was appointed Chief Pharmacist and Head of Pharmacy Department, Health Management Board (HMB), Kaduna State, becoming the first pharmacist to hold the position in the newly restructured state (after Katsina was carved out). He was responsible for pharmacy services across fourteen general hospitals in Kaduna State. The GM/CEO was the late Dr. Sunday Samuel Umar. Other senior officers included Dr. D. Bok (Chief Medical Officer), Mr. Kure (Chief Nursing Officer) and Mallam Musa Yarima (Financial Controller). While serving as Chief Pharmacist, Ahmed organized a landmark workshop, titled *“Towards Improving Hospital Pharmacy Practice in Kaduna State”* at Hamdala Hotel, Kaduna. The Honourable Commissioner of Health, Pharm (Mrs). Sarah J.L. Hassan, was impressed and declared the workshop open as Special Guest of Honour.

4.5 Ministry of Health (MoH), Kaduna State

In May 1989, Ahmed was transferred to the Kaduna State Ministry of Health Headquarters as Chief Pharmacist. In 1990, he became the first Director of Pharmaceutical Services (DPS) in the Kaduna State Civil Service. Key colleagues at the ministry included Pharm. (Mrs.) Sarah Jamima Ladi Hussan (Hon. Commissioner of Health), Dr. Yohana Madaki (Director of Primary Health), Mr. Ayuba Mairabo (Chief Nursing Officer), Dr. Hannatu Yako Shehu (Director, PHC), Dr. Hussaini

Yahaya (Director, PRS) and Mr. Shekari (Chief Accountant, who later became the Accountant-General of Kaduna State).

Ahmed also had longstanding family ties with prominent figures in the MoH from Zaria, including Alh. Abubakar Ladan (DG, MoH) and Alh. Awwalu Muhammad Makarfi (later DG, MoH), both of whom were elder family friends. They both serve as Director Generals in the MoH when Ahmed served as the DPS.

4.6 Project Implementation Unit (PIU) – AfDB/AfDF Kaduna Health Rehabilitation Project (KHRP), Ministry of Health Headquarters, Kaduna

In November 1992, Ahmed was appointed the first Project Manager (PM) of the Project Implementation Unit (PIU) for the African Development Bank (AfDB) and African Development Fund (AfDF)-assisted Kaduna Health Rehabilitation Project (KHRP) in the headquarters of the State Ministry of Health (MoH), Kaduna. As Project Manager, he was responsible for coordinating the implementation of the multimillion-dollar loan-funded project, which rehabilitated five (5) general hospitals across Kaduna State. He successfully supervised procurement, logistics and installations of hospital equipment, medical devices and pharmaceutical supplies. The project significantly improved the state's health infrastructure and was regarded as one of the most successful AfDB/AfDF health projects at the time.

4.7 Disposable Syringes Manufacturing Plant Project, Dakace, Zaria

In 1992, Ahmed was appointed Project Coordinator (PC) for the Disposable Syringes Manufacturing Plant, Dakace, Zaria, under the Ministry of Health Headquarters, Kaduna. The project aimed to establish Nigeria's first state-owned manufacturing plant for disposable syringes and needles. Ahmed oversaw planning, construction, equipment procurement and staff recruitment phases for

28 months till commissioning. This role provided the foundation for his later leadership of the Zaria Pharmaceutical Company Ltd.

4.8 Zaria Pharmaceutical Company (ZPC) Ltd., Kaduna State

In 1997, Ahmed became the first Pharmacist Managing Director and Chief Executive Officer (MD/CEO) of Zaria Pharmaceutical Company (ZPC) Ltd, (manufacturers of *ZARINJECT* disposable syringes and needles). As MD/CEO, he provided visionary leadership that positioned ZPC as one of the first indigenous companies to produce disposable medical consumables locally. His tenure emphasized quality assurance, staff training and market expansion.

4.9 Zazzau Pharmaceutical Industries Ltd. (ZPIL), Zaria

In 2000, Ahmed assumed the role of Managing Director and CEO of Zazzau Pharmaceutical Industries Ltd (ZPIL) located at Kofar Doka, Zaria, Kaduna State. The company specialized in the manufacture of both solid and liquid pharmaceutical products. Under his leadership, ZPIL enhanced local drug production and contributed to reducing Nigeria's dependence on imported medicines.

4.10 Pharmacists Council of Nigeria (PCN), Abuja

In 2003, Ahmed was appointed the Registrar and Chief Executive Officer (CEO) of the Pharmacists Council of Nigeria (PCN), with headquarters at Idu Industrial Layout, Abuja, FCT, through the interview process. As Registrar, he provided regulatory oversight for pharmacy education, licensing and practice across the nation. He spearheaded reforms that enhanced pharmacy training curricula, strengthened inspection frameworks and promoted professional ethics nationwide. During his tenure, he was also elected Chairman, Committee of Registrars of the Health Professions (CRHP), where he coordinated inter-professional collaboration among regulators of health professions for an unprecedented three (3) terms totaling six (6) years (2006 - 2012).

4.11 Governing Council of the Pharmacist's Council of Nigeria (PCN)/Pharmacy Council of Nigeria (PCN)

After completing his tenure as Registrar/CEO, Ahmed continued to serve the profession at the highest level. He was appointed Chairman of the Governing Council of the Pharmacists Council of Nigeria (PCN), Idu, Abuja by His Excellency, President Muhammadu Buhari, *GCFR*, in June 2020 and served for 3 years. In August 2022, the National Assembly changed the name to Pharmacy Council of Nigeria, which the President assented to. In this capacity, he provided strategic leadership and policy direction for the regulation of pharmacy practice in Nigeria. His chairmanship further cemented his reputation as one of the most respected pharmacists and health sector leaders in the country.

5.0 Involvement in the Formation and Management of Learned Organizations

Ahmed has played significant roles in establishing and leading professional and community-based associations dedicated to service and development. Notably, he pioneered the Committee of Project Managers and Procurement Officers of the five (5) States involved in the Project Implementation Units (PIUs) of the Multistate Health Rehabilitation Projects funded by the African Development Bank (AfDB) and the African Development Fund (AfDF). These States were Kaduna, Kebbi, Sokoto, Ondo and Ogun, with Ahmed serving as Chairman for five (5) consecutive, unbroken years. His leadership has extended to other important associations as outlined below:

5.1 Zaria Educational Development Association (ZEDA)

ZEDA was established in 1992 by young men and women seeking to improve the quality of western-style education in Zaria, Zazzau Emirate, and Kaduna State through community participation. Ahmed was a founding member and a member of the Central Working Committee (CWC) from its inception in 1992 until 2011, when he was

elevated to the Board of Trustees (BoT) as the only foundation member appointed on individual merit, rather than by statutory provision. He has retained this position to date.

Ahmed's father, the late Alhaji (Dr) Abdurrahman Mora, of blessed memory, served as the first Chairman of the BoT. The Board was inaugurated by His Highness, Alhaji Shehu Idris, *CFR*, the Emir of Zazzau, at the Emir's Palace, Zaria, on 5th December, 1992. Since then, other distinguished leaders, such as Amb. Muhammadu Lawal Sambo, *mni (Walin Zazzau)*, Engr. Abdullahi Nuhu Bamalli (*Ciroman Zazzau*) and Prof. Idris Abdulkadir, have served as Chairmen. The current Chairman is Dr. Stephen C. Kitchener, *OON*.

Ahmed was also the first Chairman of the Prominent Indigenes and Friends Scholarship Scheme (PIFSS) a Standing Committee of ZEDA, which was launched in 1995. He led until 2003, when he relocated to Abuja for national service. Other ZEDA programmes include the Remedial Programme, Vocational Training Programme, JAMB Clinic and Quiz/Debate Competitions for primary and secondary schools within Zazzau Emirate and Kaduna State. He also chaired the organizing committees for ZEDA@Ten (2002) and ZEDA@Twenty (2012) anniversary celebrations. The current CWC Chairman is Dr. Alimi Bello, with Prof. Isa Abbass as Secretary.

5.2 Ahmadu Bello University Pharmacy Alumni Association (ABUPAA)

ABUPAA was conceived in 1994 in Ahmed's office at the PIU, Ministry of Health, Kaduna, as a platform to unite pharmacy graduates of:

- The old School of Pharmacy, Zaria (1933–1968)
- The Department of Pharmacy and Pharmacology, Faculty of Science, ABU
- The Faculty of Pharmaceutical Sciences, ABU, Zaria

Ahmed was the Interim National Chairman at inception. Under his leadership, sensitization campaigns were conducted among ABU Pharmacy graduates at the Pharmaceutical Society of Nigeria (PSN) conferences in Jos (1995) and Ilorin (1996). In March 1997, ABUPAA was formally inaugurated by Maj. Gen. M.T. Kontagora (Rtd.), the Sole Administrator of ABU, Zaria. Ahmed was confirmed as the Substantive National Chairman at the first AGM.

Key achievements under his leadership included:

- Complete overhaul of the faculty's plumbing system and installation of new water tanks.
- Renovation of the Dean's office with new furniture.
- Repainting of faculty walls and repairs of lecture theatre furniture.
- Lamination of all student records for preservation.

In March 2008, ABUPAA held another AGM and homecoming event chaired by Pharm. Danbaba Danfulani Suntai, Governor of Taraba State, with Prof. Gabriel E. Osuide, the foundation Dean, as Guest of Honour. Governor Suntai pledged to build twin 250-seater lecture theatres in honor of Prof. Osuide, a project completed in 2011 at the cost of ₦300 million. Other notable interventions included:

- Donation of a brand-new Peugeot 407 saloon car (₦3.7 million) for the Dean, the first such gesture in ABU's history.
- Enhanced faculty security, improved internet services and further facility renovations.

5.3 Barewa Old Boys' Association (BOBA): – (1970 Intake Class Group)

This Class Group was founded at Ahmed's residence in 1994 to foster camaraderie and fellowship among students admitted into Barewa College, Zaria in January 1970. His initiative inspired the formation of other BOBA Class Groups. Ahmed served as Chairman for eighteen (18) years until 2012, when he was unanimously elected Secretary

General of BOBA (National). Distinguished members of the Class Group include:

- His Eminence Alhaji Muhammad Sa'ad Abubakar, *CFR, mni*, Sultan of Sokoto;
- HRH Alhaji (Dr) Muhammad Abali Ibn Muhammadu Idrissa, Emir of Fika, Yobe State;
- HRM Chief Jonathan P. Zamuna, JP, *Ukurma* of Piriga Kingdom, Lere LGA, Kaduna State;
- HRH Alhaji Shehu Usman Ya'u Kauru, Emir of Kauru, Kauru LGA, Kaduna State;
- Prof. Abdulmumini Hassan Rafindadi, former CMD of ABUTH and first Vice-Chancellor, FUL, Lokoja (2011-2016);
- His Excellency Lt. Gen. A.B. Dambazau, *CFR*, former COAS and Minister of Interior and now Nigeria's Ambassador designate;
- Alh. Zakari Mohammed Sada, Chartered Accountant (current Chairman) of the class Group);
- Alhaji Tijjani Musa Isa (*Danmasanin Hadeja*), former ICAN President;
- Mallam Musa Tugga Karam (*Barden Piriga*), retired Federal Director and proprietor of Saminaka Hotels, Lere LGA, Kaduna State;
- Dr. Muhammad Ali Hussein, *mni*, Consultant Physician and former Permanent Secretary, Bauchi State
- Ambassador Haroun Umar, *mni (Dandarman Piriga)*;
- Engr. James H. Yaro (*Dallatun Piriga*) (retired Permanent Secretary, Kaduna State).
- Alh. Abdurrahman Mahmood (retired Permanent Secretary, Kaduna State)
- Hon. Musa Salisu Kafinsoli (former member, House of Representatives, Abuja).

Appendix Two highlights the contributions of the 1970 Class Group to tertiary education in Nigeria over the last 51 years.

5.4 Barewa Old Boys' Association (BOBA) – National

Ahmed's association with BOBA dates back to his admission to Barewa College, Zaria (1970–1974). His passion for the College has remained strong and he has actively participated in branch activities in Kano (1980–1984), Kaduna (1986–2002 & 2012–date), Zaria (1997–2003) and Abuja (2003–date). BOBA, originally founded in 1939 as the Katsina Old Students Association, has had a distinguished history. Ahmed's involvement at the national level allowed him to serve under the following Presidents

- Justice Muhammadu Bello, *GCON* (Retired *CJN*)
- Prof. B.D. Musa (2004–2008)
- Dr. Umaru Abdulmutallab, *CON* (2008–2017)
- Engr. Dahiru Ibrahim (2017–2025)

The current BOBA President from 20th September 2025 is Maj. Gen. AJ Kazir, *CFR*. Ahmed held several leadership roles in BOBA National, including:

1. Member, Barewa College Management Board (2003);
2. NEC Member (2002–2004);
3. Second Assistant Secretary General (2004–2012) (two terms);
4. Secretary General (2012–2021) (two terms);
5. NEC Member (2021–2025).

Key Innovations as Secretary General (2012–2021)

- Nationwide branch revival tours (2013) in Katsina, Zamfara, Kebbi, Sokoto, Plateau, Bauchi, Taraba, Adamawa and Kaduna.
- Hosting AGMs in Barewa College annually, with fundraising and infrastructure projects.

- Expansion of Annual Lecture, Dinner, and Awards beyond Zaria/Kaduna/Abuja to Dutse, Katsina, Sokoto and Kano.
- Production of no fewer than 23 BOBA publications (lecture series, newsletters, almanacs, fliers, scorecards, compendiums).
- Establishment of a BOBA Library at the National Secretariat.
- Sponsorship of teacher capacity-building workshops (2014, 2016) in collaboration with the NTI, Kaduna.

In October 2015, the BOBA Board of Trustees was reactivated and chaired by His Excellency, General Yakubu Gowon, *GCFR, PhD* (B783). Gowon's historic connection with Ahmed's late father, Mallam (Dr) Abdurrahman Mora, added depth to the occasion, as Gowon was once taught Geography by Mallam Mora in the 1950s at Kaduna College (later Barewa College).

6.0 West African Pharmaceutical Manufacturers Association (WAPMA)

In June 2000, the Centre for the Development of Enterprise (CDE), an EU initiative, invited some pharmaceutical manufacturers from West, South and Central Africa to a workshop in Douala, Cameroon. Ahmed was invited by virtue of being the Executive Chairman of Zazzau Pharmaceutical Industries Limited (ZPIL), Zaria. The primary objective was to enhance the technical capacities of pharmaceutical manufacturers and increase the production of essential medicines in Africa. Participants included representatives from Nigeria, Ghana, Senegal, Côte d'Ivoire, Zimbabwe, Niger, Benin Republic and Cameroon.

In December 2002, another meeting was convened at the Excellence Hotel, Ikeja, Nigeria by the same organization (then renamed the Centre for the Development of Industries (CDI). At this meeting, a committee was established with Ahmed as Chairman, tasked with

exploring ways of improving formal trade in pharmaceutical products within West Africa. The committee recommended the creation of the West African Pharmaceutical Manufacturers Association (WAPMA), with headquarters in Accra, Ghana. The recommendation was unanimously accepted and Ahmed was appointed the first Interim Chairman of WAPMA. He formally communicated the objectives of the new Association to manufacturers across Anglophone and Francophone countries through official letters written in English and French.

7.0 Ahmadu Bello University (ABU), Main Campus, Kongo Campus & ABUTH

Ahmed's passion and lifelong commitment to Ahmadu Bello University (ABU), Zaria and its affiliates have been exceptional. Since his admission into the School of Basic Studies for his A-Level programme in June 1974, he has maintained an unbroken connection with the University.

His major associations and roles with ABU over the years include:

- a) Advanced Level Student, SBS, Main Campus, Samaru (1974–1975);
- b) Pharmacy Undergraduate, Faculty of Pharmaceutical Sciences (1975–1978);
- c) Internship Training, ABUTH, Kaduna (1978–1979);
- d) MBA Student, Faculty of Administration, Kongo Campus (1983–1985);
- e) Assistant Secretary, ABU Alumni Association (Zaria Branch) (1983 & 1985);
- f) Part-time Lecturer, Faculty of Pharmaceutical Sciences (1986–date);
- g) Founding Member, Interim National Chairman, National Chairman, and later Chairman, Board of Trustees (BoT), ABU Pharmacy Alumni Association (1994–date);

- h) Part-time Lecturer in International Business, MBA Part-time Programme, Faculty of Administration, Kongo Campus (1998–2000).

Because of these long-standing contributions, Ahmed was invited in 2001 to serve on the committee that drafted the Constitution of the ABU Alumni Association.

He contested the presidency of the Association three times:

- In 2002, he lost narrowly to Mallam Abubakar Gimba.
- In 2010, he lost again to Princess Henrietta Ingo Ogan.
- In August 2015, he was unanimously elected National President on his third attempt. He completed two terms, serving until February 2021, a period of 66 months.

Key Achievements as National President (2015–2021)

7.1 Nationwide Tours & Branch Revitalization

He toured over 20 states between 2017 and 2018 (including Bauchi, Borno, Sokoto, Kano, Rivers, Anambra, Lagos and Abuja), holding interactive sessions that reactivated dormant state branches.

7.2 Infrastructure Development

- He built the National Secretariat of ABU Alumni Association using personal funds in the main campus, Samaru
- Secured the donation of an N80 million worth pedestrian bridge at ABU North Gate, built by H.E. Atiku Abubakar, *GCON*.

7.3 Student Support Initiatives

- Launched a Laptop Loan Scheme for students through a partnership with Omatek, Plc.
- Initiated an Intra-Campus Bus Shuttle Service, funded by H.E. Umaru Tanko Al-Makura (Governor of Nasarawa State) and supported by Hajiya Hadiza Bala Usman (MD, NPA), who donated two (2) brand new buses.

7.4 Hostel Development

Laid the foundation of a 104-unit postgraduate hostel at the Main Campus, with the SGF, Mr. Boss Mustapha, pledging further support.

7.5 Capacity Building

Sponsored the first-ever capacity-building workshop for ABU Deans and Directors in 2018, in collaboration with ASCON, Badagry.

7.6 Water Supply Project

Reactivated water supply to 420 staff houses in Kongo Campus funded by H.E. Ibrahim Hassan Dankwambo, Governor of Gombe State.

7.7 International Collaboration

Led a delegation to the USA in 2019, engaging with the New York City University Alumni Association and the MIT Alumni Association, Boston. The delegation also paid courtesy visits to top Nigerians in the UN, including Hajiya Amina Mohammed (UN Deputy Secretary-General) and H.E. Prof. Tijjani Muhammad Bande (UN General Assembly President, 74th Session).

7.8 High-Level Engagements

Paid courtesy visits to prominent Nigerians, including:

- H.E. Aisha Muhammadu Buhari (the First Lady of Nigeria);
- H.E. Atiku Abubakar, *GCON* (the former Vice President);
- Lt. Gen. T.Y. Danjuma, *GCON* (the former Minister of Defense);
- Asiwaju Bola Ahmed Tinubu (the former Governor of Lagos State, now President of Nigeria);
- HH Shehu Idris, *CFR* (the Emir of Zazzau);
- HRM Oba of Lagos and HRM *Alake of Egbaland*.

Legacy

Ahmed became the 4th elected National President of the ABU Alumni Association, after:

1. Mallam Abubakar Gimba (2002–2006);
2. Engr. Bunu Shereef Musa (2006–2010);
3. Princess Henrietta Ingo Ogan (2010–2015);

Others who served before 2002 included Arc. Fola Alade the Foundation President and Arc. Mohammed Abdullahi Dewu. The current President Worldwide of the ABU Alumni Association is Mallam Adamu Mamman Kontagora (*Barden Kontagora*).

In ABU's 63-year history, Ahmed's family has produced 45 alumni spanning children, grandchildren and great-grandchildren of the late Mallam Mora, the largest single-family representation in the University's history. Ahmed himself studied at ABU for his A-Levels (1974–1975), undergraduate Pharmacy (1975–1978), and MBA (1983–1985).

8.0 The Conference of the Alumni Associations of Nigerian Universities (CAANU) (Now Association of Nigerian Universities Alumni – (ANUA)

The Conference of the Alumni Associations of Nigerian Universities (CAANU) was established in approximately 1984 as the umbrella body for all alumni associations of Nigerian universities. Its primary objective was to provide a platform for peer evaluation and collaboration, particularly in supporting universities through alumni interventions. In its first two decades, however, CAANU struggled to achieve its core objectives. The structure adopted involved an elected National Executive Committee (NEC) under a National Chairman, with quarterly delegates' conferences hosted rotationally by different universities.

8.1 Ahmed's First Association with CAANU

Ahmed's first official engagement with CAANU came in April 2013, when the ABU Alumni Association hosted the first-quarter delegates' conference at the Main Campus, Samaru, Zaria. At the time, Ahmed was Deputy National President of the ABU Alumni Association and represented the National President, Princess Henrietta Ingo Ogan, who was away on official assignment. As the second most senior officer of the ABU Alumni NEC, Ahmed received the then Chief Richard Ohanagoro-led CAANU NEC and the delegates (two per university alumni association, as tradition required).

8.2 The 2016 Election

By 2016, cracks had emerged within CAANU's NEC. Tensions arose between the National Chairman, Chief Richard Oma Ahonaruogho (now *SAN*) (also the Worldwide President of the University of Benin Alumni Association) and the Secretary, who was backed by some members. At the ABU Delegates' Conference, these divisions became clear. At the National Convention held on 24th September 2016 at the Assembly Hall of Abubakar Tafawa Balewa University (ATBU), Bauchi, Ahmed was unanimously elected unopposed as National Chairman, following Chief Richard's tenure. His election was seen as a major endorsement of his leadership qualities, despite Chief Richard having spent personal resources in an attempt to stabilize the Association. Notably, a faction staged a walkout during Chief Richard's report, but there were still enough accredited delegates to conduct a free and fair election. The convention was graced by HRM *Igwe* (Dr.) Nnaemeka Alfred Ugochukwu Achebe, *CFR*, *nni*, the Obi of Onitsha and Chancellor of ABU, who delivered the keynote address.

8.3 Tenure and Re-Election

Ahmed served as National Chairman of CAANU from September 2016 to October 2018. At the 2018 National Convention, held on

October 13, 2018, at the Federal University of Agriculture, Makurdi (FUAM), he was re-elected after defeating the Secretary, who had served alongside him since 2016. His second tenure ran until December 2022, in line with CAANU's amended constitution.

8.4 Key Achievements as National Chairman (2016–2022)

a) Courtesy Visits

- Hon. Minister of Education, Mallam Adamu Adamu (27th October 2016), where CAANU requested:
 - Office accommodation in Abuja;
 - Inclusion of Alumni Presidents in University Governing Councils;
 - Allocation of land in Abuja for the CAANU Secretariat;
- Executive Secretary, National Universities Commission (NUC) – Prof. Abubakar Adamu Rasheed, *MFR, mni*;
- Executive Secretary, TETFund – Dr. Abdullahi Baffa Bichi;
- Several State Governors (Gombe, Nasarawa), Deputy Governors (Kaduna, Ebonyi), and SSGs (Edo, Cross River);
- Numerous Royal Fathers during delegates' conferences, including the Emirs of Bauchi, Zazzau, Lafia, the *Tor Tiv* (*Gboko*), the *Oba* of Benin, the *Ooni* of Ife, the *Obi* of Onitsha, *Alafin* of Oyo, among others.

b) Successful Organization of Delegates' Conferences

Ahmed successfully organized 17 delegates' conferences and conventions between 2017 and 2022 across various universities, including:

- i. University of Abuja (March 2017)

- ii. Igbinedion University, Okada (May 2017)
- iii. Baze University, Abuja (September 2017)
- iv. Ahmadu Bello University, Zaria (December 2017)
- v. Gombe State University (April 2018)
- vi. Ambrose Alli University, Ekpoma (July 2018)
- vii. FUAM, Makurdi (Convention – October 2018)
- viii. LAUTECH, Ogbomoso (December 2018)
- ix. FUTA, Akure (September 2019)
- x. University of Calabar (February 2020)
- xi. Nasarawa State University, Keffi (August 2020)
- xii. Kaduna State University (November 2020)
- xiii. Ebonyi State University, Abakaliki (March 2021)
- xiv. Nnamdi Azikiwe University, Awka (December 2021)
- xv. University of Ibadan (March 2022)
- xvi. Niger Delta University, Yenagoa (July 2022)
- xvii. Bayero University, Kano (Convention – December 2022)

c) Patrons of CAANU

Instituted the appointment and investiture of Patrons, including:

- HRH Alh. (Dr.) Rilwanu Suleiman Adamu, *CFR* – Emir of Bauchi (Northeast) (October 2020)
- HRH Justice Sidi Bage Muhammad I, JSC (Rtd), *CFR* Emir of Lafia (North Central) (November 2020)
- HH Amb. Mallam Ahmad Nuhu Bamalli, *CFR*, Emir of Zazzau (Northwest) (November 2020)

- HRM, *Igwe* (Dr) Nnaemeka Alfred Ugochukwu Achebe, *CFR*, *mni* Obi of Onitsha (December 2021)
- The *Oba* of Benin is yet to be decorated

d) National Secretariat

- Renovated and commissioned CAANU's National Secretariat at the Federal Ministry of Education Annex, CBD Abuja, with funding of ₦10 million from Gov. Abdullahi A. Sule of Nasarawa State. Commissioning took place on 25th November 2021.
- Furnished with support of Sen. Ahmed Babba Kaita (Katsina North), who donated ₦2 million.

e) Constitutional Amendments

- Extended NEC tenure to four years (renewable once).
- Created six Vice-Chairmen positions.
- Approved at the Delegates' Conference in AAU, Ekpoma (July 2018).

f) Regional Initiative

- Initiated the formation of the West African University Alumni Association (WAUAA), with support from the Association of Vice-Chancellors of Nigerian Universities (AVCNU).

g) Advocacy

- Presented a Memorandum on Sustainable Funding for Tertiary Education in Nigeria at:
 - A joint workshop by the Federal Ministries of Finance and Education, Presidential Villa, Abuja.
 - The 35th Annual Conference of AVCNU.
 - To the Senate Committee on Tertiary Education.

8.5 End of Tenure

At the December 2022 Convention held at Bayero University, Kano (BUK), Ahmed handed over the leadership of CAANU after a transparent election, in which Prof. Yakubu Aboki Ochefu emerged as the new National Chairman.

9.0 The Nigerian Association of Pharmacists and Pharmaceutical Scientists in the Americas (NAPPSA)

The Nigerian Association of Pharmacists and Pharmaceutical Scientists in the Americas (NAPPSA) was established by Nigerian pharmacists practicing in the United States and Canada to promote ethical and professional collaboration among themselves. Ahmed's involvement with NAPPSA began right from its inception. In May 2007, two U.S.-based Nigerian pharmacists, Nnodum Iheme and Jonathan Obiagwu, visited his office at the Pharmacists Council of Nigeria (PCN), where he was serving as Registrar and Chief Executive Officer at the time. They introduced themselves as officials of NAPPSA and explained the association's objectives. The PCN fully supported NAPPSA in its mission to foster collaboration and to educate Nigerian pharmacists in diaspora on the requirements for registration and licensure should they wish to return home to practice. The PCN law provided for a six-week intensive training programme, known as the Foreign Pharmacy Graduate Orientation Programme (FPGOP), conducted in approved Faculties of Pharmaceutical Sciences in Nigeria. As Registrar of the PCN, Ahmed was invited to present position papers at the inaugural Annual Scientific Conference and Exposition of NAPPSA, held in Houston, Texas, in September 2007.

Participation in NAPPSA Conferences

Subsequently, Ahmed attended the Association's annual scientific conferences in the following U.S. cities:

- Houston (2007)
- Atlanta (2008)

- Bethesda (2009)
- Los Angeles (2011)
- Chicago (2012)
- Philadelphia (2013)
- Orlando (2014)
- Detroit (2015)
- Las Vegas (2016) – Joint conference with the Association of Nigerian Physicians in the Americas (ANPA)
- Houston (2017) – 10th Anniversary
- Charlotte (2018)
- Baltimore (2021)
- Tampa (2022)
- Columbus (2023)
- San Antonio (2024)
- Chicago (2025)

Appointment to the Board of NAPPSA

In 2017, Ahmed was appointed to the Board of NAPPSA, becoming the first non-U.S.-based Nigerian pharmacist to hold such a position. That year, he attended two Board meetings in Houston, Texas (May and September), and in 2018, he attended two more in Charlotte, North Carolina (May and September).

Missed Conferences

Ahmed missed only two conferences:

- Dallas (2010) – when he was a participant in the Senior Executive Course (SEC) at the Nigerian Institute for Policy and Strategic Studies (NIPSS), Kuru, near Jos.
- Atlanta (2019) – due to delays in obtaining a U.S. entry visa.

Professional Standing and Collaboration

Ahmed remains in good standing with all past and present Presidents of NAPPSA. Over time, he worked closely with the Association both as Registrar/CEO of the PCN (2007–2012) and later as Chairman of

the Governing Council of the Pharmacists Council of Nigeria (PCN) (2020–2023) until the dissolution of Federal Government Boards and Councils in June 2023. This enduring relationship with NAPPSA highlights Ahmed’s commitment to strengthening professional ties between Nigerian pharmacists at home and in the diaspora, while advancing the practice of pharmacy globally.

10.0 Member of the Nigerian Medical Team for the Care of Nigerian Muslims in Saudi Arabia During the Hajj Exercise

The Government of the Federal Republic of Nigeria, through what is now known as the National Hajj Commission of Nigeria (NAHCOM), has over the years constituted teams of healthcare professionals, including medical doctors, pharmacists, nurses and other allied health workers, to travel to the Kingdom of Saudi Arabia during the annual Hajj pilgrimage to provide medical care to Nigerian pilgrims. Ahmed was first appointed as a member of the Nigerian Medical Team to Saudi Arabia in 1989, when he served as Chief Pharmacist at the Headquarters of the Ministry of Health, Kaduna State. He was subsequently reappointed to serve in the same capacity in 1992, 1995 and 2000. As a pharmacist, Ahmed rendered professional pharmaceutical services to Nigerian pilgrims and occasionally to pilgrims from other countries across the following locations:

1. **Jeddah:**
At the airport, where he attended to arriving and departing Nigerian pilgrims.
2. **Makkah:**
During his period of service, three (3) clinics operated under the Nigerian Medical Team, located in:
 - Jarwal District
 - Al-Misfalah District
 - Ajyad District

3. **Madinah:**
Providing care to pilgrims visiting the Prophet's Mosque and other holy sites.
4. **Mina:**
Offering pharmaceutical services during the three-day stay of pilgrims at Mina.
5. **Mount Arafat:**
Attending to pilgrims on the 9th day of the Islamic month of *Dhul-Hijjah*, the climax of the Hajj rites.
6. Through his participation in these missions, Ahmed gained invaluable international pharmaceutical experience, particularly in large-scale medical logistics, cross-cultural healthcare delivery and emergency response under unique and demanding conditions.

11.0 Honors, Awards and Fellowships

Ahmed has received almost 100 fellowships, honors, awards and recognitions from learned organizations and institutes over the last thirty (30) years:

1. Fellow, Pharmaceutical Society of Nigeria (FPSN) (1997).
2. Fellow and Industrial Leadership Award, Institute of Corporate Administration of Nigeria (1998).
3. Fellow, Institute of Corporate Executives of Nigeria (FICEN) (1998).
4. Fellow, Institute of Healthcare Management, Nigeria (FIHMN) (2012).
5. Fellow, Nigerian Institute of Management (Chartered) (FNIM) (2017).
6. Benefactor Award, Faculty of Pharmaceutical Sciences, ABU Zaria (2004).
7. Labour-Friendly Award, Nigerian Labour Congress (NLC), Abuja Branch (2005).

8. Eminent Persons Award, Nigerian Association of Industrial Pharmacists (NAIP) (1998).
9. Pharmaceutical Leadership Award as Africa's Pharmacist Executive of the Year (2009), conferred by African Leadership Magazine, Paris (2010).
10. Most Distinguished Alumnus, Faculty of Pharmaceutical Sciences, ABU Zaria (2012).
11. Traditional Title of *Wakilin Maganin Zazzau*, conferred by the Zazzau Emirate Council, Zaria, under His Highness, the Emir of Zazzau, Alhaji Shehu Idris, *CFR* (2013).
12. Traditional Title of *Iyan Piriga*, conferred by the Piriga Kingdom, Lere LGA, Kaduna State, by HRM Chief Jonathan Zamuna (*Ukurma Piriga*), 1st December 2018.
13. Meritorious Award as Grand Patron, Pharmacists Council of Nigeria Multipurpose Cooperative Society (PCNSMCS), 28th March 2018.
14. Icon of Societal Development Award, National Association of Northern Nigerian Students (NANNS) (2023).
15. Corporate Merit Award, African Students Union Parliament (2020).
16. Award of Recognition, Nigerian Association of Pharmacists in Academia (NAPA), (Kaduna State University Chapter), for invaluable contributions to the establishment and development of the Faculty of Pharmaceutical Sciences (11th October 2021).
17. Award of Excellence, National Association of Kaduna State Pharmacy Students (NAKAPS) (National Body), for Outstanding Contributions to Pharmacy Education in Nigeria through Meaningful and Effective Leadership (2020).
18. Keynote Speaker Award, Pharm. D. S. Wali Book Project Committee, at the 8th Public Lecture in Honour of the Late Pharm. Dalim Suleiman Wali, FPSN – Lecture on “*The Teaching and Practice of Pharmaceutical Care in Nigeria: The Perspective of a Practitioner*” (23rd July 2022).

19. National Honour – *Officer of the Order of the Niger (OON)*, Federal Republic of Nigeria (May 2023).
20. Award of Excellence, Pharmaceutical Association of Nigeria Students (PANS), (Kaduna State University Chapter), for Selfless Service to Humanity and Immense Contribution to the Growth of Pharmacy (22nd August 2019).
21. Individual Award for National Development, Ahmadu Bello University Students' Representative Council (ABUSRC) (8th November 2014).
22. Award of Appreciation, Newcare Home Health Services Inc. Baltimore, USA, for Dedication and Commitment to Excellence in Pharmacy Practice (26th February 2005).
23. Fellowship Award, Nigerian Institute of Management (Chartered), for Management Excellence (20th August 2017).
24. Award of Excellence, presented by Pharm. E. V. Ekwu (Chairman, PSN Rivers State), in recognition of service as Registrar, Pharmacists Council of Nigeria (PCN).
25. Award of Recognition, Barewa Old Boys Association (BOBA), for multiple achievements: election as BOBA Secretary-General, turbaning as *Wakilin Maganin Zazzau*, appointment as Pioneer Dean of the Faculty of Pharmaceutical Sciences at KASU, and election as National President of ABU Alumni Association (6th November 2015).
26. Traditional Title of *K'ayayen Sarkin Musulmi*, conferred by the Sokoto Sultanate Council under His Eminence, Alhaji Muhammad Sa'ad Abubakar, *CFR, mni*, Sultan of Sokoto (4th April 2021).
27. Merit Award, Ahmadu Bello University Pharmacy Alumni Association (ABUPAA), for Excellent and Dedicated Service to the Pharmacy Profession (July 2004).
28. Distinguished Pharmacist & Excellent Leadership Award, Pharmaceutical Association of Nigeria Students (PANS) National Secretariat (11th December 2006).

29. Merit Award, Pharmaceutical Society of Nigeria (Ebonyi State Chapter), for Leadership and Contributions to Pharmacy Practice (6th October 2008).
30. Award of Recognition, BOBA, on election as National President, ABU Alumni Association, at the 2015 BOBA Lecture and Dinner (10th October 2015).
31. Merit Award, Pharmaceutical Society of Nigeria (Pharmacy Week, Abuja 2006), for Selfless and Meritorious Service to Humanity and the Profession.
32. Award of Honour, Nigerian Association of Hospital & Administrative Pharmacists (NAHAP), Enugu State, in recognition of contributions to Hospital and Administrative Pharmacy (19th October 2007).
33. Merit Award, NAHAP (NAUTH, Nnewi), for Service to Pharmacy and Humanity (7th October 2008).
34. Award, by the Zaria Education Development Association (ZEDA), for Contributions to Education through the PIFSS Scholarship Programme, presented at *ZEDA @ 20 Anniversary* (8th January 2012).
35. Patronship Award, Pharmaceutical Association of Nigeria Students (PANS), (National), ABU Zaria (28th May 1997).
36. Merit Award, Pharmaceutical Society of Nigeria (Kaduna State Branch), for Dedicated and Consistent Service to the Pharmacy Profession (July 1994).
37. Award of Excellence, Tudun Wada Elders Association, Zaria, for Professional Attainment and Contributions to Humanity (31st January 2010).
38. Appreciation Award, Committee of Registrars of Health Professions (CRHP), at the 2nd Advocacy Workshop on *“Harmony in the Healthcare Delivery System: The Role of Health Regulatory Agencies”* (31st March 2011).

39. Award, Pharmacists Council of Nigeria, for Service as Chairman, Pension Scheme Board of Trustees (October 2003 – December 2011).
40. Award of Honour, PANS (ABU Zaria), for Contributions to the Growth of the Association (2005/2006).
41. Eminent Person Award, Nigerian Association of Industrial Pharmacists (NAIP), for Upliftment of Industrial Pharmacy Practice (1998).
42. Award of Honour, Zazzau English Attractive Lesson (ZEAL), Sarki Sambo Primary School, Zaria City, for Outstanding Contributions to Education (17th April 2016).
43. Award of Excellence, BOBA (Funtua Branch), in recognition of service as Secretary-General, BOBA (2013).
44. Gold Award, Corporate & Media Africa Communications (D'LINGA), for Distinguished Leadership in National Development (13th May 2006).
45. Distinguished Benefactor Award, Faculty of Pharmaceutical Sciences, ABU Zaria, in recognition of numerous contributions to the Faculty (14th May 2004).
46. Award of Honour and Recognition, PANS (ABU Zaria), from Students and Staff of the Faculty of Pharmaceutical Sciences (10th August 2000).
47. Recognition Award, HMA Medical Ltd, Ilorin, for commitment to industrialisation during commissioning of facilities (March 2022).
48. Meritorious Award, Pharmacists Council of Nigeria Staff Multi-Purpose Cooperative Society (PCNSMCS), for Initiating, Nurturing, and Growing the Society as Grand Patron (28th March 2018).

49. Congratulatory Award by Kwara Chemical co. Ltd, Ilorin, Kwara State, Ltd, Ilorin *“May the Almighty Allah grant you sound health and wisdom that will spur you to greater heights”* on 15th June, 2020.
50. Merit Award Presented by NIM (CHARTERED) ZARIA CHAPTER *“In honour of your Election as a Fellow of the NIGERIAN INSTITUTE OF MANAGEMENT (CHARTERED), (2018).*
51. Award of Honour Presented by the FACULTY OF PHARM. SCIENCES, ABU, ZARIA:
In recognition of your outstanding contributions to the Faculty and Pharmacy Profession in Nigeria (1st December 2016).
52. Certificate of Merit award by PANS (ABU) PHARMACEUTICAL ASSOCIATION OF NIGERIAN STUDENTS (PANS) on 17th day of June '99 in conferment of His/Her Noble Role as PATRON: *(PRESIDENT: DUKE DANIELS, DEAN: DR. E.M. ABDURAHMAN, SEC. GEN FOLAKE R. ODEDIRAN);*
53. Honorary Membership Certificate of Honour Award by PHARMACEUTICAL ASSOCIATION OF NIGERIAN STUDENTS (PANS) (Ahmadu Bello University, Zaria), Nigeria for *Very satisfactory and Excellent Service to the Association as ADVISER AND MORAL SUPPORTER by: President PANS, Dean FPS/ABU (PANS) PATRON, 1989/1990.*
54. Presented Award of Honour by KASU MAJLIS WELFARE INITIATIVE KADUNA STATE UNIVERSITY *“In recognition of your invaluable contributions to KASU MAJLIS” (2nd March, 2017).*
55. Presented the Award of Honour by PROF. SAMUEL G. EDOUMIEKUMO VICE CHANCELLOR NDU, Yanagaoa (July 2022).
56. Award of Honour presented by KADUNA STATE UNIVERSITY, *Compliments of PROF. MUHAMMAD TANKO Vice Chancellor, Kaduna State University (November, 2020)*

57. Award of Honour by NASARAWA STATE UNIVERSITY, KEFFI *With the Compliments of the Vice Chancellor, Nasarawa State University, Keffi, Nasarawa State, Nigeria, (August 2020);*
58. Congratulatory Award on Appointment as Chairman of PHARMACISTS COUNCIL OF NIGERIA – by - NIGERIA ASSOCIATION OF FOREIGN TRAINED PHARMACISTS
We believe that under your great Leadership, Wisdom, and Guidance Pharmacists Council of Nigeria will strive to greater heights on (July 20th, 2020)
Present were Dr. John C. Ejezie (President), Pharm. Faith O. Ayosin (Secretary General).
59. Outstanding Service Award Presented by NIGERIAN ASSOCIATION OF PHARMACISTS AND PHARMACEUTICAL SCIENTISTS IN THE AMERICAS (NAPPSA) 2013 NAPPSA Annual Scientific Conference and Exposition “*In Appreciation of years of support and contribution to the Mission of NAPPSA*” (September 19 – 22, 2013);
60. Merit Award presented by NIGERIA ASSOCIATION OF FOREIGN TRAINED PHARMACISTS (NAFTraPh) “*In Recognition of your Dedication and Contributions to the Profession and Humanity*” *Congratulations*, 11th October, 2019 (present were: Dr. John C. Ejezie (President), Pharm. Faith Ayosin (Secretary General).
61. Award presented by PHARM. D. S. WALI, FPSN, BOOK PROJECT COMMITTEE ‘*On the occasion of the 8th in the series of Public Lectures in Honour of the late Pharm. Dahiru Suleiman Wali, FPSN on the topic “THE TEACHING AND PRACTICE OF PHARMACEUTICAL CARE IN NIGERIA: THE PERSPECTIVE OF A PRACTITIONER” (Saturday, 23rd July, 2022).*
62. Award of Excellence Presented by NATIONAL ASSOCIATION OF KADUNA STATE PHARMACY STUDENTS (NAKAPS) NATIONAL BODY “*As Icon of Pharmacy*

Profession and Good Governance in Recognition of your outstanding contribution to the Development of Pharmacy Profession in Nigeria Through meaningful and Effective Leadership NAKAPS 2020.

63. Award of Meritorious Service & Selfless Dedication presented by PHARMACEUTICAL SOCIETY OF NIGERIA (PSN) (NIGER STATE CHAPTER) *"In Recognition of your leadership role and quality contributions in uplifting the Pharmacy Profession"* (11th January, 2023).
64. Award of Honour Presented by FEDERAL COLLEGE OF EDUCATION ALUMNI ASSOCIATION National Headquarters Kontagora, Niger State *"In Recognition of your unrelentless Services to the entire worldwide Alumni Association and a well-detailed paper presentation on the role of Alumni Association in Nation Building."*
65. Award by PHARMACISTS COUNCIL OF NIGERIA in collaboration with WEST AFRICAN POST-GRADUATE COLLEGE OF PHARMACISTS (NIGERIA CHAPTER) *"In appreciation of your contribution as a lecturer presented on the Topic "Effective Implementation of Pharmacy Consultant cadre for Improved Healthcare Delivery in Nigeria (May 18, 2021).*
66. Award presented by PHARMACEUTICAL ASSOCIATION OF NIGERIAN STUDENTS (PANS) (ABU CHAPTER) *"Having been admitted into the Hall of Fame of PANS (ABU CHAPTER) at the PA Ishaku Lecture Theatre, Faculty of Pharmaceutical Sciences, ABU, Zaria*
67. Award of Excellence presented by the ASSOCIATION OF HOSPITAL AND ADMINISTRATIVE PHARMACISTS OF NIGERIA (AHAPN), ENUGU STATE.
"In recognition of the outstanding roles played in the actualisation of the Consultant Pharmacy cadre" (24th June, 2021).
68. PAN-AFRICAN LEADERSHIP PRIZE FOR EXCELLENCE Presented by AFRICAN STUDENTS UNION PARLIAMENT

- (ASUP) *“For HARD WORK AND DEDICATION TO NATIONAL SERVICE: This is in recognition and appreciation of your hard work and dedication to service, selfless leadership, ideology, high administrative prudence, human capital development and dedicated service to God, Humanity and country (2021).*
69. AFRICAN LEADERSHIP AWARD, 2009 as *“African Pharmaceutical Executive of the year 2009”* (20th February, 2010) (Paris, France);
 70. Award of Outstanding Support Presented by MUSLIM PHARMACISTS ASSOCIATION OF NIGERIA (MPAN).
 71. Award of Outstanding Achievement in Leadership presented by PHARMACEUTICAL SOCIETY OF NIGERIA (PSN) ENUGU STATE BRANCH) *“In recognition of your commitment and meritorious service to the Pharmacy Profession”* (24th June, 2021).
 72. Award of Golden Leadership by PHARMACEUTICAL ASSOCIATION OF NIGERIA (PANS) Ahmadu Bello University, Zaria (2016/2017 Session) *“In appreciation of your unwavering support and unrelentless guidance towards our path to success. You are honoured with much gratitude*
 73. Appreciation Award Presented by PROFESSOR AZUKA C. OPARAH, Dean, Faculty of Pharmacy, UNIBEN *“As the Special Guest of the 46th Induction Ceremony of the 2020 / 2021 Pharmacy Graduands”* (March 17, 2023).
 74. Presented Award of Honour with compliments by NATIONAL AGENCY FOR FOOD AND DRUG ADMINISTRATION AND CONTROL (NAFDAC) (2020).
 75. ZARIA EDUCATION DEVELOPMENT ASSOCIATION (ZEDA): This Plaque is presented to: Prof. Ahmed T. Mora, FPSN, mni (K'ayayen Sarkin Musulmi) (Wakilin Maganin Zazzau) as GUEST SPEAKER for PRE-AGM LUNCHEON

- this 2nd day of November, 2022 at the Stonehedge Hotel, Behind New Nigeria Newspaper, Bank Road, Kaduna
76. Appreciation Award to PROF AHMED TIJJANI MORA, *mni* (*K'ayayen Sarkin Musulmi*) MY TEACHER, A MENTOR AND THE CHAIRMAN OF THE PCN GOVERNING COUNCIL (2020-DATE). Courtesy: Pharm Ibrahim Babashehu Ahmed, Registrar/CEO, Pharmacy Council of Nigeria, Given at Bida, (November 2023).
 77. Award as FRIEND OF THE STATE. By the NASARAWA STATE GOVERNMENT, NIGERIA
 78. Award of Honour of Dedication and Honesty as PHARMACY CHAMPION: In recognition of immense appreciation of your unwavering and unequivocal support towards the growth of the Pharmaceutical Association of Nigeria Students, ABU Zaria and the Pharmacy Profession in Nigeria in August 2023
 79. Distinguished Alumni Service Award: Presented to Prof. Ahmed Tijjani Mora, *OON, mni*, for distinguished and outstanding service to the association, the University and the Nation at large. By ABU Alumni Association (National), November 26, 2024.
 80. Award of Honour: Presented to Prof Ahmed Tijjani Mora in recognition of your Exceptional Leadership Qualities, Remarkable Contribution to the Society and Nation and large Congratulations. By - Baba Ahmed Primary School Old Students' Association (BAPSOSA) (2024).
 81. Icon of Pharmacy Award: Presented to Prof Ahmed Tijjani Mora, *FPSN, mni*, In Recognition of your immense contribution to Pharmacy Practice and Profession – NAIP (KANO CHAPTER) (19th May 2022).
 82. PHARMACEUTICAL ASSOCIATION OF NIGERIAN STUDENTS (Igbinedion University, Okada) presents this “AWARD OF APPRECIATION” Prof. Ahmed Tijjani Mora, *FPSN, FPCPharm, FICEN, PhD, FNIN, OON, mni*, “For your unwavering dedication, tireless efforts, and ultimate sacrifice to

the 400L class throughout the industrial visits. This 22nd day of May, 2025, congratulations.

83. PANS (ABU, Zaria) presented LIFETIME ACHIEVEMENT AWARD to Professor Ahmed Tijjani Mora, *OON, mni (K'ayayen Sarking Musulmi, Wakilin Maganin Zazzau)*, LIFE PATRON, PANS (ABU, ZARIA): This Award stands as a testament to the positive change you have championed and the legacy you continue to build on this (23rd August, 2023).

12.0 The Family of Prof. Ahmed T. Mora

Ahmed T. Mora has a very stable family life consisting of Hajiya Aisha Uwa from Talatan Mafara, Zamfara State (Married in August 1981 and Hajiya Asiya Suleiman from Dogon Bauchi, Zaria (April 1993). He is blessed with (10) children. Those who have completed university are listed in the tabulation below:

S/N	Name	University / Education	Postgraduate / Other Certificates	Present Position
1	Imran Ahmed Mora, <i>CFA</i>	Sidi Mohamed Ben Abdallah University, Fes, Morocco	MSc International Business, Coventry University, England; Chartered Financial Analyst (CFA)	Manager, Securities and Exchange Commission (SEC), Abuja
2	Arc. Hamza Ahmed Mora	Ecole Nationale d'Architecture, Rabat, Morocco	PG Diploma in Education (PGDE); MSc Architecture; Diploma in French Language	Senior Lecturer (SL), Kaduna Polytechnic, Kaduna
3	Barr. Zubaida Mora	Ahmadu Bello University, Zaria (LLB); Nigerian Law School, Bwari, Abuja (BL); Kaduna State University (Master's in Criminal Justice)	Member, Chartered Institute of Mediators and Conciliators	Legal Unit KASU Kaduna
4	Barr. Faizah Mora	Ahmadu Bello University, Zaria (LLB); Nigerian Law School, Bwari, Abuja (BL); Nasarawa State University, Keffi	PGD in Alternative Dispute Resolution (ADR)	Legal Unit NTI, Kaduna

S/N	Name	University / Education	Postgraduate / Other Certificates	Present Position
5	Yasmin Mora	Ahmadu Bello University, Zaria (B.A. Industrial Design);	MBA (KASU), Kaduna	Administrative Officer, Kaduna State University (KASU), Kaduna.
6	Hauwa Mora	Ahmadu Bello University, Zaria (BSc Accounting);	PGD in Accounting and Finance (ABU, Zaria)	Internal Audit Officer, Jaiz Bank, Abuja.
7	Maryam Mora	Ahmadu Bello University, Zaria (BSc Anatomy);	MPH (Nile University Abuja)	Branch Supervisory Manager, Medicaid Radio Diagnostics and Clinics
8	Pharm. Asiya Mora	Ahmadu Bello University, Zaria (B. Pharm)	Internship training at PCN Zonal Office, Kaduna	NYSC Pharmacist Gen. Hospital Asokoro, Abuja

- Fatima Mora and Muhammad Mora are yet to complete their studies

13.0 Board and Council Appointments (In Alphabetical Order)

Ahmed has served, or continues to serve, as a member of the following Boards and Councils, over the past thirty (30) years:

- 1) Ahmadu Bello University Governing Council — Member, Governing Council (August 2015 – February 2021).
- 2) Ahmadu Bello University Pharmacy Alumni Association (ABUPAA) — Chairman, Board of Trustees (since 2010).
- 3) Association of Nigerian Universities Alumni (ANUA) — Board Member (since 2024).
- 4) Badawiyya Pharmaceutical Plant Project — Consultant (since 2021).
- 5) Committee on the Book Project of Flagbearers of the Sokoto Caliphate Book Project— Chairman of Committee (since 2022).
- 6) Global Public Health University (GPHU) (ONLINE University) - Chairman of the Board (August 2025).

- 7) Igbinedion University Governing Council — Member (since April, 2022); Pro-Chancellor and Chairman of the Governing Council (since July 2025).
- 8) Igbinedion University Teaching Hospital (IUTH), Okada, Edo State — Member, Board of Management (2020–2022).
- 9) Institute of Human Virology, Nigeria (IHVN) — Board Member (since January 2025).
- 10) Nigerian Association of Pharmacists and Pharmaceutical Scientists in the Americas (NAPPSA) — Member (since 2007); Member of the Board (2017–2020).
- 11) Peoples Assembly of Northern Kaduna Initiative for Peace and Harmony (Chairman, 2025)
- 12) Pharmaceutical Association of Nigerian Students (PANS), ABU Chapter — Grand Patron (circa 2000) and Life Patron (2025).
- 13) Pharmaceutical Association of Nigerian Students (PANS), KASU Chapter — Grand Patron (circa 2012);
- 14) Pharmacy Scholars Initiative (PSI), Grand Patron (Since 2017).
- 15) The Abdurrahman Mora Educational Foundation (TAMEF) — Board Member (since 2012).
- 16) The Pharm. D.S. Wali, FPSN Book Project Committee — Chairman of Committee (since 2011).
- 17) Tudun Wada Zaria Elders Association member (Since 2007)
- 18) Yagaga Pharmaceutical Ltd, Ilupeju, Lagos (a logistic drug company (Chairman of the Board, May 2025).
- 19) Zaria Education Development Association (ZEDA) — Member, Board of Trustees (since 2011).
- 20) Zaria Pharmaceutical Company Ltd (ZPC), Zaria — Member, Board of Directors; Managing Director and CEO (1997–1999).
- 21) Zazzau Emirate Community Advancement Foundation (ZECAF) — Member Steering Committee (since 2022).

- 22) Zazzau Professors Forum (ZPF) (Since 2023 and as Vice-Chairman 1)
- 23) Arewa Consultative Forum (ACF) member of BoT (since 2024)

14.0 Travels and Papers Presented

Ahmed is widely travelled, having visited all the state capitals in Nigeria and 88 cities across 33 countries. Most of these trips were to attend pharmaceutical conferences, seminars and workshops, numbering over 400 since 1988. He has delivered more than 140 presentations at these events, either as a sole lecturer, a keynote speaker or a symposium panelist on topics related to pharmacy and university alumni associations. A list of some of the scientific papers presented is itemized in the last section of the book.

15.0 Chairman, D.S. Wali Book Project Committee

(a) Origins and Purpose

The D.S. Wali Book Project Committee was established to honour the legacy of Pharm. Dahiru Suleiman Wali, *FPSN* (1935–2020), who was a pioneering figure in Community Pharmacy practice in Nigeria. In 2015, on what would have been his 80th birthday (July, 23), the committee organized a public lecture in his memory. This marked the beginning of the enduring Pharm. D.S. Wali Annual Lecture Series, held every year on or around 23rd July. The committee has also produced a biography-cum-textbook, titled “*D.S. Wali: An Icon of Community Pharmacy Practice in Nigeria*,” authored by five members of the committee.

(b) Composition and Key Members

As of the latest update, the committee comprises the following distinguished members:

- Prof. Ahmed Tijjani Mora — Chairman
- Pharm. Aliyu Jibrin, *FPSN* — Secretary
- Prof. Ibrahim Adamu Yakasai — Bayero University, Kano
- Prof. Umar Usman Pateh — Ahmadu Bello University, Zaria

- Pharm. Ahmed Gana Mohammed — Aminu Kano Teaching Hospital, Kano

(c) Activities and Achievements

Annual Lectures: Since 2015, the committee has hosted lectures delivered by renowned academics and professionals:

S/NO	YEAR	CHAIRMEN	TOPIC	VENUE
1.	2015	Prof. U.U. Patch	<i>“Pharmaceutical Care in Community Pharmacy Practice: The Way Forward”</i>	Dialogue Institute, Kaduna
2.	2016	Prof. H.A. Kaita	<i>“Food Additives and Household Chemicals Exposure: Pharmaceutical Information, Education and Counselling.”</i>	Arewa House, Kaduna.
3.	2017	Prof. Magaji Garba	<i>“Exploits of a Pharmaceutical Chemist in Pharmacy Practice (Drug–Drug Interaction).”</i>	Arewa House, Kaduna
4.	2018	Prof. E.M. Abdurrahman	<i>“The Role of Pharmacists in the Campaign Against Drug Abuse: Responsibility of the Pharmacists as Sole Custodians of Medicines.”</i>	Arewa House, Kaduna
5.	2019	Prof. Ibrahim Adamu Yakasai	<i>“Strict Adherence to Ethical Pharmacy Practice: The Responsibility of the Pharmacist as Sole Custodian of Medicines.”</i>	Arewa House, Kaduna
6.	2020	Pharm. Aliyu Jibrin	<i>“Rational use of Medicines, Self-Medication and the Pharmacist.”</i>	Dialogue Institute, Kaduna.
7	2021	Pharm. Ahmed Gana Mohammed	<i>“Investment Opportunities in the Pharmaceutical</i>	Kaduna State University

			<i>Industry: Partnership Between Pharmacists and the Public."</i>	(KASU), Kaduna.
8.	2022	Prof. Ahmed Tijjani Mora, <i>nni</i>	<i>"The Teaching and Practice of Pharmaceutical Care in Nigeria: The Perspective of a Practitioner."</i>	Dialogue Institute Kaduna.
9.	2023	Prof. Mrs. M.N. Femi- Oyewo, <i>MFR</i>	<i>"Evolution of Pharmacy Practice Across the Globe: The Product to the Patient. "</i>	1000-Seater Capacity Lecture Theatre, KASU, Kaduna.
10	2024	Prof. (Mrs.) Hajara Ibrahim	<i>"The Use of Herbal Medicine in Pharmacy Practice. "</i>	1000-Seater Capacity Lecture Theatre, KASU, Kaduna.
11	2025	Prof. Ibrahim Iliya	<i>"Beyond Basic Nutrition: How Bioactive Compounds in Foods Promote Healing. "</i>	Bayero University, Kano

Book Donation Legacy: Earlier, Pharm. Wali donated half of his personal collection of pharmacy-related books to Ahmadu Bello University (ABU), Zaria and Kaduna State University (KASU), Kaduna. The Committee has since reinforced this educational legacy through further book donations and knowledge-sharing initiatives.

16.0 Who is the Most Renowned Pharmacist in Nigeria?

Nigeria has produced several highly respected pharmacists across academia, industry, regulation and public service. "Renowned" may mean influence, recognition, leadership or global impact. Notable names include:

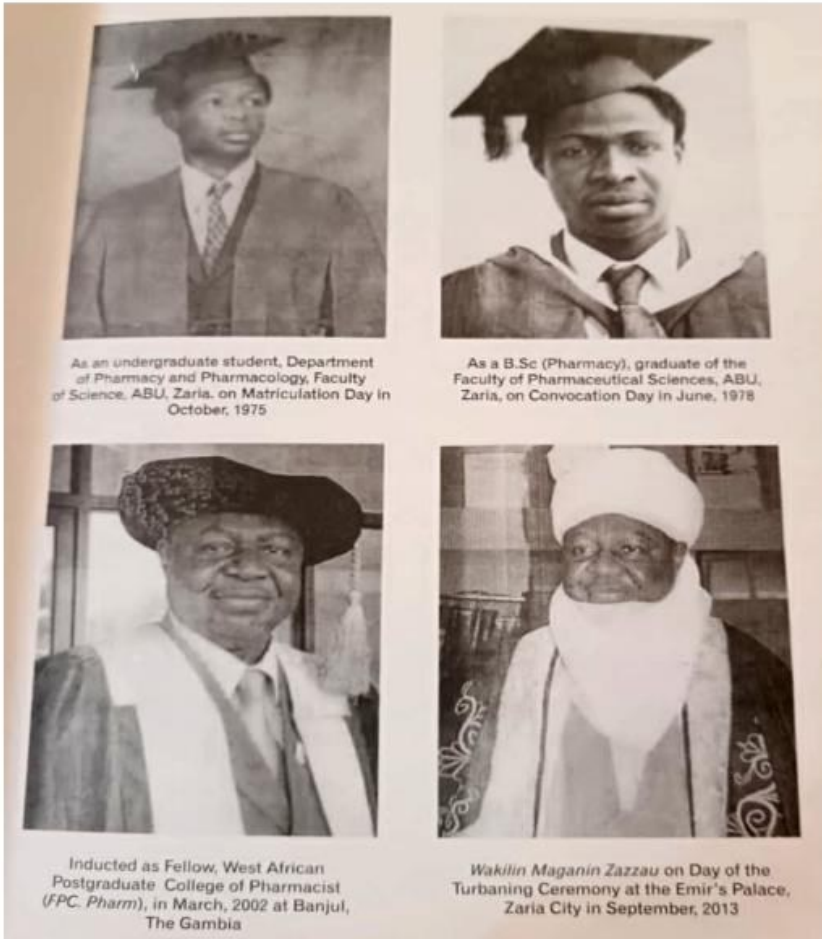
1. **Prof. Ahmed Tijjani Mora**

- The former Registrar, Pharmacists Council of Nigeria (PCN).
- The former Chairman, West African Pharmaceutical Manufacturers Association (WAPMA).

- The pioneer Dean, Faculty of Pharmaceutical Sciences, Kaduna State University; first Head, Department of Clinical Pharmacy and Pharmacy Management. Lecturer in Pharmacy in ABU, Zaria and Igbinedion University, Okada (IUO) Okada, Edo State.
 - The former Chairman, Pharmacists/Pharmacy Council of Nigeria (PCN);
 - Distinguished for contributions to pharmacy education, regulation, and policy development.
 - Author of the first book on Drug Distribution Nigeria-*“The Lizard Shape Model in Drug Distribution in Nigeria. (2014, 2018)”*
2. **Prof. Dora Akunyili (Late)**
- The former Director-General, NAFDAC.
 - Internationally acclaimed for her anti-counterfeit medicines campaign.
 - Recipient of numerous global public health awards.
3. **Prof. Chinedum Peace Babalola**
- The first female pharmacist Vice-Chancellor in Nigeria (Chrisland University).
 - Renowned researcher in pharmacokinetics and clinical pharmacy.
 - Recipient of multiple scientific excellence awards.
4. **Pharm. Sam Oluwabunwa**
- The former MD/CEO, Pfizer Nigeria and Neimeth International Pharmaceuticals.
 - The past President, Pharmaceutical Society of Nigeria (PSN).

- The advocate for Industrial Pharmacy and Public Policy.
- 5. **Prof. Isa Marte Hussaini (Ahmed's schoolmate in pharmacy school and bosom friend)**
 - Neuroscientist and pharmacist, specializing in cancer research from African medicinal plants.
 - Fellow of several international scientific academies.
 - Significant contributions to pharmaceutical research in Nigeria.

The milestone achievements and enduring legacies of Prof. Ahmed Tijjani Mora stand as a testament to his visionary leadership, selfless service and unwavering commitment to the advancement of pharmacy education, regulation and practice in Nigeria. His pioneering roles as an academic, administrator, regulator and nation builder have shaped generations of pharmacists and strengthened the institutions that safeguard public health. His contributions remain an indelible part of the history of pharmacy in Nigeria—an inspiration for present and future practitioners.



VARIOUS PORTRAITS OF PROF. A.T. MORA ABOVE

TOP LEFT: As an undergraduate Pharmacy Student on Matriculation Day in 1975 in ABU Zaria.

TOP RIGHT: As a graduate of Pharmacy on convocation day in ABU, Zaria in 1978.

Bottom Left: As a graduate of the West African Postgraduate College of Pharmacists (WAPCP) on graduation day in Banjul, Gambia in March, 2022.

Bottom Right: As "Wakilin Maganin Zazzau" on day of turbaning in Zaria in September, 2013



Mallam Haladu Tanko Soba (now retired Judge) and primary school teacher of Pharm. A. T. Mora (2nd left) hosted his former pupils at LEA Primary school Tudun Wada, Zaria) in his house some times in early 80s. Seated right Prof. A. T. Mora



Members of the Barewa College, Zaria Cadet Force. Pharm A.T. Mora is 2nd right others (L-R) Abdurrahman Mahmud, Babangida Musa and 1st right is Yahaya Hassan



Members of the Exco of PANS (ABU Chapter) posed for a photo with the Dean, FPS, ABU, Zaria Prof. G.E Osuide (4th from left). Pharm A.T. Mora, the PRO is seated 2nd from Left



(L-R) Pharm A.T. Mora, Najamuddeen Abubakar Imam (now deceased), Dr. M.S. Mora and Surajuddeen Abubakar Imam posed for photo on graduation day, December, 1978 at the residence of Dr. Abubakar Imam.

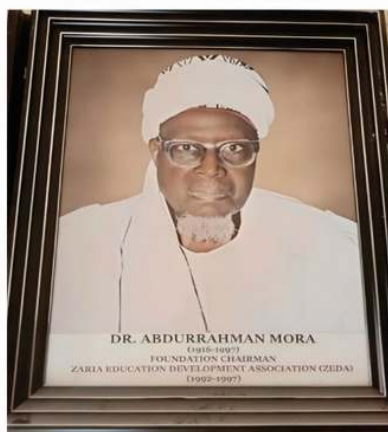


Dr. Abubakar Imam (Centre) flanked by Dr. Muhammad Saifuddeen Mora (MBBS) (ABU) (Left) and Pharm A.T. Mora (B.Sc Pharmacy). Graduation Celebration at home, in Zaria in December, 1978.



Mall (Dr.) Abdurrahman Mora (Centre) flanked by Dr. Muhammad Saifuddeen Mora (MBBS) (ABU) (Left) and Pharm A.T. Mora (B.Sc Pharmacy) (right). Graduation Celebration at home, in Zaria in December, 1978.

The Father of Prof. A.T. Mora



His Excellency Ambassador (Dr) Abdurrahman, Mora, *D. Litt*

Katsina Higher College Certificate (1938), Certificate of Education (Institute of Education, University of London), (1945-1946)

Teachers Certificate in Geography (Wosley Hall, England) 1952; Teacher Training

Fellowship, Iowa State Teachers College

(now University of Iowa) Cedar Falls, Iowa USA (1952-1953)

D. Litt (FUT Minna) (1991)

ZARIA NATIVE AUTHORITY:

- * Teacher; Zaria Middle School, (1938-1946)
- * Headmaster, Zaria Middle School, (1950-1954)

NORTH REGIONAL GOVERNMENT:

- * Teacher, Kaduna College, Kaduna (1946-1950)
- * Adult Education Officer, Zaria Province (1954-1955)
- * Chief Adult Education Officer
- * Northern Region (1955-1958)
- * Senior Assistant Secretary (MoH, Kaduna (1958-1960))

FEDERAL GOVERNMENT OF NIGERIA:

- (Federal Permanent Secretary)
- * Ministry of Lagos Affairs (1962-1966)
- * Ministry of Internal Affairs (1966-1967)

FEDERAL GOVERNMENT OF NIGERIA:

- (Foreign Diplomatic Service)
- * Pilgrims Officer;
- * First Secretary and later chargé d'affaires, Nigerian Embassy, Jeddah Saudi Arabia (1960-1962)
- Nigeria's High Commissioner (Ambassador) to the Islamic Republic of Pakistan (1967-1971)



Mall (Dr.) Abdurrahman Mora (Centre) being decorated with the robe at the conferment of Honorary Doctorate Degree (Honoris causa) by the Chancellor of the Federal University of Technology (FUT), Minna, HH, Alh. Shehu Idris, CFR, the Emir of Zazzau on 2nd February, 1991.



Mall. (Dr.) Abdurrahman Mora reading his speech after receiving the Honorary Doctorate Degree. From FUT, MINNA.



Being decorated with the “*mni*” badge by Hajiya Asiya Ladi, the wife of Pharm A.T. Mora after his graduation at NIPSS, Kuru, near Jos in November, 2010.



(L-R) Pharm. A.T. Mora, mni, Brig. Gen. IU Babangida, MFR, mni, the Monitor General of SEC 32, Mrs Nyoagbe and Pharm Joseph Nyoagbe the Registrar of the Pharmacy Council of Ghana (PCN) at the graduation.



His Excellency Arc. Muhammad Namadi Sambo, *GCON*, the Vice-President of the Federal Republic of Nigeria (R) presenting the Certificate of Membership of the National Institute (*mni*) at NIPPS, in November, 2010 after Pharm. A.T. Mora successfully completed the Senior Executive Course (SEC) (32/2010).



Maigirma Alh. Abubakar Tunau Mafara MFR (K'ayayen Sardauna) (R) with Pharm. A.T. Mora (2nd Left) and the late Ibrahim Tunau (backing camera) at the Convocation grounds in BUK, December, 1983.



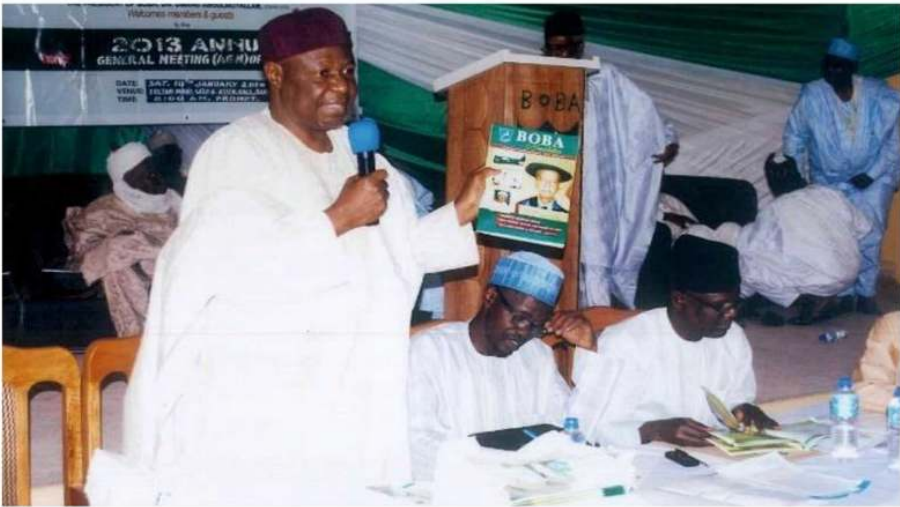
(L-R) Unidentified guest, Mallam Ibrahim Tunau (now late), Pharm A.T. Mora, Aisha Abubakar Tunau (wife of Pharm A.T. Mora) Maigirma Alh. Abubakar Tunau Mafara MFR (K'ayayen Sardauna) (Father-In-Law) (now deceased) and others at BUK Convocation grounds in December, 1983.



(L-R) Pharm. A.T. Mora the Secretary General of BOBA with Alh. (Dr.) Umaru Abdulmutallab, CON, the President of BOBA at the BOBA National Secretariat in Kaduna after an event.



His Excellency General Yakubu Gowon, GCFR, (B.783) former Nigeria's Head of State and Chairman of the Board of Trustees (BoT) of BOBA (R) with Pharm. A.T. Mora at an event in Abuja.



Pharm. A.T. Mora, the Secretary General of BOBA at an event at the Muhammad Sa'ad Abubakar Assembly Hall, Barewa College Zaria at an AGM event in 2013.



TOP: His Eminence Sultan of Sokoto Alh. Muhammad Sa'ad Abubakar, CFR, mni, making a presentation to Prof. A.T. Mora, Secretary General of BOBA at a BOBA event in Gen. Hassan Usman Katsina House, Kawo, Kaduna in October, 2015.



(L-R) Pharm. A.T. Mora, Chairman of the BOBA (1970 – Intake) Class Group making a presentation in the Palace of His Eminence the Sultan of Sokoto, a member of the Class Group in the Palace in July, 2008. The Sultan is seated first from right, the former Secretary General of BOBA, Alh. Shehu Muhammad Kaika is seated to the right of the Sultan.



Dr. (Mrs) Saratu Olorukoola (now late), Chemistry teacher of the BOBA Class (1970 - Intake) making a presentation in the Sultan's palace Sokoto in July, 2008



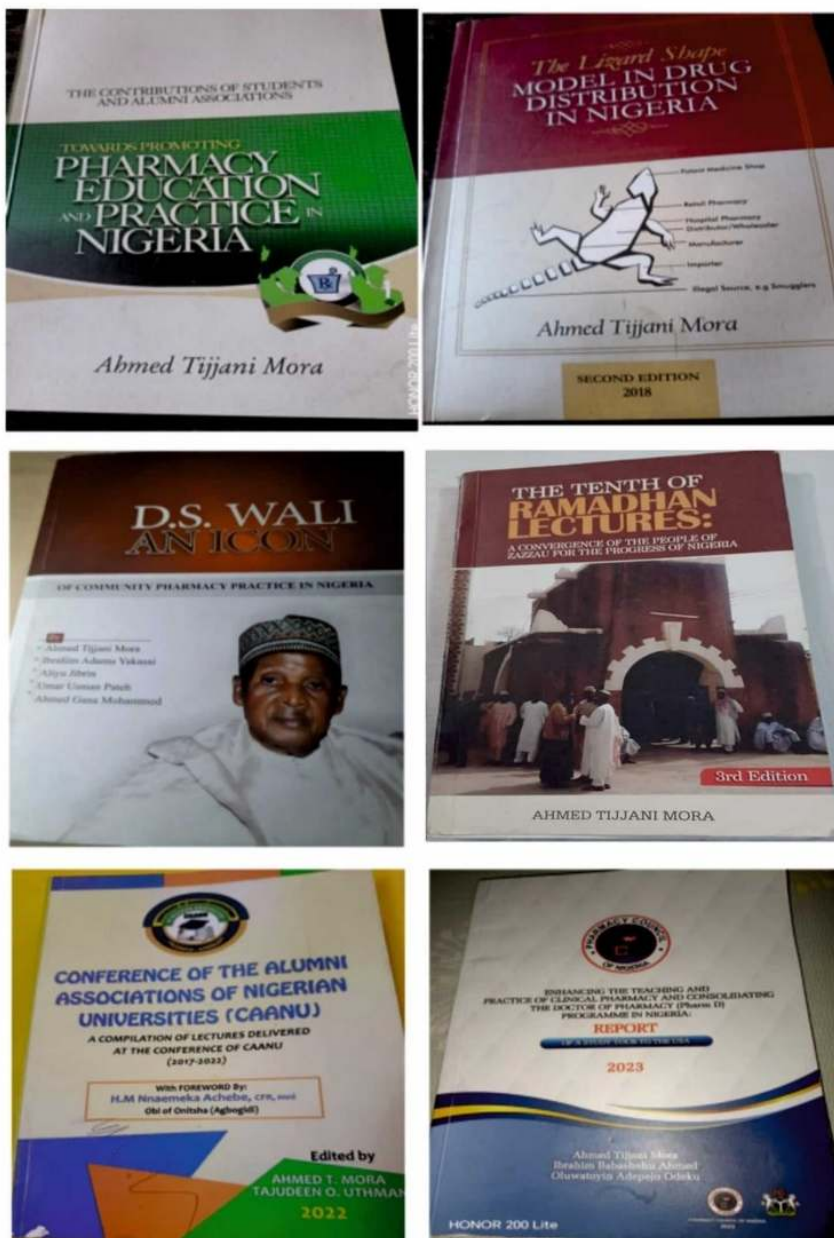
(L-R) Alh. Muhammadu Jibo, MFR (one time Bursar, ABU, Zaria) Now late and Pharm. A. T. Mora at home in Zaria



(L-R) Pharm. Aliyu Jibrin, Prof. Ibrahim Adamu Yakasai, an unidentified guest, Pharm Tanko Ayuba (Current President of the PSN) in the Family House of Pharm A.T. Mora posed for a photo session after a wedding event in Zaria



His Eminence Alh. Muhammad Sa'ad Abubakar, CFR, mni the Sultan of Sokoto, the Chancellor ABU Zaria at an event in the Assembly Hall ABU, Zaria in 2013. The Pro-chancellor and Chairman of Council, Arc. Mohammed Abdullahi Dewu and Pharm. A. T. Mora to the far left of the Sultan as the Deputy President worldwide of the ABU Alumni Association



Some of the titles authored and/ or co-authored by Prof. A.T. Mora

Chapter 2

Pharmacists Board of Nigeria (PBN), As Chief Pharmacist/ DPS, MoH, Kaduna) (1989-1992)

PHARMACISTS' BOARD OF NIGERIA (PBN): 1989 – 1992

The journey of Prof. A. T. Mora to becoming a regulatory guru started with his membership of the then Pharmacists Board of Nigeria (PBN). It was then the body responsible for, among other things, determining the standards of the knowledge and skills to be attained by persons seeking to become registered members of the pharmacy profession and reviewing those standards from time to time as circumstances may require. It was established by the Pharmacists Act, *Cap 357 LFN*.

The membership of the Board was made up of:

- a) The Chairman, who shall be a registered Pharmacist of not less than 15 years post-registration experience, appointed by the President and Commander-in-Chief of the Armed Forces of the Federal Republic of Nigeria on the recommendation of the Honorable Minister of Health.
- b) A representative of the Federal Ministry of Health, who shall be the Head of the Foods and Drugs Administration and Control Department in the ministry.
- c) The President of the Pharmaceutical Society of Nigeria (PSN), which is the umbrella body of all Pharmacists in the country.
- d) The Director of Pharmaceutical Services, or however called, of all the state Ministries of Health, including the Federal Capital Territory (FCT).
- e) The Deans of the recognized faculties of pharmacy (or Pharmaceutical Sciences) in Nigerian universities.
- f) Eight members from the states of the federation appointed by the Minister on the recommendation of the Pharmaceutical

Society of Nigeria, with no state having more than one representative.

- g) One representative of the armed forces who shall be a registered pharmacist.
- h) The Director - General of the National Institute for Pharmaceutical Research and Development (NIPRD).

The Pharmacists Board of Nigeria (PBN) existed until 1992, when the Pharmacists Act Cap 357 LFN was replaced by Decree No. 91 of 1992, which led to the establishment of the Pharmacists Council of Nigeria (PCN). It is, however, important to note that the functions as well as the composition of the Board of the PCN remained the same as those of the PBN.

Membership of the PBN and PCN Boards

Prof. Mora became a member of the Board of PBN in 1989 by virtue of being the Chief Pharmacist of the Ministry of Health, Kaduna state. That marked the beginning of his journey to becoming a pillar in the regulation of pharmacy practice in the country. In 1990, he became the Director of Pharmaceutical Services (DPS) at the Ministry of Health, Kaduna State and remained a member of the PBN Board. He was a member of the Board when the name was changed to Pharmacists Council of Nigeria (PCN) in 1992 and remained in this position until 1993. It is worth noting that in the same year, 1989, Prof. Mora was also the Chairman of the Pharmaceutical Society of Nigeria (PSN) Kaduna State branch. He was elected in absentia on Sunday, 19th February, 1989. So, while being a member of the PBN, he was at the same time a member of the Council of the PSN as a state branch Chairman. Usually, Chairmen and Secretaries of the state branches are members of the PSN Council. He voluntarily relinquished the position of chairman of the PSN in 1990 when he became the DPS, so as to be able to squarely face the duties of pharmacy administration and regulation in Kaduna State. For four good years, from 1989 to 1993, Prof. Mora remained a member of both the PBN and the PCN. But he

took a break in 1993 when he was appointed by the Kaduna state government as the Project Manager (PM) for the Project Implementation Unit (PIU) for the African Development Bank/African Development Fund (AFDF) (AfDB/AfDF) Assisted Kaduna Health Rehabilitation Project (KHRP), Ministry of Health, Kaduna.

In 1992, Kaduna state was chosen as one of the five (5) states in Nigeria to benefit from an African Development Bank facility for the rehabilitation of secondary healthcare facilities in the states. A project Manager for the Project Implementation Unit (PIU) was required. Many people lobbied to get the position. But the then Commissioner of Health, Alhaji Yusuf Hamisu Abubakar, felt that Mora, who was then the DPS, was most suitable for the job, considering his proven record of management and discipline, and subsequently appointed him as the first Project Manager of the KHRP. As usual, Mora left a mark everywhere he worked, representing Kaduna state with distinction and eventually, he was elected the Chairman of all five Project Managers. During the same period, he was again assigned the responsibility of the Project Coordinator (PC) for the disposable syringes manufacturing plant in Zaria, a project of the Ministry of Health, Kaduna state, for the establishment of Zaria Pharmaceutical Company (ZPC). In 1997, Mora was deservedly appointed the first Pharmacist Managing Director and Chief Executive Officer of the ZPC, the manufacturer of the famous *Zarinject* syringes and needles, which was the largest disposable syringes manufacturing plant in West Africa. As the Chief Executive Officer of ZPC, he continued to demonstrate a capacity for hard work, innovation, visionary leadership and professional leadership, as well as resilience.

However, in November 1999, political circumstances led to his recall from the position of MD/CEO of ZPC. Even though it lacked proper jurisdiction, a commission of inquiry was constituted by the state government in 2002 to investigate the activities at ZPC. Without hesitation, Mora appeared before the commission, defended his tenure

with facts and transparency and emerged vindicated, thereby maintaining his professionalism and integrity.

Coincidentally, in the same year, 1999, Mora was the chairman of the Conference Planning Committee (CPC) of the 88th annual national conference of the Pharmaceutical Society of Nigeria (PSN), tagged 'Coal City 99', which was held in Enugu in November. It was also one of the best conferences organized by the PSN. Being a very prominent and active member of the PSN, Mora was appointed as one of the eight (8) PSN representatives on the Pharmacists Council of Nigeria (PCN) Governing Council in December 1999. This was his second term on the Governing Council of the PCN, the regulatory body for the Pharmacy profession in the country, and part of his long journey at the PCN that has made him a regulatory Guru. Even after the three-year tenure of the Council ended in 2002, he was reappointed for a second time to represent the PSN to serve among its eight (8) representatives in August 2003, when the Council was reconstituted.

As the Chief Pharmacist and later DPS of Kaduna state, as far back as 1989 and as PSN representative, Mora played significant roles in the activities of the Boards of both the PBN and PCN as a member of so many standing committees of the Boards. Back in 1989, the meetings of the then PBN were usually held at different venues across the country, as there was no Head office at that time. For example, while one of the meetings was held in Owerri, Imo State, on 7th December 1989, the next meeting, on 7th May, 1991, was held in Yola, the capital of the then Gongola State. That meeting was declared open by the then Deputy Governor of the state. At that meeting, a committee on Pharmacy Practice was formed, with Mora as a member. The Chairman of the committee was Prof. D. T. Okpako. Other members of the committee included Prof. Fola Tayo, Dr. B. Sowemimo, Mrs. Fajemirokun, Mr. G. L. Eradiri, and Mr. A. A. Brown, who served as the secretary of the committee, being the Registrar of the PBN. Subsequent meetings of the Board were held in Ilorin, Kwara State,

on July 30, 1991, in Ibadan, Oyo State, on October 8, 1991 and in Uyo, Akwa Ibom State, on May 21, 1992.

Apart from serving as a member of the Pharmacy Practice Committee, Mora has, over the years, been a member of various standing committees of both the PBN and later the PCN. The PCN has six standing committees with specific functions, usually constituted as follows:

1. Disciplinary Tribunal

The Chairman of the Governing Council of the PCN is the Chairman of this committee. Other members are made up of the President of the PSN, one representative of the Deans of faculties of Pharmacy, one representative of the Directors of Pharmaceutical Services, one representative of the PSN, one representative of the Armed forces, an Assessor appointed by the Attorney-General of the Federation and the Registrar of the PCN who is the secretary of the committee.

2. Investigating Panel

This committee is chaired by a representative of the PSN. Other members are representatives of Deans, DPSs, the PSN, the PCN legal officer and a nominee of the Registrar who also serves as the Secretary of the committee.

3. Appointment, Promotion and Disciplinary Committee

This Committee is typically chaired by the Director of Food and Drug Services at the Federal Ministry of Health. Other members are representatives of DPSs, representative of the PSN, representative of the Deans of Faculties of Pharmacy, the Registrar of the PCN and a nominee of the Registrar who serves as secretary.

4. Finance and General-Purpose Committee

The PCN Chairman is the Chairman of this committee. The membership comprises one representative from PSN, representative from DPSs, representative from the Deans, the Registrar, and a

nominee of the Registrar, who serves as the secretary of the committee.

5. Law and Ethics Committee

The President of the PSN is usually the Chairman of this committee, which is made up of representatives of DPSs, representatives of PSN, representative of the Armed Forces, the Registrar, representative of NAFDAC nominated by the DG, NAFDAC and the Council's legal officer as secretary.

6. Education Committee

The Chairman of this committee is the PCN Chairman. Other members are the PSN President, all Deans, DG, NIPRD, representatives of DPSs, representative of PSN and a nominee of the Registrar who is also the secretary of the committee.

From Member to Registrar and Chief Executive Officer of the Governing Council

As mentioned earlier, after Mora served as a member of the Council as one of the eight (8) representatives of the PSN from 1999 to 2002 (a three-year term), he was again reappointed to represent the PSN in August 2003 as one of its eight (8) representatives. In April 2003, the Council lost its then Acting Registrar, Pharm. A. S. Rafindadi. Thus, there was a need to appoint a new Registrar for the Council. Following the usual laid down procedures, the position was advertised and some Pharmacists applied, including Mora, who already had tangible experience in the workings of the Council. Therefore, when the interview was conducted, Mora came out on top and was subsequently appointed the new Registrar and Chief Executive Officer of the PCN on 23rd September 2003, via a letter with reference No. PCN.C4051/VOL IV/383 signed by the then Chairman of the Council, Prof. F. A. Ogunbona, FPSN. With Mora as the Registrar and CEO of the PCN, the stage was set for the council's total transformation during his eventful two terms tenure totaling eight (8) years and three (3) months. This was attested to by the Opening Remarks of the Chairman

of the Council, Prof. F. A. Ogunbona at the 14th ordinary meeting of the Council on 19th February 2004. The Chairman said ***“Let me quickly congratulate you on your choice of Alhaji Ahmed T. Mora as Registrar and Secretary to this Council. He has proven to be a very astute, very hardworking and highly diligent manager of men and resources. With him at the helms of affairs in the Registry, your Council is set to march forward and take its rightful place as the most respectable regulatory body in the country. Congratulations, our dear Registrar for your brilliant performance so far and we say more grease to your elbows”***.

The chairman was right. The transformation Mora brought to the Council within the eight years he served as Registrar and CEO of the PCN was phenomenal, as will be seen in the following chapters.



Pharm A.A. Brown (1937 - 2026), one-time Registrar of the PBN and Pharm A.T. Mora at the Pharmaceutical Inspectors Workshop, in 2005 in Abuja



(L-R) Pharm (Mrs) Elizabeth Bob-Manual the DPS of Rivers State receiving the Attendance Certificate at the end of the Pharmaceutical Workshop in Abuja in 2005. The Registrar, Pharm. A.T. Mora at the Centre



(L-R) Lady Eme Ufot Ekaette, former President of the PSN, Professor Festus Adio Ogunbona (now Emeritus Professor), the Chairman of the PCN Council Pharm A.T. Mora, the Registrar and the Chairman of Patent Medicine Dealers at the Pharmaceutical Inspectors Workshop in 2005 in Abuja.

**CHAIRMEN/WOMEN AND REGISTRARS OF THE PBN AND PCN
WHO WORKED WITH PROFESSOR A.T. MORA**



Dr. Philip Emafo (Chairman PBN
(1989 -1990)



Prof. E.E Essien
Chairman PBN (1990 -1991)



Prof. G.E. Osuide
Chairman PBN (1991-1992)



Pharm (Dame) Oviasu
Chairperson PCN (1992- 1993)



Dr. F.B. Adenika
Chairman PCN (1999 -2002)



Prof. F.A. Ogunbona
Chairman, PCN (2003 –2006)
& (2006 -2007)



Prof. E.N. Sokomba
Chairman PCN (2009 -2011)



Pharm (Mrs) J. Okunsanya
Registrar (1989 -1991)



Pharm A.A. Brown
Registrar, PBN (1991 -1992)



Pharm (Ms) H.D Kayit
Registrar (1992 - 2000)



Pharm. A.S. Rafindadi
Ag. Registrar (2000 - 2003)



Pharm. NAE Mohammed
Registrar (2014 - 2022)

**PICTURES OF HEAD OFFICE/REGISTRY AND ZONAL/STATE
OFFICES**



PBN OFFICE, LAGOS



PCN LAGOS
LAGOS ZONAL OFFICE



SW ZONAL OFFICE & OYO STATE
OFFICE, IBADAN



SE ZONAL OFFICE & ENUGU
STATE OFFICE



SS ZONAL OFFICE & AKWA IBOM
STATE OFFICE, UYO.



NC ZONAL OFFICE & NIGER STATE
OFFICE, MINNA



NE ZONAL OFFICE & BAUCHI
STATE OFFICE, BAUCHI



NW ZONAL OFFICE & KADUNA
STATE OFFICE, KADUNA



ABUJA ZONAL OFFICE
(ABZO) ABUJA



Pharm Ibrahim Babashehu Ahmed
Registrar, Pharmacy Council of Nigeria (PCN)
(2022 - Date).



HEAD OFFICE & REGISTRY, PCN IDU, ABUJA



Pharm. A.T. Mora the Grand Patron of *PANS* (ABU Chapter) with Ibrahim Amidu the *PANS* (ABU Chapter) President with his exco members at an event in ABU Zaria.



Pharm A. T. Mora with some members of the PCN at the Annual Scientific Conference of the F. I. P in Seville, Spain in 2021



Pharm A. T. Mora (R) as Chairman of the PCN with the Registrar, Pharm Ibrahim Babashehu Ahmed (First left), Prof (Mrs.) Oluwatoyin Adepeju Odeku and a pharmacist posed during an official trip to Philadelphia, USA in May, 2023



The late Alhaji Muhammadu Inuwa Mora (1940 - 2001) (2nd right) with his very close friend, Alhaji Aliko Mohammed (D'an Iyan Misau) (3rd right) and others at an event in the late 1960s



90th Birthday anniversary of Prof. Gabriel E Osuide, FAS the foundation Dean of the Faculty of Pharmaceutical Sciences, ABU., Zaria and pioneer DG and CEO of NAFDAC on 15th March, 2025 in Lagos. Prof. Osuide can be seen seated with the wife Mrs. Clemensia Osuide. Prof Mora and Prof. Lere Ba'ale are standing behind.

Chapter 3

(Pharmacists Council of Nigeria (PCN): (i) DPS (1992/1993) and (ii) Rep of PSN (1999 - 2002)

Setting the National Scene — Pharmacy in Transition

By the dawn of the 1990s, Nigeria's pharmaceutical landscape was restless for reform. The industry had matured beyond colonial-era dispensaries, yet its regulatory framework remained trapped in the architecture of the 1960s. The Pharmacists Board of Nigeria (PBN), established in 1964, had served dutifully, ensuring the registration of pharmacists and premises, but its reach was limited. The explosion of new pharmacy schools, the emergence of industrial manufacturing and the challenges of counterfeit drugs demanded a stronger, more autonomous regulator. The need for a new law was championed by forward-thinking professionals within the Pharmaceutical Society of Nigeria (PSN), senior administrators in the Federal Ministry of Health and state-level Directors of Pharmaceutical Services. Among that generation of reformers stood Ahmed Tijjani Mora, then a rising technocrat in Kaduna State's Ministry of Health. The government's response came in the form of Decree 91 of 1992, and codified in the Laws of the Federation, (2004, *Cap 17*), which birthed the Pharmacists Council of Nigeria (PCN). The new body was conceived not merely as a continuation of the PBN but as its evolution, vested with expanded powers to regulate pharmacy education, license pharmaceutical premises, supervise inspections and discipline errant practitioners. The PCN was to be the custodian of the profession's conscience. Its creation symbolized Nigeria's intent to align its pharmaceutical governance with global best practices.

The Young Reformer from Zaria

At the time the PCN was inaugurated, Ahmed Tijjani Mora was serving as Director of Pharmaceutical Services (DPS) in the Kaduna State Ministry of Health. His appointment to the First Governing Council of the PCN in 1992 was, therefore, automatic, conferred by virtue of his position as state DPS. Barely in his thirties, Mora represented a new generation of pharmacists who combined technical competence with managerial discipline. Having served earlier in academia and administration, he possessed an unusual dual perspective: the rigour of scholarship and the pragmatism of policy. His calm voice, measured diction and insistence on ethical precision quickly earned him respect among senior colleagues, many of whom were industry veterans and professors, decades his senior.

At the inaugural meeting, chaired by Pharm. (Mrs.) Oviasu of Edo State, Mora listened attentively as the new Council deliberated on how to navigate its constitutional infancy, no office space large enough for its members, no permanent staff structure and no clear financial autonomy yet. But where others saw difficulty, Mora saw opportunity: *“If regulation must protect the public,”* he later wrote, *“then regulators must first regulate themselves, through discipline, order and purpose.”* That attitude became the quiet tone of his service.

The First Governing Council (1992–1993); Building on Empty Ground

The First Governing Council of the PCN began its operations under conditions that could easily have discouraged lesser professionals. The Council inherited from the old Pharmacists Board of Nigeria (PBN), a small, cramped office in Yaba, Lagos ill-equipped to host its sixty-plus members. Undeterred, the Council decided to rotate its quarterly meetings among the state capitals. This innovation, both pragmatic and inclusive, ensured national visibility for the newborn institution and fostered a sense of belonging across the states. At these peripatetic meetings, often held in modest conference halls provided by host state

governments, Mora proved invaluable. As a Director of Pharmaceutical Services, he understood the practical realities of state health administration and often bridged the gap between theory and practice. When issues arose concerning inspection logistics or coordination with local ministries, his contributions were decisive. The Chairperson, Pharm. (Mrs.) Oviasu steered the Council with admirable foresight, and within its brief lifespan, the body achieved foundational milestones that still shape Nigeria's pharmacy profession today.

Milestones of the First Council

(i) Appointment of the First Registrar

Through a transparent process endorsed by the Honourable Minister of Health, the Council appointed Ms. H. D. Kayit as the first substantive Registrar of the PCN. This move institutionalized the Council's professional independence and marked the end of the era when pharmacy regulation was managed as a mere desk under the Federal Ministry of Health (FMoH)

(ii) Introduction of Clinical Pharmacy and Pharmacy Practice in Nigerian Universities

Perhaps the most transformative decision was the Council's approval for the establishment of Departments of Clinical Pharmacy and Pharmacy Practice in all Faculties of Pharmaceutical Sciences across the country. To achieve this, the Council, working closely with the Committee of Deans, sponsored two study teams of Nigerian pharmacy educators to the United Kingdom and the United States:

- The U.S. team visited the Philadelphia College of Pharmacy (University of the Sciences) and Howard University College of Pharmacy, Washington D.C.
- The U.K. team studied the models at the University of Strathclyde and University of Bradford.

Their reports became the blueprint for transforming Nigerian pharmacy from product-centered dispensing to patient-centered clinical practice, a shift that elevated the profession's global relevance.

(iii) Standardization and Inspection Initiatives

Even within its short existence, the Council began harmonizing inspection guidelines for retail, hospital and industrial pharmacy, a task that would later mature into full-fledged directorates.

A Council Interrupted

Just as momentum was gathering, Nigeria's fragile political structure gave way. On 17 November, 1993, when the Chief Ernest Shonekan-led Interim National Government was dissolved by the military, all Governing Boards and Councils of Federal Parastatals were disbanded. The PCN was not spared. The first Council, in which Mora served diligently, thus ended abruptly. Yet, its short tenure had sown enduring seeds: institutional autonomy, academic modernization and professional confidence. For Mora, the experience was formative. He had witnessed firsthand how national policies were shaped, negotiated and sometimes undone by politics. It taught him resilience and the patience required to build lasting systems in an unpredictable environment.

Between Silence and Rebirth (1993–1999)

The six years that followed were years of administrative dormancy but professional fermentation. With the Council dissolved, the PCN Secretariat in Lagos continued only skeletal operations under the supervision of the Federal Ministry of Health. For practitioners, it was a season of uncertainty. Those "silent years" became an apprenticeship in patience. When democracy returned in 1999 and the federal government moved to reconstitute dormant boards, his name naturally re-emerged—this time, not as a bureaucratic nominee but as a professional representative of the PSN.

The Second Governing Council (1999–2002) — From Renewal to Reform

In December 1999, the Second Governing Council of the Pharmacists Council of Nigeria was inaugurated. It was a council of renewal, coming at the dawn of Nigeria's Fourth Republic, when democratic optimism filled the air. Dr. Fred B. Adenika (of blessed memory) was appointed Chairman, bringing to the role decades of pharmaceutical and administrative experience. Mora, by now an accomplished academic-administrator, took his seat as one of eight representatives of the PSN. The new Council confronted daunting tasks: the need to modernize operations, establish a permanent headquarters in Abuja, enforce discipline within an expanding profession and restore public confidence after years of institutional inactivity.

Achievements of the Second Council

(i) The Abuja Headquarters Project

The crowning achievement of this Council was the decision to construct a befitting PCN Headquarters and Registry at the Idu Industrial Area, Abuja. At the time, all operations were still in Lagos, but Abuja's rise as the Federal capital demanded relocation. The project required enormous financial prudence. In one memorable meeting at Abeokuta, Dr. Adenika privately sought Mora's support for a proposal to reduce the Council's quarterly meetings from four to two per year, diverting the savings to the building fund. When the issue came to the floor, Mora rose first to speak, articulating the logic and long-term vision behind the proposal. His speech was persuasive, his argument unassailable. The Council unanimously approved the motion. The money saved was used to augment the buildings of foundation of the headquarters that today stands as a symbol of regulatory permanence. Mora later remarked with characteristic humility, "*I never knew I was voting to build the office where I would one day serve as Registrar and again as Chairman.*"

(ii) Strengthening the Inspectorate and Logistics

The Council approved the procurement of brand-new inspection vehicles and operational equipment to enhance field supervision across the six geopolitical zones. For the first time, PCN inspectors could cover more premises and collect reliable compliance data.

(iii) Central Control of PPMV Licensing

Recognizing the chaos within the patent-medicine trade, the Council centralized and standardized the issuance of Patent and Proprietary Medicine Vendors' Licences. This reform curbed multiple licensing, reduced counterfeiting, and empowered the Council to maintain a national register of vendors.

(iv) Phased Relocation and Staff Welfare

The movement from Lagos to Abuja was executed in phases to maintain operational continuity. Key departments—registration, finance, and inspectorate—were relocated first. Staff were retrained and compensated and new policies were introduced for efficiency and morale. These initiatives signified the beginning of the PCN's institutional modernization.

Mora's Influence and Leadership Style

Within the Council, Mora's style was distinctive. He was not loud but effective—listening more than he spoke, yet when he spoke, decisions crystallized. His background as a state administrator gave him a practical understanding of the implications of every policy. He constantly reminded colleagues that “regulation must never punish; it must protect.” As the PSN representative, he served as an interpreter between the regulator and the regulated—ensuring that Council policies aligned with professional realities. He wrote detailed memoranda, offering evidence-based suggestions on training, inspection, and education. His integrity became his signature. He was trusted across divides—by bureaucrats, academics and private

practitioners alike. Many later acknowledged that his quiet diplomacy often defused potential rifts between the PSN and the Council.

The End of the Second Council and Transition to New Leadership

By December 2002, the Second Council's statutory three-year tenure had expired. Its record was impressive: the Abuja project initiated, inspectorate logistics upgraded, vendor-licensing system reformed, and financial discipline restored. Sadly, only weeks later, tragedy struck. Dr. Fred B. Adenika, the visionary Chairman under whom these achievements blossomed, passed away in the United States on 20 February, 2003. The shock reverberated across the profession. In August 2003, the Third Governing Council was inaugurated chaired by Professor Festus Adio Ogunbona, another icon of pharmacy education. Mora, recognized for his institutional memory and exemplary service, was reappointed as one of the PSN's eight representatives. Following the death of Acting Registrar Pharm. Abdulazeez Sani Rafindadi in April 2003, the Council advertised the post of Registrar. Mora applied. On 2 October 2003, he resumed duties as the Registrar and Chief Executive Officer of the PCN. It was a symbolic circle completed: the young DPS who had helped lay the Council's foundation in 1992 now returned, eleven years later, to administer the house he helped build.

The Adenika Memorial and the Culture of Excellence

Mora's deep respect for his late Chairman, Dr. Adenika, inspired him to institutionalize a culture of remembrance and capacity building. On 20 February 2004, exactly one year after Adenika's passing, and barely four months into Mora's tenure as Registrar, he organized a memorial lecture delivered by Sir Ifeanyi Atueyi, publisher of *Pharmanews*. That lecture became the seed of the Annual PCN Management Training Workshop for senior staff held every February in honour of Dr. Adenika. For eight consecutive years (2004–2011), Mora sustained the tradition, inviting distinguished pharmacists to

deliver lectures on leadership, ethics and administration. Each lecture was followed by a week-long training on modern management practices, equipping PCN personnel for excellence. Though the tradition ceased after his exit in 2012, it remains one of his enduring legacies, proof that he viewed institutional memory as the soul of organizational growth.

The Topics Delivered at the Lecture are Listed Below:

S/NO	YEAR/VENUE	CHAIRMEN	KEY NOTE ADDRESS SPEAKER	TOPIC
1.	2004/PCN Headquarters, Abuja	Prof. F. A. Ogunbona	Pharm. I. Atueyi Publisher, Pharmanews Ltd	Change Management and Organizational Effectiveness in a Volatile Regulatory Environment
2.	2005/PCN Headquarters, Abuja	Dr. Chinyere Mercellina Chukwuani	Pharm. S. Z. Nuhu, Ph.D Deputy Governor, Niger State	Managing for Excellence: The Role of a Regulatory Authority
3.	2006/PCN Headquarters, Abuja	HRH, Pharm. Luka Panya Baba Esu Karu	Prof. Femi Oyewo Dean, Faculty of Pharmacy, Olabisi Onabanjo University, Ago-Iwoye, Ogun State	The Contribution of Research Institutes and Universities in Pharmaceutical Research in Nigeria
4.	2007/PCN Headquarters, Abuja	Chief (Mrs.) Stella Okoli	Prof. Fola Tayo Chairman, Joint Faculty Board, West African Post-graduate College of Pharmacists (WAPCP)	Management and Control of Pharmaceutical Products Distribution System in Nigeria: The Role and Responsibilities of Regulatory Bodies

(Pharmacists Council of Nigeria (PCN): (i) DPS (1992/1993) and (ii) Rep of PSN (1999 - 2002)

5.	2008/PCN Headquarters, Abuja	Pharm. Lady Eme Ekaette	Prof. E. M. Abdurahman Vice-Chancellor, Kaduna State University, Kaduna	Manpower Training and Development as a Management Tool for Achieving Organizational Mandate in a Challenging Regulatory Environment
6.	2009/NIIA, Victoria Island, Lagos	Prince Julius Adelusi-Adeluyi	Prof. O. K. Udeala, Former Vice Chancellor, University of Nigeria, Nsuka.	Excellence in Pharmacy Practice Management: The Fred Adenika Legacy
7.	2010/Sheraton Hotel, Abuja.	Prof. E. E. Essien	Dr. Philip Emafo, Chairman, NAFDAC	Control of Medicines in Nigeria.
8.	2011/Ikeja Sheraton Hotel, Lagos.	Prince Julius Adelusi-Adeluyi	Prof. K. S. Gamaniel, DG, NIPRD	Standard Operating Procedure (SOP) as a Critical Tool for Effective Regulation and Control

The Fred Adenike Memorial Lecture as a Foundation was eventually established after the 2011 edition of the lectures.

The Broader Significance of His DPS and PSN-Representative Years

Between 1992 and 2002, Mora's engagement with the PCN transitioned from observation to influence.

- As Director of Pharmaceutical Services, he had witnessed the birth of a national regulator and understood its challenges from the ground up.

- As PSN Representative, he learned to navigate the complex interplay of professional politics, federal bureaucracy and ethical regulation.

These two experiences formed the perfect apprenticeship for his later leadership. They honed his ability to balance principle with pragmatism, vision with patience. They also instilled in him an abiding belief that the regulator and the profession must remain partners, not adversaries. Looking back, those years represented the architecture of his destiny: the scaffolding upon which he would later build Nigeria's most stable era of pharmacy regulation.

Reflection — The Architect Before the Blueprint

When history judges the evolution of the Pharmacists Council of Nigeria, it will note that its legal birth came by decree, but its moral birth came through the character of men and women who served selflessly in its infancy. Among them, Professor Ahmed Tijjani Mora stands out as both witness and builder. From his early seat as DPS delegate in 1992 to his influential tenure as PSN representative seven (7) years later, he exemplified the rare blend of administrative discipline, academic insight and professional humility. The policies he supported, the institutions he helped create and the integrity he displayed would later define the PCN's identity. The Council began as a bureaucratic experiment but matured into a national institution. Mora began as a participant but evolved into its conscience. The same man who once voted to reduce meeting allowances for the sake of a building would later occupy that very building as its Registrar and again as its Chairman, living proof that vision, when pursued with sincerity, circles back as destiny fulfilled.

Epilogue — The Seeds that Became Structures

The decade between 1992 and 2002, through with a break from 1993 to 1999, now reads like the prologue to his larger story, the years in which a young reformer learned the anatomy of regulation, the politics

of policy and the art of institutional stewardship. Those who knew him in those years remember his modest car, his neatly kept notebooks and his habit of writing reflections after every Council meeting, notes he still preserves. In them, one line recurs: *“Regulation must serve the people or it will lose the right to regulate.”* It was this philosophy that he would later carry into his tenure as Registrar and Chairman. But in truth, the journey began here, in the twin assignments of Director of Pharmaceutical Services and Representative of the Pharmaceutical Society of Nigeria, when a young pharmacist from Zaria helped lay to the intellectual and moral foundations of Nigeria’s highest regulatory body for the pharmacy profession. The seeds had been planted; the structure was rising. The next chapter would reveal how the builder became the master architect.



The Overhead Pedestrian Bridge at the North Gate, ABU Main Campus, Zaria constructed during the time Pharm A. T. Mora was the President Worldwide of the ABU Alumni Association (2015-2021)



Maigirma Yarima Zazzau (now Madaki) Alh. Muhammad Ja'afaru, *mni* commissioning two (2) Buses acquired by the ABU Alumni Association Exco under President Mora at the Secretariat in February 2019.



Pharm A.T. Mora, President Worldwide of the ABU Alumni Association with Professor Olufunmilayo Braide, the Deputy National President (DNP) delivering his acceptance speech after his unanimous election, unopposed as the President ABU Alumni Association in the Senate chamber, ABU, Zaria on 15th August, 2015



(L-R) Prof. Abdullahi Mustapha (former VC of ABU, Zaria (2010 - 2015)), His Excellency Governor Danbaba Danfulani Suntai (now late) Prof. Gabriel Osuide the Foundation Dean Faculty of Pharmaceutical Sciences ABU, Zaria (and after whom the Twin 250 Seater Capacity Lecture Theatres, FPS/ABU) were named and Pharm A.T. Mora, the National Chairman of the ABUPAA. On 15th October, 2011

Pharm. A.T. Mora (L) President Worldwide of the ABU Alumni Association with Prof. (Mrs) Fumilayo Braide, the *Deputy National President (DNP)* of the Association at an event.



(L-R) Pharm A.T. Mora National Chairman of the ABU Pharmacy Alumni Association (ABUPAA) showing His Excellency Pharm. Danbaba Danfulani Suntai, the Governor of Taraba State (now late) and member of ABUPAA something of interest on 15th October, 2011 during the Commissioning of Gabriel Osuide Lecture Theatre (GOLT).



With Prof. E.N. Sokomba Chairman of the PCN (2009-2011) at the Public Presentation of the 2nd edition of the book – *"The Lizard Shape Model in Drug Distribution in Nigeria"* in Ikeja in February, 2018.



(L-R) Pharm. A.T. Mora with Chief (Dr.) Stella Okoli, *MON, OON*, the Executive Chairperson of Emzor Pharmaceutical Ltd, Lagos at the Public Presentation of the 2nd edition of the book.



(L-R) Pharm. A.T. Mora, HRM Igwe (Pharm) Alex Uzo Onyido, *FPSN* the Ezechuamagha of Ogidi, Anambra State and Chief (Dr.) Stella Okoli discussing an issue of interest at the Public Presentation of the 2nd edition of the Book.

(Pharmacists Council of Nigeria (PCN): (i) DPS (1992/1993) and (ii) Rep of PSN (1999 - 2002)



His Eminence Alh. Muhammad Sa'ad Abubakar, *CFR, mni* (B.2612), the Sultan of Sokoto presenting plaque to the Keynote Speaker Lt. Gen. (Rtd) A.B Dambazau, *CFR* at the BOBA (1970-Intake) Class Group 50 years Re-Union on 29th June, 2024. Pharm A.T. Mora, *OON, mni* (B.2511) looks with admiration.





Pharm. A.T. Mora (3rd from left), the immediate Past National President of the Conference of the Alumni Associations of Nigerian Universities (CAANU) (now ANUA) at an event in Abuja in 2025.



Prof. U.U. Pateh at the Public Presentation of the Book – *The Lizard Shape Model in Drug Distribution in Nigeria* (2nd Edition) authored by Pharm. A. T. Mora at Ikeja, Lagos in February, 2018. Prof. Pateh was book Reviewer.



Pharm A.T. Mora having a warm handshake with the late Prof. Abdullahi Mahadi (former VC of ABU and Gombe State University) at a function of CAANU in Gombe State University (GSU) in April, 2018 in his capacity as the National Chairman of CAANU (now ANUA).



HH Alh. Shehu Idris, CFR, LLD. The 18th Fulani Emir of Zazzau (now late) on 27th September, 2013 before the Turbanning Ceremony of Pharm. A. T. Mora as “Wakilin Maganin Zazzau”
At the Emir’s Palace, Zaria



Prof. Idris A. Abdulkadir (L) one-time ES of NUC and one-time Chairman of Zaria Pharmaceutical Company (ZPC) Ltd when Pharm A.T. Mora was MD and CEO (1997 - 2000) congratulating Pharm A.T. before the Turbanning ceremony in the Palace, Zaria



The “Wakilin Maganin Zazzau” acknowledging greetings from well-wishers while on horseback after the Turbanning in the palace on his way to the family house in Zaria.



(L-R) Alh. Umar Faruk Mora, Dr. M.S Mora, "Wakilin Maganin Zazzau", Alh. Muhammad Aminu Mora and Alh. Abbas Mora. Celebrations at 23/25 Mora Road T/Wada Zaria after the Turbaning of the "wakilin Maganin Zazzau" On Friday, 27th SEPT, 2013.



Pharm A.T Mora (*Wakilin Maganin Zazzau*) flanked by mums Hajiya Aminatu Dikko (left) (now late) and Hajiya Binta Atti (right).



Dignitaries at the Family House of Pharm A.T. Mora after the Turbaning Ceremony (L-R) Maj. Gen. (Rtd) M.N Jega (now *Baraden Gwandu*), Pharm Abba Dasuki (now *Durbin Sokoto*) Alh. Ahmed Nuhu Bamalli (*Magajin Garin Zazzau*) (now HH the 19th Fulani Emir of Zazzau), Pharm. A.T Mora (*Wakilin Maganin Zazzau*) and Dr. Haruna Mohammed Jumare (*Maradin Zazzau*), on Friday, 27th September, 2013.

(Pharmacists Council of Nigeria (PCN): (i) DPS (1992/1993) and (ii) Rep of PSN (1999 - 2002)



**DR. AHMED TIJJANI MORA, FPSN, FPC, Pharm; Ph.D, mni
Wakilin Maganin Zazzau**



Mammoth crowd witnessed and escorting the "Wakilin Maganing Zazzau" to the Family House after the Turbaning.



Maigirma Iyan Zazzau, Alh. Muhammad Bashari Aminu (now late) seated to the left, pharm A.T. Mora and Prof A.H Rafindadi (former VC FUL, Lokoja) and others at the family house in Zaria.

Chapter 4

The Pharmacists Council of Nigeria (PCN) (Registrar and CEO) (2003-2012) Appointment as the Registrar and Chief Executive of PCN

The appointment of Pharm. Ahmed Tijjani Mora (FPSN), as the Registrar and Chief Executive officer of the Pharmacists Council of Nigeria (PCN), was considered at the special meeting of the Governing Council on 25th September, 2003, where the Council accepted the recommendations of the selection panel on the appointment of the Registrar for PCN and appointed him as the Registrar/Secretary to the Council. This appointment was not a matter of chance or luck but rather the culmination of a career meticulously built on a foundation of diverse experience, proven leadership and a visionary understanding of the pharmacy profession. The key reasons why Pharm A. T. Mora was the most suited candidate for appointment as the registrar of the Pharmacists Council of Nigeria (PCN) are:

1. Unparalleled and Diverse Professional Experience

Several key reasons established Pharm. A. T. Mora as the most suitable candidate for the appointment. He presented an unparalleled and diverse professional experience that covered all major practice areas in pharmacy. His career began in hospital and administrative pharmacy, where he served as a Pharmacist-in-Charge at General Hospital, Potiskum and rose to become the first Director of Pharmaceutical Services for Kaduna State, giving deep insight into public sector drug management, policy and administration. His background also included industrial pharmacy, having worked as a Pharmaceutical Representative for Grünenthal GmbH, which provided private sector marketing experience. Crucially, he later became the Pioneer Managing Director and CEO of Zaria

Pharmaceutical Company and Zazzau Pharmaceutical Industries Ltd, where he was responsible for local manufacturing, quality control and navigating the challenges of Nigeria's pharmaceutical industry. Furthermore, his service as a Part-Time Lecturer at Ahmadu Bello University, Zaria for seventeen years at that time and his continued *pro bono* lecturing even as Registrar gave him direct involvement in shaping the curriculum and mentoring the next generation of pharmacists, aligning perfectly with PCN's role in accrediting pharmacy schools. Adding to this comprehensive background was his prior regulatory exposure, having served as a Member of the Pharmacists Council of Nigeria representing the PSN from 1999 to 2002, which provided him with firsthand knowledge of the Council's internal workings and challenges. This 360-degree experience enabled him to understand the perspectives of the community pharmacist, the hospital pharmacist, the industrial manufacturer, the academic and the student, making him an empathetic and effective regulator.

2. Proven Leadership and Managerial Prowess

The role of Registrar is fundamentally a leadership and CEO position. Mora had a demonstrable track record of successfully managing complex organizations and projects. He acquired and exhibited astonishing project management skills, where he excellently managed the \$36 million AfDB/AfDF Kaduna Health Rehabilitation Project. In this project, Mora demonstrated an ability to handle large budgets, complex procurement, and multi-stakeholder coordination, skills directly transferable to managing the PCN secretariat. Pharm Mora also has Corporate Leadership experience, having served as the MD/CEO of ZPC and ZPIL. In this capacity, he turned around and managed manufacturing entities, overseeing production, staffing, finance and marketing; this demonstrated his ability to lead a large organization. In addition, he has demonstrated Association Leadership capability, having held positions, such as National Chairman of the ABU Pharmacy Alumni and other Alumni Associations, influential professional bodies, build consensus and drive agendas.

3. Deep-Rooted Commitment to Education and Profession Building

PCN is responsible for maintaining educational standards. Mora was not just an academic; he was a *builder* of pharmaceutical education, particularly in his role as a founding member and long-time Chairman of the Ahmadu Bello University Pharmacy Alumni Association (ABUPAA). It is not surprising that in this capacity he eventually spearheaded massive infrastructural and developmental projects for his *alma mater's* faculty, showing a lifelong commitment to elevating pharmacy education.

4. Pre-existing Relationship and Understanding of PCN

Unlike an external candidate, Prof. Mora was already intimately familiar with the Council. His prior service as a Council Member in (1992 - 1993) and (1999-2002) meant he understood the governance structure, the key challenges facing the profession and the internal dynamics of the Council. This allowed for a seamless transition and immediate impact, avoiding a long learning curve.

5. National and International Reputation and Network

The Registrar must be a credible ambassador for the profession. Mora had built a formidable reputation. He was a Fellow of multiple professional institutes (*FPSN, FPCPharm, and FICEN*). He played a key role in forming the West African Pharmaceutical Manufacturers Association (WAPMA) and eventually its Interim Chairman giving him a regional perspective. His extensive travels and presentations at conferences positioned him as a thought leader in pharmacy practice and management. Considering the status of PCN in 2003, the Council needed a reform-minded, dynamic and authoritative leader. Pharm Ahmed Tijjani Mora was not merely a qualified pharmacist; he was a unique polymath within the profession. His career was a deliberate and successful navigation through every critical pillar of pharmacy—Practice, Industry, Academia and Regulation. Therefore, following a competitive interview process in 2003, Mora was appointed

the Registrar and Chief Executive Officer (CEO) of the Pharmacists Council of Nigeria (PCN). Headquartered in Idu Industrial Layout, Abuja, the PCN is the statutory regulatory body responsible for regulating pharmacy education, training and practice in Nigeria. His appointment placed him at the helm of an institution critical to safeguarding public health by ensuring the integrity and competence of the pharmacy profession. His appointment as Registrar was a strategic decision to place a visionary with a proven track record of building institutions, managing complexity and passionately advocating for the profession at the helm of its regulatory body. It is a well-known fact amongst all pharmacists that by 2003, Mora had already excelled in every role that would prepare him for the pinnacle of pharmacy regulation in Nigeria.

Roles and Responsibilities as Registrar

As Registrar/CEO of PCN, Pharm. Mora's responsibilities included:

The Registrar's roles and functions are multifaceted, spanning administration, regulation, enforcement and advocacy. They can be categorized as follows:

1. Administrative and Chief Executive Role

- Head of Secretariat: Oversees the entire PCN Secretariat, including all the departments and staff.
- Secretary to the Governing Council: Serves as the Secretary to the Council Board, its committees and the Disciplinary Tribunal. This involves preparing agendas, recording minutes and implementing all decisions and resolutions.
- Resource Management: Manages the human, financial and material resources of the Council.

- **Spokesperson:** Often acts as the official spokesperson for the PCN, communicating its policies and positions to the public and the press.

2. Regulatory and Licensing Functions

- **Registration of Pharmacists:** Supervises the process of registering qualified pharmacists who have met the Council's requirements (including passing the PCN Examinations as the case may be).
- **Licensing of Premises:** Oversees the issuance and renewal of licenses for all pharmaceutical premises, including:
 - Community and Hospital Pharmacies;
 - Pharmaceutical Manufacturing Facilities;
 - Importation and Distribution Outlets;
 - Patent Medicine Stores
- **Accreditation of Institutions:** Manages the process of accrediting faculties of pharmacy in Nigerian universities and other training institutions for pharmacy technicians and other cadres to ensure they meet the prescribed standards.

3. Enforcement and Compliance

- **Inspections:** Directs the Inspectorate Department to conduct routine and spot-check inspections of all pharmaceutical premises to ensure compliance with existing laws and regulations.
- **Sanctions:** Initiates and oversees enforcement actions against defaulters, which can include sealing illegal premises, prosecute offenders and recommend disciplinary measures against erring pharmacists.

- **Maintaining Sanity:** Leads the Council's continuous fight against the proliferation of illegal drug shops and unqualified personnel dispensing medicines.
- 4. Educational and Professional Development**
- **Liaison with Universities:** Serves as a key link between the PCN and the Deans of Pharmacy Faculties to ensure alignment of curricula with national needs and global best practices.
 - **Internship Coordination:** Oversees the mandatory intern-ship program for fresh pharmacy graduates.
 - **Continuing Professional Development (CPD):** Promotes and facilitates CPD programmed for practicing pharmacists to ensure lifelong learning and competency.

The State of the Pharmacists Council of Nigeria (PCN) Pre-2003: Challenges and the Global Context

To understand the significance of Mora's appointment as Registrar in 2003, it is essential to examine the pre-existing conditions of the PCN. Before this period, the Council, like many public institutions in Nigeria, was grappling with a series of systemic challenges that hampered its regulatory effectiveness and stature, both nationally and in comparison, to its global counterparts.

1. Regulatory and Enforcement Capabilities

The PCN's regulatory and enforcement capabilities were significantly limited. Its capacity to enforce its own regulations was weak, with a proliferation of illegal pharmacy outlets and patent medicine stores operating beyond their scope. The inspection and enforcement teams were often under-resourced and under-motivated, facing political and logistical hurdles that led to a weak regulatory presence on the ground. The process for sanctioning erring practitioners and premises was

often slow and bureaucratic, which diminished the deterrent effect of the Council's regulations and created an environment of impunity.

2. Administrative and Structural Deficiencies

The PCN also faced significant administrative and structural deficiencies that had hampered its ability to merely process and issue licenses. This can be attributed to chronic underfunding by the government and a flawed funding structure. As a parastatal, it was heavily reliant on federal allocations, which were often insufficient and unreliable, limiting its ability to undertake capital projects, recruit competent staff or fund robust regulatory operations. Its physical infrastructure and zonal presence were limited. Before the tenure of Mora, there was no significant presence of PCN at the zonal and state levels. This vacuum was weakly filled by the Directors of Pharmaceutical Services at the state level, who typically carried out their functions as Directors at the Ministries and also served as representatives of the PCN. This lack of a decentralized structure was a major handicap for a regulator overseeing a vast country like Nigeria. Furthermore, the Council likely operated with outdated administrative and data management systems, characterized by manual record-keeping for licensure and registration that was inefficient and prone to errors, thereby slowing down all regulatory processes and exacerbating political challenges.

3. Educational and Professional Standards

A significant challenge within the pharmacy profession was the emerging misalignment between the educational foundations provided by Nigerian universities and the progressive demands of international pharmacy practice. The global landscape was increasingly embracing a patient-centered paradigm known as "pharmaceutical care," yet the curriculum within the country's institutions risked becoming outdated. As the statutory body responsible for accrediting pharmacy programmes, the Pharmacists Council of Nigeria (PCN) bore the critical responsibility of steering this educational evolution. However,

there was a discernible lag in the timely and comprehensive revision of the Benchmark Minimum Academic Standards (BMAS), which created a gap between the skills of new graduates and the competencies required in a modern healthcare environment.

Compounding this issue at the educational level was a parallel weakness in the mechanisms for lifelong learning among already practicing pharmacists. Although the concept of Continuing Professional Development (CPD) programme was acknowledged, its implementation lacked the structured and mandatory framework necessary to ensure its universal adoption and effectiveness. Without a robust system to enforce ongoing skill enhancement and knowledge updating, there was a tangible risk that the expertise of licensed pharmacists would gradually become obsolete. This deficiency not only impacted individual professional growth but also had broader implications for the overall quality of pharmaceutical care delivered to the public, as the profession struggled to keep pace systematically with the rapid advancements in medicine and patient management.

4. Inter-professional and Political Challenges

The Council was facing difficulties in clearly defending the scope of pharmacy practice from encroachment by other healthcare professionals and unqualified people, with the chaotic drug distribution system being a clear symptom of this failure. The fight against counterfeit drugs and the sanitization of the pharmaceutical sector have lacked sustained political priority, leaving regulatory bodies like the PCN and NAFDAC to fight an uphill battle without the necessary legislative or executive support.

5. Comparison with Global Pharmacy Regulatory Bodies (Pre-2003)

A comparison with global pharmacy regulatory bodies before 2003 highlights the PCN's significant disadvantage. Where established regulators in countries like the United Kingdom, the United States or

Canada enjoyed financial autonomy through registration fees, the PCN was reliant on unpredictable government funding. Global counterparts had strong statutory powers and well-funded inspection units, whereas the PCN had weak enforcement and a slow sanctioning process. While other regulators utilized advanced digital platforms, the PCN relied largely on manual processes. The scope of practice for pharmacists was proactively expanded abroad; however, the PCN was reactive. Mandatory Continuing Professional Development programme was a cornerstone of license renewal globally, but it was not well-structured in Nigeria. In brief, prior to 2003, the PCN was an institution struggling to fulfill its core mandate, marked by structural weaknesses, operational inefficiencies and a reactive regulatory approach. Mora's appointment was therefore a critical juncture, with a mandate to fundamentally reform and reposition the Council as a modern, effective and respected regulatory body.

In brief, prior to 2003, PCN was an institution struggling to fulfill its core mandate. It was characterized by structural weakness, operational inefficiency and a reactive rather than proactive regulatory posture. It lacked the funding, technological tools, decentralized structure and political clout that its global peers enjoyed. The challenges of a chaotic drug distribution system, inter-professional friction and an outdated educational framework overwhelmed its limited capacities. Mora's appointment in 2003 was therefore a critical juncture. His mandate was not merely to manage the Council, but to fundamentally reform, revitalize and reposition it as a modern, effective and respected regulatory body capable of meeting both national challenges and global standards. His background in industry, academia and hospital pharmacy provided him with the holistic vision needed to address these deep-seated issues.

Meetings Held

As the Registrar and Secretary to the Council, Pharm. Mora directs the affairs of the Registry, prepares the agenda for Council meetings,

presents issues that require a policy position from the Council and presents issues to the Council pertaining to the regulation of Pharmacy education. Therefore, at various Council sittings, Pharm. Mora had sought the Council's decisions on key issues that had left a lasting mark on the history of PCN. In his first Council meeting of 19th February 2004, Pharm. Mora presented many issues to the Council, and the Council passed thirty-five 35 recommendations, including:

1. That the auditorium of the Pharmacist Council of Nigeria at its Head Office in Idu, Abuja be named after the late Chairman of the Council, Pharm. Dr. F.B. Adenika, in appreciation of his contributions to the Council.
2. He further presented a proposal to the Council for the creation of three additional zonal offices for the South-South, North-East and North Central geo-political zones in the following locations:
 - i. South-South - Uyo
 - ii. North-East - Bauchi
 - iii. North Central - Minna

The following criteria were considered in developing the proposal:

- Status of state capital
 - Presence of a University/Faculty of Pharmaceutical Sciences
 - Premises concentration and
 - Centrality and likelihood of office accommodation being offered.
3. Approval in principle of a committee's recommendation for repositioning the registry for more effective regulation and control functions. However, details of implementation were referred to the APDC for study and recommendations to the Council.
 4. Approval of a Committee's proposal of a special forum for the DPSs to meet with the Chairman during a future meeting of

Council aimed at increasing awareness on the roles of DPSs in an effective and successful implementation of PCN's PPMVL mandate. This is important in consideration of the fact that the Hon. Minister of Health had, on 14th April 2003, reverted the authority to issue Patents and Proprietary Medicines Vendors Licenses (PPMVL) to the PCN. Therefore, the role of DPSs is crucial in administrative processes involved in the issuance of PPMVL, especially since the PCN presence is nonexistent at the state level. The initiation of action on PPMVL was stalled due to the absence of the Council. Hence, with the inauguration of the Council and the appointment of Registrar Pharm. Mora, he initiated processes for the implementation of PPMVL issuance and control.

5. That the Registry produce, in the form of a booklet format, the minimum required standards of practice in all aspects of pharmacy practice and forward the same to all DPSs. This led to the publication of numerous booklets that pertain to the functions of PCN guidelines, which are involved in all aspects related to regulation in Pharmacy.

Key Council's Decisions at its 15th Regular Meeting of May 20, 2004

In this meeting, thirty-two (32) recommendations were considered and approved. The following are the key decisions:

1. That identity cards be issued at N1,000 each to expire after 5 years
2. That license fees from PPMVL be invested in shares, landed property and treasury bills.
3. That the Registry establish and maintain registers for the registration of hospital pharmacies at secondary and tertiary levels, commencing from 2004, while subsequent years would focus on the renewal of licenses, operating premises in hospitals already granted full or provisional registration status.

4. That Hospital Pharmacies in the satellite and complementary centers of some institutions (such as teaching hospitals) be treated as separate operating premises for the purpose of registration, the payment of Annual and Renewal Fees and the issuance of an operating license, insofar as they are in different locations, in consonance with the provisions of community pharmacy practice.
5. That the Certificate of Registration for Pharmacy Technicians /Assistants shall be signed by the PCN Registrar and such certificates should be recognized by the government and other employees for appointment and promotion.
6. 25% of the revenue accrued from annual permits to Pharmacy Technicians would be given to the Pharmacy Technicians Association.
7. That the Curriculum for the orientation programme of Pharmacy Technicians/Assistants be reviewed every five years, starting with the first review in 2005.
8. The Council further approved twenty (20) recommendations from the Education Committee of the Council for upgrading the accreditation process and tools for B.Pharm and Pharmacy Technicians/Assistant programmes.

Key Council's Decisions at its 16th Regular Meeting of August 19, 2004

At this meeting, the Council took twenty-seven (27) decisions, out of which sixteen (16) are on reorganizing and strengthening the Mandatory Continuing Professional Development (MCPD) programme. In this meeting, the decision to construct an outdoor round hut facility was taken and it was financed from the internally generated revenue as approved by the Council.

Key Council's Decisions at its 17th Regular Meeting of December 9th, 2004

The 17th meeting of the Council resulted in broadly thirty-four (34) recommendations on the finances of the Registry and issues related to contracts for the day-to-day running of the Registry. The meeting also approved eight (8) recommendations on MCPD, nine (9) on Pharmacy Technicians, including:

1. The Council upheld its decision on the issue of a national examination for final year Pharmacy Technician students;
2. Uniform programme for the Pharmacy Technician training programme as against a uniform calendar;
3. The Council directed that the guidelines be followed on the issue of the preference for Pharmacy Technicians in the issuance of PPMVL;
4. The Council accepted the need to draw up a 'Code of Ethics for Pharmacy Technicians', which would include dressing in schools and the workplace;
5. That a letter be written to the government at all levels, requesting them to ensure that drugs are handled by Registered Pharmacists and Pharmacy Technicians, and
6. On the dress code for Pharmacy Technicians, the inscription on their overall shall read "P. Technician" as against the earlier approved "P.T," which was rather ambiguous.

At this meeting, the Council also reviewed the registration fees for the Orientation Programme for Foreign-Trained Pharmacists and the fees for the processes involved in the programme. The meeting further decided on the Professional gowns for the induction ceremony of fresh graduates, which are still in use. Lastly, the meeting approved for PCN to organize a summit for the Pharm. D. programme, where the curriculum would be discussed and course content developed.

Key Council's Decisions at its 18th Regular Meeting of February 17, 2005

This meeting yielded twenty-three (23) recommendations on running the Registry, PPMVL signpost, MPCD and, of course, issues related to Pharm. D. The Council also approved that the *Compendium on Minimum Standards of Practice for the Assurance of Pharmaceutical Care* in the four areas, that is, hospital, retail, importation, wholesaling and manufacturing, which was an initiative of the Registrar.

Key Council's Decisions at its 19th Regular Meeting of May 19, 2005

1. The Council approved thirty-nine (39) recommendations for this meeting. As a good administrator and motivator, Pharm. Mora recommended to the Council that PCN staff be paid one month's basic salary as an end-of-year bonus, in appreciation of their hard work in 2004, and it was approved. Other key recommendations are:
2. The Council approved funds for:
 - i. A workshop for inspectors in Abuja;
 - ii. The inauguration of Zonal Offices;
 - iii. The presentation of multimedia equipment to all nine (9) Nigerian universities with approved Faculties of Pharmacy.
3. The Council approved the MCPD Brochure as proposed amendments.
4. The Council approved the PCN-MCPD exemption list consisting of twenty officers.

Key Council's Decisions at its 20th Regular Meeting of August 16, 2005

The 20th meeting yielded fourteen (14) decisions that are basically approvals for funds, reconstitution of Council's Standing Committees and approval of the proposed amendment to the *Code of Ethics for Pharmacists*. In addition, the Council approved a guideline for Public-

Private-Partnership (PPP) in Hospital Pharmacies, reviewed the proposal for the MCPD programme and the procedure for registration of Pharmacists.

Key Council's Decisions at its 21st Regular Meeting of December 8, 2005

The Council had passed twenty-two (22) decisions. Amongst the key decisions are:

1. The approval of the amended Code of Ethics for Pharmacists as presented;
2. The Council approved the amendments to the annual license to practice as presented;
3. The Council considered the proposal for the building of a multi-purpose conference center in the empty space at the Head Office/Registry of PCN and approved the concept, with a proviso that it would be executed strictly on B.O.T. arrangements;
4. The Council considered and approved the prototype sketches for the PCN Zonal Offices and that the Federal Government Capital Grant for 2006 be used for the building of the Uyo and Minna Zonal Offices, where PCN had already secured plots of land.
5. The Council approved the payment of a full month's salary to the Registry staff as the 13th month "*ex gratia* award."
6. The Council approved the development of a Compulsory Continuing Education Programme for Pharmacy Technicians (CCEPT).
7. The Council directed that a standard Hospital Experience Assessment Book be developed for use by Pharmacy Technicians-in-training.
8. The Council approved the constitution of the committee on the Pharm. D. Programme, and its terms of reference are:
 - a. To update the Pharm.D. degree curriculum developed in 2002.;

- b. To provide justification for the change to “Pharm. D.” from the “B. Pharm.” Programme in Nigerian Universities;
- c. To make recommendations on the ways and means of facilitating the commencement of the programme in Nigeria; and;
- d. To consider other related matters and make appropriate recommendations to Council.

Key Council’s Decisions at its 22nd Regular Meeting of March 30, 2006

The March 2006 Council meeting approved thirteen (13) decisions, with the most important one being the consideration of the recommendations of the Law and Ethics Committee on the request for clarification on the draft laws from the Legal Unit of the Federal Ministry of Health, and passed six separate decisions. Other key decisions passed in this meeting include:

1. The Council directed that 50% of the fines collected for the sealing of pharmacy premises be returned to the states to assist them in the enforcement of PCN standards
2. The Council approved the distribution of books to various institutions and stakeholders as presented and further decided to donate some of the books to the University of Liberia.
3. The Council approved the purchase of eight (8) vehicles, following the approval of the President, in the following order of priority:
 - i. Six (6) double-cabin pick-ups, one for each Zone, for inspection
 - ii. One (1) station wagon car for the Head Office
 - iii. One (1) Prado jeep for the Head Office
4. The Council approved the Benchmark – Minimum Academic Standards for the Pharm. D. Degree programme and the justification for the Pharm. D. degree programme as presented.

Key Council's Decisions at its 23rd Regular Meeting of July 13, 2006

The July meeting resulted in thirty-one decisions, with the majority of these decisions featuring in the submissions from the Law and Ethics and Finance and General-Purpose Committees. The issues discussed in this meeting include setting standards for Hospital Pharmacies and how they will be registered, decisions on Pharmacies located in Army barracks and “mammy markets”, awarding contracts and making payments for commitments and issues related to discipline and litigation.

Key Council's Decisions at its 24th Regular Meeting of December 7, 2006

The Council passed forty-four (44) decisions at its 24th sitting. The decisions include:

1. Renaming of PCN Zonal Offices to PCN Area Offices and the approval of twelve (12) Area Offices for PCN as recommended by PCN-RIC, as in Table 4.1

Table 4.1: List of the newly created PCN Area Offices with their area of coverage and headquarters.

S/No.	Area Office	States to be covered by the Area Office	Headquarters
1	Lagos	Lagos	Lagos
2	Ibadan	Oyo, Osun, Ogun	Ibadan
3	Benin	Edo, Ondo, Delta, Ekiti	Benin
4	Enugu	Enugu, Anambra, Ebonyi	Enugu
5	Owerri	Imo, Abia, Bayelsa	Owerri
6	Uyo	Akwa Ibom, Cross Rivers, Rivers	Uyo
7	Kano	Kano, Katsina, Jigawa	Kano
8	Kaduna	Kaduna, Zamfara, Sokoto Kebbi	Kaduna
9	Bauchi	Bauchi, Gombe, Taraba, Plateau	Bauchi
10	Maiduguri	Borno, Yobe, Adamawa	Maiduguri
11	Minna	Niger, FCT, Nasarawa	Minna
12	Ilorin	Kwara, Kogi, Benue	Ilorin

2. The Council approved the payment of one month's basic salary to all registry staff as an end-of-year ex gratia award.

3. The Council considered and approved the draft *Inspector's Handbook*.

Key Council's Decisions at its 25th Regular Meeting of April 5, 2007

The 25th regular meeting broadly yielded twenty-eight (28) decisions. The key decisions in this meeting include:

1. The approval of the reviewed draft of the *Continuing Education Handbook for PPMVs* with the amendments as presented. Other key decisions are:
2. The Council considered and approved the proposed Form MI for renewal of the PPMV License, which is retitled Application for Issuance of PPMV License, while a new form MII should be titled Application for Renewal of PPMV License
3. Council approved the proposal that the list of graduands for induction from the Faculties of Pharmacy be endorsed by both the Dean of Pharmacy and the Registrar of the University or his representative.

Key Council's Decisions at its 26th Regular Meeting of August 2, 2007

The Council, at its 26th meeting, deliberated and passed nineteen (19) decisions. In this meeting, the Council approved the:

1. The proposal that a statement which would empower the PCN to make claims and counter claims to recover all cost of litigation instituted against it by registered Pharmacist and/or registered premises be written or made at the back of PCN's forms for the registration of both Pharmacists and Pharmaceutical premises to wit, "*Take notice that the Pharmacist Council of Nigeria (PCN) shall make a claim and recover all costs of litigation incurred by it in defense of any court action instituted against it at the instance of any registered Pharmacist and/or registered Pharmaceutical Premises and whereby the suit is struck out, withdrawn or the Pharmacist or the premises loses the case.*"

2. The Council considered the recommendation on the attitudes of pharmacy graduates and noted the need for collective responsibility for better professional behavior through:

- a. Proper dress code,
- b. Proper dispensing habit,
- c. Respect for seniority, and
- d. Character molding from school.

During Pharm Mora's tenure as the Registrar of the Pharmacists Council of Nigeria, many other key decisions were taken by the Council, where he served as the Secretary, who was responsible for documenting Council proceedings and drafting the Council's agenda of meetings and indeed the agenda of the various standing committees of the Council. His visionary leadership had helped in running the Registry and in steering the Council towards impactful and meaningful contributions for better pharmaceutical services, education and training.

Major Achievements and Challenges

Addressing Pre-2003 Challenges: Mora's Strategic Reforms

Of course, Pharm. Mora's tenure as Registrar of the PCN (2003-2012) was fundamentally a period of strategic reform and institutional transformation. He addressed the pre-2003 challenges with a clear, multi-pronged approach, moving the PCN significantly closer to the operational standards of global regulatory agencies. Through focused hard work, resilience and the ability to manage human resources, he had addressed each of the challenges, and his key contributions are:

1. Weak Enforcement & Proliferation of Illegal Outlets

During Pharm Mora's tenure as Registrar of PCN, the Council recognized that the Pharmaceutical Inspection Committees (PIC) at the state level were not functioning and, in some states, they were nonexistent. This situation has hampered effective monitoring of pharmaceutical activities in the states. Therefore, the Council

mandated the Registry to conduct a nationwide assessment of PICs at the end of which it was realized that the PICs lacked vehicles, budgetary allocations from State Governments, honoraria or allowances for the inspectors. The PICs are thought to be the responsibility of the PCN, and in some States, they are not constituted. These shortcomings led the Council to recommend that the vehicles be purchased for the State PICs in addition to the recommended minimum welfare package for the inspectors. The Council also reviewed the 50% inspection fee being paid to State PICs to include remitting a portion of the registration fee as well. The Council further recommended that the inspectors be provided with jackets bearing the inscription "PCN Inspector," annual identification cards and the provision of proper security during inspections in the form of co-opting Police into the teams.

A workshop for pharmaceutical inspectors was organized to ensure that only drugs of proven efficacy, safety and quality reach the healthcare delivery system. Additionally, the Council organized an interactive meeting with the Pharmaceutical Inspectors in order to harmonize their activities nationwide and impart practice requirements for Good Manufacturing Practice (GMP) and Good Layout Practice (GLP). The measures taken significantly led to robust Inspection through the "Task Force" Approach method. Therefore, Pharm Mora revitalized and empowered the Council's inspection directorate. He advocated for a "Task Force on Fake Drugs" initiative, leading to the systematic sealing of hundreds of illegal and non-compliant pharmacy premises. His collaborative approach further strengthened collaboration with NAFDAC and other security agencies, creating a more formidable front against the counterfeit drug market and sanitizing the drug distribution chain.

2. Administrative & Structural Deficiencies

In June 1998, the Pharmacist Council of Nigeria (PCN) solicited the assistance of the Office of Establishments and Management Services

to assess the staffing levels of the Council, with a view to creating additional positions to enhance efficiency in the registry. The terms of reference for the team were to conduct a staff audit of the departments and units within the PCN and examine the job descriptions of existing positions, with a view to determining their staffing levels. The second term of reference was to conduct a study of the present and proposed functions of the Council in relation to the Pharmacist Council of Nigeria Decree No. 91, which established the Council, with the aim of designing an appropriate organizational structure and prescribing proper job descriptions for positions within the Council. Lastly, to consider other matters which, in the opinion of the Consulting Team, might contribute to the overall efficiency and effectiveness of the Council. The team's recommendations suggested the need for restructuring of the respective departments, employing additional staff to improve the Council's staffing capacity and a review of job descriptions for the principal officers. Pharm Mora took advantage of this report and initiated changes that impacted the structure and functions of the departments, addressing the acute staff shortage and, for the first time, introduced the innovative decentralization model known as Zonal Structures.

Previously, most regulatory activities were centralized at the PCN headquarters in Abuja, along with three zonal offices in Lagos, Enugu and Kano. This created bottlenecks and made it difficult for the Council to effectively oversee the vast and diverse Nigerian pharmaceutical landscape, given the country's size and population. The decentralization of the Pharmacists Council of Nigeria (PCN), primarily through the establishment of state offices and zonal structures, brought several significant advantages. Decentralization was aimed at improving regulatory efficiency and speed, enhancing monitoring and enforcement, increasing accessibility and stakeholder engagement and contextualizing regulation and governance. Other advantages of decentralization include convenient professional Development and capacity building, especially with the localization

of Continuing Professional Development (CPD) Programmes, as well as the provision of Career Opportunities for Regulators.

Pharm Mora established additional zonal offices in Uyo, Minna and Bauchi, enabling the PCN to cover each of the geopolitical zones. Additionally, Area Offices were established in Benin City, Owerri, Ilorin, Ibadan, Kaduna and Maiduguri. This brings the total of Zonal and Area Offices to twelve. By 2008, 35 state offices of the PCN had been established nationwide. Therefore, Pharm A. T. Mora increased the offices of PCN from the four he met to forty-four by 2009.

Similarly, Pharm Mora met thirteen (13) official vehicles at the Council out of which only nine (9) were roadworthy. Through his foresight and dedication towards having a well-regulated Pharmacy practice, Pharm Mora laid a gradual plan for making the Headquarters and the area office have an adequate number of vehicles for smooth discharge of its function. By the end of his tenure, he had purchased an additional thirty-three (33) brand new vehicles for the Council, thereby leaving behind forty-two functional vehicles. The phenomenal growth of PCN automatically led to the growth of its staff from 99 in 2003 to 272, thereby matching infrastructural expansion with employment opportunities for our trained Pharmacists.

3. Outdated Educational Standards

At the beginning of Pharm. Mora, in his capacity as Registrar, presented to the council the need for a review of accreditation tools. The Council, at its 15th Regular Meeting on May 20, 2004, deliberated and decided on the amendments to PCN B. Pharm accreditation tools. The modification includes adjusting the academic to non-academic staff ratio, which is to be the same as that of the National Universities Commission (NUC). Similarly, the scores for the percentage of pharmacists in the ratio for professional and non-professional staff were adjusted to encourage faculties to have more pharmacists in their workforce. Other considerations, such as funding for academic activities, teaching support facilities, and student interaction, were

revised to enhance teaching, learning and performance. Therefore, Pharm. Mora was known to have strengthened accreditation exercises. He reinforced the accreditation process for faculties of pharmacy, ensuring institutions were held to higher standards of infrastructure, staffing and educational quality. In the same vein, Pharm. A. T. Mora embarked on Curriculum Modernization. He played a pivotal role in the review and updating of the Benchmark Minimum Academic Standards (BMAS) for pharmacy education in Nigerian universities. This ensured the curriculum began aligning with global shifts towards patient-focused "pharmaceutical care" and clinical pharmacy.

4. Erosion of Professional Boundaries & Low Political Will

The unprecedented three-term chairmanship of the Committee of Registrars of the Health Professions by Pharm. A. T. Mora, while a testament to his personal influence and diplomatic skill, unfolded against a complex backdrop that arguably contributed to an erosion of professional boundaries for pharmacy and exposed a concerning level of low political will for substantive healthcare reform. From the perspective of many within the pharmacy profession, this prolonged period, while potentially beneficial for inter-professional dialogue, created a strategic dilemma. The Registrar of the PCN, the paramount defender of pharmacy's unique scope of practice, was simultaneously chairing a committee comprising the Registrars of often competing professions, such as medicine and nursing. This dual role, though prestigious, risked blurring the lines of professional advocacy. In the delicate and often contentious ecosystem of Nigerian healthcare, where turf wars over prescribing rights, ownership of premises, and clinical responsibilities are frequent, the need for a strong, unambiguously partisan voice for pharmacy is critical. There was a palpable concern that the imperative to maintain collegial harmony and consensus within the committee could inadvertently soften the firm, assertive stance required to resist encroachments from other

professions and to forcefully advocate for the expansion of pharmacy's role in patient care.

This situation was compounded and perhaps even enabled by the pervasive low political will from the federal government to decisively address the structural and regulatory ambiguities within the health sector. The very fact that such a committee required a chairman to serve for three consecutive terms suggests a systemic stagnation, a lack of political drive to implement the clear, binding policies that would have made such a prolonged consensus-building role less necessary. True political will would have been demonstrated by empowering each professional regulatory body with the legislative and executive backing to firmly govern its own domain without constant inter-professional negotiation over fundamental practice rights. Instead, the government's apparent inertia created a vacuum where leadership within committees like the CRHP became a substitute for decisive governmental action. Consequently, Mora's long tenure, rather than being a sign of progressive reform, can be interpreted as a symptom of a system in a holding pattern, where professional boundaries were maintained through continuous negotiation rather than being solidified and protected by robust, unambiguous laws and regulations. The erosion was subtle, not in overt concessions, but in the potential for a defensive, rather than offensive, posture in advancing the pharmacy profession, all while the political machinery that should have been the ultimate arbiter remained disengaged from making the tough decisions needed to modernize and clarify the roles of Nigeria's health professionals.

Therefore, the election of Pharm. Mora, as Chairman of the Committee of Registrars of the Health Professions (CRHP) for three terms (2006-2012), was a masterstroke. This platform allowed him to foster dialogue and collaboration, reducing friction and building a united front on common health issues, thereby strengthening the PCN's political standing.

This further elevated his Pharmacy profession's profile through his active leadership and the Council's more visible enforcement actions. He raised the political and public profile of pharmacy regulation, garnering more attention and, by extension, stronger political will.

5. Ethical Re-orientation and Professional Development:

Pharm Mora placed a strong emphasis on professional ethics and continuous development. He mandated and facilitated Mandatory Continuing Professional Development (MCPD) programme for pharmacists, ensuring they remained current with evolving knowledge and practices. In fact, the Council, in an effort to strengthen the MCPD programme, provided one multimedia projector and one computer system to all the MCPD provider centers across the country in 2005. The Council also introduced quality assurance measures in the MCPD programme and mandated the centers to computerize all their activities as far back as 2004. The Council further laid down a foundation for hosting the MCPD programme on the PCN website. He also used his platform to consistently advocate for the highest standards of ethical conduct, reinforcing the pharmacist's role as the custodian of public health in the medicine use process.

6. Improvement of Hospital Pharmacy Practice

Under the leadership of Pharm Mora, PCN dedicated 2007 as the year for hospital pharmacy practice. This was prompted by the realization that the hospital pharmacy was the oldest and most patronized specialty of pharmacy in Nigeria. Moreover, hospital pharmacies are the largest employers of pharmacists, especially when considering the primary, secondary and tertiary healthcare systems in Nigeria. In that year, the focus of the PCN was on improving hospital pharmacy in Nigeria. The Council organized a Nationwide Advocacy Visit and Interactive Sessions with hospital pharmacists, followed by a Follow-up Workshop on Improving Hospital Pharmacy Practice in Nigeria.

The essence of the interactive sessions with Hospital Pharmacists was to enlighten the professionals in the hospital sector on the new

developments that emerged, such as the shift towards PharmD as the minimum degree for practice and what it entails in the provision of Pharmaceutical Care through follow-ups, ward rounds and provision of drug information to other professionals in the hospital and the patient as well. Other important areas that were articulated and presented to hospital pharmacists included the issue of Private Public-Partnership (PPP), which was a concept at the time, following the apparent failure of the Drug Revolving Fund (DRF). Similarly, the hospital pharmacist is encouraged to update themselves through the MCPDs programmes and prepare themselves to address emerging diseases, such as HIV/AIDS and ACTs in the treatment of malaria. Importantly, issues of *pharmacovigilance*, *adverse drug reactions*, *concordance*, etc are raised in the interactive sessions. Generally, the interactive session was organized to promote the advancement of hospital pharmacy practice in providing quality and improved services to patients, the healthcare team and the general public.

To consolidate the benefits derived from the interactive session with hospital pharmacists, the Council further organized a follow-up workshop and charted a direction for the PCN's renewed commitment to hospital pharmacy. All Heads of pharmacy departments in Nigerian tertiary health institutions were invited. At the end of the workshop, twenty-three (23) resolutions were taken to strengthen hospital pharmacy and align it with what obtains in the developed nations.

Key Contributions to the PCN

1. **Institutional Strengthening and Expansion:** His most tangible contribution was the physical and administrative expansion of the PCN, moving it from a centralized entity to a nascent nationwide regulator with developing zonal structures.
2. **Modernization of Pharmacy Education:** By driving the update of the BMAS, he directly shaped the competence of future pharmacists, ensuring they were better prepared for modern roles.

3. **Enhanced Regulatory Muscle:** The aggressive inspection and enforcement campaigns under his leadership restored a significant degree of authority and fear to the PCN's regulatory mandate. It was no longer a "paper tiger."
4. **Fostering National and Regional Collaboration:** His role in the CRHP improved inter-professional relations in Nigeria's health sector, while his prior involvement with WAPMA provided a valuable regional perspective.
5. **Ethical Re-orientation and Professional Development:** He championed Continuing Professional Development (CPD) programme, embedding the culture of lifelong learning into the profession, which is a cornerstone of modern regulatory practice.

Moving the PCN Forward: A Comparison with Global Agencies Post-Reforms

Pharm Mora's reforms, while not instantly elevating the PCN to the level of long-established Western agencies, succeeded in bridging a significant portion of the gap and, most importantly, set the institution on a corrective and ambitious trajectory. Prior to 2003, the PCN was a weak, under-resourced and slow-moving body; however, under Mora's leadership from 2003 to 2012, it underwent a fundamental transformation. This progress is evident across several key areas. In terms of enforcement, the PCN shifted from being ineffective to establishing a proactive inspection regime with tangible consequences, such as the sealing of non-compliant premises, thereby moving it closer to a credible enforcement model. Geographically, the agency began its crucial journey, transitioning from a centralized and ineffective structure in Abuja to initiating decentralization, laying the foundational work for zonal offices that would enable true nationwide coverage. Furthermore, the static educational standards, which had become increasingly disconnected from global practice, were addressed through the modernization of the BMAS curriculum,

aligning Nigerian pharmacy education more closely with the international patient care model.

The PCN's professional stature was also elevated. Once marginalized and often embroiled in territorial disputes, its leadership of the Committee of Registrars of Health Professions (CRHP) granted it a respected seat at the highest table of health sector policy, mirroring the collaborative approach seen in advanced systems. Finally, a culture of Continuing Professional Development (CPD) programme was established, transitioning it from a voluntary and unstructured concept to a formalized framework, laying the essential groundwork for it to become a mandatory requirement for license renewal. In essence, Pharm Mora inherited a struggling, reactive regulator and reshaped it into a more proactive, respected and effective institution. He addressed its core weaknesses not with isolated fixes, but with a comprehensive strategic vision that targeted its fundamental structure, core processes and external relationships. While challenges remained, his tenure successfully positioned the PCN at the center of Nigeria's healthcare landscape, setting it on a definitive path toward global best practices by recognizing that effective regulation is built on the pillars of strong enforcement, modern education and strategic collaboration.

Concrete Actions PCN Took (2003–2012)

During this period, the Pharmacists Council of Nigeria (PCN) embarked on a series of key, documented initiatives and operational actions to strengthen its regulatory mandate. A cornerstone of this effort was the significant decentralization of its operations, marked by a deliberate expansion of its physical presence nationwide. Under the leadership of Pharm. Mora, the number of PCN offices rose substantially from a mere handful to a much wider network of zonal and state offices, a major reform that directly improved local licensing and inspection capabilities. This expanded reach was coupled with a rigorous campaign of strengthened inspections and premises

regulation, whereby the Council increased the frequency of routine inspections targeting pharmacies, wholesalers and various drug outlets. This was reinforced by a decisive escalation in enforcement actions, including closures, warnings and license sanctions against illegal operators and poorly managed premises, with a particular emphasis on sealing non-compliant facilities and conducting diligent follow-up visits.

Concurrently, the PCN established tighter control over licensing and professional registers, placing a strong emphasis on maintaining accurate and up-to-date rolls of pharmacists, pharmacy technicians, and licensed premises. This was supported by a more robust vetting process for licensing applications, all of which formed part of a broader strategy to professionalise drug distribution points and reduce unqualified practice. Recognizing the complex nature of the pharmaceutical landscape, the Council also prioritized collaboration with other agencies and launched public awareness campaigns. It worked closely with NAFDAC, the Police, the Pharmaceutical Society of Nigeria (PSN) and Customs Services on joint enforcement drives against open drug markets and counterfeit medicines, with its public messaging often highlighting the proliferation of unqualified vendors as a key driver of the fake drug trade, thereby elevating the prominence of multi-agency enforcement. Furthermore, the PCN sharpened its focus on education, training and accreditation, undertaking systematic accreditation and inspection of pharmacy training institutions. It enforced stringent standards for pre-registration training and internships, making critical interventions to harmonize curricula and practical training and publicly articulating the necessity to address the fundamental challenges affecting pharmacy education. To underpin these wide-ranging reforms, the Council pursued institutional strengthening and publicly advocated for IT modernization, with its leadership articulating clear plans to computerize critical aspects of the drug distribution and regulatory processes, as well as to professionalize internal systems for registers

and record-keeping, all aimed at improving traceability and overall oversight.

These concerted actions yielded several tangible improvements, most notably a greater regulatory footprint and a raised profile for the Council. The establishment of more PCN offices and a stronger inspectorate presence directly translated into more frequent inspections and more efficient local licensing administration, which in turn enabled the agency to identify and act against illegal premises with greater speed. Simultaneously, the PCN, under this leadership, became a more visible and influential voice in the public debate concerning fake drugs and professional standards, actively working with partners such as NAFDAC and the PSN to elevate the importance of regulation.

However, significant and deeply entrenched challenges persisted despite these comprehensive efforts. The pervasive issue of counterfeit and substandard medicines continued to plague Nigeria, as the problem proved resilient to enforcement and public awareness campaigns. This persistence was due to the problem's multi-factorial nature, involving immense supply chain complexity, porous national borders, a vast network of non-professional sellers and overarching resource constraints. As subsequent scholarly work and policy reviews have confirmed, counterfeit drugs remained a major public health challenge long after this period of reform, underscoring the limits of even a robust regulatory campaign against such a systemic issue.

Contributions of Pharm. A. T. Mora to the Pharmaceutical Society of Nigeria (PSN)

During his tenure as the Registrar and Chief Executive Officer of the Pharmacists Council of Nigeria (PCN) from 2003 to 2012, Mora made profound contributions that fundamentally strengthened the Pharmaceutical Society of Nigeria (PSN) by reinforcing the very regulatory framework that underpins the profession. As the chief regulatory officer, he spearheaded critical reforms that modernized pharmacy

education and practice standards nationwide, initiatives directly aligned with the PSN's core mission of advancing the profession. He championed the revision and modernization of pharmacy training curricula to meet contemporary needs, ensuring that new graduates were better equipped for the evolving landscape of healthcare. Furthermore, he significantly strengthened the PCN's inspection and enforcement framework, raising the bar for professional ethics and operational standards within community pharmacies, hospital pharmacies and industrial practices, thereby enhancing the integrity and public perception of the profession the PSN represents. His leadership also fostered greater inter-professional collaboration and provided robust regulatory oversight, which collectively served to elevate the status of pharmacists and solidify the PSN's role as a key stakeholder in the Nigerian health sector. Through this foundational work in regulation and standards enforcement, Pharm Mora provided an indispensable service to the PSN, creating a more structured, respected and professional environment for all its members.

Professor Mora's contributions extended significantly into the realm of pharmacy regulation, which is intrinsically linked to the PSN's objectives of maintaining professional standards. His long and distinguished service with the Pharmacists Council of Nigeria (PCN), the profession's regulatory body, saw him occupy nearly every possible leadership position. He served as a Member of the Council representing the PSN from 1999 to 2002, then as the Registrar and Chief Executive Officer from 2003 to 2012. In these capacities, he was a direct agent of the PSN's mandate to uphold ethics and standards. As Registrar, he spearheaded critical reforms that enhanced pharmacy education curricula, strengthened inspection frameworks for pharmaceutical premises and promoted robust professional ethics nationwide, thereby elevating the practice of pharmacy and safeguarding public health.

The contributions of Pharm Mora are many and multifaceted. In 1999, he served as the Chairman of the PSN Conference Planning

Committee (CPC) for the “Coal City” Conference held in Nike Lake Hotel, Enugu. In this conference, Pharm Mora invited Prof. Umar Shehu to serve as the Keynote Speaker and talk on the “*Resurgence of Tuberculosis*”; The Conference was a huge success. In the same vein, Pharm Mora played a crucial role in the PSN national conference in Bauchi in 2004. His close relationship with Governor Ahmed Adamu Mu’azu of Bauchi, who was his friend and contemporary at the university, led him to donate the sum of five million naira (N5,000,000.00) to PSN.

Pharm Mora had the Pharmaceutical Society at heart; his observation that some professional bodies were securing grants from the Federal Government, while the PSN was not, led him to utilize his contacts to secure a thirty-five-million-naira (N35,000,000.00) grant in 2008. However, only twenty-five million naira (N25,000,000.00) was released through PCN’s subhead and subsequently transferred to PSN. In 1993 and 1994 at PSN Annual National Conferences in Ibadan and Port Harcourt, Pharm Mora contested for the position of Ex-officio member of the PSN and won.

The Challenges Faced by Pharm A. T. Mora as the Registrar of PCN

The greatest challenge for Pharm. Mora was involved in the conflict between PCN and NAFDAC, both of which were government parastatals, as well as his conflict with PSN. The conflict with NAFDAC was a result of the differing personalities of the respective Chief Executive Officers of the two agencies and, of course, the thin boundary that separates some aspects of their respective duties. Both CEOs share a common zeal for discharging their responsibilities, hard work, determination, boldness and, indeed, they are all fearless. While the conflict between Pharm Mora and the PSN could be attributed to the existence of some cabals in PSN headquarters that insist on having their way all the time for their selfish interests and, of course, the “stubborn” Mora, who will not allow them to have their way.

1. The Clash of Mandates: NAFDAC vs. PCN (2004)

The conflict between the National Agency for Food and Drug Administration and Control (NAFDAC) and the Pharmacists Council of Nigeria (PCN) in 2004 was a significant and profound event in Nigeria's public health sector, representing not a simple dispute but a deep-seated institutional clash over regulatory territory, professional jurisdiction and the very interpretation of the law. The root cause was the overlapping and ambiguous mandates granted to both agencies concerning the regulation of pharmaceutical premises and products, which created a protracted power struggle over who held the ultimate authority to license, inspect and sanction pharmacies and patent medicine stores.

2. Ambiguity and Overlap in Enabling Laws

This foundational conflict arose from provisions within the enabling legislation of both NAFDAC and the PCN, which, when interpreted differently, led to direct confrontation. The PCN, established by Decree 91 of 1992, was mandated to regulate and control the practice of pharmacy in all its aspects, a scope that explicitly included the registration of pharmacists, the registration of premises for the manufacture, sale and distribution of drugs and the critical function of issuing licenses to practice and operate pharmaceutical premises. Conversely, NAFDAC, created by Decree 15 of 1993 (later the Act Cap N1 LFN 2004), was endowed with a broader public health focus, specifically to regulate and control the importation, exportation, manufacture, advertisement, distribution, sale and use of all food, drugs, cosmetics and other regulated products, which included conducting inspections of production and distribution premises. Critically, the NAFDAC Act contained a "supremacy clause" stating that its provisions would prevail over any other law in the event of a conflict, a point NAFDAC would heavily rely upon. The central conflict point emerged because both agencies had legal backing to "register premises" and "inspect" facilities involved with drugs; a pharmacy was simultaneously a "pharmaceutical practice" under the

PCN's purview and a "premise for the distribution/sale of a regulated product" under NAFDAC's, creating a classic and intractable case of duplicated functions and jurisdictional overlap.

3. The Catalyst: NAFDAC's Aggressive Anti-Counterfeiting Campaign

This latent tension was ignited by the context of NAFDAC's aggressive anti-counterfeiting campaign led by its formidable Director-General, the late Professor Dora Akunyili. This widely praised national and international effort created a powerful operational impetus for the agency. Professor Akunyili and NAFDAC argued that to effectively combat the dangerous counterfeit drug trade, which constituted a public health emergency, the agency needed uncompromised control over the entire supply chain, including the final point of sale—the pharmacy and patent medicine store. From this perspective, PCN's more traditional regulatory approach was perceived as neither stringent nor swift enough to address the crisis. Consequently, NAFDAC began independently licensing, inspecting and sealing pharmaceutical premises it deemed non-compliant, often without coordinating with the PCN, an action that, from the Council's viewpoint, was a direct and unacceptable invasion of its core professional territory.

4. The Immediate Trigger: The "Port Harcourt Raid"

The conflict escalated from a war of words to a physical and public confrontation in 2004, crystallized in the "Port Harcourt Raid." During a routine enforcement operation, NAFDAC officials raided and sealed several pharmacies in Port Harcourt, Rivers State, for allegedly stocking unregistered or counterfeit drugs. The PCN, under its Registrar, Mora, and the PCN Council responded forcefully, arguing that these pharmacies were already properly registered and licensed by the Council. They publicly accused NAFDAC of overstepping its legal bounds by shutting down premises that were legally operating under PCN's regulations. This event triggered an acrimonious public

standoff, with the PCN insisting that NAFDAC's role was confined to regulating products, while the PCN's role was to regulate premises and practitioners. NAFDAC countered that it was impossible to separate the two, asserting that unsafe premises and unqualified personnel were the very channels through which dangerous products reached the public.

5. Professional Jurisdiction vs. Public Health Mandate

Ultimately, at its philosophical core, the conflict represented a fundamental clash between two different mandates and worldviews. The PCN, as a professional council, saw itself as the guardian of the pharmacy profession tasked with ensuring that only qualified pharmacists operated in environments that met strict professional standards, with its primary focus being on the practitioner. In contrast, NAFDAC, as a public health agency, viewed its mission as protecting the entire Nigerian populace from harmful products, regardless of the seller's professional status, with its focus unequivocally on the product's safety for the consumer. It was this divergence in primary focus, professional jurisdiction versus a sweeping public health mandate that underpinned the clash between NAFDAC's aggressive, enforcement-heavy approach and the PCN's more traditional, profession-oriented regulatory style.

6. Government Intervention and the "MoU"

The public and operational conflict between NAFDAC and the PCN escalated to a point where it became profoundly disruptive to the national drug distribution system, compelling the Federal Government to intervene directly as a mediator and arbiter. In 2004, the government officially mandated that the two agencies resolve their differences and establish a clear and workable framework for cooperation. The principal outcome of this high-level intervention was the creation and signing of a Memorandum of Understanding (MoU), a critical document that definitively delineates their roles and responsibilities. This agreement established a fundamental and lasting distinction

between the two regulatory bodies. Within this new framework, NAFDAC was formally recognized as the primary agency responsible for guaranteeing the quality, safety and efficacy of drugs and all other regulated products throughout the entire distribution chain, from importation and manufacture to the final point of sale. Concurrently, the PCN was unequivocally reaffirmed as the sole agency with the authority to license all pharmacy premises and patent medicine stores and regulate the practice, conduct and qualifications of the pharmacy professionals themselves. In essence, the MoU created a symbiotic regulatory dichotomy: while NAFDAC ensures *what*, that the drug product itself is genuine, safe and effective, the PCN ensures the *where* and the *who*, that the premises from which it is dispensed are properly approved and that the personnel handling the medicines are fully qualified and professionally accountable.

6. The Clash with PSN

Pharm Mora had a serious clash with PSN that had administrative and legal implications as the Registrar of PCN. The PSN claimed that Pharm. Ahmed Tijjani Mora, as Registrar of the PCN, altered or "changed the list" of nominees submitted by the PSN for membership into the Pharmacists Council of Nigeria (PCN), a claim that was a highly severe and contentious accusation.

First, it is essential to understand that the Pharmacists Council of Nigeria (PCN) is a statutory body with a governing council composed of members from various stakeholders. The Pharmaceutical Society of Nigeria (PSN) is, by law, entitled to nominate eight (8) number of individuals to be appointed as members of the Council. The Directors of Pharmaceutical Services of the 36 states, including the Federal Capital Territory, Deans of the approved Faculties of Pharmacy and the representative of the Armed Forces, who is also a pharmacist and the DG of NIPRD. Additionally, the President of the Pharmaceutical Society of Nigeria is a member, as is the Director of Foods and Drugs in the Federal Ministry of Health who must be a pharmacist.

Pharm Mora vehemently denied any wrongdoing. His defense rested on a technical and legal interpretation of the process that the final authority to appoint council members lies with the Honourable Minister of Health, not the PSN or the Registrar. The PSN submits its recommendations, but the Minister has the prerogative to approve, reject or request new nominations. Specifically, the Pharmacists Council of Nigeria (PCN) has denied the allegation by the Pharmaceutical Society of Nigeria (PSN) that its registrar, Mora, appointed some members to the PCN Council single-handedly. The Council further stated that the Registrar is not the appointing authority and could not have unilaterally appointed members of the Council as being peddled.

The specific issue became a key part of the broader conflict that led to an intense public feud between the PSN and Pharm. Mora and his supporters. Eventually, it led to Legal and Political battles. The PSN primarily decided to "de-fellow" Pharm. Mora and placed him on a "roll of dishonour." The matter was subject to litigation and the final outcome was a ruling by an appellate court that the PSN's punitive actions against Mora were unconstitutional because he was not given the right to a fair hearing. The court also determined that the PSN lacked jurisdiction over the matter, as the issues related to Mora's role as the former PCN Registrar, which fell under the PCN's jurisdiction, not the PSN's. *(see Appendix III for the Judgement of the Court of Appeal which ruled in favour of Pharm. Mora in 2012).*

Similarly, the PSN petitioned the Independent Corrupt Practices and Other Related Offences Commission (ICPC) to investigate Pharm Mora for alleged abuse of office and corruption. The ICPC concluded its investigation and found no evidence of wrongdoing on the part of Pharm. Mora. The ICPC's investigation not only cleared him but also validated his core defense against the PSN's allegations. The ICPC's verdict was a major turning point in the public perception of the clash between Pharm Mora and PSN because it shifted the narrative from

the story of "one Registrar accused of corruption" to "a reformist regulator falsely accused of doing his job." It further validated Mora's stance and weakened the PSN's position. The failure of such a serious petition to a major anti-corruption agency significantly weakened the credibility of the allegations against Mora. ***(See Appendix IV for the Letter from ICPC clearing Pharm. Mora of any malfeasance).***

In conclusion, Mora inherited a struggling, reactive regulator in 2003 and transformed it into a more proactive, respected and effective institution despite the enormous challenges. His key contributions included institutional strengthening and expansion through decentralization, the modernization of pharmacy education, enhanced regulatory muscle through aggressive enforcement and fostering national and regional collaboration. He also championed ethical re-orientation and professional development. While challenges undoubtedly remained, such as the persistence of counterfeit drugs and the need for full technological integration, his tenure successfully addressed the PCN's core weaknesses with a comprehensive strategic vision. He moved the PCN from being a peripheral player to a central actor in Nigeria's healthcare landscape and set it on the correct trajectory toward achieving global best practices in pharmacy regulation.

Pharm. A.T. Mora's Major Achievements as Registrar from 2003 to 2012

S/N	Parameter	As at 2003	December, 2012	Remarks
1.	Staff Strength	99	272	PCN Is the Professional Regulatory body with highest number of staff
2.	Total No of Offices	4	44	Expansion in keeping both the approved morning level and taking regulatory activities of the PCN to grassroots level
	State Offices	None	30	
	Zonal Offices	3	8	
3.	Vehicle (Cars, Buses, Pick-up)	9	50	Inspection of Pharmaceutical Premises greatly improved.
4.	Prototype-built Zonal Office	None	5 at Minna, Bauchi, Ibadan, Uyo and Kaduna	North west Zonal Office, Kaduna almost complete
5.	Remodeled Office	None	Two (at Lagos and Enugu)	Lagos, Enugu a Head Office remodeled to accommodate more staff and attend to more stakeholders
6.	Total Value of Land Properties	N252,172,325,00 (as per valuation in 2006)	N891,043,888,00 (as per Valuation in March 2011)	353% increase in value
7.	Minimum Standards for Practice	None	Available to Professionally edited and standardized publication	Publicly presented in 2006 by Prof. Eytayo Lambo. 2 nd edition 2023
8.	Number of Universities with approved B.Pharm Programme	9	13	Ten (10) other Universities have the programme in varying degrees of approval

9.	Patent and Proprietary Medicine Vendors Licenses (PPMVL) (Supervision)	None	Fully in place with PPMVL Committees in each state an FCT	Inaugurated the PPMVL Committees in each state an FCT in 2004																																													
10.	Mandatory Continuing Professional development (MCPD) Programme	In place	Fully Restored with participation of development partners such as PATHS, DFID Technical groups and Diaspora Pharmacists	The PCN is the only regulatory body with a comprehensive MCPD Programme since year 1998 after the National Health Summit																																													
11.	Computers	10 Systems (Desktop)	185 71 Desktops 144 Laptops 2 Servers 10 Projectors	To ensure that all staff are I.T compliant																																													
12.	Staff Training	Minimal and disjointed	<table> <tr> <th>Year</th><th>CMD</th><th>ASCON</th><th>Others</th><th>Staff Beneficiary</th></tr> <tr> <td>2004</td><td>24</td><td>-</td><td>56</td><td>80</td></tr> <tr> <td>2005</td><td>25</td><td>-</td><td>17</td><td>42</td></tr> <tr> <td>2006</td><td>5</td><td>37</td><td>22</td><td>64</td></tr> <tr> <td>2007</td><td>21</td><td>15</td><td>1</td><td>37</td></tr> <tr> <td>2008</td><td>10</td><td>45</td><td>46</td><td>101</td></tr> <tr> <td>2009</td><td>153</td><td>10</td><td>103</td><td>266</td></tr> <tr> <td>2010</td><td>7</td><td>71</td><td>2</td><td>80</td></tr> <tr> <td>2011</td><td>82</td><td>-</td><td>55</td><td>137</td></tr> </table>	Year	CMD	ASCON	Others	Staff Beneficiary	2004	24	-	56	80	2005	25	-	17	42	2006	5	37	22	64	2007	21	15	1	37	2008	10	45	46	101	2009	153	10	103	266	2010	7	71	2	80	2011	82	-	55	137	To Training and develop all cadre of Staff toward ensuring effective performance
Year	CMD	ASCON	Others	Staff Beneficiary																																													
2004	24	-	56	80																																													
2005	25	-	17	42																																													
2006	5	37	22	64																																													
2007	21	15	1	37																																													
2008	10	45	46	101																																													
2009	153	10	103	266																																													
2010	7	71	2	80																																													
2011	82	-	55	137																																													
13.	Nomination for the Senior Executive Course (SEC) NiPSS, Kuru, Near Jos	None	Registrar was nominated a successfully completed the SEC 32, 2010 at NIPPS, Kuru	PCN Registrar became the first Registrar at health Regulatory Body to attend the SEC and be awarded the Member of the national Institute (Mni) Certificate																																													

The Pharmacists Council of Nigeria (PCN) (Registrar and CEO) (2003-2012)

14.	Nomination for Participation at the Executive Intelligence Management Course Four (EIMCIV) course of National Security Institute, Abuja	None	A staff was nominated to attend the course at the Department of State Services	The PCN staff became the first from any Regulatory Agency under FMOH to participate in the programme



Pharm A.T. Mora after Turbaning as “*K'ayayen Sarkin Musulmi*” in the Palace of the Sultan on Sunday, 4th April, 2021.



The “*K'ayayen Sarkin Musulmi*” greeting the Sultan with a clenched fist after the Turbaning.



Members of the ABU Alumni Association (Sokoto & Zamfara States Branches) hosted the "*K'ayayen Sarkin Musulmi*" to a reception after the turbaning, (R-L) Alh. A.C Ladan (*Barden Gwandu*) and Alh. Muhammad Anka (*Santurakin Anka*).



The "*K'ayayen Sarkin Musulmi*" with In-Laws in the Residence of *Maigirma* the late Alh. Abubakar Tunau Mafara (*K'ayayen Sardauna*). First left is the wife of the late *K'ayayen Sardauna*, Hajiya Nana (Fatima) and right is the first daughter of the *K'ayayen Sardauna* Hajiya Ladi (Khadijat)

Chapter 5

The Council of Nigeria: (Chairman of the Governing Council) (2020 - 2022)

When Mora as the Professor of Clinical Pharmacy and Pharmacy Practice assumed the mantle of leadership as Chairman of the Governing Council of the Pharmacists Council of Nigeria (PCN), he brought with him a clear vision and a profound passion for elevating the pharmacy profession. His tenure was marked by transformative reforms, bold innovations and an unrelenting pursuit of excellence in pharmaceutical regulation and practice. With a deep sense of purpose and unwavering dedication, he guided the Council through a period of renewal, strengthening its structures, empowering its workforce and building bridges of collaboration across academia, industry and government. The legacy of his leadership continues to resonate in the Council's renewed efficiency, credibility, and national impact.

However, the appointment of Professor Ahmed Tijjani Mora as the Chairman of the Governing Council of the Pharmacists Council of Nigeria (PCN) by President Muhammad Buhari, *GCFR* as par Decree 91 of 92 (LFN No. 17 OF 2004) in 2019 was challenged by the leadership of the PSN. Apparently, the PSN was still not comfortable with the appointment of Pharm. Mora despite his credentials (See Appendix V for the Letter of Appointment of Prof. Ahmed Tijjani Mora as Chairman of the Governing Council of the PCN). The leadership of the PSN and some past PSN Presidents wrote petition to President Muhammadu Buhari, *GCFR* opposing the appointment of Pharm. Mora as Chairman, which was carried in major Newspapers in Nigeria. *(See Appendix VI for the widely circulated petition by the Leadership of the PCN against the appointment of Prof. Mora as Chairman of the PCN)*. Eventually, on the day, 15th June 2020, His Excellency President Muhammadu

Buhari, *GCFR*, represented by the Honourable Minister of Health, Dr. Osagie Ehanire, inaugurated the Governing Council of the PCN with Prof. Mora as Chairman. (*See Speech of the Minister at Appendix VII*).

During Professor Mora's tenure as Chairman, the Pharmacists Council of Nigeria (PCN) experienced a remarkable institutional transformation and operational renewal. He championed far-reaching regulatory reforms that strengthened compliance monitoring, streamlined the accreditation of pharmacy training institutions and enhanced the licensure process for practitioners. Under his leadership, the Council introduced digital platforms for registration, documentation and inspection, thereby improving efficiency, transparency and data integrity. Infrastructural development was also given priority, with the establishment and upgrading of zonal and state offices to improve regulatory coverage nationwide. Professor Mora placed strong emphasis on stakeholder engagement—fostering productive collaboration with academia, professional associations and international partners to advance pharmaceutical education, quality assurance and public health outcomes. His leadership also encouraged professional discipline, ethical practice and continuous professional development, setting new standards for excellence in pharmacy regulation and governance.

Achievements and Initiatives Under the Leadership of Professor A. T. Mora

1. Regulatory Reforms and Policy Strengthening

Professor A. T. Mora's tenure was marked by far-reaching regulatory reforms aimed at reinforcing the statutory mandate of the Pharmacists Council of Nigeria (PCN). He provided strategic direction in the review and modernization of existing regulations governing pharmacy practice, ensuring alignment with contemporary professional and global standards. His leadership emphasized compliance enforcement and the institutionalization of best practices

across all sectors of pharmaceutical service delivery from manufacturing and distribution to community and hospital pharmacy. These reforms enhanced the Council's regulatory credibility, streamlined processes, and fostered greater accountability among practitioners and stakeholders.

2. Institutional Strengthening and Capacity Development

Recognizing the centrality of strong institutions to effective regulation, Professor Mora prioritized the internal restructuring and capacity building of the PCN. His administration enhanced the operational efficiency of the Council through improved administrative frameworks, staff training and the establishment of clear performance benchmarks. He facilitated the expansion and equipping of zonal and state offices across the federation, thereby improving accessibility and responsiveness to regulatory needs at the grassroots. The emphasis on institutional growth under his leadership ensured that the PCN remained proactive, adaptive and capable of meeting the dynamic challenges of the healthcare sector.

3. Digital Transformation and Process Automation

A key highlight of Professor Mora's achievements was his commitment to digital innovation. He spearheaded the development and deployment of electronic platforms for the registration, documentation and monitoring of pharmacists and pharmaceutical premises. The adoption of these digital systems enhanced operational transparency, minimized bureaucratic bottlenecks and enabled real-time data management and analytics for policy decision-making. This digital shift not only modernized the Council's administrative processes but also positioned the PCN as a forward-looking regulatory body in line with Nigeria's broader digital governance agenda.

4. Infrastructural Development and Resource Optimisation

Under Professor Mora's chairmanship, the Council recorded significant progress in infrastructural development and resource

management. He facilitated the construction, renovation and equipping of PCN offices and inspection facilities across several states, thereby strengthening the Council's physical presence nationwide. These infrastructural investments improved work efficiency, enhanced visibility and created an enabling environment for effective regulatory operations. His prudent resource utilization and commitment to sustainable growth ensured that infrastructural projects were executed with transparency and long-term impact.

5. Stakeholder Collaboration and Professional Engagement

Professor Mora recognized that effective regulation thrives on partnership and collaboration. He cultivated productive relationships with key stakeholders, including the Federal Ministry of Health, universities, professional associations such as the Pharmaceutical Society of Nigeria (PSN) and international organizations like the World Health Organization (WHO). Through these collaborations, the Council enhanced its policy influence, promoted research-driven regulatory decisions and improved alignment with global health and pharmaceutical standards. His inclusive leadership style encouraged dialogue, cooperation and unity within the profession, strengthening the collective voice of pharmacists in national health discourse.

6. Promotion of Professionalism, Ethics and Continuing Education

One of Professor Mora's enduring legacies was his unwavering commitment to professionalism, ethical conduct and lifelong learning within the pharmacy profession. His administration strengthened the disciplinary framework of the PCN to ensure that practitioners adhered to the highest standards of integrity and service. He also supported continuous professional development (CPD) initiatives that encouraged pharmacists to update their knowledge and competencies in line with emerging trends in pharmaceutical science and practice. This emphasis on ethics and continuous

learning significantly elevated the image and credibility of the profession in Nigeria and beyond.

7. Legacy and Impact

The cumulative impact of Professor Mora's visionary leadership is reflected in a more efficient, transparent and respected Pharmacists Council of Nigeria. His reforms laid a solid foundation for sustainable growth, digital modernization and enhanced professional accountability. Beyond institutional achievements, his tenure inspired a renewed sense of purpose among pharmacists, reaffirming the Council's pivotal role in safeguarding public health and advancing the frontiers of pharmaceutical regulation in Nigeria.

Professor A. T. Mora's tenure as Chairman of the Governing Council of the Pharmacists Council of Nigeria (PCN) was distinguished by strategic reforms and forward-looking initiatives that reshaped the Council's operations and strengthened its regulatory impact. He spearheaded the review and modernization of key regulatory frameworks governing the practice of pharmacy, ensuring that professional standards reflected contemporary realities and global best practices. Under his direction, the Council enhanced its institutional capacity through the restructuring of administrative systems, targeted staff development and the expansion of zonal and state offices for improved national coverage. His leadership championed digital transformation by introducing automated systems for the registration and monitoring pharmacists and pharmaceutical premises, thereby improving transparency, efficiency and accountability. Professor Mora also prioritized infrastructural development, overseeing the establishment and upgrading of office facilities, inspection units and resource centers that enhanced service delivery and visibility across the country. Furthermore, he strengthened stakeholder collaboration through constructive engagement with professional associations, academia, government agencies and international partners—fostering synergy in advancing

pharmaceutical education, regulation and public health. His commitment to professionalism, ethical standards and continuous learning led to the reinforcement of disciplinary procedures and the expansion of Continuing Professional Development (CPD) programmes, which collectively elevated the image and credibility of the pharmacy profession in Nigeria.

Professor Mora's leadership of the Governing Council of the Pharmacists Council of Nigeria (PCN) was characterized by vision, integrity and an unwavering commitment to public service. He led with a rare blend of intellectual depth, administrative discipline and moral conviction, qualities that inspired confidence and respect among colleagues, staff and stakeholders. His approach to governance was both inclusive and strategic, grounded in the belief that sustainable progress arises from collaboration, transparency and accountability. Professor Mora consistently emphasized the importance of professionalism, ethical responsibility and evidence-based decision-making, which became guiding principles for the Council's policies and operations.

Beyond his administrative achievements, Professor Mora's leadership was deeply humanistic, anchored in mentorship, empathy and the cultivation of excellence among younger professionals. He created an enabling environment where innovation was encouraged and where the contributions of every stakeholder were valued. His tenure fostered a culture of service and purpose within the PCN, ensuring that the Council's regulatory actions were always aligned with its ultimate mission of safeguarding public health and promoting rational drug use in Nigeria. The enduring legacy of Professor Mora lies not only in the tangible reforms he implemented but also in the values he embedded within the institution. His vision of a modern, efficient and ethically driven regulatory body continues to guide the PCN's strategic direction and inspire future generations of pharmacy leaders. Through his exemplary service, Professor Mora

has left an indelible mark on the Nigerian pharmacy profession, one defined by integrity, innovation and a lifelong dedication to the advancement of healthcare and humanity.

In recognition of his distinguished service, exemplary leadership and enduring contributions to the advancement of pharmacy practice and regulation in Nigeria, Professor A. T. Mora stands as a beacon of integrity, excellence and commitment to the public good. His visionary guidance not only repositioned the Pharmacists Council of Nigeria as a model regulatory institution but also strengthened the foundation upon which the nation's pharmacy profession continues to thrive. The values he upheld, discipline, diligence, transparency and ethical responsibility, remain a lasting source of inspiration to all who serve in the noble pursuit of pharmaceutical excellence. The legacy of Professor Mora endures as a testament to purposeful leadership and an unwavering dedication to the betterment of humanity through the pharmacy profession.



(L-R) Maigirma Alh. Shehu Sambo (*Danmasanin Gusau*) Pharm A.T. Mora and Mall. Yahaya Ibrahim in a Mosque in *Danmagaji* in Zaria for the Wedding Ceremony of one of the daughters of Pharm



HRM Igwe (Dr.) Nnaemeka Ugochukwu Alfred Achebe, *CFR, mni*, the *Obi* of Onitsha and Chancellor, ABU Zaria at the "*Ofala*" Festival of HRM Igwe Alex Onyido in Ogidi, Anambra State in 2020.



BOTTOM: His Royal Highness Chief Jonathan P. Zamuna, JP, the Ukarmo Piriga, Lere LGA, Kaduna State (Centre) on the day of the turbaning of Prof. A.T. Mora as "*Iyan Piriga*" in Gure, Lere LGA, Kaduna State. On the right is Mr. Jame H. Yaro, also turbaned "*Dallatun Piriga*" on 1st December, 2018 in the Palace, Gure.



Photo of Pharm. A. T. Mora Relaxing with some of his grandchildren

Chapter 6

The Pharmacy Council of Nigeria (PCN) (Chairman of the Governing Council (2022-2023))

The Custodian of a New Regulatory Era

By 2022, the Nigerian pharmacy profession reached a decisive milestone. When President Muhammadu Buhari signed the Pharmacy Council of Nigeria (Establishment) Act 2022, the law that birthed the new Council, a 30-year cycle of reform came full circle. The Act repealed the old Pharmacists Council of Nigeria Decree 91 of 1992, replacing it with a statute that expanded the regulator's reach and modernized its powers for a new generation. At the helm of this reborn institution stood Professor Ahmed Tijjani Mora, who, having chaired the seventh Governing Council of the defunct body since 2020, now became the first Chairman of the Pharmacy Council of Nigeria. His appointment symbolized continuity and renewal, the bridge between the founding generation and a new era of accountable, science-based pharmacy regulation.

6.1 From Transition to Transformation

Inaugurated on 15 June 2020 amid the global *COVID-19* pandemic, Mora's Council inherited an institution long without a governing Council. Over the next three years and especially through 2022 – 2023, he steered the transformation from the Pharmacists Council to the Pharmacy Council with quiet discipline and strategic clarity. The 2022 Act broadened jurisdiction to include Pharmacists, Pharmacy Technicians, Patent and Proprietary Medicine Vendors (PPMVs), Manufacturers, Distributors and medical-device operators. It empowered the Registrar to revoke licenses administratively, made the office a statutory member of Council and gave legal status to zonal and state offices as regulatory arms of the national body.

Mora's task was to translate these legal provisions into operational reality. He convened harmonization meetings, revised disciplinary procedures and established transition committees for staff and asset integration. His leadership was inclusive yet time-bound, encouraging debate while insisting on timely delivery.

6.2 Announcing the Rebirth

On 13 October, 2022, at a landmark press conference in Abuja, Mora publicly presented the gazetted Act. Beside him stood Pharm. Ibrahim Babashehu Ahmed, newly appointed Registrar / CEO and the senior directors of the Council. Mora declared the legislation “*a renaissance for pharmacy regulation in Nigeria, an evolution from prescription to protection, from structure to system.*” He highlighted that only twenty-three universities nationwide were accredited to run a Pharmacy or Doctor of Pharmacy (PharmD) programmes, signalling the Council's uncompromising quality standards. Enforcement data showed that over 19,000 illegal outlets had been sealed, providing proof that the Council's authority was now being felt nationwide. That day marked the formal public birth of the Pharmacy Council of Nigeria under his chairmanship.

6.3 Strengthening Enforcement and Ethics

Between 2022 and 2023, the Council's enforcement drive reached unprecedented scale. Inspection teams visited more than 27,000 premises and shut down those failing to meet minimum standards, including unregistered pharmacies and substandard PPMV shops. Mora insisted that enforcement be “*evidence-based and transparent.*” Each operation was followed by an audit and public disclosure “*sanitizing the pharmaceutical space, not stigmatizing practitioners.*” In June 2023, he personally chaired a Disciplinary Tribunal at the Fred Adenika Auditorium, Abuja, where four pharmacists were suspended for serious professional breaches. In his closing address, he reminded colleagues: “*Ethics is the immune system of our profession. When it fails, the whole body of public trust becomes sick.*”

6.4 Consolidating Professional Reforms

Mora's years as Chairman also witnessed policy victories that reshaped pharmacy practice:

1. **Pharmacy Consultancy Cadre (2021):** Through persistent engagement with the Office of the Head of Civil Service of the Federation, he secured approval of the consultancy cadre for pharmacists, formally recognizing graduates of the West African Postgraduate College of Pharmacists as Consultants in the public service, a goal pursued for decades.
2. **Recruitment and Capacity Building (2022):** With advocacy to the same office, he obtained a waiver to recruit 100 new staff, strengthening the Council's human-resource base for national coverage.
3. **Infrastructure Expansion:** He commissioned the Lagos Zonal Office, the largest and busiest in operations, enhancing regulatory efficiency across the South-West.
4. **Logistics Support:** Leveraging longstanding relationships, he secured four vehicles from the Nigeria Customs Service to bolster field operations, an example of how personal credibility translated into institutional gain.
5. **Hospital Pharmacy Inspection Regulations (2023):** Under his chairmanship, the Council approved, for the first time, detailed the regulations governing the inspection of hospital pharmacies nationwide, an achievement ratified at the final Council meeting, which he chaired in June 2023.

6.5 Advancing Education and Practice Standards

Throughout his tenure, Prof Mora viewed education as the seedbed of regulation. Despite post-pandemic restrictions, the Council under him reactivated accreditation and re-accreditation visits to universities. New or renewed approvals were granted to Chukwuemeka Odumegwu Ojukwu University, Enugu State University of Science and Technology and Bingham University, Karu. Advisory visits were conducted to other emerging pharmacy

programmes across to universities Afe Babalola, Adeleke, Lead City, Calabar, Abia, King David University, Ebonyi, City University, Kano and Bauchi State University, Gadau.

In May 2023, he led, with the Registrar and a representative of the Committee of Deans, a study tour to the United States, visiting Philadelphia College of Pharmacy, Saint Joseph's University and Howard University, Washington, D.C. The tour revisited models first explored in 1992, reaffirming the global vision behind Nigeria's (PharmD) transformation. The same month, he convened a Leadership Training Workshop for all 23 Deans of Pharmacy, linking academia with regulation in pursuit of one continuous quality culture. Under his watch, Council also approved the Second Edition of the Compendium of Minimum Standards for Pharmacy Practice—updating the seminal 2005 work he had produced as Registrar. The revised edition captured evolving trends in hospital, industrial, community, and academic pharmacy. It has since been printed and made available to practitioners.

6.6 Institutional Development and Resource Mobilization

To sustain long-term capacity, Mora secured a TETFund intervention of ₦50 million through the National Institute for Pharmaceutical Research and Development for the construction of a Pharmaceutical Sciences Resource Centre at the Council's headquarters, Idu-Abuja, with ₦100 million additional mobilization under negotiation. He also oversaw a comprehensive review of Pharmacy Technician Training Programmes in Colleges of Health Technology, ensuring equitable distribution and quality assurance across states.

6.7 Regulatory Diplomacy and Stakeholder Engagement

Prof. Mora's diplomacy extended beyond the profession. In February 2022, he led a PCN delegation to the Nigeria Customs Service to strengthen collaboration on control of pharmaceutical imports—resulting in a joint framework for border inspections. He also fostered cooperative ties with NAFDAC, the Federal Ministry of Health,

universities and industry bodies, thereby positioning the Pharmacy Council as a hub of multi-agency coordination. His calm negotiation style was again visible in the Kanawa Coordinated Wholesale Centre dispute: when a faction of medicine dealers challenged the Council's relocation policy, the Federal High Court in June 2023 upheld the Council's authority, affirming PCN's statutory mandate under the 2022 Act.

6.8 Challenges and Strategic Responses

Implementing the 2022 Act demanded redesigning frameworks for technicians and PPMVs, re-training inspectors and educating thousands of practitioners. The Council operated under tight budgets and residual pandemic disruptions, yet Mora's pragmatic leadership-maintained momentum in decentralizing enforcement through zonal offices, adopting digital registration platforms and pursuing dialogue where confrontation would have stalled progress.

6.9 Closing a Chapter, Opening a Legacy

On 19 June, 2023, a presidential directive dissolved all federal boards, concluding Mora's tenure. By then, he had guided the profession through one of its most transformative epochs. Under his leadership:

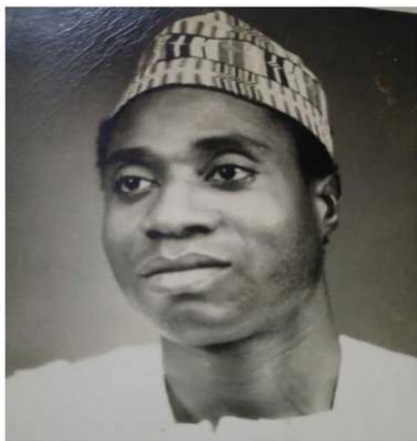
- The Pharmacy Council of Nigeria was fully established and operationalized;
- A Registrar was appointed through transparent recruitment;
- Nationwide enforcement and compliance structures were consolidated;
- Educational and ethical standards were elevated;
- The Council's institutional partnerships reached new breadth;
- The Consultant Pharmacy cadre was approval;

Mora's departure marked not an end but the completion of a historical circle: the young reformer of 1992 had become the elder custodian of 2022 – 2023.

6.10 Reflection - The First Chairman

History will record Professor Ahmed Tijjani Mora as the first Chairman of the Pharmacy Council of Nigeria, the pioneer custodian of its modern identity. His brief but consequential tenure transformed legislative intent into a living institution, guided by the conviction that regulation is a service, not a means of power. *“The law gave us a name,”* he once reflected, *“but integrity will give us a legacy.”* That legacy endures in the Council's strengthened authority, its culture of ethics and evidence and the generation of professionals he inspired. The Act of 2022 may have provided structure, but Mora's leadership supplied the conscience and that, more than any statute, is what will sustain Nigeria's pharmacy regulation into the future.

*The Pharmacy Council of Nigeria (PCN) (Chairman of the Governing Council
(2022-2023)*



Pharm. Muhammadu Inuwa Musa (A. K. A Dispenser) (he died in December, 2024)



(L-R) Pharm. Mustapha Abubakar Imam (1938-2024) and Alhaji Muhammadu Inuwa Mora (1940 - 2001) sometimes in late 1950s/ early 1960s



Dr. Abubakar Imam (1911-1981) in his Farm



Prof. Ahmed Tijjani Mora with Pharm. Adams, one time President PANS (ABU Chapter) at an event in ABU Zaria in 2025



Prof. Ahmed Tijjani Mora with members of PANS (KASU CHAPTER), KASU at an event in the University in 2026.



: (L-R) His Excellency Chief Lucky Igbinedion, the Deputy Chancellor of Igbinedion University Okada (IUO) and two - term Governor of Edo State, Professor Ahmed Tijjani Mora (centre) and Prof Lawrence Ikechukwu Ezemonye, the Vice Chancellor, IUO, Okada. The picture was taken in July, 2025 at Oti Hotel, Sapele Road, Benin City during a meeting of the Governing Council of Igbinedion University Okada during which Prof Mora was announced as the Pro-Chancellor and Chairman of the Governing Council of Igbinedion University Okada.

Chapter 7

Tributes

AN ICON, A PHARMACIST PAR EXCELLENCE, A DISTINGUISHED ABU ALUMNUS, AND A PATRIOT PROFESSOR AHMED TIJJANI MORA, OON, mni, @ 70

Prof. Ahmed Tijjani Mora evokes a range of sentiments among those who know or have worked with him. What unites these perspectives, however, is an abiding recognition of his unwavering dedication to his profession, his community, and humanity at large. He is, quite simply, a man who sets clear goals and attains them.

My first encounter with Prof. Mora was nearly two decades ago at Ahmadu Bello University (ABU), Main Campus, Samaru, Zaria during the Annual General Assembly/Homecoming of the ABU Alumni Association. Then a candidate for the Association's presidency, his manifesto spoke for itself precise, thoughtful and focused on tangible service to the University. We also learned that he had been teaching *pro bono* in the Faculty of Pharmacy. That singular act of giving back endeared him to many and remains emblematic of his enduring ethos of service. Beyond the academy, Prof. Mora has distinguished himself in alumni and community leadership. He served as Secretary, General of the Barewa Old Boys' Association (BOBA), where his innovative ideas and tireless efforts helped attract impactful intervention projects to Barewa College, Zaria. Within the ABU Alumni Association, he served two terms as Deputy President and two terms as President, during which the Association recorded notable progress through carefully designed initiatives that benefitted staff, students and host communities.

Noteworthy projects championed under his stewardship include:

- The Pedestrian Overhead Bridge linking the North Gate to Samaru village, addressing a long, standing safety concern that had tragically claimed lives.
- Intra, Campus Shuttle Bus Services, facilitating safer and more efficient movement for students and staff across campuses.
- The Students' Laptop Loan Scheme, enabling less privileged students to acquire laptops through convenient instalment plans.
- The proposed Twin (Male and Female) Postgraduate Hostel, conceived to ease the acute shortage of student accommodation; its foundation, laying ceremony has already been conducted.

Those who meet Prof. Mora soon discover a warm and affable personality that belies any initial perception of reserve. He is quick to set aside personal views when confronted with superior arguments and bears no grudges. Generosity and compassion run through his dealings; many can attest to his readiness to listen, mentor and support, even where differences once existed.

A hallmark of his leadership is meticulous planning. When entrusted with responsibility, he defines objectives, maps out the path to achieving them and sets realistic timelines. This disciplined approach consistently leaves behind a legacy that successors can readily build upon. Little wonder that as one assignment ends, a greater call to service often follows; like a "golden fish," he has no hiding place.

Within the profession, his contributions remain exemplary. He initiated the first Faculty Chapter of the ABU Alumni Association (Faculty of Pharmacy) and currently chairs the Board of Trustees of the Faculty of Pharmacy Alumni Association, ABU Zaria. He also led significant fundraising efforts during ABU's 50th anniversary and has been instrumental in resource mobilization, both material and intellectual, for the Faculty of Pharmacy. His mentorship transcends

disciplines, inspiring generations of professionals to pursue excellence with integrity.

There is no doubt that Prof. Mora's life and work have touched countless individuals. His legacy, rooted in professional excellence, community service and principled leadership, will endure well beyond his lifetime.

On behalf of the ABU Alumni Association Worldwide, I extend heartfelt congratulations to Prof. Ahmed Tijjani Mora, *OON, mni*, as he turns 70. We wish him many more years in robust health and continued service to our university, our nation and humanity, *amin*.

Congratulations!

Adamu Mamman Kontagora

(Barden Kontagora)

President, ABU Alumni Association

Mallam Adamu Mamman Kontagora is an ABU-trained mathematician and current President Worldwide of the ABU Alumni Association. Professor Mora previously served in the same capacity from 2015 to 2021. Mallam Kontagora was elected President in September 2023 and re-elected unopposed in November 2025. He holds the traditional title "*Barden Kontagora*".

TRIBUTE TO PROFESSOR AHMED TIJJANI MORA, *FPCN, mni,*

By Air Commodore E. K. Jekada (Rtd), *PhD, mni*

It is with great delight that I write this tribute in honour of my younger brother and mentee, **Professor Ahmed Tijjani Mora, FPSN, mni**, whose family I have known for nearly five decades, dating back to my days as a student at the then St. Enda's College, later Basawa Teachers College, Zaria. His relation, Ahmed Sabir, was my classmate at St. Enda's and later a colleague in the military. We became bosom friends and through that bond I came to know the Mora family well. I fondly recall spending time at their home in Tudun Wada, including joining them for *Eid* prayers during festive *Sallah* periods.

A Family of Integrity

Their father, popularly known as **Baba Mora**, was a rare and distinguished personality in Zaria. He embodied the very essence of *Mutumin Kirki* (a man of intrinsic goodness). His virtues included *gaskiya* (truthfulness), *amana* (trustworthiness), *karimci* (generosity), *hakuri* (patience), *ladabi* (good manners and obedience), *hikima* (wisdom), *hankali* (sound judgment) and *adalci* (fairness). To a large degree, Professor Ahmed Mora is a true reflection of his father, carrying forward these values into his professional and personal life.

Professional Achievements

Professor Mora, a pharmacist by training, rose to prominence as both an academic and an administrator. His distinguished career led him to serve as **Registrar of the Pharmacists Council of Nigeria (PCN)** and later as **Chairman of the Governing Council of PCN** appointed by President Muhammadu Buhari (of blessed memory). In these roles, he transformed the Council, raising its reputation to an enviable height and earning it national respect. His father's values, deeply ingrained in him, were evident in his leadership and success.

At the National Institute for Policy and Strategic Studies (NIPSS)

Providence brought our paths together again at NIPSS, Kuru, when Professor Mora (then Dr. Mora) was a Participant in the **Senior**

Executive Course 32 (2010), while I was serving as a Directing Staff and later Director of Studies. At NIPSS, Participants are divided into Study Groups and I happened to be the Directing Staff assigned to his group at the initial stage, while he served as the Chairman (Monitor). He displayed outstanding leadership qualities, guiding his group with wisdom and dedication. His contributions during discussions and local study tours to Kogi and Bayelsa States, as well as the international study tours to Burundi and Germany were invaluable. His determination and leadership ensured his syndicate consistently excelled.

I also had the privilege of part supervising his Individual Project, titled *“The Problem of Fake Drugs in Nigeria: A Study of the Operations of the National Agency for Food and Drug Administration (NAFDAC).”* Despite our personal relationship, I maintained strict academic standards. He rose to the challenge with perseverance and commitment, eventually earning an “A” grade. He was one of the top three participants in that course alongside Mathias Kurah of the Central Bank of Nigeria and Brigadier General (later Major General, (Rtd) Tilewa Amosun of the Nigerian Army Medical Corps.

Qualities and Character

Professor Mora has always stood out for his diligence, discipline and determination. He is time conscious, reliable and willing to go above and beyond expectations. His intellectual capacity is remarkable and he demonstrates academic competence and organizational ability at the highest levels. He worked harmoniously with colleagues, showed deep concern for their welfare and always strived to achieve group objectives with commitment and cooperation.

It is no surprise, therefore, that he rose to become a Professor, having taught at three universities, Ahmadu Bello University (ABU), Kaduna State University (KASU), where he was the pioneer Dean of the Faculty of Pharmaceutical Sciences and Igbinedion University, Okada (IUO), Edo State, where he now serves as Pro-Chancellor and Chairman of the Governing Council.

Conclusion

Professor Mora is an erudite scholar, a disciplined leader and a humble personality with exceptional qualities. He leads by example, remains focused and resilient and possesses the rare blend of wisdom, humility and vision. His contributions continue to benefit Nigeria greatly and I believe his wealth of experience and resourcefulness will remain invaluable to the nation in the years ahead.

On this joyous occasion of your 70th birthday, I salute you, my brother and mentee. May you enjoy continued good health, long life and the fulfilment of seeing the fruits of your labour in the generations you have inspired.

Air Commodore E. K. Jekada (Rtd), PhD, mni

The Former Directing Staff/Director of Studies

National Institute for Policy & Strategic Studies (NIPSS), Kuru, Jos

Air Commodore (Rtd) E. K. Jekada, *mni*, was a Directing Staff at the National Institute for Policy and Strategic Studies (NIPSS), Kuru, when Professor Mora attended the Senior Executive Course (SEC 32/2010). A disciplinarian renowned for efficiency, he played a central role in administering the programme. He and Professor Mora first knew each other in the 1970s while Air Commodore Jekada was a student at St. Enda's College (now Basawa Teachers' College), Zaria. They have maintained cordial relations since then.

CELEBRATING PROF. AHMED TIJJANI MORA @70

Bismillahir Rahmanir Raheem!

The request by members of the Prof. A. T. Mora Book Writing Committee for me to write a tribute in honour of Prof. Ahmed Tijjani Mora, whom I fondly call “*Baba*” is both a profound honour and a rare privilege. I am deeply grateful to the Committee for considering me worthy, despite my relatively young age, to be among those contributing to this special publication in commemoration of *Baba*’s 70th birthday on 13th May, 2026, *In Sha-Allah*.

For me, it is not about the date of our first meeting or the number of years we have known each other, but rather the profound impact he has had on my life. From the very beginning, *Baba* has treated me as a son. Though he is old enough to be my father, he always makes me feel completely at ease in his company and frequently asks after my own father, though they have never met, they are of the same generation.

I cannot fail to acknowledge *Baba*’s invaluable assistance in the establishment of Badawiyya Pharmaceutical Products Manufacturing Company in Kano. Though I am an industrialist with ventures in fertilizer blending, cooking oil production and now oil and gas, I found his guidance on the design and layout of our pharmaceutical plant to be unique and transformative. Over the past two years, his consistent advice and direction have been both enlightening and inspiring.

Every encounter with *Baba* is akin to attending a lecture. I vividly recall how he guided my team and me on our responsibilities through the organogram he personally designed for the company. His emphasis on the optimal utilization of scarce resources left a lasting impression on us all. He taught us to regard the “5Ms”, Man, Money, Materials, Machines and Methods, as valuable resources that must

be efficiently harnessed. This lesson was a revelation for my entire team and me.

Though a Professor of Clinical Pharmacy and Pharmacy Practice, *Baba* is equally versatile in other spheres of human knowledge. I remember one afternoon within the premises of our facility in Kano, when over lunch, he enlightened us on the *Jihad* of the 19th century Fulani Islamic reformer, Sheikh Usmanu *bin* Fodio (may Allah be pleased with him), and the subsequent transformations in Hausaland supported by Sheikh Abdullahi *bin* Fodio (Abdullahi Gwandu) and Caliph Muhammad Bello. It is therefore no surprise that His Eminence, the Sultan of Sokoto, Alhaji Muhammad Sa'ad Abubakar, *CFR, mni*, honoured him with the turbaning as “*K'ayayen Sarkin Musulmi*.”

As *Baba* attains the milestone age of 70, I join countless others in praying to Allah (SWT) to bless him with long life, excellent health, enduring happiness and the grace to witness many more fruitful years, Amin.

Once again, I remain profoundly grateful for the privilege of being called upon to contribute to this historic Book Project.

Bissalaam.

Alh. Abdulbasir Abubakar Jargab'a
Kano

Alhaji Abdulbasir Abubakar Jargaba is a Kano-based industrialist and businessman. Encouraged by Professor Mora, he ventured into pharmaceutical manufacturing as part of his diversified industrial portfolio. He regards Professor Mora as a father figure.

**PROF. AHMED T. MORA, OON, mni:
A PHARMACIST PAR EXCELLENCE**

From his professional choice and training to his career and service at private, public and academic levels, one keyword defines him: “Pharmacy.” Prof. Ahmed T. Mora has been actively and diligently serving Nigeria, Nigerians and, by extension, humanity for over four decades. He began his journey at the entry point of the profession and rose to the highest position as CEO and overall Regulator of the National Pharmaceutical Regulatory Body. His passion, dedication and unparalleled commitment to the sector earned him the prestigious traditional title of *Wakilin Maganin Zazzau* from the Zazzau Emirate. Indeed, it is all about pharmacy!

The name Prof. Ahmed T. Mora is iconic in the modern history and development of pharmaceutical services in Nigeria and beyond. My personal relationship with Prof. Mora began more or less at the family level. Though we were born in the same neighborhood, Tudun Wada Zaria Ward, we did not connect during childhood. Later, through a friendship with his elder brother, Dr. Saifuddeen Mora, I became acquainted with most members of the Mora family. The Mora name remains prominent not only in Tudun Wada but throughout the Zazzau Emirate.

In his early career, as a representative of one of the major pharmaceutical companies, Prof. Mora joined us in Kano in 1980. We lived near each other and maintained a close relationship. As God willed it, I was appointed Director General of the Kaduna Ministry of Health in 1990, where I met Alh. Ahmed T. Mora as the Director of Pharmaceutical Services for the state. He was the youngest among the eight directors under my supervision and the only one younger than myself. More importantly, he was the only one I knew personally before joining the Ministry. These circumstances fostered my confidence in him and led to a harmonious working relationship and enduring friendship throughout my tenure (January 1992) and beyond.

Through close association with him, I had the opportunity to witness both his overt and hidden leadership qualities. Perhaps his brilliance and diligence were to be expected, given his background in the private sector. Yet, his energy, resilience under pressure, sincerity, focus and service-oriented mindset were unmatched. He exhibits confidence and determination in every task combined with loyalty, ambition and a rare ability to lead without offending others.

I am truly delighted that these positive traits and many more, continue to manifest as he advances in life.

Prof., while 70 may sound long and be considered a milestone by global standards, in your case, it must be said that complete retirement is only for the weak or the infirm. Blessed with good health and abundant energy, you are encouraged to keep moving forward, but at a reasonable pace.

I wish to conclude this humble tribute with a quotation from an unknown author:

"... You may rest. You may change lanes. But you must never stop moving.

To stop is to rust.

To rust is to rot.

And to rot is to die ..."

Congratulations on your forthcoming 70th birthday anniversary!

May Allah (SWT) bless and reward your tireless efforts and outstanding service abundantly. Aamiin!

Signed:

Alh. Awwalu M. Makarfi

Mallam Awwalu M. Makarfi served as Director-General at the Headquarters of the Kaduna State Ministry of Health during Professor Mora's tenure as Chief Pharmacist and later Director of Pharmaceutical Services. He is regarded as an elder brother, having grown up in the same Tudun Wada Zaria community.

**A TRIBUTE TO A FATHER, MENTOR AND PATRIARCH AT
70:**

PROF. AHMED T. MORA

Today, we gather to celebrate not just a milestone, but a man; a man whose life, legacy and character have touched all who have had the privilege of knowing him. I count it one of the greatest blessings of my life to be among those fortunate enough to call him father.

I first met my father-in-law at his home in Tudun Jukun, Zaria, a day forever entrenched in my memory. I had come with humility and a hopeful heart, seeking his blessing to court his first daughter, Zubaida. I expected formality, but what I received was family. The warmth and generosity with which he welcomed me and my friends was overwhelming. I did not feel like a guest; I felt like a son. From that very first day, he has never treated me as anything less.

In him, I have found not only a father-in-law but also a guide, someone who treats me with dignity and respects whose words carry the wisdom of a life devoted to purpose, discipline and service. Our conversations, whether about politics, travel, academia or the philosophies of life, have always left me enriched. Listening to him is like attending a masterclass, yet never once does he make you feel small. Such is the humility that defines his greatness.

His personality and charisma are undeniable. He carries himself with a quiet confidence that commands respect, yet he remains approachable; firm, yet fair; strong, yet deeply compassionate. His contributions to the community, though vast, are often understated. But for those who know, they know: he is a pillar, a reference point and a moral compass.

Perhaps his greatest legacy is the family he has nurtured. The values he instilled, discipline, kindness, integrity and excellence, are evident

every day in the woman I married and in each of her siblings. For this, I owe him my deepest gratitude.

In a society that so desperately needs wisdom, values and integrity, men like him should never truly retire. Their presence alone is a source of strength and guidance. Though today we mark his 70th year, I believe his work and his impact are far from over.

Baba, thank you for everything, for the home you have built, the love you have shown and the example you continue to set. May Almighty Allah grant you good health, long life and peace of mind; Amin

Happy 70th Birthday, sir. You are deeply loved, admired and celebrated.

With deepest respect, love and gratitude.

Mallam Sani Ibrahim Mohammed

Mallam Sani Ibrahim Mohammed is a chartered accountant married to the most senior daughter of Professor Mora, “Barrister Zubaida Mora”. He submitted this tribute on behalf of the sons-in-law of Professor Mora. He is currently a staff member of the “Standards Organisation of Nigeria (SON)”.

A PILLAR OF PHARMACY REGULATION IN NIGERIA

Professor Ahmed Tijjani Mora, OON, mni, Wakilin Maganin Zazzau.

Professor Ahmed Tijjani Mora is a household name in Nigeria and beyond, a dedicated professional whose expertise, compassion and commitment to healthcare have left a profound impact on our nation. His tireless efforts to advance pharmacy practice and foster a culture of care have earned him the deepest respect and admiration. We celebrate Professor Mora's remarkable achievements, especially his passion for his profession and his unwavering dedication to the service of Nigeria.

An illustrious son of the Zazzau Emirate, Professor Mora is an extraordinary individual, who has left an indelible mark on Nigeria's educational landscape and the field of pharmacy. His contributions to the growth of education and the development of pharmacy practice have been truly exceptional.

As a prolific author, his books on pharmacy and related fields have enlightened countless students, professionals and researchers, cementing his legacy as a thought leader. His commitment to knowledge, sharing has had a lasting influence on generations of pharmacists.

His dedication to community service is evident in his active participation in numerous events within the Zazzau Emirate, Kaduna State and across the country, efforts that promote unity and shared purpose. His jovial nature has endeared him to many, while his hard work and integrity have earned him the respect of his peers and the wider public.

His numerous awards, including the prestigious National Honour, as well as his traditional titles of *Wakilin Maganin Zazzau* and *Kayayen Sarkin Musulmi*, are a testament to his outstanding achievements. In

addition, his leadership roles in various professional associations, coupled with his tenure as Chairman of the Governing Council of Nigeria's first private university, Igbinedion University, Okada, Edo State, demonstrate his commitment to excellence.

We celebrate Professor Mora's achievements, his passion and his unwavering dedication to making a difference. He is an inspiration to us all and we are proud to be associated with his remarkable career and contributions to the progress of our country.

Please Professor Mora, accept our deepest appreciation and admiration for all that you have accomplished. May your legacy continue to inspire and motivate future generations.

Alhaji Shehu Garba

Dokajen Zazzau

Senior Special Assistant to the Emir (SSA)

Maigirma Alhaji Shehu Garba is the “*Dokajen Zazzau*” and Senior Special Assistant to His Highness the Emir of Zazzau. He and Professor Mora have been childhood friends in Zaria. The *Dokaje* served loyally as a dependable aide to the late 18th Fulani Emir of Zazzau, “His Highness Alhaji Shehu Idris, *CFR* (1975–2020)”, and continues to serve with the same dedication under the present Emir, “His Highness Ambassador (Dr) Ahmad Nuhu Bamalli, *CFR*”. He is an authority on the Zazzau traditional institution and other emirates in Northern Nigeria.

**A TRIBUTE TO PROFESSOR AHMED TIJJANI MORA,
*OON, mni,***

It is my distinct honor and privilege to pay tribute to a friend whose life story continues to inspire across generations. Professor Ahmed Tijjani Mora, *OON, mni*, is not only a distinguished pharmacist and academic but also a man whose journey reflects discipline, commitment and a deep sense of service to society. For me, it is especially meaningful to write these words, having known him from childhood and witnessed, up close, the traits that have shaped the remarkable man he has become.

From his earliest years, Ahmed's warmth and aura set him apart. He had a gift for connecting with people, putting others at ease and leaving a lasting impression through sincerity and charm. Over time, these qualities shone in everything he did: in the classroom, in leadership positions, in professional circles and in the community. That blend of intellect, versatility and human touch is what makes him such an outstanding personality.

Professionally, Professor Mora's career spans a broad and impactful spectrum. He began in the marketing of pharmaceutical products, where he displayed sharp business acumen and leadership instincts. He later brought his expertise into the Civil Service, rising to become Director of Pharmaceutical Services, a role in which he shaped national policy and advanced the practice of pharmacy in Nigeria. His time as Project Manager of Zaria Pharmaceutical Company Limited further demonstrated his ability to balance technical knowledge with managerial skill.

Perhaps one of his most notable contributions was in regulatory leadership. As Registrar and later as Chairman of the Pharmacists Council of Nigeria, Professor Mora brought vision, reform and integrity to the profession's highest body. His tenure left behind structures and standards that strengthened the credibility of pharmacy practice in our country.

In recognition of his immense contributions to health in Kaduna State, Professor Mora was conferred with the distinguished chieftaincy title of *Wakilin Maganin Zazzau* by the late Emir of

Zazzau and Chairman of the Kaduna State Council of Chiefs, Alhaji Shehu Idris, *CFR*. This honor was not only a personal milestone but also a recognition of his significant impact on healthcare delivery and his dedicated service to the community.

Over the years, Professor Mora has practiced pharmacy in nearly all the core areas of the profession, amassing a wide range of experience that few can match. His forays into lecturing and university administration reflect the same breadth. He taught and mentored students at Ahmadu Bello University, Zaria, before moving to Kaduna State University, where he served as foundation Dean of the Faculty of Pharmaceutical Sciences and pioneer Head of the Department of Clinical Pharmacy and Pharmacy Management (2012–2016). His academic journey has since taken him to Igbinedion University, Okada, Edo State, where he is not only a Professor but also holds the distinction of being the first Professor of Clinical Pharmacy and Pharmacy Practice in the university. Today, he continues to provide leadership as the Pro-Chancellor of the same university, guiding its growth.

His intellectual contributions are not confined to the classroom. As a prolific essayist, Professor Mora has written extensively on both academic and social issues, always with clarity, courage and a sense of responsibility to the public good. Through essays, lectures and public commentaries, he has engaged in national discourse in ways that deepen understanding and promote reform.

Community service is another area where his dedication shines. A founding member and trustee of the Zaria Emirate Development Association (ZEDA) and the Zazzau Emirate Council Advancement Forum (ZECAF), Professor Mora has been at the forefront of initiatives aimed at uplifting his community. He also served with distinction as National President of the Ahmadu Bello University Alumni Association, building bridges between generations of graduates and strengthening the legacy of one of Nigeria's greatest institutions.

Traditional honors have come to him naturally, recognizing his contributions to society and his standing among his people. Along

with his title of *Wakilin Maganin Zazzau*, he also holds the title of *Kayayen Sarkin Musulmi* of Sokoto, a further mark of the respect and trust he enjoys at the highest levels of traditional authority. Beyond titles and offices, what endears Ahmed most to many of us is his humanity. He is approachable, deeply rooted in his faith and values and always willing to guide others. His versatility as a writer, his humor in conversation and his ability to nurture friendships across decades are part of the legacy he continues to build.

As I reflect on our long friendship, I am reminded of how consistently he has been. From childhood to the heights of national recognition, Ahmed has remained true to himself: curious, disciplined, sociable and driven by a desire to make a meaningful contribution. It is this authenticity that inspires trust and admiration from colleagues, students, professionals and community members alike.

I am proud to call him my friend and prouder still to see the heights he has attained. May the example of this icon, whose impactful journey and guiding light continue to inspire, serve as a beacon for future generations of Nigerians to pursue excellence with integrity, versatility and a commitment to service to humanity.

Alh. Surajuddeen B. Yakubu, FBR, FCIoD
Santuraki Babba, Zazzau

Maigirma Surajuddeen Balarabe Yakubu, the respected “*Santuraki Babba of Zazzau*”, is an ABU-trained lawyer and a childhood friend of Professor Mora. He served as a banker, Honourable Commissioner in the Kaduna State Executive Council, and formerly as Executive Secretary of the “Nigerian Investment Promotion Council (NIPC)”. A thorough and articulate legal luminary, the *Santuraki Babba* currently heads a successful private human-development enterprise in Abuja.

PRICELESS ENCOUNTER – A TRIBUTE TO A PHARMACY REGULATOR:

PROF. A. T. MORA, OON, mni

By Divine providence, after many years of “*long time no see*,” I had the privilege of meeting and interacting once again with Prof. Ahmed Tijjani Mora at the Asokoro, Abuja office of the National Affordable Medicines Initiative (NAMI), an initiative championed by the late Pharmacist Sam Nda, Isaiah (the founder and publisher of Leadership newspapers) with the support of the Federal Government of Nigeria. From that encounter, our relationship has continued to wax stronger ever since. Prof. Mora is a man of many parts, having traversed all aspects of pharmacy practice, academia, regulation and industry, where he has carved a distinguished niche for himself. He can rightly be likened to a giant elephant, whose description depends on the part being touched.

Academia

In academia, Prof. Mora is celebrated by his students for his legendary intellectual curiosity, resilience, hard work and meticulous attention to detail. As Pro-Chancellor at JUO, a renowned university, he is a teacher of teachers and a scholar of scholars, sowing seeds of knowledge and nurturing generations of pharmacists and academics who continue to enrich humanity. Indeed, he has seeded the pharmacy community with academic offspring of diverse identities.

Regulation

In pharmacy regulation, he has had the rare privilege of serving both as Registrar and later as Chairman of the Governing Council of the Pharmacists Council of Nigeria (PCN), a feat unmatched in history. Staff at the Council testify to his thoroughness, strict scrutiny and unwavering insistence that things must always be done properly. As Chairman, despite opposition from adversaries, he remained unstoppable. His legendary legacy at the Council is a testament to his firm “*can, do*” spirit.

Industry

In industrial pharmacy practice, where my path crossed more closely with his, Prof. Mora’s vast knowledge of regulation and his strong

academic grounding facilitated seamless services for clients. As a business associate, his influence commands attention from the highest quarters. I am proud to call him my business partner, an embodiment of intellect, charisma and integrity.

Personal Impact

Undoubtedly, Prof. Mora has engraved his name forever in the hearts of all who have encountered him. I am honored to call him my person, for he is unstoppable. This world is better and richer because he has lived in it, leaving indelible footprints in the history of the pharmacy profession. Through him and my association with him, I have found enduring examples of forthrightness, diligence, compassion and resilience.

Conclusion

As you celebrate three scores and ten (70 years) of your sojourns on this earth, it is my heartfelt prayer and that of my family that Almighty Allah bless and keep you, open the doors of His mercy and grant you fulfillment beyond your expectations. We congratulate you and send you 70 encapsulated and pharmaceutically packaged cheers on your birthday. May Allah provide you and yours with abundant doses of His grace throughout the rest of your life.

Alhaj (Pharm.) Musibau Ade Mustapha, *PhD*
Managing Consultant / CEO
TAFAPHARM Consult Limited, Lagos

Dr. Musibahu Mustapha, *PhD*, is a Nigerian pharmacist and one of the leading experts in Pharmaceutical Manufacturing in Nigeria. He and Professor Mora have jointly designed and established Pharmaceutical Manufacturing Plants for many clients across the country.

Tribute to Prof. A.T. Mora By:

**Dr. Haruna Muhammed Jumare, FCCA, FIPFM, Maradin
Zazzau,**

Introduction

It is with immense pleasure, deep respect and heartfelt admiration that I write this tribute in honor of Professor Ahmed Tijjani Mora, *OON, mni*, as he celebrates his remarkable 70th birthday. Today, we gather to celebrate a rare gem a distinguished academic, visionary leader and compassionate mentor whose life embodies the highest ideals of scholarship, leadership and service to humanity.

A Pioneer in Pharmaceutical Excellence and Academic Leadership

Professor Mora's career has been defined by extraordinary contributions to pharmacy practice, regulation and academia. Throughout his professional journey, he has exemplified the highest standards of integrity, dedication and leadership, leaving an indelible mark on pharmacy education, administration and regulation across Nigeria and beyond. His approach to teaching, regulation and administration has been both rigorous and inspiring. His transformative contributions to pharmacy regulation have strengthened Nigeria's healthcare system and positioned our pharmaceutical sector as a model for others. His rare ability to balance scientific rigor with practical application has ensured that regulatory frameworks serve both public health and professional development.

The Mentor and Teacher of Teachers

What truly sets Professor Mora apart is not only his professional achievements but also his unwavering passion for mentorship. As a mentor par excellence, he has nurtured countless young minds, guiding them toward excellence while instilling in them values of integrity, discipline and service. His door has always been open to pharmacists and academics seeking guidance. His advice, delivered with warmth, wisdom and practicality, has shaped the careers and

lives of many. His inclusive and empowering leadership style demonstrates that true leadership lies not in wielding authority but in inspiring others to reach their highest potential. Under his guidance, many have grown not just as professionals, but as men and women of character.

Institutional Leadership and Administrative Excellence

Professor Mora's brilliance as an administrator is evident in the many leadership positions he has held, most notably as a two-term President of the Ahmadu Bello University Alumni Association Worldwide. His tenure was marked by renewed vibrancy, expanded outreach and stronger bonds among alumni across the globe. Every institution he has served has been left stronger, more vibrant and more impactful. His ability to combine intellectual excellence with selfless dedication to institutional growth has made him not only a leader of leaders but also a builder of enduring legacies.

Lasting Impact and Enduring Legacy

Professor Mora's contributions extend far beyond academia and administration. His work in pharmaceutical curriculum development, regulatory policy and professional ethics has created frameworks that will continue to shape the profession for generations. His scholarly works, policy papers and educational initiatives remain vital references for pharmacy professionals today. Most importantly, he has elevated the status of the pharmacy profession. Through advocacy for pharmacists in healthcare delivery, emphasis on continuing professional development and alignment with global best practices, he has ensured that pharmacy is recognized as a cornerstone of national healthcare.

Personal Character and Values

On a personal level, Professor Mora exemplifies integrity, humility, generosity and compassion. Despite his towering achievements, he remains approachable, deeply caring and committed to the welfare of others. His life demonstrates that greatness lies not only in achievements, but in character and service. His recognition with

national honors, including the officer of Order of the Niger (*OON*) and Member of the National Institute (*mni*), is a fitting acknowledgment of his exceptional service to the nation and humanity.

Looking Forward: A Blueprint for the Future

As Professor Mora marks this milestone, we celebrate not only his past achievements but also the continuing influence of his work and mentorship. The principles he has championed, innovation anchored in ethics, regulation balanced with compassion, and leadership rooted in service, remain timeless blueprints for the future of healthcare, academia and public service.

Conclusion: A Life Well Lived, A Legacy That Endures

Professor Ahmed Tijjani Mora, *OON*, *mni*, your seven decades on earth have been defined by service, excellence, and impact. You are a teacher of teachers, a leader of leaders and a patriot whose life continues to inspire countless individuals across generations. Your legacy lives in the professionals you have mentored, the policies you have shaped, the standards you have set and the institutions you have strengthened. As you celebrate this milestone, may you find fulfillment in knowing that your contributions will continue to benefit humanity for generations to come. May the years ahead be filled with health, joy and peace. Your legacy is our inspiration and your example lights the path for those who follow. Happy 70th Birthday, Professor Mora. Thank you for your service, mentorship, leadership and unwavering commitment to excellence.

Maigirma Dr Haruna Muhammed Jumare, *PhD*, is the “*Marad'in Zazzau*” and a former Bursar of the Federal College of Education (now Federal University of Education), Zaria. During Professor Mora’s tenure as President Worldwide of the ABU Alumni Association, the *Marad'in Zazzau* served in several capacities, including Financial Secretary and Auditor, among others. Presently, he is the National Treasurer, Ahmadu Bello University Alumni Association & Director of Finance and Administration, Sir Ahmadu Bello Memorial Foundation He remains a member of the “National Executive Committee (NEC)” of the ABU Alumni Association.

Tribute to Prof. Ahmed Tijjani Mora, OON, mni
by Dr Ibrahim Mu'azzam Tunau, NBBS, FRCSI, Dip. LS, Dublin
Ireland

Bismillahi ar, Rahman Ar, Rahim

I am highly honoured to be invited to contribute to the book, on Professor Ahmad Tijjani Mora, *OON, mni*”.

I was brought up by my maternal grandfather, Alhaji Abubakar Tunau, *MFR (Kayayen Sardauna)*. My grandfather and Professor Ahmad Tijjani Mora’s father, Dr Abdurrahman Mora, were intimate friends. In addition, Professor Mora’s step-mother, Hajiya Binta Atti Abubakar Imam, and my step-grandmother, Hajiya Fatima Nana Bello Kagara, are first cousins. So, our two families are very close.

At a very young age, I knew most of Tijjani’s younger brothers and sisters but never knew or met him until sometime in early 1981 when he frequented our home in Sokoto. I did not realise what was happening until my favourite aunt, Aisha (*Uwa*) Tunau, told me she was going to marry him. I could see she was definitely in love, so I paid attention to him.

Professor Ahmad Tijjani Mora (Tijjani) is very charismatic, outspoken, sincere, kind and generous. He is a man of integrity and it shows in everything he does. He is very supportive and caring and he treats people with respect. I will always remember the short holiday I spent in Kano with him and Auntie Uwa in his house in late 1981. He made that visit very special. I was 18 years of age and he trusted me and handed me his car keys to drive to wherever I liked in Kano. He introduced me to some of his friends and all treated me like a friend. When he found out that I was planning to study Medicine, I got advice and guidance from him and his friends that would leave a lasting impact on me. He made me feel valued. His presence in my life is a blessing I never take for granted. Tijjani has a quiet way of

helping others and eventually, his home in Kaduna became a gathering place for so many friends and extended family.

Another great strength of Tijjani is that he is very hard working and devoted to his profession. He is renowned for scholarship and public service and he is prudent. As a pharmacist he made a significant contribution to the pharmaceutical world. What stands out for me is his contribution to the study and learning of pharmacy. Despite his busy National and International engagements, he serves as a part-time lecturer in the Faculty of Pharmaceutical sciences, Ahmadu Bello University, Zaria, his *Alma Mata*, teaching courses in Pharmacy Management to undergraduate Level 400 students from 1986 to date, *pro bono*, a period of about 40 years. He also teaches students registered for MSc Clinical Pharmacy and Pharmacy Practice and supervises PhD students at the same University.

He has been honoured with several awards by different bodies in Nigeria and abroad. One of such is the African leadership magazine award as Africa's most distinguished pharmacist of the year 2009 (Paris, France).

His input on the community stands out clearly. In Sep 2013, the 18th Emir of Zazzau, His Highness, Alhaji Shehu Idris awarded Tijjani with the title of *Wakilin Maganin Zazzau* (Royal Representative of Medicine) in recognition of his immense contributions to health in Kaduna State. In April 2021, the current (20th) Sultan of Sokoto, his eminence, Alhaji Muhammadu Sa'ad Abubakar III, CFR, awarded our Tijjani a new traditional title of *Kayayen Sarkin Musulmi*. He was also awarded with the National honour, the officer of the Order of the Niger (OON) by the 15th President of Nigeria, Asiwaju, Muhammadu Buhari, CFR in 2023.

I have certainly learnt a lot from this great man. I am grateful and proud to be associated with him as family (uncle-in-law). He is also a friend, a mentor and role model. May Allah grant Tijjani many more blessed and healthy years, amin.

Alhamdulillah.

Dr Ibraheem Mu'azzam Tunau, *MBBS, FRCSI, Dip. LS*

Dublin, Ireland.

Dr Mu'azzam Ibrahim Tunau is a Nigerian medical doctor based in Dublin, Ireland. He is the eldest grandchild of the late Alhaji Abubakar Tunau Mafara, *MFR (K'ayayen Sardauna)*, and a close family relation of Professor Mora. A surgeon by profession, he has practiced in Ireland for nearly three decades. The Tunau and Mora families share a long history of mutual respect and close ties.

TRIBUTE ON PROF. (PHARM) A.T. MORA

As founding member of Zaria Education Development Association, we established *ZEDA* together with Pharm. Ahmed Tijjani Mora in December 1992. He was the pioneer Chairman of PIFSS, Prominent Indigenes and Friends Scholarship Scheme Committee that offers scholarship to indigent students. He chaired *ZEDA* at 10 and 20, years anniversaries respectively, with live radio coverage. He was a member of the Central Working Committee of *ZEDA* before moving to become a member of the Board of Trustees of the Association. Pharm. Mora contributed immensely toward the book launch of *ZEDA*. And he is reliable, tenacious and forthright.

Dr. Abdul Alimi Bello

Chairman, Central/Working Committee
Zaria Education Dev. Association (*ZEDA*)

Dr Abdul-Alimi Bello is an ABU-trained veterinary doctor. In 1992, he, Professor Mora, and other young professionals founded the “Zaria Education Development Association (*ZEDA*)” to address declining standards of Western education in Zaria and Zazzau Emirate. Now in its 33rd year, *ZEDA* remains a model community-based educational initiative in Northern Nigeria, with Dr Bello as Chairman of its Central Working Committee and Professor Mora as a member of its Board of Trustees.

**A TRIBUTE IN HONOR OF PROF. A. T. MORA, *OON, mni*,
(*WAKILIN MAGANI ZAZZAU EMIRATE*)**

Presented by Dr. Lawal Ladan

Distinguished ladies and gentlemen, esteemed colleagues and noble friends of the profession, today, we gather not only to launch a book but also to celebrate a man whose very life has been a living testament of wisdom, courage and service: Professor Ahmed Tijjani Mora, *OON, mni*, (*Wakilin Magani of Zazzau Emirate*), a towering figure in pharmacy, education, administration and regulation, Prof. Mora remains a rare gift to our nation and indeed to the world.

He is a leader whose vision has shaped institutions, a mentor whose hands have lifted countless professionals and a pillar whose presence has steadied the noble field of pharmacy. His story is not merely about achievements, but about a life wholly devoted to service, excellence and humanity.

Permit me a personal reflection. Through the kind introduction of the *Dokajin Zazzau*, I first encountered this remarkable man. From that moment, a journey of partnership and fraternity began. Together, we pioneered the establishment of the Ahmadu Bello University Alumni Association in the Americas, a dream he embraced with passion and commitment. Prof. Mora stood firmly with us, laying the foundation for a bridge that continues to connect scholars across continents.

He was also present at the inaugural meeting of the Nigerian Association of Pharmacists and Pharmaceutical Scientists in the Americas (NAPPSA), a historic moment that charted a new course for our profession. Since then, he has remained a constant and shining presence at NAPPSA conferences, where ideas are exchanged, knowledge is shared and the spirit of mentorship flourishes. Year after year, he has engaged colleagues, inspired young professionals and challenged us all to pursue excellence without compromise.

Prof. Mora's influence transcends borders. In Nigeria, his imprint on regulatory frameworks has given structure, direction and integrity to pharmacy practice. Abroad, his wisdom and guidance have inspired generations of pharmacists and pharmaceutical scientists. He is both a bridge and a beacon, connecting the past with the future and Nigeria with the wider world.

As we launch his "Part" biography as a pharmacy regulator, we are reminded that Prof. Mora's life itself is a living memoir—rich in lessons, adorned with achievements and illuminated by service. His legacy will not only be preserved in the pages of books but also in the hearts and lives of those who have been touched by his wisdom and guided by his example.

Distinguished colleagues, let us rise to celebrate this great son of Nigeria, Prof. A. T. Mora, *OON, mni*, a man of vision, a mentor to many, a statesman of integrity and a colossus in the field of Pharmacy.

May his life continue to inspire us, and may his legacy endure for generations to come, Amin

Dr. Lawal Ladan
Atlanta, Georgia, USA.

Dr. Lawal Ladan is a USA-based Nigerian pharmacist practicing in Atlanta, Georgia. He has lived in the United States since the mid-1970s. Dr Ladan regularly attended the Annual Scientific Conferences and Expositions of NAPPSA with his African-American wife and frequently spent time with Professor Mora during such events. Originally from Suleja, Niger State, he visits Nigeria regularly and always pays courtesy visits to Professor Mora at his Abuja residence.

A Heartfelt Tribute to Professor Ahmed Tijjani Mora, OON, mni

Dr. Rahmatu Hassan, FWACS
Sarauniyar Zazzau Suleja

It is both gladdening and an honour to celebrate the life and legacy of Professor Ahmed Tijjani Mora, a distinguished Nigerian pharmacist, academic and administrator, who has left an indelible mark on the pharmacy profession and beyond.

With over four decades of dedicated service, Professor Mora excelled as a lecturer, researcher and practitioner, inspiring generations of students and professionals alike. His unwavering commitment to the advancement of pharmacy education and practice earned him numerous recognitions and national honours, including the Officer of the Order of the Niger (*OON*).

A trailblazer in his field, Professor Mora became the first Professor of Clinical Pharmacy and Pharmacy Practice at Igbinedion University, Okada. His remarkable leadership journey includes serving as Chairman of the Governing Council of the Pharmacy Council of Nigeria (*PCN*), Registrar/Chief Executive Officer of the *PCN* and National President of the Ahmadu Bello University (ABU) Alumni Association.

Beyond his outstanding professional achievements, Professor Mora's humility, kindness and dedication to mentorship distinguished him as a role model to many. His selfless service and deep commitment to humanity continue to inspire those privileged to have known or worked with him.

We celebrate his extraordinary contributions, his passion for excellence and his generosity of spirit. Indeed, his legacy will continue to shape the future of pharmacy, academia and leadership in Nigeria and beyond.

Dr (Mrs) Rahmatu Hassan is an ABU-trained medical doctor and consultant ophthalmologist. She was a schoolmate of Professor Mora at SBS/ABU, Zaria, in 1974. She later served as Managing Director of the "Federal Medical Centre (FMC), Birnin Kebbi", after years of service as a consultant ophthalmologist at UDUS. She holds the traditional title "*Sarauniyar Zazzau Suleja*"

TRIBUTE TO PROF. A. T. MORA, OON, mni

On the Occasion of His 70th Birthday

It gives me immense pleasure and a deep sense of alumni unity to write this tribute to our distinguished alumnus, **Professor Ahmad Tijjani Mora, OON, mni** on the occasion of his 70th birthday and the launch of a Special Publication in his honour.

I have known Prof. A. T. Mora over the years, particularly as a devoted alumnus of **Ahmadu Bello University (ABU)**, who served in various leadership capacities within the ABU Alumni Association with great honour and distinction. He was Deputy National President from 2010 to 2015 and National President from 2015 to 2021, during which he rendered exemplary service and oversaw numerous achievements, including:

- Construction of the first National Secretariat of the ABU Alumni Association at Area A, Main Campus, Samaru, Zaria.
- Building of an overhead pedestrian bridge at the North Gate, enhancing safety and movement between Samaru village and the Main Campus.
- Purchase and deployment of five buses to provide subsidised transportation for staff and students between Kongo and Samaru campuses.
- Restoration of water supply to 420 staff quarters in Kongo and *Banzazzau*, after more than 20 years without supply from the Kaduna State Water Board.
- Sponsorship of a one week capacity, building workshop for Deans and Directors of the University in partnership with the Administrative Staff College of Nigeria (ASCON), Badagry.
- Introduction of a laptop loan scheme in collaboration with OMATEC Computers, with 50 students as the first beneficiaries.

- Commencement of construction of a 104, room ensuite postgraduate hostel to improve living and learning conditions for postgraduate students.

It is therefore not surprising that he was unanimously elected, unopposed, as **National Chairman of the Conference of Alumni Associations of Nigerian Universities (CAANU)** from September 2016 to December 2022.

On a personal note, I must record my sincere appreciation to him for the invaluable support he accorded me during my tenure as Chairperson of the Ahmadu Bello University Alumni Association, FCT Chapter from 2010 to 2012.

Prof. Mora's record of service goes far beyond the Alumni Association. He has also distinguished himself as a leader, a mentor and a pillar in the field of pharmacy education, administration and regulation. His passion for the profession and unwavering devotion to excellence have earned him national and international recognition.

In appreciation of his dedication, patriotism, sacrifice and selfless service at various levels of our communities, he has been honoured with several traditional titles, including:

- *Wakilin Maganin Zazzau (2013)*
- *Iyan Piriga of Lere LGA (2018)*
- *K'ayayyèn Sarkin Musulmi (2021)*

As you turn 70, Prof., it is our prayer that Almighty Allah will continue to bless you and your family, grant you many more years in good health and peace of mind and enable you to continue contributing positively to our nation and humanity.

Happy 70th Birthday!

Dr. Salamatu Hussaini

- The former Chairperson, ABU Alumni Association (FCT Chapter)
- Member, Ahmadu Bello University Endowment Fund, Board of Trustees
- The former Minister of Women Affairs and Social Development
- The former Minister of State, Foreign Affairs

Barrister (Dr.) Salamatu Suleiman is an alumna of the Faculty of Law, Ahmadu Bello University, Zaria. She has had a distinguished national career, including serving as the Honourable Minister of Women Affairs and Social Development. Her professional path crossed with that of Professor Mora when she served as Chairperson of the ABU Alumni Association (FCT Chapter) while Professor Mora was the Deputy National President of the Association. They share deep mutual respect and a strong passion for their *Alma Mater*, ABU, Zaria.

**Tribute to Professor Ahmed Tijjani Mora, OON, mni,
FWAPCPharm, FPSN**

Astute Leader, Bridge Builder and Alumni Pillar

It is with profound honour and gratitude that I pay this tribute to **Professor Ahmed Tijjani Mora, OON, mni**, a distinguished scholar, pharmacist, administrator and, above all, a consummate leader whose towering contributions to university alumni development in Nigeria will remain evergreen.

I had the rare privilege of serving as his Deputy Chairman during his eventful leadership of the **Conference of Alumni Associations of Nigerian Universities (CAANU)** from September 2016 to December 2022. Those six years were, without doubt, a defining era in the history of CAANU marked by unprecedented achievements, national integration and transformational leadership.

A Unifying and Visionary Leader

Prof. Mora provided exemplary, nation, building leadership by uniting alumni associations from universities across Nigeria and fostering integration across the six geo-political zones. He was not merely a chairman in title, but a true unifier who positioned alumni associations as partners in the growth of universities while strengthening ties across regions, cultures and traditions.

Historic Achievements in Alumni Mobilisation

One of the landmark achievements of Prof. Mora's tenure was the successful organisation of **fourteen (14) Delegate Conferences** across universities in Nigeria's six geo, political zones, an unmatched record in CAANU's history. These conferences provided vibrant platforms for dialogue, policy engagement and strengthened the voice of alumni in national discourse while promoting inclusivity and integration. He also presided over **two highly successful National Conventions in Makurdi and Kano**, which set new standards in alumni mobilisation and participation. These Conventions attracted

record attendance, strengthened networks and produced far, reaching resolutions that enriched alumni university relations nationwide.

Partnership with Traditional Institutions

Another historic milestone under his leadership was the appointment and investiture of **six eminent traditional rulers as Patrons of CAANU**, representing each geo-political zone. These included:

- The Emir of Bauchi, Dr. Rilwanu Suleiman Adamu, *CFR*
- The Emir of Lafia, Justice Sidi Bage Muhammad I, JSC (Rtd.), *CFR*
- The Emir of Zazzau, Ambassador Ahmad Nuhu Bamalli, *CFR*
- The *Obi* of Onitsha, His Majesty Igwe Alfred Nnaemeka Achebe, *CFR, mni*
- The *Ooni* of Ife, Oba Adeyeye Enitan Ogunwusi, Ojaja II, *CFR*
- The *Oba* of Benin, His Royal Majesty Oba Ewuare II, *CFR*

Each investiture, conducted in their palaces, elevated CAANU's national profile and cemented enduring partnerships with Nigeria's revered traditional institutions.

A Champion of Peace and Reconciliation

Prof. Mora also distinguished himself as a pillar of peace and reconciliation. Alumni associations, like many human organisations, occasionally faced disputes and divisions. At such moments, he intervened with wisdom, patience and diplomacy to restore harmony. He consistently encouraged dialogue, mutual respect and compromise, preferring out-of-court settlements as the pathway to lasting peace. By mediating disputes across alumni bodies, he preserved unity and ensured that alumni energies were channelled toward supporting their alma maters.

Institution Building and Lasting Legacies

Prof. Mora's leadership extended to strengthening alumni institutions. As **President of the Ahmadu Bello University Alumni Association**, he championed and completed the construction of a befitting National Secretariat. Later, as CAANU Chairman, he facilitated the donation of office space at the Federal Ministry of

Education Annexe, Abuja, courtesy of then the Honourable Minister of Education, Malam Adamu Adamu. Under his guidance, this was renovated and transformed into CAANU's **first, ever National Secretariat**, a legacy that stands as a testament to his vision. He also led CAANU into constructive engagement with the **Federal Government, National Assembly and relevant agencies**, making meaningful contributions to national conversations on university education and offering practical solutions to challenges within the system. His statesmanship ensured that the alumni community gained recognition as a credible stakeholder in shaping the future of higher education in Nigeria.

A Leader with a Human Face

Beyond institutional accomplishments, Prof. Mora led with empathy and deep concern for the welfare of his cabinet members and alumni leaders across the country. He nurtured growth, encouraged professional development and created a sense of belonging within CAANU. His fatherly care inspired loyalty, strengthened teamwork and made his leadership not only effective but also humane.

A Lasting Inspiration

Prof. Ahmed Tijjani Mora, *OON, mni*, is not only a trailblazer in pharmacy and public service but also a visionary in alumni leadership. His tenure as CAANU Chairman stands as a **golden era**, marked by unity, institutional growth, peacebuilding and national impact. His humility, God-fearing nature and love for humanity shone through all his engagements, while his authorship of numerous books further underscored his intellectual depth and commitment to knowledge.

As one who worked closely with him during those six years, I can affirm that Prof. Mora's leadership was nothing short of transformative. He remains an enduring inspiration to all who aspire to serve with **integrity, vision and selflessness**.

Indeed, **Prof. Ahmed Tijjani Mora, OON, mni**, is a worthy alumnus of Ahmadu Bello University, a beacon of hope for the alumni community and a true nation builder.

Dr. Tajudeen Olusegun Uthman

The Former Deputy Chairman, CAANU

Dr. Tajudeen Olusegun Uthman is a former President Worldwide of the Tai Solarin University of Education Alumni Association and former Deputy National Chairman of the “Conference of Alumni Associations of Nigerian Universities (CAANU)” from 2018 to 2022. He had earlier served during Professor Mora’s first term as Chairman of CAANU as Treasurer and has remained in touch with him ever since.

Prof. Ahmed Tijjani Mora, OON, mni

Ahmed Mora – The Regulator

It with great pleasure that I accept the invitation to reflect on my professional interactions with Prof Ahmed Tijjani Mora.

One Monday night in 2023, I was watching the late night news. My phone rang. With eyes focused on the TV screen, I answered the call. Then I heard a familiar voice. I glanced at the phone. It was a little after 10 pm (US Eastern Time). And it was a direct, dial call from Nigeria (I never installed the WhatsApp program on my phone until recently).

So, I said, *“Ahmed, it is after 3 am in Abuja. Why are you calling me in the middle of the night? Is everything OK?”*

He explained that a mutual friend had called him earlier that evening, saying that I wanted Ahmed to call me. He forgot the message and went to sleep. Around 3 am in the night he woke up and remembered that he hadn’t called me. And so, he was calling to make sure that everything was OK!

I was so moved that I said, *“Ahmed, you are no longer a friend”*. To which he responds: “Oh, oh... how come?” I continued, *“You are now my brother, because only my brother would wake up in the middle of the night to call to check if I am OK.”*

This incident sheds some light on Prof Ahmed Mora’s humanity and his sense of duty and caring for people both personally and as a Pharmacy Regulator.

In September 2007, I was the founding Program Committee Chair of the Nigerian Association of Pharmacists & Pharmaceutical Scientists in the Americas (NAPPSA). We had invited Prof Dora Akunyili, the then Director General of Nigeria’s National Agency for Food and Drug Administration and Control (NAFDAC) as our Keynote Speaker. We also invited the Registrar of the "Pharmacists Council of Nigeria" (PCN) as the Pharmacy Council of Nigeria was known at that time.

Ahmed Mora, the then PCN Registrar, attended and made an impressive Presentation at that inaugural NAPPSA Conference in Houston in September 2007. That was my first time of meeting Ahmed Mora.

In July 2009, Ahmed Mora as the PCN Registrar, in collaboration with NAFDAC, arranged a series of 2-day Training Sessions for Pharmacists and the Pharmaceutical Industry in Lagos, Abuja and Enugu. I was an invited Speaker at these sessions. This gave me a close up opportunity to work with and understand him.

Over the past eighteen or so years, we have maintained a professional relationship. He is a frequent attendee and Presenter at NAPPSA Annual Conferences. He worked hard for “Reciprocity” to streamline the process by which foreign trained Pharmacists are registered to practice in Nigeria, a process that was finally achieved under his successor as Registrar of PCN, Pharm NAE Mohammed. NAPPSA presented Ahmed with an Award in recognition of his participation, commitment and leadership in NAPPSA matters. Ahmed Mora, is a Pharmaceutical Regulator’s Regulator! He is simply the best!

Echeazu Ogu, *PhD*

President,

BonScience Inc

Delaware, USA

Phone: +1 302 521 5366

September 30 2025

Dr Echeazu Ogu, *PhD* (popularly known as “*Wazirin Delaware*”), is a USA-based Nigerian pharmaceutical scientist residing in Delaware. Through NAPPSA, Professor Mora became acquainted with him. An expert in pharmaceutical manufacturing validation, Dr Ugwu was engaged by Professor Mora during his tenure as Registrar and CEO of the PCN to conduct two-day workshops in Lagos, Abuja, and Enugu for manufacturing and quality-control personnel in pharmaceutical manufacturing companies nationwide.

A TRIBUTE TO PROFESSOR A. T. MORA, OON, mni

I feel deeply privileged and with immense gratitude to Almighty Allah to be deemed worthy of paying this tribute to my father, a man whose giant footprints have left a golden mark on the sands of history, particularly in our family, community, profession, academia and in Nigeria as a whole.

When I contemplated writing this tribute, I felt humbled and inadequate, for my perception of my father, who is the third of the nine sons out of sixteen children of my grandfather, Dr. Abdurrahman Mora, from his two wives, Hajiya Aminatu Dikko and Hajiya Binta Abubakar Imam, is so vast and glorious that words can scarcely capture it.

He is my mentor and a mentor to many whose lives he has touched. An accomplished Professor of Pharmacy, a seasoned administrator, an acclaimed educationist and a scholar of international repute, he has spent the greater part of his life serving humanity, molding lives and adding value wherever he goes. His focus has always been to leave things better than he met them.

Daddy is an exemplary husband, father and a successful family man, who goes to great lengths to ensure the needs of his immediate and extended family members are met. He is also a philanthropist, ever ready to open his doors to widows, orphans, the poor and the needy, who come knocking.

We are grateful for having him as our moral compass, the guiding star that the younger ones in the family look up to. His pivotal role in championing family causes and initiatives, such as the establishment of the Abdurrahman Mora Education Foundation (TAMEF) in honor of his father (my grandfather), is a heritage that reflects his visionary leadership. It is a lasting legacy that will continue to provide opportunities for generations of our family.

His open arms and diverse support, both temporal and spiritual, together with his trusted medical advice, have been a source of strength, success and well-being for our family and cannot be appreciated enough.

Indeed, I lack the perfect words to fully describe my admiration and gratitude for him. I only hope that this brief tribute captures a glimpse of our collective appreciation of his qualities and greatness as the icon of the Abdurrahman Mora family. We pray to continue imbibing and exemplifying the virtues he stands for integrity, honesty, humility, truthfulness and sincerity, without hypocrisy or sycophancy.

In conclusion, I thank Almighty Allah, the Most Merciful, for granting Professor Ahmed Tijjani Mora the grace of attaining the glorious age of 70 years. Our fervent prayers are that Allah grants him many more years in good health, wisdom and blessings as he continues to remain our pride and banner of honor.

THANK YOU AND CONGRATULATIONS, DADDY!

Halima Inuwa Mora (PhD)

(For the Abdurrahman Mora Offspring)

Dr Halima Inuwa Mora, *PhD*, is the eldest grandchild of the late Mallam (Dr) Abdurrahman Mora (1916–1997), father of Professor Mora. She is the first daughter of the late Alhaji Muhammadu Inuwa Mora (1940–2001). A lecturer at “Kaduna Polytechnic”, she submitted this tribute on behalf of the brothers, sisters, nephews, nieces and grandchildren of Professor Mora.

**TRIBUTE TO PHARMACIST PROFESSOR AHMED
TIJJANI MORA, *mni, FPSN, FPCPharm*, ON THE
OCCASION OF HIS 70TH BIRTHDAY**

Today, as we celebrate the 70th birthday of Pharmacist, Professor Ahmed Tijjani Mora, we honour not merely the passing of time, but the remarkable life of an exceptional professional, administrator, innovator and nation builder whose footprints remain deeply imprinted on Nigeria's healthcare and pharmaceutical landscape.

My association with Prof. Mora dates back to my tenure as Honourable Commissioner for Health in Kaduna State, when circumstances placed before me a decision that would shape the course of our professional relationship. Following the dissolution of the Kaduna State Health Management Board and its merger with the Ministry of Health, I had to choose between two Directors of Pharmaceutical Services, the incumbent from the Ministry and the DPS from the defunct HMB. After careful evaluation, I chose Prof. Mora. It was a decision I have never regretted, as he went on to serve with distinction, competence and integrity.

In 1992, Kaduna State was selected as one of only five states in Nigeria to benefit from an African Development Bank (AfDB) facility for the rehabilitation of secondary healthcare facilities. The position of Project Manager for the Project Implementation Unit (PIU) attracted keen competition and lobbying from many within and outside the Ministry. Prof. Mora never lobbied, nor did he ask me for the position. Yet, I was convinced he was the most qualified, armed with an MBA from Ahmadu Bello University in addition to his pharmacy training and with a proven record of management discipline. With the Governor's approval, I appointed him as the first Project Manager.

In this role, Prof. Mora not only set up and equipped the PIU but also earned the respect of his peers across the four other participating states. He was eventually elected Chairman of all five Project Managers and Procurement Officers. He represented Kaduna State

with excellence, including at the AfDB headquarters in Abidjan, and played a central role in the signing of the landmark agreement between the Bank, the Federal Government and the five states in May 1993. During the same period, under my guidance and support, Prof. Mora worked tirelessly towards the establishment of the Zaria Pharmaceutical Company, manufacturers of the renowned *ZARINJECT* syringes and needles, the largest disposable syringe manufacturing plant in West Africa at the time. His leadership as Project Coordinator, while simultaneously serving as DPS and Project Manager of the AfDB, assisted health rehabilitation programme, reflected his exceptional capacity for hard work, innovation and vision. By the time the plant was commissioned in May 1996, Prof. Mora had laid the foundation for what became an enduring industrial legacy in Nigeria's health sector.

His career trajectory continued to reflect resilience and excellence. Even when political circumstances led to his recall from the position of MD/CEO of the ZPC in 1999, he upheld professionalism and integrity. In the early 2000s, when a Commission of Inquiry, constituted without proper jurisdiction, was set up to investigate the Zaria Pharmaceutical Company, Prof. Mora did not hesitate to appear. With candour, facts and transparency, he defended the project and emerged vindicated.

In December 2003, I had the honour of chairing the reception organised by the Pharmaceutical Society of Nigeria (Kaduna State Branch) to celebrate his appointment as Registrar and CEO of the Pharmacists Council of Nigeria (PCN) after a rigorous, merit-based selection process. This appointment marked yet another milestone in his illustrious career, placing him at the helm of the statutory body regulating pharmacy education, training and practice in Nigeria.

Prof. Ahmad Tijjani Mora's professional life is distinguished by three enduring qualities: **integrity, merit-driven achievement and an unwavering commitment to service**. His journey is a compelling

testament to what is possible when competence is combined with character.

On this milestone 70th birthday, we salute a thoroughbred professional, a committed public servant, a dedicated teacher and mentor, a community leader, a patriot and a loyal friend. May the years ahead bring you continued good health, fulfilment, and the joy of knowing that your life's work has touched countless lives and will continue to inspire generations.

Happy Birthday, Prof. Mora.

Your legacy is secure and your name will forever be associated with excellence and service in the annals of Nigeria's health sector.

Hon. Yusuf Hamisu Abubakar

Alhaji Yusuf Hamisu Abubakar, popularly known as *Mairago*, is a lawyer and a graduate of the Faculty of Law, Bayero University, Kano (BUK). He is a respected politician in Kaduna State and served as Honourable Commissioner at the Ministry of Health, Kaduna State, in the early 1990s during the administration of the late Alhaji Dabo Mohammed Lere. He was Commissioner of Health at a time when Professor Mora served as Chief Pharmacist and Director of Pharmaceutical Services, Project Coordinator of the Zaria Pharmaceutical Syringes Production Company, and first Project Manager of the Project Implementation Unit (PIU) of the AfDB-assisted Kaduna Health Rehabilitation Project at the Ministry of Health Headquarters, Kaduna. He is currently the Chairman of the Nigerian Maritime Administration and Safety Agency (NIMASA). There exists a strong relationship of mutual respect between him and Professor Mora.

**A Tribute to Prof. A. T. Mora, OON, mni
A Paragon of Strength and Exemplary Leadership**

Reflecting on the life and legacy of **Prof. A. T. Mora, OON, mni** fills me with immense pride, gratitude and admiration for a man who has been a constant presence in my life. As a fellow alumnus of the esteemed **Barewa College, Class of 1970 intake**, I have had the privilege of witnessing Mora's remarkable evolution, from a bright and ambitious young man into a distinguished leader, mentor, and stalwart in the realms of pharmacy education, administration and regulation.

Throughout his life, Mora has consistently demonstrated an unwavering commitment to the welfare of others. His selflessness, generosity, and kindness have profoundly impacted countless lives. His guidance, support and encouragement have been a beacon of strength for many. Indeed, one of his most remarkable qualities is his deep empathy; whatever affects those around him resonates deeply with him, and he never rests until a solution is found.

Mora's illustrious career in pharmacy education, administration and regulation stands as a model of **dedication, diligence and pursuit of excellence**. His contributions to the field have earned both national and international recognition, culminating in prestigious honours, including the esteemed titles of ***Wakilin Maganin Zazzau*** and ***Kayayen Sarkin Musulmi***, conferred by the Emir of Zazzau and the Sultan of Sokoto, respectively.

As we commemorate Mora's **70th birthday**, we acknowledge the Divine blessings that have adorned his life. We pray for continued guidance, protection and many more years of good health, happiness, and service to humanity.

To my dear friend, Mora, I extend my heartfelt congratulations on this milestone. May this special day usher in yet another chapter of joy, love and fulfilment in your remarkable life. I am honoured to call you a friend, brother and colleague.

May Allah continue to bless you and your family. *Alhamdulillah!*

With warm regards and best wishes,

HRH. Alh. Ya'u Shehu Usman

(Zakari Ya'u II)

**Emir of Kauru,
Kauru LGA, Kaduna State**

His Royal Highness Ya'u Shehu Usman (*Zakari Ya'u II*) is the Emir of Kauru, Kauru Local Government Area of Kaduna State. He and Professor Mora were classmates at Barewa College, Zaria (1970–1974), and maintained a close relationship, particularly during their periods of service in the Kaduna State Civil Service. The Kauru Emirate is an ancient Hausa community with an establishment history spanning over 500 years.

A TRIBUTE IN HONOUR OF PROF. A. T. MORA, OON, *mni*

Professor Ahmed T. Mora is a renowned authority in pharmaceutical practice, not only in Nigeria but also across the West African sub-region. I first met this quintessential gentleman in 1970, when we both gained admission into Barewa College, Zaria. We were in the same class. My admission number was B 2510, while his was B 2511. From the onset, he displayed brilliance, meticulousness and exemplary conduct. These attributes earned him the unanimous confidence of our Form Master and classmates, leading to his election as Class Monitor from Form Three in 1972 until we completed our training at Barewa College. Later, he was appointed one of the two prefects of Nagwamatse House, one of the students' dormitories in Barewa College, Zaria.

Over the years, I personally observed his remarkable leadership qualities of integrity, justice and fairness. A few instances stand out:

An Act of Fairness at Barewa College (1970): During our first year, I had a quarrel with a classmate, a huge fellow, who attempted to jump the queue into the dining hall and later chased me to the fives court near Dan Hausa House. Prof. Mora, sensing a fight was about to erupt, immediately intervened in my defense. He held the other student accountable, despite the fact that I was a Christian and non-Hausa, while my assailant was Hausa. Religion and ethnicity were never factors in his judgment of right and wrong, a reflection of Barewa College's enduring tradition of fairness since its founding in 1921.

Restoration of Justice in Piriga Chiefdom: Another profound example of Prof. Mora's moral courage was his role in advocating for my reinstatement as the Paramount Traditional Leader of Piriga Chiefdom in Lere Local Government. Upon hearing of my deposition, he personally investigated the matter and found the action

unjustifiable. Even while attending an international conference abroad, he strongly condemned the decision, pointing out that no query or warning had ever been issued to me. He rightly recognized it as a case of impunity, high, handedness and abuse of power. Prof. Mora's acceptance and influence across the state and the nation, particularly among traditional rulers, is remarkable. He holds prestigious titles such as *Kayaye of the Sultanate*, *Wakilin Magani of Zazzau Emirate* and *Iyan Piriga Chieftdom*.

His contributions to pharmaceutical education, practice and research are iconic. Having served two terms (2003–2012) as Registrar and Chief Executive Officer (CEO) of the Pharmacy Council of Nigeria (PCN), he is indeed an institution within the pharmacy profession. For over thirty years, he has also been delivering free lectures to undergraduate and postgraduate pharmacy students at Ahmadu Bello University, Zaria, a testament to his passion for mentorship and knowledge sharing.

It is also a matter of pride to Barewa Old Boys that one of our own was appointed Pro-Chancellor and Chairman of the Governing Council of Igbinedion University, Okada (IUO), Edo State, the first private university in Nigeria, established in 1999. This appointment was in recognition of his wealth of experience, integrity and outstanding academic credentials.

Undoubtedly, his credibility and towering profile may attract envy from retrogressive elements in society. Yet, his faith in God, hard work, transparency and the goodwill of patriotic Nigerians will continue to sustain him. In my humble view, the Federal Government should consider technocrats of Prof. Mora's caliber for Ministerial appointments, where his expertise can greatly improve the nation's incritical Sectors.

Finally, this Book Launch could not have come at a more fitting time, as Prof. Mora celebrates his 70th birthday. It is my fervent prayer that

Almighty God blesses him with long life, sound health and continued grace as he remains steadfast in his service to humanity, amen.

HH Jonathan Phraguwa Zamuna

Ukarma Piriga I

Lere LGA, Kaduna State

His Royal Highness Jonathan Phraguwa Zamuna JP, is the “*Ukarma* of the Piriga Kingdom” in Lere Local Government Area of Kaduna State. He attended Barewa College, Zaria, and sat next to Professor Mora in Form 1B upon their admission into the College. His Royal Highness had admission number “B.2510”, while Professor Mora had “B.2511”. An alumnus of ABU, Zaria, and a university in Atlanta, Georgia, USA, where he obtained his Master’s degree, the *Ukarma* worked in the Kaduna State and Federal Public Services before retiring as “Director of Administration” at the National Eye Centre, Kaduna. In April 2018, he was appointed and installed as the *Ukarma* of Piriga Kingdom by the Kaduna State Government. On the same day and at the same venue, he appointed Professor Mora, his former classmate and longtime friend, as the “*Iyan Piriga*”, a traditional title the *Ukarma* held prior to his elevation as a Royal Father.

**A TRIBUTE OF PROFESSOR AHMED TIJJANI MORA,
OON, mni
LIEUTENANT GENERAL AB DAMBAZAU (RTD) *PhD,*
*CFR***

It is both a privilege and a duty to offer this tribute to Professor A. T. Mora, *OON, mni* a man whose life embodies the values of service, leadership and unwavering commitment to the advancement of our nation. For decades, Prof. Mora has stood at the intersection of pharmacy education, professional regulation and public service, leaving an indelible mark on the health sector and the many lives he has touched.

I have known Professor Ahmed Mora since 1970 when we met as students of Barewa College Zaria and I found in him a rare combination of intellect, discipline and humility. As a leader, he possesses the vision to chart new paths, the courage to defend what is right and the patience to guide others towards excellence. In a country where the health of the people is both a moral duty and a national security imperative, Prof. Mora has been a steadfast guardian of standards, ensuring that pharmacy practice in Nigeria remains anchored on professionalism, ethics and competence.

His tenure in various capacities, as an educator, administrator and regulator, has been defined by results. Under his leadership, pharmacy regulation became more robust, with systems that not only protect the public but also elevate the profession. He understood that the regulation of pharmacy practice is not merely about compliance, but about building trust, trust between the professional and the patient, between institutions and the society they serve.

What has always struck me most about Prof. Mora is his humanity. Behind the distinguished titles and official responsibilities is a man

deeply committed to mentoring others. Many who now lead in the pharmacy profession, academia and public service trace their growth to his guidance. He is a mentor who challenges without discouraging, corrects without humiliating and inspires without seeking the spotlight. In an age where leadership is often measured in titles, he reminds us that its true measure lies in the number of people empowered to carry the vision forward.

I have also admired his discipline and work ethic. Coming from a background where standards and accountability are non-negotiable, I recognised in him the same ethos. He approaches every task with meticulous preparation, engages with clarity of thought and carries himself with the dignity befitting his office. It is no surprise that his contributions have been recognised nationally and internationally and that he has earned the respect of colleagues, policymakers and the general public alike.

Beyond his professional accomplishments, Prof. Mora is a patriot in the truest sense, one who sees service to Nigeria not as an obligation, but as a calling. His dedication to improving healthcare delivery, strengthening pharmacy institutions and mentoring the next generation is, to my mind, a legacy that will endure long after titles fade and official duties are handed over.

As he marks his 70th birthday, we not only celebrate the milestones of his career but also the character and values that have defined his journey. Prof. Mora's life is a reminder that leadership is not about personal gain, but about stewardship; that influence is not about control, but about example and that true success is measured in the lives bettered and the systems strengthened through one's service.

On behalf of all those who have had the privilege to work alongside him and in my personal capacity as one who has benefited from his friendship and wise counsel, I extend my heartfelt congratulations. May the years ahead be filled with health,

peace and the continued satisfaction of seeing the seeds you have planted bear fruit across the nation.

Nigeria is better because you served. The pharmacy profession is stronger because you led. And those of us who know you are richer for the example you have set.

Lieutenant-General Abdurrahman Bello Dambazau, *CFR, PhD*, is one of the finest officers produced by the Nigerian Armed Forces. He and Professor Mora were classmates at Barewa College, Zaria, and have remained close ever since. He was among three classmates admitted into the Nigerian Defence Academy (NDA) who all rose to the rank of General before retirement. The others were “His Eminence Sultan Muhammad Sa’ad Abubakar, *CFR*”, and an Air Commodore fighter pilot who died in 2018. General Dambazau served as “Chief of Army Staff (2008–2010)” and later as “Minister of Interior (2015–2019)”. He has recently been nominated and confirmed as an Ambassador by President Bola Ahmed Tinubu, *GCFR*. He holds the traditional title “*Baraden Kano*”.

TO MY BIG BROTHER, PROF. A. T. MORA, WHO SEES THE BEST IN EVERYONE.

From our Barewa College days to adulthood, I have always admired your positive nature, it truly makes you a rare gem. If I were to describe Prof. Mora, I would begin with my vivid memories of our Barewa days, moments that perfectly capture the generous spirit you bring to the world.

One moment later in life that I will forever treasure was making a turnaround trip from my base in Los Angeles, California, to Las Vegas, Nevada, just to spend time with you and the team during a NAPPSA conference in June, 2017. That experience remains very special to me.

I have nothing but the utmost respect for you. Your achievements are limitless, and I am proud to be your junior friend. May the Almighty Allah continue to bless, guide and protect you always. Ameen. Stay blessed.

**Moqhtari Mohammed Danmallam
Malumfashi**

Moqhtari Muhammad Danmallam attended Barewa College, Zaria where he was a year junior to Prof. Ahmed Mora. His father, Mallam Muhammad Danmallam, was a highly respected personality from Malumfashi and served as an Honourable Commissioner in the former North Central State (now Kaduna and Katsina States) in Northern Nigeria. Mallam Moqhtari has lived in Los Angeles, USA, for almost four decades, where he has built a thriving business and has a wife and children. Since their days at Barewa College, Zaria Prof. Mora has maintained a warm and cordial relationship with Mallam Moqhtari, who continues to visit Nigeria regularly.

**TRIBUTE IN HONOUR OF A GENIUS: PROF. A. T.
MORA, OON, mni**

All glory, honour and adoration be unto Almighty God for this special day, as *Babalawo*, a Yoruba word for pharmacist or herbalist, fondly called by me, marks his 70th birthday.

I sincerely appreciate the committee members of this publication for their wisdom and tireless efforts in putting together this book in honour of **Prof. A. T. Mora**, a great man of our time. I am elated and humbled to be counted worthy to contribute to this tribute.

My journey with Prof. Mora began in 2008 under the umbrella of the **Committee of Registrars for Health Professions (CRHP)**. The aims and objectives of CRHP are to foster unity among health professionals, ensure patients receive the best possible care from duly registered professionals and fight quackery to the barest minimum.

During one of the most challenging periods of my life, I benefitted immensely from CRHP under Prof. Mora's watch as Chairman. His leadership, wisdom and guidance left an indelible mark on my professional journey.

Over the years, Prof. Mora proved himself to be a seasoned administrator and an efficient leader. The welfare of CRHP members remained a top priority throughout his tenure as Chairman of this revered committee.

A landmark achievement of his leadership was the transformation of the nomenclature of Health Regulatory Registrars from pensionable appointments to executive cadre positions with tenureship. This change professionalised the office, strengthened institutional integrity and elevated the profile of health regulatory agencies in Nigeria.

My brother, my friend and my *Babalawo*, I celebrate you today. I wish you nothing but the very best that life has to offer. By God's grace, your latter days shall be greater, richer and more impactful than your former.

Congratulations! Congratulations!! Congratulations!!!

Mrs. Olusola Orimoloye

The Former Registrar/CEO

Dental Technologists Registration Board of Nigeria

Mrs. Olusola Orimoloye is a former Registrar and Chief Executive Officer of the Dental Technologists Registration Board of Nigeria. She also served as Secretary of the Committee of Registrars of Health Professions (CRHP) during the period when Professor Mora was the Registrar and CEO of the PCN and Chairman of the Committee. Professor Mora held this chairmanship for an unprecedented six (6) years, from 2006 to 2012.

**NATIONAL PILLAR: REFLECTIONS ON THE LIFELONG
CONTRIBUTIONS OF PROFESSOR AHMED TIJJANI
MORA, *FPSN, FPCPharm, FNIM, OON mni*.**

**A PAPER PRESENTED AT THE SECOND ANNUAL
SYMPOSIUM AND SECOND LECTURE IN HONOR OF
PROF. A. T. MORA, *OON, mni* (K'AYAYEN SARKIN
MUSULMI, WAKILIN MAGANIN ZAZZAU AND IYAN
PIRIGA) ORGANIZED BY THE PHARMACEUTICAL
ASSOCIATION OF NIGERIAN STUDENTS (PANS), ABU,
HELD AT THE FACULTY OF PHARMACEUTICAL
SCIENCES, AHMADU BELLO UNIVERSITY, ZARIA ON
Saturday, 23RD AUGUST, 2025.**

BY

**PHARM. ALIYU JIBRIN, *FPSN, FPCPharm, MNIM*
MANAGING DIRECTOR, NAMAMA PHARMACY
LIMITED KADUNA.**

1. PREAMBLE.

Let me firstly express my appreciation to PANS, ABU for inviting me as a speaker at this annual symposium, which also incorporate a lecture in honor of Prof. A. T. Mora, *OON, mni*, the life Patron of PANS, ABU, Zaria. I'm told that the lecture aims to bring into focus the life of who they described as, and truly so, a 'National Icon', who has impacted the lives of many positively and in so many ways and continue to do so till date. This is an honor well-deserved. Some of us are not surprised at all as it is something we have been expecting. The only surprise is that it is the PANS that took the first position. But I believe there will be more of such events.

Again, this is the second in the series of the lectures with the first one being held in August last year with the Registrar of the *PCN*, Pharm. Ibrahim Baba Shehu Ahmed, *FPSN, FPCPharm, FNAPharm* as the Guest Lecturer. There couldn't have been a better choice than the current Registrar, who has worked directly under Prof. Mora and, therefore, witnessed all the transformations brought about by this

Phenomenal Pharmacist first as Registrar/ CEO for eight years and then as the Chairman of the Governing Council of the PCN for three years. I'm glad to be the second Speaker in this very important and laudable initiative.

2. REMINISCENCE

Some thirty-nine or so years ago (1986/87), we were intern Pharmacists at the ABUTH Zaria, Kaduna and Malumfashi now in Katsina State. Our Chief Pharmacist then was late Mr G. O. Obiaga of blessed memory. He was also the Financial Secretary of the PSN Kaduna State branch. So, we could not escape the payment of our membership annual dues, which was sixty naira (N60.00) only. We were of course thought to attend PSN general meetings usually held on Sundays and rotated between Kaduna, Zaria, Kafanchan and Katsina.

At every meeting, we used to notice one very cute, well-dressed Pharmacist, who was full of energy and seemed to be involved in almost every activity of the PSN. One of the main issues for discussion then at every meeting was what they called COPHARM. And the chairman would say let us hear progress report on COPHARM project from Alhaji Mora.

Alhaji Mora would come out and speak very good English giving details of his report, demonstrating that he knew his onions. That was really inspiring. He appeared to be one of the driving forces of PSN Kaduna state branch then. Of course, it wasn't long before he became the Chairman of the branch in 1989. One amazing thing is that we still see the same energy and passion for his profession even after he has celebrated his 69th birthday in May last year.

3. WHO IS PROFESSOR MORA?

Simply put, Prof. Mora can be said to be an accomplished Pharmacist. In the paper presented last year during the first lecture, the Registrar of the PCN, Pharm. Ibrahim Baba Shehu Ahmed, *FPSN, FPCPharm, FNAPharm* described accomplished individuals

as “those who inspire others through their achievements, mentorship and passion. They also demonstrate commitment to excellence, driving positive change and leaving a lasting legacy”. He went on to give us the characteristics of accomplished individuals as;

- ✓ Significant contribution and input
- ✓ Strong work ethic and dedication
- ✓ Resilience and adaptability
- ✓ Visionary thinking and innovation
- ✓ Continuous learning and self-improvement.

With his benevolence and majestic outlook, Prof. Mora is a representation of a successful professional of great achievement whose works have cut across all aspects of pharmacy practice. His wealth of knowledge, length of experience and wisdom has earned him vast popularity unparalleled by those of his contemporaries. Over the years, he has shown immense ability in being a mentor as well as a leader to depend on. He is indeed a pillar as far as a pharmacy profession is concerned in this country and even beyond. Meriam Webster dictionary described a “Pillar” as someone who is an important member of a group; a supporting, integral or upstanding member of a society. Prof. Mora fits into this definition.

4. THE CONTRIBUTIONS OF PROF. A. T. MORA

The lifelong contributions of Prof. Mora to humanity are legendary. All his life he has been making significant contributions, leaving lasting legacies and foot-prints everywhere he set his foot on. Let me give a very brief chronicle of his contributions generally as follows:

4.1 CONTRIBUTION TO PSN (STATE AND NATIONAL)

- From being a member of committees, to secretary of committees to Chairman of committees, he became the PSN Kaduna State Chairman.
- Chief Pharmacist Kaduna State Health Management Board and later DPS Kaduna State.

- By virtue of the above, he was a member of PSN Council and
- Member of the then Pharmacists Board of Nigeria (PBN)
- In 1999 again, he was amongst the eight (8) members representing PSN in the Council of PCN.
- He was the CPC Chairman of the PSN conference tagged “COAL CITY 99” held in Enugu. In November, 1999.
- In 2002, he was the Chairman of the Committee that organized what was called “The First Special Pharmacists Summit” in Kaduna where leaders in all aspects of pharmacy practice in Nigeria converged to fashion a way forward for pharmacy practice in the country.

4.2 CONTRIBUTION TO PHARMACY EDUCATION AND TRAINING

- From 1986 to date, Prof. Mora has been teaching Pharmacy Management in this Faculty free of charge.
- He was founding Dean of the Faculty of Pharmaceutical Sciences, Kaduna State University (KASU), Kaduna.
- He was the first Professor of Clinical Pharmacy and Pharmacy Practice at Igbenedion University Okada (IUO), Edo state.
- Not quite long, he was appointed as the Pro-chancellor and Chairman of the 7th Governing Council of Igbenedion University Okada (IUO), Edo state.

4.3 CONTRIBUTION TO PHARMACY PRACTICE AND REGULATIONS

The contributions of Prof. Mora to Pharmacy practice and regulation through the Pharmacists Council of Nigeria and later Pharmacy council of Nigeria (*PCN*) are vital.

Prof. A. T. Mora was appointed the Registrar/CEO of the *PCN* on merit from October 2003 to January 2012. In his opening remarks at

the 14th ordinary meeting of the Council held on February 14th 2004, the then Chairman of the council, Prof. Festus Adio Ogunbona had this to say: *“let me congratulate you on your choice of Alhaji Ahmed Mora as the Registrar and Secretary to this Council. He has proven to be a very astute, very hardworking and highly diligent Manager of men and resources. With him at the helm of affairs in the Registry, your Council is set to match forward and take its rightful place as the most respectable regulatory body in the country. Congratulations, our dear Registrar for your brilliant performance so far and we say more grease to your elbow”*.

Of course, PCN did not only take its rightful place but the PCN Registrar was elected to become the Chairman of the Committee of Registrars of Health Professions (CRHP), thereby projecting the image of Pharmacy and Pharmacists. He was the first Chairman to serve for three terms in the history of that Committee.

As they say, the reward for good work is more work. In March 2019, His Excellency President Muhammadu Buhari, *GCFR* appointed him as the Chairman of the Governing Council of the PCN. Although this was followed by lots of bickering, insinuations and bad publicity from some quarters, he was nevertheless inaugurated as the Chairman of the Governing Council of the PCN on Monday, 15th June 2020.

In his response during the inauguration, Prof. Mora said, *“As I conclude this post inauguration speech, let me seize this golden opportunity towards the process initiation of healing of wounds perceived or indeed inflicted on persons or groups that are stake holders in pharmacy profession over the years. I was subjected to all manner of humiliation, persecution and bad publicity by past and present leadership of the PSN including being dragged to the ICPC during the past ten years (2009 to date) which coincided with greater part of my second term as Registrar and beyond. I was exonerated by the ICPC however. I had to resort to courts to redeem my name and image and at a particular instance instituted libel case against the Society and which I was prevailed upon by some leaders of the*

pharmacy profession to withdraw as way forward for reconciliation. I withdrew the case against the advice of my Lawyer just to allow peace to reign in 2014. I want to say here and now that despite the anguish, psychological trauma and unnecessary anxiety by the past and present leadership of the PSN these past years on myself, family members and many of my admirers among whom are my former and present students in ABU, KASU and IUO, Okada, professional colleagues, especially the thirty seven (37) Directors of Pharmaceutical services in the states and the FCT that came out to fully support my appointment by His Excellency, President Muhammadu Buhari, GCFR, I bear no animosity or grudge to the Society or anyone. I hope as Pharmacists, we should towards strengthening the profession and not breaking it”.

As two-term Registrar/CEO and later Chairman of the Governing Council of the PCN, Prof. Mora introduced radical re-structuring and transformation of the council into a global player in pharmacy regulation. From just four (4) offices when he assumed duty, he created forty (40) additional State and Zonal offices, including the ones he constructed from foundation to commissioning in Minna, Bauchi, Ibadan, Uyo and Kaduna, thereby bringing regulation closer to practitioners and creating more job opportunities for Pharmacists and others as well.

Again, from 99 staff when he became the Registrar the staff strength of PCN rose to 274 by January 2012 making PCN the regulatory body with the highest number of staff out of the fourteen bodies under the Federal Ministry of Health. He met only nine (9) operational vehicles in PCN and he increased that forty-two (42) by procuring thirty-three (33) additional vehicles.

One of the characteristics of a good leader and mentor is visionary thinking and innovation. Prof. Mora demonstrated this when he led a delegation on a courtesy visit to the office of the Head of Service of the Federation, Dr Folashade Yemi-Esan, on Tuesday, 18th August, 2020 when he made just three (3) requests;

1. Creation of additional department specifically for ICT as the Council intends to computerize tracking of the drug distribution system in the country in liaison with relevant regulatory agencies such as NAFDAC.
2. Approval of waiver for the employment of additional staff to enable PCN adequately respond to regulatory demands across the country.
3. Increase in the number of GL 17 positions at the PCN beyond the number of available departments to enable more staff members reach that ultimate position.

It is also worth mentioning that Prof. Mora as the Chairman of the Governing Council of PCN conducted a seamless and transparent recruitment exercise that produced the current Registrar, our dear, vibrant and dynamic Pharm. Ibrahim Baba Shehu Ahmed, *FPSN, FPCPharm, FNAPharm*. This is in tandem with the sayings that “leaders don’t create followers; they create more leaders”. And to become truly great, one has to stand with people not above them. It also demonstrates that leadership is no longer about your position. It is more about your passion for excellence and making a difference. Prof. Mora no doubt has left indelible marks at PCN. And we can confidently say that the story of PCN can never be complete without the mention of Prof. Mora.

4.4 CONTRIBUTIONS TO SOCIETY THROUGH INVOLVEMENT IN THE FORMATION AND/OR RUNNING OTHER ORGANISATIONS

Prof. Mora is a very versatile professional having his hands in many meat pies at the same time. His contributions do not end with his profession only. He has traversed many segments of the society.

4.4.1 ZARIA EDUCATION DEVELOPMENT ASSOCIATION (ZEDA)

Prof. Mora is a founding member and member of the Central Working Committee of ZEDA from 1992 to 2011. ZEDA is non-

political, voluntary and community-based association dedicated to the improvement of education in Zaria, its environs and Nigeria in general.

From 2011 to date, he has been a member of the Board of Trustees of ZEDA.

4.4.2 BAREWA OLD BOYS ASSOCIATION (BOBA)

Prof. Mora can be said to be one of the most active members of BOBA. Since 1994, he established BOBA (1970 SET), his own class set and served as its chairman for eighteen (18) years up to 2012 when he voluntarily resigned having been appointed as the Secretary-General of the main body, a position he held up to 2021. Before then, he has been a member of the National Executive Committee since 2002 becoming second Assistant Secretary-General from 2008 to 2012. As chief scribe of BOBA from 2012 to 2022 and who worked with three (3) different Presidents of the Association, he accomplished so many novel programs and projects as intervention for the College.

- ✓ Held AGM consistently throughout his tenure when funds were raised and roads were named in the College.
- ✓ Held Annual Lecture, Dinner and recognition Awards events as well as Centenary Celebrations.
- ✓ Produced and presented many publications of the Association (BOBA newsletter, Almanacs, Scorecards etc.) to inform members and the general public of the activities of BOBA.
- ✓ Requested for books and established a library at the National Secretariat in Kaduna.
- ✓ Sponsorship of capacity building workshops for Teachers of Barewa College.

4.4.3 AHMADU BELLO UNIVERSITY (ABU) ALUMNI ASSOCIATION

Prof. Mora is deeply involved in the activities of ABU Alumni Association. He was elected as Deputy President of the Association

from 2010 to 2015. In August 2015, he was unanimously elected as the President. After completing his tenure, he was re-elected for a second final term on 17th August, 2017 till 28th February, 2021. His contributions to ABU Alumni Association were to say the least, spectacular. A few need to be mentioned.

- ✓ Ensured the building of the overhead pedestrian bridge at North gate Samaru at the cost of eighty million naira.
- ✓ Launched the laptop computer loan scheme for the students of the University.
- ✓ Initiated the intra campus bus shuttle services between Samaru and Congo campuses with five (5) busses flying the routes.

4.4.4 THE CONFERENCE OF THE ALUMNI ASSOCIATIONS OF THE NIGERIAN UNIVERSITIES (CAANU)

This Association now known as Association of Nigerian Universities Alumni (ANUA) is the umbrella body for all Nigerian Universities Alumni Associations. At the National Convention of the Association held at Abubakar Tafawa Balewa University (ATBU) Bauchi, on 24th September, 2016, Prof. Mora was elected unopposed as the new National Chairman of the Association for a period of two years in accordance with the constitution of CAANU. He was re-elected in 2018 in keenly contested election. His tenure did not end until 2022 after serving for six (6) years consecutive years. His achievements?

- ✓ Appointed six (6) Patrons of CAANU and performed the investiture of five (5) of them before leaving office.
- ✓ Renovation and commissioning of the National Secretariat of CAANU at the Federal Ministry of Education annex, Abuja.
- ✓ Amended the Constitution of CAANU to allow for four-year tenure which is renewable and also created six Vice Chairmen positions.
- ✓ Initiated the West African University Alumni Association (WAUAN)

4.4.5 AHMADU BELLO UNIVERSITY PHARMACY ALUMNI ASSOCIATION (ABUPAA)

ABUPAA was formed in his office in Kaduna in 1994 with him as the Interim Chairman. He was made the Substantive Chairman at the AGM held in 1997. Re- elected in 2008. ABUPAA is the first faculty-based Alumni Association in A.B.U. He ensured ABUPAA got registered with Corporate Affairs Commission (CAC) in 2010 with him as the Chairman Board of Trustees (BOT). Through ABUPAA, Prof. Mora engineered the largest single intervention project in ABU when he got the then Governor of Taraba State, Pharm. Danbaba Danfulani Suntai, of blessed memory a member of ABUPAA to build this particular twin 250 – seater capacity lecture theatre, which was commissioned on 15th October, 2011. And named Gabriel Osuide Lecture Theatres (GOLT)

4.4.6 PHARM. D. S. WALI BOOK PROJECT COMMITTEE

The initiation of the book, '*D. S. WALI: AN ICON OF COMMUNITY PHARMACY PRACTICE IN NIGERIA*' started on Monday, 7th February, 2011 at the Dean's Office, Faculty of Pharmaceutical Sciences, ABU Zaria. It was after the Induction and oath taking ceremony for fresh Bachelor of Pharmacy degree graduates of the Faculty administered by the then Registrar, Prof. A. T. Mora *mni*. Discussions and testimonies among us present in the office on the contributions of Mallam D. S. Wali to the Faculty led to the spontaneous decision to write a book on him. Prof. Mora offered to be the Chairman of the Committee with myself as the Secretary. Other members of the committee are Prof. Ibrahim Adamu Yakasai, Prof. Umar Usman Pateh, and Pharm. Ahmed Mohammed Gana. Suffice it to say that the book has been written and launched at Mambayya House, Kano. Thereafter, the Committee continued to arrange series of annual public lectures in honor of Mallam Wali since his 80th birthday in 2015 to date. This year's lecture, which is the eleventh, was held at the Faculty of Pharmaceutical Sciences, Bayero University, Kano. But who bankrolled all the activities of this

committee? None other than Prof. A. T. Mora, *mni*. He has been the one funding it single-handedly all these years. And he has been doing that cheerfully from the bottom of his heart. You will agree with me he has a big heart. But one good thing deserves another, as people say. Today, we are holding a lecture in his honor and I want to believe that this is just the beginning. Our people say ‘*Alheri ba ya faduwa kasa banza*’.

5. CONCLUSION

I would like to once again thank the leadership of PANS, ABU for giving me this opportunity to say a little about my leader my very reliable elder brother, a pharmacist of repute, a colossus in pharmacy practice, education and regulation. A highly organized, focused and very objective personality who always strives for excellence and not given to mediocrity. A leader who is able to surmount the usual challenges of leadership; strong but not rude, kind but not weak, bold but not bully, thoughtful but not lazy, humble but not timid and proud but not arrogant. And to put the icing on the cake, God Almighty has endowed him with good health and physique, enormous energy and capacity as well as excellent vision and brain.

No wonder, ChatGPT has identified Prof. Mora as the most renowned Pharmacist in Nigeria. “Renowned” can mean different things – influence on the profession, national recognition, leadership role or global impact. Nigeria has produced several highly renowned Pharmacists over the decades in academia, industry, regulatory practice and public service. These include the late Prof. Dora Akunyili, FPSN, Prof. Chinedom Peace Babalola, FPSN, Prof. Isa Marte Hussain and Pharm (Mazi) Sam Ohuabunwa, FPSN, FPCPharm, MFR.

Prof. Mora has proven to be a game changer and no doubt a pillar. Little wonder in the last thirty (30) years he has received close to one hundred awards and recognitions from many quarters. The history of pharmacy in Nigeria and beyond and indeed many other

organizations he served would never be complete without the mention of Prof. Ahmed Tijjani Mora, *OON*, *mni* in gold.

Thank you for listening.

**PHARM. ALIYU JIBRIN, *FPSN*, *FPCPharm*, *MNIM*
MANAGING DIRECTOR,
NAMAMA PHARMACY LIMITED KADUNA.**

Pharm. Aliyu Jibrin is an ABU-trained pharmacist and Chairman of the Book Project Committee. He and Professor Mora have worked together across various pharmacy practice platforms in Kaduna State and Nigeria. Both served on the “Pharm. D. S. Wali Book Project Committee”, which culminated in the presentation of “D. S. Wali: An Icon of Community Pharmacy Practice in Nigeria” in April 2013, Kano. Pharm. Jibrin was one-time Chairman of the PSN (Kaduna State Branch).

**A TRIBUTE TO PROFESSOR AHMED TIJJANI
MORA, OON, mni**

(Dr. Ibrahim Jatau Abubakar)

Professor Ahmed Tijjani Mora stands as a remarkable figure whose life and career embody dedication, integrity and excellence. His journey through the field of pharmacy and public service reflects not only professional competence but also a deep commitment to humanity and the advancement of the health sector in Nigeria.

From his early days as a student to his distinguished roles in academia, administration and regulation, Professor Mora has consistently displayed visionary leadership. His contributions to the Pharmacy Council of Nigeria (PCN), as Registrar and Chief Executive Officer, left an indelible mark on the regulation and practice of pharmacy in the country. Through his reforms, he enhanced standards, strengthened compliance, and elevated the professional status of pharmacists nationwide. Beyond his administrative brilliance, Professor Mora is a teacher at heart. Generations of students and professionals have benefited from his mentorship, wisdom and humility. His scholarly works and numerous publications continue to inspire intellectual growth and policy advancement within the health system.

He is also a man of deep faith and character, whose humility and moral rectitude are as admirable as his academic and professional achievements. His traditional title of *Wakilin Maganin Zazzau* is not merely honorary but a reflection of his enduring commitment to service, community and cultural heritage.

As a leader, Professor Mora's life demonstrates that true greatness lies in service, service to knowledge, to one's profession and to humanity. His enduring legacy is that of a mentor, reformer and patriot who has continuously raised the bar of excellence.

We celebrate Professor Mora not only for what he has achieved but also for what he continues to represent, a beacon of inspiration, discipline and integrity for all who follow in his path.

May Allah, the Most Merciful, continue to bless him with sound health, wisdom and fulfillment in all his endeavours.

— *Dr. Ibrahim Jatau Abubakar, Ph.D*
Melbourne, Australia

Dr Ibrahim Jatau Abubakar is a Nigerian pharmacist practicing in Australia. Trained at ABU, Zaria, he was an undergraduate student of Professor Mora and later served at the PCN during Professor Mora's tenure as Registrar and CEO. He later relocated to Tasmania, obtained his PhD, and recently moved to Melbourne. In 2017, he founded the "Young Pharmacists Scholars (now Pharmacists Scholars Initiative – PSI)", with Professor Mora as Grand Patron.

Prof. Ahmed T. Mora, OON, mni, FPSN

A Bosom Friend and Classmate

Pharm. Nuhu Isah Abdullahi, FPSN, JP (Dankaden Gumel)

I am deeply honored to say a few words about Prof. A. T. Mora, fondly known to us, his classmates, as “A.T.”

Our journey together began in 1975 at the Department of Pharmacy, and Pharmacology Faculty of Science, Ahmadu Bello University (ABU), Zaria. Along with other colleagues, we graduated with a Bachelor of Science degree in Pharmacy in 1978. A.T. Mora gained admission into the Department through the School of Basic Studies (SBS) Zaria, having earlier attended Barewa College. I, on the other hand, gained admission through the College of Advanced Studies (CAS), Kano, after attending Government College Birnin Kudu.

Within a short period, A.T. Mora and I became very close friends, together with several others, including the late Auwalu Abdulsalam, the late Nuhu Namallam Ibrahim, Mansir Ali, the late Sani Baba, the late Sadauki, Abu Sada, Zagbayi Nuhu, the late Salisu Ibrahim, the late Lubabatu, Abubakar, the late Hauwa Aliyu Jafiya, Maryam Isa Dutse, Ramatuh Yarkano, Lawal Balah and many others too numerous to mention.

A.T. Mora could best be described as a man of excellent character, responsible, humane, disciplined, supportive and remarkably friendly. He was admired by all students, irrespective of ethnicity, gender or religious background.

Even at the undergraduate level, A.T. Mora demonstrated exceptional academic ability and leadership. He often organized and led student discussion groups, motivating us to study diligently. His study group was particularly successful; none of its members ever had to resit an exam or repeat a year. His commitment and mentorship were instrumental to our collective academic success. It is, therefore, no surprise that his intellectual prowess and dedication to learning ultimately earned him the distinguished rank of Professor.

A.T. Mora's human nature and genuine friendship continued well beyond our university years. He maintained close and cordial relationships with most of our classmates, including those residing in Kano. Personally, I, together with my late friend Auwalu Abdulsalam, remain deeply grateful for his consistent concern, care and support.

Allow me to share a few memorable examples:

- (i) In 1994, when I was appointed Director, General/Permanent Secretary, Jigawa State Ministry of Health, Social Welfare and Community Development, A.T. Mora was among the first to identify with and celebrate me.
- (ii) In 2012, when I was turbaned with the traditional title of *Dankaden Gumel*, he traveled all the way from Kaduna to Kano and organized a befitting reception in my honor.
- (iii) In 2018, when the late Auwalu Abdulsalam and I were conferred as Fellows of the Pharmaceutical Society of Nigeria (FPSN), Prof. Mora again graced the occasion in Kano and celebrated with us.
- (iv) He has always been present at the wedding *fatiha* of our children and stood by us during times of loss, offering comfort and solidarity.

His contributions to the development and progress of our dear nation, Kaduna State, his immediate community and the pharmacy profession at large, are immeasurable. His dedication has rightly earned him several prestigious honors: *Officer of the Order of the Niger (OON)*, *Member of the National Institute (mni)*, *Fellow of the Pharmaceutical Society of Nigeria (FPSN)* and the revered traditional titles of *Wakilin Maganin Zazzau* and *Kayayen Sarkin Musulmi*

We, his classmates and friends from ABU Zaria, are exceedingly proud to be associated with Prof. A. T. Mora and his remarkable life of excellence, service and distinction.

Maigirma Pharm. Nuhu Isa Abdullahi, FPSN, the “*Dankaden Gumel*”, is a classmate and close friend of Professor Mora from their days at the Faculty of Pharmaceutical Sciences, ABU, Zaria. He had a distinguished career in the public service of Kano and later Jigawa State, rising to the rank of “Permanent Secretary” before retirement.

PROF. A. T. MORA: Lessons from the Career of an Unmatched and Accomplished Pharmacist

A Paper Presented by

**Pharm. Ibrahim Babashehu Ahmed, FPSN, FPCPharm,
FNAPharm, FCAI**

Registrar, Pharmacy Council of Nigeria (PCN)

At the **Annual Symposium and Lecture** organized by the
Pharmaceutical Association of Nigeria Students (PANS),
Ahmadu Bello University, Zaria, in Honour of

Prof. Ahmed Tijjani Mora, OON, mni
(*Wakilin Maganin Zazzau, K'ayayen Sarkin Musulmi*)

Ice Breaker

Quotations on Great Personalities

- “A great person is not great because they are famous or wealthy; a great person is great because they make others great.” – *Phylicia Rashad*
- “Born with personality is an accident, but dying with a personality is an achievement.” –
(*www.AwesomeQuotes4u.com*)

Who is an Accomplished Person?

An accomplished person is an individual who is highly skilled, experienced and often recognized or awarded in their field. Such a person possesses a unique blend of expertise, knowledge and values that distinguish them from others.

The Characteristics of an Accomplished Person

- Exceptional expertise and mastery in their field.
- Significant contributions and impact.
- Strong work ethic and dedication.
- Resilience and adaptability.
- Visionary thinking and innovation.
- Effective leadership and collaboration.
- Continuous learning and self, improvement.

- Integrity, professionalism, and ethical standards.

Accomplished individuals inspire others through mentorship, achievements and passion. They embody a commitment to excellence, driving positive change and leaving a lasting legacy.

In pharmacy, an accomplished person like **Prof. A. T. Mora** exemplifies these qualities through his outstanding achievements and service.

The Prospects of the Pharmacy Profession

Pharmacy is a noble, respected and dynamic profession. It offers graduates diverse career paths:

- Industrial Pharmacy;
- Community Pharmacy;
- Hospital Pharmacy;
- Pharmacy Administration;
- Pharmaceutical Research;
- Academia;
- Public service and other areas of human endeavour

Prof. Mora's career is a shining example, spanning these fields with exceptional accomplishments.

The Journey to Accomplishment – The Story of Prof. A. T. Mora

Prof. Mora's illustrious career demonstrates versatility, dedication and leadership:

1. **Hospital Practice:** Began as an intern and youth service pharmacist, later, Chief Pharmacist through clinical excellence.
2. **Administration:** Served as Chief Pharmacist, Kaduna State and became the first Director of Pharmaceutical Services (DPS), Kaduna State Ministry of Health. He also represented the State on the defunct Pharmacy Board of Nigeria.

3. **Industrial Pharmacy:** Managing Director/CEO of Zaria Pharmaceutical Company (ZPC) Ltd, (Manufacturers of *ZARINJECT* Syringes and Needles) and CEO of Zazzau Pharmaceutical Industries Ltd (ZPIL), (Producers of Liquid and Solid Formulations).
4. **Academia:** Foundation Dean, Faculty of Pharmaceutical Sciences, Kaduna State University (KASU); & pioneer Head of Dept, Clinical Pharmacy & Pharmacy Management. At Igbinedion University, Okada (IUO) he became a Professor. He has also taught Pharmacy Management *pro bono* at ABU Zaria since 1986.
5. **Regulatory Leadership:** Registrar/CEO, Pharmacists Council of Nigeria (2003–2012); Chairman, Governing Council of PCN (2020–2022) and the first Chairman, Governing Council of the Pharmacy Council of Nigeria (2022–2023).

His tenure witnessed innovations, such as the review of the *Compendium of Minimum Standards of Practice*, a U.S. study tour and enhanced logistics for PCN operations through vehicle allocation from the Nigeria Customs Service. At every stage, he left unmatched footprints.

Testimonials on Prof. Mora

Dr. Abubakar Sha’aban

PSI Co, the Founder and Vice President, Cardiff University, UK

“Prof. Mora exemplifies mentorship at its finest. His dedication and willingness to engage with us, despite his demanding schedule, inspire us to strive for excellence. His presence is a constant reminder of the value of nurturing the next generation.”

Dr. Ibrahim Jatau

The tFounder, Pharmacy Scholars Initiative (PSI), Department of Health, Tasmania, Australia

“Prof. Mora embodies true mentorship. As Grand Patron of PSI since 2017, his consistent guidance has been invaluable. His distinguished career reflects his devotion to pharmacy, education, and public health. His passion for developing the next generation of pharmacists is unmatched.”

Career Milestones

- Pioneer National Chairman, ABU Pharmacy Alumni Association (ABUPAA) (1994–2010); now BoT Chairman (2010–date).
- National Chairman, Conference of Alumni Associations of Nigerian Universities (CAANU) (2016–2022).
- President, Worldwide ABU Alumni Association (2015–2021).
- Secretary, General, Barewa Old Boys Association (BOBA) (2012–2021).
- Chairman, Committee of Health Regulatory Professionals (CHRP) (2008–2012).
- Interim Chairman, West African Pharmaceutical Manufacturers Association (WAPMA) (2002–2003).

Awards and Honours

- Fellow, Pharmaceutical Society of Nigeria (FPSN).
- Fellow, Nigerian Association of Industrial Pharmacists (NAIP) – Industrial Leadership Award.
- Fellow, Institute of Corporate Executives of Nigeria (FICEN).
- Fellow, Institute of Health Management of Nigeria (FIHMN).
- Fellow, Nigerian Institute of Management (FNIM).
- African Leadership Magazine Award – *African Pharmacist of the Year* (2009).
- Officer of the Order of the Niger (OON), Federal Government of Nigeria.
- *Wakilin Maganin Zazzau* – traditional title, Zazzau Emirate Council (2013).

- *Iyan Piriga* – traditional title, Piriga Chiefdom, Kaduna State (2018).
- *K'ayayen Sarkin Musulmi* – traditional title, Sokoto Sultanate Council (2021).

Lessons Learned

1. **Versatility** – Adaptability across hospital, industry, academia and administration.
2. **Leadership** – Rising to the top in every field of engagement.
3. **Continuous Learning** – Fellowships, titles and ongoing education reflect growth.
4. **Service** – Commitment to alumni, councils and community.
5. **Excellence** – Pursuit of high standards, awards and recognition.
6. **Resilience** – Overcoming challenges across diverse roles.
7. **Networking** – Building lasting professional and social relationships.
8. **Balance** – Harmonizing professional and family life.
9. **Legacy** – Lasting contributions to pharmacy, education and community.
10. **Humility** – Recognizing his “modest contributions” despite unmatched achievements.

Prof. Mora’s journey demonstrates that success is not defined solely by personal achievements but by the positive impact left on others.

“Success is not a destination, but a journey. It’s the culmination of efforts, sacrifices and perseverance.” – Malcolm Gladwell

Lessons for Success

- **Commit** – Set realistic goals, stay disciplined and adapt when needed.
- **Learn from the Journey** – Celebrate small victories and grow from experiences.
- **Have Fun** – Let passion fuel productivity.
- **Think Positively** – Believe in yourself and embrace challenges as opportunities.
- **Minimize Distractions** – Focus on what matters and surround yourself with positive people.
- **Plan and Track** – Set SMART goals (Specific, Measurable, Achievable, Relevant, Time, bound).
- **Persist** – Hard work, dedication, and resilience are non-negotiable.

Conclusion

Success in pharmacy, or any field, requires dedication, perseverance and lifelong learning. Prof. A. T. Mora’s career embodies this truth: a lifetime of service, leadership and impact.

As **Madisyn McKee** aptly stated:

“Being successful takes time and a lot of practice. Success does not happen overnight. Often it takes years of repeated effort, or even failure, before one can reap the rewards.”

Prof. Mora's life and career remind us that true success lies in service, resilience and legacy.

Thank you for listening.

Pharm. Ibrahim Babashehu Ahmed is the current Registrar and Chief Executive Officer of the Pharmacy Council of Nigeria (PCN), Abuja. He was taught by Professor Ahmed Tijjani Mora at the undergraduate level in the Faculty of Pharmaceutical Sciences, ABU, Zaria. During Professor Mora's tenure as Registrar and CEO of the PCN (2003–2012), Pharm. Babashehu joined the Council from the Niger State Civil Service and rose through the ranks to the position of Director. In 2020, when Professor Mora was appointed Chairman of the Governing Council of the PCN, Pharm. Babashehu emerged as the highest-scoring candidate among thirty-two applicants in the written and oral examinations and was subsequently appointed Registrar and CEO by the Governing Council.

CELEBRATING 70 YEARS OF EXCELLENCE

A Tribute to Prof. A. T. Mora, OON, mni

Happy 70th birthday to the illustrious **Prof. Ahmed Tijjani Mora**! As a seasoned pharmacist, administrator, regulator and the founding Dean of our Faculty, you have left an enduring legacy that continues to inspire generations of students. Although you left the university before our time, your impact resonates through the stories shared by our senior colleagues, who speak highly of your dedication to their academic, moral and financial growth.

Your leadership as the foundation Dean of our Faculty set a high standard of professionalism and organisation among pharmacy students at **Kaduna State University (KASU)**. Under your guidance, our Faculty became known for its discipline, corporate dress code and commitment to the values of the profession. This legacy continues to shape our identity and inspire future generations of pharmacists.

During my tenure as **PANS President at KASU**, I was privileged to experience your mentorship and support firsthand. Your encouragement and trust meant the world to us. I vividly recall when you insisted that we attend the **PANS National Convention (Unleash, 21)** in Uyo, Akwa Ibom State despite our security concerns. Not only did you encourage us, but you also generously sponsored my flight alongside my colleague, **Pharm. Huzaifa Ahmad Muhammad**, then PANS President of ABU's Faculty of Pharmaceutical Sciences. Your advice, ***“Take every good chance that will boost your leadership”***, has remained a game changer in my journey and I am deeply grateful for your mentorship and selflessness.

What touched me most was your return to lecture my immediate juniors. I witnessed your commitment through your guidance, provision of writing materials and distribution of free textbooks. Such dedication extended beyond KASU, as echoed by students from ABU and

Igbinedion University, who benefitted from your generosity. Your devotion to students' growth and well, being is truly remarkable.

Dear Professor, you stand out in every sense of the word. Your legacy extends far beyond your tenure as dean, shaping countless lives, including mine. Here's to many more years of inspiring and empowering pharmacy students! May your 70th birthday be a joyous celebration of your remarkable life's work and profound impact.

We thank God for your achievements in both your personal and professional life. Your humility, hard work, intellect and digital engagement are worthy of emulation and continue to inspire us all. Personally, I thank you for being a father figure and a role model. We celebrate you today and look forward to many more celebrations ahead.

God bless you abundantly.

With best regards,

Pharm. Salisu Bashir Bawa

Former PANS President, Faculty of Pharmaceutical Sciences, KASU

Pharm. Salisu Bashir Bawa is a pharmacist and a graduate of the Faculty of Pharmaceutical Sciences, Kaduna State University (KASU), Kaduna. Professor Mora was the foundation Dean of the Faculty and has served as the Grand Patron of the Pharmaceutical Association of Nigerian Students (PANS), KASU Chapter. Pharm. Bawa once served as President of PANS, KASU Chapter.

TRIBUTE TO AN ICON
PROF. AHMED TIJJANI MORA, *PhD, mni.*

Today, we gather to celebrate a remarkable milestone – the **70th birthday** of our esteemed senior colleague and mentor. Seventy years of life, filled with experiences, achievements and contributions that have left an indelible mark on our profession and on our lives.

As we reflect on **Prof. A. T. Mora's** illustrious career, we are reminded of his unwavering dedication, his passion for excellence, and his commitment to mentoring and guiding others. With a career spanning decade, Prof. Mora has been a beacon of inspiration, a role model and a shining example of professionalism.

Throughout his journey, he has demonstrated **exceptional leadership, vision, and expertise**. His contributions to pharmacy practice, education and professional development have been groundbreaking, innovative and far-reaching. He has inspired generations of professionals, including myself, with his tireless work ethic, meticulous attention to detail and relentless pursuit of excellence.

One of **Prof. A. T. Mora's** most enduring qualities is his ability to connect with people from all walks of life. His warm smile, infectious enthusiasm and genuine interest in others have made him a beloved figure in the profession and beyond. He possesses a rare gift for bringing people together, fostering collaboration and creating a strong sense of community.

I recall a personal encounter with **Prof. Mora in 2017**, when he was appointed Chairman of the Pharmacists Council of Nigeria (PCN) Board by the then President Muhammadu Buhari, of blessed memory. Unfortunately, he was initially denied inauguration due to a petition orchestrated by a PSN cabal. I personally led a delegation of the **Association of States' Ministries of Health Directors of**

Pharmaceutical Services to meet the then Minister of Health. Through God's grace and collective effort, the matter was resolved speedily and the Board was inaugurated. Regrettably, much valuable time had already been lost and the tenure elapsed with the end of that administration on **May 29th, 2023**.

On this special day, however, we do not dwell on challenges but celebrate **Prof. Mora's resilience, achievements and character**. His integrity, empathy and kindness have earned him the deep respect and admiration of his peers. He is a true professional, a master of his craft and an enduring role model for us all.

As we wish him a happy 70th birthday, we also acknowledge the **lasting impact** he has had on our lives and careers. His legacy will continue to inspire, motivate and guide us as we navigate the complexities of the health sector and the wider society.

To **Prof. Ahmed Tijjani Mora**, we offer our heartfelt congratulations on this milestone birthday. May this special day be filled with love, laughter and celebration. May your years ahead be as bright, impactful and remarkable as your past. Here's to many more decades of wisdom, creativity and joy!

Pharm. Yakubu Akawu

Permanent Secretary

Taraba State Ministry of Energy

Pharm. Yakubu Akawu is a pharmacist and civil servant in the Taraba State Government and currently serves as the Permanent Secretary, Ministry of Energy, Taraba State. He was an undergraduate student of Professor Mora in the Faculty of Pharmaceutical Sciences, ABU, Zaria. Over the years, a strong and enduring bond developed between Professor Mora as a lecturer and Pharm. Akawu as his student.

**TRIBUTE TO PROF. AHMED TIJJANI MORA *FPSN*,
FPCPharm,
FICEN, FNIM, OON, mni.**

It is a rare privilege and an honour of a life time to have this opportunity to pen down this tribute to my mentor and role model. Prof. A T Mora whom I first met as an undergraduate at ABU Zaria where he has been teaching for free since 1986. His unique teaching skills and deep understanding of Pharmacy education, career path and kindness has continued to impact and shape students not only in ABU Zaria but also in Kaduna State University where he was the pioneering Dean of the Faculty of Pharmaceutical Sciences. The Collage of Pharmacy, Igbinedion University has continued to benefit from his academic experience and mentorship. His period in the industry as MD/CEO of Zaria Pharmaceutical Company and later Zazzau Pharmaceutical Company was remarkable. Prof. Mora brought them into Pharmaceutical Manufacturing Group of Manufacturers Association of Nigeria (PMG-MAN) and enhanced their production capacities. He left an indelible foot-print in the industrial sector. As a regulator, I was privileged to take my oath to practice pharmacy when he served as Registrar of Pharmacy Council of Nigeria [PCN] where he served for 8 years overseeing the review of pharmacy curriculum and opening of PCN offices in every state of the Federation, strengthening regulation and promoting quality pharmacy practice.

Prof. A T Mora an administrator per excellence pioneered the Directorate of Pharmaceutical Sciences in the Ministry of Health Kaduna State. His policy frame work enhanced the distribution of health commodities as well as human capacity building. This unique pedigree of being an administrator, regulator, industrialist and astute academic has placed Prof. A T Mora at a vintage position enabling him to have a bird view of pharmacy practice in Nigeria, which he articulated in his book, “*The Lizard Shape Model in Drug Distribution in Nigeria*”. This epic book has become an invaluable manual for drug manufacturers, drug importers, wholesalers,

distributors, retail and hospital pharmacies, etc. As the chairman, Governing Council of Pharmacy Council of Nigeria, the Council provided policy direction which revamped pharmacy education and raised the standard of Accreditation of Faculty of Pharmaceutical Sciences in universities. He also ensured that the PCN ACT 31 OF 2022 was signed into law replacing PCN Decree 91 of 1992. His quest for improved quality of healthcare in Nigeria is profound. As Pro-Chancellor and Chairman of the Governing Council of Igbinedion University, the Okada Council has set in motion a policy direction aimed in the transformation of Igbinedion University Teaching Hospital into a centre of medical innovation, research and quality patient care that will not only serve Edo State but also impact lives across Nigeria and beyond.

Finally, as we celebrate a pharmacist of distinction, regulator, administrator, industrialist and astute academic at 70 years, my mentor and role model, may God continue to grant you good health and strength to continue in your distinguish service to humanity.

Signed

Pharm. Yoosu Tersoo Jonathan (*FPCPharm*)

Assistant Director Pharmaceutical Science.

Federal Medical Centre, Makurdi.

Pharm. Yoosu Tersoo Jonathan is a pharmacist at the Federal Medical Centre, Makurdi. Trained at ABU, Zaria, he was a student of Professor Mora during his undergraduate degree program. He later served as Chairman of the “Pharmaceutical Society of Nigeria (Benue State Branch)”. He remains in close contact with Professor Mora.

Prof. Ahmed Tijjani Mora, OON, mni

Prof. AT Mora: A Teacher, Mentor and Beacon of Nigerian Pharmacy My aim is to share my personal reflections on the remarkable teaching, leadership and mentorship of Prof. Ahmed Tijjani Mora from the perspective of his former student in Zaria. Prof. Mora (also known as Alhaji AT Mora), is a prominent leader and distinguished figure in the Nigerian pharmacy profession, who served as Chairman of the Pharmacists Council of Nigeria, having previously held the position of Registrar.

I had the distinct privilege of being Prof. AT Mora's student at Ahmadu Bello University, Zaria in the 1990s, where he taught us Pharmacy Management courses as well as aspects of pharmacy law and ethics. Even then, his reputation as an accomplished pharmacist, technocrat, visionary leader and committed educator preceded him. At that time, Prof. Mora was also the Managing/Director of Zaria Pharmaceutical Company Ltd, the manufacturer of the renowned *Zarinject*[®] syringe. Beyond his corporate and professional responsibilities, he was a dedicated patron of both the Pharmaceutical Association of Nigerian Students (PANS) and the Pharmaceutical Muslim Students Society (PMSS), which I was a member. His engagement with students went far beyond formal teaching; he was a mentor, guide and role model whose guidance inspired many of us to aspire to excellence in pharmacy and life.

Prof. Mora's teaching style was marked by clarity, depth, professionalism and an emphasis on practical applications. He was not just an academic, but a thinker, a strategist and a technocrat who instilled in his students the importance of hard work, ethical practice and the pursuit of excellence. Even after nearly three decades, I vividly recall the lessons he taught me – lessons not only in pharmacy but in leadership, vision and

character.

AT Mora's generosity and commitment to the growth and advancement of pharmacy education and the profession in Nigeria, across Africa and globally have been truly legendary. During our time, he consistently supported the Faculty of Pharmacy in Zaria and student initiatives, both morally and financially. I personally benefited from his support during the launch of the Kano/Jigawa Pharmaceutical Students Association (KAJIPSA) magazine in 1998/99, when he generously donated N10,000, a substantial sum at the time. He also contributed numerous educational materials to the Faculty, playing a tangible role in nurturing the next generation of pharmacists.

I must say that Prof. Mora's career trajectory exemplifies dedication to both professional practice, education and national service. After his tenure in the corporate sector, he transitioned into regulatory affairs, shaping the Pharmacists Council of Nigeria as Registrar and later, as Chairman, ensuring that pharmacy practice in Nigeria adheres to the highest standards. He also contributed to academia as the founding Dean of Pharmacy at Kaduna State University, demonstrating his unwavering commitment to teaching and mentorship. In all my interactions with Prof. Mora, whether as his student, mentee or his junior colleague, I have consistently been struck by his humility, kindness and approachable nature. He embodies the qualities of a true visionary leader, one who not only achieves excellence personally but also elevates those around him.

I am proud to have been shaped, guided and inspired by Prof. Ahmed Tijjani Mora, a man whose legacy will continue to influence generations of pharmacists in Nigeria and beyond.

In conclusion, Prof. Ahmed Tijjani Mora (*Wakilin Maganin Zazzau*) has profoundly shaped my journey as a pharmacist and professor of pharmacy, not only through his teaching and mentorship but also

through his visionary leadership, generosity and unwavering commitment to the profession.

It is an honor to celebrate his remarkable legacy and accomplishments. Happy 70th Birthday, Sir. May Allah *Azza Wa Jallah* continue to elevate him, bless him with abundant health and wealth and grant him many more years of success and fulfillment, *ameen*.

Ahmed Awaisu, Ph.D,
MPharm (Clinical)
B.Pharm. Professor &
Head of Department
Department of Clinical Pharmacy and Practice
College of Pharmacy
Building I – 06, Second Floor Room SA2.12
Qatar University, P.O.Box 2713
Doha, Qatar.

Professor Ahmed Awaisu is a Nigerian pharmacist based in Doha, Qatar. A graduate of ABU, Zaria, and former student of Professor Mora during his undergraduate degree program, he is a highly talented and purposely-driven scholar whose innovations and patents have placed Nigeria on the Qatari and world pharmaceutical platform. He maintains regular contact with his mentor.

**FOUNDATIONS OF EXCELLENCE: THE FORMATIVE
YEARS OF PROFESSOR AHMED TIJJANI MORA DURING
THE 1960S, TUDUN WADA, ZARIA**

Prof. Aminu Salihu Mikailu

Department of Accounting

Usmanu Danfodiyo University, Sokoto

The landscape of a person's character is often mapped in the fertile and unsuspecting plains of youth. For Professor Ahmed Tijjani Mora, this early terrain was the conservative yet vibrant city of Zaria and its environs during the 1960s, a period of post-colonial transformation in Nigeria, teeming with promise and social vitality. His early life, deeply intertwined with family, friendship and rigorous education laid the essential groundwork for the renowned scholar and respected individual he would later become.

My story of a strong and enduring relationship with Professor Mora began in 1962, during a period when life followed the rhythm of communal relations and family responsibility. I had traveled from Zaria City, where I was born, to Tudun Wada to reside with my uncle, Mallam Salihu Mikailu, who served as the *Alkali* (a Judge in the Islamic Court). Our home, known colloquially as *Gidan Alkali*, stood on Siliki Road, later renamed Baba Ahmed Road. This placed us near Mora Road, home of the late Dr. Abdurrahman Mora, a respected figure and father of Professor Ahmed Mora. Our connection was thus shaped not by chance but by the close-knit geography of Tudun Wada, Zaria.

Within the compound of the late Dr. Abdurrahman Mora were many children of my age. Yet, my strongest affinity was with two brothers in particular: Ahmed and Aminu. During that memorable period, our lives were intertwined through shared experiences. We frequently visited each other's homes and our days were filled with childhood play that seamlessly blended into the rich cultural and intellectual atmosphere of our homes.

Our formal education began together in 1963 at LEA Primary School, Tudun Wada (now Baba Ahmed Primary School). Established in 1937, the school was a beacon of learning in the region. Even then, it was known for its excellence, quality teachers and well, equipped teaching and sporting facilities. Parents from all over the Local Government Area, Sabon Gari, Zaria City, Wusasa and Muchiya, considered it worthwhile to send their children to study there, often traveling long distances, sometimes on foot, in pursuit of quality education. We remained classmates until the beginning of Primary Six in 1968, when my uncle's posting to Rigachikun necessitated my transfer. Despite the move, the strength of our early friendship continued to grow and endures to this day. Even at this tender age, the contours of Professor Mora's personality were already evident. Some defining traits include:

Home and Heart Discipline: Ahmed Mora carried the strict ethical discipline of home into the classroom. It was evident in his immaculate cleanliness, unwavering respect for teachers and classmates and calm, focused demeanor, all remarkably mature for his age. He embodied concentration and sheer will, qualities both natural and nurtured.

Academic and Sports Excellence: Academically, he was among the top performers throughout our primary school days, with special aptitude in English, Geography, Islamic Religious Knowledge and Arithmetic. Yet, he was no bookworm; he balanced studies with active participation in sports, displaying excitement and team spirit on the field.

An Inquiring Mind and Leadership: A telling sign of his future potential emerged in Primary Five. Classmates Ibrahim Sambo, Ismaila Datti and Dalhatu Yusuf, formed an unofficial study group, himself and others formed a rival one. During mid-morning breaks, they focused not on play but studying current affairs and dissecting challenging lessons. This initiative, undertaken without instruction

from teachers, reflected his curiosity, scholarly zeal and early leadership abilities.

In conclusion, Professor Ahmed Mora's brilliance is no accident of later life; it is the product of the nurturing he received from home and his early school environment. The seeds of hard work, humility, simplicity, honesty and benevolence were sown early, and his respect for authority and compassion for the less privileged were evident even in the sun-baked playgrounds and classrooms of LEA Tudun Wada. It was here, in these formative years, that the diligent, friendly and brilliantly promising young boy Ahmed Mora began his journey toward the distinguished scholar and global pharmacy icon we know today.

Professor Aminu Salihu Mika'ilu is a distinguished academic and one of the few Nigerians to have served as Vice-Chancellor in three Nigerian universities. He and Professor Mora enrolled together at L.E.A. Primary School, Tudun Wada, Zaria, in January 1963 and were childhood neighbours. Together with Sa'idu Shafi'i, they were the first three pupils from the school admitted in 1963 to graduate with bachelor's degrees from ABU, Zaria, in 1978—in Accounting, Agriculture, and Pharmacy respectively. Professor Mika'ilu is currently with “Usmanu Danfodiyo University, Sokoto (UDUS)”, where he previously served as Vice-Chancellor.

A TRIBUTE TO A TITAN OF PHARMACY:
CELEBRATING PROFESSOR AHMED TIJJANI MORA,
OON, mni

Professor Isa Odidi

It is with immense privilege and profound respect that I honour a true giant of our profession, **Professor Ahmed Tijjani Mora, OON, mni**, on the auspicious occasion of his 70th birthday.

To write about Professor Mora is to attempt to describe the ocean: vast, deep and powerful. For many of us in pharmacy, both in Nigeria and beyond, Professor Mora has been that ocean, constant, sustaining and inspiring.

A Career Woven in Excellence

The tapestry of Professor Mora's career is uniquely threaded with **excellence, integrity and vision** across the four sacred pillars of our profession: **Education, Industry, Public Service and Regulation**. He remains the first Nigerian pharmacy practitioner to reach the pinnacle in all four domains, a feat that sets him apart as an icon.

His academic foundation was laid at **Barewa College, Zaria** followed by a **Bachelor of Pharmacy** from **Ahmadu Bello University (ABU), Zaria** in 1978. He went on to earn an **MBA (Pharmaceutical Marketing)** from ABU and a **Ph.D. in Management Studies** from Usmanu Danfodiyo University, Sokoto, with a pioneering thesis on "*Drug Distribution in Nigeria.*" His intellectual journey further took him to the **National Institute for Policy and Strategic Studies (NIPSS), Kuru**, where he graduated from the Senior Executive Course (SEC 32), earning the prestigious designation ***mni***.

Pillar of Education and Academia

As an academic, Professor Mora did not merely teach, he built institutions and inspired generations. His lecturing career began in 1986 at ABU and spans **over 36 years of continuous pro bono service**, a testament to his unmatched generosity. He was the **Foundation Dean of the Faculty of Pharmaceutical Sciences at**

Kaduna State University (KASU) and pioneer Head of the Department of Clinical Pharmacy and Pharmacy Management. Later, he made history as the **first Professor of Clinical Pharmacy and Pharmacy Practice at Igbinedion University, Okada.** Beyond teaching, his influence extended to governance, serving on the Governing Councils of **Ahmadu Bello University and Igbinedion University**, thereby shaping policies that strengthened pharmacy education nationwide.

Architect of Regulatory Reform

As **Registrar and CEO of the Pharmacists Council of Nigeria (PCN) (2003–2012)**, Professor Mora transformed pharmacy regulation in Nigeria. He decentralized the Council's operations, expanding its presence from just **4 offices to 44 nationwide**, thereby ensuring effective oversight of practice. In **June 2020**, upon his inauguration as **Chairman of the PCN Governing Council**, he vowed to confront pressing issues, such as chaotic drug distribution and inadequate internship placements for young pharmacists. His leadership culminated in the enactment of the **Pharmacy Council of Nigeria Act, 2022**, which today provides the Council with the legal framework to better regulate all stakeholders in the pharmaceutical value chain.

Industry Leader and Public Servant

Professor Mora's impact extended to industry and governance. He served as **MD/CEO of Zaria Pharmaceutical Company Ltd. (manufacturers of ZARINJECT syringes) and Zazzau Pharmaceutical Industries Ltd.** In public service, he was the **pioneer Director of Pharmaceutical Services, Kaduna State Ministry of Health** and also managed the **ADB/ADF-Assisted Kaduna Health Rehabilitation Project**, bringing both vision and discipline to healthcare reform.

Mentor and Unifying Figure

Beyond titles and positions, Professor Mora's legacy lies in his **mentorship and community building.** As **National President of the Ahmadu Bello University Alumni Association (2015–2021)** and **Foundation National Chairman of the ABU Pharmacy**

Alumni Association (ABUPAA), he tirelessly worked to strengthen bonds within the profession.

For many of us, myself included, he was more than a professor or superior. He was a guide. He saw potential long before we did, pushed us to grow with firmness and fatherly compassion and opened his doors for counsel, encouragement, and insight.

Honours and Recognition

His immense contributions have been acknowledged through the **National Honour of Officer of the Order of the Niger (OON)**, alongside revered traditional titles, such as *Wakilin Maganin Zazzau* and *K'ayayen Sarkin Musulmi* from the Sokoto Sultanate Council.

Conclusion

Professor Ahmed Tijjani Mora, your life has been a gift to pharmacy. You have been our **teacher, pioneer, regulator, leader and mentor**. On this milestone 70th birthday, we celebrate not just your years, but the monumental impact of your work, etched permanently into the fabric of Nigeria's healthcare system. As you mark this day, may you look back with pride, forward with joy and continue in good health to inspire generations yet to come.

Thank you for your **service**, your **leadership** and your **mentorship**.

Happy 70th Birthday, Sir!

With the highest esteem,

Professor Isa Odidi

Professor Isa Odidi is a Toronto-based Nigerian pharmacist who trained at the Faculty of Pharmaceutical Sciences, Ahmadu Bello University (ABU), Zaria, graduating in 1979, just one year after Professor Mora. The two have remained close friends ever since. Professor Odidi obtained his PhD from the University of London in Pharmaceutics and Pharmaceutical Technology. He is an internationally acclaimed pharmaceutical manufacturer and the founder of "Intellipharmaeconomics International Inc", Canada, a company quoted on the New York Stock Exchange (NYSE). He also owns pharmaceutical facilities in China and is currently developing another in Oman, in the Arabian Peninsula.

**70TH BIRTHDAY TRIBUTE TO A BELOVED TEACHER
PROF. A. T. MORA, *OON, mni* (WAKILIN MAGANIN
ZAZZAU)**

It is with immense joy and gratitude that I join millions around the world in celebrating a truly remarkable milestone, the 70th birthday of a teacher, mentor, adviser and inspirer, Pharm. (Prof.) A. T. Mora. You have touched not only my life but also the lives of countless others with your kindness, wisdom and grace.

Prof. Mora, you have been more than just an educator. You have been a mentor, a guide, a cheerleader and a friend. With your warm smile and welcoming spirit, you made every encounter, even in the classroom, feel like a safe space, where learning flourished and every student felt seen and valued. I cannot forget those free handouts you gave us. To you, it may have seemed insignificant, but to some of us from humble backgrounds, it meant the world.

You taught not only from books but from the heart. Your lessons extended far beyond the curriculum. You taught us patience, respect, resilience and the power of a kind word. You taught us love. Thank you, sir. You believed in us, even when we doubted ourselves under the intimidating weight of pharmacy as a course. That belief made all the difference.

Over the years, your influence has spread like ripples, shaping minds, building character and leaving an imprint that time cannot erase. Former students still speak your name with fondness and I remain one of them. Even now, as your junior colleague, I continue to admire your calm wisdom, advice and encouragement. You always want your students to succeed. You celebrate us and even those who now occupy national positions of authority, you treat with the utmost respect, sometimes to our embarrassment. Personally, I am uplifted by your ever, encouraging nature.

Seventy years of life filled with dedication, laughter and compassion; what an incredible legacy you continue to build. Today, I say thank you. Thank you for your years of service, your heartfelt care and the countless lives you have changed. May this special birthday shine as brightly as the light you have shared with me and so many others.

Happy 70th Birthday, Your Highness, Prof. A. T. Mora! You are cherished more than words can express.

Prof. Martins Emeje, *PhD, FAAS, FNAPharm, FPSN*

Director, General/Chief Executive

**Nigeria Natural Medicine Development Agency (NNMDA),
Nigeria**

Professor Martins Emeje is a pharmacist and a product of the Faculty of Pharmaceutical Sciences, ABU, Zaria. He was a student of Professor Mora at undergraduate level. An extremely hardworking, energetic, and brilliant scholar, Professor Emeje obtained his MSc and PhD degrees from first-generation Nigerian universities, where he consistently distinguished himself academically, just as he did at ABU, Zaria. He is currently the Director-General and Chief Executive Officer of the “Nigerian Natural Medicine Development Agency (NNMDA)”, Victoria Island, Lagos.

**OPENING REMARKS
BY THE CHIEF HOST
PROF. MOHAMMED GARBA MAGAJI
DEAN, FACULTY OF PHARMACEUTICAL SCIENCES
AT THE 2ND NATIONAL SYMPOSIUM OF THE
PHARMACEUTICAL ASSOCIATION OF NIGERIAN
STUDENTS (PANS),
IN HONOUR OF PROFESSOR AHMED TIJJANI MORA,
OON, mni
ON THE THEME:
“SHAPING THE FUTURE OF PHARMACY: CONNECTING
KNOWLEDGE, INNOVATION AND OPPORTUNITIES”**

The Registrar, Pharmacy Council of Nigeria (PCN);
The Chairman of the Occasion;
Our distinguished guest lecturers;
Respected colleagues;
Honoured guests;
And my dear students—good morning.

It is my singular honour, as Dean of the Faculty of Pharmaceutical Sciences and Chief Host of this year's National Symposium, to warmly welcome you all to this gathering on the profound theme:

“Shaping the Future of Pharmacy: Connecting Knowledge, Innovation and Opportunities.”

Today, we are not only here to exchange forward, looking ideas...

Not only to nurture the next generation of pharmacy innovators...

But also, to celebrate and pay tribute to a towering figure in our profession:

Professor Ahmed Tijjani Mora, *OON, mni*

A man whose life and career embody the seamless blend of knowledge, innovation, service and generosity.

Professor Mora's story is one of impact, one of vision and one of selflessness:

As a distinguished academic and public servant, he has served in some of the highest leadership positions in our profession: Registrar and CEO of the Pharmacists Council of Nigeria, where he transformed pharmacy regulation; Chairman of the Governing Council of the PCN; Foundation Dean of the Faculty of Pharmaceutical Sciences, Kaduna State University; and today, Pro Chancellor and Chairman of the Governing Council of Igbinedion University, Okada, the first private university in Nigeria.

As a legendary teacher, he has taught *pro bono* for more than three decades in this very faculty. He shared wisdom freely, inspired countless students and shaped careers, never seeking anything in return.

As a proud alumnus and leader, he served as President of the Ahmadu Bello University Alumni Association and today chairs the Board of Trustees of the ABU Pharmacy Alumni Association. (ABUPAA) Under his stewardship, our alumni community flourished.

His visionary leadership inspired collective action; alumni efforts, guided by him, led to the construction of a 250, seater twin lecture theatre proof that when vision meets commitment, great things are possible. And as a national icon, he has been honored with the *Officer of the Order of the Niger (OON)* and conferred with distinguished traditional titles, among them *Wakilin Maganin Zazzau* and *Kayayen Sarkin Musulmi*.

Distinguished ladies and gentlemen, these achievements represent more than personal milestones.

They embody the very heart of this symposium:

- **Knowledge:** sustained by scholarship.
- **Innovation:** driven by leadership and reforms.

- **Opportunities:** created for students, alumni, and the profession at large.

Prof. Mora, your legacy is profound.

You have built not just structures, but people.

Not just classrooms, but generations.

Not just history, but a future.

Fellow students and colleagues, as we deliberate today, may we be inspired by his example. Let us connect our own scholarship... our creativity... and our innovation... to shape a brighter more impactful future for pharmacy in Nigeria and beyond.

On behalf of us all, Professor Mora, we say thank you.

Thank you for your leadership.

Thank you for your generosity.

And thank you for your unrelenting service to this noble profession.

Thank you all.

And may this symposium inspire, empower and energize us for the great task ahead.

Professor Mohammed Garba Magaji is the current Dean of the Faculty of Pharmaceutical Sciences, Ahmadu Bello University (ABU), Zaria, and a Professor of Pharmacology. He was an undergraduate student of Professor Ahmed Tijjani Mora in the Faculty of Pharmaceutical Sciences, ABU, Zaria. Professor Magaji holds Professor Mora in very high esteem, regarding him as a distinguished academic, mentor, and lecturer who has served the faculty selflessly for thirty-nine (39) consecutive years, *pro bono*, since 1986.

**PROFESSOR AHMED TIJJANI MORA, OON, mni,
(Wakilin Maganin Zazzau, Kayayen Sarkin Musulmi): –
The Man I Know**

It was on **17th January 1970** that Prof. Mora and I began our secondary school journey at the famous **Barewa College, Zaria** (then known as Government College, Zaria). Our sojourn at Barewa lasted four and a half years, ending successfully in **June 1974**.

Ahmed, as we fondly called him, was in Form 1B with school number **B.2511** and a member of **Nagwamatse House**. He was undoubtedly among the brightest in our stream, often emerging top of the class after most end-of-session examinations. Our set comprised students drawn largely from the then 14 Local Government Areas of the **North, Central State** (later Kaduna State, comprising Zaria and Katsina Provinces), as well as others from the then North-Western, North-Eastern, Kano, Kwara, Benue and Plateau States. We even had two international classmates—**Michael Okpo** from the South, Eastern State and **Sanjoy Hore** from India (the only day student, as his father lectured at Ahmadu Bello University). The rest of us were boarders.

Barewa College: The Foundation of Brotherhood

Barewa College, which gave us the foundation for much of our success in life, has an illustrious history. Established in 1921 as **Katsina College**, it was the first secondary school founded by the colonial government in Northern Nigeria. Over the years, it bore different names, **Katsina Higher College, Zaria Secondary School and Government College Zaria** before finally being renamed **Barewa College** in 1972, when we were in Form 3.

When we were admitted in 1970, about **145 students** joined (though some never turned up), and divided into four streams, 1A, 1B, 1C and 1D. It was the largest admission at the time, reflecting the growing demand for education. The school then had both a **secondary** and a **Higher School Certificate (HSC) section**, which admitted

outstanding students to prepare for GCE London exams before proceeding to university. By 1975, the school's population was **1,162 students**; today (2025), it has grown to **2,600 students**.

All students were boarders except for Sanjoy. The seven hostels, **Lugard, Clifford, Dan Hausa** (South wing) and **Malam Smith, Nagwamatse, Bieneman, Mort** (North wing), were separated by the dining hall, clinic and tailor's bay. These houses were named after senior colonial officials, who shaped the Northern Region's history.

Our school life was regimented yet fulfilling, designed to instill **discipline, education, nationalism and physical/mental development**. We were treated equally, irrespective of social background. Uniforms varied: white shirt and shorts for class, brown shirt and blue shorts after school and elegant white kaftan and trousers for weekend town visits. Meals were communal and the timetable was strict: breakfast after morning lessons, siesta after lunch, sports in the evening, dinner and night prep before "lights out" at 10 pm.

Remarkably, all essentials, uniforms, books, beddings and food, were provided by the school. Fees were only **£3 annually**, with parents paying for a maximum of three children in school. Additional benefits included free soap and transport allowances for holidays. For many, these provisions outweighed the modest fees. Such an environment fostered **brotherhood**, explaining the lifelong bond we Barewa alumni cherish.

The Mora Family Connection

Prof. Ahmed Mora hailed from **Tudun Wada, Zaria**, barely a kilometre from the college. His family was well known across Zazzau Emirate; his father having taught many of the early educated elites in Zaria Province. Indeed, **11 members of the Mora family** passed through Barewa from 1933 onward. His elder brother, **Dr. Saifuddeen**, studied medicine, while another brother, **Aminu**,

became a banker. His younger brother, **Abbas**, entered Barewa after our time. Coming from such an enlightened family, the Moras enjoyed certain educational and social advantages. Yet, Ahmed never showed off; rather, he blended humility with brilliance. My younger brother and I often accompanied him home on Sundays, where we enjoyed his family's warmth and generosity.

I also share another bond with Ahmed: our fathers, **Malam Hassan Rafindadi** (my father) and **Malam Abdulrahman Mora** (his father), were contemporaries at Katsina College in the 1930s and lifelong friends. Thus, when we arrived at Barewa, his family became our guardians, a kindness for which we remain grateful.

The Barewa Old Boys' Association (BOBA) Class of 1970

In 1994, our set formally established the **BOBA Class Group of 1970**, creating a platform for fellowship and brotherhood. This initiative was driven by a few, notably **Prof. Ahmed Mora, Zakari Sada, Germane Kemba Ya'u** and **Ahmed Sambo**, with immense support from eminent members, such as **His Eminence, the Sultan of Sokoto**, and **His Royal Highness, the Emir of Fika**.

Prof. Mora served as the **pioneer Chairman** of our class group from 1994 until 2012, a remarkable **18 years**, before resigning following his election as **Secretary, General of the National BOBA**. His leadership cemented our bonds, ensured regular meetings and provided welfare structures for members, including health and financial support. His organisational skills were exemplary, sourcing funds and logistics for gatherings across the country. Under his leadership, our class also contributed to significant projects, notably the **Sultan Muhammad Sa'ad Abubakar Assembly Hall** at Barewa, commissioned in 2010 and now a landmark of the college.

Of the **143 students** admitted in 1970, about **80 are still alive** as of January 2025. 51 have passed away and the whereabouts of 9 remain unknown.

Professional Journey

Beyond Barewa, I have keenly followed Prof. Mora's illustrious career across **pharmaceutical, academic, regulatory and traditional spheres**:

- He began in the pharmaceutical industry, gaining deep insight into the drug distribution chain.
- He rose to **Chief Pharmacist** at the Kaduna State Ministry of Health/Health Management Board.
- He became the **pioneer CEO of ZPC Ltd (manufacturer of the ZARINJECT brand of disposable syringes)**, producing high, quality disposable syringes that gained national and West African acceptance.
- He served as **Registrar/CEO of the Pharmacists Council of Nigeria (PCN)**, later becoming **Chairman, Governing Council** of the same body (now Pharmacy Council of Nigeria).
- Academically, he holds a **B.Pharm, MBA, and PhD**, lectured at ABU *pro bono* and was the **pioneer Dean** of the Faculty of Pharmaceutical Sciences at Kaduna State University. He is also **Pro-Chancellor and Chairman of Council at Igbinedion University, Okada**.
- Traditionally, he was honoured as **Wakilin Maganin Zazzau** by the Emir of Zazzau and as **Kayayen Sarkin Musulmi** by the Sultan of Sokoto.

Conclusion

Prof. Mora is undoubtedly one of the most accomplished members of the **BOBA Class of 1970**. His brilliance, leadership, humility and devotion to service make him a shining star of our generation. We are immensely proud of him and continue to pray for greater successes in his life.

Professor Abdulmumini Hassan Rafindadi (B.2489), *MBBS, FMCPath, FICS, FNAMed*

Professor of Pathology

Department of Pathology, Faculty of Basic Clinical Sciences

College of Health Sciences, Ahmadu Bello University, Zaria

Consultant Pathologist, ABU Teaching Hospital, Zaria

Professor Abdulmumini Hassan Rafindadi is a Professor of Pathology. He and Professor Mora were classmates at Barewa College, Zaria (1970–1974), and among the pioneer students of the one-year experimental ‘A’ Level programme at the School of Basic Studies (SBS), ABU, from June 1974 to June 1975. They studied Mathematics with Physics, Chemistry, and Biology and successfully completed the programme in 1975. Professor Rafindadi has had a distinguished academic and medical career, serving as Head of Department and Dean in the Faculty of Medicine, ABU, Zaria, before becoming the Chief Medical Director of Ahmadu Bello University Teaching Hospital (ABUTH), Shika. He later served with distinction as the first Vice-Chancellor of the Federal University, Lokoja (FUL). Notably, the late Alhaji Hassan Rafindadi (*Sarkin Yakin Katsina*), his father, and the late Mallam (Dr.) Abdurrahman Mora, the father of Professor Mora, were contemporaries and close friends at Katsina College (now Barewa College, Zaria) in the 1930s.

A TRIBUTE TO PROFESSOR AHMED TIJJANI MORA (ATM), OON, mni

Professor Ahmed Tijjani Mora, often called ATM, dispenses not cash, but knowledge, a contrast to the Automated Teller Machine, yet equally valuable. He is a quintessential administrator, a pharmacist par excellence and an astute academician of high repute. A man who cherishes merit and standards, he is the kind of personality from whom you always gain and add to your knowledge after any meeting or discussion.

Professor Mora is a God-gifted, energetic individual, constantly on the move, a key secret to his many successes and achievements. He is a dedicated mentor who ensures his mentees succeed. He is a great achiever both academically and administratively. As a teacher, he has nurtured many professors and notable figures in Nigeria and beyond. He is the envy of his critics, particularly for his transformative leadership as Registrar of the Pharmacists Council of Nigeria (PCN). He modernized the Council's operations and infrastructure and the PCN will never forget him as its best performing Registrar. It was during his tenure that the PCN Head Office and Registry in Idu, FCT, initiated by the then Chairman, Dr. Fred Adenika, in 2000, was commissioned in 2005.

Professor ATM provided exemplary leadership as Chairman of the Committee of Registrars of Health Professionals under the Federal Ministry of Health. He executed this role with passion and stylistic alacrity. My brother and *Oga* is always confident and optimistic in executing any task. As the Chair, he successfully led the Committee on numerous retreats within and outside the shores of Nigeria.

An accomplished author, Professor Mora's most popular work is *"The Lizard, Shaped Model of Drug Distribution in Nigeria."* He was also the principal author of the book *"D.S. Wali: An Icon of Community Pharmacy Practice in Nigeria."*

In a rare privilege, Professor Mora later became the Chairman of the PCN Governing Council, where he once again performed marvelously. Currently, he serves as a board member of the Institute of Human Virology and as the Pro-Chancellor and Chairman of Council for the highly prestigious Igbinedion University, Okada, the first government, approved and registered private university in Nigeria.

It is worthy of note that, to date, only Professor A.T. Mora and I have had the privilege of attending the Senior Executive Course (SEC) at the National Institute for Policy and Strategic Studies (NIPSS), Kuru, while serving as Registrars.

I could continue this tribute without end. I have presented my brother, Professor A.T. Mora: a senior citizen, Member of the National Institute (*mni*), seasoned administrator, a quintessential academician and a pharmacist par excellence, who today celebrates his septuagenarian years.

I wish him many more useful years ahead in good health and prosperity. May Allah (SWT) continue to be our Guide and bless the celebrant and us abundantly, *amin*.

Professor Abdulmumini Ibrahim, DSc, *mni*

Professor Abdulmumini Ibrahim, *mni*, is a medical practitioner who served as Registrar and Chief Executive Officer of the “Medical and Dental Council of Nigeria (MDCN)”. He was Registrar at the same time Professor Mora served as Registrar and CEO of the Pharmacists Council of Nigeria (PCN). A strong bond of friendship and mutual respect has existed between them ever since. Professor Ibrahim succeeded Professor Mora as Chairman of the “Committee of Registrars of Health Professional Bodies (CRHP)” after the latter completed his tenure. Both attended several Annual Scientific Conferences and Expositions of “NAPPSA” in various cities across the United States.

**A TRIBUTE IN HONOR OF MY FRIEND AND BROTHER,
PROFESSOR AHMED TIJJANI MORA, OON, mni**

Professor Ahmed Tijjani Mora — *B.Sc. (Pharmacy), MBA, FPCPharm, Ph.D., FPSN, FNIM, FIHMN, FICEN, OON, mni; K'ayayen Sarkin Musulmi (Wakilin Maganin Zazzau) (Iyan Piriga)* — is a colossus in pharmacy, whose impact on pharmacy education, administration and regulation transcends national boundaries, a distinction that truly earns him the title of a “Global Pharmaceutical Icon.”

Professor Mora's tenure as Registrar of the Pharmacists Council of Nigeria (PCN) witnessed unprecedented innovations and transformative developments across the academic, industrial, hospital and regulatory subsectors of the profession.

I first encountered him during my time as Pioneer Dean of the Faculty of Pharmaceutical Sciences at Nnamdi Azikiwe University, Awka, Anambra State. His visionary leadership empowered us to:

- Host the Mandatory Continuing Professional Development (MCPD) Programme.
- Launch an Internship Programme at the Faculty, providing early career mentorship for young pharmacists in academia.
- Facilitate the training of foreign, trained pharmacists.
- Establish a Model Pharmacy.

The seeds Prof. Mora planted in our then young Faculty have since grown into a vast repository of human capacity, serving not only Nigeria but the global community. His support also garnered respect and recognition for me within my university, a factor that contributed to my eventual appointments as Deputy Vice Chancellor (Academic) and later, Vice Chancellor of that great citadel of learning.

On the global stage, Prof. Mora's tenure as Registrar of PCN was marked by the establishment of the Nigerian Association of

Pharmacists and Pharmaceutical Scientists in the Americas (NAPPSA). From 2007 to date, he has consistently attended its annual scientific conferences in the USA, often as a key resource person or participant. After completing his successful tenure at PCN, Prof. Mora's passion for pharmacy education took him to Kaduna State University, where he again served as Pioneer Dean of the Faculty of Pharmaceutical Sciences. From this position, he made significant contributions to the advancement of pharmacy management and administration within the Nigerian university system and beyond.

As we celebrate this great pharmacy doyen and icon, who has attained the distinguished age of 70, I can only pray that Almighty God grants him long life, good health, peace, progress and continued intellectual endowment for the advancement of mankind.

Kudos and seventy hearty cheers to my dear friend and brother, Professor Ahmed Tijjani Mora.

Professor Charles Okechukwu Esimone, FAS, FPSN, FNAPharm
Immediate Past Vice Chancellor, Nnamdi Azikiwe University, Awka

Professor Charles Okechukwu Esimone is a Professor of Pharmacy and the Immediate Past Vice-Chancellor of Nnamdi Azikiwe University, Awka (NAU), Anambra State, Nigeria. He is a distinguished academic with proficiency in several European languages. Professor Esimone has known Professor Ahmed Tijjani Mora for many years; both have attended numerous professional pharmacy events together, during which they developed mutual confidence, respect, and professional camaraderie.

A. T. MORA: AN ICON OF SCHOLARSHIP ADMINISTRATION AND LEADERSHIP

It is indeed an honor and a privilege to be invited to contribute a tribute to the life and times of Professor Ahmed Tijjani Abdurrahman Mora on the occasion of his 70th birthday celebrations. My reflections are based on three memorable encounters I had with him.

The first was in 1988, shortly after I returned from Canada, where I had completed my doctoral studies and fellowship. Following the tragic death of my younger brother, Lieutenant Colonel Mustapha M. Jumare, in a ghastly motor accident, Professor Ahmed visited our family to console us. He extended his sympathy both to our extended family in Zaria and to Mustapha's immediate family in Kaduna. My first impression of him was that of an imposing personality underscored by humility and generosity. He shared deeply in our grief, demonstrating genuine sympathy and compassion throughout the condolence visits.

The second occasion was in 2010 at the National Institute for Policy and Strategic Studies (NIPSS), Kuru, Plateau State, where he was a participant in the Senior Executive Course (SEC) 32, running from February to November that year. The ten-month program was a rigorous multi-disciplinary and multi-professional training in policy formulation, implementation and evaluation aimed at nation-building. It culminated in the award of the prestigious *mni* certificate. The participants included senior military officers of colonel rank and above (and their equivalents in the Air Force and Navy), senior police officers from the rank of commissioner, directors in the federal civil service, permanent secretaries at state level, as well as senior technocrats from government agencies and corporate organizations. Professor Ahmed fit seamlessly into this distinguished group, bringing to bear his academic and professional expertise in pharmacy combined with his rich bureaucratic and technocratic experience in the federal civil service. It was through this interaction that I came to

appreciate how he easily traversed various disciplines in the humanities, despite being a trained scientist and pharmacist.

Within SEC 32, Group 6 elected him as their Chairman. He led them effectively in all training activities and study tours. These included daily syndicate discussions, lecture series, national tours to Kogi and Bayelsa States, a continental study tour to Burundi, a world study tour to Germany and an industrial tour of the Kaduna Petrochemical Refining Company. I later joined the program in April as a Study Group Director, taking over leadership from him, as my arrival had been delayed by the violent crises in Jos. In fact, the Director General of the Institute, Professor Danfulani, had to arrange a security escort for me to reach NIPSS safely. During the course, Ahmed undertook original research and presented a project titled ***“The Problem of Fake Drugs in Nigeria: A Case Study of the Operation of the National Agency for Food and Drug Administration and Control (NAFDAC).”*** I attended his viva voce and witnessed how the external examiner commended the quality of his work, recommending it for publication. It stood out as one of the best projects in SEC 32, earning recognition from fellow participants and study group directors alike. At the end of the course, his generosity was again demonstrated through thoughtful gifts and souvenirs he shared with colleagues and directors in appreciation of their support and collaboration.

The third significant encounter was related to his part-time PhD candidature in the Faculty of Management Sciences, Usmanu Danfodiyo University, Sokoto, where I worked. When he mentioned his doctoral studies to me in 2010 during our time at NIPSS, I followed up with his supervisors. Both spoke highly of his capacity to successfully complete the program. Encouraged by this, I attended one of his meetings with the supervisory team to discuss his progress and timelines. Alhamdulillah, in 2012 I received the joyful news that he had successfully defended his PhD thesis, entitled ***“Distribution Strategies in the Pharmaceutical Industry: A Case of Selected***

Pharmaceutical Companies in Nigeria.” The thesis, praised for its originality, was recommended for publication to inform policymakers and the public on how pharmaceutical companies distribute medicines and related products.

Another noteworthy aspect of his intellectual life is his commitment to scholarship demonstrated through his numerous publications in biography, pharmacy and related fields, which he generously distributed to individuals and libraries. His leadership roles extend beyond academia into professional and alumni associations, such as the Barewa Old Boys Association (BOBA) and the Ahmadu Bello University (ABU) Alumni Association. In addition, he holds esteemed traditional titles in the palaces of the Sultan of Sokoto and the Emir of Zazzau, where he contributes to both traditional and modern systems of leadership. Professor Ahmed Tijjani Mora is truly an erudite scholar, administrator and leader. I trust other contributors will add to these reflections on his remarkable life. May Allah continue to guide and protect him.

Professor Ibrahim M. Jumare
Department of History & International Studies
Usmanu Danfodiyo University, Sokoto, Nigeria

Professor Ibrahim Muhammad Jumare is a Professor of History at UDUS and elder brother of the late Lt. Col. Mustapha M. Jumare. Both the late Jumare and Professor Mora were among the founding members of ZEDA. Professor Jumare also served as Directing Staff at NIPSS during Professor Mora’s participation in SEC 32/2010, supervising Group 6, where Professor Mora served as Chairman. He provided significant mentorship during the programme.

A TRIBUTE TO A PILLAR OF PHARMACY EDUCATION AND REGULATION

Alhamdulillah!

It is a great honor for me to be part of this celebration.

As a health broadcast journalist of over two decades at the Nigerian Television Authority (NTA), I had the privilege of crossing paths with Prof. Ahmed Tijjani Mora during his tenure as Registrar of the Pharmacists Council of Nigeria (PCN).

I was a cub reporter then, but not too inexperienced to recognize a “reporter’s delight” among the numerous resource persons and experts we relied on to enrich our reports with knowledge. Covering the activities of the PCN during Prof. Mora’s tenure was indeed a delight for me and the rest of the crew. His humility, goodwill and hospitality ensured that his assignments never failed to receive coverage. Despite the distance from the NTA Headquarters at Area 11, Garki to Idu where the PCN Headquarters was located, he always ensured that we were assigned a vehicle and a camera crew.

While covering the Council’s activities, we journalists also became more knowledgeable about regulatory processes. We learned, under Prof. Mora’s watch, the difference between registered and unregistered premises. His relentless fight against quackery and his efforts at strengthening professionalism and safety left an indelible impression on us.

Prof. Mora’s contributions to pharmacy education are immeasurable, a true legacy of excellence. His commitment to nurturing the next generation of pharmacists has significantly shaped the profession.

Having risen through the ranks to Management cadre at the NTA, I later conceptualized and produced the health programme **Health Options** until I crossed over to the Nigerian Natural Medicine Development Agency. This move brought me closer to men and women of honor in Nigeria’s healthcare space. During this period, when Prof. Mora was later appointed Chairman of the Pharmacists Council of Nigeria, I had

the privilege of hosting him as a guest on **Health Options**, where he outlined strategies to reposition pharmacy practice in Nigeria.

Prof. Mora's legacy goes beyond his impressive resume. He has inspired countless individuals, myself included, to pursue careers in healthcare. Today, I am undergoing studies at the Nigerian College of Natural Medicine Technology, the training arm of the Nigerian Natural Medicine Development Agency.

I must stress the importance of accomplished professionals being role models for the next generation. Prof. A. T. Mora stands out as a clear example worthy of emulation. I recall when he led final year students of Igbinedion University, Okada, on an excursion to the Laboratory Complex of the Nigerian Natural Medicine Development Agency in Lagos. The bond between him and the students was striking and gave further credence to his qualification as a role model to both the present and future generations. His dedication to excellence has raised the bar for pharmacy education, administration and regulation, leaving an indelible mark on the profession.

As we celebrate Prof. Ahmed Tijjani Mora's remarkable career, we honor not only his achievements but also the countless lives he has touched. His work stands as a testament to the power of dedication, expertise and compassion in shaping the future of healthcare.

May the book continue to inspire generations of healthcare professionals to follow in his footsteps.

Rabi Abdallah

Health Broadcast Journalist/Natural Medicine Advocate

Hajiya Rabi Abdallah is a Nigerian journalist who gained prominence during her career with the "Nigerian Television Authority (NTA)", where she served notably as a health correspondent. During Professor Mora's tenure as Registrar and Chief Executive Officer of the "Pharmacists Council of Nigeria (PCN)", she interviewed him on several occasions. She also covered numerous PCN events, reporting them accurately and professionally.

MY TRIBUTE TO PROFESSOR A. T. MORA @ 70

A PILLAR OF INTEGRITY, VISION AND UNWAVERING PURPOSE

It is a true privilege to pay tribute to Professor A. T. Mora, *OON, mni*, a remarkable leader whose lasting influence on pharmacy education, administration and regulation has inspired countless individuals.

My connection with Professor Mora did not begin in the prestigious halls of a university or the professional corridors of pharmacy. It began on the shared grounds of Barewa College, Zaria. Although our professional paths diverged, I pursued a different field while he rose to prominence in pharmacy, the core values of character, discipline and integrity that we embraced during those formative years have continued to shape our lives. From this unique perspective, as both a lifelong brother and an admirer of his extraordinary journey, I am honored to share this tribute.

From the very beginning, it was clear that Professor Mora possessed immense discipline and quiet determination. His leadership was never about chasing the spotlight; it was rooted in genuine, selfless service. This unwavering commitment has been the hallmark of his career. I have watched with immense pride as he skillfully navigated the intricate world of pharmacy education and regulation, not just as a professional, but as a true reformer. His contributions to the field stand as a testament to his visionary leadership and have significantly raised the standards of pharmacy practice in Nigeria.

Beyond his well, documented accomplishments, one particular act highlights his character for me. I was deeply touched and filled with pride when I learned of his role in chairing the book committee that honored our beloved uncle, Pharm. Dahiru Sulaiman Wali, FPSN. By establishing the annual lecture series in Pharm. Wali's memory, Professor Mora did more than fulfill a duty, he celebrated a legacy. This gesture of grace and deep respect for his mentor's memory speaks volumes about his values. The initiative not only honors an extraordinary life but also emphasizes the importance of valuing our heritage and acknowledging the contributions of those who came before us.

Professor Mora's leadership in this project truly reflects his commitment to remembrance and education, creating a bridge between the past and present in the pharmacy field. He embodies the principle that real greatness is not just about personal achievements but also about celebrating and preserving the legacies of others. This is the man I have known since our school days, someone who builds institutions and uplifts those around him.

As we celebrate his 70th birthday, we are not merely marking a milestone; we are honoring a life devoted to integrity, hard work and service. Professor Mora, your journey from Barewa to becoming a cornerstone of the pharmacy profession is nothing short of inspiring.

In closing, I express my sincere gratitude to Professor A. T. Mora for his unwavering dedication, exceptional leadership and profound compassion for the profession. Let us all strive to uphold the values he represents and continue to build on the strong foundation he has established. May your life be filled with good health, happiness and peace.

Congratulations on your remarkable journey and here's to celebrating many more milestones in the years ahead!

Sincerely,

Salahudeen Na'ibi Sulaiman Wali, MSNA, BA, MSc, MBA
Barewa Old Boys' Class of '72
(Representative of Pharm. D. S. Wali, FPSN Family)

Salahuddeen Na'ibi Suleiman Wali attended Barewa College, Zaria, and was two years junior to Professor Mora. Their relationship became closer during Salahuddeen's service at the National Teachers' Institute (NTI), Kaduna. Their bond further strengthened when Professor Mora, as Chairman of the **"Pharmacist D. S. Wali, FPSN Book Project"**, spearheaded the public presentation of **"D. S. Wali: An Icon of Community Pharmacy Practice in Nigeria"** and the subsequent annual lectures in honour of the late Pharm. D. S. Wali (1935–2020). Salahuddeen Wali is a nephew of the late Pharm. Wali.

PROF. AHMED TIJJANI MORA, OON, mni

Dear Prof A. T Mora,

On this special occasion of your 70th birthday and the anticipated launch of your new book, it is with great honor and appreciation that I pay tribute to you, a distinguished figure whose influence has shaped the pharmaceutical landscape in Nigeria.

Your unwavering commitment to promoting harmony and collaboration within the industry is truly commendable. Through your roles as former Registrar of the Pharmacists Council of Nigeria and Chairman of BOT, Pharmacy Council of Nigeria, you have demonstrated a remarkable ability to reconcile differences and foster unity amongst various stakeholders.

As a mentor and a friend, your guidance and wisdom have been invaluable to me, both personally and professionally. Your scholarly contributions in the form of published works and books have not only showcased your profound expertise but have also served as a source of inspiration for many within the pharmaceutical community.

As we celebrate this significant milestone in your life and eagerly anticipate the unveiling of your new book, I extend my heartfelt congratulations and best wishes. May this next chapter be filled with continued success, good health and boundless joy. May your latest literary endeavor resonate with readers and serve as a testament to your enduring legacy in the field.

Prof, your dedication and vision have left an indelible mark on the pharmaceutical sector. It is a privilege to learn from someone of your caliber. Here's to a future brimming with accomplishments and continued influence in shaping the trajectory of our industry.

With the utmost respect and admiration,

Steve Ozioko,

Managing Director/CEO

New Health Pharmacy Group, Abuja.

Mr. Steve Ozioko is a graduate of the “University of Nigeria, Nsukka (UNN)”. He ventured into pharmaceutical wholesaling and has made significant contributions to the distribution and availability of pharmaceutical products, particularly in Nigeria’s Federal Capital Territory, Abuja. Mr Ozioko and Professor Mora have maintained a cordial friendship since Professor Mora’s tenure as Registrar of the PCN.

**TRIBUTE ON BEHALF OF THE FAMILY OF ALHAJI
ABUBAKAR ALHAJI
(SARDAUNAN SOKOTO)**

I am honored to stand on behalf of the families of the late Inna (*Matan Mallam*), Hajiya Rahina, and the *Sardauna* of Sokoto, Sir Alhaji Abubakar Alhaji, KBE, to express our deepest gratitude for the opportunity to pay tribute to Professor Ahmed Tijjani Mora, *nni*, on the remarkable occasion of his 70th birthday. As we celebrate this milestone, we are reminded of the profound impact that “*Uncle TJ*,” as he is affectionately known to both families, has had on our lives.

Uncle TJ's relationship with our family predates his marriage to our beloved Auntie Asiya Ladi Mora. He was more than just a family member by marriage, he was a son to *Inna*, our late grandmother.

To my father, *Uncle TJ* is a brother. Their bond is built on mutual respect, trust and deep understanding. For me personally, he has been a tower of strength and inspiration. For as long as I can remember, he has been a constant presence in my life, offering guidance, support and encouragement whenever I needed it. I recall with fondness his visit to me in Abidjan, Côte d'Ivoire, where I was working; I cherished every moment we spent together.

Despite his many professional responsibilities, *Uncle TJ* remained devoted to his family and extended family members. His visits to my marital homes in Lagos and Kano were always filled with warmth and love. Although he was friends with my late husband, he always made me his priority, ensuring that I felt loved and cared for.

Uncle TJ's influence extends far beyond his immediate family. He possesses a rare gift for connecting with people. Many have benefited from his generosity, kindness and guidance. His integrity and character have earned him widespread respect and admiration, reflected in the traditional titles conferred upon him: *Wakilin*

Maganin Zazzau in Zaria, *Iyan Piriga* in Lere and *Kayayen Sarkin Musulmi* in Sokoto.

As we celebrate his 70th birthday, we are reminded of the enduring importance of family, friendship and community. We are deeply grateful for the lessons he has taught us; the love he has shared with us and the unwavering support he has provided. We wish him many more years of good health, happiness and fulfillment and we pray that he continues to be a source of inspiration and strength to us all.

In conclusion, I extend, on behalf of our family, our heartfelt appreciation to *Uncle TJ* for being such an integral part of our lives. Your love, guidance and support have meant the world to us and we are forever grateful. We look forward to celebrating many more years of your remarkable life and enduring legacy.

Honourable Aisha Abubakar

**On Behalf of the Family of Alhaji Abubakar Alhaji, KBE
(*Sardaunan Sokoto*)**

The family of Alhaji Abubakar Alhaji (*Sardaunan Sokoto*) has a close and longstanding relationship with Professor Mora. “Hajiya Aisha Abubakar”, a former Honourable Minister of the Federal Republic of Nigeria, is the eldest daughter of the *Sardauna*, Hajiya Aisha wrote this tribute on behalf of her late maternal grandmother, “Hajiya Rahina (popularly known as “*Matan Mallam*”)”. Professor Mora’s second wife, Hajiya Asiya, is the younger sister of Hajiya Aisha’s mother, further strengthening the family ties.



(L-R) Arc. Hamza Ahmed Mora, Dr. Halima Inuwa Mora, *PhD* and Imran Ahmed Mora, *CFA* during the wedding ceremony of Hamza in December, 2025



Alh. Muhammadu Inuwa Mora (1940 - 2001) as the Chairman of the Governing Council of Yaba College of Technology. (1985-1993)



(R-L) His Eminence, Alhaji Muhammad Saad Abubakar, *CFR*, *mni* (B.2612), the Sultan of Sokoto: Prof. Ahmed Tijjani Mora, *mni* (B.2511) (middle) and Lt. Gen Abdurrahman Bello Dambazau, *CFR* (B.2479) all classmates in Barewa College Zaria at a BOBA function in the Muhammadu Dikko hall, Barewa College Zaria, sometimes in the mid 90's



Visit of the Chairman of the Petroleum Trust Fund (PTF) Major General Muhammadu Buhari *GCFR* to the Zaria Pharmaceutical Company (ZPC) Ltd, Zaria Kaduna State, on 20th April, 1997 (L-R) Standing The Managing Director and CEO Of ZPC, Pharm. Ahmed Tijjani Mora showing the Chairman of the PTF (at the centre of the picture) sample of the ZARINJECT brand of the disposable syringes while the Military Administrator of Kaduna state, Col. Hameed Ibrahim Ali looks admiration.



The National President of the Ahmadu Bello University (ABU) Alumnae Association Pharm. Ahmed Tijjani Mora, *FPSN, mni (Wakilin Maganin Zazzau)*, making a presentation of a plaque to the Deputy Secretary General (DSG) of the United Nations, Hajiya Amina J. Mohammed in her Office in the United Nations Headquarters, Manhattan, New York City when he led the members of the National Executive Committee (NEC) of the Association on a courtesy visit to the DSG on Thursday, 5th December, 2019



A Courtesy Visit on His Excellency President Muhammadu Buhari, *GCFR* Members of the Management of Graham Foggs Ltd, a Pharmaceutical Company on 23rd November, 2018 in the Presidential Villa, Aso Rock, Abuja

COPYRIGHT CLEARANCE STATEMENT

Reproduced Conference Papers Presented at the Annual Scientific Conferences of the Nigerian Association of Pharmacists and Pharmaceutical Scientists in the Americas (NAPPSA).

Some of the scholarly papers included in this book were originally presented by Professor Ahmed Tijjani Mora at various Annual Scientific Conferences and professional meetings, including those organized by the Nigerian Association of Pharmacists and Pharmaceutical Scientists in the Americas (NAPPSA).

These papers are reproduced in this publication as part of the intellectual and professional contributions of Prof. Ahmed Tijjani Mora to the advancement of pharmacy education, pharmaceutical policy, professional regulation, and healthcare development, both in Nigeria and internationally.

All such papers included in this volume were solely authored by Prof. Ahmed Tijjani Mora, unless otherwise indicated. The inclusion of these works in this book is intended to preserve the scholarly record of the author's contributions and to make them more widely accessible to researchers, policymakers, students, and members of the pharmacy profession.

To the best of the author's knowledge, the reproduction of these papers in this book does not infringe any copyright restrictions, as the intellectual content remains the original work of the author. Proper acknowledgement is hereby made to the respective conferences and professional platforms where the papers were first presented.

Where any paper was subsequently published in conference proceedings, journals, or other formats, such material has been reproduced in good faith for academic and historical purposes, with due recognition of the original presentation platforms.

The author expresses appreciation to the Nigerian Association of Pharmacists and Pharmaceutical Scientists in the Americas (NAPPSA) for providing a reputable international professional forum through which many of these scholarly contributions were first shared with the global pharmacy community.

This compilation is intended strictly for academic, historical, professional, and non-commercial scholarly purposes, consistent with established academic conventions regarding the republication of an author's original works.

Every effort has been made to ensure appropriate acknowledgement of prior presentation venues. Should any clarification be required regarding the status of any paper reproduced herein, the publisher and author remain willing to provide necessary information or make appropriate adjustments in subsequent editions.

Prof. Ahmed Tijjani Mora, OON, mni
B.Sc (Pharmacy), *MBA, PhD*
FPCPharm, FPSN, FICEN, FIHMN
Author

Chapter 8:

Some of the Papers Presented by Professor Ahmed Tijjani Mora at Conferences and Seminars (1988-date)

**Mandatory Continuing Professional Development (MCPD):
Successes and Challenges**

A Lecture by

Pharm. Ahmed T. Mora

At the One-Day MCPD Train-the-Trainer Workshop

Fred Adenika Auditorium Hall, PCN, Idu, Abuja

Wednesday, July 27, 2005

1.0 Introduction

1.1 The World Health Assembly Resolution WHA 27.31 recognized Continuing Education as an integral part of the health and education system, highlighting its importance to health authorities in achieving and maintaining high-quality healthcare.

1.2 The need for **Mandatory Continuing Professional Education** in the recertification of health professionals was acknowledged at the maiden National Health Summit in 1995. Subsequently, in 1996, the Honourable Minister of Health directed all professional regulatory bodies to begin discussions on modalities for the programme's early implementation.

1.3 In line with government directives, the Pharmacists Council of Nigeria (PCN) commenced the first cycle of **Mandatory Continuing Professional Development (MCPD)**, formerly known as MCPE for the recertification of pharmacists in April 1998. PCN was the first healthcare regulatory body in Nigeria to initiate MCPD.

Defaulting pharmacists were given a 16-month grace period (January 2004 – April 2005) to complete the three required modules.

1.4 The Federal Ministry of Health has since accepted the need for mandatory recertification of all health professionals. Processes are ongoing to define policy, regulations and legal frameworks to make MCPD a statutory requirement for recertification.

2.0 Objectives of MCPD

The objectives of MCPD are to:

- a. Update pharmacists' knowledge and keep them abreast of advances in pharmaceutical science and pharmacy practice.
- b. Supplement initial training, expanding knowledge and skills to meet consumer needs.
- c. Improve skills to ensure pharmacists remain relevant members of the healthcare team.
- d. Provide a platform for sharing ideas and experiences, enhancing competence and commitment, and improving the quality of products and services.
- e. Serve as an ongoing process of adaptation, enabling pharmacists to contribute effectively to change in healthcare delivery.
- f. Ensure pharmacists remain statutorily fit to offer licensed services, in line with Section 14(1) and (6) of the Pharmacists Council of Nigeria Act 91 of 1992.

3.0 Overview of the First Cycle

3.1 From April 1998 to April 2005, a total of **6,870 pharmacists** participated in the MCPD programme, translating to **17,099 completed modules** (MCPD database, July 26, 2005).

3.2 Participation statistics as of July 26, 2005, showed varying levels of completion across the required modules.

3.4 Registry Records and Complaints

A set of three lists of pharmacists who had completed **three, two and one MCPD modules** respectively was produced by the Registry and circulated in September 2004 to:

- All Directors of Pharmaceutical Services (DPSs),
- All MCPD Centres,
- The Federal Ministry of Health, Abuja, and
- The Pharmaceutical Society of Nigeria (PSN) National Secretariat, Lagos.

This circulation was intended to cross-check records available in the Registry and allow pharmacists to lodge complaints where necessary. Many complaints were received (and are still being received), and their treatment is ongoing at the Registry. As a result of this process, an updated list of participants who had completed the three MCPD modules was produced and circulated to all DPSs in March 2005.

4.0 The Present Focus of the Programme

4.1 Change of Nomenclature

The Council, in its repositioning of the Registry, decided to change the nomenclature from **Mandatory Continuing Professional Education (MCPE)** to **Mandatory Continuing Professional Development (MCPD)**. This change was based on the understanding that pharmacists have already been educated; what is now required is continuous **development**, updating their knowledge and skills to keep pace with advancements in pharmaceutical science and modern trends in pharmacy practice.

4.2 MCPD Curriculum and Credit Units

The MCPD curriculum covers core and elective areas with assigned credit units as follows:

- **Public Health Pharmacy** – 6 Credit Units
- **Health Education and the Pharmacist** – 2 Credit Units
- **Information Technology and Pharmacy** – 2 Credit Units

- **Contemporary Issues** – 4 Credit Units
- **Pharmacy Practice** – 4 Credit Units
- **Pharmacy Laws and Consumer Rights** – 5 Credit Units
- **Recent Advances in Management and Therapy of Prevalent Diseases** – 3 Credit Units
- **Principles of Management** – 2 Credit Units
- **Pharmaceutical Sales and Marketing** – 2 Credit Units
- **Laboratory Investigations** – 2 Credit Units
- **Total Quality Assurance** – 3 Credit Units

Special Electives (existing CEP activities of PSN and related bodies) – 14 Credit Units

4.3 Feedback and New Direction

A general appraisal of the first MCPD cycle highlighted the need to make programmes more **practical, participatory and interactive**.

This led the Governing Council to organize a **Train-the-Trainer (TOT) Workshop** to:

- Prepare resource persons,
- Standardize delivery across MCPD centres, and
- Strengthen monitoring and feedback mechanisms.

4.4 Involvement of Practitioners

The Council recognized the importance of engaging **experienced practitioners** from diverse fields, not just academics, as resource persons. Their expertise would complement the scholarly contributions of universities and broaden the programme's relevance to real-world practice.

4.5 Scope of the TOT Workshop

Participants in the workshop were drawn from:

- All 11 MCPD Centres,
- The Pharmaceutical Society of Nigeria (PSN),
- Technical groups outside academia,

- Zonal offices, and
- The PCN Headquarters/Registry, Abuja.

4.6 Objectives of the TOT Workshop

The objectives of the Train-the-Trainer Workshop included:

- Improving the quality of seminar delivery and overall programme organization.
- Ensuring acceptable uniformity of MCPD programmes across centres.
- Providing feedback to MCPD centres for self-evaluation and continuous improvement.
- Strengthening coordination between PCN and MCPD centres.
- Making recommendations for a sustainable MCPD programme for registered pharmacists in Nigeria.
- Enhancing the capacity of participants to cascade knowledge to other resource persons.

5.0 Further Challenges

Being the pioneer regulatory body in Nigeria to implement training linked to professional recertification, PCN must continuously refine MCPD, particularly in **programme delivery**.

The psychology of **adult learning** must be factored into MCPD design. If teaching remains mechanical, even well-structured topics may fail to achieve their objectives. Adult learners require interactive, participatory approaches to derive value and maintain engagement.

The **second MCPD cycle** aims to correct lapses experienced in the first by introducing this new focus. Presentations should evolve into **interactive exercises** that foster understanding, engagement, and professional satisfaction for participants.

Thank you all, and God bless.

**Pharmacists Council of Nigeria (PCN)
PCN Standards for Monitoring Registered Premises**

A Paper by

Pharm. Ahmed T. Mora, *FPSN, FPCPharm*

Registrar and Secretary to the Council, Pharmacists Council of
Nigeria (PCN)

Presented at the National Pharmaceutical Inspectors Workshop

Organized by the Pharmacists Council of Nigeria

Merit House, Abuja

Tuesday 2nd – Wednesday 3rd August, 2005

Introduction

The repositioning plan by the Registry for more effective regulatory control of the profession by the Pharmacists Council of Nigeria (PCN) submitted to and approved by the Governing Council in 2004 gave birth to the Inspection and Monitoring Department. Previously, the Department was responsible only for the inspection of premises already registered for one form of pharmaceutical activity or another, as well as for new premises seeking approval. Inspection, as we all know, focuses on personnel and equipment in the environment (the premises). However, the mere presence of these elements in any premises does not automatically translate into excellent service to the public. In other words, well-kept premises with the required facilities and well-trained personnel may not necessarily guarantee high-quality professional service.

Inspection vs. Monitoring

Monitoring, on the other hand, focuses on the actual delivery of professional services to patients within the premises. It evaluates how core services are rendered by utilizing the instruments, facilities and personnel earlier observed during the inspection phase. A premises under monitoring must have previously been inspected, approved and licensed by the Council. Thus, the distinction is clear: **inspection**

is about compliance with physical and structural requirements, while **monitoring** assesses the quality of pharmaceutical care delivered.

A Compendium of Minimum Standards of Practice

The search for benchmark standards in pharmacy practice led the Registry to initiate the compilation of **Minimum Standards of Practice for the Assurance of Pharmaceutical Care** across the following areas:

- Hospital Pharmacy
- Retail Pharmacy
- Wholesale Pharmacy
- Importation and Manufacturing

This initiative was approved by the Governing Council, resulting in a four-part compendium that defines the practice in a standardized manner. The formal presentation of the compendium was scheduled for August 18, 2005, during the commissioning of the PCN Head Office/Registry by the Honourable Minister of Health. The compendium ensures that every inspected, approved and licensed premises has access to minimum standards. Pharmacists can use it to gauge expectations, while inspectors can rely on it as a tool to evaluate compliance and service quality.

Monitoring Activities

Monitoring activities are carried out through the following methods:

- **Observation** of interactions between the pharmacist and patients.
- **Discussions** between the inspector and the pharmacist on practice-related issues to evaluate competence and adherence to professional standards.
- **Reference to previous inspection reports** to assess continuity of compliance.

The goal is to ensure that practices meet or exceed benchmark standards. Falling below such standards is considered a failure in professional service delivery.

The Future of Monitoring

- Monitoring is expected to provide a **two-way feedback system** between licensed premises and the regulatory body on the adequacy of services rendered.
- It is the responsibility of PCN to ensure society receives the pharmaceutical care it deserves. This is one reason why the **Mandatory Continuing Professional Development (MCPD)** programme was introduced to update pharmacists' knowledge.
- With monitoring fully established, PCN will design **specialized continuing development programmes** tailored to different practice areas (hospital, retail, wholesale, importation and manufacturing).
- Updated versions of the compendium will serve as reference materials for continuing professional development.
- PCN also has the responsibility to periodically update the knowledge base of licensed premises, outside MCPD cycles, to ensure that patients consistently receive quality services.

The Nigerian public is becoming increasingly sophisticated, demanding only the best. Patients may seek second opinions or even resort to legal action if professional service falls short of expectations. To protect both patients and pharmacists, monitoring must be meaningful, comprehensive and proactive.

The best time to entrench this system is now.

Thank you all, and God bless.

**Presentation by the Registrar, Pharmacists Council of Nigeria
(PCN)**

**Pharm. Ahmed T. Mora, *FPSN, FPCPharm*
on the Laws and Regulations Governing Pharmacy Practice in
Nigeria and Reciprocation**

**At the First Annual Convention of the Nigerian Association of
Pharmacists and Pharmaceutical Scientists in the Americas
(NAPPSA), Houston, Texas, USA
September 14–16, 2007**

Introduction

It is a pleasure to be here today at the maiden convention of the Nigerian Association of Pharmacists and Pharmaceutical Scientists in the Americas (NAPPSA). When this invitation first came, through phone calls, emails and a subsequent visit to my office in Abuja by your President, Mr. Nndum Iheme, accompanied by Mr. Jonathan Obiagwu, I realized that Nigerian pharmacists in the USA and across the Americas have not forgotten their motherland. I congratulate the President, the Board, the Executive Committee and indeed all members of NAPPSA on this important convention. Please, keep the flag flying.

I have been asked to speak on the topic: **“Laws and Regulations Governing Pharmacy Practice in Nigeria and Reciprocation.”**

1.0 Historical Perspective of Pharmacy and Drug Laws in Nigeria

a. Origins of Pharmacy in Nigeria

The practice of pharmacy in Nigeria is closely linked to the arrival of colonial missionaries along the West African coast in the 17th century. Archives reveal that the Portuguese, who came with doctors and nurses to the coastal towns, also brought medicines in the form of mixtures, lotions and tablets. By 1887, Mr. Richard Zacheus

Bailey had established a drug store along Balogun Street in Lagos. Before then, hospitals and dispensaries had already been opened in Lagos, Warri, Asaba and Calabar. With the expansion of British colonial influence in the 19th and 20th centuries, Nigeria was fully integrated into British administration. From this perspective, one can safely argue that modern pharmacy practice in Nigeria is rooted in Great Britain, guided by British ethics of practice, bilateral agreements between sovereign nations and Memoranda of Understanding under the World Confederation of Pharmacists. As with all established professions, pharmacy has historically sought to maintain standards in qualification and practice. In Nigeria, this led to the institutionalization of regulatory bodies established by legal instruments.

Today, several regulatory agencies influence pharmacy practice in Nigeria, including:

- **Pharmacists Council of Nigeria (PCN)**
- **National Agency for Food and Drug Administration and Control (NAFDAC)**
- **National Drug Law Enforcement Agency (NDLEA)**
- **Ministerial and Executive Policies** of the Federal Government on health matters (e.g., the Draft Health Act).

b. Brief History of Pharmacy Laws in Nigeria

The earliest laws controlling pharmacy practice date back to **1878**, when the Lagos Pilotage and Harbour Ordinance was enacted to regulate medical treatment and the use of medicines on board ships docked in Lagos Harbour.

Other key milestones include:

- **1881:** Hospital Ordinance.
- **1889:** Ereko Dispensary Rules.
- **1902:** Pharmacy Ordinance No. 8, which regulated the sale and distribution of drugs and poisons (initially restricted to Lagos

Colony and a few protectorates). Under this law, dispensers and druggists were examined and certified.

- **1925:** Establishment of the Medical College, Yaba, Lagos, which produced trained dispensers. This prompted the enactment of the Poison and Pharmacy Ordinance, strengthening registration of dispensers, chemists, and druggists.
- **1936:** Poison and Pharmacy Ordinance revised.
- **1958:** Poison and Pharmacy Act (Cap. 152), which amended earlier provisions.

In recent judicial review, the history of pharmacy regulation was summarized in the case: **Nigeria Association of General Practice Pharmacists Employers (NAGPPE) vs. PCN, Suit No. FHC/KD/CP/8/2001**. In this case, Justice Liman of the Federal High Court, Kaduna (2001) observed:

“Both Counsel did not bother to familiarize themselves with the history of pharmacists and pharmaceutical chemists, when it should have been obvious that the Poison and Pharmacists Act 1958 was partially repealed in 1964, and retained as the Pharmacy Act Cap 26 (1990 as Cap 357 LFN). Section 26(1) of the Pharmacists Council of Nigeria Decree No. 91 of 1992 repealed Cap 357. Therefore, the only law regulating the practice of pharmacy in Nigeria today is **Decree 91 of 1992.**”

2.0 Laws Governing Pharmacy Practice in Nigeria – Repeals and Amendments

It is the desire of every government to provide good healthcare services to its citizens. Under **Chapter II** of the **Constitution of the Federal Republic of Nigeria (1990, Cap 62)**, Section 17(2)(c) provides that:

“The State shall direct its policy towards ensuring that there are adequate medical and healthcare facilities for all persons.”

In line with this constitutional provision, successive governments have enacted legislations on health and pharmacy practice. Key laws include:

2.1 Pharmacists Council of Nigeria (PCN) Decree No. 91 of 1992

- Established the **Pharmacists Council of Nigeria (PCN)**, replacing the former Pharmacy Board of Nigeria (PBN), which was created in 1964.
- Charged with responsibilities including:
 1. Determining the standards of knowledge and skills required for registration as a pharmacist.
 2. Establishing and maintaining registers of persons entitled to practice, and publishing same.
 3. Reviewing and enforcing codes of conduct for pharmacists.
 4. Regulating and controlling pharmacy practice in all its aspects.
- Also established:
 - **Pharmacists Disciplinary Tribunal**
 - **Investigating Panel** (for cases of professional misconduct).
- **Sanctions:** Punishments are limited to removal from register, suspension or warning. The highest punishment (Section 23(4)) is a fine of ₦1,000 or imprisonment up to 2 years—a grossly inadequate deterrent in modern times.

2.2 Food and Drugs Act, Cap. 150 LFN 1990 (as amended by Decree No. 21 of 1999)

- Regulates the **sale, manufacture, distribution, exportation, and advertisement** of food, drugs, cosmetics and devices.

- Initially empowered the **Minister of Health** to issue certificates to manufacturers (delegable authority), subject to the presence of a Superintendent Pharmacist.
- Enforcement functions are now carried out by **NAFDAC** though the Act technically remains valid law.
- **Penalties:** Fine not exceeding ₦1,000 or imprisonment up to 2 years (grossly outdated).

2.3 NAFDAC Decree No. 15 of 1993 (Deemed effective from 1st October 1992)

- Established the **National Agency for Food and Drug Administration and Control (NAFDAC)** as successor to the Food and Drug Administration Department of the FMoH.
- Functions include regulating and controlling the importation, exportation, manufacture, distribution, advertisement and use of drugs, cosmetics, foods, chemicals and medical devices.
- NAFDAC is also empowered to inspect premises, raw materials and enforce compliance.
- **Penalties:** Offenders face fines up to ₦5,000 or imprisonment up to 2 years (again inadequate).

2.4 NDLEA Decree Cap. 253 LFN 1990 (as amended by Decree 15 of 1992)

- Established the **National Drug Law Enforcement Agency (NDLEA)**, with full police powers.
- Empowered to inspect premises dealing in narcotics, check records and collaborate with other regulatory bodies.
- **Penalties:** Range from 5–25 years imprisonment, with fines up to ₦2 million and possible forfeiture of goods.

2.5 Poisons and Pharmacy Act, Cap. 535 LFN 1990

- Regulates the sale and distribution of poisons and certain classes of drugs.
- Provides for appointment of inspectors authorized to enter premises and enforce compliance.
- **Penalties:** Ridiculously low—₦10 to ₦200 fines or up to 12 months imprisonment.

2.6 Drug and Related Products (Registration, etc.) Decree, 1993 (as amended by Decree 20 of 1999)

- Prohibits the manufacture, importation, advertisement, sale, or distribution of unregistered drugs, cosmetics and medical devices.
- Enforced by **NAFDAC** through the Drug Registration Committee.
- **Penalties:** Individuals liable to ₦50,000 fine or 2 years imprisonment; corporate bodies liable up to ₦100,000.

2.7 Dangerous Drugs Act, Cap. 91 LFN 1990

- Regulates importation, exportation, manufacture, sale and use of **opium, cocaine and other dangerous drugs**.
- Grants regulatory authority to the **President**, Customs and Excise officers.
- **Penalties:** Fine of ₦200 or imprisonment up to 2 years, including the forfeiture of goods.

2.8 National Drug Formulary and Essential Drugs List Act, Cap. 252 LFN 1990

- Provides for a **National Drug Formulary and Essential Drugs List**.

- Prohibits the importation/manufacture of drugs outside the list except with ministerial approval (e.g., for rare diseases).
- Establishes a Review Committee and Secretariat under the FMoH.
- **Penalties:** ₦1,000 fine or imprisonment up to 5 years.

2.9 Counterfeit and Fake Drugs and Unwholesome Processed Foods (Miscellaneous Provisions) Decree No. 25 of 1999

- Prohibits the **manufacture, importation, sale, and distribution** of counterfeit/fake drugs and unwholesome processed foods.
- Provides for the establishment of a **Federal Task Force**, working with the Nigerian Police.
- **Penalties:** Fine not exceeding ₦500,000 or imprisonment between 5–15 years or both.

3.0 Rules and Regulations Governing Pharmacy Practice

It is important to note that the **subsisting law** regulating and controlling the practice of pharmacy in Nigeria, including the retention of pharmacists' names in the Register, is the **Pharmacists Council of Nigeria (PCN) Act No. 91 of 1992**. The Act empowers the PCN to make **rules and regulations** to guide its operations. These rules and regulations are compiled, published, and gazetted from time to time. They include the following:

3.1 Key PCN Publications and Guidelines

- i. **Disposal of Poisons Book (2003)** – Used for the disposal and control of dangerous drugs in hospitals, pharmacies and other health institutions.
- ii. **Code of Ethics for Pharmacists in Nigeria (2003)** – Outlines the moral and professional obligations of

pharmacists and prescribes standards of conduct appropriate to the profession.

- iii. **Procedures for Registration (2005)** – Provides detailed procedures for the registration of pharmaceutical premises across various activities regulated by PCN.
- iv. **Guidelines on the Issuance of Patent and Proprietary Medicines Vendors Licences (2003)** – Defines the process for granting licences to vendors. The PCN remains the sole licensing authority.
- v. **Approved Patent Medicine List (3rd ed., 2003)** – Provides an updated list of medicines approved for sale in licensed patent medicine shops nationwide, in line with the **National Drug Policy**.
- vi. **Pharmacy Technicians Training Manual (Rev. ed., 2005)** – Ensures uniformity in curriculum for training pharmacy technicians across Schools of Health Technology in Nigeria.
- vii. **Internship Programme (Rev. ed., 2005)** – Introduces and guides fresh pharmacy graduates into professional practice.
- viii. **Internship Logbook (2005)** – Designed for interns to record weekly work activities during training, signed by preceptors for accountability.
- ix. **Orientation Programme for Patent Medicine Vendors (Rev. ed., 2004)** – Provides structured training and orientation for patent medicine vendors to ensure safe practice, given the limited number of pharmacists nationwide.
- x. **Foreign Pharmacy Graduate Orientation Programme (FPGOP) Manual (2005)** Guides foreign-trained pharmacy graduates on adapting to Nigerian practice

standards, laws, ethics, tropical medicine, and public health realities.

- xi. **Pharmacists Council of Nigeria Act No. 91 of 1992** – The principal legislation regulating pharmacy practice in Nigeria.
- xii. **Mandatory Continuing Professional Development (MCPD) Brochure (2005)** - Guides registered pharmacists on procedures for accessing Continuous Professional Development (CPD) programmes, necessary for re-certification.
- xiii. **A Four-Part Compendium of Minimum Standards for the Assurance of Pharmaceutical Care in Nigeria (2005)** – Prescribes minimum acceptable standards for pharmacy practice in:
 - Manufacturing outfits
 - Hospital pharmacies
 - Community pharmacies
 - Wholesale/importation premises
- xiv. **Guidelines for the Establishment of Pharmacy Programmes (2005)** – Provides Nigerian universities with guidelines on establishing faculties of pharmacy and training programmes.

3.2 Gazetted Regulations (2005)

Under the authority of the **PCN Act No. 91 of 1992** and with the approval of the Honourable Minister of Health, the following regulations were gazetted in 2005:

- 1. **Inspection, Location, and Structure of Pharmaceutical Premises Regulations (2005, No. 79, Vol. 92)**
 - Established a **Pharmaceutical Inspectorate Division**.
 - Empowered inspectors to enter premises at reasonable times where pharmaceutical activities are suspected.

- Prohibited locating pharmaceutical premises in markets, kiosks or roadside stalls.
- Prescribed penalties of imprisonment (≥ 6 months) and/or fines (₦50,000).

2. **Registration of Pharmaceutical Premises Regulations (2005, No. 81, Vol. 92)**

- Specified requirements for obtaining a premises licence.
- Restricted retail practice to companies incorporated and wholly owned by registered pharmacists (or partnerships of same).
- Made the registration of **hospital pharmacies** mandatory.
- Defined penalties for defaulting premises.

4.0 **Extant Laws and Their Position on Reciprocity**

As earlier stated, the principal law regulating pharmacy practice in Nigeria is the **Pharmacists Council of Nigeria (PCN) Act No. 91 of 1992**, which formally established the PCN.

This Act provides explicit provisions on:

- **Qualifications for registration** as a pharmacist,
- **Conditions for licensing**, and
- Other related matters governing the profession.

Importantly, the Act confers on the **PCN exclusively the statutory authority** to register pharmacists and issue licences. This means that any arrangement or **Memorandum of Understanding (MoU)** that attempts to establish a parallel registration body, even if in collaboration with the PCN, would **contradict the provisions of the law**.

However, the law does not prevent the PCN from:

- Conducting **orientation programmes for foreign-trained pharmacists** in the United States of America (USA), in

collaboration with professional associations, such as NAPPSA;

- Facilitating **reciprocity arrangements** regarding the retention of Nigerian pharmacists' names on the register, provided such programmes are **not restricted** only to members of any one association (e.g., NAPPSA).

The issue of **reciprocity** has consistently been a key concern for the PCN, particularly in relation to:

1. **Annual license renewal** for Nigerian pharmacists living abroad, and
2. The **Orientation Programme for Foreign-Trained Pharmacists**, conducted annually (August/September) in rotation across accredited faculties of pharmacy in Nigeria.

This programme is also featured in the PCN's **2007–2011 Strategic Plan of Action**, which was under review by the Council at the time of this presentation. Furthermore, at its **26th Governing Council meeting** held on **Thursday, August 2, 2007**, the PCN approved an **annual flat fee of \$100 (USD)** for Nigerian pharmacists practicing outside the country to maintain their names on the Register of Pharmacists in Nigeria. Details on implementation were to be issued subsequently.

5.0 Lacuna, Loopholes and Overlap of Functions among Enforcing Agencies

5.1 Current Situation

Pharmacy practice in Nigeria today is plagued by several challenges, including:

- The prevalence of **fake and substandard drugs**,
- The **arbitrary sale of medicines in open markets**,
- The **high cost of pharmaceuticals**, and

- **Inadequate enforcement mechanisms**, particularly the shortage of pharmaceutical inspectors.

Since pharmacy practice directly concerns the **availability, safety, efficacy and authenticity** of pharmaceutical products, the **Pharmacists Council of Nigeria (PCN)** should naturally play the central regulatory role. However, overlaps in legal mandates have created systemic weaknesses.

5.2 Weaknesses in Law Enforcement

Although Nigeria has numerous laws regulating pharmacy practice, **many are outdated** and implementation remains the greatest challenge. For example:

- Under the **Poisons and Pharmacy Act (Cap. 535 LFN)**, pharmaceutical inspectors lack prosecutorial powers. They must hand offenders over to the **police** for prosecution. In practice, offenders are often released, or penalties imposed are too weak to serve as deterrents.
- Many penalties under existing laws are **ridiculously low** compared to the severity of offences, creating little incentive for compliance.

5.3 Task Forces and Regulatory Conflicts

Despite multiple legislations against counterfeit drugs and narcotics, the problem persists due to **conflicts in enforcement structures**.

- The **Counterfeit and Fake Drugs (Miscellaneous Provisions) Decree No. 17 of 1997**, now Cap. 73 LFN, stipulates that the **Chairman of the Federal Task Force** should be an **Assistant Director in the Federal Ministry of Health**. In practice, however, the role is occupied by a **NAFDAC official**, raising legal questions that could invalidate prosecutions in court.

- The **Federal Ministry of Health** has gradually lost effective **supervisory control**, weakening coordination among enforcement bodies.
- Task Forces, originally meant to **dismantle illegal drug premises** often focus instead on **raiding registered pharmaceutical premises**, while illegal outlets continue to flourish.
- Regulatory overlap also arises between **NAFDAC, NDLEA, and PCN**, as their enabling laws permit them to inspect pharmaceutical premises. Yet, under the law, only the **PCN is empowered to license such premises**.

This duplication of mandates creates **confusion, conflict and inefficiency**.

5.4 Drug Distribution Challenges

Nigeria's drug distribution system remains **chaotic** and, at times, an embarrassment at the International level. In the past, certain **West African countries outrightly banned Nigerian pharmaceutical imports** due to concerns over quality and distribution practices.

5.5 Patent and Proprietary Medicine Vendors (PPMVs)

Another loophole exists in the **regulation of Patent and Proprietary Medicine Vendors (PPMVs)**:

- By law, PPMVs are only permitted to handle drugs in their **original, sealed packs**.
- In practice, many operate far beyond this legal scope.
- Nevertheless, recent regulatory measures by the **PCN** are beginning to achieve **modest success** in improving compliance within this category of medicine vendors.

6.0 Efforts Made to Review the Laws

The **Pharmacists Council of Nigeria (PCN)** has initiated a draft proposal to amend the existing **Pharmacy Law**, particularly *Act No. 91 of 1992*. This draft has been submitted to the **Legal Department of the Federal Ministry of Health** for review and subsequent transmission to the **Federal Ministry of Justice**.

6.1 Salient Provisions of the Proposed Amendments

The proposed amended law seeks to modernize and strengthen pharmacy regulation in Nigeria. Key provisions include:

1. **Enhanced Regulatory Powers for PCN**
 - Broader authority to monitor, regulate, and enforce compliance across all areas of pharmacy practice.
 - Clearer delineation of roles to avoid overlap with other regulatory bodies.
2. **Strengthening of Inspection and Enforcement**
 - Empowerment of **pharmaceutical inspectors** with prosecutorial powers.
 - Increased penalties for offences, such as sale of counterfeit, substandard or unregistered drugs, making them **more deterrent and proportionate** to the crime.
3. **Reform of Drug Distribution Channels**
 - Establishment of a **structured and transparent drug distribution system**, addressing the chaotic and unregulated markets currently in existence.
 - Stronger legal backing for the regulation of **Patent and Proprietary Medicine Vendors (PPMVs)**.

4. **Professional Development and Re-Certification**

- Legal reinforcement of the **Mandatory Continuing Professional Development (MCPD)** programme for pharmacists as a requirement for license renewal.
- Institutionalization of periodic training to ensure pharmacists remain updated with global best practices.

5. **Alignment with International Standards**

- Harmonization of Nigerian pharmacy laws with **World Health Organization (WHO)** recommendations and global pharmaceutical practices.
- Facilitation of **reciprocity agreements** for Nigerian pharmacists practicing abroad, ensuring recognition of professional qualifications.

S/N	PCN Decree 91 of 1992	New PCN Act 2004
1	Provisions relating to the establishment of the Council and composition, including financial provisions.	Retained on the proposed draft.
2	(a) Section 3(1) – On composition of Council paragraph (10)(1) provided for one representative of the Armed Forces, which shall be a registered Pharmacist. (b) Registrar not a member.	(a) Section 2(1)(f) – The representative shall not be lower than the rank of a Colonel. (b) The Registrar of the Council is made a member of Council. (c) Provided for one member representing pharmaceutical manufacturers.
3	Section 8(3) – Registrar to keep register of Associate Members and premises.	Section 7(c), (d) – Added two registers for Pharmacy Technicians and Licensed Patent Medicine Vendors.
4	Non-existent.	Section 10 – Introduces procedures for resolving registration matters between the Registry and applicants for registration.

5	Non-existent.	Section 10(13) – Restricts the operation of retail pharmacy to pharmacists registered under the Act, makes it mandatory for pharmaceutical companies (for purposes of importation, exportation or wholesale) to have a registered pharmacist on the Board of Directors, and empowers the Council to refuse registration of premises if public health would be prejudiced.
6	Non-existent.	Section 11 – Imported from Poison and Pharmacy Act the requirement for a pharmacist to maintain direct and continuous personal control and management of the premises he superintends.
7	Non-existent.	Sections 12 and 13 – Empowers authorized persons (Pharmaceutical Inspectors) to enter any place where drugs are sold, conduct investigation, and take samples of suspected items.
8	Section 14(2) – A pharmacist with at least fifty (50) years post-registration experience shall not pay practicing fees.	Section 18(3) – Any pharmacist with at least forty (40) years post-registration experience or who has attained the age of sixty (60) years is exempted from the payment of retention fees to maintain name in the register.
9	Section 14(6) – Non-payment of practicing fees in a year attracts a fine twice the prescribed fee for the first offence and ten times for subsequent offences.	Section 18(4) – Raised the fine to 50% of the applicable fee within the year.
10	Non-existent.	Sections 19–21 – Makes provisions for internee pharmacists, approval of institutions for internship training, and regulation of practice by internee pharmacists.
11	Section 23 – Provided punishment for filing false statements with the Council	Section 28 – Raised the fines to ₦20,000.00 and ₦50,000.00 respectively.

*Some of the Papers Presented by Professor Ahmed Tijjani Mora at
Conferences and Seminars (1988-date)*

	to a fine not exceeding ₦1,000.00 on summary conviction and not exceeding ₦1,000.00 on indictment or exceeding two years imprisonment.	
12	Section 22 – Makes provisions for library facilities only.	Section 29 – Makes provision for Council to maintain library and laboratory facilities.
13	Non-existent.	Section 30 – Provided against obstruction of a Pharmaceutical Inspector.
14	Non-existent.	Section 31 – Empowers the Council to prosecute offenders in a competent court of law.
15	Non-existent.	Section 33 – Created an omnibus punishment provision on persons found liable upon conviction to a fine of ₦50,000.00 or one-year imprisonment or both.
16	Non-existent.	Section 34 – Provided for training and registration of pharmacy technicians in Schools of Health Technology approved by the Council.
17	Non-existent.	Section 35 – Empowers the Council to make guidelines for the registration of hospital pharmacies.
18	Non-existent.	Section 38 – Makes provisions for re-certification before retention of names of registered persons, pharmacy technicians, and sellers of medicine.
19	Non-existent.	Section 40 – Provides for pharmacists to offer First Aid Services where it is not reasonably practicable for the patient to consult a medical practitioner.
20	Non-existent.	Section 41 – Provided for mandatory sixty (60) days pre-litigation notice to be served on Council by a person who intends to commence a suit against the Council.

21	Non-existent.	Sections 46–53 – Made provisions for supply of restricted or dangerous drugs.
22	Schedule 1 – Prescribes minimum qualification to membership of Council to be not less than seven (7) years post-registration experience.	Schedule 1 – Raised the minimum qualification to ten (10) years.
23	Schedule 2 – The minimum qualification to become Assessor to the PCN Disciplinary Tribunal is seven (7) years standing.	Schedule 2 – Raised the Assessor’s qualification to not less than ten (10) years standing.

7.0 Conclusion

The registration and/or retention of names of pharmacists whether foreign-trained or Nigerian-trained but resident outside the country, particularly in the USA is both feasible and fully accommodated within the extant laws. Under Act 91 of 1992, the Registry is empowered to recommend to Council the appropriate guidelines on registration and reciprocity for members of NAPPSA and other registered pharmacists who wish to retain their names in the PCN Register. At present, the Registry has engaged the services of a computer programmer to upgrade its systems and enable online annual license renewal for pharmacists. Once the Council formally approves the modalities for reciprocity, the Registry will immediately communicate these details to NAPPSA. For more information about the PCN, our official website www.pcnn.org can be visited.

In conclusion, I once again congratulate the leadership of NAPPSA for extending this invitation and for providing me with the opportunity to share my thoughts with colleagues in the United States. With modern information and communication technology, the barriers between nations are rapidly narrowing. Likewise, the “divide” between Nigerian pharmacists abroad and their

responsibilities to their regulatory body the PCN is being bridged for the convenience and benefit of all.

Thank you and God bless.

**Pharm. Ahmed T. Mora, *B.Sc. (Pharm), MBA, FPSN,*
*FPCPharm***

Registrar, Pharmacists Council of Nigeria

Houston, Texas, USA

Sunday, September 16, 2007

✉ tijjanimora@yahoo.com | pcnig@yahoo.ca

BRIEF HISTORY OF PHARMACY LAWS

<i>S/No</i>	<i>Ordinance / Law</i>	<i>Year</i>	<i>Purpose</i>
1	<i>Lagos Pilotage and Harbour Ordinance</i>	<i>1878</i>	<i>Control the supervision of the applications of medicines and medical treatment on board ships docked in Lagos harbor.</i>
2	<i>Hospital Ordinance</i>	<i>1881</i>	—
3	<i>Ereko Dispensary Rules</i>	<i>1889</i>	—
4	<i>Pharmacy Ordinance No. 8</i>	<i>1902</i>	<i>Control the sale and distribution of drugs and poisons in Lagos, Calabar, Opobo, Warri, Forcados, or such other areas as the Governor in Council deems fit.</i>
5	<i>Poison and Pharmacy Ordinance</i>	<i>1925</i>	<i>Enacted when the Medical College, Yaba, was established in 1925. It was enacted to fortify the registration of dispensers, trained chemists, and druggists.</i>
6	<i>Poison and Pharmacy Ordinance of 1936</i>	<i>1936</i>	—
7	<i>Poison and Pharmacy Act, Cap 152</i>	<i>1958</i>	<i>To amend the 1936 Ordinance.</i>
8	<i>Pharmacy Act Cap 26</i>	<i>1964</i>	<i>Partially repealed the Poison and Pharmacy Act 1958.</i>
9	<i>LFRN Cap 357</i>	<i>1990</i>	<i>Retention of 1964 Act.</i>
10	<i>Pharmacists Council of Nigeria (Decree 92 of 1992)</i>	<i>1992</i>	<i>Section 26(1) of this Decree repealed Cap 357.</i>

RELATED LAWS AFFECTING THE PHARMACY PROFESSION

<i>S/No</i>	<i>Law</i>	<i>Year</i>	<i>Emphasis</i>
<i>1</i>	<i>Food and Drugs Act Cap 150 LFN</i>	<i>1990</i>	<i>Makes provisions for the control of sale, manufacturing, distribution, advertisement, importation, and exportation of food, drugs, cosmetics, and devices. Issuance of certificate to manufacture is vested in the Honourable Minister of Health who may delegate. Condition is that this must conform with the Pharmacists Act (i.e., Superintendent Pharmacist as in Cap 152).</i>
<i>2</i>	<i>Decree No. 21</i>	<i>1999</i>	<i>Amended the FDA. Sections 7, 9, and 17 address the issues of manufacturing prohibition by unauthorized persons; designation of inspectors, officers, and analysts on recommendation of the Food and Drug Advisory Council; and imposition of penalty of fine not exceeding ₦1,000 or a term of imprisonment not exceeding three years, or both respectively.</i>
<i>3</i>	<i>NAFDAC Decree No. 15 of 1993 (Retrospective to October 01, 1992)</i>	<i>1993</i>	<i>Section 28 – Makes NAFDAC the successor agency to the former Food and Drug Administration and Control Department of the Federal Ministry of Health. Section 5 – Provides the Agency the responsibility to regulate and control the importation, exportation, manufacture, distribution, and use of drugs, cosmetics, foods, water, chemicals, and medical devices. Section 27 – Allows the Honourable Minister of Health to give directives of a special character and the Agency must comply. Section 24 – Empowers officers of the Agency to enter premises to enforce provisions of the Decree.</i>
<i>4</i>	<i>NDLEA Decree 33 Cap 253 LFN</i>	<i>1990</i>	<i>Authority to deal with narcotics issues in the country.</i>
<i>5</i>	<i>NDLEA Decree (Amended) by Decree 15 of 1992</i>	<i>1992</i>	<i>This amendment gives all police powers, privileges, and authorities to the Agency to enforce provisions of the enabling Decree. Section 3 – Requires the Agency to collaborate with Government bodies carrying out functions wholly or partly analogous to those of the Agency.</i>
<i>6</i>	<i>Poison and Pharmacy Act Cap 535 LFN</i>	<i>1990</i>	<i>This Act regulates the sale and distribution of drugs and poisons. The PCN Decree 91</i>

			<i>of 1992 is charged with the enforcement of the provisions of this Act.</i>
7	<i>Drug and Related Products (Registration, etc.) Decree 1993</i>	1993	<i>Promulgated principally to prohibit the manufacture, importation, exportation, advertisement, sale, or distribution in Nigeria of unregistered drugs, cosmetics, and medical devices. NAFDAC enforces the provisions of this law through the Drug Registration Committee.</i>
8	<i>Drug and Related Products (Registration, etc.) Amendment</i>	1999	<i>Amended the 1993 Decree.</i>
9	<i>Dangerous Drugs Act Cap 91 LFN</i>	—	<i>Section 7(1) – Empowers the President to make regulations to control the handling of cocaine and other drugs, while subsection (2) provides authorization for any person running a pharmacy in accordance with the Pharmacy Act (now Cap 535) to manufacture, compound, and carry on the business of dispensing drugs. Section 17(1) and (2) – Empowers any police officer, or pursuant to a warrant issued by a magistrate, to enter the premises of any person carrying on business related to this Act to inspect any document or stock of drugs if there is reasonable suspicion of commission of an offence and to seize and detain those drugs or documents. Section 19 – Makes provisions for penalties. Maximum penalty: ₦200 fine or imprisonment for two years and/or forfeiture.</i>
10	<i>National Drug Formulary and Essential Drug List Cap 252 LFN</i>	1990	<i>This Act prescribes a National Drug Formulary and an Essential Drug List and prohibits the importation into or manufacture in Nigeria of any drug not in the list except as permitted by the Honourable Minister of Health under Section 3 on certain grounds, including where the drug is a cure for uncommon diseases requiring specialized skill for diagnosis and treatment. A Review Committee is responsible for regulating dealings in drugs. Section 9 – Provides for the establishment of a Secretariat (or the Department of FDAC of the FMOH) responsible for monitoring and implementation of the list. Section 8 –</i>

*Some of the Papers Presented by Professor Ahmed Tijjani Mora at
Conferences and Seminars (1988-date)*

			<i>Provides penalty against a guilty person who may be liable to a fine of ₦1,000.00 or imprisonment not exceeding five years.</i>
11	<i>Counterfeit and Fake Drugs and Wholesome Processed Foods (Miscellaneous Provisions) Decree No. 25</i>	<i>1999</i>	<i>This law prohibits the sale, production, importation, manufacturing, distribution, etc., of counterfeit and fake drugs and unwholesome processed foods or sale of drugs or poisons in premises not duly licensed by the appropriate authority (defined as any person or body authorized to grant, license, or register premises for sale or distribution of drugs or poisons). Section 6 – Establishes a Federal Task Force working in conjunction with the Nigeria Police Squad (established under Section 10) to enforce the provisions of the law. Penalty: A person who commits an offence is liable on conviction to a fine not exceeding ₦500,000.00 or imprisonment for a term not less than five years or more than fifteen years, or both.</i>

**TEXT OF PAPER PRESENTED
BY PHARM. AHMED T. MORA, *FPSN, FPCPharm*
THE REGISTRAR, PHARMACISTS COUNCIL OF
NIGERIA (PCN)
ON THE OCCASION OF THE FORMAL INAUGURATION
OF THE DRUG DISTRIBUTION INSPECTION
COMMITTEE (DDIC) OF THE PCN
AT THE FRED ADENIKA AUDITORIUM ON
WEDNESDAY, JULY 02, 2008**

1.0 Introduction

It is my pleasure to welcome you all to this inauguration ceremony. Today marks the introduction of a new concept in the implementation of our regulatory and control functions under *Decree (now Act) 91 of 1992*. We are all aware of the inspections carried out by Pharmaceutical Inspectors either as staff of the PCN or under the Pharmaceutical Inspection Committee (PIC) of the Council. For clarity, inspections of premises by Pharmaceutical Inspectors, which may lead to approval and licensing of outlets where pharmaceutical activities take place, include:

- **Pharmaceutical Inspection Committee (PIC):** Retail, Wholesale, and Importation outlets.
- **Registry Staff (Inspection and Monitoring):** Pharmaceutical Manufacturing Companies.
- **Registry (Pharmacy Practice):** Hospital Pharmacies.

These inspections are not only for approvals and licensing but may also take the following forms:

Types of Inspections

1. **Comprehensive/Routine Inspection** – Generally reserved for:
 - New pharmaceutical establishments;
 - Establishments applying for permits to extend their

scope of operation;

- Establishments with major changes in key personnel or premises;
- Establishments not inspected for 3–5 years;
- Cases where credible information (even informal) suggests serious lapses.

Such inspections should be **announced**.

2. **Concise Inspection** – For establishments already inspected, aimed at assessing standards of good pharmacy practice. May be **announced**.
3. **Follow-up Inspection** – To verify that corrective measures previously recommended have been implemented. Where deadlines were given, this may be **unannounced**.
4. **Special Inspection** – Triggered by specific complaints of malpractice or non-compliance. Preferably **unannounced**.
5. **Investigative Inspection** – Used to assess the performance of new establishments whose scope of operation was previously unknown.

While inspections are generally announced, unannounced visits are necessary in some cases, especially for **follow-up, special and investigative inspections**.

Frequency of Inspections:

- All premises should be inspected at least **every 12–24 months**.
- Premises with frequent contraventions may be inspected **every six months**.
- Well-compliant premises may be inspected less frequently.

Most of us here have participated in one or more of these inspection types and have also received training, whether in-house, in external facilities or in structured workshops.

2.0 Inspection of Establishments in the Drug Distribution Chain

The welfare of patients and the general public is of paramount concern in the drug distribution chain, whether the drugs are manufactured locally or imported. Inspections are therefore undertaken to ensure:

- Protection of patients and the public from malpractices by distributors and suppliers;
- Compliance with drug laws and regulations on compounding, distribution, importation, exportation and storage;
- Maintenance of high ethical and professional standards in pharmaceutical practice.

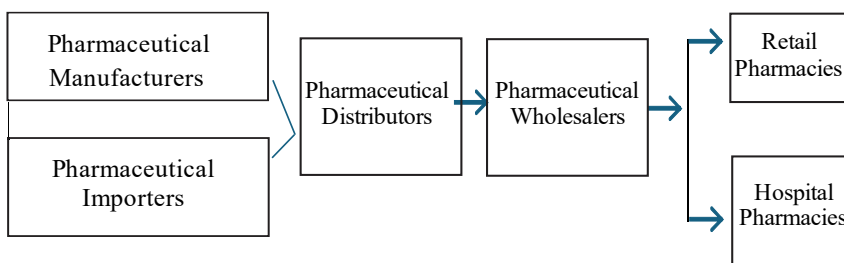
In this regard, **Section 5 of the Federal Republic of Nigeria Official Gazette (Lagos, 12th August, 2005; No. 81, Vol. 94)** provides guidelines under “*Proper Record Keeping in Premises*”:

1. Manufacturers, importers, wholesalers and retailers of drugs, poisons and devices shall maintain appropriate records of the receipt and disposal of drugs.
2. Unauthorized disposal of drugs shall attract penalties.
3. Retail outlets shall keep records of:
 - All purchases;
 - Disposal of poisons;
 - Ledgers/bin cards;
 - Dispensed prescriptions.
4. Wholesalers/Distributors shall keep similar records as required of retailers.
5. Importers shall maintain:
 - Relevant import documents;
 - Records as required under subsection (3).
6. Manufacturers shall keep records relating to:
 - Purchases of raw materials;
 - Production and quality control activities;
 - Distribution and sales of finished products.

From the above, it is evident that the **PCN is embarking on a new and broader form of inspection** different from the traditional inspections specific to particular practice areas.

In this **Drug Distribution Inspection**, the focus is on the **entire drug distribution chain**. In essence, the chain as a whole not just individual segments is under scrutiny.

A typical distribution chain may follow this flow:



This type of inspection addresses the multiplicity of practices as a system. It entails a thorough and painstaking as well as meticulous approach. In a manner, it could be investigative, as once a trail has been identified, the tracking must continue till the end, which is at the retail or hospital pharmacy levels.

3.0 Drug Manufacture, Dispensing and Medication Errors

A **medication error** is defined as any **preventable event** that may cause or lead to inappropriate medication use or patient harm while the medication is under the control of a healthcare professional, patient or consumer. Such events can arise from issues related to:

- Prescribing,
- Order communication,
- Product labeling, packaging, or nomenclature,
- Compounding,
- Dispensing,

- Distribution,
- Administration,
- Patient education, monitoring, and use.

In the specific context of pharmacy practice, **medication errors** often occur within healthcare facilities, such as hospitals, retail pharmacies and manufacturing settings, where pharmacists play a pivotal role.

Perspectives from Literature

According to Ghanaian pharmacists **H. K. Abutiate and Edward O. Amporful**, in their paper, “*Medication Errors Prevention: A Pillar for Patient Safety*” (The African Pharmacist, Vol. 1, No. 1, 2008), medication errors are incidents in which there has been an error in prescribing, dispensing, preparing, administering, monitoring or providing medicine advice, **regardless of whether harm occurred or not**. They further stated that in **England**, preventable harm from medicines costs an estimated **£750 million annually**. This underscores the economic and human burden of medication errors.

The **College of Nurses of Ontario (2006) Medication Practice Standard** emphasized preventing such errors by adhering to the **Five Rights of Medication Use**:

1. The **right drug**,
2. The **right dose**,
3. The **right patient**,
4. The **right time**,
5. The **right documentation (or note)**.

Similarly, **William A. Zellmer**, in his book “*The Conscience of a Pharmacist: Essays on Vision and Leadership for a Profession*” (ASHP, 2002), argued that pharmacists, particularly hospital pharmacists, must play a leadership role in preventing medication errors. He identified three critical “planes” of intervention:

- **First Plane – Individual Competence** Pharmacists must

recognize potential lapses in drug preparation, distribution, and administration, and build appropriate checks into their daily practice.

- **Second Plane – Hospital Systems** Hospitals must design their medication systems (ordering, dispensing, and administering) to minimize human error.
- **Third Plane – Quality Assurance** Pharmacy departments should implement continuous monitoring systems for detecting and reducing dispensing and administration errors.

Zellmer concluded that reducing medication errors requires **vigilance at every level of healthcare**, with pharmacists leading multidisciplinary programmes designed to prevent errors.

The Critical Role of Pharmacists

The **pharmacist's contribution** is central at every stage of the drug-use process:

- **Manufacturing:** Ensuring pharmaceutical products are formulated correctly.
- **Dispensing:** Providing the right medicine to the right patient with proper labeling.
- **Patient Counseling:** Giving clear, precise and unambiguous instructions for use.

Even a well-formulated and properly labeled medicine can become ineffective, or even harmful, if handled or dispensed unprofessionally. Thus, **the pharmacist stands as the decisive factor** in whether a medicine is used safely and effectively. The **World Health Organization (WHO)**, in its *2008 Summary of the Evidence on Patient Safety*, highlighted that including pharmacists in hospital medical teams significantly reduces errors. A 1999 study showed that

having a pharmacist on medical wards reduced **medication ordering errors by 66%**, from 10.4 to 3.5 per 1,000 patient days. Clearly, the **pharmacist, the premises and the practice environment** are all critical in minimizing medication errors.

4.0 The Challenge for the Committee

This Committee faces very challenging tasks for several reasons. First, there is no documented guideline or Standard Operating Procedure (SOP) for this type of inspection in Nigeria. The Committee, therefore, has to develop its own working system.

Second, inspections may be frustratingly long, as tracking a particular product, especially suspicious ones, could take considerable time.

Third, invoices, receipts and other industry documents may be deliberately hidden or falsified by outlets in an attempt to mislead the Committee's work.

Fourth, Committee members should also expect the deliberate creation of dead ends by some outlets.

Nonetheless, I am confident that the members of this Committee are seasoned professionals, who will rise to the challenge. While it is expected that much of the Committee's work will take place in Lagos, it may not end there.

For effectiveness, the Committee must equip itself with the following during inspections:

- A list of all registered premises in the country for the current year and at least the two preceding years.
- Calculators, notebooks and other tools for proper documentation, including recording invoice and receipt numbers.

I recommend that Committee members use today and tomorrow to define the modus operandi for their assignment, request all necessary materials, and set clear procedures. It is not necessary for the entire Committee to be present at every outing. I strongly recommend that the Committee be split into two groups, with each group taking turns to conduct inspections and later convening to share findings. This will ensure manageable numbers at each facility and prevent disruption of Registry work on a larger scale across the various offices from which members are drawn.

On this note, I hereby inaugurate the Drug Distribution Inspection Committee (DDIC) for the inspection of facilities, as outlined under Decree (now Act) 91 of 1992 and in accordance with Regulation 81, Vol. 904 of the Federal Republic of Nigeria Official Gazette, Lagos, dated August 12, 2005.

Thank you all and good luck.

**Pharm. Ahmed T. Mora, *FPSN, FPCPharm*
Registrar, PCN**

The Nigerian Pharmacist, Medication Errors and the Regulation and Control of Pharmacy Practice by the Pharmacists Council of Nigeria (PCN). Delivered as the Registrar and Chief Executive Officer (CEO) of the PCN at the Second Annual Convention of the Nigerian Association of Pharmacists and Pharmaceutical Scientists in the Americas (NAPPSA), held at Atlanta, Georgia, USA, from September 18–20, 2008.

1.0 Introduction

As the battle against fake, counterfeit and substandard drugs rages on, the need for an integrated and multidisciplinary effort to win the fight becomes even more apparent. It is the responsibility of every healthcare professional to ensure that medications and drugs intended for prophylactic, curative and/or diagnostic purposes within healthcare facilities are genuine, safe and efficacious.

A **pharmacist** is a person professionally qualified in **pharmacy**, the branch of health sciences dealing with the preparation, dispensing and rational use of medicines. Globally recognized and licensed, the pharmacist prepares, compounds, mixes and dispenses medicines for the benefit of patients and the healthcare delivery system.

The pharmacist's role in ensuring the successful use of medicines for treatment is total and paramount among healthcare professionals. Put simply, no matter the accuracy of diagnosis, the soundness of prescriptions, or the genuineness of medicines prescribed, if the pharmacist lacks professional competence due to inadequate updates or knowledge, medication errors are most likely to occur. It is therefore incumbent on the **regulatory body**, the Pharmacists Council of Nigeria (PCN), to ensure that only fit, proper and competent pharmacists are licensed to practice. Pharmacy practice traditionally takes place in hospital pharmacies, retail pharmacies, importation/wholesale firms and pharmaceutical manufacturing industries. The PCN registers, inspects, approves and licenses these premises in line with **Act 91 of 1992**.

2.0 The Nigerian Pharmacist

The Nigerian pharmacist is a healthcare professional registered by the PCN to practice pharmacy in Nigeria. He or she must:

- Graduate from any of the approved Faculties of Pharmacy in Nigerian universities;
- Complete internship training in an accredited centre; and
- Obtain an annual license from the PCN to practice.

Alternatively, foreign-trained pharmacists with at least a Bachelor's degree in Pharmacy must pass the **Mandatory Orientation Programme for Foreign-Trained Pharmacists** at a PCN-approved centre before being licensed. Ensuring continuous competence and fitness to practice is a basic responsibility of the PCN. This is largely achieved through **Mandatory Continuing Professional Development (MCPD)** programmes in accredited centres. MCPD exposes pharmacists to current developments in pharmaceutical sciences, thereby improving their knowledge and skills for the benefit of patients.

The ultimate objective is to present well-trained, registered and currently licensed pharmacists to the public, professionals who dispense appropriate pharmaceutical products and services within PCN-registered, inspected, approved and licensed premises.

To enforce compliance, the PCN carries out regular inspections both announced and unannounced. Depending on inspection reports, a pharmacy may:

- Continue to offer services,
- Be partially restricted, or
- Be sealed outright.

These measures are to ensure that the public accesses only those premises that meet PCN standards, as mandated by Act 91 of 1992.

3.0 Drug Manufacture, Dispensing and Medication Errors

A **medication error** is any preventable event that may cause or lead to inappropriate medication use or patient harm while the medication is under the control of a healthcare professional or patient. Errors may arise from professional practice, healthcare products, procedures and systems, including:

- Prescribing,
- Order communication,
- Product labeling and packaging,
- Compounding,
- Dispensing,
- Distribution,
- Administration,
- Patient education, monitoring, and use.

Abutiati and Amporful (2008), in *The African Pharmacist*, defined medication errors as incidents in which errors occur during prescribing, dispensing, preparation, administration, monitoring or advice—whether or not harm occurs. They estimated that preventable harm from medicines costs England over **£750 million annually**. Medication errors begin at the **prescription stage**, which can lead to downstream problems. The **College of Nurses of Ontario (2006) Medication Practice Standard** highlights human factors in errors and prescribes the five “rights”:

- Right drug,
- Right dose,
- Right client,
- Right time, and
- Right note.

William A. Zellmer (2002), in *The Conscience of a Pharmacist*, stressed that pharmacists must do all they can to prevent medication errors, particularly in hospitals where they occur more frequently. He identified three levels of action:

1. **Personal vigilance** – recognizing lapses in preparation, distribution and administration, and creating effective checks.
2. **Hospital systems** – designing medication processes that minimize human error.
3. **Quality assurance** – establishing ongoing systems to monitor and evaluate dispensing and administration errors.

He concluded that reducing errors requires vigilance at multiple levels, with pharmacists taking the lead in multidisciplinary programs.

Pharmacists' skills in formulation, dispensing and patient counseling significantly reduce errors. Every pharmaceutical product carries vital information and only pharmacists are trained to interpret and communicate this information professionally. Counseling must be clear, precise and unambiguous to ensure compliance. MCPD participation is therefore not optional; failure to comply results in non-licensure, regardless of prior registration. The **WHO (2008) Summary on Patient Safety** emphasized the pharmacist's role, citing a study in which the presence of a pharmacist reduced medication ordering errors by **66%**. Thus, the pharmacist, the premises and the facilities are all critical in ensuring patient safety.

4.0 Setting Standards for Pharmacy Practice by the PCN

Pharmacy practice in Nigeria has a long history:

- The first outlet opened in **1887** at Balogun Street, Lagos.
- The ***Pilotage and Harbor Ordinance (1878)*** and the ***Hospital Ordinance (1881)*** laid early foundations.
- The ***Pharmacy Ordinance No. 8 of 1902*** formally regulated the practice.
- The ***Poison and Pharmacy Ordinance (1923)*** and subsequent establishment of the **Board of Medical Examiners (1927)** further shaped regulation.

From 1927 to 1992, successive bodies, the Board of Medical Examiners, the Pharmacy Board of Nigeria and the Pharmacists Board of Nigeria,

regulated pharmacy until **Decree 91 of 1992 (now Act 91)** established the PCN. The PCN regulates pharmacy in all aspects. Section 26 of the Act transferred all powers of the defunct PBN to the Council. Courts have upheld its authority to regulate both public and private pharmacy practice. On **August 16, 2005**, the PCN launched the **Four-Part Compendium of Minimum Standards for the Assurance of Pharmaceutical Care**, defining the boundaries and scope of practice in hospital, retail, wholesale/importation and manufacturing pharmacy. This historic publication, presented by Hon. Minister Prof. Eyitayo Lambo, positioned PCN as a proactive and credible regulatory authority. The **Compendium** is now a required reference material in all pharmaceutical premises before registration or licensing.

5.0 Conclusion

The pharmacist plays a central role in ensuring patient compliance and reducing medication errors. Within the healthcare delivery chain, the professional responsibility for producing, compounding and dispensing medicines rests squarely on the pharmacist. The late U.S. President **John F. Kennedy** once remarked:

“The pharmacist is an indispensable link in the chain of national health protection. If we did not have the pharmacist, it would be necessary to invent him.” (*Pharmacy Practice Management in Developing Communities*, Fred B. Adenika, 1975). Today, Nigerian pharmacists stand tall among their healthcare colleagues. The PCN’s role in training, equipping and licensing competent, fit and proper pharmacists is vital. By enforcing standards, accrediting premises and mandating MCPD, the PCN ensures the public receives the **highest quality pharmaceutical care**.

Pharm. Ahmed T. Mora, FPSN, FPCPharm

Registrar, PCN

September 18–20, 2008

**Inter-Country Pharmacy Practice: The Responsibility of
National Pharmacy Councils. Presented by Pharm. Ahmed T.
Mora, Registrar and CEO of the Pharmacists Council of
Nigeria (PCN), at a Workshop on the Harmonisation of
Pharmacy Curriculum Across the West African Sub-Region,
organised by the West African Health Organisation (WAHO),
at the West African Health Community Secretariat, Yaba,
Lagos, on October 23, 2008.**

1.0 Introduction

It is a pleasure to be here on the invitation of Prof. Diallo Abdoulaye of the West African Health Organization (WAHO) to share my thoughts with distinguished colleagues. I congratulate WAHO for bringing this important topic before this august body and thank them for the opportunity to participate in this workshop.

The responsibilities of pharmacy regulatory bodies in member countries include:

- a. Determining the minimum professional knowledge required of those seeking to become pharmacists.
- b. Registering and licensing pharmacists upon successful completion of their studies.
- c. Registering and licensing pharmaceutical premises where pharmacists practice, such as hospital pharmacies, retail pharmacies and pharmaceutical wholesalers.
- d. Ensuring continuous professional knowledge through Continuous Professional Development (CPD) programmes so pharmacists remain current in practice and can provide optimal pharmaceutical care, professional advice to other healthcare professionals and support for efficient health systems.
- e. Periodic inspection and monitoring of licensed pharmaceutical premises to ensure compliance with standards, such as Good Manufacturing Practice (GMP) and other regulations stipulated by their respective Governing Councils or Boards.

Examples of pharmacy regulatory bodies within the sub-region include:

- Pharmacists Council of Nigeria (Nigeria)
- Pharmacy Council of Ghana (Ghana)
- Pharmacy Board of Sierra Leone (Sierra Leone)
- Pharmacy Board of Liberia (Liberia)
- Pharmacy Council of The Gambia (The Gambia)

Similar bodies exist in Mali, Senegal, Guinea, Togo, Côte d'Ivoire, Burkina Faso, Benin Republic, Niger, Cape Verde and Guinea-Bissau.

2.0 Inter-Country Pharmacy Practice

The need for inter-country pharmacy practice among WAHO member nations is both desirable and timely. The goals of WAHO and the broader Economic Community of West African States (ECOWAS) cannot be fully realized without bold steps toward integrating professional services, trade, and practice.

The following steps are proposed:

- **Harmonization of Pharmacy Curricula:** Align Bachelor of Pharmacy (B. Pharm) and Doctor of Pharmacy (Pharm. D) programmes across member states.
- **Establishment of a Regional Forum of Registrars/CEOs:** Create a body of Registrars and CEOs of Pharmacy Councils/Boards to compare instruments of practice, such as registrations, licenses, seals and signatures, and to meet periodically in rotation across member states.
- **Review of Enabling Laws:** Amend existing laws establishing Pharmacy Councils/Boards to allow for seamless inter-country pharmacy practice. Where this may take time, Councils/Boards should adopt resolutions to temporarily enable such practices.

The **Pharmacists Council of Nigeria (PCN) Decree** already allows for reciprocity, provided that:

- a. The applicant holds an approved qualification.

- b. The applicant passes the Council's examination in pharmacy law and ethics (or equivalent as required).
- c. The applicant acquires the required experience and residency in Nigeria before registration.

In practice, pharmacists from English-speaking West African countries can be registered to practice in Nigeria and Nigerian pharmacists can similarly be registered in Ghana. This reciprocity has largely been facilitated by professional migration (brain drain), where pharmacists from the region often seek opportunities in the UK, USA and the Middle East. Encouraging inter-country pharmacy practice within the region can help balance and cushion against this trend.

3.0 Integration of Related Regional Bodies

The West African sub-region has in recent times witnessed the establishment of several professional associations and networks, including:

- **West African Pharmaceutical Manufacturers Association (WAPMA):** Established in December 2002 with headquarters in Accra, Ghana.
- **West African Drug Regulatory Authorities Network (WADRAN):** Established to harmonize drug regulatory activities across the sub-region.
- **West African Postgraduate College of Pharmacists (WAPCP):** Formerly the West African Pharmaceutical Federation, now focused on postgraduate training.

These bodies, together with WAHO, provide platforms for harmonization. Their objectives must be integrated to fully exploit the potential of inter-country pharmacy practice.

4.0 Conclusion

The proposal to encourage inter-country pharmacy practice is excellent and long overdue. The statutory agencies capable of

actualizing it are the National Pharmacy Councils and Boards of member states. However, challenges remain:

- Differences in enabling laws.
- Variations in healthcare delivery systems, particularly drug distribution models.
- Language barriers (Anglophone vs. Francophone).

These challenges are not insurmountable. With gradual reforms, collaboration and goodwill, they can be overcome. The time to begin is now, and WAHO deserves commendation for initiating this vital step toward regional integration in pharmacy practice.

Thank you, and God bless.

Pharm. Ahmed T. Mora, B.Sc (Pharm), MBA, FPSN, FPCPharm
Registrar/CEO, Pharmacists Council of Nigeria
Lagos, October 23, 2008

**SPEECH OF THE REGISTRAR, PHARMACISTS COUNCIL
OF NIGERIA (PCN),**

PHARM. AHMED T. MORA, *FPSN, FPCPharm*

ON VISION 20-2020

**THE PHARMACIST AS AN INDISPENSABLE
PROFESSIONAL IN HEALTHCARE DELIVERY**

July 07, 2009

1.0 Introduction

The world's top economies are characterized by strong institutions and well-structured societies underpinned by good health policies, functional infrastructure, food and social security, population control, ICT development, human resource development, high standards of living, high per capita income and GDP and, most importantly, effective organization and government intervention. In Nigeria, however, the situation is far from ideal when compared with these characteristics.

2.0 Government Economic Development Efforts

The previous administration introduced the **National Economic Empowerment and Development Strategy (NEEDS)** as a response to Nigeria's development challenges. NEEDS was born out of the 2001 Kuru Declaration, Vision 2010 and extensive consultations nationwide. It aimed at reorienting national values, reducing poverty, creating wealth and generating employment. Towards the end of that administration, **NEEDS II** was proposed to bridge policy and implementation gaps. Subsequently, the **Seven-Point Agenda** of President Umaru Musa Yar'Adua, *GCFR*, identified major economic development indicators in line with global standards. This agenda was linked directly to the **Vision 20-2020 Project**, which seeks to place Nigeria among the world's top 20 economies by the year 2020.

3.0 Nigeria's Economic and Health Status

Nigeria is a developing economy with a struggling health system. The World Health Organization (WHO) in 2000 ranked Nigeria's health system **187th out of 191 countries**.

Challenges include:

- Low consumer health knowledge and awareness.
- Weak healthcare delivery systems.
- Poorly equipped and dilapidated facilities, often serving only 5–10% of their potential capacity.
- Low health expenditure, still less than \$10 per capita, compared to the \$34 internationally recommended (though improving under the present administration).

The pharmaceutical sector faces additional problems:

- Prevalence of fake, substandard, adulterated and unregistered medicines.
- Heavy dependence on imported medicines and raw materials.
- Only about **40% of medicines consumed locally are produced in Nigeria**, with **100% of Active Pharmaceutical Ingredients (APIs) imported**.
- Chaotic drug distribution and the persistence of open drug markets.

4.0 The Pharmacist's Indispensable Role in Achieving Vision 20-2020

Health is wealth and the adequate provision and supply of medicines are central to effective healthcare delivery.

Pharmacists are indispensable professionals with responsibilities spanning diverse practice areas, including hospitals, community settings, industry, administration and academia. Their contributions to Vision 20-2020 include:

1. Health policy administration.
2. Regulation and control of medicines and related products.

3. Oversight of pharmacy education and training.
4. Provision and management of medicines supply.
5. Manufacturing and drug product development.
6. Academic teaching and research.
7. Hospital pharmacy administration.
8. Monitoring drug utilization, adherence, and therapeutic outcomes.
9. Operationalizing pharmaceutical care.
10. Patient education and counseling on prescription and OTC medicines.
11. Provision of drug information to professionals, healthcare teams, and the public.
12. Promoting rational drug use through data collection, evaluation, and dissemination.
13. Maintaining robust documentation and information systems.
14. Contributing to national drug policy, formularies, and essential drug lists.
15. Conducting and documenting clinical trials.
16. Ensuring the quality, safety, efficacy, and potency of medicines.
17. Contributing to public health services and interventions.

5.0 The Pharmacist's Continued Relevance in a Changing Global Landscape

The achievement of Vision 20-2020 faces the challenge of ensuring the availability and retention of adequate numbers of pharmacists, particularly in underserved areas. This is compounded by global economic uncertainties and evolving disease burdens. To address this, the PCN has institutionalized the **Mandatory Continuing Professional Development (MCPD) programme** to ensure pharmacists remain relevant and competent.

The PCN continues to advocate for active pharmacist participation in:

- Hospital ward rounds.
- CPR programmes.
- Recording hospital admission medication histories.
- Preventing and managing drug-related problems.
- Strengthening pharmaceutical services through medicines management systems.

Postgraduate programmes are also necessary to equip pharmacists in clinical settings with the expertise required to respond to evolving health needs.

6.0 Conclusion

It remains incontestable that **health is wealth**. If we accept that the adequate provision and supply of medicines are central to healthcare delivery, then the pharmacist's role is indeed indispensable to achieving Vision 20-2020. The goals of Nigeria's **National Drug Policy**—to ensure adequate supply of safe, effective, affordable and quality medicines at all times; to promote rational use and to stimulate local production—are inseparable from the professional responsibilities of pharmacists.

Pharmacists, therefore, are critical partners in driving Nigeria toward Vision 20-2020.

Thank you for listening.

ADDRESS

By Pharm. Ahmed T. Mora, *FPSN, FPCPharm*

Registrar/CEO, Pharmacists Council of Nigeria (PCN)

**At the Opening Ceremony of a Two-Day Workshop for
Superintendent and Production Pharmacists of Pharmaceutical
Manufacturing Industries in Nigeria (Lagos & South-West
States).**

**Theme: “Achieving Excellence in Current Good Manufacturing
Practice (cGMP) for the Assurance of Quality Pharmaceutical
Products in Nigeria”**

**Venue: All Seasons Multipurpose Plaza, Agidingbi, Ikeja, Lagos
Date: Tuesday, July 12, 2009**

I am pleased to welcome you all to this two-day advocacy workshop for Superintendent and Production Pharmacists of the pharmaceutical manufacturing industries in Nigeria. On behalf of the management and staff of the PCN, I warmly welcome the Honourable Commissioner for Health, Lagos State, Dr. Jide Idris, who graciously accepted to chair this occasion. We also welcome the Managing Directors/CEOs of pharmaceutical industries, the President of the Pharmaceutical Society of Nigeria (PSN), the Directors of Pharmaceutical Services, invited guests, PCN staff from Lagos and South-West states, members of the press, ladies and gentlemen.

Pharmaceutical manufacturing requires specialized skills and compliance with strict regulatory standards. Reports from PCN inspectors and the Drug Distribution and Inspection Committee (DDIC) have revealed lapses in adherence to standards. As a regulatory authority under Act 91 of 1992, PCN has both a professional and societal duty to correct these deviations. This workshop has been designed to address the identified gaps, featuring contributions from:

- Renowned experts in pharmaceutical manufacturing.
- NAFDAC representatives.
- WHO representatives.
- PCN inspectors with practical field experiences.

We deeply appreciate their commitment and professional service.

This initiative reflects PCN's dedication to safeguarding the quality of pharmaceutical products in Nigeria and ensuring compliance with Good Manufacturing Practice (GMP).

Thank you for listening and, once again, I warmly welcome you to this important workshop. May Almighty God continue to bless Nigeria, *amin*.

Pharm. Ahmed T. Mora, *FPSN, FPCPharm*

Registrar/CEO, Pharmacists Council of Nigeria

Lagos

July 12, 2009

**Regulatory Challenges Facing Pharmacy Practice in Nigeria.
Delivered as Registrar and CEO of the PCN at the Third
Annual Convention of NAPPSA, held in Bethesda, Maryland,
USA, from September 17–20, 2009.**

Abstract

This paper enumerates the major challenges facing the implementation of the PCN's statutory mandate. It also briefly describes some of the Council's modest achievements, including its determination to integrate the principles and practice of Knowledge Management (KM) at all functional levels. The bottom line remains that ensuring the Nigerian public has access to licensed pharmaceutical outlets under the supervision of qualified and registered pharmacists will continue to be the PCN's priority.

Pharmacy is an indispensable component of an effective healthcare delivery system in any country and Nigeria is no exception. A fundamental responsibility of the Pharmacists Council of Nigeria (PCN) is to ensure the public has access to appropriate pharmaceutical products through registered, inspected, approved, licensed and monitored premises. These include retail pharmacies, wholesale/importation facilities, hospital pharmacies and manufacturing plants. This mandate is executed under the authority of the PCN Act 91 of 1992, which empowers the Council to regulate all aspects of pharmacy practice in Nigeria. The legal framework for regulating medicines and pharmaceuticals in the country has evolved through various instruments since 1887.

All pharmaceutical premises in Nigeria must be staffed by qualified and registered pharmacists, who are licensed annually by the PCN. The Council's Mandatory Continuing Professional Development (MCPD) programme ensures that only "fit and proper persons" with up-to-date professional competencies are permitted to offer pharmaceutical care to the public in approved settings. The PCN operates through an elaborate organizational structure within its Registry designed to implement regulatory mechanisms effectively across the country. Responsibilities are fulfilled at the grassroots level through a network of 35 State Offices and eight Zonal Offices,

all reporting to the Registrar, who serves as the Chief Executive Officer. As a public corporation, the PCN's Governing Council and Chairman are appointed by the President and Commander-in-Chief of the Federal Republic of Nigeria. The Council provides policy direction to the management for the proper control and regulation of pharmacy practice as stipulated in its establishing Act.

1.0 Introduction

Pharmacy practice in Nigeria today is broadly categorized into four specialties:

1. Retail Pharmacy Practice
2. Wholesale/Importation of Pharmaceutical Products
3. Hospital Pharmacy Practice
4. Pharmaceutical Manufacturing

The Pharmacists Council of Nigeria (PCN) is the federal government agency established by *Decree 91 (now an Act) of 1992*. It is vested with the responsibility of regulating and controlling all the aspects of pharmacy practice in Nigeria. This mandate encompasses the registration, inspection, approval, licensing and monitoring of all premises where pharmaceutical activities occur. Furthermore, the PCN is responsible for regulating the training, registration and licensing of "fit and proper persons" to study pharmacy and practice the profession in Nigeria. Therefore, the PCN has the onerous duty of ensuring that only individuals with relevant and appropriate professional competencies are licensed to provide pharmaceutical care to the Nigerian public. These services must be delivered from premises that have been registered, inspected, approved, licensed and are effectively monitored by the PCN.

The regulatory framework is further defined by other legal instruments, such as:

- The ***Inspection, Location and Structure of Pharmaceutical Premises Regulations 2005***
- The ***Registration of Pharmaceutical Premises Regulations 2005***

These official gazettes specify critical pharmaceutical powers and limitations.

In 2005 (and revised in 2009), the PCN prepared, edited and published the **Four-part Compendium of Minimum Standards for the Assurance of Pharmaceutical Care in Nigeria**. This document prescribes the minimum standards required for the four specified areas of pharmacy practice. Similarly, the PCN developed the **Mandatory Continuing Professional Development (MCPD)** programme, now in its eighth module of the third cycle. The MCPD ensures that pharmacy practitioners remain current in their knowledge within this dynamic profession. Participation in the MCPD and the acquisition of necessary credits are prerequisites for annual licensure to practice pharmacy in Nigeria. In summary, the PCN ensures that a properly trained individual from an approved institution is matched with an appropriately registered, inspected, approved, licensed and monitored pharmaceutical premise. This system certifies that the standards are appropriate for the Nigerian public to access safe pharmaceutical care.

2.0 A Brief History of Pharmacy and Related Laws in Nigeria

The laws controlling pharmacy practice in Nigeria date back to 1878 with the ***Lagos Pilotage and Harbour Ordinance***, which regulated the application of medicines and medical treatment for persons on board ships docked in Lagos harbor. This was followed by the ***Hospital Ordinance of 1881*** and the ***Ereko Dispensary Rules of 1889***. In 1902, ***The Pharmacy Ordinance No. 8*** was enacted to control the sale and distribution of drugs and poisons, with application restricted to the colony of Lagos and specified protectorate towns. Under this ordinance, dispensers and druggists were examined and granted certificates to practice specific pharmacy-related vocations. Subsequently, the ***Poisons and Pharmacy Ordinance of 1936*** was enacted to fortify the registration of dispensers, trained chemists, and druggists. This was substantially amended by the ***Poisons and Pharmacy Act Cap 152 of 1958***.

The historical legal framework was definitively reaffirmed by the Nigerian courts. In the case of ***Nigerian Association of General Practice Pharmacist Employers (NAGPPE) Vs PCN (Suit No.***

FHC/KD/CP/8/2001), Justice Liman of the Federal High Court, Kaduna, held in 2001:

"I should say with the greatest respect, that both Counsel did not bother to familiarize themselves with the history of pharmacist and pharmaceutical chemists (in Nigeria) ... the Pharmacy Act 1958 was... repealed by Section 26(1) of the Pharmacists Council of Nigeria Decree 1992, No. 91... Effectively therefore, the only law regulating the practice of pharmacists in Nigeria today is Decree 91 of 1992." (Emphasis added).

This judicial pronouncement provides a definitive statement on the governing law for pharmacy practice in Nigeria.

3.0 The Registry of the Pharmacists Council of Nigeria

The Head Office and Registry of the PCN is located in the Idu Industrial Area of Abuja, Nigeria's Federal Capital Territory. The Registrar serves as its Chief Executive Officer (CEO). Since 2004, under the supervision of a 15-member Governing Council appointed by the President and Commander-in-Chief of the Federal Republic of Nigeria, the PCN has initiated a decentralization process. This initiative aims to extend the Council's regulatory and control mechanisms to rural and peripheral areas where the majority of Nigerians reside.

The organizational structure now includes:

- **A Head Office and Registry** in Abuja.
- **Eight Zonal Offices**, each representing one of the country's geopolitical zones: Lagos (South West), Enugu (South East), Uyo (South-South), Minna (North Central), Kaduna (North West), Bauchi (North East), and Abuja.
- **Thirty-five State Offices** operating as 'mini-Registries' across the Federation, supervised by the Zonal Offices.

Each State Office is staffed by at least one pharmacist assisted by an administrative officer to provide a basic administrative infrastructure for the PCN's state-wide functions. This structure has significantly improved the quality and speed of PCN's services nationwide.

3.1 Responsibilities of the Pharmacist-in-Charge of State Offices

The Pharmacist-in-Charge of each State Office is responsible for effectively carrying out the following functions:

- i. Processing applications for the renewal of pharmacists' annual licenses to practice.
- ii. Processing applications for the renewal of pharmaceutical premises licenses.
- iii. Coordinating the activities of the State Premises Inspection Committee (PIC).
- iv. Coordinating the activities of the State Patent and Proprietary Medicine Vendors License (PPMVL) Committee.
- v. Supervising and conducting visits to pharmacy internship training centres.
- vi. Participating in routine and new hospital pharmacy inspections with relevant Head Office or Zonal Office staff.
- vii. Participating in inspection visits to new and existing pharmaceutical manufacturing premises with relevant Head Office or Zonal Office staff.
- viii. Participating as an observer in the supervision of Pharmacy Technicians' final examinations alongside Council-approved external examiners.
- ix. Participating in the accreditation of pharmacy programmes in Faculties of Pharmacy located within the state as part of the PCN Secretariat staff.
- x. Participating in the accreditation of Pharmacy Technician programmes at Schools of Health Technology within the state.
- xi. Attending the induction and oath-taking ceremonies for new Bachelor of Pharmacy graduates at universities within the state as part of the Registrar's or Zonal Officer's team.
- xii. Overseeing the proper implementation and documentation of Mandatory Continuing Professional Development (MCPD) programmes in the state, often organized through the State Branch of the Pharmaceutical Society of Nigeria (PSN).

- xiii. Overseeing the proper implementation and documentation of orientation or continuous education programmes for licensed patent medicine vendors in the state.
- xiv. Participating as part of the Registrar's team during Orientation Programmes for Foreign-trained Pharmacists at Faculties of Pharmacy within the state.
- xv. Receiving all enquiries and complaints from stakeholders and promptly dispatching them to the Registrar through the Zonal Officer-in-Charge. This includes managing the established hotline: 0806 605 5429.
- xvi. Ensuring the proper implementation of the State Office's administrative and financial responsibilities in conformity with PCN guidelines.
- xvii. Protecting the image and corporate status of the PCN.
- xviii. Carrying out any other professional or administrative duties as directed by the Registrar or Zonal Pharmacist-in-Charge (after due clearance with the Registrar).

3.2 The Responsibilities of the Pharmacist-in-Charge of Zonal Offices

The eight Zonal Offices supervise the professional and administrative functions of the State Offices within their jurisdiction and report directly to the Registrar. Their responsibilities include:

- i. Being directly responsible to the Registrar.
- ii. Ensuring the proper implementation of all professional and administrative responsibilities of the PCN in all State Offices within the zone by:
 - * Maintaining proper and timely contact with PCN staff in State offices to ensure they carry out their functions effectively.
 - * Ensuring the availability of Council's inspection vehicles to the States as needed and keeping the Directors of Pharmaceutical Services (DPSs) informed.

- * Ensuring the availability of appropriate working materials, such as stationery, basic furniture and equipment in good working order.
- * Fostering an excellent working relationship between State PCN Offices and State Ministries of Health, particularly the State Pharmaceutical Services Departments.
- iii. Coordinating the incomes and expenditures of all PCN State Offices in the zone on a regular basis and promptly reporting the outcomes to the PCN Head Office/Registry.
- iv. Holding regular follow-up meetings with the DPSs to ensure the timely implementation of:
 - Orientation programmes for licensed patent medicine vendors.
 - Continuous Education Programmes for eligible patent medicine vendors.
- v. Holding regular follow-up meetings with Deans of Faculties of Pharmacy in the zone regarding the timely implementation of MCPD programmes through the relevant State Offices.
- vi. Liaising with Honourable Commissioners for Health and other relevant State officials to build synergy for effective regulation of the pharmacy profession in accordance with Act 91 of 1992.
- vii. Representing the Registrar or the PCN at social or professional activities in any state within the zone, as directed.
- viii. Submitting separate, quarterly reports to the Registrar on the professional activities of every state in the zone.
- ix. Participating in PCN's periodic Management Committee meetings, as convened with the Registrar's approval.
- x. Convening meetings of the Officers-in-Charge of State PCN Offices, after obtaining necessary clearance and approval from the Registrar.

4.0 Knowledge Management in PCN Functions

The PCN is committed to applying modern Knowledge Management (KM) principles to its core functions, Registration, Control,

Monitoring, Regulation, Education and Training, to enhance the quality of pharmacy practice in Nigeria.

As a first step, the PCN commissioned an internationally renowned Nigerian KM consultancy firm to standardize all its publications. The result has been the creation of three distinct publication series:

- **Registration and Control (ReC) Titles**
- **Monitoring & Regulation (MoR) Titles**
- **Education and Training (EdT) Titles**

Each publication is assigned a unique code within its series, allowing for up to 999 titles per series. This standardization ensures all 20 PCN publications meet UNESCO and international standards. The PCN is proud of this achievement, which positions it as a pacesetter for other Nigerian public corporations in KM. This logically structured system makes it significantly easier for pharmacy practitioners, related workers, educators and trainers to search, locate, cite, recall and purchase PCN publications. A full, classified list of standardized publications to date is shown in below, with more titles scheduled for future publication.

S/NO	CODE	FULL TITLE & ISSN
1.	PCN.Rec.001	The establishment of the Pharmacists Council of Nigerian Decree (No. 91 of 1992). Reprinted. 2009. (ISBN: 708-2397-56-3).
2.	PCN.Rec.002	Conditions of Service for Employees of the Pharmacist Council of Nigeria 2 nd ed., 2009. (ISBN: 978-2397-64-4)
3.	PCN.Rec.003	A Compilation of Pharmacy, Drugs and Related Laws and Rules in Nigeria, 1935-2000. 24 ed., 2009. (ISBN: 978-2397-96-2)
4.	PCN.Rec.004	PCN's Approved Patent Medicine List 4 th ed., 2009, (ISBN: 978-2397-72-5)
5.	PCN.Rec.005	Procedures for the Registration of Pharmacist and Pharmaceutical Premises 2 nd ed., 2009 (ISBN: 978-2397-80-6)
6.	PCN.Rec.006	Meeting the Leadership Challenge of a Repositioned PCN, the Council's 2003 – 2007 Scorecard. 2 nd ed., 2009. (ISBN: 978-2397-17)

B. Monitoring and Regulation (MoR) Titles

S/NO	CODE	FULL TITLE & ISSN
	PCN.MoR.001	PCN's Guidelines for the Issuance of the Patent and Proprietary Medicines Vendor's Licence. 24 ed., 2009. (ISBN: 978-2397-25-3)
	PCN.MoR.002	Code of Ethics for Pharmacist in Nigeria. 2 nd ed., 2009. (ISBN: 978-2397-33-4)
	PCN.MoR.003	PCN's Manual for Pharmaceutical Inspectors in Nigeria. 2 ed., 2009. (ISBN: 978-2397-41-5)
	PCN.MoR.004	The PCN Sourcebook on Pharmacy Regulation in Nigeria. 2d ed., 2009 (ISBN: 978-2397-49-0)
	PCN.MoR.005	The four-part compendium of minimum Standards for the Assurance of Pharmaceutical Care in Nigeria. 2d ed., 2009. (Hardback & Paperback editions both in Matt Finis). (ISBN: 708-2397-66-0) (Handback) (ISBN: 987-2397-58-x) (Limp)

C. Education and Training (EdT) Titles

S/No	Code	Full Title & ISSN
12.	PCN.EdT.001	Akubue, Paul I, and Fred B. Adenika (eds). <i>Pharmacy Education in Nigeria: Proceedings of the Pharmacy Curriculum Conference, organized by the Pharmacists Council of Nigeria, August 28-30, 2001. 24 reprint, 2009.</i> (ISBN: 9782397-65-2).
13	PCN.EdT.002	PCN Lecture Notes for the Orientation Programme of Patent Medicine Vendors in Nigeria. 2d ed., 2009. (ISBN: 978-2397-73-3).
14	PCN.MoR.003	PCN's Guidelines for the Establishments of the Bachelor of Pharmacy Programme in Nigerian Institutions. 3 rd ed., 2009 (ISBN.978-2397-81-4).

15	PCN.MoR.004	PCN's Manual for the Foreign Pharmacy Graduate Orientation Programme (FPGOP). 2na ed., 2009. (ISBN: 978-2397-89-X).
16	PCN.MoR.005	PCN's Internship Training Programme for Pharmacy Graduates in Nigeria. 2d ed., 2009. (ISBN: 978-2397-97-0)
17	PCN.MoR.006	PCN's Internship Logbook for Fresh Pharmacy Graduates in Nigeria. 2 nd reprint, 2009. (ISBN:978-2397-18-0)
18	PCN.MoR.007	PCN's Training Manual for Pharmacy Technicians in Nigeria. 3 rd ed., 2009. (ISBN:978-2397-26-1)
19	PCN.MoR.008	PCN's Continuing Education Programme (CEP) for Holders of the Patent and Proprietary Medicines Vendor's Licence (PPMVL) in Nigeria. 2d ed. 2009.
20	PCN.MoR.009	PCN's Mandatory Continuing Professional Development Programme (MCPD) for the Recertification of Pharmacist in Nigeria. 3rd ed., 2009 (ISBN:978-2397-42-3).

5.0 Addressing PCN's Regulatory Challenges

Nigeria is a developing country with a population exceeding 140 million people. It suffers from some of the world's poorest health indices, including high rates of infant and maternal mortality, a high HIV/AIDS prevalence, low life expectancy, endemic malaria and a resurgence of tuberculosis. These issues are a major concern for all healthcare professionals, both within Nigeria and abroad, including members of associations like NAPPSA. The country's healthcare infrastructure is inadequate for its population's needs, particularly in rural areas. Health budgets across federal, state and local government levels, as well as in the private sector, are insufficient and often poorly implemented, limiting their impact. Most critically, there is a severe shortage of trained human resources to manage existing health facilities and to correct the skewed distribution and mix of healthcare professionals across the country.

Pharmacists are in critically short supply. As of December 2008, only 13,775 pharmacists were registered with the PCN, with just 5,489 renewing their annual licenses. This translates to a ratio of approximately one pharmacist for every 10,163 Nigerians based on a population of 140 million, far below the World Health Organization (WHO) recommended ratio of one pharmacist per 5,000 people. The output of the nine approved Schools of Pharmacy is low, producing only 667 pharmacists in 2008. While eight more universities have commenced pharmacy programmes at various accreditation stages, the current deficit presents significant regulatory challenges, which can be grouped into six categories:

- i. **Inadequate Number of Pharmacists:** The shortage of qualified professionals creates a vacuum easily filled by quacks and charlatans, making enforcement of rules and regulations by the limited PCN staff and their Nigeria Police Force collaborators extremely challenging.
- ii. **Inadequate Number of Registered Pharmacy Outlets:** The scarcity of licensed retail, wholesale, hospital and manufacturing premises leads to the proliferation of illegal outlets, overstretching the capacity of pharmaceutical inspectors.
- iii. **Weak Legal and Punitive Measures:** The existing corrective measures often allow offenders within the profession to escape with minor punishments, undermining regulatory authority.
- iv. **Insufficient Staff, Volunteers and Funding:** There is an inadequate number of staff and volunteer inspectors, coupled with limited funds, hindering regular and effective inspections necessary to maintain minimum standards.
- v. **Need for Enhanced Training:** There is an urgent need for

regular, effective training for inspectors and PCN staff in modern inspectorate concepts, manufacturing practices and accreditation standards. Access to overseas training facilities is crucial to ensure inspectors are more knowledgeable than the personnel they assess.

- vi. Weak Cooperation from Stakeholders:** Some pharmacists and stakeholders responsible for providing pharmaceutical care exhibit inappropriate or weak cooperation, failing to adhere to stipulated standards. For instance, participation in Mandatory Continuing Professional Development (MCPD) programmes is sometimes met with legal challenges that distract from and negatively impact programme outcomes.

6.0 Concluding Remarks

Benchmarking and standard-setting for the training of pharmacists and pharmacy technicians and for pharmacy practice itself constitute the fundamental responsibility of the PCN. The enforcement of these standards is essential to ensure that practice in training institutions and pharmaceutical premises conforms to specified norms. The PCN cannot and will not deviate from this core commitment. The Council faces an onerous task in fulfilling its statutory mandate. Despite the challenges, its leadership and staff are determined to succeed, bolstered by the goodwill of all stakeholders, including NAPPSA members. This determination and stakeholder support must be maintained and strengthened to protect patients and ensure the safety of pharmaceutical consumers nationwide. If this is done promptly, correctly, and efficiently, the pharmaceutical inspector, as an enforcer of duly constituted regulations, will have effectively discharged their responsibility to the nation.

May God continue to guide and bless our efforts to move Nigeria

forward in this critical endeavour. Thank you for your attention.

Bibliography of Sources Used

1. PCN. (2009). *The PCN Sourcebook on Pharmacy Regulation in Nigeria* (MoR.004, 2nd ed.). ISBN: 978:2397-49-0.
2. PCN. (2009). *A Compilation of Pharmacy, Drug and Related Laws and Rules in Nigeria, 1935-2000* (ReC.003, 2nd ed.). ISBN: 978:2397-96-2.
3. PCN. (2009). *The Four-part Compendium of Minimum Standards for the Assurance of Pharmaceutical Care in Nigeria* (MoR.005, 2nd ed.). ISBN: 978:2397-66-0 (Hardback), 978:2397-58-X (Limp).
4. PCN. (2009). *PCN's Manual for Pharmaceutical Inspectors in Nigeria* (MoR.003, 2nd ed.). ISBN: 978:2397-41-.
5. Mora, A. T. (2007). *Laws and Regulations Governing Pharmacy Practice in Nigeria and Reciprocity*. [Unpublished lecture]. First Convention of NAPPSA, Houston, Texas, USA.
6. Mora, A. T. (2008). *The Nigerian Pharmacist, Medication Errors and the Regulation and Control of the Pharmacy Practice by the Pharmacists Council of Nigeria (PCN)*. [Unpublished lecture]. Second Convention of NAPPSA, Atlanta, Georgia, USA.

LEARNING OBJECTIVES

Upon completion of this presentation, the audience will be familiar with:

- a. The major provisions of *Act 91 of 1992* that establish the PCN as the agency responsible for regulating all aspects of pharmacy practice in Nigeria.
- b. A brief history of pharmacy laws in Nigeria from 1878 to the present.
- c. The legal instruments available for the proper regulation of pharmacy practice.
- d. The managerial structure at PCN Headquarters and its State and Zonal Offices nationwide, designed to ensure pharmaceutical premises are registered, inspected, approved, licensed and monitored to provide effective care under the supervision of PCN-licensed pharmacists.
- e. The PCN's commitment to applying Knowledge Management (KM) principles, evidenced by the standardization of twenty (20) PCN titles to date.
- f. The key challenges currently facing the regulation and control of pharmacy practice in Nigeria.

Parm A.T. Mora

September 17-20, 2009

**Remarks by the Chairman of the Committee of Registrars of
Health Professions (CRHP), presented by Pharm. Ahmed
Tijjani Mora at the Retreat of Members of the CRHP held at
the Yankari Game Reserve, Bauchi State, from January 18–19,
2011.**

Preamble

It is my pleasure to welcome every member of the Committee of Registrars of the Health Professions (CRHP) to our third retreat here at the famous **Yankari Game Reserve, Bauchi State**. This retreat includes four new members who joined the CRHP since our last gathering at **Obudu Cattle Ranch and Resort, Cross River State, on October 7–8, 2009**. These are:

- Mr. Shiono Bennibor – Registrar/CEO, Community Health Practitioners Registration Board of Nigeria (CHPRBN)
- Mr. Samuel Akume – Registrar/CEO, Dental Therapists Registration Board of Nigeria (DTRBN)
- Prof. David Segun Oluleye – Registrar/CEO, Institute of Public Analysts of Nigeria (IPAN)
- Prof. Anthony Emeribe – Registrar/CEO, Medical Laboratory Science Council of Nigeria (MLSCN)

Included in our agenda is a dinner in honor of **five retired members**: Mallam Bashir Umar, Mrs. Mabel John, Alhaji Bola Haruna, Lady E. U. Okonkwo and Dr. Rowland Eyime. We congratulate them on their distinguished service and wish them happy retirement, hoping that we, too, may exit our duties with the same honor. This retreat is organized by **GSCL Consulting, Abuja**, a multidisciplinary firm specializing in advisories, performance improvement and outsourcing. The theme is:

**“The Need for Minimum Standards of Practice by
Health Regulatory Agencies as an Instrument for
Assuring Effective Health Care Regulation in Nigeria.”**

Health care professions are dynamic worldwide, as are their regulations, which remain our primary responsibility. In a fast-changing national environment, measurable standards are essential

for Councils, Boards and Institutes to evaluate the quality of the health care professions they regulate. This understanding informed the choice of our retreat topic. Additionally, the public, whose health care we regulate, deserves the highest standards as professional practices become increasingly sophisticated. We look forward to learning and discussing these concepts with the Consulting Firm.

History of Our Retreats

The **first CRHP retreat** was held in Asaba, Delta State, on **April 10–13, 2008** in honor of Dr. Rowland Eyime, then the Honourable Commissioner for Science and Technology, Delta State and former Registrar of the Optometrists and Dispensing Opticians Board of Nigeria. The theme was:

“Brainstorming on Ways to Improve Health Care Delivery Services in Nigeria.”

The **second retreat** took place at Obudu Cattle Ranch and Resort, featuring Prof. W. O. Aiyepku, an international consultant, who presented on: **“Empowering Knowledge Workers for Organizational Success: The Advantages of a Knowledge Management-Savvy Workforce.”**

Our **first workshop** was on **July 29, 2008**, at the Sheraton Hotel, Abuja, themed:

“The Role of Health Regulatory Bodies in Advancing Healthcare Delivery Services in Nigeria.”

Capacity-building initiatives extended internationally. In **Geneva, Switzerland (February 18–19, 2010)**, some members attended the **Second World Health Professions Conference on Regulations** organized by the **World Health Professions Alliance (WHPA)**, with the theme: **“Shaping the Future: The Regulation of Health Professionals.”**

These efforts underscore the importance of equipping ourselves with the latest professional knowledge. As custodians of laws governing health professions, we must model **Continuous Professional Development (CPD)** for our members. Currently, a request is before the Honourable Minister of Health for approval for members to attend a

2-week Professional Development Programme in Strategic Development Policy at Liverpool Hope University, UK (February 14–25, 2011). We remain hopeful for approval, as was granted for the Geneva event in 2010.

Responsibilities as Registrars

Since 1999, public service reforms in Nigeria have increased the demand for effective positioning of our Registries, requiring us to execute our regulatory and control functions efficiently. As I stated on **October 8, 2009, at Obudu:**

“The recent structural reforms in Nigeria’s Public Service have placed huge responsibilities on our shoulders that must be confidently and efficiently discharged, especially regarding our new powers of procurement. Our enhanced responsibilities and status place us in the spotlight, attracting potential envy and scrutiny from professional associations, staff, and members of our Governing Councils, Boards, or Institutes.”

These observations remain relevant today. Some of us have faced challenging times with staff, professional associations or governing bodies. While critics will always critique, CRHP provides a platform to **share ideas, learn from each other, and enhance health care services in Nigeria.** An effective health care delivery system requires a mix of professional expertise. With cooperation and shared knowledge, we can meet these challenges.

I wish all participants a **successful and fruitful retreat** here at **Yankari Game Reserve**, Nigeria’s foremost game reserve. From Abuja, Lagos, Asaba and Obudu to Geneva, our past retreats have been successes. This retreat will be no exception. **Next stop: Liverpool.**

Thank you for your attention, and God bless.

Pharm. Ahmed T. Mora, *nni*
Registrar, PCN and Chairman, CRHP
January 19, 2011

The Health Regulatory Agencies as Instruments for Professional Harmony in Nigeria. Presented by Pharm. Ahmed Tijjani Mora at a Workshop organised by the Committee of Registrars of Health Professions (CRHP) at the Sheraton Hotel, Abuja, on Thursday, March 31, 2011.

1.0 Introduction

Health Regulatory Agencies in Nigeria operate under the **Federal Ministry of Health (FMOH)** and were established by Acts of the National Assembly to regulate and control the training and practice of health care professions. Presently, there are **thirteen such agencies**:

1. Pharmacists Council of Nigeria (PCN)
2. Medical and Dental Council of Nigeria (MDCN)
3. Institute of Public Analysts of Nigeria (IPAN)
4. Medical Laboratory Scientists Council of Nigeria (MLSCN)
5. Nursing and Midwifery Council of Nigeria (NMCN)
6. Institute of Chartered Chemists of Nigeria (ICCON)
7. Dental Therapists Registration Board of Nigeria (DTRBN)
8. Medical Rehabilitation Therapists Registration Board of Nigeria (MRTB)
9. Optometrists and Dispensing Opticians Registration Board of Nigeria (ODORBN)
10. Dental Technologists Registration Board of Nigeria (DTRBN)
11. Radiographers Registration Board of Nigeria (RRBN)
12. Health Record Officers Registration Board of Nigeria (HRORBN)
13. Community Health Practitioners Registration Board of Nigeria (CHPRBN)

Each agency is headed by a **Registrar (CEO)** appointed as a public officer of the Federal Republic of Nigeria (FRN). The agencies also have **Governing Councils or Boards** with distinguished

professionals under a Chairman appointed by the President, Commander-in-Chief of Nigeria.

- The **Pharmacists Council of Nigeria (PCN)** has the largest Governing Council with 63 members.
- The **Community Health Practitioners Registration Board** has the smallest with 11 members.

(Table 1: Number of Governing Council Members for each agency)

S/NO	Institution	No. Of Governing Council Members
1.	Pharmacists Council of Nigeria (PCN)	63
2.	Medical and Dental Council of Nigeria (MDCN)	61
3.	Institute of Public Analysts of Nigeria (IPAN)	20
4.	Medical Laboratory Scientists Council of Nigeria (MLSCN)	1
5.	Nursing and Midwifery Council of Nigeria	18
6.	Institute of Chartered Chemists of Nigeria	17
7.	Dental Therapists Registration Board of Nigeria (DTRBN)	17
8.	Medical Rehabilitation Therapists Registration Board of Nigeria (MRTB)	17
9.	Optometrists and Dispensing Opticians Registration Board of Nigeria (ODORBN)	15
10.	Dental Technologists Registration Board of Nigeria (DTRBN)	13
11.	Radiographers Registration Board of Nigeria (RRBN)	12
12.	Health Record Officers Registration Board of Nigeria (HRORBN)	12
13.	Community Health Practitioners Registration Board of Nigeria (CHPRBN)	11

Health Regulatory Agencies and the CRHP

These agencies are tasked with making regulations in accordance with the Acts establishing them, defining and advancing the training and practice of members of their respective professions.

2.0 Setting Up of the Committee of Registrars of the Health Professions (CRHP)

August 19, 2004 is a landmark date for healthcare professions in Nigeria and the West African region. On this day, the then Honourable Minister of Health, **Professor Eyitayo Lambo**, inaugurated the CRHP.

The objectives of the CRHP are to:

- Raise the level of ethical conduct in healthcare delivery in Nigeria;
- Set targets for efficient performance;
- Facilitate Continuing Professional Development (CPD) for health professions;
- Enlighten the general public on health matters;
- Harmonize regulatory activities for efficiency and effectiveness in the diverse health professions;
- Act as a platform for synergy among various government health care efforts by harnessing the unique strengths of different professions; and
- Foster unity and mutual respect among health professions.

During the inauguration, the Minister stated:

“It is heart-warming that the Registrars of all regulatory bodies in the health sector are coming together under one umbrella. This is commendable and should be applauded. I urge you to use this Committee as a springboard to rejuvenate the team spirit that is fast fading away.” (Lambo, 2004)

The expectations from the Registrars are therefore significant. However, they cannot operate independently in regulating health professions. Collaboration, rather than working along parallel tracks, benefits patients and the healthcare system immensely (CRHP, 2008). Prince Julius Adelusi-Adeluyi, the distinguished pharmacist and former Honourable Minister of Health, emphasized during the first CRHP workshop on **July 29, 2008**:

“There is an urgent need for a strong advocacy of harmony among members of the healthcare team.” (CRHP, 2008)

These statements testify to the relevance of this workshop and the chosen theme.

3.0 Activities of the CRHP from Inception to Date

The CRHP members are well-versed in implementing the mandates of their respective Governing Councils/Boards. To enhance cohesion, harmony and understanding, the Committee designed activities to improve their capacities as Registrars and CEOs. Major activities include:

1. **First Retreat:** Asaba, Delta State (April 10–13, 2008), honoring Dr. Rowland Eyime, then Honourable Commissioner for Science and Technology. Theme: *“Brainstorming on Ways to Improve Health Care Delivery Services in Nigeria.”*
2. **Second Retreat:** Obudu Cattle Ranch and Resort, Cross River State. Prof. W. O. Aiyepku presented on: *“Empowering Knowledge Workers for Organizational Success: The Advantages of a Knowledge Management-Savvy Workforce.”*
3. **Third Retreat:** Yankari Game Reserve, Bauchi State (January 18–19, 2011), organized by **GSCL Consulting, Abuja**. Theme: *“The Need for Minimum Standards of Practice by Health Regulatory Agencies as an Instrument for Assuring Effective Health Care Regulation in Nigeria.”*
4. **First Workshop:** Sheraton Hotel, Abuja (July 29, 2008). Theme: *“The Role of Health Regulatory Bodies in Advancing Healthcare Delivery Services in Nigeria.”*
5. **Capacity-Building Activities:** Geneva, Switzerland (February 18–19, 2010), attending the Second World Health Professions Conference on Regulations organized by the **World Health Professions Alliance (WHPA)**. Theme: *“Shaping the Future: The Regulation of Health Professionals.”*
6. **Professional Development:** Liverpool Hope University, UK (March 14–25, 2011), attending a **2-week Strategic Development Policy Workshop**.

Healthcare professions and their regulations are dynamic. This fast-changing environment necessitates measurable standards for Councils, Boards, and Institutes to evaluate the quality of the professions they regulate.

4.0 Lessons Learnt

The primary aim of regulatory bodies is to regulate and control the training and practice of their members. In achieving this, the focus has always been to:

- Improve patient health;
- Develop and maintain the healthcare delivery system; and
- Enhance professional competencies and skills through CPD programs.

There are no quick fixes. Health professionals operate interdependently:

- Medical practitioners rely on accurate documentation from Health Record Officers.
- Laboratory Scientists ensure correct diagnosis.
- Nurses rely on Pharmacists for proper drug administration.
- Optometrists coordinate with Ophthalmologists.
- Radiographers provide accurate imaging for surgeons.
- Dental Technologists and Therapists collaborate.
- Community Health Officers make crucial referral decisions in emergencies.
- Chemists ensure proper analysis to prevent harmful interactions.

The proximity of regulatory agencies, such as in the **Federal Capital Territory (FCT), Abuja**, enhances collaboration, particularly where responsibilities overlap.

5.0 Recommendations

5 a. Key Recommendations

1. CRHP members should continue organizing activities to foster unity among themselves, their regulatory functions and the overall interests of patients.
2. CRHP and the Presidents of national health professional associations should meet periodically to improve understanding and elevate healthcare delivery.
3. The Federal Government, via the Ministry of Health, should increase funding to health regulatory agencies.
4. The FCT should allocate suitable land in Abuja for regulatory agencies still operating in Lagos, with financial support from the Ministry of Finance for constructing Headquarters.
5. The Ministry of Health should continue to support CRHP and its members to ensure efficient discharge of responsibilities.

5 b. Implementation Strategies

1. Continue organizing workshops and activities to entrench harmony among health professions.
2. CRHP should liaise with national health professional associations to convene periodic meetings.
3. Increase funding for health regulatory agencies during budget cycles.
4. Approve suitable land in Abuja for agencies still based in Lagos to build Head Offices and relocate.
5. The Honourable Minister of Health should continue motivating CRHP members and regulatory bodies for effective performance.

6.0 Conclusion

In conclusion, **Adair's Circle** provides a model for regulatory bodies:

- Achieve their health regulatory mandate;
- Build and maintain the professional health team;
- Develop individual professionals through CPD programs (Weir, 2011).

Effective implementation of these principles ensures the harmony of healthcare professionals and strengthens the overall healthcare delivery system in Nigeria.

Thank you all for listening, and God bless.

References

- CRHP (2008). *The Role of Health Regulatory Bodies in Advancing Healthcare Delivery Services in Nigeria — Proceedings of a One-Day Advocacy Workshop*, Ibadan.
- Lambo, E. Prof (2004). Address on the Inauguration of the Committee of Registrars of the Health Professions (CRHP), August 19, 2004, SGF Conference Room, Federal Secretariat, Abuja.
- Weir, D. (2011). *Cultural Diversity and Change Management in Public Services*, Liverpool Hope University, Liverpool, March 16, 2011.

**An Overview of the Regulatory Functions of the Pharmacists
Council of Nigeria: Inspectorate Activities. Presented by
Pharm. Ahmed T. Mora, Registrar and CEO of the PCN, at the
Pharmaceutical Inspectors' Workshop organised by the PCN at
the Raw Materials Research and Development Agency, Abuja,
from July 5–6, 2011.**

1.0 Introduction

If one were to summarize the primary activity of the Registry staff of the Pharmacists Council of Nigeria (PCN), it would be **inspection and accreditation**:

- Inspection of pharmaceutical premises where drugs and medical devices are manufactured, stored, distributed and/or offered for sale to the public.
- Accreditation of the Bachelor of Pharmacy (B. Pharm) programme in approved universities (Schools of Pharmacy), as well as pharmacy technician training in Schools of Health Technology.

For the PCN to effectively discharge its regulatory and control functions, pharmaceutical premises must be **registered, inspected, approved and licensed**. Such premises cannot be considered licensed unless they are under the supervision of a pharmacist, who has graduated from an accredited School of Pharmacy in Nigeria—or its recognized equivalent abroad—and designated as the superintendent pharmacist. This paper expands on the regulatory functions of the PCN with particular focus on **Inspectorate activities**, as they relate to pharmaceutical premises involved in production, storage, distribution, dispensing and sale of medicines and medical devices.

The process from a pharmacist's intention to establish a practice to the stage of its licensure by the PCN is detailed and rigorous. It involves key actors (inspectors), activities (inspections), objects (products) and facilities (premises). The **National Drug Policy**

(NDP, 2005) underscores this when it states that “*the effective inspection of drug products and manufacturing facilities is critical to ensuring the quality, safety, and efficacy of drugs.*” Within this regulatory framework lies the scope of inspectorate activities of the PCN.

2.0 Conceptual Clarification

2.1 Who is a Pharmaceutical Inspector?

A Pharmaceutical Inspector is a registered pharmacist, who has undergone organized, approved training and is formally designated by the PCN (Brown, 1999). According to the **World Health Organization (WHO, 2007)**, inspectors should normally be pharmacists with experience in community and/or hospital pharmacy.

2.2 The Attributes of an Inspector (WHO, 2007)

An inspector must possess:

- Strong knowledge of pharmacy, drugs, and poisons.
- Sound understanding of relevant laws and regulations.
- Command of technical terms and excellent communication skills.
- Ability to detect forged or falsified documents.
- Maturity, honesty, and integrity.
- Responsible conduct that commands respect.
- Capacity to organize work with minimal supervision.
- Ability to make quick, rational, and sound decisions.
- Strong interpersonal skills with firmness and fairness.
- Ability to hold post-inspection discussions with management.
- Motivation, stamina, and commitment to long hours.
- Ethical conduct, avoiding conflicts of interest.

2.3 Definition of Pharmaceutical Inspection

Pharmaceutical inspection is a **planned system of activities** aimed at ensuring that drugs, whether manufactured locally, imported, dispensed in health institutions or sold in pharmacies are of acceptable quality, safe and efficacious for diagnosis, prevention and treatment of diseases in humans and animals (Brown, 2008). In essence, it is the activity undertaken by officially designated PCN inspectors to ensure that practice in licensed premises conforms to **established standards**.

Key activities include:

- a) Inspection of drug manufacturing industries.
- b) Inspection of imported drugs at ports of entry.
- c) Inspection of pharmaceutical outlets (wholesale, retail, importation).
- d) Inspection of hospital pharmacies (PCN, 2009).
- e) Inspection of the pharmaceutical distribution chain by the **Drug Distribution Inspection Committee (DDIC)**, since July 2008.

3.0 Inspectorate Activities of the PCN

3.1 General Scope

The primary purpose of inspection is to ensure that the superintendent pharmacist and other staff of licensed premises deliver appropriate pharmaceutical care to the public. Inspection is not limited to initial licensing. It must also be **periodic**, ensuring compliance with minimum standards and ethical practice. The ***Federal Republic of Nigeria Official Gazette No. 79, Vol. 92 (2005)*** empowers inspectors to:

- Enter premises where pharmaceutical products are manufactured, stored, packaged or sold.
- Examine articles, records, and facilities.
- Collect samples or specimens for analysis.

- Open and examine containers or packages.
- Review documents and records relevant to compliance.
- Seize and detain articles suspected of contravening regulations.

3.2 Retail/Wholesale/Distribution/Importation Premises

These outlets provide the public with access to medicines, poisons and devices. Inspections are conducted under a **Public–Private Partnership (PPP)** framework, with pharmacists of proven integrity co-opted into the process.

The PCN's *Manual for Pharmaceutical Inspectors in Nigeria* reminds inspectors that “*to whom much power is given, much responsibility is expected.*” Inspections should not be limited to identifying faults, but should be an opportunity to assist, guide and motivate practitioners toward compliance with **Good Manufacturing Practice (GMP)** and **Good Pharmacy Practice (GPP)**.

3.3 Hospital Pharmacies

Hospital pharmacies operate within health institutions primarily to fill prescriptions and provide services, such as compounding, drug information and formulary management. The **Registration of Pharmaceutical Premises Regulations (2008)** mandates that every hospital pharmacy be registered with the PCN. The **PCN's Compendium of Minimum Standards** specifies staffing requirements based on bed capacity at primary, secondary and tertiary healthcare levels. These standards serve as benchmarks during inspection. In 2009, the PCN convened a workshop to develop consensus statements on hospital pharmacy practice in Nigeria, aligning with global trends to shift pharmacy's focus from *products* to *patients*.

3.4 Pharmaceutical Manufacturing

The focus here is compliance with **current Good Manufacturing Practice (cGMP)**. The inspection of manufacturing plants requires specialized expertise, as it covers diverse process areas. Between 2004 and 2009, the PCN invested significantly in training its inspectors, both locally and internationally, ensuring they acquired the skills necessary to evaluate pharmaceutical plants to the highest standards.

Table 1: Major Efforts in the Training and Development of PCN Pharmacists in Inspection of Pharmaceutical Industries

S/No	ACTIVITIES	PERIOD	Venue	Remarks
1	Meeting with Superintendent and Production Pharmacists of Pharmaceutical Manufacturing Industries in Nigeria.	October 16, 2004	Airport Hotel Ikeja, Lagos	
2	PCN Factory Inspection Training by Nigerian German Chemicals (NGC) for Superintendent and Production Pharmacists and registry Staff	September 26-20, 2005	Lagos Office, NGC Premises Lagos	Pharm. Yomi Osewa facilitated
3	Workshop for manufacturer and Importers of Pharmaceutical Products towards evolving a systematic distribution system	August, 2006	Sheraton Hotel, Abuja	The Honourable Minister of Health, Prof. Eytayo Lambo attended as Special Guest of Honour
4	Training on Fundamental plus essential of validation and Pharmaceutical water system by Rodot consulting for Registry Staff	December 2006	Sheraton Hotel, Ikeja, Lagos	For staff of PCN and NAFDAC
5	Good Manufacturing Practice (GMP training for PCN Inspectors	July 19 -20, 2008	PCN Head Office/Registry Idu, Abuja	Pharm. Yomi Osewa facilitated
6	WORKSHOP FOR TEN	June 22-25, 2009	Otta, Ogun	22 People were

	(10) Staff of the PCN by the World Health Organization (WHO) on Good Pharmaceutical Distribution		State	in attendance, 10 pharmacist from PCN, 7 Pharmacists from NAFAC, 3 Chemists and Biochemists and 3 Consultant from WHO
7	Training for Pharmaceutical Inspectors Staff of PCN on aspect of Current Good Manufacturing Practice (cGMP)	Lagos (July 13 th)	Multipurpose Centre	88 Pharmacists staff of PCN attended. Pharm. Yomi and Dr. Echeazu Ogu were trainers
		Abuja (July 23 rd)	Sheraton Hotel	
		Enugu (July 27 th 2009)	Nike Lake Hotel	
8	Two days advocacy workshop for CEO's superintend Pharmacists in Nigeria's Pharmaceutical Manufacturing Industries on c GMP	Lagos (July 14 th -15 th , 2009)	Multipurpose Centre	Total of 29 CEOs, 134 Superintendent Pharmacists and 11 Production Pharmacists attended.
		Abuja (July 21 st - 23 rd)	Sheraton Hotel	
		Enugu (July 27 th -29 th ,2009)	Nike Lake Hotel	
9	The Deputy Director, Inspection and Monitoring participated in a workshop on "United Nations Prequalification of Medicines, Diagnostic and Vaccines" at the 6 th Consultative Stakeholders Meeting with manufactures on the invitation of NAFDAC	April 4-5, 2011	Geneva switzerl and	

3.5 Drug Distribution

Drug distribution as a process requires close inspection. Any properly manufactured or imported pharmaceutical product registered with **NAFDAC** must only be distributed within premises that have been **inspected, approved, registered and licensed** by the PCN.

The **National Drug Policy (NDP, 2005)** provides explicit guidance on rational drug distribution:

- i) Drug distribution, supply, sale and dispensing shall be under the control and supervision of pharmacists at all levels.
- ii) Government shall ensure that drug manufacturing, wholesaling and retailing activities are registered as distinct enterprises.
- iii) In the private sector, the distribution channel shall flow from manufacturers/importers → wholesalers → retailers.
- iv) Supplies to public health facilities shall be based on expressed need, primarily through the Central Medical Stores.
- v) All drugs purchased or donated to government at any level must be channeled through the Central Medical Stores.
- vi) Hospitals and clinics shall maintain inventory control systems, including computerized records linked to the Central Medical Stores.
- vii) Adequate security must be provided for storage areas, especially for narcotic drugs.
- viii) All drugs distributed in the country shall be labeled in English.
- ix) Incentives shall be created to encourage pharmacists to establish practices in rural areas. Professional associations and regulatory bodies should also support rational drug use and greater access to quality medicines.

In line with this, the **Drug Distribution Inspection Committee (DDIC)** was inaugurated on July 2, 2008. Its focus is safeguarding patients and the public by ensuring:

- Protection against malpractice by distributors and suppliers.

- Strict adherence to drug laws governing compounding, distribution, importation, exportation, and storage.
- Maintenance of high ethical and professional standards.

The ***Federal Republic of Nigeria Official Gazette (No. 81, Vol. 92, 2005)*** further mandates proper record-keeping across all levels of the pharmaceutical distribution chain, requiring manufacturers, importers, wholesalers and retailers to keep accurate and auditable records of purchases, sales, prescriptions, production and quality control. To date, the DDIC has inspected **86 premises** (retail, wholesale and manufacturing) across **13 States of the Federation**.

3.6 Patent and Proprietary Medicine Shops (PPMS)

The inspection of PPMS outlets is carried out nationwide by the **Patent and Proprietary Medicine Vendors Licence (PPMVL) Committees**, chaired by the Directors of Pharmaceutical Services (DPSs). The approved drug list serves as a guide for the medicines permitted in these shops. However, numerous challenges persist and inspectors remain familiar with these difficulties during field operations.

4.0 Challenges to Pharmaceutical Inspectors in Discharging Their Regulatory Functions

Pharmaceutical inspectors face many challenges, including:

- a) Lack of cooperation from owners or superintendent pharmacists of inspected premises.
- b) Risks to inspectors' lives and inadequate logistical support.
- c) Collusion between pharmacists and business operators.
- d) Insufficient logistics and funding.

5.0 Conclusion, Recommendations and Implementation Strategies

5.1 Conclusion

As **Prof. Fola Tayo** emphasized in his keynote at the 4th Fred Adenika Memorial Lecture (2007):

“To perform effectively, National Regulatory Authorities (NRAs) must be provided with the necessary political support, legal powers, human and financial resources. However, these alone will not guarantee effective medicines regulation. NRAs also need strong public support and proper management.”

Therefore, agencies such as the **PCN** and **NAFDAC** require both governmental and public support to effectively perform their regulatory roles. Medicines must be strictly regulated to:

- Address information gaps between manufacturers/sellers and patients/consumers.
- Protect desperate patients from ineffective or toxic medicines.
- Prevent misuse (e.g., antibiotics) that may harm individual and public health.
- Ensure all actors in the drug supply chain uphold treatment outcomes.

5.2 Recommendations and Implementation Strategies

Recommendation 1: Adequate Funding

The Federal Government, through the FMOH, should increase PCN’s funding for inspections, recognizing that regulatory inspections are **not a revenue-generating venture**.

Implementation:

- PCN to prepare annual inspection budgets with clear timelines.
- FMOH to propose special budgetary allocations dedicated to inspection activities.
- Funds to be used for:
 - i. Procuring more vehicles for inspectors.
 - ii. Regular local and international training of inspectors to remain ahead of those they regulate.

Recommendation 2: Collaboration with the Police

PCN should partner with the Nigerian Police to strengthen inspections.

Implementation:

- IGP to direct State Commands to support PCN inspectors.
- Police officers to be sensitized on pharmacy laws to better understand their roles during inspections.

Recommendation 3: Judicial Enlightenment

The Judiciary must be better informed about the role of inspectors in safeguarding public health.

Implementation:

- Organize periodic workshops for judges.
- Encourage judges to carefully evaluate injunctions related to pharmaceutical inspections.

Recommendation 4: Public and Professional Cooperation

Pharmacists and the public should see inspectors as partners, not adversaries.

Implementation:

- Revise pharmacy curricula and MCPD programmes to include detailed objectives of inspection as a growth tool.
- Reframe inspections as **corrective** rather than punitive.
- Conduct public awareness campaigns to explain the benefits of inspections.

Pharm. Ahmed T. Mora, *mni*

Registrar, Pharmacists Council of Nigeria

July 05, 2011

References

1. Brown A.A. (1999). *Assuring Drug Quality through Pharmaceutical Inspection*. Gold Print Press Ltd., Lagos.
2. Brown A.A. (1999). *Who is a Pharmaceutical Inspector?* Workshop by PCN (Unpublished).
3. Federal Ministry of Health (2005). *National Drug Policy*.
4. Federal Republic of Nigeria (2005). *Inspection, Location and Structure of Pharmaceutical Premises Regulation*. Official Gazette No. 79, Vol. 92.
5. Federal Republic of Nigeria (2005). *Registration of Pharmaceutical Premises Regulations*. Official Gazette No. 81, Vol. 92.
6. Pharmacists Council of Nigeria (2009). *Manual for Pharmaceutical Inspectors*. BLISS International Ltd., Ibadan.
7. Pharmacists Council of Nigeria (2009). *Compendium of Minimum Standards for Assurance of Pharmaceutical Care in Nigeria*. BLISS International Ltd., Ibadan.
8. Pharmacists Council of Nigeria (2009). *Proceedings of Nationwide Advocacy Visits and Interactive Sessions with Hospital Pharmacists*. BLISS International Ltd., Ibadan.
9. Tayo, F. (2007). *Management and Control of Pharmaceutical Products Distribution in Nigeria: The Role of Regulatory Bodies*. 4th Fred Adenika Memorial Lecture (Unpublished).
10. World Health Organization (2007). *Quality Assurance of Pharmaceuticals: A Compendium of Guidelines and Related Materials (Vol. 2)*. Geneva.

**A Review of the Guidelines and Code of Ethics for
Pharmaceutical Inspectors. Presented by Pharm. Ahmed T.
Mora, *mni*, Registrar and CEO of the PCN, at the
Pharmaceutical Inspectors' Workshop Organised by the PCN at
the Raw Materials Research and Development Council, Abuja,
from July 5–6, 2011.**

1.0 Introduction

This paper provides guidance toward achieving improved, ethical and effective inspection practices. In academic institutions, particularly at the tertiary level, certificates and degrees are awarded to graduates who demonstrate both academic excellence and character. In line with this principle, the pharmacy profession operates under a Code of Professional Conduct, which regulates practitioners and outlines the principles governing their interactions with clients, other professionals and the public (PCN, 2009). As custodians of human life, pharmacists must uphold high ethical standards to ensure the protection and confidentiality of their patients. This is especially crucial for pharmaceutical inspectors, who are empowered by law to access premises at reasonable times as part of their statutory duties. Inspectors are therefore called to carry out their responsibilities within the confines of professional ethics.

2.0 Conceptual Clarification

2.1 Code of Conduct

According to Rosenberg (2004), a Code of Conduct is a statement of ethical practices and guidelines that an enterprise adheres to. The Longman Active Study Dictionary defines it as rules that people in a particular business agree to obey. For pharmaceutical inspectors, this means maintaining the highest standards of professionalism, decorum and etiquette, while upholding the highest standards of both qualification and character as pharmacists.

2.2 Ethics

Rosenberg (2004) defined ethics as personal beliefs about what is right or wrong. The Longman Active Study Dictionary defines ethics as a set of beliefs that influence people's behavior. For inspectors, ethics entails impartiality, transparency and skillfulness during inspections. The PCN Code of Ethics guides pharmacists in fulfilling their moral and professional obligations, ensuring conduct consistent with the dignity of the profession (PCN, 2009).

3.0 The Ethics of Pharmaceutical Inspection

No two inspections are identical, even within the same specialty. For instance, an inspection of a pharmaceutical manufacturing facility to assess compliance with current Good Manufacturing Practice (cGMP) requires inspectors to exercise sound judgment to ensure product quality, safety and efficacy. Inspectors must never disclose information obtained in one facility to another. While questions are asked during inspections, truthful and transparent answers are expected. The fine line between honesty and concealment must always be carefully navigated. The PCN ensures inspectors are competent before issuing them identity cards, with selection processes carried out by the Inspection and Monitoring (I&M) Department and the Pharmaceutical Inspection Committee (PIC). Only pharmacists with sound professional records and knowledge are recommended as inspectors. Unfortunately, instances of unethical behavior by inspectors have been recorded in the past. Inspectors must recognize that their conduct reflects not just on themselves but also on the profession, stakeholders and the PCN. Ethics extend beyond inspections—they must be evident in the inspector's professional and social life. Furthermore, inspectors are required to continuously update their skills through the Mandatory Continuing Professional Development (MCPD) programme. Inspectors who fail to do so cannot adequately carry out their roles. Indeed, “a pharmaceutical inspector cannot give what he/she does not have.”

4.0 The Regulatory Responsibility of the PCN in Maintaining Ethics in Pharmaceutical Inspection

Pharmaceutical premises may be established for retail, wholesale, importation, hospital practice or manufacturing. The PCN plays a vital role in registration, approval, licensing and monitoring. The quality of inspections during registration often determines the level of professionalism practiced in such premises. Monitoring exercises, conducted by PCN staff, complement the work of the PIC by ensuring Good Pharmacy Practice (GPP) across all categories of premises.

5.0 Conclusion, Recommendations, and Implementation Strategies

5.1 Conclusion

A PCN pharmaceutical inspector must uphold the highest ethical standards and decorum, both during inspections and in daily conduct. Their professional and social behavior must be exemplary, reflecting the integrity of the profession.

5.2 Recommendations

Recommendation 1

The Code of Ethics for PCN Pharmaceutical Inspectors should be reviewed periodically to reflect professional advancements and improve the credibility of inspections.

Implementation-Strategies:

- a) The PCN Law and Ethics Committee should periodically review the Code.
- b) All inspectors must be adequately informed of updates to the Code.

Recommendation 2

Inspectors should continually update their knowledge, skills and competencies.

Implementation Strategies:

- a) Mandatory participation in the MCPD programme.
- b) Organization of periodic seminars, workshops and conferences for inspectors.

Recommendation 3

Stakeholders should be enlightened on the identity and role of PCN inspectors to discourage impostors.

Implementation-Strategies:

- a) PCN should publish the list of inspectors periodically in major national dailies.
- b) The honorarium of inspectors should be reviewed upward to encourage and motivate them.

References

- Longman Active Study Dictionary (2004). Pearson Education Ltd.
- Pharmacists Council of Nigeria (PCN) (2009). PCN's Code of Ethics for Pharmacists in Nigeria, MoR.002, BLISS International Ltd., Ibadan.
- Rosenberg, J. M. (2004). The Essential Dictionary of Management and Human Resources. Barnes and Noble Books.

Innovative Trends in Pharmacy Practice in Nigeria: A Regulatory Perspective. Presented as Registrar and CEO of the PCN at the 2011 Annual Scientific Conference and Exposition of NAPPSA, held at the Sheraton Gateway Hotel, Los Angeles, USA, on September 5, 2011.

1.0 Introduction

This paper highlights evolutionary and innovative measures undertaken by the **Pharmacists Council of Nigeria** to keep pharmacy education and practice aligned with global developments. The overall goal has been to **standardize and upgrade pharmaceutical sciences education** at all levels and ensure that only fit and proper persons are licensed to practice, thereby meeting society's needs over time.

The **Pharmacists Council of Nigeria (PCN)** is a parastatal of the Federal Government established by *Decree (now Act) 91 of 1992*, charged with the responsibility of regulating and controlling pharmacy education and practice in Nigeria in all its aspects and ramifications.

The history of pharmacy regulation in Nigeria dates back to 1927, with the creation of the **Board of Medical Examiners**, which functioned until 1936, when the **Pharmacy Board of Nigeria** was established. In 1964, the **Pharmacists Board of Nigeria (PBN)** came into being and in 1992, the PCN was formally established.

These successive regulatory bodies were established to address the evolving needs of pharmacy education and practice, ensuring the proper training of practitioners, promoting ethical practice and regulating pharmaceutical premises—registered, inspected, approved and licensed for the manufacture, sale and display of medicines and medical devices.

The historical trajectory of regulation in Nigeria can be summarized into three broad periods:

- **1927 – 1960 (Colonial to Independence)**
- **1960 – 2000 (Post-Independence to Millennium)**
- **2000 – 2011 (Contemporary Era)**

It is within this historical context that this paper addresses the topic:

**“Innovative Trends in Pharmacy Practice in
Nigeria: A Regulatory Perspective.”**

I sincerely thank the Executive Committee of NAPPSA, under the leadership of its President, Pharm. Nnodum Iheme, for this invitation and the opportunity to share my thoughts. The conference theme, *“Building Innovation and Change: Gateway to a New Pharmaceutical Frontier,”* is indeed timely and appropriate. I congratulate and rejoice with all NAPPSA members on the success of this 2011 Annual Scientific Conference and Exposition.

1.1 Conceptual Clarification and Scope

According to the *Oxford Advanced Learner’s Dictionary* (Hornby, 2006):

- **Innovation** is “the introduction of new things, ideas or ways of doing.”
- **Trend** is “a general direction in which a situation is changing or developing.”
- **Perspective** is “a particular attitude towards something or a way of thinking about it.”

The scope of this paper covers regulatory trends from **1927 to 2011**.

1.2 Objective of the Paper and Limitations

The primary objective is to enable members of NAPPSA to appreciate the regulatory enforcement strategies of the PCN and its precursor agencies over the past **84 years (1927–2011)** in responding

to the dynamism of pharmacy education and practice in Nigeria. It is recognized that many participants may not be familiar with the ordinances, decrees, or laws that have shaped pharmacy regulation in Nigeria. Furthermore, the practice environment in Nigeria is quite different from that in the Americas. Nevertheless, this paper provides Nigerian pharmacists in the diaspora with a concise overview of pharmacy education and practice in Nigeria past and present thereby broadening participants' perspectives on the challenges faced in introducing innovative ideas to meet the changing needs of pharmacy practice.

1.2.1 Standards of Practice

An important example of regulatory innovation was the definition of **Minimum Standards of Professional Services** by the PCN. In 2005, the PCN published the **“Four-Part Compendium of Minimum Standards for the Assurance of Pharmaceutical Care in Nigeria.”** This became a valuable reference tool for pharmaceutical inspectors, guiding them in evaluating pharmacies and other outlets. The compendium marked a turning point in regulating community pharmacy practice in Nigeria. Since its publication, inspections of new or existing premises are conducted alongside ongoing monitoring of previously licensed outlets. Comparable global models exist, such as the **“Good Pharmacy Practice (GPP) Standards”** developed by the South African Pharmacy Council, which similarly emphasizes high-quality service delivery (GPPSA, 2010).

1.2.2 Continuing Professional Development (CPD) Programme

At the **1995 National Health Summit**, the need for **Continuing Professional Development (CPD)** was underscored. In 1996, the Honourable Minister of Health directed all health regulatory agencies, including the PCN, to develop modalities for CPD implementation. The PCN responded in 1998 with the **Mandatory Continuing Professional Education (MCPE)** programme, which

evolved in 2004 into the **Mandatory Continuing Professional Development (MCPD)** programme. The MCPD provided a structured pathway for all eligible pharmacists to continuously upgrade their knowledge, skills, and competencies. This innovation has earned the pharmacy profession in Nigeria immense respect, as no other health regulatory body among the 12 agencies has developed such a robust and well-established CPD programme.

1.3 Regulatory Capacity of the PCN

The PCN's ability to adapt to professional demands has been demonstrated by:

- The success of the **Compendium of Minimum Standards**, and
- The establishment of the **MCPD Programme**.

Both initiatives underscore the Council's commitment to innovation, professional excellence and responsive regulation in line with global best practices.

2.0 Landmark Innovative Responses to Pharmacy Practice by Nigeria's Regulatory Agencies

The history of pharmacy regulation in Nigeria can be traced across four major regulatory bodies, each of which introduced innovations in response to the challenges of its time:

1. **Board of Medical Examiners (1927–1936)**
2. **Pharmacy Board of Nigeria (1936–1964)**
3. **Pharmacists Board of Nigeria (1964–1992)**
4. **Pharmacists Council of Nigeria (1992–date)**

A: The Pre-Independence Period – 1927 - 1960

S/N	YEAR	MAJOR INNOVATION IN PHARMACY PRACTICE REGULATION	REMARKS
1	1927	Establishment of Board of Medical Examiners of Board	Poisons and Pharmacy Ordinance of 1928
2	1927	School of Pharmacy, Yaba set up to train dispensers	
3	1927	Association of Government dispensers (Later Pharmaceutical Society of Nigeria)	
4	1930	School of Pharmacy Zaria set up to train dispensers for Northern Protectorates	
5	1933	First Chemist and Druggists Graduates coming out of Yaba School of Pharmacy were registered	
6	1936	Pharmacy Ordinance enacted to regulate sales and licensing of dispensers	

The contributions of these agencies, viewed within the three historical periods identified earlier (1927–1960, 1960–2000 and 2000–2011), highlight the progressive regulatory innovations that have shaped pharmacy practice in Nigeria.

Sources:

- Adenika, F (1998). "Pharmacy in Nigeria", Panpharm Ltd., Ikeja.
- PCN (2008): "The PCN Sourcebook on Pharmacy Regulation in Nigeria", BLISS International Ltd, Ibadan.

B: The Post-Independence Period - 1960 - 2000

S/NO	YEAR	MAJOR INNOVATION IN PHARMACY PRACTICE REGULATION	REMARKS
1.	1962	The Federal Manufacturing Laboratory, Yaba, Lagos was established.	
2.	1964	The Pharmacists Board of Nigeria (PBN) was established, thereby making the regulation and control of the profession an all-pharmacists affair.	
3.	1985	Internship training was introduced by the PBN for fresh pharmacy graduates for twelve months.	
4.	1985	The Food and Drugs (Lagos) Act, 1965 and the Food and Drugs Act (Commencement) Order of 1967 were enacted to control the importation of drugs and other pharmacy related products into Nigeria. Pharmaceutical inspectors appointed by the Board.	
5.	1966	University of Ife (Now Obafemi Awolowo University, Ile-Ife) graduated its first B. Pharm degree holders.	
6.	1969	The first ever National Pharmaceutical Inspectors Workshop was organized by the PBN in Lagos on November 14, 1969.	
7.	1980	As a result of intense pressure from the Board and the PSN, a Directorate of Pharmaceutical Services was established in the Federal Ministry of Health and Pharm. Dr. P.O. Emafo, the Board's Chairman, became its first Director. The State Ministries of Health, taking a cue from this development, also established Directorates of Pharmaceutical Services, with registered Pharmacists as Directors, and statutory members of the PBN.	

8.	1981	An orientation training programme for overseas-trained pharmacists started.	
9.	1988	NIPRD established.	
10.	1989	Following pressure from the Board, the National Universities Commission (NUC) approved a minimum standard for a five - year training curriculum for Pharmacy.	
11.	1990	The NUC gave a directive for the introduction of a uniform Clinical Pharmacy Programme in Nigeria Universities Faculties/Schools of Pharmacy.	
12.	1992	NAFDAC established.	
13.	1992	The PBN sponsored the trips of Deans of Faculties/Schools of Pharmacy to overseas institutions running qualitative clinical pharmacy programmes. The visits gave the Deans first-hand experience of how to formally introduce the programme into their curricula in such a way as to harmonize it with the existing system of training for Pharmacy. Two Universities in the U.K and two in the U.S.A were visited. The Registrar of the Board coordinated the trips and submitted a report, which has since formed the nucleus for further development of Clinical Pharmacy Practice in Nigeria.	
14.	1993	The Pharmacists Council of Nigeria Came into existence with the enactment of the PCN Decree No. 91 of 1992.	
15.	1993	Prince Julius Adelusi-Adeluyi became the first Pharmacists in Nigeria's history to be appointed Minister of Health.	
16.	1993	Prof. O.K. Udeala became the first pharmacists to hold the position of substantive Vice Chancellor at University of Nigeria, Nsukka (UNN).	

17.	1998	The Registry introduced the MCPE programme.	
18.	1999	Following a request by the Registrar of the Council to the Presidency in 1998 for the creation of improved cadres of human resources for the Secretariat of the Council, approval was conveyed, after a thorough examination by the Office of Establishment and Management Service. Six Zonal Offices of the Secretariat of the Council were recommended for establishment.	
19.	1999	The Pharmacists Council of Nigeria (PCN) Governing Council was reconstituted and inaugurate with Dr. Fred 8. Adenika as Chairman.	

Sources:

- Adenika, F (1998): "Pharmacy in Nigeria", Panpharm Ltd., Ikeja.
- PCN (2008): "The PCN Sourcebook on Pharmacy Regulation in Nigeria", BLISS International Ltd, Ibadan.

C: The Post-Millennium Period - 2000 - Date

S/N	YEAR	MAJOR INNOVATION IN PHARMACY PRACTICE REGULATION	REMARKS
1.	2000	First complement of PCN staff posted to Abuja office PCN.	
2.	2000	MCPE was re-introduced by Council after two years suspension.	
3.	2001	Pharmaceutical Inspection Committees (PICs) were established in all the 36 States and Abuja Federal Capital Territory, with the DPS as Chairman.	
4.	2001	Two Zonal Offices of the PCN were established in Enugu and Kano.	
5.	2003	The issuing authority of PPMVLs was reverted to the PCN.	

6.	2003	Inauguration of the 3rd Governing Council of the PCN, with Prof. F.A. Ogunbona as Chairman.	
7.	2003	Appointment of a new Registrar, Pharm. Ahmed T. Mora.	
8.	2004	Re-organization of the Registry and the creation of an additional department bringing the total to seven.	
9.	2004	PCN Act 19 of 1992 cited as Laws of the Federation of Nigeria (LFN) as Cap 17 in the Statutes.	
10.		Approval of the introduction of the Doctor of Pharmacy (Pharm. D) Degree programme by the NUC in Nigerian Universities.	
11.		The 4 th Fred Adenika Memorial Lecture was delivered by Prof. Fola Tayo who spoke on "Management and Control of Pharmaceutical Products Distribution System in Nigeria: The Role and Responsibilities of Regulatory Bodies".	
12.		Pharmaceutical Inspectors Workshop in Abuja.	All Core PIC members from States
13.		Professor E. N Sokomba appointed Chairman of the PCN Governing Council by President, Commander-in- Council Chief of the Federal Republic of Nigeria.	New Governing Council reconstituted
14.		Workshop for Superintendent and Production Pharmacists of Pharmaceutical Manufacturing Industries in Nigeria in Lagos, Abuja and Enugu.	Dr. Echeazu Ugwu of Philadelphia served as Lead
15.		Pharmaceutical Inspectors Workshop in Abuja	All Core members from States PIC

2.0 Landmark Innovative Responses to Pharmacy Practice by Nigeria's Regulatory Agencies

The regulatory history of pharmacy in Nigeria reflects a progressive evolution of standards, institutions and professional oversight. Each regulatory era introduced distinct innovations in

response to emerging challenges in pharmacy education and practice.

Table 1: Evolution of Pharmacy Regulation in Nigeria (1927–2011)

Period / Agency	Innovations and Contributions
1927–1936: Board of Medical Examiners	• First attempt at formal regulation of pharmacy practice in Nigeria. • Set the foundation for examining and licensing practitioners. • Early control of drug distribution, though limited in scope.
1936–1964: Pharmacy Board of Nigeria	• Established by law to specifically regulate pharmacy. • Introduced structured examinations for pharmacists. • Strengthened control of pharmaceutical premises and medicines. • Expanded scope of practice regulation beyond medical examiners.
1964–1992: Pharmacists Board of Nigeria (PBN)	• Expanded role in pharmacy education and professional training. • Supervised pharmacy schools and curriculum development. • Increased monitoring of pharmaceutical premises. • Enhanced professional recognition of pharmacy as distinct from medicine.
1992–date: Pharmacists Council of Nigeria (PCN)	• Established by <i>Decree (now Act) 91 of 1992</i>. • Full mandate to regulate and control pharmacy education and practice in all its aspects. • Introduced the Compendium of Minimum Standards for Pharmaceutical Care (2005). • Developed and implemented the Mandatory Continuing Professional Development (MCPD) programme (1998–2004). • Enhanced licensing, inspection, and monitoring of pharmaceutical premises. • Strengthened collaboration with other agencies (e.g., NAFDAC, FMOH) to combat fake and substandard drugs. • Promoted harmonization of standards in line with global best practices.

3.0 Emerging Trends and Future Directions in Pharmacy Regulation

The regulatory journey of pharmacy in Nigeria demonstrates a steady alignment with international best practices. However, the rapidly changing landscape of healthcare and pharmaceutical sciences demands new approaches to ensure continuous relevance and impact.

3.1 Technology-Driven Regulation

- **Digital Registries and Licensing:** The PCN has begun transitioning from manual registers to digital platforms for pharmacists and pharmaceutical premises. This enhances transparency, accountability and ease of verification.
- **E-Inspection Tools:** The adoption of handheld devices and electronic software for inspection promises real-time data collection, monitoring and analysis.
- **CPD E-Learning Platforms:** Expansion of online MCPD programmes provides pharmacists nationwide with easier access to professional development opportunities.

3.2 Strengthening Pharmacy Education

- **Curriculum Modernization:** Pharmacy curricula in Nigerian universities are being reviewed to incorporate modern disciplines, such as Pharmacoeconomics, regulatory science, biotechnology, nanomedicine and pharmacovigilance.
- **Experiential Learning:** The shift towards practice-based training ensures graduates are well-prepared for clinical, industrial and regulatory roles.
- **Global Benchmarking:** Efforts are ongoing to align Nigerian pharmacy education with standards set by

international organizations, such as the International Pharmaceutical Federation (FIP).

3.3 Expanding Scope of Pharmacy Practice

- **Clinical Pharmacy Services:** There is a growing emphasis on pharmacists' direct involvement in patient care, including medication therapy management and pharmacovigilance.
- **Industrial Pharmacy Development:** Local manufacturing is being strengthened to reduce dependence on imports in line with national policy on drug security.
- **Specialization Tracks:** Pharmacists are being encouraged to pursue specialization in regulatory science, supply chain management and Pharmacoeconomics.

3.4 Collaboration with Stakeholders

- **Inter-Agency Partnerships:** Stronger collaboration with NAFDAC, the Federal Ministry of Health and state-level agencies is crucial for combating fake drugs and ensuring public safety.
- **Regional Integration:** The PCN is actively engaging with the West African Health Organization (WAHO) to harmonize pharmacy standards across ECOWAS countries.
- **Global Engagement:** Nigerian pharmacists are increasingly contributing to international discourse on pharmacy practice, innovation and regulation.

4.0 Conclusion and Recommendations

4.1 Conclusion

The evolution of pharmacy regulation in Nigeria from 1927 to 2011 demonstrates a strong trajectory of innovation and adaptation. The Pharmacists Council of Nigeria (PCN) has consistently provided leadership in ensuring that pharmacy education, professional practice, and pharmaceutical premises meet internationally accepted standards. Landmark achievements, such as the Compendium of Minimum Standards (2005) and the Mandatory Continuing Professional Development (MCPD) Programme (2004), have placed Nigeria at the forefront of pharmacy regulation in Africa.

As new challenges emerge such as nanomedicine, biotechnology and the globalization of pharmaceutical trade the regulatory role of the PCN must remain dynamic, proactive and technology-driven.

4.2 Recommendations

To consolidate gains and address future challenges, the following actions are recommended:

- 1. Digital Transformation:** Full deployment of electronic systems for registration, inspection and monitoring of pharmacists and pharmaceutical premises.
- 2. Strengthening Pharmacy Education:** Continuous curriculum review to incorporate emerging fields such as Pharmacoeconomics, biotechnology and digital health.
- 3. Regional and Global Integration:** Active participation in WAHO and FIP initiatives to harmonize regulatory standards across ECOWAS and align with international best practices.
- 4. Enhanced Collaboration:** Strengthen partnerships with NAFDAC, the Federal Ministry of Health and security agencies to combat fake and substandard medicines.
- 5. Capacity Building:** Expand the CPD/MCPD framework through e-learning, mentorship programmes, and speciali-

zation tracks to ensure pharmacists remain globally competitive.

References

- Hornby, A. S. (2006). *Oxford Advanced Learner's Dictionary of Current English* (7th ed.). Oxford University Press.
- Good Pharmacy Practice Standards in South Africa (GPPSA). (2010). South African Pharmacy Council.
- Mora, A. T. (2010). *Pharmaceutical Regulation and Distribution in Nigeria: Challenges and Prospects*. PCN Publications.
- Pharmacists Council of Nigeria (2005). *Four-Part Compendium of Minimum Standards for the Assurance of Pharmaceutical Care in Nigeria*. PCN Press, Abuja.
- National Health Summit (1995). *Proceedings of the Maiden National Health Summit*. Federal Ministry of Health, Abuja.

The Challenges of Health Regulation in Nigeria: The Big Picture. A Paper by Dr. Ahmed T. Mora, Ph.D., FPSN, FPCPharm, mni, Dean, Faculty of Pharmaceutical Sciences, Kaduna State University (KASU), Kaduna, at National Conference on Healthcare Management/Membership Induction Hosted by the Institute of Healthcare Management, Nigeria Agura Hotel, Abuja November 30, 2012

Preamble

This paper focuses on the role of health regulatory agencies in Nigeria, the use of HRH in healthcare facilities and the challenges they face.

The topic of discussion, “*The Challenges of Health Regulation in Nigeria: The Big Picture*”, highlights the critical challenges facing health regulation in Nigeria. By choosing this topic, the Institute of Healthcare Management, Nigeria, emphasizes the need for a comprehensive exploration of these challenges. The topic aligns perfectly with the broader conference theme, “*Healthcare Management in Nigeria: Challenges and Prospects*” and aims to strengthen service delivery, regulation and training—the sub-theme of this National Conference.

Healthcare service is a packaged commodity delivered by stakeholders within the healthcare system through registered, inspected, approved and licensed facilities, subject to periodic evaluation by relevant health authorities. The goal is to make healthcare easily accessible to patients whose health is compromised, to prevent disease, or to maintain bodily normalcy. Key questions arise:

- What type of healthcare package is being dispensed?
- At which facility?
- By which stakeholders?
- With what level of skill, expertise, and competence?

The evaluation of these questions through inspection or accreditation is a primary function of health regulatory agencies. These agencies

rely on Human Resources for Health (HRH) defined by the World Health Organization (WHO) as personnel, who promote and preserve health, diagnose and treat diseases and include health management and support staff who ensure the healthcare system functions efficiently (WHO, 2006). In all formal organizations, human resources are the heartbeat of service delivery.

1. Health Regulatory Agencies in Nigeria

Currently, there are thirteen (13) healthcare regulatory agencies in Nigeria:

1. Community Health Practitioners Registration Board of Nigeria (CHPRBN)
2. Dental Technologists Registration Board of Nigeria (DTRBN)
3. Dental Therapists Registration Board of Nigeria (DTRBN)
4. Health Records Officers Registration Board of Nigeria (HRORBN)
5. Institute of Chartered Chemists of Nigeria (ICCON)
6. Institute of Public Analysts of Nigeria (IPAN)
7. Medical and Dental Council of Nigeria (MDCN)
8. Medical Laboratory Science Council of Nigeria (MLSCN)
9. Medical Rehabilitation Therapists Registration Board of Nigeria (MRTRBN)
10. Nursing and Midwifery Council of Nigeria (NMCN)
11. Optometrists and Dispensing Opticians Registration Board of Nigeria (ODORBN)
12. Pharmacists Council of Nigeria (PCN)
13. Radiographers Registration Board of Nigeria (RRBN)

These agencies are empowered to make regulations under the various Acts of the Federation of Nigeria (LFN) that establish them and to define and advance training and practice within their respective professions. They operate as government parastatals under the supervision of the Federal Ministry of Health (FMOH), each governed by a separate Board or Council. The core functions of these agencies include the registration and licensing of practitioners, inspection of practice premises, accreditation of training institutions

and ensuring Continuing Professional Development (CPD) compliance. For example, the Pharmacists Council of Nigeria (PCN) was established under Decree (now Act) 91 of 1992 (LFN No. 17 of 2004) to regulate pharmacy education and practice.

1.1 Example: Community Health Practitioners Registration Board of Nigeria

The Community Health Practitioners Registration Board of Nigeria (“the Board”) is responsible for:

- a) Determining standards of knowledge and skills for community health practitioners and improving these standards over time;
- b) Establishing and maintaining a register of registered members and publishing lists periodically;
- c) Conducting examinations and awarding certificates or diplomas, with fees prescribed by the Board;
- d) Performing other functions conferred by the Act.

1.2 The Registrar

Each regulatory agency is led by a Registrar, who serves as the Chief Executive Officer (CEO). Responsibilities include administration according to Public Service Rules. Appointment of a Registrar typically involves advertisement, shortlisting, interview and selection. The Registrar must be a registered and licensed professional in the respective field.

For example, the Medical and Dental Council of Nigeria (MDCN) Act stipulates:

- The Council appoints a qualified medical practitioner or dental surgeon as Registrar.
- The Registrar maintains registers of names, addresses, qualifications, and other particulars of registered practitioners.

Registrars play a pivotal role in maintaining the integrity of professional registration and regulatory oversight.

1.3 Staff of Regulatory Agencies

Regulatory agencies recruit staff as public officers to support the Registrar in discharging regulatory functions. Staff appointments follow Public Service Rules. For instance, the Nursing and Midwifery Council of Nigeria:

- May appoint additional employees as necessary;
- Holds authority to advance, confirm, promote, transfer, discipline, or terminate staff appointments.

Staff are considered public officers, subject to retirement rules and receive salaries and benefits approved by the Office of the Accountant-General of the Federation (OAGF).

1.3 Governing Boards/Councils

All regulatory agencies operate under Governing Boards or Councils appointed by the Federal Government to serve as policy-making bodies. Membership composition varies by agency. For example, the Medical Laboratory Science Council of Nigeria (MLSCN) Act, 2003, stipulates that:

- Chairman: A qualified Fellow with at least 5 years post-qualification experience, appointed by the President;
- Representatives from the Federal Ministry of Health, private practitioners, and universities;
- Representatives from professional associations and non-governmental organizations with interest in healthcare.

Most Boards follow similar structures, balancing professional expertise with public representation. Such is the composition of most of the Boards or Councils. The number of members on such Boards is produced in the following Table:

S/NO	REGULATORY AGENCY	NUMBER OF MEMBERS ON THE BOARD
1.	Pharmacists Council of Nigeria (PCN)	63
2.	Medical and Dental Council of Nigeria (MDCN)	61
3.	Institute of Public Analysts of Nigeria (IPAN)	20
4.	Medical Laboratory Scientists Council of Nigeria (MLSCN)	1
5.	Nursing and Midwifery Council of Nigeria (NMCN)	18
6.	Institute of Chartered Chemists of Nigeria (ICCON)	17
7.	Dental Therapists Registration Board of Nigeria (DTRBN)	17
8.	Medical Rehabilitation Therapists Registration Board of Nigeria (MRTB)	17
9.	Optometrists and Dispensing Opticians registration Board of Nigeria (ODORBN)	15
10.	Dental Technologists Registration Board of Nigeria (DRTBN)	13
11.	Radiographers Registration Board of Nigeria (RRBN)	12
12.	Health Record Officers Registration Board of Nigeria (HRORBN)	12
13.	Community Health Practitioners Registration Board of Nigeria (CHPRN)	11

1.4 Variations in Board Composition

There is, however, a difference with the Institute of Chartered Chemists of Nigeria (ICCON) and the Institute of Public Analysts of Nigeria (IPAN) regarding the composition of their Boards and the title of their chairmen, who are called Presidents. For ICCON, the President and Vice President are elected by the Council. The Act provides:

“There shall be for the Institute a President and a Vice President, who shall be Fellows of the Institute, to be elected by the Council and hold office for a term of two years from the date of election.” (Chapter 112, Institute of Chartered Chemists of Nigeria Act)

The same election process applies for IPAN. While the other eleven (11) regulatory agencies have their chairmen appointed by the President of Nigeria on the recommendation of the Honourable Minister of Health, ICCON and IPAN select their leaders through elections. A common feature among all regulatory agencies is the establishment of Investigating Panels and Disciplinary Tribunals. For instance, the Medical Rehabilitation Therapists (Registration, etc.) Act provides:

“There shall be established a Disciplinary Committee, known as the Medical Rehabilitation Therapists Practitioners Disciplinary Committee (‘the Disciplinary Committee’), charged with the duty of considering and determining any case referred to it by the Panel established under this Act.” (Chapter M9, Medical Rehabilitation Therapists (Registration, Etc.) Act)

Regulatory agencies exist to ensure the proper education and practice of healthcare professionals. Deviations from standards must be corrected to maintain professionalism and protect the public. Issues of professional misconduct or ethics are investigated by the Panel after which the Disciplinary Tribunal hears cases publicly. The Attorney-General of the Federation appoints a counsel to assist Panels and Tribunals during proceedings.

2.0 The Role of Health Regulatory Agencies in Advancing Healthcare Delivery

Emeritus Professor Umaru Shehu, FAS, emphasized the role of professional regulatory bodies:

“The roles of professional regulatory bodies are usually well defined and similar worldwide. They include:

- Setting minimum entry qualifications for students;
- Approving curricula for training;
- Monitoring student training to ensure compliance;

- Certification and re-certification programmes;
- Facilitating Continuing Professional Development (CPD);
- Ensuring compliance with professional ethics and morality.” (CRHP, 2008)

Prof. Dennis Agbonlahor, former Chairman of MLSCN, stated:

“Today's patient care and healthcare practice is multi-professional and multi-disciplinary. Recognizing that all participants contribute equally valid knowledge and expertise is essential for effective teamwork.” (CRHP, 2011)

At the global level, representatives of the World Medical Association (WMA), the International Pharmaceutical Federation (FIP) and the International Council of Nurses (ICN) formed the World Health Professions Alliance (WHPA) in 1999. Dentists joined in 2005 through the World Dental Federation (WDF), collectively representing 25 million healthcare professionals worldwide. The WHPA promotes collaboration among key health professionals, governments, policymakers and the World Health Organization (WHO) to benefit patients and healthcare systems (CRHP, 2008).

The author attended the First World Health Professions Conference on Regulations in Geneva, Switzerland (May 17–18, 2008) organized by the WHPA. The conference emphasized the regulatory role of agencies in maintaining healthcare systems and the responsibility of governments to fund inspections and accreditations.

2.1 The Accreditation and Inspection of Healthcare Facilities

Healthcare training institutions must be accredited to ensure adherence to approved curricula, Benchmark Minimum Academic Standards (BMAS) or course content. Evaluations can occur in Schools of Health Technology, Polytechnics or Universities. Teams of professional experts conduct accreditation visits, assessing new and ongoing programs. Accreditation also applies to internship or housemanship centers for fresh graduates, such as medical doctors, pharmacists and radiographers. Supervising professionals must demonstrate full oversight. For instance, in PCN, a logbook must be signed by a supervising pharmacist with at least five years of

registration and countersigned by the Director of Pharmaceutical Services (DPS) of the State Ministry of Health.

Inspections ensure that healthcare facilities meet minimum standards of practice and protect the public. They may be for licensing new facilities or monitoring registered premises. Inspectors from MLSCN, PCN and NMCN assess compliance with professional standards, ensuring that fake or unqualified practitioners do not operate. Protocols, SOPs, monographs and documents, such as PCN's *Four-Part Compendium of Minimum Standards of Practice for the Assurance of Pharmaceutical Care in Nigeria* serve as evaluation tools.

Inspections are carried out in teams. While accreditations are periodic, inspections are ongoing. The PCN has Pharmaceutical Inspection Committees (PICs) in States and the FCT to inspect pharmacies and importation firms. Private pharmacists may serve as co-opted inspectors after receiving training and ID cards. However, inspections of pharmaceutical manufacturing facilities are conducted exclusively by public officers. For RRBN, inspections of imaging centers are centrally managed from Lagos due to the specialized equipment and limited number of facilities.

Inspectors act as “policemen” of healthcare premises. At PCN, inspections are corrective, not punitive. Punitive approaches may discourage cooperation from healthcare professionals.

2.2 Continuing Professional Development (CPD) Programmes

Healthcare professionals must maintain high competence to avoid errors and maintain patient confidence. CPD ensures that practitioners adapt to specialized healthcare demands, emerging diseases, drug resistance and changing environments. At the National Council on Health (NCH) meeting in Asaba, Delta State (April 2010), participation in CPDs became mandatory for licensing and license renewal. PCN launched the Mandatory Continuing Professional Education (MCPE) in 1998, later renamed Mandatory Continuing Professional Development (MCPD) in 2004. Initially, some modules were overly academic, but collaboration with Partners for Transforming Health Systems (PATHS) improved content and practical relevance.

2.3 The Committee of Registrars of Health Professions (CRHP)

On August 19, 2004, the Honourable Minister of Health, Professor Eyitayo Lambo, inaugurated the CRHP to:

- Raise ethical standards in healthcare delivery;
- Set performance targets;
- Facilitate CPD for healthcare professions;
- Educate the public on health matters;
- Harmonize regulatory activities for efficiency;
- Foster collaboration among healthcare professionals;
- Promote unity and mutual respect among health professions in Nigeria.

The Minister stated:

“It is heart-warming that Registrars of all regulatory bodies are coming together under one umbrella. I urge you to use this Committee as a springboard to rejuvenate the team spirit that is fast fading away.” (Lambo, 2004)

Interconnectivity among regulatory agencies ensures coordinated oversight across primary, secondary and tertiary healthcare facilities. For example, inspectors from PCN and MDCN may encounter each other in hospitals or clinics.

3.0 The Challenges of Health Regulation in Nigeria

The key functions of health regulatory agencies include accreditation, inspections, CPD and maintaining registers of practitioners and practice premises. Investigating Panels and Disciplinary Tribunals uphold ethics, professionalism and standards. The following are major challenges and proposed solutions:

3.1 Inadequate Numbers of HRH

Insufficient personnel affect inspection frequency, quality and integrity. Governments must recruit, train and adequately remunerate health inspectors, ensuring independence from the institutions they monitor.

3.2 Creation/Upgrading of Housemanship or Internship Centres

Graduates require accredited internship placements for skill development. Universities must ensure sufficient placements in recognized centers.

3.3 Poor Logistics Support for Inspectors

Regulatory agencies need adequate funding for vehicles, communication equipment and other logistics to conduct inspections effectively.

3.4 Lack of Adequate Training and Development for Inspectors

Inspectors require specialized knowledge. Training in Nigeria and abroad ensures technical proficiency and effective oversight.

3.5 Inadequate CPD Programmes

Some agencies lack CPD implementation or offer poorly designed modules. Experienced practitioners should contribute as resource persons. License issuance should depend on CPD compliance and fitness to practice.

3.6 Interference by Professional Associations

Professional associations, as NGOs, should not usurp regulatory authority. Regulatory staff are public officers trained to implement mandates objectively.

3.7 Frequent Dissolutions of Governing Boards/Councils

Frequent early dissolutions disrupt policy continuity and management supervision. Boards should serve full terms unless incompetence is evident.

4.0 Conclusion

Health regulatory agencies play a vital role in Nigeria. Potential mergers of parastatals should not compromise effective regulation and oversight. Government funding, well-trained public officers and

accountability are essential for maintaining healthcare quality in registered and unregistered facilities nationwide.

Thank you for listening, and God bless.

Dr. Ahmed T. Mora, PhD; FPSN; FPC Pharm; mni

Dean, Faculty of Pharmaceutical Sciences

Kaduna State University (KASU)

November 30, 2012

Agura Hotel, Abuja

References

- Acts for the Establishment of all thirteen Health Regulatory Agencies
- CRHP (2008): *The Role of Health Regulatory Bodies in Advancing Healthcare Delivery Services in Nigeria* – Proceedings of a One-Day Advocacy Workshop, Abuja, 2008, 91p
- CRHP (2011): *Harmony in the Healthcare Delivery System: The Role of Regulatory Agencies* – Proceedings of a One-Day Advocacy Workshop, Abuja, 2011, 75p
- FRN (2007): *National Human Resources for Health Strategic Plan (2008–2012)*, 2007, 83p
- Lambo, E. Prof. (200

**A Workshop to Develop Curriculum-Based Accreditation
Criteria for Pharmacy Technicians'
Training Programmes in the ECOWAS Region. Presented by
Dr. Ahmed T. Mora at a Workshop organised by the West
African Health Organisation (WAHO), Bobo-Dioulasso,
Burkina Faso, from July 2–3, 2013.**

Preamble

The Pharmacy Technician Programme is designed to provide auxiliary training to students, primarily in Schools of Health Technology (SHTs), to qualify them as Pharmacy Technicians (PTs). These professionals fill the much-needed middle-level Human Resources for Health (HRH) gap and serve as support personnel to pharmacists. Over time, Pharmacy Technicians have become recognized as a vital technical auxiliary cadre, providing services especially at Primary Health Centres and General or Cottage Hospitals for secondary healthcare delivery. It is also not uncommon to find PTs in Teaching Hospitals, Federal Medical Centres (FMCs), and other tertiary healthcare institutions, where they provide support services to pharmacists.

In Nigeria, the training of Pharmacy Technicians in SHTs is supervised by the Pharmacy Department of each school under a pharmacist designated as Head of Department (HoD). Accreditation of the PT programme is conducted by the Pharmacists Council of Nigeria (PCN) through duly constituted accreditation teams of the Governing Council. The curriculum of the PT programme is prepared by the PCN and handed down to SHTs. Accreditation follows a structured process using a specially designed form (FORM PTI) and the approved curriculum, which must be fully in place during both initial and re-accreditation exercises.

This workshop has therefore been convened to develop curriculum-based accreditation criteria for Pharmacy Technician Training Programmes in the ECOWAS region, drawing lessons from Nigeria and other countries. The ultimate goal is to harmonize accreditation

standards across the region. This paper is presented as a foundation for discussions at this two-day workshop.

1. The Roles of Pharmacy Regulatory Agencies

Pharmacy Regulatory Agencies—whether Pharmacists Councils, Pharmacy Councils, or Pharmacy Boards—are established to regulate and control pharmacy education and practice in all its dimensions within member countries of the region. These agencies must take full responsibility for accreditation, training, examinations and licensing (or issuance of permits) for Pharmacy Technicians. This ensures that graduates can properly deliver essential pharmaceutical services at the middle level, under the supervision of trained and licensed pharmacists.

The functions of Regulatory Agencies in PT training can be summarized as follows:

- Curriculum Development
- Accreditation Requirements and Evaluation Format
- Designation of Institutions for PT Training, including student entry requirements, quotas, and physical infrastructure standards
- Indexing and Examinations
- Registration, Issuance of Licenses/Permits, and Fees
- Continuing Education, Workshops, and “Train-the-Trainers” programmes

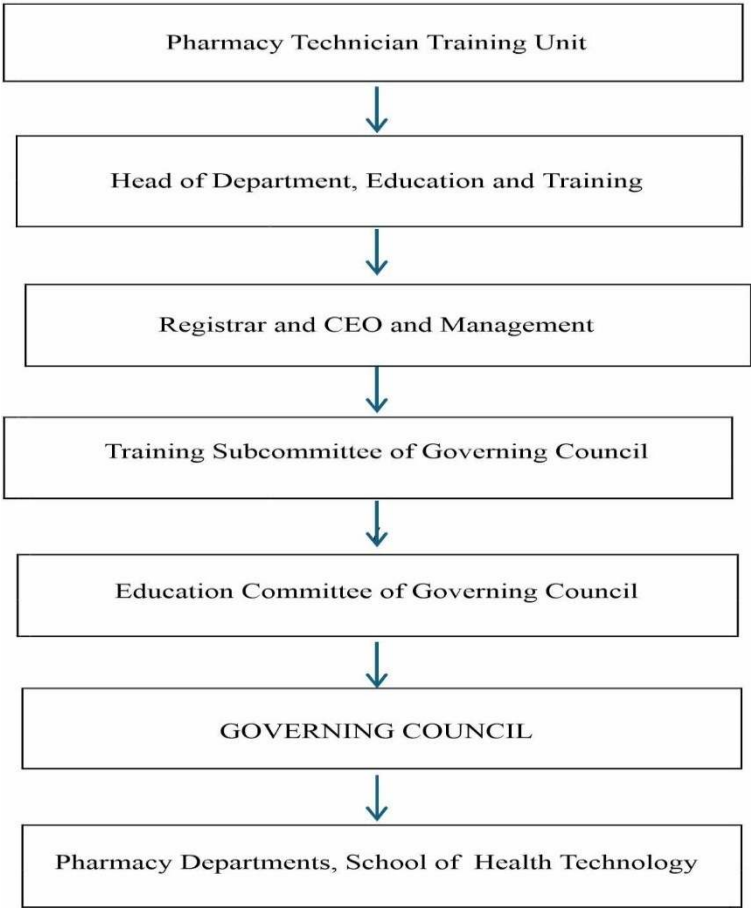
1.1 Curriculum Development

A Pharmacy Regulatory Agency should be structured in a way that places curriculum development and training manual design under the appropriate department. For example, within the PCN, this responsibility lies with the Education and Training Department, where a designated pharmacist serves as desk officer. Curriculum development—whether creating a new programme or reviewing an existing one—begins in this department and culminates in the office of the Registrar/Chief Executive Officer (CEO), working with the management team to finalize content. At PCN, the Training Subcommittee, a standing committee of the Governing Council,

reviews PT training matters. Its recommendations are passed to the Education Committee and then to the Governing Council for deliberation and approval. Only after this process is the approved curriculum issued to the Schools of Health Technology (SHTs) for implementation.

(See Table 1 below for a flow chart of this process.)

Table 1



1.2 Accreditation Requirements and Evaluation Format

The primary objective of an accreditation exercise is to ensure that pre-set standards—particularly in human resources, infrastructure and training facilities for Pharmacy Technicians (PTs)—are met. In other words, the reality on the ground must align with the established benchmarks. To uphold the integrity of the accreditation process, the **Governing Council or Board** should constitute accreditation teams based on recommendations from the Registrar and Chief Executive Officer (CEO). Wherever possible, these teams should include:

- Pharmacists from universities;
- Pharmacists from Departments of Pharmaceutical Services within State or Provincial Ministries of Health;
- Retired pharmacists with prior public service experience;
- Pharmacists in community practice or from the Pharmaceutical Society.

However, given the sensitive nature of evaluation, preference should be given to pharmacists serving as public officers or civil servants, whose accountability mechanisms discourage profit-driven bias and ensure impartiality.

Application Form for the Accreditation of Schools of Health Technology (Form PTI)

1. Particulars of the School

- a. Name of School:
- b. Address.....
- c. Local Government Area State
- d. Telephone, fax, email
- e. Date of establishment of the School
- f. Date of commencement of the Pharmacy Technician Training
- g. Other departments in the School

h. Supervising Ministry

2. The Governing Council

- a. Number of members
- b. Name of Chairman
- c. Qualifications of Chairman
- d. Number of pharmacists on the Council

.....

3. Staffing

- **Principal/Provost**

- i. Name
- ii. Qualifications (with dates)
- iii. Date of assumption of office
- iv. Years of experience in SHTs

- **Head of Department (Pharmacy)**

- i. Name
- ii. Qualifications (with dates)
- iii. Professional registration number (with date)
-
- iv. Current license number (with date)
- v. Years of experience in SHTs
- vi. Date of joining the School
- vii. Courses taught

- **Other Pharmacists as Staff**

For each:

- i. Name
- ii. Qualifications (with dates)
- iii. Years of experience in SHTs
- iv. Full-time/Part-time
- v. Courses taught
- vi. Registration number / Current license number

.....
(Use additional sheets if required)

- **Other Teaching Staff**
 - i. Name
 - ii. Qualifications (with dates)
 - iii. Years of teaching experience in SHTs
-
- Signature/Date**.....
- Official Stamp**.....

1.3 Institutions for the Training of PTs

For PTs to be available in the workforce, there must be **designated training institutions** approved by the Regulatory Agency. In Nigeria, these are primarily **Schools of Health Technology (SHTs)** operated by State Governments. Challenges have arisen where some polytechnics—and in one notable case, a Teaching Hospital—attempted to initiate PT training. Such moves were strongly resisted by the PCN to safeguard programme integrity. All designated institutions must strictly adhere to the **accreditation manual and format** prescribed by the Regulatory Agency.

Entry Requirements (Nigeria Example)

Candidates must possess a **Senior Secondary School Certificate Examination (SSSCE)** from WAEC with passes in at least four subjects, including Chemistry, Biology, Mathematics and English in not more than three sittings. Equivalent passes in GCE or NECO are accepted. The programme typically runs for **36 months** after which graduates may work as Pharmacy Technicians under the supervision of registered pharmacists.

1.4 Indexing of Students and Examinations

A student indexing system should be implemented. In Nigeria, SHTs verify students' entry qualifications within six (6) months of admission and submit them for indexing by the PCN, using prescribed forms and paying the required fees. The Regulatory Agency should also appoint **External Examiners** to supervise final examinations. Guidelines include:

- External Examiners must be pharmacists with at least **10 years post-registration experience** and proven expertise in training.
- Schools must apply at least **four weeks** in advance for appointment of an examiner.
- The External Examiner supervises examinations, validates results, and submits a report directly to the Regulatory Agency.

The **Training Subcommittee, Education Committee** and ultimately the **Governing Council** review these reports before final approval and release of results.

1.5 Registration, Licensing/Permits, and Fees

Upon qualification, Pharmacy Technicians are entered into the **Register of PTs** maintained by the Registrar of the Regulatory Agency.

- A **Permit** (to distinguish it from pharmacists' licenses) is issued, authorizing PTs to work in approved organizations.
- The Permit is also a requirement for promotion exercises in government establishments.
- The Regulatory Agency prescribes fees for **initial registration** and **periodic renewal** of the Permit.

Maintaining an up-to-date Register is essential for professional regulation and accountability.

1.6 Continuing Education, Workshops, and Train-the-Trainer Programmes

To ensure sustained competence, Regulatory Agencies must enforce **Continuing Professional Development (CPD)** for PTs, similar to pharmacists.

Key approaches include:

- Periodic **training workshops** for PT tutors (who are pharmacists), organized centrally or regionally;
- Incorporating **peer review mechanisms** to ensure adherence to curriculum standards;
- Compulsory CPD for PTs to keep pace with new developments in pharmaceutical technology; and
- Structured “Train-the-Trainer” programmes to strengthen teaching capacity within SHTs.

Conclusion

Pharmacy Technicians play a vital role in ensuring effective healthcare delivery as auxiliary staff across the **ECOWAS region**. The challenge for regulators lies in providing sufficient training and exposure to PTs while avoiding role confusion with pharmacists. Nonetheless, the harmonization of **curricula and accreditation standards** across ECOWAS is both urgent and necessary, especially in pursuit of healthcare-related **Millennium Development Goals (MDGs)** and broader access to quality healthcare services. By sharing human resources across member states and strengthening regulatory oversight, the region will be better positioned to combat parasitic and communicable diseases that transcend borders. Ultimately, a **harmonized PT curriculum** will provide a benchmark recognized across ECOWAS, fostering mobility, quality assurance and improved healthcare outcomes.

Thank you.

**Ensuring Access to Registered Pharmaceutical Products in
Licensed Pharmaceutical Premises: The Role of Regulatory
Agencies. Presented by Dr. Ahmed Tijjani Mora, *mni*, Dean,
Faculty of Pharmaceutical Sciences, Kaduna State University
(KASU), at the First International Conference of the
Nanomedicine Association of Nigeria, held at the African
University of Science and Technology (AUST), Galadimawa,
Abuja, on November 27, 2013.**

Abstract

This paper discusses access to registered pharmaceutical products—including nanomedicines—in licensed pharmaceutical premises and highlights the role of regulatory agencies, notably the **National Agency for Food and Drug Administration and Control (NAFDAC)** and the **Pharmacists Council of Nigeria (PCN)**. A breakthrough in drug invention, whether for prophylaxis or curative purposes, is only one part of the success story in drug development. The processes of drug identification, extraction, evaluation, dosage form selection and clinical trials that guarantee safety and efficacy do not automatically ensure that products are available where they are most needed—at the tail end of the distribution chain, namely retail and hospital pharmacies. The paper concludes by proposing strategies to guarantee access to safe, effective and genuine medicines—whether locally produced or imported—registered by NAFDAC and distributed exclusively within registered, inspected, approved and licensed pharmaceutical premises by the PCN.

Among the recommendations:

- **Manufacturers and importers** should utilize computerized invoicing systems to strengthen the traceability of pharmaceutical products.
- **Retail and hospital pharmacies** should adopt patient-centered dispensing software that records patient profiles, enables product recalls, and facilitates communication between patients and pharmacists.

☎ GSM: 0813-875-9537

☎ Email: tijjanimora@yahoo.com

1.0 Introduction

Pharmaceutical products prescribed, dispensed or administered within healthcare institutions fall into two broad categories:

1. **Prescription drugs**
2. **Non-prescription (Over-the-Counter, OTC) drugs**

In Nigeria, there are two officially recognized sources of pharmaceutical products:

- **Local manufacturing**, and
- **Importation** from approved overseas pharmaceutical companies.

Correspondingly, there are also two primary outlets where patients or consumers access these products:

- **Retail pharmacies**, and
- **Hospital pharmacies**.

Both outlets function at the tail end of the drug distribution chain, sourcing products either from local manufacturers or from major importers. Between these two ends of the chain are **distributors and wholesalers**, who serve as intermediaries. Positioned strategically, they often exert significant control over the pharmaceutical business in Nigeria.

2.0 DIFFERENTIATING CORE PHARMACEUTICAL SCIENCE COURSES:

The Relevance of Pharmacy Management Courses

A plant is essentially made up of its root, stem, leaves and fruits. A plant with medicinal value may contain the active pharmaceutical ingredient in any of these parts—or in all of them—in varying concentrations.

2.1 Pharmacognosy

Pharmacognosy is the branch of pharmacy education that focuses on the medicinal importance of plants. The process of separating the active pharmaceutical component from the root, stem, leaf or fruit is known as **maceration**. Over time, various sophisticated techniques have been developed to separate different constituent groups in

medicinal plants. Among these, **chromatography** is widely used, as it enables separation based on melting or boiling points of compounds, among other properties.

2.1.2 Pharmaceutical Chemistry

Once the active medicinal product has been identified, the next step is to isolate it in sufficient quantity. Often, very large amounts of raw plant material are required, which may be impractical. **Pharmaceutical Chemistry** enables us to identify the structure of the active ingredient using instruments, such as the **ultraviolet (UV) spectrophotometer**. Beyond identification, the structure can be synthesized in the laboratory and the medicinal activity of the synthetic equivalent compared with the naturally extracted form. Chemical modifications of the identified structure can also be made to optimize efficacy, enhance safety, reduce toxicity, minimize side effects, improve compliance and decrease resistance. Similarly, for inorganic chemicals of medicinal value, elements within the same group of the **Periodic Table** may be substituted until the most suitable compound for therapeutic use is achieved.

2.1.3 Pharmacology

Once the active ingredient has been isolated and characterized, the next stage is **pharmacological testing**. This involves studying its activity in living tissues. Preclinical studies often begin with laboratory animals—mice, rats, rabbits or monkeys—before progressing to **clinical trials** in human volunteers. These studies determine the **therapeutic dose**, **lethal dose**, **side effects** and the **optimum clinical dose**. The results form the basis for safe and effective therapeutic use.

2.1.4 Pharmaceutics

After pharmacological validation, attention shifts to identifying the best formulation for stability, safety and efficacy. This is the role of **Pharmaceutics**, which determines the most appropriate dosage form—whether tablets, capsules, creams, injectables or inhalation.

Critical formulation considerations include:

- Stability of the product,

- Expiry date of the active ingredient,
- Compatibility with additives such as coloring agents and preservatives,
- Dissolution and disintegration times for solid dosage forms, and
- Resistance to breakage (friability).

These parameters ensure the delivery of a high-quality pharmaceutical product.

2.1.5 Clinical Pharmacy and Pharmacy Management

With the product formulated, **Clinical Pharmacy** evaluates its effects in live patients in hospital settings. Interactions with clinicians, pharmacists and nurses are studied, including **drug–drug**, **food–drug**, and **drug–diagnostic reagent** interactions. However, before clinical use is possible, **Pharmacy Management** becomes critical. It ensures that medicines reach patients efficiently through organized systems of importation, manufacturing, distribution and retail. Pharmacy Management involves the coordinated use of scarce resources—**man, money, materials, machines and methods**—across pharmaceutical industries, wholesalers, distributors, hospital pharmacies and retail pharmacies. Poor or ineffective management can result in unavailability or unaffordability of medicines, regardless of how well they have been researched or developed. In essence, **Pharmacy Management is to pharmacy what Community Medicine is to medicine**: a discipline that ensures products reach patients in a safe, affordable and professional manner.

3.0 The Roles of Drug Regulatory Agencies

Two major regulatory agencies of the Federal Government oversee drug matters in Nigeria:

1. **The National Agency for Food and Drug Administration and Control (NAFDAC)** – Established in 1993 by Decree (now Act) 13, NAFDAC regulates and controls the importation, exportation, manufacture, advertisement, distribution, sale and use of food, drugs, cosmetics, medical

devices, bottled water and chemicals. It is responsible for the **registration of all pharmaceutical products** and ensures compliance with **current Good Manufacturing Practice (cGMP)**.

2. **The Pharmacists Council of Nigeria (PCN)** – Established in 1992 by Decree (now Act) 91, the PCN regulates and controls pharmacy education and practice in all its dimensions. It inspects, approves, registers, licenses and monitors all pharmaceutical premises, including manufacturing plants, importation firms, wholesale outlets, retail pharmacies, hospital pharmacies and patent medicine shops.

The relationship between NAFDAC and PCN is **symbiotic**:

- Only **NAFDAC-registered products** should be stocked and dispensed in **PCN-licensed premises**.
- Any product not registered by NAFDAC is **fake**, regardless of origin.
- Any outlet not licensed by PCN is **illegal**, regardless of location.

Together, NAFDAC and PCN play complementary roles in safeguarding the health of Nigerians.

4.0 Sources of Drugs in Nigeria

There are two official sources of drugs in Nigeria:

1. **Local manufacturing companies**, represented by the **Pharmaceutical Manufacturers Group of the Manufacturers Association of Nigeria (PMG-MAN)**.
2. **Licensed importers**, represented by the **Association of Pharmaceutical Importers of Nigeria (APIM)**.

Both bodies are central to any effective and transparent distribution model. Additionally, the **Federal Ministry of Health (FMOH)**, through its Food and Drugs Services Department, works in

collaboration with NAFDAC and PCN to sanitize the distribution system. If every drug produced locally or imported is **registered by NAFDAC** and **distributed only through PCN-licensed premises**, patients and consumers would be guaranteed access to safe and effective medicines.

5.0 The Situation with Nanomedicines

The current distribution system for orthodox pharmaceutical products in Nigeria is widely acknowledged as inadequate. The question arises: should this same flawed model be applied to emerging therapies such as **nanomedicines**? The answer is clearly **no**. Advances in **DNA research, nanosciences and nano-technologies** promise innovative treatments. However, without a reliable distribution model, these breakthroughs could fail to reach patients safely. As Molly Stevens and Clive Roberts note in *Pharmacy 2020*:

“...the increasing demands of understanding how modern medicines work at the molecular level, the shift towards predictive, preventive and personalized healthcare, and challenges from nanotechnology and stem cell technology have all added to the need for pharmacists to remain the experts in medicines.”

Ultimately, the responsibility of pharmacists is not only to understand medicines but also to ensure **patients have safe and timely access** to them.

6.0 Computer Software Solutions

A computerized monitoring system can revolutionize pharmaceutical distribution in Nigeria. Just as the **Central Bank of Nigeria (CBN)** monitors financial transactions nationwide through electronic systems, a similar approach can be adopted in pharmacy.

A pharmaceutically-designed software system would:

- Track suppliers, wholesalers, retailers and patients across the distribution chain;

- Require importers and manufacturers to log all transactions at the point of invoicing;
- Ensure traceability of products from source to end-user;
- Operate only within **PCN-licensed premises**.

Such an electronically monitored regime would greatly enhance transparency, accountability, and patient safety.

7.0 Conclusion

This paper reviews the current distribution model for orthodox pharmaceutical products in Nigeria, its limitations and the urgent need for reform in preparation for future innovations, such as nanomedicines. By adopting **computerized monitoring systems** and strengthening collaboration between **NAFDAC, PCN, PMG-MAN and APIM**, Nigeria can establish a robust, transparent, and patient-centered distribution model. Ultimately, the goal is to guarantee that all medicines—whether conventional or nanomedicines—are safe, efficacious and accessible to the patients who need them most.

The Challenges of Health Regulation in Nigeria: The Big Picture. Published by the Institute of Healthcare Management, Nigeria, by Dr. Ahmed T. Mora, *mni* December 2015.

Introduction

Like all formal organizations, human resources are the heartbeat of health service delivery. This paper examines the relevance of health regulation in Nigeria through the utilization of HRH in healthcare facilities and the challenges faced.

The Institute of Healthcare Management, Nigeria, acknowledges that health regulation in Nigeria faces significant challenges. The Institute has therefore prioritized a comprehensive discussion of these challenges in today's conference. This topic aligns with the broader conference theme, "*Healthcare Management in Nigeria: Challenges and Prospects*", and specifically supports the sub-theme on strengthening service delivery, regulation and training. Healthcare services are packaged commodities provided by stakeholders within registered, inspected, approved and licensed facilities. These services are subject to periodic evaluation by relevant health authorities. The aim is to make these services readily accessible to patients for treatment, prevention, or maintenance of health.

Health regulatory agencies are empowered to make regulations under the various Decrees, Acts or Laws of the Federation of Nigeria (LFN) that establish them. They define and advance the training and practice of professionals in their respective fields. These agencies are government parastatals under the supervision of the Federal Ministry of Health (FMOH), each with its own Governing Board or Council. A key question arises: *What type of healthcare service is being delivered, at which facility, by which professional, and at what level of competence?* Inspection and accreditation are the primary tools used by regulatory agencies to evaluate these questions. To carry out these functions, agencies rely on Human Resources for Health (HRH), defined by the World Health Organization (WHO) as:

“Those who promote and preserve health, diagnose and treat diseases, and include health management and support workers who do not provide health services directly” (WHO, 2006).

1. Health Regulatory Agencies in Nigeria

Currently, Nigeria has thirteen (13) healthcare regulatory agencies:

1. Community Health Practitioners Registration Board of Nigeria (CHPRBN)
2. Dental Technologists Registration Board of Nigeria (DTRBN)
3. Dental Therapists Registration Board of Nigeria (DTRBN)
4. Health Records Officers Registration Board of Nigeria (HRORBN)
5. Institute of Chartered Chemists of Nigeria (ICCON)
6. Institute of Public Analysts of Nigeria (IPAN)
7. Medical and Dental Council of Nigeria (MDCN)
8. Medical Laboratory Science Council of Nigeria (MLSCN)
9. Medical Rehabilitation Therapists Registration Board of Nigeria (MRTRBN)
10. Nursing and Midwifery Council of Nigeria (NMCN)
11. Optometrists and Dispensing Opticians Registration Board of Nigeria (ODORBN)
12. Pharmacists Council of Nigeria (PCN)
13. Radiographers Registration Board of Nigeria (PRBN)

These agencies are empowered to make regulations under the relevant Decrees, Acts or Laws of the Federation of Nigeria (LFN) establishing them. They define and advance the training and practice of their respective professions. All agencies are government parastatals under the FMOH, each with a separate Governing Board or Council. Their core functions include:

- Registration and licensing of practitioners
- Registration and licensing of practice premises
- Oversight of Continuing Professional Development (CPD) programs

These functions are highly professional and ethics-laden, ensuring compliance with standards of practice. In essence, the main functions of health regulatory agencies are:

1. **Inspection of premises** where registered professionals practice
2. **Accreditation of training institutions** to ensure compliance with approved Benchmark Minimum Academic Standards (BMAS)

For example, the Pharmacists Council of Nigeria (PCN) was established under Act 91 of 1992 (now LFN No. 17 of 2004) to control and regulate pharmacy education and practice comprehensively. A more detailed example is the Community Health Practitioners Registration Board of Nigeria (CHPRBN), established under Cap. C19:

Duties of the Board:

- a) *Determine and improve the standards of knowledge and skills for community health practitioners*
- b) *Establish and maintain a register of members, publishing lists periodically*
- c) *Conduct examinations and award certificates or diplomas, prescribing fees for these services*
- d) *Perform other functions as conferred by the Act*

1.1 The Registrar

In all agencies, the **Registrar**, who serves as the Chief Executive Officer (CEO), is responsible for administration in accordance with Public Service Rules. Recruitment follows a transparent process of advertisement, shortlisting, interviews and selection. The Registrar must be a registered and licensed professional in the relevant field. For example, under the MDCN Act:

- The Council appoints a fit medical practitioner or dental surgeon as Registrar

- The Registrar prepares and maintains registers of names, addresses, qualifications, and other relevant details of all registered practitioners

The title “Registrar” is appropriate because this officer maintains the official registers of both members and premises of practice.

1.2 Staff of Regulatory Agencies

Regulatory agencies recruit staff as public officers to assist the Registrar in discharging regulatory functions. Recruitment follows Public Service Rules, as seen in the Nursing and Midwifery Council of Nigeria:

- The Council may appoint employees as needed
- It can advance, confirm, promote, transfer, discipline, or terminate staff appointments

Staff rights and privileges are contained in the “Conditions of Service” for each agency. Staff are public officers, paid salaries and benefits approved by the Office of the Accountant-General of the Federation (AGF) and are subject to retirement in accordance with Civil Service rules.

1.3 Governing Boards/Councils

Each regulatory agency has a Governing Board or Council as a policy-making body. The composition varies across agencies. For example, under the Medical Laboratory Science Council of Nigeria (MLSCN) Act, 2003:

Composition of the Governing Board:

- a) A Chairman, a qualified fellow with at least 5 years post-qualification experience appointed by the President on the recommendation of the Minister from a list submitted by the Association of Medical Laboratory Scientists of Nigeria
A representative of the Federal Ministry of Health and Social Services

- b) Twelve members representing the six geopolitical zones, with rotational representation
- c) Two registered members from Nigerian universities offering Medical Laboratory Science, on rotation
- e) A representative of private Medical Laboratory Science practitioners
- f) A representative of the Association
- g) A representative from a health-focused NGO representing public interest.

Such is the composition of most of the Boards or Councils. The number of the members on such Boards is produced in the following table:

SN	REGULATORY AGENCY	NO OF MEMBERS ON THE BOARD
1	Pharmacists Council of Nigeria (PCN)	63
2	Medical and Dental Council of Nigeria (MDCN)	61
3	Institute of Public Analysts of Nigeria (IPAN)	20
4	Medical Laboratory Scientists Council of Nigeria (MLSCN)	19
5	Nursing and Midwifery Council of Nigeria (NMCN)	18
6	Institute of Chartered Chemists of Nigeria (ICCON)	17
7	Dental Therapists Registration Board of Nigeria (TRBN)	17
8	Medical Rehabilitation Therapist Registration Board of Nigeria (MR TB)	17
9	Optometrists and Dispensing Opticians Registration Board of Nigeria (ODORBN)	15
10	Dental Technologists Registration Board of Nigeria (DR TBN)	13
11	Radiographers Registration Board of Nigeria (RRBN)	12
12	Health Record Officers Registration Board of Nigeria (HRORBN)-	12
13	Community Health Practitioners Registration Board of Nigeria (CHPRN)	11

There is, however, a difference between the **Institute of Chartered Chemists of Nigeria (ICCON)** and the **Institute of Public Analysts of Nigeria (IPAN)** regarding the composition of their Boards and the designation of their chairpersons, who are called **Presidents**. For ICCON, the President and Vice President are **elected by the Council**. The relevant provision in the Act states:

“There shall be for the Institute a President and a Vice President who shall be Fellows of the Institute, to be elected by the Council and hold office each for a term of two years from the date of election.” (*Chapter 112, Institute of Chartered Chemists of Nigeria Act*).

The same election process applies to IPAN. In contrast, the Chairpersons of the other eleven (11) regulatory agencies are **appointed by the President of Nigeria, Commander-in-Chief of the Armed Forces, on the recommendation of the Honourable Minister of Health**. As noted, ICCON and IPAN remain unique in having their leadership **elected rather than appointed**. A common feature across these regulatory bodies is the provision in their establishing Acts for the constitution of **Investigating Panels and Disciplinary Tribunals**. For instance, the **Medical Rehabilitation Therapists (Registration, etc.) The Act** provides for a **Disciplinary Committee**, stating:

“There shall be established a Disciplinary Committee to be known as the Medical Rehabilitation Therapists Practitioners Disciplinary Committee (in this Act referred to as ‘the Disciplinary Committee’), which shall be charged with the duty of considering and determining any case referred to it by the Panel established by the following provisions of this Act” (*Chapter M9, Medical Rehabilitation Therapists (Registration, etc. Act)*)

As regulatory agencies, these Institutes, Boards and Councils were established to **regulate the education and practice of their respective healthcare professions**. Inevitably, there will be deviations among practitioners that must be corrected to **maintain professional standards**. Regulatory mechanisms act both as **punishment for offenders** and as a **deterrent for potential violators**.

Issues relating to **professional misconduct or ethical violations** are typically investigated by the Panels constituted under the various Acts. Following an investigation, cases are referred to the **Disciplinary Tribunal**, where hearings are conducted openly. The **Attorney-General of the Federation** appoints assessors to assist Panels and Tribunals during their proceedings.

1. The Role of Health Regulatory Agencies in Advancing Healthcare Delivery Services in Nigeria

Emeritus Professor **Umaru Shehu, FAS**, a renowned medic and academic, summarized the roles of professional regulatory bodies as follows:

“The roles of professional regulatory bodies are usually well-defined and largely similar worldwide. They include:

- Setting minimum entry qualifications for students aspiring to join the profession
- Approving curricula for training in various professions
- Monitoring the training of students to ensure compliance with regulations
- Implementing certification and re-certification programs
- Facilitating Continuing Professional Development (CPD) for practitioners
- Ensuring compliance with professional ethics and moral standards in practice”

(CRHP, 2008)

While each healthcare regulatory agency has its specifics, it is the **team approach to healthcare provision** that translates into excellent service delivery. **Prof. Dennis Agbonlahor**, the former Chairman of the Medical Laboratory Science Council of Nigeria (MLSCN), emphasized:

“Today's patient care and healthcare practice is multi-professional and multi-disciplinary. It involves teamwork across many healthcare professions. Recognizing that all participants contribute equally valid knowledge and expertise from their professional and personal experiences is essential to working effectively as a team.” (*CRHP, 2011*)

At the global level, this **teamwork approach** was further emphasized in 1999 when representatives of the **World Medical Association (WMA)**, the **International Pharmaceutical Federation (FIP)** and the **International Council of Nurses (ICN)** created the **World Health Professions Alliance (WHPA)**. In 2005, the **World Dental Federation (WDF)** joined the alliance. Together, these four healthcare professions, representing **25 million professionals worldwide**, aim to **facilitate collaboration among key health professionals, governments, policy makers and the World Health Organization (WHO)**. Working collaboratively, rather than along parallel tracks, enhances patient care and strengthens healthcare systems globally.

I had the privilege of attending the **First World Health Professions Conference on Regulation** in Geneva, Switzerland, from **May 17–18, 2008** organized by the WHPA. The theme was “*The Role and Future of Health Professions Regulation*”. The conference fully acknowledged the **critical role of regulatory agencies in maintaining healthcare systems** and safeguarding patients. The importance of **government funding for inspections and accreditation** was repeatedly emphasized during the event.

2.1 Accreditation and Inspection of Healthcare Facilities

Institutions that train healthcare professionals must be **accredited by the relevant health regulatory agencies** to ensure that the curriculum, course content or BMAS (as applicable) complies with the standards approved by the agencies. Accreditation evaluations may cover programs in **training institutes, Schools of Health Technology, Polytechnics or Universities**. A team of experts from within the profession is typically assembled by the **Governing Council, Board or Management of the Regulatory Agency**, depending on the program being evaluated. This evaluation may concern either **new or ongoing programs**. Another form of accreditation applies to **Housemanship or Internship centers**, such as those for medical doctors, pharmacists and radiographers. These programs usually take place within **secondary or tertiary health institutions** or recognized private healthcare facilities.

Supervising professionals must **actively oversee the trainees**. For instance, the **Pharmacists Council of Nigeria (PCN)** requires a **logbook** signed by the **Supervising Pharmacist** (who must have at least five years of post-registration experience) and countersigned by the **Director of Pharmaceutical Services (DPS)** of the State Ministry of Health. This applies to all internship centers, whether publicly or privately owned.

Inspections of healthcare facilities ensure that services meet **minimum standards of practice**. Inspections vary in purpose:

- **Licensing inspections:** For new healthcare facilities
- **Compliance inspections:** For registered facilities to ensure adherence to standards and public safety

For example, the **MLSCN Inspector** verifies the license status of laboratory scientists in both private and public diagnostic centers, while the **Pharmaceutical Inspector** inspects manufacturing plants,

hospital pharmacies, retail pharmacies and wholesale or importation companies.

The **Nursing and Midwifery Council of Nigeria (NMCN)** faces a significant challenge in inspecting over **200,000 nurses, midwives and auxiliary nursing staff**, ensuring that quack practitioners are identified and removed. Similarly, PCN addresses the menace of fake drugs through **regular inspections**. Regulatory agencies must maintain **standards of practice** through consistent accreditation and inspections.

To facilitate this, agencies use **protocols and evaluation tools**, including:

- Evaluation forms
- Standard Operating Procedures (SOPs)
- Monographs and books on Minimum Standards

The PCN, for example, has published the “**Four-Part Compendium of Minimum Standards of Practice for the Assurance of Pharmaceutical Care in Nigeria**”, which serves as both a **practitioner’s guide** and an **inspector’s evaluation tool**.

Inspection teams carry out evaluations, with accreditation occurring periodically and inspections continuously. Agencies, such as **MDCN, PCN and MLSCN** are regularly involved in one form of inspection or another. In PCN, the **Governing Council** has established **Pharmaceutical Inspection Committees (PICs)** at the state level led by the DPS of the State Ministries of Health and the Federal Capital Territory (FCT). These committees inspect **retail pharmacies, wholesale and importation pharmaceutical firms**. Similarly, **Patent and Proprietary Medicine Vendor License (PPMVL) Committees** inspect outlets for patent and proprietary medicines. These committees may include **professionals not employed by the PCN**. Most are private pharmacists and pharmacy owners who undergo **intensive training by PCN** and are issued

Pharmaceutical Inspector ID cards. In contrast, inspections of pharmaceutical manufacturing facilities are conducted **exclusively by public officers** from the PCN and State Ministries of Health.

In the case of the **Radiographers Registration Board of Nigeria (RRBN)**, inspections of imaging centers in general and teaching hospitals, whether public or private are **centrally coordinated from the RRBN Registry in Lagos** due to the **specialized nature of the instruments** and the **limited number of facilities nationwide**.

Inspectors function as the “**police**” of **healthcare facilities**. At PCN, inspections are **corrective, not punitive**. Emphasizing punishment may discourage stakeholders from presenting their facilities for inspection.

2.2 Continuing Professional Development (CPD) Programmes

The **competence, expertise and skills** of healthcare professionals (HRH) must be maintained at an **optimal level**. Regulatory agencies ensure practitioners maintain **high professional competence** to:

- Avoid medication errors
- Maintain confidence among peers and patients
- Effectively collaborate with co-healthcare professionals

As healthcare delivery becomes more specialized, from **primary to tertiary levels**, the need for **lifelong learning** grows. The complexity of healthcare, changing environments, lifestyles, re-emergence of diseases and resistant microorganisms demands that healthcare professionals **continually update their knowledge and skills**. **Continuing Professional Development (CPD) programs** provide this framework. At the **National Council on Health (NCH) meeting in Asaba, Delta State (April 2010)**, participation in CPDs and acquisition of necessary credit units became **mandatory for all healthcare professionals** for license issuance. CPD emphasizes the “**sharing**” aspect of learning, which enhances understanding. The

concept was highlighted at the **National Health Summit of 1996**, which emphasized the need for CPDs due to falling standards in tertiary education.

In 1998, PCN launched the **Mandatory Continuing Professional Education (MCPE)** program, with faculties of Pharmacy or Pharmaceutical Sciences serving as provider centers. The program was later renamed the **Mandatory Continuing Professional Development (MCPD)** program in 2004, engaging **university faculty and private practitioners** as resource persons. Over time, some pharmacists expressed dissatisfaction, noting that lecture notes were repetitive of undergraduate material and lacked practical application. In response, PCN collaborated with **Partners for Transforming Health Systems (PATHS)** to **revamp course content and implement train-the-trainer programs**, making CPD more practical and impactful.

2.3 The Committee of Registrars of Health Professions (CRHP)

August 19, 2004, marked a landmark in Nigerian and West African healthcare regulation. On this date, **Honourable Minister of Health, Professor Eytayo Lambo**, inaugurated the **CRHP** with the following objectives:

- Raise ethical standards in healthcare delivery
- Set performance targets for efficiency
- Facilitate CPD for health professionals
- Enlighten the public on health matters
- Harmonize regulatory activities for efficiency
- Promote synergy across government healthcare efforts
- Foster unity and mutual respect among health professionals

In his inauguration address, Minister Lambo stated:

“It is heart-warming that Registrars of all regulatory bodies in the health sector are coming together under

one umbrella. This is commendable and should be applauded. I urge you to use this Committee as a springboard to rejuvenate the team spirit that is fast fading away.” (*Lambo, 2004*)

Interconnectivity among health regulatory agencies is **essential for coordinated healthcare delivery**, especially as inspectors from different agencies may operate in overlapping facilities. Collaboration ensures **harmony among healthcare professionals** and optimizes service delivery across **primary, secondary, and tertiary healthcare facilities**.

3.0 The Challenges of Health Regulation

Key functions of health regulatory agencies include **accreditation, inspections, CPD and maintenance of practitioner registers**. Provisions for **Investigating Panels and Disciplinary Tribunals** are crucial for **professional ethics and conduct**. Some major challenges in Nigerian health regulation include:

1. **Inadequate Numbers of HRH** – Limits inspection frequency, quality, and integrity. Governments should recruit, train and fairly compensate inspectors.
2. **Housemanship or Internship Centre Management** – Graduates require adequate placement for practical tutelage.
3. **Poor Logistics Support for Inspectors** – Agencies need vehicles and communication equipment for effective inspections.
4. **Inadequate Training of Inspectors** – Inspectors must receive national and international training to remain technically competent.

5. **Limited or Poorly Packaged CPD Content** – CPD programs should be practical, delivered by professionals with field experience, and tied to license issuance.
6. **Interference by Professional Associations** – Regulatory functions must remain independent from NGOs or voluntary associations.
7. **Frequent Dissolution of Boards/Councils** – Premature board dissolution undermines policy implementation and supervision.

3.1 Inadequate Numbers of Human Resources for Health (HRH)

A **shortage of HRH** directly affects the **frequency, quality and integrity of inspections**. Just as the government recruits' personnel for the **Police, Army, Air Force and Navy**, it must also **recruit, employ, train and develop inspectors** for health regulatory agencies. Inspectors should be **well-compensated** to maintain focus and avoid compromising integrity. They should **never be drawn from the pool of those being inspected**, except in cases requiring intelligence gathering.

3.2 Creation or Upgrading of Housemanship and Internship Centres

Graduates from **medical schools, faculties of pharmacy and other relevant healthcare programs** require structured tutelage. Adequate **placement for Housemanship or Internship** is essential to ensure practical training. Universities must admit and forward graduates to properly **accredited centers** to complete their required professional training.

3.3 Poor Logistics Support for Inspectors

Many inspectors **lack essential logistics** to carry out inspections effectively. Regulatory agencies must be **adequately funded** to procure **reliable vehicles, communication equipment, and other necessary tools** to facilitate inspections.

3.4 Inadequate Training and Development of Inspectors

Inspectors often **lack adequate knowledge and technical training** to perform their duties effectively. They should receive **comprehensive training, both locally and internationally**, to remain technically competent and ahead of the facilities they inspect.

3.5 Inadequate Course Content in CPD Programmes

Despite the **National Health Summit's directive sixteen (16) years ago**, some agencies have **not implemented CPD programs**, while others offer content that is **too academic and not practical**. Practitioners who have excelled in their fields should be **engaged as resource persons** for CPD programs. **License issuance or renewal** should be contingent upon **successful participation in CPD programs**, ensuring that all healthcare professionals remain fit and proper to practice.

3.6 Interference by Professional Associations

Professional associations are **non-governmental organizations (NGOs)** and **voluntary in nature**. However, some associations have attempted to **usurp the regulatory roles of health agencies** through intimidation, blackmail, threats or coercion. This is **unacceptable**, as regulatory staff are **public officers trained with public funds** to implement their mandates. Regulatory functions must **remain independent** and not be influenced by professional associations.

3.7 Frequent Dissolution of Governing Boards/Councils

Frequent **dissolution of Governing Boards or Councils** before the completion of their terms disrupts **policy-making and effective management supervision**. Boards and Councils should serve their **full terms**, except in cases where the **Federal Ministry of Health** determines that members are non-performing or lack focus.

4.0 Conclusion

The **relevance of health regulatory agencies in Nigeria** cannot be overstated. While recommendations such as the **Orosanye Committee on Parastatals** suggest potential mergers of some parastatals—including health regulatory agencies—the **fundamental need remains effective regulation and control** of healthcare education and practice. This can only be achieved through **public officers who are well-trained, accountable and governed through proper public service channels**. Government should **fully fund all health regulatory agencies**, as the **quality of healthcare accessible to Nigerians**—in both registered and unregistered health facilities—**depends on the effectiveness and efficiency of these regulatory activities**.

References

- **Acts for the Establishment of all Thirteen Health Regulatory Agencies**
- CRHP (2008): *The Role of Health Regulatory Bodies in Advancing Healthcare Delivery Services in Nigeria* — Proceedings of a One-Day Advocacy Workshop, Abuja, 2008, 91p
- CRHP (2011): *Harmony in the Healthcare Delivery System: The Role of Regulatory Agencies* — Proceedings of a One-Day Advocacy Workshop, Abuja, 2011, 75p
- FRN (2007): *National Human Resources for Health Strategic Plan (2008–2012)*, 2007, 83p
- Lambo, E. Prof. (2004): *Address by Prof. Eyitayo Lambo on the Inauguration of the Committee of Registrars of Health Professions (CRHP), August 19, 2004, SGF Conference Room, Federal Secretariat, Abuja*

Interfacing Practitioners with Students to Achieve an Apex Healthcare Learning Environment. Presented by Dr. Ahmed Tijjani Mora, Dean, Faculty of Pharmaceutical Sciences, KASU, at a Workshop for Staff of Barau Dikko Teaching Hospital (a Tertiary Healthcare Provider) held at Hotel Seventeen, Lafia Road, Kaduna, on Wednesday, April 13, 2016.

Preamble

The Barau Dikko Teaching Hospital (BDTH), Kaduna is not the first to migrate from being a General Hospital (a secondary healthcare provider facility) to a Teaching hospital (a tertiary healthcare provider facility). Indeed, most teaching hospitals and/or tertiary healthcare institutions were once secondary healthcare providers. Even before the establishment of the Faculty of Medicine, Ahmadu Bello University (ABU), Zaria in 1967, the Ahmadu Bello University Teaching Hospital (ABUTH), Zaria had been in existence since colonial days and it was known as "*Asibitin Kofar Doka*". It was the facilities of this hospital later named General hospital, Zaria that were upgraded as ABUTH, Tudun Wada, Zaria before the present permanent structure at Shika, which was officially commissioned on 4th November, 2005, some thirty (30) years since construction started. (Mahmoud H et al, 2012).

Similarly, the Federal Neuro-Psychiatric hospital, Kaduna was established by the defunct North Central State Government as a psychiatric, 100-bed hospital in 1975. The Federal Government took over the hospital from the Kaduna State Government in 1996, following the recommendations in 1994 of the National Committee for the Review of Psychiatric Hospitals and Services in Nigeria (FMOH, 2007).

The upgrading of the facilities of the former Barau Dikko Specialists Hospital to transform it to BDTH is therefore not unusual. With this upgrade, the institution will, in addition to offering healthcare services to patients, now train healthcare personnel to perform professional health care services within the hospital as trainees and, upon qualifying, add to the much-needed human resources in health (HRH) to service the nation's health institutions at different tier-Primary, Secondary or Tertiary as the case may be. In the case of

BDTH, its conversion by law is to effectively serve as the institution with the appropriate facilities and expertise for medical students of the Kaduna State University (KASU) to be trained during their clinical years of training presently and indeed for other courses whose students would require the facilities for their professional training as well.

It is from the point of view of this migration from being a secondary to a tertiary healthcare facility that BDTH, as it is, will be discussed in this paper, especially with the now new addition of students who would be taught (hence teaching hospital) in the hospital scenario to the hitherto only healthcare professionals and other staff and the patients and their relatives that transverse the corridors of the hospital. The additional work of "Teaching" in the name of the hospital says it all. The hospital staff need this kind of workshop in order to be re-oriented as to the new status, focus, aims and objectives of the hospital now as a teaching hospital of a state-owned University which is KASU.

Again, a new cadre of staff will be recruited as part of the upgrade requirement to ensure proper teaching of the students is affected. Healthcare regulatory agencies, such as the Medical and Dental Council of Nigeria (MDCN), the Pharmacists Council of Nigeria (PCN), the Medical Laboratory Science Council of Nigeria (MLSCN) will now assemble team with greater expertise to visit the hospital more often for inspection and accreditations of one healthcare professional programme or the other, as the case may be, due to the new tertiary status of the facility. For this too/ the conduct of staff; their skills and competencies in handling patients who will now be received on referral basis from general hospitals within the state capital and beyond must be enhanced and this workshop provide one of such environments.

Vision and Mission of BDTH, Kaduna

Vision: To be a Tertiary Healthcare facility of highest ethical standard providing total healthcare training and services to our clients.

Mission: To offer highly specialized collaborative and integrated research and tertiary healthcare services and training that would meet

the healthcare expectation of our clients in a conducive setting that promotes hope/ motivation and dignity to mankind,

2.0 THE OF HARMONIOUS LEARNING AND TEACHING ENVIRONMENT

One of the greatest advantages in establishing BDTH as a teaching hospital is its presence within Kaduna Metropolis, which has four (4) other public Specialist tertiary healthcare provider facilities as follows: -

- ❖ The Federal Neuro-Psychiatric hospital in Barnawa;
- ❖ The National Eye Centre by the western bye-pass, Kaduna;
- ❖ The National Ear Care Centre, just across the road and;
- ❖ The 44 Armed Forces Reference Hospital by the Sir Kashim Ibrahim House.

The emergence of BDTH as a fifth public tertiary healthcare facility in Kaduna Metropolis will certainly elevate the quality of healthcare services to the populace at specialist level albeit on referral basis. Needless to add, there would be the much-expected collaboration and referral of patients across these tertiary healthcare provider facilities and more importantly, professional exchanges and advocacies in terms of workshops, seminars and such other informed and learned meetings to sharpen skills and expertise of the staff. The greatest beneficiaries would be the students specially the medical students, who could benefit from specialist knowledge from consultants in these four (4) other public tertiary healthcare Institutions that would, as time goes on, be invited or be integrated within the BDTH teaching structure as per their specialized healthcare training as consultants, to the students due to proximity.

All healthcare professional practices entail long-life learning and as such the competencies, skills and expertise of the practitioners as specialists or consultants in the new teaching hospital must be sharpened from time to time to meet up with the dynamic nature of healthcare sciences. For instance, the defunct Institute of Health, ABU, Zaria that midwifed the ABUTH to provide training environment for the medical students at inception was made up of the following constituent institutions:

- The Ahmadu Bello University Hospitals, Clinics and Dispensaries;
- The Ahmadu Bello University School of Nursing;
- The Ahmadu Bello University Medical Auxiliaries Training School;
- The Ahmadu Bello University School of Hygiene;
- Dala Orthopedic Hospital Kano with linkages to the Royal National Orthopedic Hospital, UK;
- General Hospital Malumfashi and associated Health Centres and Village Dispensaries;
- Guinness Eye Centre and the Maxillo-facial Unit.

These constituent units provided facilities for training personnel in the fields of medicine and health sciences while providing services and the medium for conducting research into health problems of the community. It is necessary for BDTH to make research into various health sectors that affects the immediate community.

The availability of a library with relevant titles for the various practitioners to refer and consult to is needed. The teaching hospital must see itself automatically as a provider centre for Continuing Professional Development (CPD) programme for all the healthcare professions. It is not just enough for BDTH to operate as apex referral centre within the provision of the National Health Policy (NHP) alone, it must be a leading centre in the provision of data, information and expertise in all the health care professions. For instance, the Pharmaceutical Services Department should be equipped with state-of-the-art equipment or instruments, such that pharmacy students from KASU are not only taught practical cases in clinical pharmacy and pharmacy management in the hospital, but that other hospital pharmacy departments in the surrounding general hospitals should look up to the departments for guidance. The various departments and units should be providing Bulletins for their various professional updates for the entire hospital community to benefit from. In other words, a village structure of healthcare professionals, supporting staff and students should evolve and work in harmony with each other towards achieving the set goals and objectives of the teaching hospital.

3.0 PROFESSIONALISM AND WORK ETHICS

A solid foundation of professionalism and work ethics is needed among the various healthcare personnel in the teaching hospital. All healthcare professionals in the teaching hospital should be role models for the students and serve as epitome of hope for the patients and patient's relations. As healthcare professionals, the staff members are bounded by the following attributes:

- Expertise in specialized body of knowledge;
- A registration body;
- Possession of license to practice;
- Having been administered oath;
- Code of Conduct;
- Ethics of their various profession;
- Necessity for CPD to update professionalism and;
- Altruistic motivation for choosing to be healthcare professionals.

With these attributes, it means the expectations from our various regulatory and licensing agencies must always be prominent in our minds as we practice or teach the students or trainees placed before us. One of the best ways to live up to the expectations of our various regulatory agencies is drafting and observance of Standard Operating Procedure (SOPS) for the implementation of every major component of our professional services. In that way, professionals would have properly checked and armed themselves to ensure repetitive and system approach in handling every case.

Which are these regulatory agencies? They are as follows:

- ❖ Community Health Practitioners Registration Board of Nigeria;
- ❖ Dental Technologists Registration Board of Nigeria;
- ❖ Dental Therapists Registration Board of Nigeria;
- ❖ Health Records Officers Registration Board of Nigeria;
- ❖ Medical and Dental Council of Nigeria;
- ❖ Medical Laboratory Science Council of Nigeria;

- ❖ Medical Rehabilitation Therapists Registration Board of Nigeria;
- ❖ Nursing and Midwifery Council of Nigeria;
- ❖ Optometrists and Dispensing Opticians Registration Board of Nigeria;
- ❖ Pharmacists Council of Nigeria;
- ❖ Radiographers Registration Board of Nigeria;
- ❖ Institute of Chartered Chemists of Nigeria;
- ❖ Institute of Public Analysts of Nigeria.

4.0 QUALITY AND QUANTITY IMPROVEMENT OF LEARNING ENVIRONMENT

One of the first major challenge for all the staff of the new teaching hospital is handling or relating with students. From the days of the old Nursing Home, to Sani Abacha Children Hospital, to the New General hospital, to Barau Dikko Specialist hospital and now Barau Dikko Teaching Hospital, staff members have always been by themselves alone and their patients and/or patient's relatives to contend with. Now with addition of students among the hospital community, there must be proper structure on how the professionals and even other non-professionals who do not teach relate with the students.

4.1 Quality

Seeing and examining patients that hitherto takes few minutes will now take longer period due to the teaching nature of the practice. Students would have to be given the opportunity to review patients' medication, ask questions and be asked too and to document all observations.

4.2 Quantity

The operation at BDTH will now be a 24 hour, seven days a week continues activity with lots of documentation, especially from one shift team to another. Students will need to be allowed time and space to document findings. Statistics would have to be kept, data analyzed and findings shared. There will be more work, no doubt.

5.0 A TEMPLATE FOR HARMONIOUS RELATIONSHIP BETWEEN' HEALTHCARE PROFESSIONAL STAFF AND MEDICAL AND PHARMACY STUDENTS.

The students from the Faculty of Medicine, KASU would practically be all over the teaching hospital once they've passed their pre-clinical years and their learning shifts to the teaching hospital. Pharmacy students would be present in the hospital in their 4th and 5th year for courses in Clinical Pharmacy.

- ❖ Dental Department;
- ❖ Ear, Nose and Throat Department; Other areas are as follows:
 - Laboratory for Haematology, Medical Microbiology, Chemical Pathology,
 - Morbid Anatomy and Forensic Pathology experiences.
- ❖ Radiology Department;
- ❖ The Wards to observe and review patients;

5.1 Medical Students

The MB, BS students on clinical posting to BDTH will have to continually be in contact with the following departments or units:

- ❖ General Outpatient Department (GOPD);
- ❖ The operation theatres;
- ❖ Obstetrics and Gynaecology Department;
- ❖ Paediatrics Department;
- ❖ Ophthalmology Department;

The physiotherapy department to observe response of patients undergoing physiotherapy. Pharmacy department to observe the compounding of medications and be informed of the Hospital formulary which should include different brands of drugs and their strength and dosage forms. Social Welfare, Administration, etc.

5.2 Pharmacy Students

The B. Pharm. Students of KASU start their Clinical Ward rounds during their fourth (4th) year and essentially, their areas of linkages are as follows: -

Pharmacy Department of the teaching hospital where they would be expected to observe and partake in dispensing of prescribed drugs to patients and how they counsel them. They should have proper understanding of the computerized dispensing system as well as Compounding of some extemporaneous preparations on a large scale. Students should examine prescriptions critically to avoid drug-drug, food-drug and drug-diagnostic reagent interactions and avoid medication error generally.

Their ward round is to ensure that the prescribed drug for the inpatients, duly dispensed through the ward nurse, is acting in the patient and giving the desired result as expected. Side effects and patient compliance will be monitored and recorded. There must be harmony between the matron in charge of the Wards and the hospital pharmacist, along with Faculty of Pharmaceutical Sciences lectures that will both serve as preceptors to guide and teach the students in the wards. Concepts like the unit dose dispensing, the Drug Revolving Found (DRF) Scheme operation as well as entries for narcotics, dispensed drugs for patients enrolled for the National Health Insurance Scheme (NHIS) that the students have been lectured the theory aspect would now be practicalised in the teaching hospital.

5.3 The Nursing Care Personnel

The services of the nurse are crucial in any teaching hospital, as the services of the nursing care personnel goes beyond patient care, drug administration and management of the wards. The nurses should provide the environment within the teaching hospital for effective learning and teaching, most of which is in the wards. Attitudes of the medical and pharmacy students and the doctors and pharmacists who takes them round would have to be moderated to confer respect to the Wards and the patients as being managed by the nurses. Entry into the wards would have to be timed at a particular part of the day to ensure that certain categories of patients are not disturbed. In any ward round, the nursing personnel of the Ward must be present and participate in the review of the patient by way of offering valuable information about the patient to the medical or pharmacy teams at any point in time.

6.0 CONCLUSION, RECOMMENDATIONS AND IMPLEMENTATION

STRATEGIES

The teaching of medical courses goes hand-in-hand with the establishment of a teaching hospital, hence the need to upgrade the facilities at the former BDSH, from being of secondary healthcare provider status to BDTH, a tertiary healthcare institution to enable the MB. BS students of Kaduna State University (KASU) an environment for effective learning and practice during their clinical years of training. What took place in ABU, Zaria in 1967 with the establishment of a Faculty of Medicine and closely followed by that of Pharmaceutical Sciences in 1970 appears to be repeating itself in KASU with Faculties of Medicine and Pharmaceutical Sciences established in 2009(?) and 2012 respectively.

The teaching component in the aims, objectives and indeed mission of BDTH makes all the difference in the way and manner the philosophy of provision of healthcare services is provided to members of the public henceforth. The presence of healthcare professionals as staff in BDTH and whose regulatory agencies were established with a sound foundation to control and regulate their various education and practices again emphasized the need for a conducive environment for teaching, learning, research and healthcare delivery of tertiary nature. Similarly, the life-long learning mantra associated with every healthcare profession by way of practitioners subjecting themselves periodically to Continuing Professional Development (CPD) programmes appropriate to their specialisation again necessitate the emphasis of periodic advocacies as in conferences, workshops, seminars and the likes by the different professionals to enhance their skills, expertise and competencies, since BDTH has now been upgraded to a tertiary public healthcare provider facility.

Fourthly, the presence of at least four (4) other specialist and/or tertiary public healthcare facilities within the state capital is a big motivation for BDTH to initiate collaborative discourse with them at inter - professional intellectual basis in addition to the obvious horizontal rapport that will be expected among the five (5) public

healthcare institutions within the state capital. More to the point, the medical and pharmacy students of KASU who would be the direct beneficiaries in the teaching hospital should count themselves very lucky, compared to their counterparts elsewhere in the country.

There is hardly a city in Nigeria with the possible exception of Lagos where one can find concentration of consultants or specialists in various and diverse healthcare fields, such as in Eye, Ear, neuropsychiatry and in trauma (as in Armed Forces reference hospital) as in Kaduna. If one extends it on a state-wide basis, we have the National Tuberculosis and Leprosy Centre in nearby Saye and of course the Federal Government owned conventional University teaching hospital in Shika all in nearby Zaria just an hour drive from Kaduna. There are certainly areas for teaching and learning in terms of collaboration across tertiary/specialists healthcare levels and vertically on referral basis from the multitude of general hospitals that are secondary healthcare provider facilities within the state capital.

In conclusion, I will round up this presentation with some recommendations and implementation strategies as follows:

6.1 Recommendation Number One

The Barau Dikko Teaching Hospital (BDTH) should fully exploit the presence of the four (4) other tertiary/specialist public healthcare provider facilities within Kaduna to improve skills, expertise and competencies of its healthcare professionals' staff which would ensure the imparting of the appropriate advancements in the various specialist areas of tertiary healthcare services to the medical and/or pharmacy students of KASU.

6.1.1 Implementation Strategies

The management of the teaching hospital to organize periodic seminars, conferences and workshops addressing topical subjects of contemporary healthcare importance along with the four (4) other tertiary/specialist public healthcare provider facilities within Kaduna and to be attended by all professionals as well as medical and pharmacy students of KASU; BDTH as the only conventional University teaching hospital among the tertiary/specialist public

healthcare provider facilities within Kaduna to take the lead as the key provider centre for all manner of Continuing Professional Development (CPD) programmes for the healthcare professions within the metropolis to benefit from on a regular basis. The teaching hospital to establish a healthcare journal where research studies from the four (4) other public healthcare teaching/specialist facilities and BDTH itself would be published periodically.

6.2 Recommendation Number Two

The management of the teaching hospital should provide an enabling environment to fully integrate and accommodate the medical and pharmacy students that would now, for the first time in the more than thirty (30) years of the existence of the hospital (as secondary healthcare facility) be additional cadre within the hospital community.

6.2.1 Implementation Strategies

All healthcare professionals regulated by any of the thirteen (13) healthcare regulatory agencies in the country, who are presently under the employment of BDTH to be given some teaching allowances; since beside their specializations duties as healthcare professionals, they will be expected to teach medical and or pharmacy students of KASU in respect of aspects of their professional practices. Adequate lecture rooms to be provided within the hospital to enable students a place to stay in between lectures and ward rounds to avoid wandering into units or departments that may not be ready to receive them. The Wards that are the "main" theatre for teaching the students to be accessible to the later at specific times in order to avoid infringement. Upon the work of the ward nurses or patients rest period. A cafe, restaurant and common room to be established in order to enable both staff and students to interact and socialize outside of units or departments where professional practices take place.

6.3 Recommendation Number Three

The management of the teaching hospital should equip a modern library area with the latest ICT gadgets to ensure access to updated healthcare information at all times for both staff and students.

6.3.1 Implementation Strategies

An electronic library with modern and up-to-date facilities to be installed to aid easy access to relevant healthcare information at all times. A Drug Information Centre (DIC) to be linked to all departments to ensure easy retrieval of information on drugs by both staff and students for the benefit of the patients and for research. Attendance to national and international conferences within Nigeria and abroad to be encouraged through periodic funding of budget code for such.

Thank you all for listening

Dr. Ahmed T. Mora
Wednesday, 13th April, 2016

The Role of Health Regulatory Agencies in Enhancing Effective Healthcare Delivery Services in Nigeria. Presented by Dr. Ahmed Tijjani Mora, *mni*, on the occasion of the 2016 Joint Scientific Conference and Exposition of NAPPSA and ANPA, held in Las Vegas, Nevada, USA, from June 23–26, 2016.

1.0 Preamble

This Paper examines four (4) healthcare regulatory agencies, notably the Health Record Officers Registration Board of Nigeria (HRORBN), the Pharmacists Council of Nigeria (PCN); the Medical and Dental Council of Nigeria (MDCN) and the Dental Technologists Registration Board of Nigeria (DTRBN). A questionnaire was designed and administered to the four (4) agencies and was completed, retrieved and analysed.

The history of regulating healthcare services in Nigeria dates back to 1927, when the Board of Medical Examiners (BME) was established by the British Colonial administration. From the BME, other regulatory agencies evolved at different times to the present thirteen (13) health regulatory agencies that cater for the control and regulation of one healthcare profession or the other and as agencies of the Federal Government under the supervision of the Federal Ministry of Health (FMOH).

The responsibilities of these health regulatory agencies under the leadership of their various Registrars are similar even though the healthcare focus of each of the professions is different. Essentially, they regulate and control the education and practice of these healthcare professions at the Primary, Secondary and Tertiary healthcare provider centres responsible for the provision of Primary healthcare, Secondary healthcare and Tertiary healthcare services in public health institutions, respectively.

Similar control and regulation also take place in all private health care provider facilities. The health regulatory agencies do more. They

ensure the competencies of the healthcare professionals are enhanced as per their skills, expertise and competencies through mounting Continuing Professional Development (CPD) programmes in accredited centres and allotting points to ensure that these healthcare professionals are up-to-date in the knowledge of their various professions for the interest of their patients, students and fellow healthcare colleagues to benefit from

2.0 The Role of Health Regulatory Agencies in Advancing Healthcare Delivery Services in Nigeria

The renowned medic and academic, Professor Emeritus Umaru Shehu, FAS stated as follows in respect of the roles of professional regulatory bodies The roles of professional regulatory bodies are usually well defined and they are more or less the same all over the world. They include the following:

- (i) Setting minimum entry qualifications for students aspiring to Join the profession;
- (ii) Approving the curricular for training in the various professions;
- (iii) Monitoring the training of students to ensure compliance with all the regulations;
- (iv) Certification and re-certification of programmes;
- (v) Facilitating Continuing Professional Development (CPD) programme for practitioners of the profession and;
- (vi) Ensuring compliance with professional ethics and morality in practice (CRHP, 2008).

2.1 The Establishment of the Committee of Registrars of the Health Professions (CRHP)

The date, August 19, 2004 will forever be a special one to members of the healthcare professions in Nigeria and indeed the West African

region. It was on that day that the then Honourable Minister of Health, Professor Eyitayo Lambo, inaugurated the CRHP. The objectives of the CRHP are as follows:

- (i) ***Raise the level of ethical conduct in healthcare delivery services in Nigeria;***
- (ii) ***Set targets for efficient performance***
- (iii) ***Facilitate Continuing Professional Development (CPD) for the Health Professions in Nigeria;***
- (iv) ***Enlighten the general public on health matters;***
- (v) ***Harmonise all regulatory activities to ensure efficiency and effectiveness in the performance of the various functions of the diverse professionals in the health sector;***
- (vi) ***Act as a platform for achieving synergy of the various healthcare delivery efforts of government by harnessing the peculiar and unique strength of the various professionals/ and***
- (vii) ***Foster unity and mutual respect among the health professions in Nigeria.***

While inaugurating the CRHP, the Honourable Minister made the following statement, among others:

It is, therefore, heart-warming that the Registrars of all the regulatory bodies in the health sector are coming together under one umbrella. This is commendable and should be applauded. I want to urge you to use this committee as a springboard to rejuvenate the team spirit that is fast fading away (Lambo, 2004). These highly encouraging but weighty words were re-echoed by Dr. A. A. Ibrahim, the present Registrar of the MDCN and current Chairman of the CRHP, when he stated, while delivering a paper, as the Keynote Address Speaker in Kaduna, on the occasion of Sensitisation Workshop for Staff of Barau Dikko Teaching Hospital (BDTH) of Kaduna State University

(KASU) on becoming a tertiary healthcare provider facility, as follows:

...I must confess that the most single issue, that is threatening to overwhelm every investment that has been made in the healthcare delivery system in Nigeria is not always about the availability of facilities for healthcare delivery, but the issue of inter-professional rivalry among the health professions... (Ibrahim, 2016).

3.0 The Role of Health Regulatory Agencies in Enhancing Effective

Healthcare Delivery Services in Nigeria: A Study of HRORBN; PCN; MDCN and DTRBN

The parameters compared were as follows:

- (a) Year of establishment;
- (b) Applicable Law of the Federation of Nigeria (LFN) establishing the Agency;
- (c) Location of the Head office/Registry;
- (d) Staff strength as at January 31, 2016;
- (e) Basic Responsibilities:
 - (i) Accreditations;
 - (ii) Inspections;
 - (iii) Examinations;
 - (iv) Preparations of Benchmark Minimum Academic Standards (BMAS);
 - (v) Conducting Continuing Professional Development (CPD) programmes;
 - (vi) Preparations of Minimum Standards of Practice;
 - (vii) Registration and Licensing of Professionals;

- (viii) Registration and Licensing Practice Premises;
 - (ix) Capacity building programmes for staff;
 - (x) Others (Specify).
- (f) Ranking of where most of the Agency's regulatory functions take place –
- (i) University;
 - (ii) Tertiary Healthcare facilities;
 - (iii) Secondary healthcare facilities;
 - (iv) Primary healthcare facilities;
 - (v) Private Practices;
 - (vi) Schools for Health Technologies;
 - (vii) Institutions for the training of the regulated professional cadre;
 - (viii) Missionary health Institutions;
 - (ix) Ministries of Health;
 - (x) Others (Specify)
- (g) Ranking in percentages collaboration with sister healthcare regulatory bodies
- i. Community Health Practitioners Registration Board of Nigeria (CHPRN);
 - ii. Dental Technologists Registration Board of Nigeria (DTRBN);
 - iii. Dental Therapists Registration Board of Nigeria (DTRBN);
 - iv. Health Record Officers Registration Board of Nigeria (HRORBN);
 - v. Institute of Chartered Chemists of Nigeria (ICCON);
 - vi. Institute of Public Analysts of Nigeria (IPAN);

- vii. Medical and Dental Council of Nigeria (MDCN);
 - viii. Medical Laboratory Science Council of Nigeria (MLSCN);
 - ix. Medical Rehabilitation Therapist Board Nigeria (MRTBN);
 - x. Nursing and Midwifery Council of Nigeria (NMCN);
 - xi. Optometrists and Dispensing Opticians Registration Board of Nigeria (ODORBN);
 - xii. Pharmacists Council of Nigeria (PCN);
 - xiii. Radiographers Registration Board of Nigeria (RRBN).
- (h) Percentage ranking of challenges faced by the Healthcare regulatory bodies: -
- i. Lack of personnel;
 - ii. Poor funding;
 - iii. Lack of adequate logistics;
 - iv. Unsuitable office accommodation;
 - v. Interference by the supervising Federal Ministry of Health (FMOH)
 - vi. Lack of training and development of registry staff;
 - vii. Interference by the professional Association;
 - viii. Infighting among registry staff;
 - ix. Lack of staff motivation;
 - x. Others.

3.1 THE HEALTH RECORD OFFICERS REGISTRATION BOARD OF NIGERIA (HRORBN)

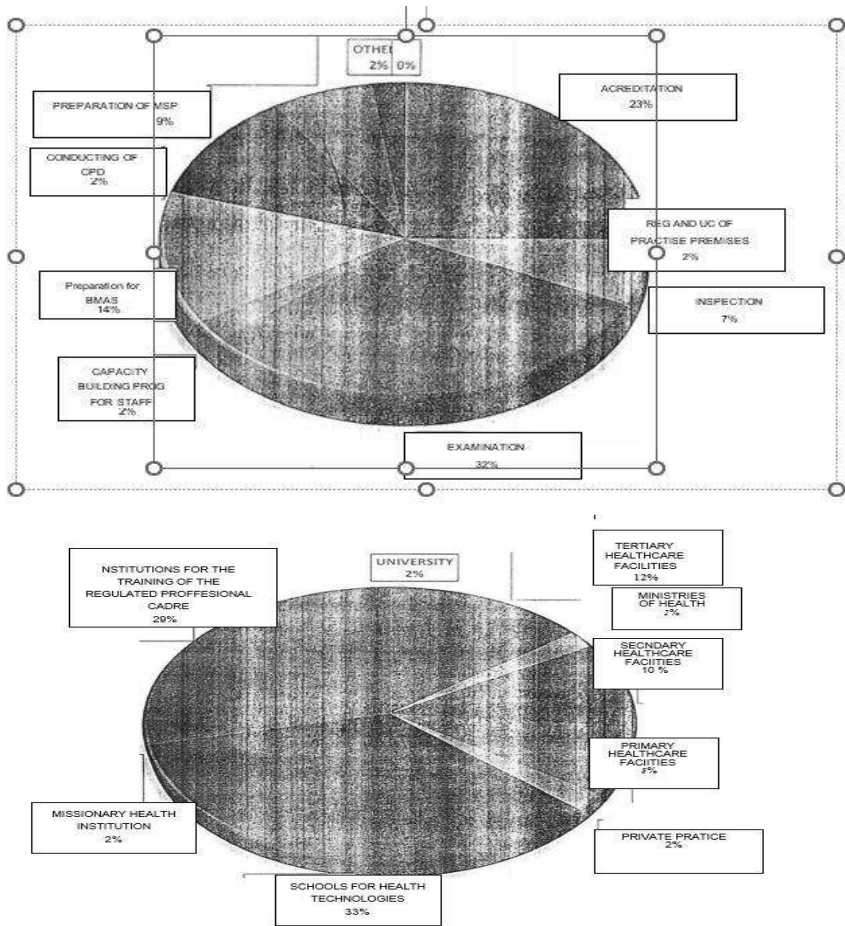
The Board was established in 1989 and the establishment was applicable to

DECREE 39,1989 (CAP 166 OF 1990 LFN) with its head office located at 16, Kinshasa Street, Wuse, Zone 6, Abuja. The Board has total staff strength of 137 staff.

(a) Percentage score of the Board's Responsibility to the listed Functions Interpretation.

According to the respondent rating of the Board's responsibility to the listed functions, it is responsible for:

3.1 (b) Places Where the Board's Regulatory Activities often takes Place.



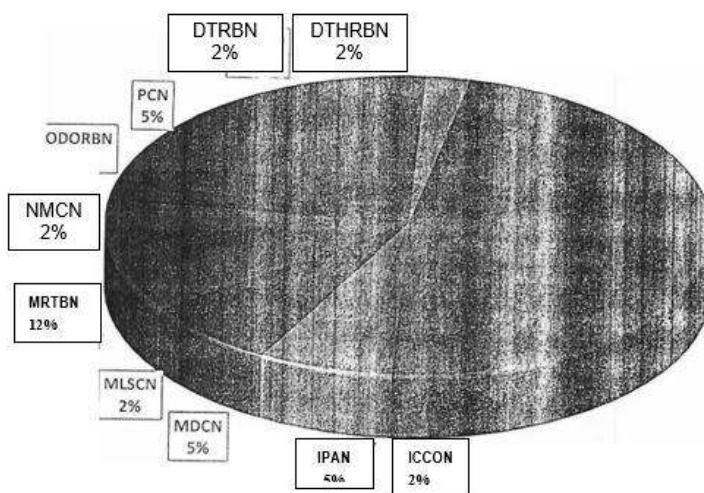
Interpretation

According to the respondent rating, the Board's regulatory functions take place in the following places (with percentage frequency):

- i. 33% on Schools for Health Technology
- ii. 29% on institution for the training of the regulated professional cadre Hence, the major places where the Boards regulatory functions take place are at Schools of Health Technologies and institutions for training the regulated professional cadre and healthcare facilities, among others.
- iii. 29% Poor funding
- iv. 24% Interference and the duplication of regulatory roles

Hence, we can conclude that the Council's major challenges are poor funding, interference and duplication of regulatory roles and lack of personnel respectively, among others.

3.1 (C) Sister Regulatory Bodies that the Board often Collaborates with.



Interpretation

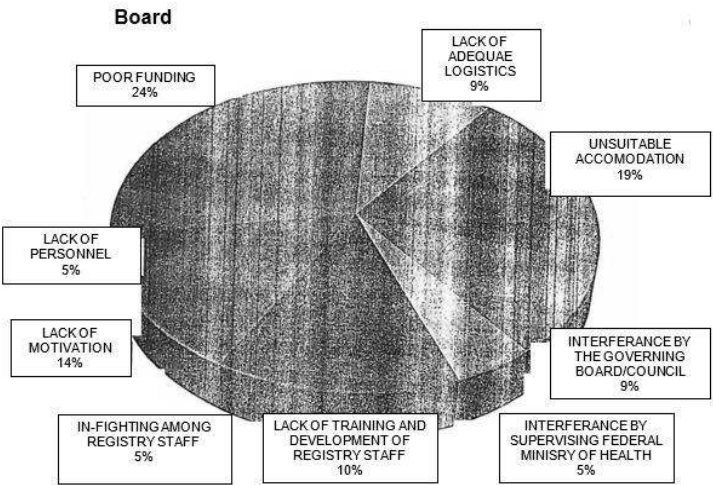
According to the respondents, the sisters' bodies the Board often collaborates with (together with their frequency of collaboration) are:

- i. 12% with the Medical Rehabilitation Therapist Board of Nigeria (MRTBN)
- ii. 10% with the Community Health Practitioners Registration Board of Nigeria (CHPRN).

As such, we conclude that the Board collaborates more with the Medical Rehabilitation Therapist Board of Nigeria (MRTBN) and the Community Health Practitioners' Registration Board of Nigeria (CHPRN) respectively, among others.

The Board/Council/Institute enjoys a cordial relationship with other professional associations.

3.1 (d) Percentage Rating of the Challenges Faced by the Board Interpretation



Challenges faced by the Board (with their percentage rating) are:

- i. 24% Poor funding

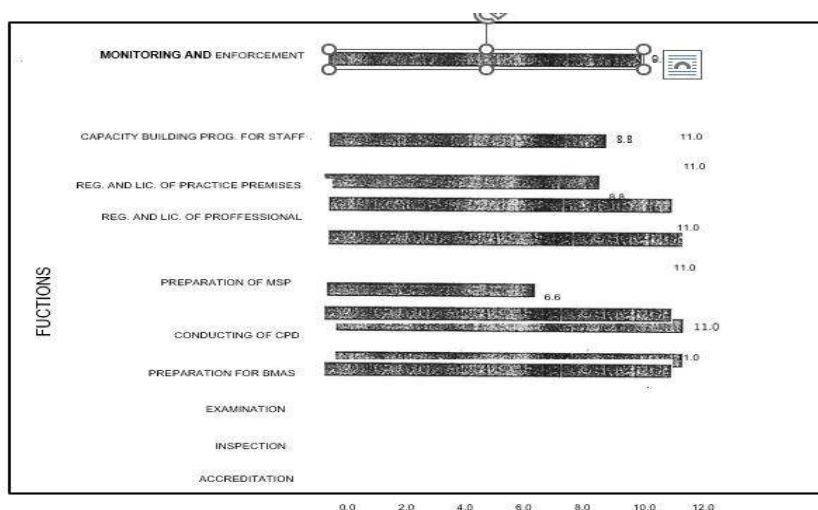
- ii. 19% Unsuitable office accommodation
- iii. 10% Lack of training and development of registry staff

Hence, we can conclude that the Board's major challenges are poor funding, unsuitable office accommodation and lack of staff motivation respectively, among others

3.2 THE PHARMACIST'S COUNCIL OF NIGERIA (PCN)

The above Council was established in 1936 and the establishment was applicable to Decree 91 of 1992 with a head office located at Plot 7/9 Idu Industrial Layout, Idu, Abuja. The Council has total staff strength of 282.

3.2a. Percentage Score of the Council's Responsibility to the Listed Functions.



Interpretation

According to the respondent rating the Council is responsible for

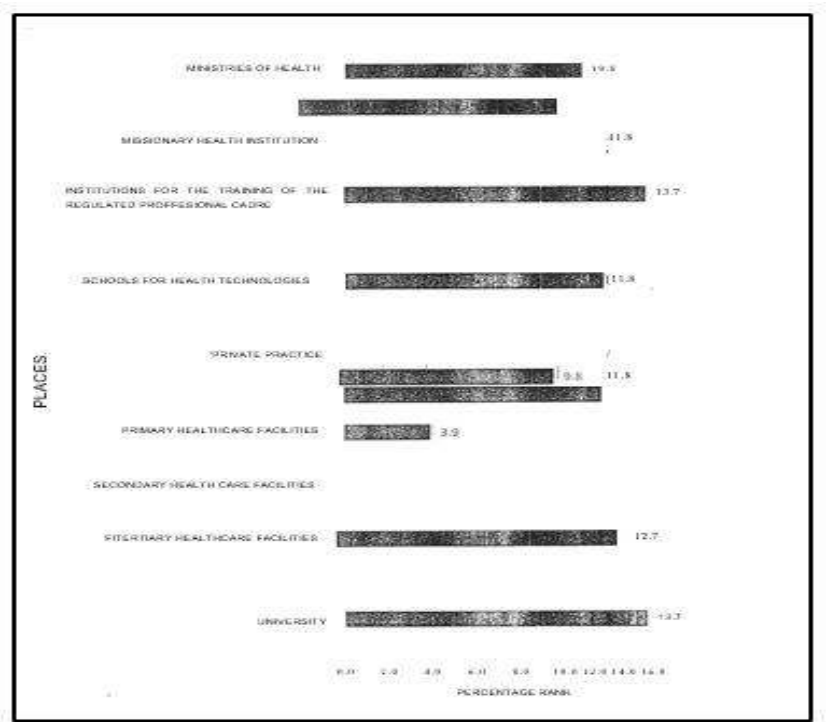
- i. 11 % accreditation.
- ii. 11 % inspection.
- iii. 11 % preparation of benchmark minimum academic

standards (BMAS)

- iv. 11% conduction of continuing professional development (CPD) programmes
- v. 11 % preparation of minimum standards of practice
- vi. 11 % registration and licensing professionals.
- vii. 11 % registration and licensing practice premises.

Hence, the Council performs majorly the functions of accreditation, inspection, preparing benchmark minimum academic standards (BMAS), conducting continuing professional development, registration and licensing professionals and practice premises, among others.

3.2b. Places where the Council’s Regulatory Activities often take place.



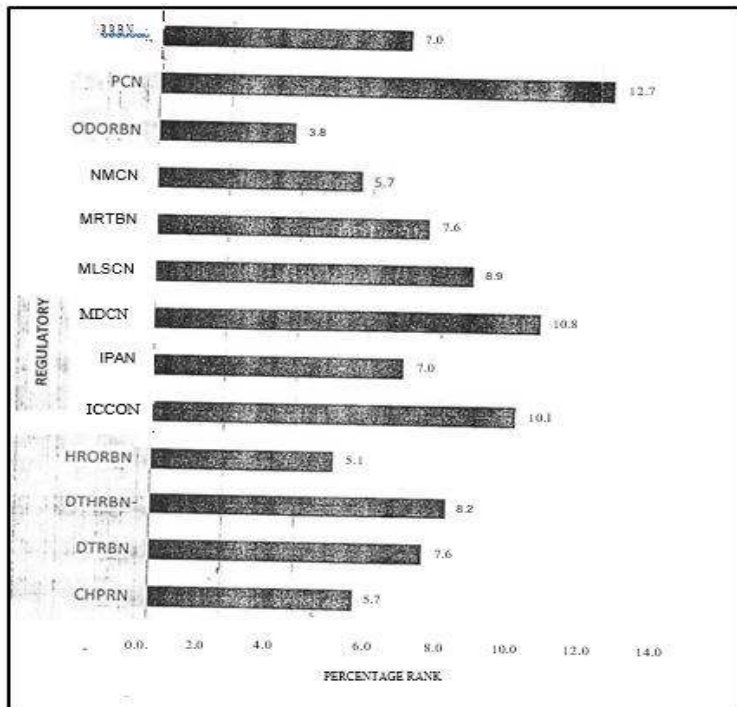
Interpretation

According to the respondent rating, the Council's regulatory functions take place in the following places (with percentage frequency):

- i. 13.7% on university grounds
- ii. 13.7% on institution for the training of the regulated professional cadre

Hence, the Council uses all the above listed places majority for their regulatory functions, except for primary health facility with a 3.9% usage.

3.2c. Sister regulatory Bodies that the Council Collaborates with



Interpretation

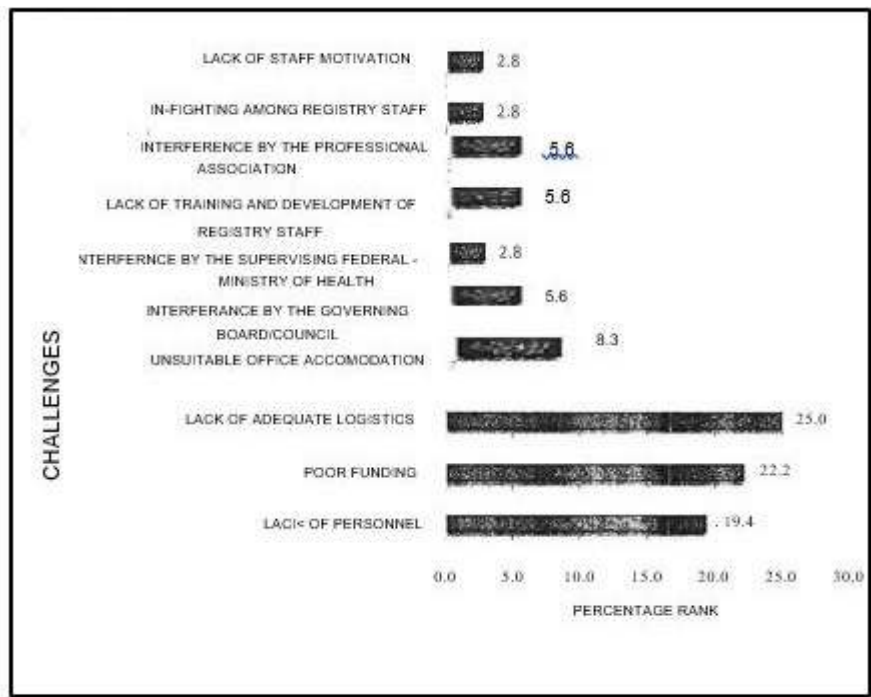
According to the respondents, the sisters' bodies the Council often collaborates with (together with their frequency of collaboration) are:

- i. 9.9% with the Institute of Chartered Chemists of Nigeria (ICCON)
- ii. 10.8% with the Medical and Dental Council of Nigeria (MDCN)
- iii. 8.9% with the Medical Laboratory Science Council of Nigeria

As such, we conclude that the Council collaborates with all the listed bodies effectively and efficiently.

8.0 The Council enjoys a very cordial relationship with other professional associations

3.2d. Percentage Rating of the Challenges Faced by the Council.



Interpretation

The challenges faced by the Council (with their percentage ratings) are:

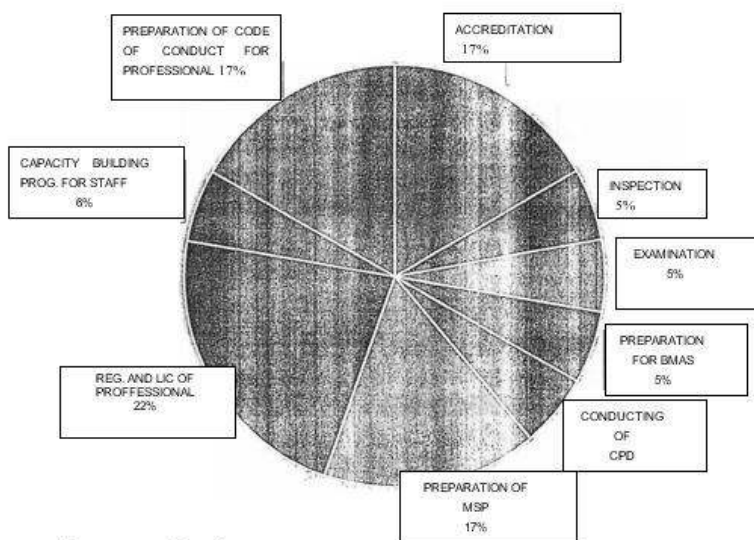
- i. 19.4% Lack of personnel
- ii. 22% Poor funding
- iii. 25% Lack of adequate logistics

Hence, we can conclude that the Council's major challenges are lack of adequate logistics, poor funding and lack of personnel, respectively.

3.3 THE MEDICAL AND DENTAL COUNCIL OF NIGERIA (MDCN)

The above council was established on 18th December, 1963 and the establishment was applicable to CAP MS LFN 2010. The council has total staff strength of 121 staff.

3.3a. Percentage Rank of the Council's Responsibility to the outlined functions.

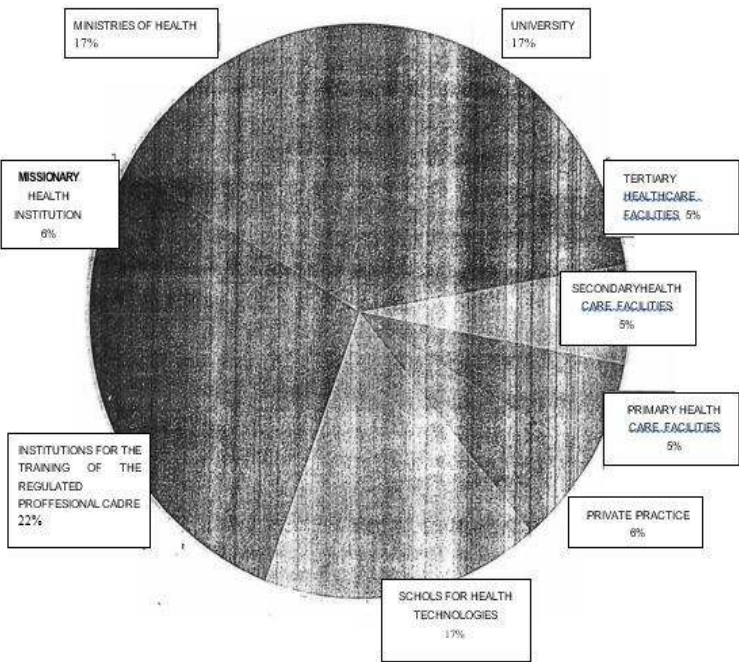


Interpretation.'

According to the respondent rating, the Council is responsible for

- i. 17% accreditation.
- ii. 22% registration and licensing of professionals.
- iii. 17% preparation of code for professions.
- iv. Hence, the Council performs majorly the function of the registration and licensing of professionals, disciplining members and the preparation of the code of conduct for the professionals, among others.

3.3b. Percentage Rank of the Places where the Council’s Regulatory Activities takes place



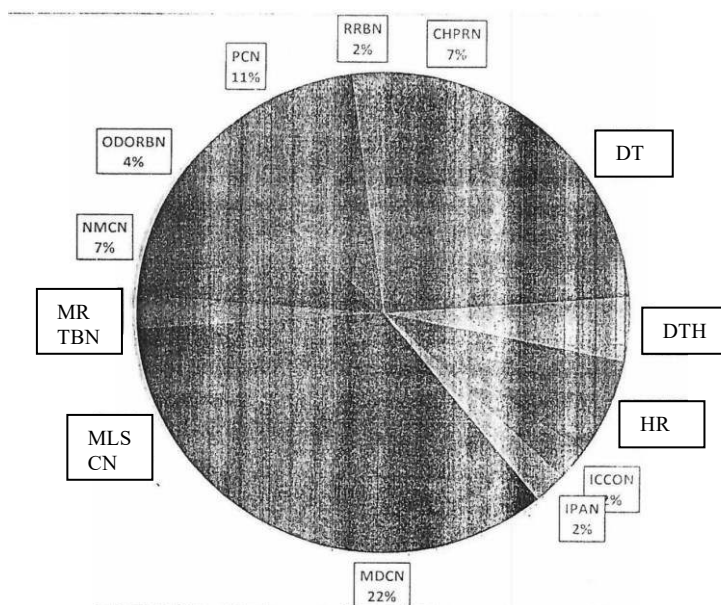
Interpretation

According to the respondent rating, the Council's regulatory functions take place in the following place (with percentage frequency):

- i. 17% on university grounds
- ii. 25% on tertiary health care facilities
- iii. 17% schools of Health Technologies (SHTs)

Hence, the Council uses universities and the institutions for the training of the regulated professional cadre majorly for their regulatory functions.

3.3 (c) MDCN do not collaborate with any sister regulatory body. Hence, they operate in isolation.



Interpretation

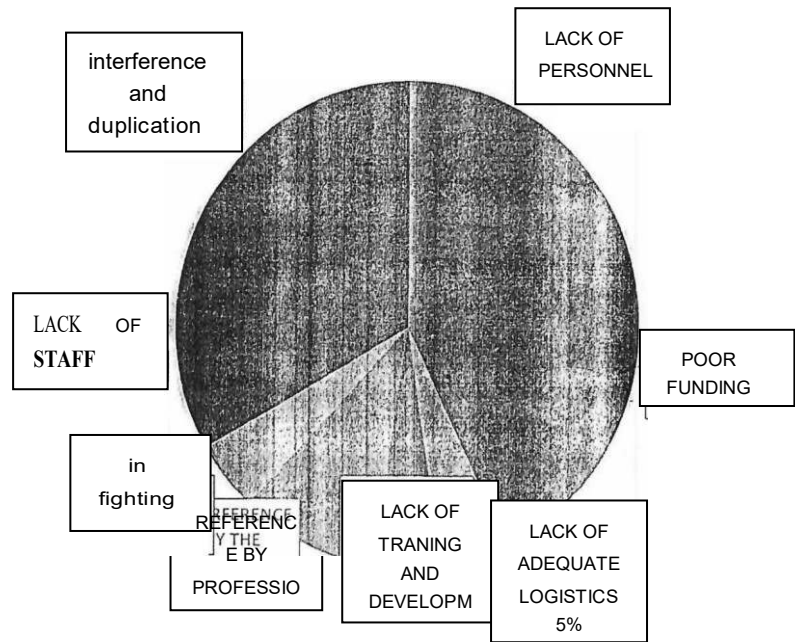
According to the respondent, the council often collaborates more with itself, Dental Therapist Registration Board of Nigeria (DTRBN) and Medical Laboratory Science Council of Nigeria (MLSCN) respectively among others.

Interpretation:

- (i) 17% DTRBN;
- (ii) 13% MLSCN;

(iii) 11 % PCN.

3.3 (d) Percentage Rating of the Following Challenges Faced by the Council



Interpretation

The challenges faced by the Council (with their percentage ratings) are:

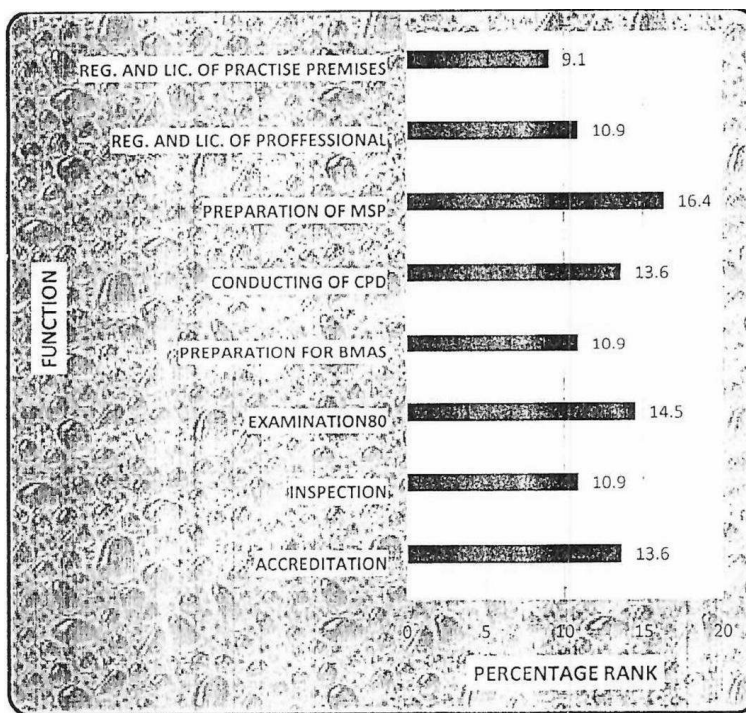
- i. 14% Lack of personnel
- ii. 29% Poor funding
- iii. 24% Interference and the duplication of regulatory roles

Hence, we can conclude that the Council's major challenges are poor funding, interference and duplication of regulatory roles and lack of personnel respectively, among others.

4.0 THE DENTAL TECHNOLOGISTS' REGISTRATION BOARD OF NIGERIA (DTRBN)

The above Board was established in 1987. The establishment was applicable to Decree 43 of 1987 now Cap d6 of 2004 Abuja. The Board has total staff strength of 85.

4.4(a) Percentage Rank of the Board's Responsibility to their Functions



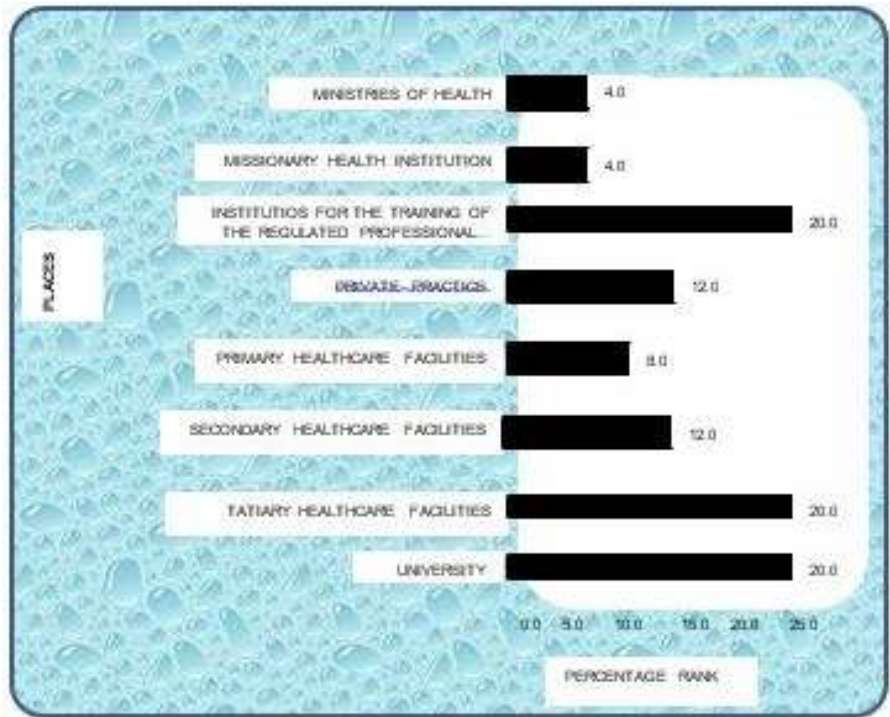
Interpretation

According to the respondent rating, the Board is responsible for

- i. 13.6% accreditation.
- ii. 14.6% examination.
- iii. 16.4% preparation of benchmark minimum academic standards (BMAS)
- iv. 13.6% conduction of continuing professional development (CPD) programmes

Hence, the Board performs majorly the functions of the registration and licensing professionals, the examination and accreditation, conduct of continuing professional development (CPD) and the preparation of minimum standards of practice respectively, among others.

4.4(b) Percentage Rank of Places where the Board's Regulatory Activities Takes Place



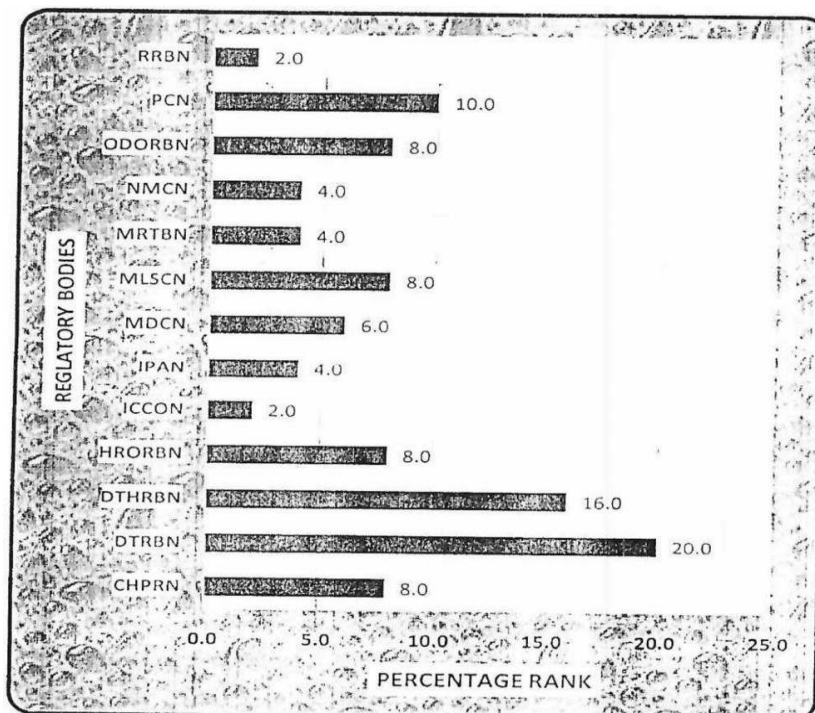
Interpretation

According to the respondent rating, the Board is responsible for
13.6% accreditation.
14.6% examination.
16.4% preparation of benchmark minimum academic standards (BMAS)

1v. 13.6% conduction of continuing professional development (CPD) programmes

Hence, the Board performs majorly the functions of the registration and licensing of professionals, the examination, and accreditation, conduct of continuing professional development (CPD) and the preparation of minimum standards of practice respectively, among others.

4.4(b) Percentage Rank of Places where the Board's Regulatory Activities Takes Place

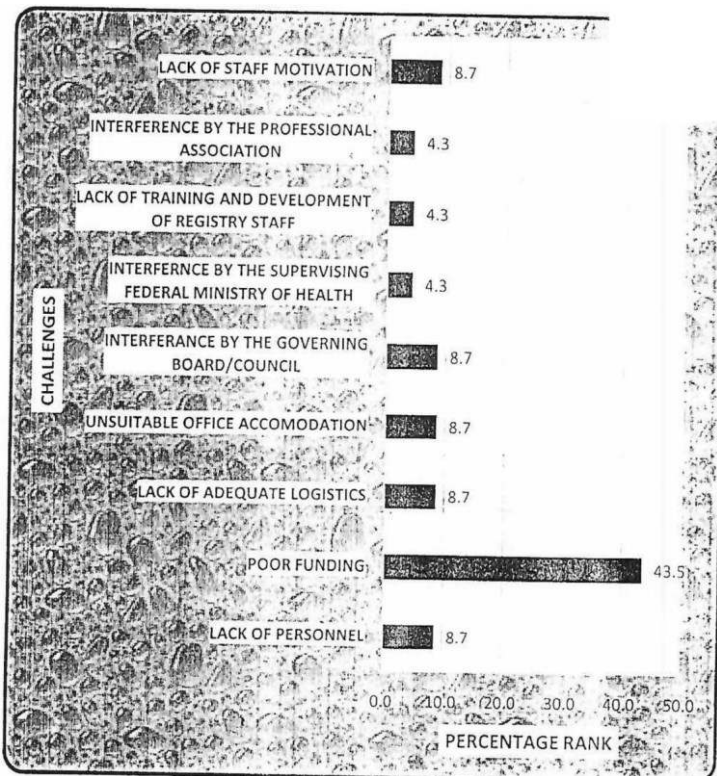


According to the respondent rating, the Board regulatory functions take place in the following places (with percentages frequency)

- 20% on University grounds;
- 20% on tertiary health care facilities
- 12% on secondary health care facilities

Hence, the Board uses tertiary institutions, University and the institute for the training of the regulated professional cadre majorly for their regulatory functions.

4.4{d) Percentage Rating of the Following Challenges Faced by the Board



Interpretation

The challenges faced by the Board (with their percentage ratings) are:

- (i) 8.7% Lack of personnel
- (ii) 43.5% Poor funding
- (iii) 8.7% Lack of adequate logistics

- (iv) 8.7% Unsuitable office accommodation
- (v) 8.7% Interference by the Governing Board
- (vi) 8.7% Lack of staff motivation.

Hence, we can conclude that the Board's major challenges are poor funding, lack of adequate logistics, unsuitable office accommodation and lack of staff motivation respectively, among others.

5.0 Conclusion, Recommendations and Implementation strategies

5.1 Conclusion:

(A) HRORBN:

The responsibility of the Board is high on Examinations (32%) and Accreditation (23%) while it is low on conducting CPD (2%). Indeed, in Registration and the licensing of practices and capacity building programmes for staff, it is 2% for each. In places where the Board's Regulatory function takes places, SHT and Institution-for the training of the regulated professional cadre are high with 33% and 29%, respectively. It is low in university (2%) and Missionary Health Institution (2%) while MoHs and Private practice are equally low with 2% each. In dealing with sister regulatory bodies, HRORBN has more dealings with MRTBN (12%) and CHPRN (10%) but low with DTRBN (2%) and ICCON (2%). It has no relationship with RRBN (0%). On the challenges faced by HRORBN, poor funding is high (24%) as with unsuitable office accommodation (19%). Lack of personal is at 5%. Interferences are very low with FMOH at (5%).

(B) PCN

The responsibility of PCN is high on Accreditation (11%); Inspection (11%); Preparation of BMAS (11%); Conducting CPD (11%), Registration and Licensing Professionals (11%) and Registration and Licensing practice premises (11 %). It is only low on examination (6.6%). Places where the Council's regulatory functions often take

place are high at university (13.7%); Tertiary healthcare facilities (12.7%); Secondary healthcare facilities (9.8%); Private Practice (11.8%); SHTS (11.8%); Institutions for the regulation of the regulated professional Cadre (13.7%); Missionary Health Institution (11.8%); and Ministries of Health (10.8%). It is only low at PHCs (3.9%). In collaborating with sister regulatory bodies, PCN scored 100% high for most of the other twelve (12) regulatory bodies, possibly due to presence and necessity of drug products in each of the agencies for them to function effectively.

On the challenges, PCN indicated lack of adequate logistics (25%), poor funding (22.2%) and lack of personnel (19.4%), among the highs. Lack of staff motivation (2.8%) is the only parameter with a single digit as a challenge. In-fighting among registry staff is 0%.

(C) MDCN

The responsibilities of the MDCN are high on registration and licensing professionals (22%), preparations of MSP (17%), Preparation of a code of conduct for the profession (17%) and Accreditation (17%). It is low on each of Inspection (5%), Examination (5%), preparation of BMAS (5%), conducting CPD (6%) and capacity building programme for staff (6%). In places where the MDCN's regulatory work takes places, they are high in university (17%) and Institutions facilities for the training of the regulated professional cadre (22%). It is low on Missionary Health Institution (6%) and Primary Health Care Facilities (5%). On collaboration with other health regulatory agencies, MDCN has more to do with DTRB (17%). MLSCN (13%) and PCN (11 %). It is low on collaboration with MRTBN, IPAN, ICCON, RRBN at 2% in each case.

On the challenges facing the MDCN, it indicated poor funding (29%) and Interference/duplication of regulatory role (24%) among the highs while in fighting among registry staff (5%) is low, as well as lack of logistics (5%). Indeed, no challenges are faced in respect of

unsuitable office accommodation (0%), interference by the Governing Council (0%) and the supervising FMOH (0%).

(D) DTRBN

The responsibility of the Board is high on registration and licensing professionals (10.9%); Examination (14.6%) and Accreditation (13.6%), conducting CPD (13.6%) and the preparation of MSP (16.4%). It is low on capacity building programme for staff (5%) and none in the preparation of a code of conduct for professionals (0%) and discipline for members (0%). In places where the Board's regulatory function takes place, we have University (20%); tertiary healthcare facilities (20%) and institutions for the training of the regulated professional cadre (20%) among the highs. Missionary health institution (4%) and MOHS (4%) are low, with no presence at SHT (0%).

In dealing with sister regulatory bodies, DTRBN claimed, to have high relationship with DTHRBN (16.0%), MLSCN (8.0%) and PCN (10%). It is low with nurses at 4. % and similar percentage with MRTBN, IPAN and very low with RRBN at 2%. On the challenges faced by DTRBN, it indicates poor funding (43.5%) as the most critical. Interference by Governing Council (8. 7%), lack of motivation (8. 7%) and lack of personnel is 8. 7%. with professional association it rated the challenge at 4.3%.

5.2 Recommendations

(a) Recommendation Number 1: HRORBN needs to address the following:

- (i) Regulating the activities of the professionals and practice areas;
- (ii) Conducting CPD programmes;
- (iii) Inspection of premises;
- (iv) Preparations of BMAS;
- (V) Registration and licensing Professionals;

- (vi) Should be given mandate to close down both government and private facilities that do not comply with the minimum standards.

(b) Recommendation Number 2:

PCN Needs to address the following:

Setting and enforcing standards that would promote safe and effective healthcare delivery in a harmonious atmosphere that recognizes and respects the boundaries of all the players in the team; Provision of adequate pharmaceutical inspectors and inspection vehicles in all its offices.

(c) Recommendation Number 3:

MDCN needs to address the following:

- (i) Maintain and improve standards of practice among professionals;
- (ii) Eliminate quackery in the professions;
- (iii) Improve on the quality of training of professionals;
- (iv) Improve on the regulation / registration of professionals;
- (v) Improve on the quality of patient care.

(d) Recommendation Number 4:

DTRBN needs to address the following

- (i) Inspection;
- (ii) Preparation of BMAS;
- (iii) Registration and licensing of practice premises;
- (v) Capacity building programmes for staff.

5.3 Implementation Strategies

The members of CRHP must continue to collaborate with each other to peer review their regulatory and control functions, especially in critical areas of inspections, accreditations, registration & licensing, CPD programmes and in setting minimum standards of practice. There are no two ways about these in regulating and controlling their various profession's education and practices. Similarly, all the regulatory bodies, singly and as CRHP members should establish linkages with international bodies like the World Health Professional Alliance (WHPA) and partner at the global level towards imbibing the best global practices in the dispensation of regulatory and control services in Nigeria in a most corrective and ethical manner as public institutions.t

In the final analysis, it is the resolution of the WHPA that should prevail and which emphasized that facilitating collaboration between key health professionals and major international stakeholders such as Governments, Policy makers and the World Health Organization (WHO). That by working in collaboration instead of along parallel tracks, patients and the Healthcare systems of our world would benefit tremendously.

Thank you.

Dr. Ahmed T. Mora *PhD, mni*

Las Vegas, Nevada,

June 23-26, 2016

REFERENCES

- CRHP (2008); "The Role of Health regulatory Bodies in Advancing Healthcare Delivery Services in Nigeria" - Proceedings of a One-Day Advocacy Workshop held in Abuja, 2008, 9lp.
- Lambo, E. Prof (2004); Address by Prof. Eyitayo Lamibo, on the Inauguration of the Committee of Registrars of the Health Profession (CRHP) on Thursday, August 19, 2004 in the SGF's Conference Room, Federal Government Secretariat, Abuja.
- Ibrahim A.A. (2016: Address by Dr. A.A Ibrahim, the Registrar, MDCN
- while deliver the Keynote on "Minimum Standards Required in tertiary Healthcare Facilities for effective Service Delivery" at the Banquet Hall of Hotel Seventeen Lafiya Road, Kaduna on Wednesday, 13" March, 2016
- International Journal of Healthcare Management, (Vol. 1 No. 1); December, 2015 P 36- 57.

**Text of Remarks on the Opening Ceremony of a Sensitisation
Workshop to Kick-Start the Implementation of the Pharmacist
Consultant Cadre within the Healthcare System in Nigeria.**

**Presented by Prof. Ahmed Tijjani Mora, *PhD, mni*, at the
Banquet Hall, NAF Conference Centre and Suites, Abuja, on
May 18, 2021.**

We are privileged to have in our midst for this Opening Ceremony:

- The Honourable Minister of Health, **Dr. Osagie Ehanire**;
- The Head of the Civil Service of the Federation, **Dr. Folasade Yemi-Esan**;
- Leadership of the Committees on Health and Establishment from the Senate and the House of Representatives.

Your presence today underscores the significance of this healthcare and establishment event, hosted by the **Pharmacists Council of Nigeria (PCN)**. The *LFN Act No. 17 of 2004*, which establishes the PCN with the mandate of regulating and controlling pharmacy education, training and practice, is instrumental to this historic gathering of leaders from both the Executive and Legislative arms of government under one roof.

Historical Context

In September 2006, the PCN received the “*Report of the Pharmacists Council of Nigeria (PCN) Visitation Panel to the West African Postgraduate College of Pharmacists (WAPCP)*.” Exactly fourteen (14) years later, on **11th September, 2020** the Head of the Civil Service of the Federation released a circular titled: “*Approved Scheme of Service for Pharmacist Consultant Cadre in the Federal Civil/Public Service*.” Between 2006 and 2020, pharmacists in Nigeria and four other Anglophone West African countries underwent active training for the Fellowship programme of the WAPCP, which was established **45 years ago in Monrovia, Liberia**, initially as the **West African Pharmaceutical Federation (WAPF)**.

The National Council on Establishment (NCE), the PCN, the WAPCP and other stakeholders actively engaged to ensure recognition of the College's Fellows within the Pharmacist Consultant cadre in the Federal Civil/Public Service. Notably, the now-defunct **Pharmacists Board of Nigeria (PBN)** initiated the route to the Pharmacist Consultant cadre through visits by Nigerian Deans of Pharmacy to selected Schools of Pharmacy in the **UK and USA** in 1992. This initiative aimed to study Clinical Pharmacy curricula in advanced countries and was sponsored by the Federal Ministry of Health.

The focus areas were:

- Prescribing decisions;
- Monitoring and modifying drug therapy;
- Counselling patients on the ward prior to discharge; and
- Benefits to patients in the community.

Following this initiative, the **National Universities Commission (NUC)** directed the creation of **Departments of Clinical Pharmacy and Pharmacy Practice/Management** in all Nigerian Faculties of Pharmacy. The WAPCP Fellowship programme aligns directly with Clinical Pharmacy topics at the undergraduate Bachelor of Pharmacy level in Nigerian universities.

The Faculties of WAPCP

1. *Faculty of Drugs Production and Quality Assurance*
2. *Faculty of Social and Administrative Pharmacy*
3. *Faculty of Clinical Pharmacy*
4. *Faculty of Community Pharmacy*
5. *Faculty of Public Health Pharmacy*

The Purpose of the Workshop

In recognition of the Pharmacist Consultant cadre by the Federal Government, the PCN—mandated by law to regulate pharmacy education, training, and practice—is organizing this workshop to

kick-start a series of workshops in Lagos, Enugu and Kaduna. These workshops will induct the newly approved Pharmacist Consultants as they embark on this new phase of professional practice in:

- Federal, State, and Local Government health services; and
- Organised private sector healthcare facilities, for the benefit of patients and the overall healthcare system.

Keynote Address

At this workshop, the **Keynote Address** will be delivered by **Professor N. N. Wannang, *FPCPharm*** of the Faculty of Pharmaceutical Sciences, University of Jos and Immediate Past Secretary General of the WAPCP. The Pharmacist Consultant must be properly inducted into this officially designated role, emphasizing collaboration with other healthcare professionals to provide **advanced and integrated healthcare services**, with specific focus on **pharmaceutical care** in healthcare facilities across the country.

Collaboration with Other Healthcare Professions

Representatives from the **Pharmaceutical Society of Nigeria (PSN)**, **Nigerian Medical Association (NMA)**, **Nigerian Association of Nurses and Midwives (NANM)**, **Medical Laboratory Scientists Council of Nigeria (MLSCN)**, and other healthcare professions have been invited. Pharmacist Consultants are expected to work **side by side with Medical Doctors, Nurses, Medical Laboratory Scientists** and others in public healthcare facilities, ensuring holistic patient care.

Message to Fellows

To my colleagues—Fellows of the College and now Pharmacist Consultants—this is a **defining moment** for the pharmacy profession. This workshop comes at a critical time, as Nigeria and the world continue to battle the *COVID-19* pandemic, which has persisted for the past fifteen (15) months. We anticipate a **highly educational, informative and professionally enriching workshop**.

Thank you.

Prof. Ahmed Tijjani Mora, *PhD, FPSN, FPCPharm, mni*

Chairman, Governing Council, Pharmacists Council of Nigeria (PCN)

**The Implementation of the Regulatory Functions of the
Pharmacists Council of Nigeria (PCN) in Respect of Education
and Continuing Professional Development (CPD) Programmes.
Presented by Prof. Ahmed Tijjani Mora, PhD, mni, Chairman,
Pharmacists Council of Nigeria (PCN), at the 2021 Scientific
Conference and Exposition of NAPPSA, held in Baltimore,
Maryland, USA, from September 23–26, 2021.**

PREAMBLE

The Pharmacists Council of Nigeria (PCN) is a Federal Government agency established by *Act CAP P17, LFN 2004*. It is mandated to regulate all aspects of pharmacy education, training, and practice in Nigeria. The pharmacy profession in Nigeria comprises two cadres:

- **Pharmacists**
- **Pharmacy Technicians**

Pharmacists are trained in approved universities for a minimum of five years (B.Pharm) or six years (PharmD). Pharmacy Technicians are trained in colleges or schools of health technology for a minimum of three years to become certified.

The key regulatory functions of the PCN in education and training include:

- Determining the standard of knowledge and skills required to become a pharmacist or pharmacy technician in Nigeria.
- Approving institutions in Nigeria permitted to train pharmacists and pharmacy technicians.
- Recognizing qualifications granted by foreign institutions for the purpose of practice in Nigeria.

This paper outlines the PCN's responsibilities in the following areas:

1. Pharmacy Education and Continuing Professional Development (CPD)
2. Pharmacy Technician (PT) Education

3. Training of Patent and Proprietary Medicine Vendors License (PPMVL) Holders

1.0 PCN RESPONSIBILITIES: PHARMACY EDUCATION & CONTINUING PROFESSIONAL DEVELOPMENT (CPD) PROGRAMME

1.1 Benchmark Minimum Academic Standard (BMAS): The PCN is responsible for establishing the Minimum Academic Requirements (MAR) for the Bachelor of Pharmacy (B.Pharm) and Doctor of Pharmacy (PharmD) degree programs.

- The PCN's Deans Committee prepares the MAR, which is then reviewed by the Education Committee and recommended to the Governing Council for approval.
- Once approved, the MAR is forwarded to the National Universities Commission (NUC) for incorporation into the final BMAS document.
- The approved BMAS is disseminated to all Faculties of Pharmacy/Pharmaceutical Sciences for implementation.
- The BMAS is subject to a mandatory review every five years.

1.2 Accreditation of Pharmacy Schools: The PCN develops and maintains scoring formats to ensure objective and uniform assessment of facilities and human resources during accreditation visits.

- This format is designed to minimize subjective assessment and standardize evaluations across all institutions.
- The scoring format collects data on:
 - **Human Resources:** Academic staff (both professional and non-professional) and non-academic staff (administrative and technical). Professional staff are classified as senior academics (Lecturer I and above).
 - **Facilities:** Buildings, including offices, classrooms, and laboratories.

- **Teaching Support Facilities:** Library, herbarium, botanical garden and animal house.
- The Education Committee constitutes accreditation teams and recommends them to the Governing Council for approval.
- The report from each accreditation visit is considered by the Education Committee before a final recommendation is made to the Governing Council.

1.3 Other Key Programmes

The PCN also oversees:

- **Internship Training**
- **Pre-registration Examination for Interns**
- **Foreign Pharmacy Graduate Orientation Program (FPGOP)**
- **Mandatory Continuing Professional Development (MCPD) Programme for Pharmacists**

2.0 PCN RESPONSIBILITIES: PHARMACY TECHNICIAN (PT) EDUCATION

The PCN regulates the training and certification of Pharmacy Technicians through:

- Accreditation of schools and colleges of health technology offering PT programs.
- Development of the course content for PT education.
- Administration of the National Pre-Certification Examination for PTs.
- Development of CPD program content for licensed PTs.

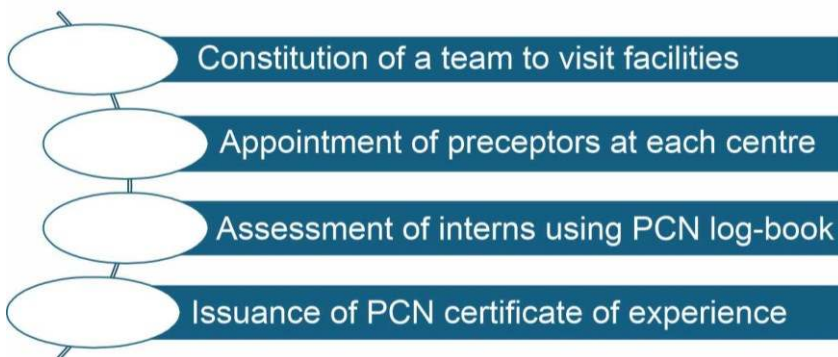
3.0 PCN RESPONSIBILITIES: TRAINING OF PATENT AND PROPRIETARY MEDICINE VENDORS (PPMVL)

The PCN ensures the competency of PPMVL holders through:

- Developing the course content for the orientation program for new licensees.

- Developing the course content for mandatory refresher training for existing PPMVL holders.

Regulation of Internship training program



1.4 Pre-Registration Examination (PEP)

The Pre-registration Examination (PEP) is a quality assurance mechanism introduced by the PCN to ensure all intern pharmacists meet a consistent national standard before full registration, aligning with global best practices.

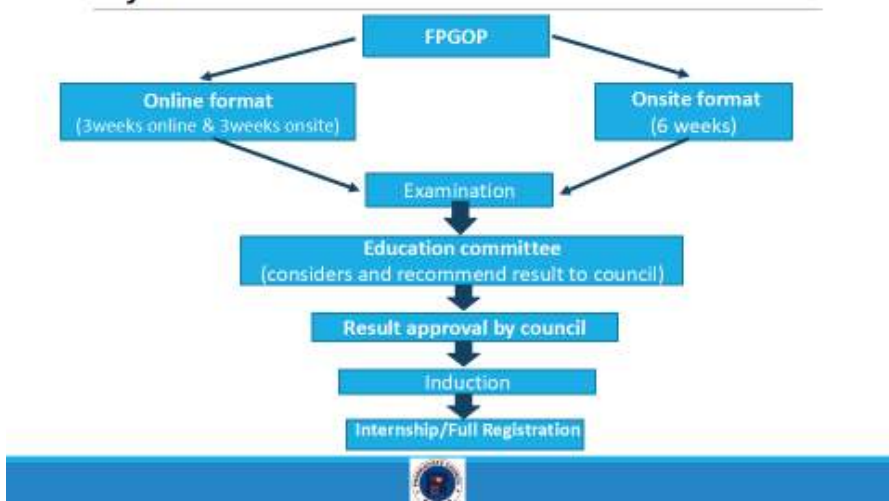
- The PEP was approved by the Governing Council in 2015, with the first examination conducted in October 2017.
- A dedicated committee appointed by the Council sets the examination questions based on the approved curriculum.
- The examination is administered as a computer-based test at designated centers across Nigeria.
- Results are collated and presented to the Governing Council via the Education Committee for final approval.
- Successful candidates are eligible to be registered as pharmacists to practice in Nigeria.

1.5 Foreign Pharmacy Graduate Orientation Program (FPGOP)

The FPGOP is designed for Nigerian pharmacists trained in foreign universities who wish to be registered to practice in Nigeria.

- The PCN is responsible for developing the course content and administering this program.
- The program assesses the competencies of foreign-trained graduates and orients them to the Nigerian healthcare context and legal framework.

Steps for registering Foreign trained pharmacists by PCN



1.6 Mandatory Continuing Professional Development (MCPD) for Pharmacists

The PCN holds full responsibility for administering the Mandatory Continuing Professional Development (MCPD) program to ensure pharmacists maintain and enhance their competencies throughout their careers. Key activities include:

- Developing and periodically reviewing the MCPD modular system.
- Designing MCPD courses, curricula, and modules that address contemporary advancements in pharmacy practice.
- Accrediting MCPD provider centers nationwide.

- Developing course content and allocating credit points for various activities, including face-to-face and online conferences, workshops and seminars.
- Implementing and managing an online MCPD platform for streamlined record-keeping and accessibility.
- Allocating credit points to pharmacists who successfully complete MCPD programs.

2.0 PCN RESPONSIBILITIES: PHARMACY TECHNICIAN (PT) EDUCATION & DEVELOPMENT

2.1 PT Training Curriculum: The Pharmacy Technician (PT) is a middle-level cadre within the pharmacy profession.

- The PCN's Governing Council approves the 3-year training curriculum delivered at accredited Colleges/Schools of Health Technology.
- The PCN Training Sub-Committee is responsible for curriculum development, which covers basic sciences, principles of PT practice, basic dispensing, pharmacology, primary healthcare, anatomy and physiology.
- The program includes a mandatory minimum of six months of experiential learning in hospitals and community pharmacies.

2.2 National Pre-Certification Examination: The PCN administers the National Pre-Certification Examination for Pharmacy Technicians.

- The examination is held annually. Eligibility requires graduation from a PCN - accredited College of Health Technology.
- An Ad-hoc Committee, constituted by the PCN Training Committee from Heads of Department of approved colleges, sets the examination questions.

- The PCN administers the exam and the Ad-hoc Committee moderates the results.
- Results are recommended to the Governing Council via the Education Committee for approval before release to candidates through their colleges.
- Successful candidates are inducted as Certified Pharmacy Technicians in Nigeria.
- Foreign-trained Pharmacy Technicians must undergo certificate evaluation before being eligible to sit for the national examination.

2.3 Continuing Professional Development (CPD) Programme for Pharmacy Technicians

- The PCN develops the course content for CPD programme for licensed Pharmacy Technicians.
- The Council is currently reviewing the MCPD guidelines for PTs, which includes developing new manuals, facilitator guides and presentation slides.

3.0 PCN RESPONSIBILITIES: TRAINING OF PATENT AND PROPRIETARY MEDICINE VENDORS (PPMVL)

The PCN, as the licensing authority, is responsible for educating PPMVL holders to ensure the safe and legal sale of patent medicines. This involves:

- **Orientation for New Licensees:** Developing course content for mandatory orientation programs, covering:
 - Legal and regulatory frameworks governing the sale of patent medicines.
 - Operational responsibilities of vendors.

- Elementary aspects of product procurement and handling.
 - The approved patent medicine list.
- **Continuing Education:** Organizing refresher training and Continuing Education Programs (CEP) for existing licensees.
- **Resource Development:** Preparing and updating manuals for both orientation and continuing education programs.

**Policy Reforms and Regulatory Challenges in Nigeria's
Pharmaceutical Sector. Presented by Prof. Ahmed Tijjani Mora,
PhD, mni, as the Keynote Speaker at the Pharmacy Week
(Academic Pharmacy Summit) Organised by the
Pharmaceutical Association of Nigerian Students (PANS),
Sa'adu Zungur University, Gadau, Bauchi State, on Friday,
July 25, 2025.**

It gives me great pleasure to be invited by the Comrade Abubakar Umar Abdullahi-led Executive Committee of the Pharmaceutical Association of Nigerian Students (PANS), Sa'adu Zungur Chapter, Sa'adu Zungur University, Gadau, Bauchi State, to deliver the keynote address on the occasion of your Pharmacy Week (Academic Pharmacy Summit). Let me begin by congratulating the entire membership of PANS, Sa'adu Zungur Chapter, on this important occasion. The Faculty of Pharmaceutical Sciences at Sa'adu Zungur University, Gadau, is relatively young, yet I am pleased to note that its students have already organized themselves into a vibrant professional association. The topic assigned to me — **“Policy Reforms and Regulatory Challenges in Nigeria's Pharmaceutical Sector”** — is both appropriate and timely. As pharmacy students, it is essential that you understand the policy directions, reforms and challenges within the profession you are preparing to enter. Pharmacy, as a healthcare profession, involves the scientific knowledge and practical expertise necessary for the **manufacture, formulation, storage, dispensing and distribution of pharmaceutical products**, among other vital responsibilities.

2.0 THE NEED FOR HEALTH PROFESSIONS

The human body is designed in such a way that it may not always remain in perfect condition throughout life — whether in infancy, childhood, adolescence, adulthood, or old age. Over thousands of years, mankind has developed **curative, preventive and diagnostic interventions** to address illnesses and maintain well-being. Over

time, specialization in healthcare delivery evolved — from the days of the ancient apothecaries to the sophisticated system we have today.

In Nigeria, the first legal framework for regulating healthcare professionals was established in **1927** through the creation of the **Board of Medical Examiners (BME)** under British colonial administration. From that single regulatory body in 1927, Nigeria now has **thirteen (13)** health regulatory agencies — a reflection of the growth and dynamism of the nation's healthcare sector.

These are:

1. **Pharmacy Council of Nigeria (PCN)** – for pharmacy and pharmacists
2. **Medical and Dental Council of Nigeria (MDCN)** – for medicine, dentistry, physicians, and dentists
3. **Nursing and Midwifery Council of Nigeria (NMCN)** – for nursing and midwifery
4. **Medical Laboratory Science Council of Nigeria (MLSCN)** – for medical laboratory science
5. **Community Health Officers Registration Board of Nigeria (CHORBN)** – for community health officers
6. **Radiographers Registration Board of Nigeria (RRBN)** – for radiography and radiographers
7. **Optometrists and Dispensing Opticians Registration Board of Nigeria (ODORBN)** – for optometry and opticians
8. **Medical Rehabilitation Therapists (Registration) Board of Nigeria (MRTB)** – for physiotherapy and rehabilitation therapists
9. **Health Records Officers Registration Board of Nigeria (HRORBN)** – for health records officers

10. **Dental Therapists Registration Board of Nigeria (DTRBN)**
– for dental therapy
11. **Dental Technologists Registration Board of Nigeria (DTRBN)** – for dental technology
12. **Institute of Public Analysts of Nigeria (IPAN)** – for public analysts
13. **Institute of Chartered Chemists of Nigeria (ICCON)** – for chemistry and chemists

Each of these regulatory agencies operates under the supervision of the **Federal Ministry of Health (FMoH)** and was established by an Act of the National Assembly to oversee education, training and professional practice in its respective field.

3.0 THE PHARMACY COUNCIL OF NIGERIA (PCN)

The **Pharmacy Council of Nigeria (PCN)** was established by the **Pharmacy Council of Nigeria (*Establishment*) Act, 2022**, to regulate and control pharmacy education, training, and practice in all aspects.

Predecessor bodies to the PCN include:

1. **Pharmacy Board of Nigeria** (1936)
2. **Pharmacists Board of Nigeria (PBN)** (1964)
3. **Pharmacists Council of Nigeria (PCN)** (1992)

3.1 REGULATION OF PHARMACY EDUCATION AND TRAINING

The PCN regulates pharmacy education and training through the **Education and Training Department** of its Registry and Head Office. The Council's responsibilities include:

- Accreditation of **PharmD** and **B.Pharm** programme in Nigerian universities

- Accreditation of **Pharmacy Technician programme** in colleges or schools of health technology
- Accreditation of **internship centres** in healthcare and pharmaceutical manufacturing facilities
- Supervision of the **Foreign Pharmacy Graduates Orientation Programme (FPGOP)**
- Coordination of **Mandatory Continuing Professional Development (MCPD)** programme in collaboration with universities and the National Institute for Pharmaceutical Research and Development (NIPRD), Abuja

All examination results arising from these programs are subject to review and approval by the Council before release.

3.2 REGULATION OF PHARMACY PRACTICE

Pharmacy practice is regulated by the Governing Council of the PCN through three core departments:

- **Inspection and Monitoring (I&M)**
- **Pharmacy Practice (PP)**
- **Enforcement**

Key reference materials include:

- *Compendium of Minimum Standards for the Provision of Pharmaceutical Care in Nigeria*
- *PCN Guidelines for the Issuance of Patent and Proprietary Medicine Vendor Licences (PPMVL)*
- *Manual for Pharmaceutical Inspectors*
- *Code of Ethics for Pharmacists in Nigeria*
- *PCN Sourcebook on Pharmacy Regulation in Nigeria*

Inspections and monitoring are conducted by:

- **State Pharmaceutical Inspectors Committees** chaired by Directors of Pharmaceutical Services
- **PPMVL Inspection Committees** in all states and the FCT

- **Hospital Pharmacy Inspection Committees**
- **Inspection teams for pharmaceutical manufacturing facilities**
- **PCN Enforcement Department** for compliance with licensing and operational standards

The overarching goal is to ensure that the public has access only to duly inspected, approved, registered and licensed pharmaceutical premises. Those operating outside regulatory standards face appropriate sanctions.

4.0 THE CHALLENGES OF REGULATION AND POLICY REFORMS

In fulfilling its statutory mandate, the PCN encounters several challenges similar to those faced by other regulatory agencies. These include:

1. **Inadequate inspection personnel**, including registry staff
2. **Limited logistics**, especially vehicles for inspection, accreditation and enforcement
3. **Insufficient funding** for staff training and development
4. **Jurisdictional overlaps** with other boards and agencies (e.g., NBTE, NVRC) due to unclear boundaries
5. **Periodic tensions** with professional associations related to pharmacy practice

Despite these challenges, the PCN continues to perform commendably. Over the past two decades, it has expanded its operational presence across the federation. In 2003, there were only **four offices** — Abuja (Head Office and Registry), Enugu, Kano, and Lagos. Today, there are **forty-four (44)** state and zonal offices across Nigeria.

CONCLUSION

The Pharmacy Council of Nigeria has made remarkable progress in fulfilling its mandate to uphold professional standards and ensure quality healthcare delivery. I commend the students of Sa'adu Zungur University for organizing this intellectually stimulating summit. Your choice of theme reflects foresight and a deep sense of professional responsibility.

Thank you for your kind attention and for the opportunity to share my thoughts with the students and faculty of this great institution — named after a great Nigerian.



Mallam Abdurrahman Mora, Headmaster of the Zaria Middle School (1950-1954) Cedar Falls, USA in (1952/1953).



Pictures of Mallam Abdurrahman Mora, Headmaster of the Zaria Middle School (1950-1954) (standing first from right) at Iowa State University (now University of Iowa), Cedar Falls USA, with other trainees on US Teacher Training Fellowship programme (1952-1953).



H.E. Alhaji A. Mora chatting with the Charge d'Affaires of Cuba and Algeria.



H.E. Alhaji A. Mora shaking hands with Ambassador of Kuwait, wife of the Kuwait Ambassador. 1967/68.



H.E. Alhaji A. Mora shaking hands with the wife of the Malaysian High Commissioner.



Pictures of His Excellency Alhaji Abdurrahman Mora, Nigeria's High Commissioner (Ambassador) to the Islamic Republic of Pakistan (1967-1971) with fellow diplomats at functions in Karachi and Islamabad

APPENDIX ONE

THE MEMBERS OF BOBA 1970 INTAKE CLASS GROUP AND THEIR CONTRIBUTIONS TO THE DEVELOPMENT OF TERTIARY EDUCATION IN NIGERIA

The members of the BOBA 1970 Intake Class Group have made significant contributions to the development of tertiary education in Nigeria. They have served in various leadership, administrative, and academic roles, as outlined below:

1. Vice-Chancellors

- *Prof. A. H. Rafindadi – Vice-Chancellor, Federal University Lokoja (FUL), Lokoja.*

2. Bursars

- *Alh. Muhammad Bashir Azeez – Bursar, Adamawa State University (ADSU), Mubi, Adamawa State.*

3. Registrars

- *AB. Dambazau – Registrar, Nigerian Defence Academy (NDA).*
- *Mohammed Abdu Arzika – Registrar, Federal College of Education (Technical), Bichi, Kano State; Registrar, Waziri Umaru Federal Polytechnic, Kebbi State; Registrar, Adamu Augie College of Education, Argungu.*

4. Deans

- *Prof. A. H. Rafindadi – Dean, Faculty of Medicine, Ahmadu Bello University (ABU), Zaria.*
- *Prof. Ahmed Tijjani Mora – Dean, Faculty of Pharmaceutical Sciences, Kaduna State University (KASU), Kaduna.*

5. Pro-Chancellors and Chairmen of University Governing Councils

- *Lt. Gen. A. B. Dambazau, CFR – Pro-Chancellor and Chairman, Capital City University (CCU), Kano.*

- *Prof. Ahmed Tijjani Mora, OON, mni – Pro-Chancellor and Chairman, Igbinedion University, Okada (IUO), Edo State.*

6. *Members of University Governing Councils*

- *Prof. Ahmed Tijjani Mora – Member, Governing Council, Ahmadu Bello University (ABU), Zaria.*
- *Alh. Tijjani Musa Isa – Member, Governing Council, Federal University Lafia, Nasarawa State.*

7. *Chancellors of Universities*

- *His Eminence, Alhaji Muhammad Sa’ad Abubakar, CFR, mni, Sultan of Sokoto – Chancellor, University of Benin (UNIBEN), Benin City; Chancellor, Ahmadu Bello University (ABU), Zaria; Chancellor, University of Ibadan (UI), Ibadan (current).*
- *His Royal Highness, Alhaji (Dr.) Muhammadu Abali Ibn Muhammadu Idrissa, CFR, Emir of Fika, Yobe State – Chancellor, University of Uyo (UNIUYO), Akwa Ibom State; Chancellor, Federal University Lokoja (FUL), Kogi State (current).*

8. *Heads of Departments, Rectors, Provosts, Chief Medical Directors (CMDs), and Principals*

- *Prof. A. H. Rafindadi – Head of Department, ABU; Chief Medical Director, Ahmadu Bello University Teaching Hospital (ABUTH), Zariap.*
- *Prof. Ahmed Tijjani Mora – Head of Department, KASU.*
- *Prof. Abdussalaam Yayo Manu – Head of Department, Bayero University, Kano (BUK).*
- *Bldr. A. S. Abdulhameed – Head of Department, Kaduna Polytechnic (KADPOLY).*
- *Alhaji Sani Rabi’u – Provost, Katsina College of Education (KT-COE).*

- *Alh. Abba Sada – Principal, College of Nursing, Katsina.*

9. ***Other Distinguished Classmates Serving in Tertiary Institutions***

- *Dr. Abdullahi Tafida – Lecturer, Kaduna State University (KASU).*
- *Wing Commander (Dr.) Sule Dan Abdu – Lecturer, Ahmadu Bello University (ABU).*

Note: Other members of the class also served meritoriously in academia, including the late Dr. Irimiya Aboi, PhD (of blessed memory).

APPENDIX TWO

PROF. AHMED TIJJANI MORA'S ASCENDENCY PATHWAY TO THE HEIGHT OF EACH PHARMACY PRACTICE AREA AND REGULATORY BODIES (A)

S/N	PRACTICE AREA	PERIOD	INSTITUTION/ ORGANIZATION	REMARK
1.	Hospital and Administrative Pharmacy	1978 – 1979	ABUTH (Intern)	
		1988 – 1989	Kaduna State Health Management Board, Kaduna	Chief Pharmacist and Head of Pharmacy dept
		1989 – 1992	Ministry of Health, Kaduna	Chief Pharmacist/ Director of Pharm. Services
2.	Industrial Pharmacy	1980 – 1984	Messrs Grunenthal GMBH (c/o Pharco (Nig) Ltd, Kano	Pharm. Rep. Of the North
3.		1997 – 1999	Zaria Pharmaceutical Ltd (ZPC), Zaria (ZARINJET)	MD and CEO
4.		2000 – 2003	Zazzau Pharm. Industries Ltd (ZPIL), Zaria	MD and CEO
5.	Community Pharmacy	1986 – 1988	Kaduna State Distribution Agency (KSDA) Ltd, Kaduna	Pharmaceutical Divisional Manager
6.	Academic Pharmacy	1986 – date	Faculty of Pharmaceutical Sciences, ABU, Zaria	Part - Time Lecturer
		2012 – 2016	Faculty of Pharmaceutical Sciences, KASU, Kaduna	Foundation Dean and HOD Clinical Pharmacy and Pharmacy Administration

7.		2016 – date	<i>College of Pharmacy Igbinedion University Okada (IUO), Okada</i>	<i>Professor Clinical Pharmacy and Pharmacy Practice</i>
8.		2025	<i>IU, Okada</i>	<i>Pro-Chancellor and Chairman of Governing Council</i>
9.	<i>Pharmacy Regulation</i>	1989 – 1992	<i>Pharmacists Board of Nigeria (PBN)</i>	<i>Member of Board</i>
		1992 – 1993	<i>Pharmacists Council of Nigeria (PCN)</i>	<i>Member of Council (1st council)</i>
		1999 – 2002	<i>Pharmacists Council of Nigeria (PCN)</i>	<i>Member of Council (Rep of council)</i>
		2003 – 2012	<i>Pharmacists Council of Nigeria (PCN)</i>	<i>Registrar and CEO</i>
		2020 – 2022	<i>Pharmacists Council of Nigeria (PCN)</i>	<i>Chairman of the Governing Council (Last Chairman)</i>
		2022 – 2023	<i>Pharmacists Council of Nigeria (PCN)</i>	<i>Chairman of the Governing Council (1st Chairman)</i>

APPENDIX TWO

THE CONTRIBUTIONS TO PHARMACY EDUCATION AND PRACTICE IN KADUNA STATE BY PROF. AHMED TIJJANI MORA (1982 - DATE) (B).

S/NO	ACTIVITY	PERIOD	POSITION AS
1	<i>Clinical Trials and Launch of branded Cotrimoxazole tabs (NEVIN) and branded Tramadol Hydrochloride Capsules (TRAMAL) pharmaceutical products.</i>	1982, 1983 in Zaria	<i>Pharmaceutical Rep. Grunenthal, GmBH of Germany under Pharco (Nig) Ltd, Kano.</i>
2	<i><u>Establishment of Pharmaceutical Division of KSDA Ltd:</u> - - Procurement /Importation of Pharmaceutical Products and sold at reduced prices to retailers and wholesalers within former Kaduna State (present Kaduna and Katsina States)</i>	1986-1988	<i>Pharmaceutical Divisional Manager, Kaduna State Distribution Agency (KSDA) Ltd, Kaduna (now defunct)</i>
3	<i>(i) Erection of Perimeter wall at the State Medical Stores (sms), <u>Kakuri, Kaduna.</u></i>	1988-1989	<i>As the Chief Pharmacist of the Health Management Board (HMB) Kaduna State.</i>
	<i>(ii) Initiated the DRF Scheme in 1989 at New Gen. Hospital, Kaduna and subsequently extended to Gen. Hospital, T/Wada, and Kakuri in Kaduna and Gen. Hospitals in Zaria and Kafanchan</i>	1989	
	<i>(iii) Expansion of the compounding unit at "Magajin Gari's" Health office, Kaduna.</i>		
	<i>(iv) Establishment of the Drug Information and Formulary Centre (DIFC), New Gen Hospital, Kaduna</i>		
4	<i>(i) Establishment of a Quality Control Unit for testing of procured drugs for the State at the State Medical Stores (SMS) Kakuri, Kaduna</i>	1990-1993 by ELSTOW (Nig) Ltd), Lagos	<i>As Chief Pharmacy and later Director of Pharm. Services, MOH, Kaduna</i>

	<i>(ii) Recommended for sponsorship of the first Pharmacist in the Kaduna State Civil Service for In-Service M.Sc. (Pharmaceutical Analysis) -Pharm Peter G. Usman (late) at UNILAG. He was followed by Pharm. Isah Baba for M.Sc. (Pharmaceutics)in ABU, Zaria.</i>	<i>1990-1993</i>	<i>Director of Ph arm. Services, MOH, Kaduna. Both successfully completed their studies and Isah Baba is now professor</i>
	<i>(iii) Initiated the disposable syringe plant concept in Zaria-Zaria Pharmaceutical Company (ZPC) Ltd, as Project Coordinator.</i>	<i>Agreement signed in 1993</i>	<i>Supply and installation of machinery and equipment by Consortium Voest Alpine-Austroplan of Austria</i>
	<i>(iv) Movement of the Pharmaceutical Services Dept along with other Depts from Lugard Hall to the present location at MOH, Independence Way, Kaduna</i>	<i>1992</i>	<i>Selected the present offices for the Pharmaceutical Services Department, MOH, Kaduna</i>
<i>5</i>	<i>(i) First and only Pharmacist and Pioneer Project Manager at the Project Implementation Unit PIU), AfDAB/AfDF - Assisted Kaduna Health Rehabilitation Project (KHRP), MOH, Kaduna.</i>	<i>1992- 1998</i>	<i>Project Manager PM, PIU at KHRP, MOH, Kaduna</i>
	<i>(ii) Initial Works for the Rehabilitation of five (5) hospitals at Yusuf Dantsoho, Gwamna Awan in Kaduna, and General Hospitals in Kafanchan, Saminaka & B/Gwari</i>	<i>1992-1998</i>	<i>Messrs Norman to Daburn as Architects and Consultants</i>
	<i>(iii) Procurement of Hospital Equipment for hospitals in Kaduna State.</i>	<i>1992- 1998</i>	<i>Suppliers include Hiltron (UK) Ltd.</i>
	<i>(iv) Procurement of Drugs to Improve the DRF Schemes in the hospitals in Kaduna State</i>	<i>1992- 1998</i>	<i>Suppliers include Europharm Ltd, Jos.</i>
	<i>(v) Training of Health Personnel at UNIMAID in Health Management</i>	<i>1992- 1998</i>	<i>University of Maiduguri (UNIMAID)</i>

	<i>Information System (HMIS)</i>		
	<i>(vi) Procurement of Furniture for Hospitals in Kaduna State</i>	<i>1992-1998</i>	
	<i>(i) Project Coordinator for the 135 million capacity disposable syringes production plant of Kaduna State- The Zaria Pharm. Company (ZPC) Ltd, (manufacturer of 2ml ZARINJECT syringes) as the biggest disposable syringes plant in West Africa. Plant Commissioned on 22/05/1996</i>	<i>Project Coordinator for 28 months and Coordinated the Supply and Installation of machinery and / equipment for the factory.</i>	<i>Initiator, Project Coordinator leading to completion of project.</i>
	<i>(ii) MD/CEO, ZPC Ltd, Zaria: First Pharmacist MD/CEO of the Company.</i> ❖ <i>Employed Pharmacists that have been rejected even after passing interviews- Peter Usman; Isah Baba; Benjamin Dangiwa.</i>	<i>1997—1999</i>	<i>MD/CEO, ZPC ZARIA</i>
	❖ <i>Trained Pharmacists, Engineers and laboratory Staff in Austria to run the ZPC Plant.</i>	<i>1997- 1999</i>	<i>MD/CEO, ZPC</i>
	❖ <i>Reduced the number of expatriated staff from Nine (9) to four (4) within the FIRST year as MD and to only 2 by the 1999.</i>	✓	✓
6	•• <i>Supplied syringes worth N42 million to PTF and N50 million worth of syringes for the 1998 National Immunisation Days(NIDs) of the NPI.</i>	✓	<i>Petroleum Fund National Programme Immunisation (NPI) as beneficiaries.</i>
	❖ <i>Created 42-Pharmacist distributors nationwide from the ONLY distributor at inception in</i>	✓	✓
	❖ <i>Subjected the syringes to Clinical trials in ABUTH, Zaria and OAUTH, Ile-Ife and launching of the syringes in Sheraton Hotel, Lagos and Novotel Hotel, Accra, Ghana in</i>	<i>1998- 1999</i>	<i>MD/CEO, ZPC</i>

	<i>1998 and 1999 respectively. Follow-up launchings in several Teaching Hospitals and Federal Medical Centres in Nigeria.</i>		
	❖ <i>Trained ABU Pharmacy Students in Quality Control of medical devices using the sterility tests: LAL- method and other pyrogen testing methods. - Professors from ABU and NIPRD who assisted in the teaching - PF Olurinola, J. Ojile and Kolo Ibrahim.</i>	<i>1998- 1999</i>	<i>Pharmacy Students of ABU, Zaria as beneficiaries</i>
7	<i>Part-Time Lecturer in "Pharmacy Management" for Part III (later 400 Level) B.Pharm. students of the Faculty of Pharm. Sciences, ABU Zaria from 1986 to date (39 years) continuously PRO-BONO</i> ❖	<i>1986 to Date 40 years)</i>	<i>- Lecturer II (1986-2001); - Senior Lecturer (2001) - Professor (Present)</i>
8	<i>Foundation Member, Interim National Chairman, National Chairman and presently Chairman, Board of Trustees (BoT) of the ABU Pharmacy Alumni Association (ABUPAA).</i>	<i>1994 to 2010 (Chairman); 2010–Date (BoT Chairman)</i>	<i>National Chairman; Chairman, Board of Trustees (BoT) of the ABUPAA</i>
	<i>(i) Rehabilitation of the Faculty lecture rooms.</i>	<i>1994–1998</i>	<i>Funds from Fundraising exercises in 1997 & 2008</i>
	<i>(ii) Repairs of Plumbing System, Procurement of tanks to enable free water flow to the laboratories.</i>	<i>1994–1998</i>	<i>Funds from Fundraising exercises in 1997 & 2008</i>
	<i>(iii) Supply of furniture for lecturer's offices and the Dean's office.</i>	<i>1994–1998</i>	<i>(As above)</i>
	<i>(iv) Lamination of the students' records (1972-1997).</i>	<i>1994–1998</i>	<i>(As above)</i>

	<i>(v) Supply of Reference books and other textbooks to the faculty.</i>	<i>1994–1998</i>	<i>(As above)</i>
	<i>(vi) Procurement of brand-new car - Peugeot 306 Car for the official use of the Dean.</i>	<i>First in the History of ABU for any Dean</i>	<i>(As above)</i>
	<i>(vii) Painting of the Faculty buildings.</i>	<i>1994–1998</i>	<i>(As above)</i>
	<i>(viii) Sinking of bore hole for the Faculty.</i>	<i>1994–1998</i>	<i>(As above)</i>
	<i>(ix) ICT facilities supplied at the old building.</i>	<i>1994–1998</i>	<i>(As above)</i>
	<i>(x) Initiation and supervision of the building of the twin 250-seat capacity lecture theatre for the Faculty of Pharmaceutical Sciences, ABU, Zaria by the Taraba State Government. under the leadership of the Governor, Pharm. Danbaba D. Suntai, a 1984 graduate of the faculty. Cost of the theatres, (named after Gabriel Osuide, the founder and first Dean of the Faculty) by the Taraba State Government was to the tune of N300 million.</i>	<i>Commissioned on 15th October, 2011</i>	<i>Biggest project erected by any alumni body in the history of ABU, Zaria.</i>
9	<i>Consolidation of the activities of Pharmacists within the State from 1987 to 1990 at Association level—the Pharmaceutical Society of Nigeria (PSN) (Kaduna State Branch)</i>	<i>1987–1990</i>	<i>Vice Chairman (1987-1989); Chairman (1989 to 1990) Unanimously elected unopposed and in absentia.</i>
10	<i>Taking over the Zazzau Pharmaceutical Industries (ZPIL) Zaria on a Ten (10) year lease from ABUTH, Zaria</i>	<i>2000–2010</i>	<i>Chairman of Pharmaceutical Consultancy Ltd, (PCL) the company leased the facilities at ZPIL</i>
	<i>(i) Rehabilitated the equipment and building of the facility.</i>	<i>2000–2010</i>	

	<i>(ii) Registered the products with NAFDAC—Paracetamol (tabs and syrups); Chloroquine (tabs and syrups); Folic Acid (tabs);</i>	2000–2010	
	<i>(iv) Won the Award for the best producer of Paracetamol Tablets for the whole of West Africa. Award given at Sheraton Hotel, Lagos on 24/4/2007.</i>	2007	ZPIL
11	<i>Pharmacists Council of Nigeria (PCN) (i) Created Kaduna State and NW Zonal Offices in Kaduna.</i>	2003–2012	Registrar/CEO of PCN
	<i>(ii) Building of new, Prototype NW Zonal Office and Kaduna office within Kaduna at the By-Pass area, Kaduna. Plot approval by H.E. Arc. Mohammed Namadi Sambo. Initial offices at Obasanjo House given during Arc. Namadi Sambo's regime as Governor</i>	<i>Foundation laying ceremony by the Gov. of Kaduna State ably represented by the Hon. Commissioner of Health, Dr. Yari Evarton on Monday, October 26, 2009.</i>	Registrar/CEO of PCN (Commissioned)
12	<i>Faculty of Pharmaceutical Sciences, Kaduna State (KASU), Kaduna</i>	2012–2016	
	<i>(i) Chaired the committee of the University established to the establishment of a new Faculty of Pharmaceutical Sciences in University</i>	February – May, 2012	Chairman of Committee
	<i>(ii) Appointed foundation Dean, Faculty of Pharmaceutical Sciences, KASU.</i>	October 2012 to 2016	Dean of Faculty.
13	<i>Public Presentation of two books on Pharmacy Authored: (i) "The Contributions of Students and Alumni Associations Towards Promoting Pharmacy Education</i>	May 17, 2014	Arewa House, Kaduna as Author

	<i>and Nigeria" (ISBN: Soft 978-978-071-7, Hard 978-083-070-0)</i> <i>(ii) "The Lizard Shape model of Drug Distribution" (ISBN: 978-978-083-MORA, A.T. - Author).</i>		
14	<i>Pharmacy Department, Barau Dikko Specialists Hospital, Kaduna</i> <i>(i) Initiated the concept of remodeling of the Pharmacy Department to enable the dept provide appropriate pharmaceutical care, especially with the conversion of the hospital to a teaching hospital.</i>	January, 2015	<i>Dean of Faculty of Pharmaceutical Sciences, KASU and member of the Committee for the conversion of the hospital to a teaching hospital.</i>
	<i>(ii) Promoted the dispensing software for Hospitals of the MOH. Accepted the idea and contract for the supply of computers and training of the system.</i>	January, 2015	<i>Supplies to be by Trustee Drugs Ltd, Lagos.</i>

APPENDIX THREE

□

1. PHARMACISTS COUNCIL OF NIGERIA
2. ALHAJI AHMED TIJANI MORA

V

1. PHARMACEUTICAL SOCIETY OF NIGERIA
2. NATIONAL COUNCIL, PHARMACEUTICAL SOCIETY OF NIGERIA

*COURT OF APPEAL
(LAGOS DIVISION)*

KUMAI BAYANG AKAAHS JCA (*Presided*)
RITA N. PEMU JCA (*Read the Lead Judgment*)
MOHAMMED A. DANJUMA JCA

CA/L/144/2011

FRIDAY, 10 FEBRUARY 2012

CONSTITUTIONAL LAW - Constitutional provisions

JUDGMENT AND ORDERS - Court's judgment - Conclusiveness of as between parties

JUDGMENT AND ORDERS - Judgment - Where delivered in violation of constitutional provisions - Invalidity thereof

Issue:

Whether the learned trial judge was not in error to have delivered the ruling on appeal outside the open court contrary to Order 23, rule 1 of the Federal High Court (Civil Procedure) Rules, 2009 and section 236(1) of the Constitution of the Federal Republic of Nigeria, 1999.

Facts:

The appellants as plaintiffs before the Federal High Court, Lagos,

instituted action by way of originating summons against the respondents, wherein they sought declaratory and injunctive orders against the respondents. They also sought exemplary damages in the sum of N50,000,000.00 (fifty million naira).

The trial court granted the appellants' claim in part, granting reliefs 1, and refusing reliefs 2 and 3. The trial judge did not read its judgment in open court, but in its secretary's office as the judge's chamber door was rammed and the judge could not robe.

Dissatisfied with the decision of the trial court, the appellant appealed to the Court of Appeal.

Held: (*Allowing the appeal*)

1. *Conclusiveness of court's judgment as between parties - Subject to appeal and to being amended or set aside, a judgment is conclusive as between the parties and their privies, and is conclusive evidence against all the world of its existence, date and legal consequences. [P. 1001, paras. A - B]*
2. *Invalidity of judgment delivered in violation of constitutional provision - Unless and until the court has pronounced a judgment upon the merits or by consent, it has the power to revoke the expression of its coercive power when that has only been obtained by a failure to follow any rules of procedure, but this admits of the exception that a judgment given, but not according to the constitutional requirement, is profoundly void and void it is. It renders the entire proceedings from beginning to the end null and void and of no effect whatsoever. It is as if no proceedings ever took place. The parties cannot consent to render it valid. They have no moral or constitutional power to do so. In the instant case, the irregularity occasioned by the learned judge's delivering of the judgment in his secretary's office is fundamental. This lacuna touches on the legality of the entire proceedings, including the judgment in all its ramifications. [P. 1003, paras. C - E]*
Per DANJUMA JCA: [Pp. 1004 - 1005, paras. F - E]

the said date. The purported ruling, though existing in fact, but in law, i.e. *dejure*, did not exist as non-compliance with the constitutional requirement that a trial, (including judgment) shall be held in a court or tribunal which is opened to the members of the public, robbed it of legal existence. Section 36(3) of the Constitution of the Federal Republic of Nigeria, 1999 as amended, provides thus:

‘The proceedings of a court or the proceeding of any tribunal relating to the matters mentioned in subsection (10) of this section including the announcement of the decision of the court or tribunal shall be held in public.’

There being no delivery of the ruling in the public, as members of the public had no right of access to the learned trial judge’s secretary’s office as of right, the proceedings thereat was a nullity.

A violation of the Constitution, the grundnorm of the Nigerian legal system, which is binding on all persons, courts and authorities in Nigeria vitiated the said ruling.

Appeal against same has merit. Indeed, the learned trial judge has no jurisdiction to sit as he did, as one of the conditions precedent to the validity of the ruling was the venue of session and the constitutional character of same.

This therefore means that there was a feature in the case that prevented it from being valid.

In *Skenconsult v. Ukey* (1981) 1 SC 6; *Madukolu v. Nkemdilim* (1962) 1 All NLR (Pt. 4) 557; *Utih v. Onoyivwe* (1991) 1 NWLR (Pt. 166) 166 at 206, the Supreme Court held that proceedings conducted in a state where any of the conditions precedent for its validity is absent is void and has no legal efficacy.

It is for this and upon the well reasoned questions raised by the lead judgment, which I adopt as mine that I agree and subscribe to the

view that this appeal must succeed.

Indeed, even if the ruling were delivered in a public place (court, so designated) it is my judgment that this appeal would still have been meritorious as the ruling was not only arrived at without affording the parties the right of fair hearing on the issue *suo motu* raised by the learned trial judge: The learned judge, indeed arrived at the decision in perversity (with due respect) having blown hot and cold as to the cause of action and the right procedural steps to prosecute the claim.”

Nigerian Cases Referred to in the Judgment:

Madukolu v. Nkemdilim (1962) 1 All NLR (Pt. 4) 587

Skenconsult v. Ukey (1981) 1 SC 6

Utih v. Onoyivne (1991) 1 NWLR (Pt. 166) 166

Foreign Cases Referred to in the Judgment:

Macfoy v. U.A.C. (1962) A.C. 152

Smith v. Clay (1967) 3 Bro. CC. 639

Nigerian Statutes Referred to in the Judgment:

Constitution of the Federal Republic of Nigeria, 1999, section 36(3)

Human Rights Act, 1998, section 1(3), Schedule 1 Article 6(1)

Pharmacists Council of Nigeria Act, No. 91 of 1992, sections 1, 3(1)(f), 17, 18, 21, 23, 24 and 27

Nigerian Rules of Court Referred to in the Judgment:

Federal High Court (Civil Procedure) Rules, 2009, Order 23, rules 1, 3, and Order 46, rules 1 and 2

Book Referred to in the Judgment:

Halbury's Laws of England, 4th Edition-Reissue, Vol. 37, paragraph 1059, page 333

Counsel:

Josiah Daniel-Ebune - *for the Appellant.*

J. O. Excamacchi Esq. - *for the Respondent.*

A PEMU JCA (Delivering the Lead Judgment): This is an appeal against
nigh Cuc Archibong defier 10 Court, Ligeria, whiint, the ajoturadl to the
ardndureliefs het ansises of appeal.

B Facts culminating in this appeal

affomectis the slurt ne ivitag a ta thons divlecl; wprnsed reloveds, anlis
reliefs, and elthier pharraes of montristions, pages 26.02. See pages 524-
appeal.

The attenuating cone to podsund in lire apge riltefs wtb plamadid: theavalls is afor:

1. A declaration dat eby by the commuon of the defendants
18,21, 21, 24,20 Ploymaciss on f áthes confesa bapes what
act No. So No. 91 1992, ond the relifin, and the stantent and
1.301, Sôciog villl, Suppers and hounmer in on bowels fin
in arser to statuary afiliss onforechderals on easord dade
r dor opateruthe and frölvellf, purposed with caferad ing oc flued-
nbarion ded 2011, pages 29e 1 Pcd 2010.

the beliefs reforets dios cy cers.

1. A declaration that the ielisiardi isfdaras of the plarmacy professions of Act No. 11992.

2. A declaration that article 14 of the constitution of the defendants is subject to sections 1, 17, 18, 21, 23, 24 and 27 of the Pharmacists Council of Nigeria, Act No. 91 of 1992 and article 14 of the 1st defendants' constitution is null and void for all purposes to the extent of any inconsistency with the said provisions of.

3. A declaration that the plaintiffs have no statutory and/or any obligations with regard to the nomination, refusal or acceptances and/or inauguration of persons who constitute the board of the 1st plaintiff especially under section 3(1)(f) of the Pharmacists Council of Nigeria Act, No. 91 of 1992 and such could not have acted in any way or manner whatsoever in connection with the said issues as to amount to a professional misconduct.

- H 4. A declaration that the defendants acted illegally, *ultra vires*, and unconstitutionally in the purported act of the de-fellowing of the 2nd plaintiffs as well as the suspension of 2nd plaintiff as its member on the grounds of unethical misconduct as a pharmacist, a subject matter over which it has no jurisdiction - pharmacol. 20.

H

- as its fellow and suspension as a member of the 1st defendant, the defendants acted in breach of the provisions of section 36 of the Constitution of the Federal Republic of Nigeria, 1999, the rules of natural justice and equity and articles 14 and 5(v) of the constitution of the 1st defendant A
6. An order of this honourable court vacating nullifying quashing and/or setting aside all the proceedings leading to and the purported act of the defendants and/or its servants, privies, agents and principles in de-fellowing the 2nd plaintiff as a fellow of the 1st defendant as well as his suspension as a member of the 1st defendant on the purported allegation of unethical misconduct as a member of the 1st defendant. B C
 7. A declaration that the 2nd plaintiff is a member and a fellow of the Pharmaceutical Society of Nigeria.
 8. An order of perpetual injunction restraining the defendants, whether by themselves, servants, agents, privies whosoever and however defined from usurping and/or impersonating the disciplinary functions of the 1st plaintiff by virtue of the provisions of the Pharmacists Council of Nigeria, Decree No. 91 of 1992. D
 9. ₦50,000,000.00 (fifty million naira) being exemplary damages against the defendants for the psychological trauma, embarrassment and harassments that their illegal and unconstitutional activities have caused the plaintiffs. E
 10. Further or other relief(s).
 11. That provisions may be made for the cost of this proceedings.
- The case of the appellants is in effect, *inter alia*, challenging the legality of the disciplinary powers carried out by the defendants in purported resolutions embedded in a communique published in several dailies, suspending the 2nd appellant as a member of the 1st respondent and de-fellowing him as a fellow of the 1st respondent. The said resolutions affected also other professional pharmacists registered in Nigeria by the 1st appellant, who were not a party to the meeting where the purported resolutions contained in the purported communique were made. F G
- The 2nd appellant was not aware of such proceedings, nor given opportunity, to make any sort of representations to defend the premediated allegations against him by an aggrieved member.
- The grounds on which the resolutions were made were: H
- (a) "unethical misconduct,"
 - (b) "unlawful activities of State Health Regulatory Agencies."
- The allegation of "unethical misconduct was tied to what the 1st

- A respondent called "glaring acts calculated to embarrass, ridicule and discomfit the PSN". Based on this, the respondents resolved and approved the suspension of Alhaji Ahmed T. Meraus a member of the PSN and placed him permanently on the PSN roll of dishonor. Consequent upon this, the 2nd appellant's membership of PSN was suspended and he was delisted as fellow of PSN. By this, he can no longer participate in any PSN national and state functions or "represents pharmacists within and outside Nigeria". (Refers to exhibits D, E, EI and E2). The said exhibits also purported to approve the suspension as well as placing the names on roll of dishonor. Pharmacists Uche Madumere and Mrs. H. Nene Isandiyidighi, on the alleged grounds that "by their action truncated the bid of the PSN to dialogue with them on professional matters"

- C In their further and better affidavit in support of the originating summons, exhibits "G" and "H" were attached, which is a certified true copy of a newspaper publication showing, that the appellants became aware for the 1st time that the defendants had in July 2009, penciled down and indicted the 2nd plaintiff ever before the 1st plaintiff board in contention was constituted. Exhibit "G" is at page 306 of the record of appeal while exhibit "H" spans pages 308-319 of the record of appeal.

- D The plaintiffs were not heard, and the 1st respondent was aware that there is a pending court action on the basis of the very allegation(s) it acted against the plaintiffs before the Federal High Court, but went ahead to hold an "extraordinary meeting" to suspend the 2nd appellant, de-fellow him as well as suspend and also de-fellow other duly registered Nigerian Pharmacists. The appellant grieves that on the date fixed for the ruling, the open court was filled to capacity but the court instructed its secretary to invite counsel to the parties to take the ruling in his secretary's office, and noteven in his chambers.

- F The learned trial judge deviated from the issues before him by completely departing from the crux of the originating summons, but based his decision on issues not before the court, nor between the parties.

- G The respondents, in a counter-affidavit of thirty-nine paragraphs, and filed on 6 April 2010 answered to the facts deposed to in the affidavit in support of the originating summons, Pages 159 - 166 of the record of appeal.

- The learned trial judge, having heard learned counsel for the respective parties, on 15 November 2010 read out his ruling (but not in open court).

- H For purposes of elucidation, I deem it necessary to reproduce the proceedings of 15 November 2010, shorn of the actual ruling, page 520 of the record of appeal.

*"In the Federal High Court
Holden at Lagos,*

<i>Nigeria</i>		A
<i>On Monday, 15 November 2010</i>		
<i>Before the Honourable</i>		
<i>Justice Charles Efang Archibong</i>		
<i>Judge</i>		
<i>Suit No: FHC/L/CS/284/2010</i>		B
Between:		
1. Pharmacist Council of Nigeria		
2. Alhaji (Pharm) Ahmed Tijani Mora	Plaintiffs	
And		
1. Pharmaceutical Society of Nigeria		
2. National Council, Pharmaceutical Society of Nigeria	Defendants	C
Parties absent		
Josiah Daniel Ebune Esq. - for the Plaintiffs		
J. O. Ezeamachi Esq. - for the Respondent		
Court:	Ruling read	D
	Not read in open court. Judge's chamber door was rammed and judge could not robe.	
	(Sgn.)	
	C. E. Archibong	
	Judge	E
	15 November 2010".	
At pages 4-5 of the ruling which is reflected at pages 524-525 of the record of appeal, the learned trial judge had this to say		
"I will grant the plaintiffs' 1st relief sought. Reliefs 2 and 3 relate to non-issues.		
I do not see that the defendant acted illegally, <i>ultra vires</i> and unconstitutionally in "de-fellowing" a person whom they had made fellow of their society in the first place nor that they are not unable to suspend one of their own members from the roll as a member.		F
All other reliefs are denied.		
	C. E. Archibong	G
	Judge	
	15 November 2011"	
The appellants, dissatisfied with this decision has appealed it. in line with the practice direction of this court, they filed a notice of appeal on 26 November 2010 which was amended on 4 March 2011 but was deemed filed on 2 June 2011, encapsulating three grounds of appeal. They are:		H
Ground 1	"The learned trial judge erred in law when, on the pretext	

- A that he could not access his chambers, he sat unrobed in his secretary's office to deliver the ruling in the presence of counsel to both parties who were robed.
- Ground 2 "The learned trial judge erred in law when he held that the proceedings by way of judicial review and not originating summons is the proper procedure *vide* which he could entertain certain aspects of the plaintiffs' case relating to the disciplinary proceedings wherein the 2nd defendant was purportedly suspended and de-fellowed and thereby came to a wrong decision occasioning a miscarriage of justice.
- B
- C Ground 3 "The learned trial judge erred in law when he misconstrued the case of the plaintiffs as challenging the powers of the defendants to make rules regulating its members when the case submitted to him for determination was not whether the defendants had such powers but whether in the exercise of such powers *qua* their members, they are bound to comply with the provisions of section 36 of the Constitution of the Federal Republic of Nigeria, 1999, their own, 2005 constitutional provisions relating to fair hearing and the rules of national justice."
- D
- E The appellants filed their brief of argument on 31 March 2011, while the respondents' brief of argument was filed on 3 May 2011.
- The appellants' brief settled by Josiah Daniel-Ebune, Esq. had identified, formulated and proffered three issues for determination (as discerned at page 12 - 13 of their brief of argument. They are:
- F 1. "Whether the learned trial judge was not in error to have delivered the ruling on appeal outside the open court contrary to Order 23, rule 1 of the Federal High Court (Civil Procedure) Rules, 2009 and section 36(3) of the 1999 Constitution of the Federal Republic of Nigeria. (Ground 1 of the amended notice of appeal).
- G 2. "Whether the learned trial judge was right when he held in effect, *suo motu*, and without giving the parties to the case the opportunity to address him on that point, at pages 521 - 522 and 24 of the record as follows:
- H "The originating summons poses several questions for determination not all of which are justiciable in this form of action. This is an originating summons not an action for the judicial review of an action or order given by the

- body in question here; the Pharmaceutical Society of Nigeria. No record of proceeding are before us but we are asked whether the final action taken by the body in question was *ultra vires* or otherwise. The only valid question for determination is question one" ...
- "The defendants have not joined issues on this and their counter-affidavit details other matter that cannot be countenanced in this form of action."
- And whether that holding did not amount to a denial of fair hearing to the appellants as well as substantially affected the premises for refusing the appellants the other reliefs they sought and whether in the process, the learned trial judge has not occasioned the appellants a miscarriage of justice. (Grounds 2 and 4 of the amended notice of appeal).
3. "Whether the learned trial judge was not in error when after holding in effect, that an action by way of judicial review, rather than the originating summons proceedings is the proper procedure where the action of the respondents complained on the originating summons could be questioned, he still went ahead to pronounce on the merits of the originating summons by holding at page 525 of the record, as follows:
- "I do not see that the defendant acted illegally, *ultra vires* and unconstitutionally in "de-fellowing" a person whom they had made a fellow of their society in the first place nor that they are not unable to suspend one of their own members from the roll as a member."
- And whether such holding has not occasioned the appellants a miscarriage of justice. Ground 3 of the amended notice of appeal).
- The respondents had formulated just one issue for determination at page 2 of his brief, but adopsis issues 2 and 3 of the appellants' brief. The brief of argument settled by Jude O. Ezeamaechi Esq., is:
- "whether the learned trial judge was right when he delivered the ruling of 16 November 2010 in his secretary's office on the ground that the door to his chambers was rammed and could not open (Ground 1 of the amended ground of appeal)"
- In considering this appeal, I am poised strongly to see how tenable ground 1 of the grounds of appeal is. It is from this ground that issue No 1 of the respective parties issues for determination is distilled.
- If this ground succeeds, it cuts across the entire appeal in the positive as a life wire that must take life when exposed to life. If ground No. 1

A succeeds, it renders null and void the entire proceedings, no matter how well and elegantly conducted.

It is trite that, subject to appeal, and to being amended, or set aside, a judgment is conclusive as between the parties and their privies, and is conclusive evidence against all the world of its existence, date and legal consequences.

B The requirement that hearing of a case by the courts should be in public is sacrosanct, subject to certain exceptions in our rules of court. In Nigeria, it is a constitutional requirement. It is also entrenched in our rules of court including the Federal High Court (Civil Procedure) Rules, 2009.

C Section 36(1) and (3) of the Constitution of the Federal Republic of Nigeria, 1999 is apt. Let me reproduce same *verbatim*:

Section 36(1)

"In the determination of his civil rights and obligations, including any question or determination by or against any government or authority, a person shall be entitled to a fair hearing within a reasonable time by a court or other tribunal established by law and constituted in such manner as to secure its independence and impartiality."

D

Section 36(3)

"The proceedings of a court or the proceedings of any tribunal relating to the matters mentioned in subsection (1) of this section (including the announcement of the decision of the court or tribunal) shall be held in public."

E

This matter came before the Federal High Court, sitting in Ikeja Lagos, and was determined there.

F Order 23(1) of the Federal High Court (Civil Procedure) Rules, 2009 is worthy of reproduction. It says:

"The judge shall, after trial deliver judgment in open court."

Section 1(3), schedule 1 article 6(1) of the Human Rights Act, 1998, provides that:

G

"In the determination of his civil rights and obligations, everyone is entitled to a fair and public hearing. Judgment is to be pronounced in public, but the press and public may be excluded from all or part of the trial in the interests of morals, public order or national security in a democratic society, where the interests of juveniles or the protection of the private life of the parties so require, or to the extent strictly necessary in the opinion of the court in special circumstances where publicity would prejudice the interests of justice."

H

Paragraph 1059, page 333 of Halbury's Laws of England, 4th Edition,

Reissue, Vol. 37.

A

It is obvious, from the record of appeal, that the judgment read on 15 November 2010, the subject matter of this appeal, was not read in open court.

The fact that the judge's chamber door was rammed, thereby preventing him from robing cannot afford him any excuse.

B

Learned counsel for the respondents, Jude O. Ezeamacchi Esq. had argued that the learned trial judge is permitted to deliver his judgment in his secretary's office, by the provisions of Order 46, rules 1 and 2 of the Federal High Court (Civil Procedure) Rule, 2009 which provides as follows:

1. "A court may at its discretion appoint any day(s) and anyplace from time requires.

C

2. The sittings of a court for the hearing and determination of the rights and obligations of parties shall be in public but subject to the provisions of the Constitution of the Federal Republic of Nigeria, the court may, for special reasons, hear any particular action in the presence only of the parties, their legal practitioners (if any) and the officers of the court."

D

See page 5 paragraph 5.0.1 of the respondents' brief of argument.

I beg to disagree with this line of thought. This is simply because the provision of Order 46, rules 1 and 2 of the Federal High Court (Civil Procedure) Rules must bow to the constitutional requirement that hearing of matters, including the delivering of judgment must be in public.

E

Moreso, while the constitutional provision in section 36(3) is mandatory, that in Order 46, rule 1 of the Federal High Court (Civil Procedure) Rules, 2009 is directory only. Indeed, the provisions of Order 46, rule 2 is expressly subject to the provisions of the Constitution of the Federal Republic of Nigeria, 1999.

F

Learned counsel's argument in paragraph 5.0.2 that rules of court are not mere rules, but that they partake of the nature of subsidiary legislation, by virtue of section 18(1) of the Interpretation Act, and therefore have the force of law is misconceived, in the face of the glaring travesty of the provisions of the Constitution by the learned trial judge. This is because rules of court are subject to the provisions of the Constitution.

G

I find the argument canvassed by learned counsel for the respondents as utterly misconceived.

What stopped the learned trial judge from reading his decision in the open court? If the court hall was filled to the brim, was his chair in court also occupied?

H

Learned counsel had also argued in paragraph 5.0.7, at page 8 of his

The success of that ground of appeal disposes of the matter, and I find in consequence that the appeal is meritorious. A

The failure of the learned trial judge to read his judgment of 15 November 2010 in public, in other words in open court, has rendered the entire proceedings null and void.

The result is that the appeal succeeds and it is hereby ordered that the judgment of Honourable Justice Charles Efianga Archibong of 15 November 2010 at the Federal High Court holden at Lagos, Nigeria, be and is hereby set aside. B

Parties shall bear their own costs.

AKAAHS JCA: I read in draft the judgment of my learned brother, Pemu, JCA. I am in agreement that the appeal has merit and should be allowed. The reason given by the learned trial judge to justify sitting in chambers instead of sitting in open court is to say the least, laughable and unserious. Consequently, the appeal must be allowed and the judgment set aside. I abide by an *abid*. C D

DANJUMA JCA: I have had the benefit of reading the draft of the judgment just rendered and I agree completely with the reasoning and conclusion arrived thereat that this appeal has merit and should be allowed. The facts of the case as led before the trial Federal High Court have been so meticulously set out in the lead judgment that they should suffer no repetition. The issues and the arguments had also been aptly set out. Suffice it to state that the issue No. 1 formulated and addressed by the parties had effectively knocked off the bottom of the purported judgment by the Federal High Court delivered on 15 November 2010 by C. E. Archibong, J. There was in law, no valid ruling delivered on the said date. The purported ruling, though existing in fact, but in law, i.e. *de jure*, did not exist as non-compliance with the constitutional requirement that a trial, (including judgment) shall be held in a court or tribunal which is open to the members of the public robbed it of legal existence. Section 36(3) of the Constitution of the Federal Republic of Nigerin, 1999, (as amended), provides thus: E F G

“The proceedings of a court or the proceeding of any tribunal relating to the matters mentioned in subsection (10) of this section, including the announcement of the decision of the court or tribunal shall be held in public.” H

There being no delivery of the ruling in the public, as members of the public had no right of access to the learned trial judge’s secretary’s office

A as of right, the proceedings thereat was a nullity.

A violation of the Constitution, the grundnorm of the Nigerian legal system, which is binding on all persons, courts and authorities in Nigeria vitiated the said ruling.

B Appeal against same has merit. Indeed, the learned trial judge has no jurisdiction to sit as he did, as one of the conditions precedent to the validity of the ruling was the venue of session and the constitutional character of same.

This therefore means that there was a feature in the case that prevented it from being valid.

C In *Skenconsult v. Ukey* (1981) 1 SC 6; *Madukolu v. Nkemdilim* (1962) 1 All NLR (Pt. 4) 587; *Utih v. Onoyivwe* (1991) 1 NWLR (Pt. 166) 166 at 206, the Supreme Court held that proceedings conducted in a state where any of the conditions precedent for its validity is absent is void and has no legal efficacy.

D It is for this and upon the well reasoned questions raised by the leading judgment, which I adopt as mine that I agree and subscribe to the view that this appeal must succeed.

E Indeed, even if the ruling were delivered in a public place (court, so designated) it is my judgment that this appeal would still have been meritorious as the ruling was not only arrived at without affording the parties, the right of fair hearing on the issue *suo motu* raised by the learned trial judge. The learned judge, indeed arrived at the decision in perversity (with due respect) having blown hot and cold as to the cause of action and the right procedural steps to prosecute the claim.

F Appeal succeeds. I abide by the consequential orders setting aside the said ruling of Honourable Justice Charles Efang Archibong in suit No. FHC/L/CS/284/2010 delivered on 15 November 2010 at the Federal High Court, sitting in Lagos.

Appeal allowed

G

APPENDIX FOUR



**INDEPENDENT CORRUPT PRACTICES AND OTHER
RELATED OFFENCES COMMISSION**

☎: 09-5230898, 5230873
Fax: 09-5236717
E-mail: icpcnews@yahoo.com
icpcnews@icpcnigeria.com
Web: www.icpcnigeria.com
ICPC/P/SW/159/2011

Plot 802, Zone A9,
Constitution Avenue,
Central Area,
Abuja - Nigeria.
November 23, 2012

Our Ref: Date:

Pharm. Azubike Okwor FPSN FPCPharm FFIP
President, Pharmaceutical Society of Nigeria,
Pharmacy House,
32, Faramobi Ajike Street,
Anthony Village,
Lagos.

Dear Sir,

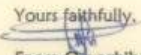
PETITION AGAINST ALHAJI TIJANI MORA, REGISTRAR AND SECRETARY OF
PHARMACISTS COUNCIL OF NIGERIA (PCN) FOR FINANCIAL MISAPPROPRIATION,
MALFEASANCE AND FRAUD

The petition and remainder dated December 21, 2011 on the above subject refers please.

2. I am directed to inform you that after discreet investigation of the allegations
contained in the petition, the Commission finds no infraction of the provisions of the
Corrupt Practices & Other Related Offences Act 2000.

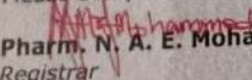
3. It is hoped that the role played by the Commission and the above information
will serve your purpose.

Accept please, the assurances of the Hon. Chairman's regards and high esteem.

Yours faithfully,

Enosa Omoghibo
For: Hon. Chairman, ICPC.

Cc: Pharm. Ahmed T. Mora
23/25 Mora Road,
Zaria, Kaduna State.

APPENDIX FIVE

	PHARMACISTS COUNCIL OF NIGERIA <i>Established by Act P17, LFN 2004</i>	HEAD OFFICE/ REGISTRY Plot 7/9 Idu Industrial Area P.M.B. 419, Garki, Abuja Tel: 8806 - 885 - 5429 8803 - 887 - 5867
REGISTRAR Pharm. N.A.E. Mohammed, B. Pharm, MBA, MScPharm, FPSN All Correspondence to be addressed to the Registrar E-mail: pcnig@yahoo.co.uk Website: www.pcn.gov.ng Our Ref: PCN/C.4046/VOL.XV/666 Your Ref:		April 17, 2019
Prof. Ahmed T. Mora, mni No. 23/25, Mora Road Zaria Kaduna Kaduna State		
INVITATION TO THE INAUGURATION OF THE PCN GOVERNING COUNCIL AND THE INAUGURAL MEETING OF THE COUNCIL		
May I humbly draw your attention to letter reference number DHS/890/1/II dated April 16, 2019 notifying the Registrar of the inauguration of the newly constituted Governing Council of Pharmacists Council of Nigeria under the chairmanship of Prof. Ahmed T. Mora, mni by the President, Commander-in-Chief of the Federal Republic of Nigeria.		
The Honourable Minister of Health will be inaugurating the Governing Council among other Governing Councils on Thursday, April 25, 2019.		
It is on this note that I write to invite the Chairman to the inauguration and inaugural meeting of Governing Council of the Pharmacists Council of Nigeria with details as follows:		
Date:	Thursday, April 25, 2019	
Venue:	Executive Hall International Conference Centre (ICC) Central Business District Abuja	
Time:	11.00 a.m.	
Inaugural Meeting:	Immediately after the Inauguration	
Venue:	Pharmacists Council of Nigeria Plot 7/9, Idu Industrial Layout Idu, Abuja	
Please accept the assurances of the Registrar's highest consideration.		
 Pharm. N. A. E. Mohammed, FPSN FNIM, FPCPharm, FNAPharm Registrar		

APPENDIX SIX

THISDAY NEWS

Breaking News

Latest Headlines

Pharmacists Reject Mora as PCN Chairman, Threaten Nationwide Strike

Pharmacists have rejected the appointment of Mr. Ahmed Tijjani Mora as chairman of the Governing Board of the Pharmaceutical Council of Nigeria (PCN).

Opposition to Mora's appointment is being spearheaded by President of the Pharmaceutical Association of Nigeria (PSN), Mr. Sam Ohuabunwa, and all his predecessors, who have called on President Muhammadu Buhari to stop the inauguration of Mora as the board chairman.

In an open letter to the president yesterday, and signed by all nine former and present presidents of the association, the pharmacists said should Mora be inaugurated, PSN would mobilise pharmacists to embark on a nationwide strike.

Signatories to the open letter are Ohuabunwa, and past presidents of PSN – Ahmed Yakassai, Olumide Akintayo, Azubike Okwor, Anthony Akhimien, O. Uwaga, Mohammed Yaro Budah, Eme Ekaite, and Philip Emafo.

They said: "The PSN observes with satisfaction a seeming tendency to appoint fit and proper persons as chairman of the PCN, which is a parastatal of the Federal Ministry of Health that regulates and controls pharmacy in all aspects and ramifications which therefore grounds the PCN as the template of ethics and values of the pharmacy profession.

"In the circumstance, our pedigree and heritage are determined by the moral and ethical quality of the person who is appointed chairman of the Governing Council of the PCN.

“From all the evidence adduced above, it is our view that Mora carries a heavy moral and ethical burden, which gravely damages his suitability for the sacred job of chairman of the PCN.

“We strongly enjoin the federal government to continue its tradition of consulting with the PSN as part of due diligence to ensure quality control of persons who emerge as chairman of the council.”

They called on the president to allow PCN produce three nominees through the Federal Ministry of Health (FMOH) from which the federal government will pick one to chair the board, adding that alternatively, Buhari can appoint any other pharmacist from the same zone with Mora.

“It may interest the president to note that the person the PSN originally recommended to the minister as chairman is Yaro Budah, who actually was nominated by the minister of health to the secretary to the federal government in a letter dated May 2018.

“In mysterious way, another letter dated October 2018 with the same reference number was written by the same minister of health to the SGF nominating Mora. Two letters nominating two different people with the same reference number signed by the same person without any reference to the previous letter.

“In the interim, we strongly enjoin the federal government to indefinitely suspend or cancel the inauguration of the PCN Governing Council with Mora as chairman for the reasons earlier stated. We believe that whoever is careless with the truth in small matters cannot be trusted with important matters,” they stated.

They said if Mora is inaugurated, PSN will “call on all hospital and administrative pharmacists in government hospitals, community and industrial pharmacists in the private sector to proceed on a total strike and withdrawal of their services. For the records, this might affect the logistics of production and supply of essential as well as other pharmaceutical care services and drugs to the entire health system, including the isolation centres that cater for COVID-19 cases in Nigeria.

“This will also lead to a directive to all registered pharmacists to boycott all activities of the PCN forthwith. This will include Mandatory Continued Professional Development (MCPD), registration of pharmacists and pharmaceutical premises as well as other routine statutory activities of the PCN.”

They also threatened to solicit the support of all other health workers for solidarity action to resist the imposition on the pharmacists in Nigeria that may signal an inadvertent move to destroy professional ethics and integrity.

Related Articles

Founded on January 22, 1995, THISDAY is published by THISDAY NEWSPAPERS LTD., 35 Creek Road Apapa, Lagos, Nigeria with offices in 36 states of Nigeria, the Federal Capital Territory and around the world. It is Nigeria’s most authoritative news media available on all platforms for the political, business, professional and diplomatic elite and broader middle classes while serving as the meeting point of new ideas, culture and technology for the aspirational and millennials. The newspaper is a public trust dedicated to the pursuit of truth and reason covering a range of issues from breaking news to politics, business, the markets, the arts, sports and community to the crossroads of people and society.

APPENDIX SEVEN

FG SHUNS PHARMACISTS, INAUGURATES PCN COUNCIL

June 18, 2020 1:23 am

Dr Osagie Ehanire
Minister of Health, Dr Osagie Ehanire

By
Friday Olokor, Abuja

Despite protest against his choice by the Pharmaceutical Society of Nigeria, the Federal Government has inaugurated Mr A. T. Mora as the chairman, Governing Council of the Pharmacist Council of Nigeria.

At the event, the government charged pharmacists to think of new ways of doing things as “a situation where the focus is on importation of finished products and compounding of raw materials is clearly not acceptable.”

A statement issued on Wednesday by the Director (Information, Media and Public Relations) in the Ministry of Health, Olujimi Oyetomi, said the inauguration would provide guidance and direction for the efficient discharge of the functions of the council as stipulated in its enabling Act.

According to him, the development of a national pharmaceutical plan for Nigeria, in collaboration with all major stakeholders, had commenced.

The Minister of Health, Dr Osagie Ehanire, who inaugurated the board reiterated that the PCN was vital to the health care system as reflected in its functions of regulating training of pharmacist, pharmacy technicians and the practice of the profession.

He said, “Key issues that should engage the attention of this council include standard of training, code and ethics of practice,

proliferation of illegal premises, unsatisfactory distribution of medicines and drug abuse.

“It is therefore paramount that the profession of pharmacy in Nigeria immediately starts to think of new ways of doing things, a situation where the focus is on importation of finished products and compounding of raw materials is clearly not acceptable.

Recall that the PSN had rejected Mora based on some allegations including unauthorised alteration of portions of the PCN Act, involvement in changing the names of PSN’s statutory nominations to the PCN Council in 2009.

Contact: theeditor@punchng.com

APPENDIX EIGHT

TEXT OF SPEECH

BY

PROF. AHMED TIJJANI MORA, mni, (Wakilin Maganin Zazzau)

***THE CHAIRMAN OF THE PHARMACIST'S COUNCIL OF
NIGERIA (PCN) PRESENTED TO MEMBER OF THE
GOVERNING COUNCIL OF THE PCN***

AT ITS INAUGURAL MEETING

ON

MONDAY, JUNE 15, 2020

***TEXT OF THE SPEECH BY PROF. AHMED TIJJANI MORA,
mni (WAKILIN MAGANIN ZAZZAU), THE CHAIRMAN OF
THE PHARMACIST'S COUNCIL OF NIGERIA (PCN)
PRESENTED TO MEMBERS OF THE GOVERNING COUNCIL
OF THE PCN AT ITS INAUGURAL MEETING ON MONDAY,
JUNE 15, 2020***

Protocol

My dear colleagues, members of the reconstituted Governing Council of the Pharmacists Council of Nigeria (PCN). I address you all as colleagues, because that is what we all are; members of the pharmacy profession assembled here as council recently constituted by His Excellency, President Muhammadu Buhari, GCFR, the Commander-in-Chief of the Armed Forces of the Federal Republic of Nigeria in line with LFN No 17 of 2004 establishing the PCN and just inaugurated by the Honourable Minister of Health (HMoH), Dr. Osagie Ehanire.

Let me begin by giving thanks to God almighty for witnessing this day, as we are now saddled with the responsibility of regulation and

control of pharmacy education and practice in Nigeria in all aspects and ramifications, among other functions as a Governing Council.

- (2) *Permit me to go down memory lane to 1999 when the Governing Council of the PCN was inaugurated by the then HMoH with Dr. Fred B. Adenika of blessed memory as the Chairman. I have a feeling of 'deja vu' as I am making this speech. That Council, which lasted its full course of three years till 2002, had myself as one of the eight (8) representatives of the Pharmaceutical Society of Nigeria (PSN) as member.*

That today, almost twenty (20) years on, I would be addressing a Governing Council of the PCN with myself as the Chairman, and comprising of no fewer than three (3) former Presidents of the Society as well as the current one as representatives on the Council is phenomenal and says so much on how the aspect of pharmacy regulation and control had progressed in Nigeria during these past years. With many other members of the reconstituted Governing Council here, we have certainly known one another in the practice of the profession or its regulation, with some dating as far back as during the days of the now defunct Pharmacists Board of Nigeria (PBN) (1964 to 1993) when I was the Chief Pharmacist and later the pioneer Director of Pharmaceutical Services in the headquarters of the Ministry of Health (MoH), Kaduna State. To others, I have had the honour and privilege of being a lecturer or fellow academic staff whether in the Faculties of Pharmaceutical Sciences in Ahmadu Bello University (ABU), Zaria, for 34 years or Kaduna State University (KASU) as foundation Dean for 4 years or indeed the Dora Akunyili College of Pharmacy, Igbinedion University, Okada (IUO), Okada, Edo State, where I am in my fifth (5th) year now as Professor.

Yet with others here, we have interacted when I was the Registrar of the PCN for two terms totalling eight (8) years. Thus, all in all, none of us is meeting the other for the first time, and because of this, I can only implore you to match your integrity and seasoned disposition with the expectation of this call to serve on the Governing Council of the PCN. I have no doubt in my mind that if we can work as a team, we should be able to attain great success.

- (3) *I wish to therefore acknowledge with gratitude the immense contributions in terms of the regulation and control of pharmacy education and practice by the past Governing Councils from 1992 to date under the Chairmanships of Pharm. (Mrs.) P. Oviasu (1992 to 1993); the late Dr. Fred B. Adenika (1999 to 2002); Prof. Festus Adio Ogunbona (2003 to 2006) and (2006 to 2007); Professor Elijah N. Sokomba (2009 to 2011) and Pharm. Bruno Nwankwo (2013 to 2015) as the case may be. On your behalf too, I once again thank His Excellency, President Muhammadu Buhari, GCFR for deeming it necessary to reconstitute the PCN Governing Council and giving his approval for our reconstitution as members based on the sound recommendations by the HMoH. We are most grateful.*

It was during the inauguration of the Governing Council of the PCN under the Chairmanship of Prof. Ogunbona, in August, 2003, when the then HMoH, Prof. Eyitayo Lambo emphasized on GROWTH, DEVELOPMENT and STABILITY as the agenda of the Federal Government which himself as the HMoH directed to be conceptualized in the functions and responsibilities of the PCN in order to be in tune with the policy thrust of the administration. There is no denying the fact that the PCN of today has indeed gone through many phases of rapid growth, sustained development and continued stability, many thanks to the various managements

of the PCN under the direct control of the various Registrars as well as the staff members of the Registry.

- (4) *As a Governing Council, we are expected to give policy direction and such other components of oversight functions to the Management of the PCN under the control of the Registrar. In actualizing these basic responsibilities, Committees would be established in tune with global best practices and ensuring that members of the three main 'blocks' on the Council, namely: the Directors of Pharmaceutical Services (DPSs), the Deans and the representatives of the Pharmaceutical Society of Nigeria (PSN) are appointed in commensurate proportions in such a way that there is presence of at least a member from the three bodies in Standing and/or Ad hoc Committees of the Governing Council as much as possible.*

The burden of regulating and control of pharmacy education and practice in Nigeria today, resting squarely on the Governing Council of the PCN, has never witnessed more challenges in the present and immediate past compared to what obtained now with COVID pandemic for the first time in the last 100 years. Similarly, the issue of the menace of drug abuse is highly topical and the voice and action of the Council must be heard by all Nigerians and decisive actions taken.

- (5) *There is the issue of review of the topics in the B.Pharm programme and the emerging Doctor of Pharmacy (Pharm.D) degree programme in the Faculties of Pharmacy in our Universities. The curriculum for the Pharmacy Technician (PT) programme mounted in the Schools of Health Technologies (SHTs) in the country should reflect the advancements in the teaching of the various pharmacy courses if the Pharmacy Technician graduate is to be of any*

meaningful help to the pharmacist in the practice of the profession.

The administrative governance within the Registry would further be looked into for the stakeholders to take full advantage of the decentralization process started years back and to enable maximum utilization of the Zonal and State offices structure as veritable units established to promote quality of operation by the management and staff of the PCN. I vividly recall that in 1991, the then PBN set up an Ad hoc Committee on Pharmacy Practice and Pharmacists Board of Nigeria's functions, a similar Ad hoc Committee would be set up.

Similarly, the standards in respect of practices in the four areas notably, pharmaceutical manufacturing, importation and wholesaling, retailing and hospital pharmacy must be seen to be in conformity with the elements entrenched in the book: 'The 4 Part Compendium of Minimum Standards for the Assurance of Pharmaceutical Care in Nigeria'.

As the very first publication on minimum standard of practice in any healthcare discipline in Nigeria, the PCN has indeed set a standard for itself which and other healthcare profession must not only be maintained but continuously improved upon. There is the need to back this with a Drug Safety and Efficacy Committee to assist Pharmacy in the hospital and in the community practice.

- (6) *As I conclude this post inauguration speech, let me seize this golden opportunity towards the process of initiation of healing of wounds perceived or indeed inflicted on persons or groups that are stakeholders in the pharmacy profession over the years.*

I was subjected to all manner of humiliation, persecution and bad publicity by past and present leadership of the PSN

including being dragged to the ICPC during the past ten years (2009 to date) which coincided with greater part of my second term as the Registrar and beyond. I was exonerated by the ICPC; however, I had to resort to the courts to redeem my name and image, and at a particular instance instituted libel case against the society and which I was prevailed upon by some leaders of the pharmacy profession to withdraw as way forward for reconciliation. I withdrew the case against the advice of my lawyer just to allow peace to reign in 2014 (six and a half years ago). I want to say here and now that despite the anguish, psychological trauma and unnecessary anxiety by the past and present leadership of the PSN these past years on myself, family members and many of my admirers among whom are my former and present students in ABU, KASU and IUO, Okada, professional colleagues, especially the thirty seven (37) Directors of Pharmaceutical Services in the States and the FCT that came out to fully support my appointment by His Excellency, President Muhammadu Buhari, GCFR, I bear no animosity or grudge to the Society or anyone. I hope as pharmacists, we should look towards strengthening the profession and not breaking it. In the end, it is the pharmacy profession, especially the regulation of the practice and education.

That we have just been inaugurated as a Governing Council established through LFN No 17 of 2004 to regulate and that should be paramount in our minds at all times.

I thank you all for listening and for your attention.

Prof. Ahmed Tijjani Mora PhD, FPCPharm, FPSN, FICEN, FIHMN, FNIM, mni

(Wakilin Maganin Zazzau),

The Chairman of the Pharmacists Council of Nigeria (PCN)

June 15, 2020

APPENDIX NINE (A,B,C) (A)



PHARMACEUTICAL SOCIETY OF NIGERIA
(FOUNDED 1927)

NATIONAL SECRETARIAT
32, Faramobi Ajike Street, Anthony Village, P.O. Box 531, Mushin, Lagos, Nigeria.
Tel: 01-4701069, 4978077, 5550267, E-mail: psn1927@yahoo.com
<http://www.psnigeria.org>

AS MEN OF HONOUR
WE JOIN HANDS

President
Pharm. (Ab.) Mohammed T. Bello, FPSN

1st Deputy President
Pharm. (Dr.) Ebube M. Arigbo-Osula, FPSN

2nd Deputy President
Pharm. Usman O. Bumbo

National Secretary
Pharm. Pius S. Abanum

Asst. National Secretary
Pharm. (Dr.) Oluwajoba O. Kudu

National Treasurer
Pharm. R. A. E. Mohammed

Nat. Financial Secretary
Pharm. Adedapo A. Ogunbadejo

Nat. Publicity Secretary
Pharm. Jerry Olorin

Editor-in-Chief
Pharm. (Dr.) Anthony Ogbochi

Internal Auditor
Pharm. Austine Ighuorude

Unofficial Members
Pharm. Oluwaseun U. Apokoma, FPSN
Pharm. Virginia L. Major, FPSN

Immediate Past President
Pharm. (Lady) Enne U. Ekaette, FPSN

8th August 2003

Pharm. Ahmed T. Mora FPSN
Ministry of Health, Kaduna
Kaduna State

Dear Pharm. Ahmed T. Mora

LETTER OF APPRECIATION.

This is to express the profound appreciation of the President, the NEC, the Council and the entire members of the Pharmaceutical Society of Nigeria (PSN) to you for your contribution to the Pharmacy profession as a member of the Governing Council of the Pharmacists Council of Nigeria (PCN) from November 1999 to November 2002.

The Society is particularly happy with your level of participation in the meetings and other activities of the Council. Be re-assured that your contributions have not gone unnoticed by the PSN.

We sincerely wish you more success in all your other endeavours and in the service of the Pharmacy profession.

Thank you and God bless.

Yours faithfully,

For: **Pharmaceutical Society of Nigeria**


Pharm. Pius S. Abanum
National Secretary.

PHARMACISTS WARN: HIV/AIDS IS REAL



(B)

**OFFICE OF THE PRESIDENT**
PHARMACEUTICAL SOCIETY OF NIGERIA
(FOUNDED 1927)
NATIONAL SECRETARIAT
192, Ikorodu Road, Palmgrove, P.O.Box 531, Mushin, Lagos, Nigeria.
Tel: 01-4701069, 827336, Fax: 01-827336. E-Mail: pen1927@yahoo.com

AS MEN OF HONOUR
WE JOIN HANDS

12th March, 2001

Alh. Ahmed T. Mora, FPSN
Zazzau Pharmaceuticals Ltd,
Zaria.

My dear Alhaji Mora,

LETTER OF APPRECIATION

I write to express my sincere appreciation for the wonderful dinner you organised in my honour, under the auspices of ABU Pharmacy Alumni Association, ABUPPA.

There is no gainsaying the fact that it is your singular effort, and magic touch that culminated in the wonderful gathering that we all witnessed.

I must confess that I was personally touched, and completely overwhelmed, and I thank you.

I pray that Allah continue to protect and guide you, and give you good health to sustain your athletic abilities. I pray also, that Allah present us with the necessary synergies to be able to ensure that your energy and wisdom are put into good use for the benefit of our nation, Ameen.

I remain.

Yours sincerely,
PHARMACEUTICAL SOCIETY OF NIGERIA


Pharm Mohammed Yaro Budah, FPSN, FPCPharm
President

PHARMACISTS WARN: HIV/AIDS IS REAL 

President
Pharm. (Alh.) Mohammed Y. Budah, FPSN

1st Deputy President
Pharm. (Dr.) Eyiade M. Arigbo-Osula, FPSN

2nd Deputy President
Pharm. Usman D. Sambo

National Secretary
Pharm. Azubike B. Okwor

Asst. National Secretary
Pharm. (Dr.) Olobayo O. Kuntle

National Treasurer
Pharm. (Lady) Chinedum N. Iloror

National Financial Secretary
Pharm. N.A.E. Mohammed

National Publicity Secretary
Pharm. (Hon.) Olinna J. Obichere

Editor-in-Chief
Pharm. Emmanuel N. Anyika

Internal Auditor
Pharm. (Hajja) Wadlat O. Oliva, FPSN

Unofficial Members
Pharm. Uchenna U. Apakama, FPSN
Pharm. Sebastian A. Bassey, FPSN

Immediate Past President
Pharm. (Lady) Ene U. Ekaette, FPSN

(C)



AS MEN OF HONOUR
WE JOIN HANDS

OFFICE OF THE PRESIDENT
PHARMACEUTICAL SOCIETY OF NIGERIA

(FOUNDED 1927)

NATIONAL SECRETARIAT

32, Faramobi Ajike St., Anthony Village, P.O.Box 531, Mushin, Lagos, Nigeria.
Tel: 01-4701069, Fax: 01-872335. E-mail: pan1927@yahoo.com

President
Alh. J. Mohammed Y. Budah, FRSC

3rd June, 2002

1st Deputy President
Dr. Egbade M. Arigbo-Osula, FRSC

Pharm. A.T. Mora, FPSN
P.O.Box.7426
Kaduna.

2nd Deputy President
Pharm. Usman D. Sambo

LETTER OF APPRECIATION

National Secretary
Pharm. Azubike B. Okwor

It is with great delight that I write on behalf of myself, the NEC and the National Council of PSN to express our profound appreciation to you for attending the FIRST SPECIAL PHARMACISTS SUMMIT, held in Kaduna on 29th to 30th May, 2002.

Asst. National Secretary
Pharm. (Dr.) Olobayo O. Kunle

Your presence and participation greatly enriched the proceedings at this landmark Summit.

National Treasurer
(Lady) Chinedum N. Ilazor

National Financial Secretary
Pharm. N.A.E. Mohammed

I implore you to propagate the spirit of peace and progress that pervaded the Summit, for the ultimate attainment of the spirit of the PSN motto - *AS MEN OF HONOUR, WE JOIN HANDS*.

National Publicity Secretary
(Hon.) Obinna J. Obichere

I pray that God continue to guide and protect us and shower Pharmacy and Pharmacists with prosperity and His blessings - Amen.

Editor-in-Chief
Emmanuel N. Anyika

Warm regards.

Internal Auditor
(Mr.) Wosilat O. Giwa, FPSN

Yours sincerely,
PHARMACEUTICAL SOCIETY OF NIGERIA

National Members
Pharm. U. Apakama, FPSN
Pharm. A. Bassey, FPSN

Pharm. M. Y. Budah FPSN, FPCPharm.
PRESIDENT

National Past President
(Hon.) Ernie U. Ekaette, FPSN

PHARMACISTS WARN: HIV/AIDS IS REAL



Appendix Ten

THE SANITARIAN.WORDPRESS.COM

***EVERY INFORMATION ABOUT ENVIRONMENTAL AND
PUBLIC HEALTH ISSUES IN NIGERIA, FOR MORE
INFORMATION, CONTACT THESANITARIAN@YAHOO.COM***

NEW PCN CHAIRMAN APPRECIATES PHARMACISTS

PROFESSOR AHMED TIJJANI MORA

It is with gratitude to the Almighty God that I write this letter of appreciation to all Nigerian Pharmacists for their congratulatory messages to me since the inauguration of the Governing Council of the Pharmacists Council of Nigeria (PCN) by His Excellency President Muhammadu Buhari, GCFR, ably Represented by the Honorable Minister of Health, Dr. Osagie Ehanire in the Federal Ministry of Health on Monday, 15th June, 2020. I also appreciate to over 250 Pharmacists who logged-in and watched the historic event via Zoom. I sincerely believe that all Pharmacists as well as other relevant stakeholders in Nigeria are still full of gratitude to the Honourable Minister for actualizing the approval and directives if His Excellency, the President and Commander In-Chief of the Armed Forces of the Federal Republic of Nigeria by the reconstitution and inauguration of the Governing Council since 2015. With a Governing Council now in place, the implementation of the routine activities in the Registry by the management under the leadership of the Registrar and CEO is expected to be much easier.

Professor Ahmed Tijjani Mora, Registrar Pharmacist Coucil Of Nigeria

The seventy two (72) of us as members of Governing Council would be working in harmony to ensure that the regulation and control of Pharmacy education and practice in Nigeria is implemented in tune with the LFN CAP 17 of 2004 establishing the PCN and in line with best global practices in similar Agencies worldwide. I have been associated with the members of three (3) main blocks of the Governing Council and would ensure that there is synergy among the entire membership in the best interest of the Pharmacy profession

and other allied stockholders that are equally regulated by the PCN. My expectations with my colleagues from the three (3) main blocks would be from the perspective of our past collaboration either in the regulation when I was a member of the defunct Pharmacists Board of Nigeria (PBN) and the present PCN, of in the academics or the practice the various specialties of the pharmacy profession. For the avoidance of doubt, I will briefly discuss each of the three (3) blocks as follows:

(1) The Directors of Pharmaceutical Services in the States Ministries of Health and the Federal Capital Territory (FCT):

This is the largest of the three groups made up of DPSs from the 36 States and the FCT and making up 51.4% of the membership of the Governing Council of the PCN.

I was Chief Pharmacist and later Director of Pharmaceutical Services in the Ministry of Health Headquarters in Kaduna from 1989 to 1993 and has served as a member of the now defunct PBN from 1989 to 1992 and among the first set of members of the Governing Council of the PCN initially established through Decree 91 of 1992 (now LFN NO 27 of 2004) from 1992 to 1993 when I ceased being the DPS having been appointed the Pioneer Project Manager at the Project Implementation Unit (PIU) of the AfDB/AjDF Assisted Multi-State Kaduna Health Rehabilitation Project.

I appreciate the fellowship and mutual respect that ensued between me and fellow DPSs. After my appointment as the Chairman of the Governing Council of the PCN became public in April of 2019, and in the heat of the tension that necessitated the deferment of the inauguration, the entire DSPSs supported my appointment as the Chairman of the PCN by His Excellency President Muhammadu Buhari GCFR. I was humbled by such unsolicited massive support without dissent by anyone. We shall certainly work together as we are the Ministries where I was as DPS as stated earlier.

(2) The Deans of Faculties of Pharmacy in Nigerian Universities Offering Courses in Pharmacy;

This is the second largest group among the three being made up of twenty two (22) Deans or 30.5% of the membership of Governing

Council. As an academic myself who had Lectured and still lecturing in Federal Government University Zaria (ABU, Zaria) (1986 to date) as part time Lecturer); State Government University (Kaduna State University (KASU, Kaduna) as foundation Dean of the faculty of Pharmaceutical Sciences and Pioneer Head of Department of Clinical Pharmacy and Pharmacy Management (2012 to 2016) and at the first Private University in Nigeria (Igbinedion University Okada (IUO), Okada, Edo state) (2016 to date) as the first professor in the Department of Clinical Pharmacy and Pharmacy Practice. To some of the Deans especially those that graduated from ABU, Zaria, I had the honour and privilege of being their lecturer, or fellow academic colleague and with IOU, Okada as Colleague.

(3) The Pharmaceutical Society of Nigeria (PSN);

This is the third of the three (3) groups made up of nine (9) members or 12.5% of the membership.

My involvement with PSN dates back to the my days in the Faculty of Pharmaceutical Sciences, ABU, Zaria when, as One of the PANS Exco (ABU Branch) attended the Golden Jubilee celebration of the PSN in the then newly opened Eko Hotel, Victoria Island, Lagos in November, 1977. That opened my eyes and served the society in the following capacities

Contested and won the post of Vice Chairman, PSN (Kaduna State Branch) in 1987; (1987-1989) (2 years);

Was returned as the Chairman of the PSN (Kaduna State Branch) Unopposed and in absentia in 1989 (1989-1990);

Contested and won the post of EX-Officio Member during the PSN Annual National Conference in Ibadan in 1993;

Contested won the post of Ex-Officio Member during the PSN Annual Conference in Port Harcourt in 1994;

Conferred with the Merit Award of the Kaduna state chapter of PSN in 2004;

Conferred with the fellowship of the Pharmaceutical Society of Nigeria in November 2007 at the Lagos Airport Hotel, Ikeja, Lagos by the then Chief of Defense Staff General Abdussalami Abubakar;

Served as the Chairman of the Conference Planning Committee (CPC) for 'Coal City 99' the Annual National Conference of the PSN in Enugu in 1999;

Recommended and appointed among the eight (8) PSN representatives in the reconstituted PCN Governing Council in December, 1999 and served for three (3) years till December, 2002;

Recommended and appointed among the eight (8) PSN representative in the reconstituted PCN Governing Council in August, 2003 and lasted for only six (6) weeks before taking up the employment as Registrar and CEO of the PCN from 1st October, 2003;

Served as the Chairman of the planning and Organizing Committee for the First Special Pharmacists Summit of the PFN in Kaduna in 2002.

It is therefore evident that none of us is meeting the other for the first time, and because of this, I can only implore you to match your integrity and seasoned disposition with the expectation of this call to serve the Governing Council of the PCN. I have no doubt in my mind that if we work as a team, we should be able to attain great success. We must perform and take the pharmacy profession and allied stakeholders that are also regulated and controlled by the PCN to higher levels. We have scheduled a post inaugural regular meeting of the Governing Council of the PCN on 9th July, 2020 in the PCN Registry and Head office, Idu, Abuja and hopefully by then the ban on interstate travels is lifted and all the members would see themselves face to face, otherwise the meeting would be virtual. We must hit the ground running as there is no time to waste.

It therefore follows that my contributions to PSN in particular and Pharmacy profession in general is legendary in spite of whatever any one or group says. I am not unaware of the myriad of problems and challenges facing Pharmacists in their various areas of practice which need to be addressed. Our colleagues in the Industry, Hospital and Community are merely surviving and not flourishing.

With the reconstitution of our Council, we must all take the opportunity to salvage our profession which is almost slipping away

from our hands. This is what should occupy the mind of every Pharmacist that has the interest of the profession in this heart.

The Registrar as the Chief Executive Officer and Registry forty four (44) State and Zonal offices of the Registry of the without Governing Council for five (5) years and are exited that at last His Excellency President Muhammadu Buhari, GCFR has reconstituted one since last year and inaugurated on 15th June, 2020 by the Honorable Minister.

Let us join hands and cooperate with one another, forget the past and work hard so that we can take Pharmacy profession to higher levels.

Finally, I once again thank you all Nigerian Pharmacists for your understanding, show of support as well your prayers. I hereby solicit for more of such prayers; cooperation and Support to enable us deliver on this National assignment.

Thank you all.

Yours Sincerely.

Prof. Ahmed Tijjani Mora, PhD, mni.

Chairman of the Governing Council of the Pharmacists Council of Nigeria (PCN).

Like us on Facebook.com/dailytrust
Follow us on Twitter @dailytrust



e-mail opinion@dailytrust.com

On appointments of Board Chairmen of Pharmacists Council

By Khalid Garba Mohammed

The negative treatment to most pharmacists appointed into leadership positions by the Pharmaceutical Society of Nigeria (PSN) is no longer news. What is new is taking this hatred so far as to challenge the authority of President Muhammadu Buhari in his constitutional right of appointing a Chairman of the Pharmacists Council of Nigeria (PCN) recently by the leadership of the PSN.

The PSN leadership, in a bizarre display of arrogance and contempt of public service reputation, is demanding that the newly appointed Board Chairman of PCN, Prof. Ahmed Tijani Mora, should apologise to it before any inauguration takes place. Unfortunately, the Federal Ministry of Health is paying attention to the Society when the latter has no say by the Law establishing the PCN (LPN No 17 of 2004) that any entry other than the President has a say on who is appointed the Chairman of the PCN.

It could be recalled that in December, 1999, President Olusegun Obasanjo, a southerner, appointed the late Dr. Fred Adenika from Ondo State as the Chairman of the PCN after many years without a Governing Council. No one complained. Dr. Adenika completed his three (3) year tenure in December, 2002. The same President Obasanjo appointed Prof. Festus Adigun from Ogun State as the Chairman of the PCN in August, 2003, and no one complained. Prof. Adigun completed his three (3) year tenure in August, 2006 and was re-appointed for a second and final tenure in August 2008 and no one complained. The Council was dissolved in November, 2007 by the then newly elected President, Alhaji Umaru Musa Yaradua, of blessed memory.

President Yaradua, a northerner, appointed Prof. Iliyah N. Sokomba, from Niger State, as the Chairman of the PCN in July 2009, thus making Prof. Sokomba the

first Northerner to ever head any of the past and present pharmacy regulatory agencies (1927 to 2009) as Chairman, and there were complaints by the southern-dominated PSN leadership. The PSN dragged the Hon. Minister of Health, the Hon. Minister of Justice and the PCN to court over this, and as expected, lost and the Society did not appeal. Prof. Sokomba, along with the then Registrar, a fellow Northerner, Prof. Ahmed Tijani Mora, were both subjected to all manners of humiliation and bad publicity by the PSN. Prof. Sokomba, who falls from Eruwa State, other leaders of the PSN and their agents support the Chairman and Registrar being pharmacists of Northern extraction.

President Goodluck Ebele Jonathan, a southerner, dissolved the Council in November, 2011, and in 2013, Prof. Bruno Nwankwo, from Anambra State, was appointed Chairman of the PCN, and no one complained.

President Muhammadu Buhari, dissolved the Council in June 2015 and in April 2019, appointed Prof. Ahmed Tijani Mora, from Kaduna State as the Chairman of the PCN, and the 22nd person to Chair past and present pharmacy regulatory agencies, as well as the second Northerner, and the PSN lashed and complained.

It would be recalled that Prof. Mora served as the Registrar and CEO of the PCN for eight (8) years from 2003 to 2012 and was, along with the first northerner to serve as the Chairman of the PCN, Prof. Sokomba, subjected to all manners of persecution by the southern-dominated leadership of the PSN from 2009 when the latter was appointed Chairman, to date, even after Prof. Mora had left the post of Registrar after serving his two terms.

The premise of the PSN's opposition to Prof. Sokomba's appointment as the new Chairman of the PCN Governing Council by President Muhammadu Buhari was that Prof. Mora had 'wronged' the PSN and

that he (Prof. Mora) must apologise to the PSN before the Society 'allows' the PMH to inaugurate the PCN Governing Council.

During the time Prof. Sokomba was appointed Chairman, it was a different reason, which was that both the Chairman, Prof. Sokomba, and the Registrar, Prof. Mora were both northerners. The PSN leadership failed to raise the fact that for sixty-six (66) years as of 2009 both the past Chairmen and Registrars of the various pharmacy regulatory agencies were from the Southern part of the country. The distribution of the Chairmen of the various past and present pharmacy regulatory agencies from 1927 to date (Board of Medical Examiners (1927-1936); Pharmacy Board of Nigeria (1936-1964); Pharmacists Board of Nigeria (1964-1982) and Pharmacists Council of Nigeria (PCN) (1982-date)) is as follows: British Colonial Administration (07); Southern Nigeria (13); Northern Nigeria (22) (Sokomba and Mora).

This is the position now and the PMH has not inaugurated the now 71-member PCN Governing Council despite presidential approval and directive. It is also imperative to note that, the Board has been dissolved since June, 2015. Why did the PSN leadership not complain and/or make effort to remind the government on the importance of the PCN Board which has been non-existing for nearly 4 years?

My take and my personal opinion: PSN should know that enough of these partisan and skewed objections on this appointment which affects the entire Nigerian pharmacists and the pharmacy profession in Nigeria as a whole. PSN should be a mother to all practicing pharmacists in Nigeria and not just to certain group of people. Moreover, PSN is not willing a good script for its the young generation pharmacists and the future custodians of the Society.

Khalid Garba Mohammed, B.Pharm, M.Sc (LRI), PhD Student, University of Milton, Italy

Re: NSA: Who the cap fits?

By Mohammed Bashir Ramal

In Nigeria, we are a people neck-deep in the act of politicising everything under the sun. So, I was amused when I recently read some politicised commentaries from an on-line analyst on Major General Salisu (Dil) Alayu (rtd), being part of the struggle for the NSA office.

Media houses are sometimes unconsciously conscripted into the arena of wrongful speculation, as they allude to Maj. Gen. SD Alayu to be among the army officers that were allegedly involved in the Dapagase when he was the Nigerian Defence Attaché to China.

For those who may not know, Major General Salisu (Dil) Alayu (rtd) was born on 23rd September, 1961 at Garkida. He is an indigene of Benue State and had his early education in Benue and Maiduguri, both in Benue state. A graduate of the Nigerian Defence Academy, Kaduna Polytechnic, Command and Staff College (Jag), University of Abuja, National Defence University, Beijing China and Tsinghua University, Beijing, China, he was commissioned as an officer into the Nigerian Army Corps of Engineers in 1984 and rose to the rank of Major General before retirement in 2016. A Soldier is always a hero because of his first decision to live for his country.

Indisputably, the life of a successful soldier is laborious, woven around these ethos and very demanding. General Alayu holds an MSc International Affairs and Strategic Studies, Masters Degree in Business Administration, Post Graduate Diploma in Security and Strategic Studies, Post Graduate Diploma in Management, Higher National Diploma in Electrical Engineering, Certificate in Security and Strategic Studies. He is Fellow of the National Defence University (UK) and a Member of the Nigerian Society of Engineers (MNSSE).

During his over 33 years career in the Nigerian Army, General Alayu was at different times the Chief Instructor at Military Engineering School, a UN Military Observer in Macedonia, Commanding Officer, Brigade Commander, Nigeria's Defence Attaché to China, with concurrent accreditation to Japan, Vietnam, Mongolia, North Korea, Iran and Bhutan. He was also at one time the Deputy Chief of Defence Intelligence at Defence Intelligence Agency.

General Alayu is a highly decorated senior military officer with a UN Preventive Deployment Medal, General Service Star (GSS), Distinguished Service Star (DSS), Meritorious Service Star (MSS), Peace Staff Course (PSC), Fellow National Military Attaché Medal from China Ministry of National Defense.

To render service in Nigeria or elsewhere is not something akin to a simple nursery rhyme pupa recite in class assembly. It requires faith and commitment; it needs energy, self-discipline, discipline, determination, unwavering and undiluted focus on goals and targets as well as vigorous pursuit of the targets.

General Alayu has never relied on his own and has demonstrated this time and time again in a combination of efficiency and deployment of several or all attributes to earn success and with

a positive image in public service is very necessary for a professional soldier, especially top officers who have risen to the status of leaders, offering services that would deposit laudable marks of success on specified assignment.

He has never concealed his contempt and disdain for the poor shape and status of the Army he made invaluable contribution to uplift, which was powered by professionalism, undisciplined, disloyal and traitorous personnel.

He thinks Nigeria as at times, He thinks the security, peace and unity of Nigeria, is immensely gifted with enough dosage of enviable leadership qualities. He is a patriotic, selfless, dutiful, hardworking, humane and committed soldier, who is imbued with faith of swift responsiveness to emergencies, anywhere, duty beckons. His patriotism is unequalled.

Quite interestingly, and contrary to the trajectory aiming portrait of the present in the social media or online publications that could still be fully information.

What I have found difficult to understand is it not dear to me what we stand to gain, when others tend to vilify.

Married with children and someone who enjoys swimming, intelligence & security research into military strategy, diplomacy and international affairs, SD Alayu cannot be compromised or influenced, no matter the amount in millions offered to him to believe. His devotion, tenacity and patriotism to the service of the fatherland are the inevitable secrets of his winning streak.

I am glad that Gen. Alayu has proved to some doubting Thomases that despite being retired he continues in professional engagements and the service of the country. I dedicate this glorious ode to Maj. Gen. SD Alayu who ceaselessly courts for fresh wars to bequest a respected Army to Nigeria.

The narrative of success, crown of victory, celebrations of valour in all special assignments of valour Alayu handles is signification of his effective utilisation of these working virtues.

Some Nigerians may not appreciate the burden of workload and excess strain on the shoulders of the.

I believe the narrative has changed for the good. And this is affirmed by the improvements we can see and feel on security around the country, despite the deliriously orchestrated drums of war. But whatever has been done to create neither appreciation it has not blurred the positive differences and winning streak of Maj. Gen. S. D. Alayu.

It is an incurable workaholic and a goal getter. His optimism in determination, hard work and commitment as recipes for success is unrivalling. He believes to make any impact, a man needs to stretch his energies and talents beyond the normal.

SD Alayu has proven to be a soldier of a man, thick in character, principle and performance.

It's easy to find in today's Nigeria, the duty of a public servant in his category, which is the environment and rich. It explains why he is a globally acknowledged in the arena of hard work and splendour.

Dr. Col. Mohammed Bashir Ramal is a retired military officer

Prof Mora's 100th Day In Office At PCN

BY YOOSU TERSOO JONATHAN



The PCN Chairman has also revealed his intention to computerize drug distribution in Nigeria which was made known by a Daily Thrust Newspaper publication dated 21st June 2020.

September 22, 2020 Marked the 100th day in office of Prof Ahmed Tijjani Mora as Chairman, Governing Council of Pharmacists Council of Nigeria (PCN), after a historic inauguration by the Minister of Health, Dr Osagie Ehanire.

The appointment of the Wakili Maganin Zazzau rekindled hope and high expectations across the diverse segments making up the pharmaceutical sector in the country. The reason is not farfetched considering his wealth of experience in all the four areas of pharmacy practice since licensed to practice pharmacy in Nigeria.

The most notable as a regulator was when appointed as registrar of PCN where he served for two terms and oversaw the restructuring of the agency and opening of offices in the 36 States of the federation, bringing regulation closer to practice and

improving the quality of pharmaceutical services across the country.

This background distinguished Prof A T Mora FPSN, mni for the revered job of Chairman PCN Governing Board and enabled him stand on a professional tetrapod giving him a clearer bird view of the myriad of challenges facing each facet of the pharmaceutical sector and the much needed solutions.

In spite the lockdown occasioned due to the Covid-19 pandemic, his 100days in office has been punctuated with a series of progressive events, but to mention a few!!

Prof AT Mora has successfully convened the 1st Council Meeting of the Reconstituted PCN Governing Board which had an overwhelming attendance of 91%.

Various Committees and Ad hoc Committees have been constituted and inaugurated with responsibilities and targets allocated to drive the policies of the Council.

Prof AT Mora FPSN, mni have brokered and led high profile courtesy visit to the Secretary to the Government of the Federation, The Head of Service of the Federation, the Inspector General of Police, a critical partner of PCN etc These are geared towards collaboration to reposition the agency to actualize her mandate.

The PCN Chairman has also revealed his intension to computerize drug distribution in Nigeria which was made known by a Daily Thrust Newspaper publication dated 21st June 2020.

As a member of the Presidential Committee on Pharmaceutical Sector Reform formed in 2003 by the Olusegun Obasanjo administration, the assiduous work of this committee midwived the National Drug Distribution Guidelines which was unveiled in 2012 by the Honourable Minister of Health, Prof Onyebuchi Chukwu, which is poised to solve the pandemic of chaotic

drug distribution in Nigeria if implemented.

The academic in his debuted book titled The Lizard Shape Model in Drug Distribution in Nigeria, a must read for all stakeholders in this sector, Prof AT Mora FPSN mni outlined the challenges bedeviling this sector and the way forward.

Therefore, it was of no surprise to stakeholders and professional colleagues when his intension to computerize drug distribution in Nigeria was made known which received wide commendation by stakeholders at all levels.

Conclusively, concerted collaboration amongst regulatory and security agencies, stakeholders and professional colleagues is the panacea for repositioning Pharmacy Practice in Nigeria to meet global best practices.

— Jonathan, a Pharmacist, wrote in from Abuja

The Leadership Newspaper, a daily paper of repute in Nigeria on October 26, 2009 ran an editorial

LEADERSHIP Monday, OCTOBER 26, 2009

3

OUR STAND

Mora's Stewardship At The PCN

The importance of health regulatory bodies in advancing health care delivery services in Nigeria informed the establishment of the Pharmacists Council of Nigeria (PCN), a Federal Government organ created through Act 91 of 1992 and saddled with the responsibilities of regulating and controlling the practice of pharmacy in all its ramifications. The PCN's functions include the registration, inspection, approval, licensing as well as monitoring of the premises where pharmaceutical activities take place.

The PCN, therefore, has the onerous duty of ensuring that only persons with the appropriate professional competencies are presented as practitioners to the Nigerian public to provide pharmaceutical care in premises that have been registered, inspected, approved and licensed and are being effectively monitored by the PCN.

Since coming into office in 2003, Pharm. Ahmed T. Mora, the registrar of the PCN, and his executive have embarked on repositioning the PCN, aimed at ensuring better regulation

and control of the practice and education of pharmacy in Nigeria. Under him the PCN initiated a decentralisation process to ensure that PCN's regulatory and control mechanism for the practice of pharmacy in Nigeria are taken to the rural areas. This entailed, apart from the PCN's head office and registry in Abuja, and three other zonal offices in Lagos, Enugu and Kano, the creation of three new zonal offices in Uyo, Minna and Bauchi making a total of six PCN zonal offices, thereby making sure that each geopolitical zone of the country has one. Area offices have also been created at Benin City, Owerri, Ilorin, Ibadan, Kaduna and Maiduguri bringing the total to twelve area offices of the PCN and by the year 2008, 35 state offices of the PCN have been established throughout the country.

The growth of the PCN from an organisation with four offices in 2003 to one with 44 in 2009 is phenomenal. Similarly, it also grew its staff strength of 99 to 260 during the same period, an indication that the infrastructural expansion is appropriately matched with creation of employment opportunity for those trained in pharmacy to effectively carry out its functions.

As the PCN commissions four of its brand new prototype zonal office buildings at Minna for the North-central, Bauchi for the North-east, Ibadan for the South-west and Uyo for the South-south this week, it is only proper that members of the public are aware of the process put in place and meant to take care of their pharmaceutical needs and education. In essence, therefore, retail pharmacies, wholesale and importation firms for drugs, hospital pharmacies, pharmaceutical manufacturing companies and indeed patent and proprietary medicine vendor stores which most Nigerians patronize for their basic pharmaceutical needs would be subjected to frequent inspections due to the presence of PCN offices at the states.

Surely, this would mean access to better and widely spread registered, approved, licensed and frequently monitored drug outlets capable of safeguarding the health of the Nigerian people. Similarly, students of pharmacy undergoing their internship trainings would be subjected to close supervision and this is made possible through the created zonal offices across the country.

ABOUT THE AUTHORS



PHARM. ALIYU JIBRIN, *FPSN, FPCPharm, MNIM*

Managing Director

Namama Pharmacy Limited, Kaduna

Pharm. Aliyu Jibrin was born on 22 October 1962 in Jushin Waje, Sabon Gari, Zaria. He attended L.E.A. Primary School, Sabon Gari, Zaria; Sardauna Memorial College, Kaduna; and the School of Basic Studies, Ahmadu Bello University (ABU), Zaria. He later proceeded to the Faculty of Pharmaceutical Sciences, ABU, Zaria, where he obtained his Bachelor of Pharmacy (B.Pharm.) degree in 1986.

He is a seasoned pharmacist with several decades of experience in community pharmacy practice. After completing the National Youth Service Corps (NYSC), he worked briefly as a hospital pharmacist at Ahmadu Bello University Teaching Hospital, Zaria, and later at

Barau Dikko Hospital, Kaduna. In 1990, he joined Osbud-K Pharmacy Limited, where he served meritoriously and rose to the position of Executive Director before resigning in 2017 to establish Namama Pharmacy Limited, which he currently heads as Managing Director.

Pharm. Jibrin attended the West African Postgraduate College of Pharmacists (WAPCP) and was admitted as a Fellow of the College (FPCPharm) in 2005. He has rendered distinguished service to the pharmacy profession through various platforms, notably the Association of Community Pharmacists of Nigeria (ACPN) and the Pharmaceutical Society of Nigeria (PSN).

He is a professional manager and active member of the Nigerian Institute of Management (NIM), Kaduna Chapter, where he once served as Chairman. His leadership roles in the profession include:

- Former Chairman, ACPN Kaduna State Branch
- Former Chairman, PSN Kaduna State
- Former National Vice Chairman, ACPN
- Former National Financial Secretary, PSN
- Former Vice Chairman, Board of Fellows, PSN

He is a Fellow of the Pharmaceutical Society of Nigeria (*FPSN*) and a Fellow of the West African Postgraduate College of Pharmacists (*FPCPharm*). He has received numerous merit awards from PSN (national and state levels), ACPN (national and state levels), and the Nigerian Institute of Management.

Pharm. Aliyu Jibrin is married with children. His hobbies include reading and travelling.



PROFESSOR IBRAHIM ADAMU YAKASAI
Dean, Faculty of Pharmaceutical Sciences
Bayero University, Kano (BUK)

Personal Information

- Name: Professor Ibrahim Adamu Yakasai
- Nationality: Nigerian
- Profession: Pharmacist / Pharmaceutical Chemist
- Current Position: Dean, Faculty of Pharmaceutical Sciences, Bayero University, Kano
- Contact: +234 806 549 9007; +234 802 714 7615
- Email: yakasaiibrahimadamu8@gmail.com

Academic Qualifications

- PhD (Pharmaceutical Chemistry) – Ahmadu Bello University, Zaria, 1995

- MSc (Pharmaceutical Chemistry) – Ahmadu Bello University, Zaria, 1991
- B.Pharm. – Ahmadu Bello University, Zaria, June 1986

Professional Experience

- Dean, Faculty of Pharmaceutical Sciences, Bayero University, Kano – 2024 to date
- Professor of Pharmaceutical Chemistry, Bayero University, Kano – 2012 to date
- Head, Department of Pharmaceutical and Medicinal Chemistry, BUK – 2016–2024
- Deputy Director (Innovation), Directorate of Research, Innovation and Partnership, BUK – 2020–2024
- Member of various university and national committees on pharmacy education, research, regulation, and quality assurance

Membership of Professional Bodies

- Pharmaceutical Society of Nigeria (PSN)
- Pharmacists Council of Nigeria (PCN)
- West African Postgraduate College of Pharmacists (WAPCP)
- Member/Collaborator, PVAC and other relevant professional bodies

Areas of Research Interest

- Drug analysis and quality control
- Pharmaceutical chemistry and drug development
- Regulation and standardisation of medical consumables
- Selected Publications and Contributions
- Author and co-author of over forty (40) peer-reviewed journal articles in pharmaceutical chemistry and drug quality control

- Author of Analysis and Production of Packaged Water
- Co-author of D. S. Wali: An Icon of Community Pharmacy Practice in Nigeria
- Supervisor of numerous undergraduate and postgraduate research projects

Awards and Honours

- Recipient of several academic and professional awards for excellence in teaching, research, and service to the pharmacy profession.

Hobbies

- Reading, football, travelling, and engaging in academic discourse on politics and educational development.



PROFESSOR UMAR USMAN PATEH, *B.Pharm, MSc, PhD, MPSN, MAW*

Professor Umar Usman Pateh is a distinguished academic and pharmacist from Misau, Bauchi State. His academic training was obtained at Ahmadu Bello University (ABU), Zaria, where he earned a Bachelor of Pharmacy (B.Pharm.) in 1993, an MSc in 2000, and a PhD in Pharmaceutical Chemistry in 2006.

He began his professional career with an internship at the Ahmadu Bello University Teaching Hospital, followed by National Youth Service at St. Louis General Hospital, Owo. He subsequently gained practical experience in community pharmacy with Wonu Cosmopharm Limited, Bauchi, before establishing Kiram Pharmacy Limited in 1996.

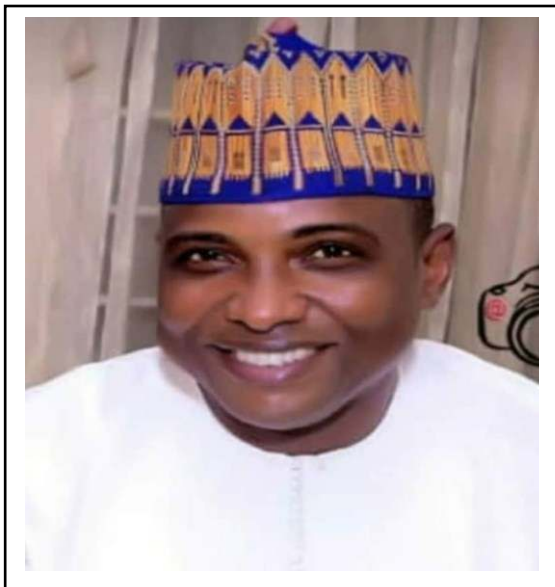
Driven by a strong passion for teaching and research, Professor Pateh joined the academic staff of Ahmadu Bello University, Zaria, in 1997, marking the beginning of a highly accomplished

academic career. He rose through the ranks and was promoted to Professor of Pharmaceutical Chemistry in 2012.

Professor Pateh has held several strategic academic leadership positions, including Head, Department of Pharmaceutical and Medicinal Chemistry (2009–2013) and Dean, Faculty of Pharmaceutical Sciences (2013–2017). He has served extensively on the ABU Senate, participated in professional regulatory activities with the Pharmacists Council of Nigeria (PCN), and served as Managing Editor of the Nigerian Journal of Pharmaceutical Sciences. He also contributed to professional development as Chairman of the Pharmaceutical Society of Nigeria (PSN), Bauchi State Branch, in 2008.

A prolific scholar and mentor, Professor Pateh has taught numerous undergraduate and postgraduate courses, supervised many postgraduate students, authored over fifty (50) peer-reviewed journal articles, presented more than twenty (20) conference abstracts, and co-authored three (3) academic books. His outstanding contributions have earned him several recognitions, including the Pharmaceutical Society of Nigeria Merit Award (2016) and the Ahmadu Bello University Grant Fellow of the Year Award (2025).

Outside academia, Professor Pateh enjoys cycling and reading. He is a devoted family man, blessed with children.



AHMED IBRAHIM UMAR, *B.Pharm, FPCPharm*

Deputy Director & Head of Training

Drugs Manufacturing Unit,

Department of Pharmaceutical Services,

Aminu Kano Teaching Hospital (AKTH), Kano, Nigeria,

Email: [ahmedibrahimumar1971@gmail.com]

(mailto:ahmedibrahimumar1971@gmail.com)

Phone: +234 803 838 698

PROFESSIONAL SUMMARY

Ahmed Ibrahim Umar is a highly experienced, licensed pharmacist with over twenty-five (25) years, of progressive professional experience spanning clinical pharmacy, pharmacotherapy, pharmaceutical manufacturing, regulatory compliance, and healthcare administration. He is widely recognised for his leadership in drug manufacturing, extemporaneous compounding, capacity

building, and public–private partnership (PPP) initiatives, within the Nigerian health sector.

EDUCATION AND PROFESSIONAL QUALIFICATIONS

- Fellow Drug Production and Quality Assurance (*FPCPharm*), West African Postgraduate College of Pharmacists
- Bachelor of Pharmacy (B.Pharm.), – University of Jos, Nigeria

PROFESSIONAL EXPERIENCE

- Deputy Director & Head of Training,
- Drugs Manufacturing Unit, Aminu Kano Teaching Hospital (AKTH), Kano, (2018 – Present)
- Leads training and capacity development programmes in pharmaceutical manufacturing
- Oversees research and development of oral liquid dosage forms, antiseptics, and disinfectants
- Ensures compliance with Good Manufacturing Practice (GMP), quality assurance standards, and regulatory requirements
- Assistant Director Drugs Manufacturing Unit, AKTH, (2014 – 2018)
- Supervised pharmaceutical manufacturing operations
- Championed the expansion of public, private partnership (PPP), initiatives
- Chief Pharmacist & Head of Department, Abubakar Tafawa Balewa University Teaching Hospital (ATBUTH), Bauchi, (2012 – 2014)
- Assistant Chief Pharmacist, PEPFAR Programme, AKTH, (2008 – 2012)
- Led antiretroviral therapy services and patient counselling activities

LEADERSHIP AND NATIONAL SERVICE

- Former Chairman, Pharmaceutical Society of Nigeria (PSN), Kano State
- Former, National Financial Secretary, Pharmaceutical Society of Nigeria (PSN)
- Former Pharmaceutical Inspector, Pharmacists Council of Nigeria (PCN)
- Member, PCN Accreditation and Examination Committee,

CERTIFICATIONS AND TRAINING

- * Digital Marketing, Google EMEA,
- * Health Sector Reform
- * Antiretroviral Treatment, Howard University, USA,
- * Administrative Management, Administrative Staff College of Nigeria (ASCON),

KEY SKILLS

- Pharmaceutical Manufacturing and Compounding
- Pharmacotherapy and Clinical Pharmacy
- Quality Assurance and Regulatory Compliance
- Healthcare Administration
- Public–Private Partnership Development
- Training and Mentorship

AWARDS AND HONOURS

- Distinguished Merit Award, PSN Bauchi State – 2010
- Exemplary Leadership Award, PSN Kano State – 2017



PROF. KABIRU ABUBAKAR GULMA, *PhD*

Kabiru Abubakar Gulma is a Professor of Global Health and Health Systems Optimisation at Euclid University and a Senior Associate with the Clinton Health Access Initiative (CHAI), Nigeria. He holds a PhD in International Public Health and has led multiple high-impact initiatives in health systems strengthening, supply-chain optimisation, and primary healthcare financing across Nigeria and West Africa.

Prof. Gulma has pioneered data-driven innovations aimed at improving commodity availability, transparency, and accountability within primary healthcare systems. His work has focused on performance-based financing models, supply-chain diagnostics, and system-level reforms designed to enhance efficiency and sustainability in healthcare delivery.

A prolific academic and committed mentor, Prof. Gulma has published over thirty (30) peer-reviewed articles in reputable international journals and serves on the editorial boards of several

public health journals. He is also the developer of McGulma's Pharmaceutical Dictionary, Nigeria's first indigenous pharmaceutical dictionary, created to promote pharmaceutical literacy and improve access to accurate drug information.

Through his research, teaching, and programme implementation, Prof. Kabiru Abubakar Gulma continues to make significant contributions toward advancing equity, efficiency, and sustainability in health systems across Africa.



IBRAHIM MOHAMMED AMIDU, *B.Pharm, MSc, GPPC, FPCPharm*

Ibrahim Mohammed Amidu is a consultant pharmacist and seasoned pharmacovigilance expert, as well as a Certified Global Pharmacovigilance Professional, with over a decade of experience in medicines regulation, policy development, and health-system strengthening in Nigeria. His career spans regulatory institutions, national programmes, industry, and digital health initiatives, where he has contributed significantly to improving the quality, safety, and governance of pharmaceutical services.

He currently serves as Head of Pharmacy at the Defence Headquarters Medical Centre, where he provides strategic leadership for pharmaceutical services within a high-responsibility military healthcare environment.

A graduate of Ahmadu Bello University (ABU), Zaria, Ibrahim served as President of the Pharmaceutical Association of Nigerian Students (PANS). He is the first Nigerian pharmacist selected for the

Kiani Global Interprofessional Patient Safety Fellowship and is a Fellow of Digital Health Skills and Innovation in Africa. He has undertaken extensive professional training through internationally recognised programmes, including specialist courses supported by the World Health Organisation (WHO) and advanced training by the Uppsala Monitoring Centre.

Ibrahim has contributed to several landmark national initiatives in pharmacy regulation and policy. He supported the development of Nigeria's first National Electronic Pharmacy Policy, officially launched on 22 December 2025 by the Coordinating Minister of Health and Social Welfare. He also assisted the Pharmacy Council of Nigeria (PCN) in developing operational guidelines for electronic pharmacies and supported the National Agency for Food and Drug Administration and Control (NAFDAC) in drafting key national guidance documents. Additionally, he has participated in inter-ministerial and technical working groups on medicines regulation and digital health governance.

He is an active member of the International Society of Pharmacovigilance (ISoP), including its African Chapter, and serves on the Coordinating Committee of the ISoP Student Community, contributing to professional development and mentorship initiatives.

Ibrahim Mohammed Amidu's career reflects a strong commitment to public service, institutional development, and the advancement of pharmacy practice in Nigeria. His professional values align closely with those exemplified by Professor Ahmed Tijjani Mora—namely, professional integrity, regulatory excellence, and dedication to strengthening systems that safeguard public health.

Bibliography

1. *A Compilation of Pharmacy, Drugs and Related Laws and Rules in Nigeria, 1935–2000. 2nd ed., 2009. Pharmacists Council of Nigeria. ISBN: 978-2397-96-2.*
2. *Acts for the Establishment of all Thirteen Health Regulatory Agencies (Nigeria).*
3. *Adenika, F. (1998). Pharmacy in Nigeria. Panpharm Ltd., Ikeja.*
4. *Akubue, P. I., & Adenika, F. B. (Eds.). (2009). Pharmacy Education in Nigeria: Proceedings of the Pharmacy Curriculum Conference (Aug. 28–30, 2001). Pharmacists Council of Nigeria. ISBN: 978-2397-65-2.*
5. *Brown, A. A. (1999). Assuring Drug Quality through Pharmaceutical Inspection. Gold Print Press Ltd., Lagos.*
6. *Brown, A. A. (1999). Who is a Pharmaceutical Inspector? PCN Workshop (Unpublished).*
7. *Code of Ethics for Pharmacists in Nigeria. 2nd ed., 2009. Pharmacists Council of Nigeria. ISBN: 978-2397-33-4.*
8. *Committee of Registrars of Health Professions (CRHP). (2008). The Role of Health Regulatory Bodies in Advancing Healthcare Delivery Services in Nigeria. Proceedings of a One-Day Advocacy Workshop, Abuja/Ibadan.*
9. *Committee of Registrars of Health Professions (CRHP). (2011). Harmony in the Healthcare Delivery System: The Role of Regulatory Agencies. Proceedings of a One-Day Advocacy Workshop, Abuja.*
10. *Conditions of Service for Employees of the Pharmacists Council of Nigeria. 2nd ed., 2009. ISBN: 978-2397-64-4.*

11. *Federal Ministry of Health. (2005). National Drug Policy.*
12. *Federal Republic of Nigeria. (2004). National Agency for Food and Drug Administration and Control Act, Cap. NI, LFN 2004.*
13. *Federal Republic of Nigeria. (2004). Pharmacists Council of Nigeria Act, Cap. P17, LFN 2004.*
14. *Federal Republic of Nigeria. (2005). Inspection, Location and Structure of Pharmaceutical Premises Regulations. Official Gazette No. 79, Vol. 92.*
15. *Federal Republic of Nigeria. (2005). Registration of Pharmaceutical Premises Regulations. Official Gazette No. 81, Vol. 92.*
16. *Federal Republic of Nigeria. (2007). National Human Resources for Health Strategic Plan (2008–2012).*
17. *Good Pharmacy Practice Standards in South Africa (GPPSA). (2010). South African Pharmacy Council.*
18. *Hornby, A. S. (2006). Oxford Advanced Learner's Dictionary of Current English (7th ed.). Oxford University Press.*
19. *Ibrahim, A. A. (2016). Address on Minimum Standards Required in Tertiary Healthcare Facilities for Effective Service Delivery. Kaduna (Unpublished).*
20. *International Journal of Healthcare Management. (2015). Vol. 1, No. 1, pp. 36–57.*
21. *Lambo, E. (Prof.). (2004). Address at the Inauguration of the Committee of Registrars of Health Professions (CRHP), August 19, Abuja.*

22. *Longman Active Study Dictionary. (2004). Pearson Education Ltd.*
23. *Mora, A. T. (2007). Laws and Regulations Governing Pharmacy Practice in Nigeria and Reciprocation. NAPPSA Convention, Houston, USA (Unpublished).*
24. *Mora, A. T. (2008). The Nigerian Pharmacist, Medication Errors and Pharmacy Regulation. NAPPSA Convention, Atlanta, USA (Unpublished).*
25. *Mora, A. T. (2010). Pharmaceutical Regulation and Distribution in Nigeria: Challenges and Prospects. PCN Publications.*
26. *National Health Summit. (1995). Proceedings of the Maiden National Health Summit. Federal Ministry of Health, Abuja.*
27. *PCN Lecture Notes for the Orientation Programme of Patent Medicine Vendors in Nigeria. 2nd ed., 2009. ISBN: 978-2397-73-3.*
28. *PCN's Approved Patent Medicine List. 4th ed., 2009. ISBN: 978-2397-72-5.*
29. *PCN's Continuing Education Programme (CEP) for Patent and Proprietary Medicines Vendors. 2nd ed., 2009.*
30. *PCN's Guidelines for the Establishment of the Bachelor of Pharmacy Programme in Nigerian Institutions. 3rd ed., 2009. ISBN: 978-2397-81-4.*
31. *PCN's Guidelines for the Issuance of Patent and Proprietary Medicines Vendor's Licence. 2nd ed., 2009. ISBN: 978-2397-25-3.*
32. *PCN's Internship Logbook for Fresh Pharmacy Graduates in Nigeria. 2nd reprint, 2009. ISBN: 978-2397-18-0.*

33. *PCN's Internship Training Programme for Pharmacy Graduates in Nigeria. 2nd ed., 2009. ISBN: 978-2397-97-0.*
34. *PCN's Mandatory Continuing Professional Development (MCPD) Programme for Pharmacists in Nigeria. 3rd ed., 2009. ISBN: 978-2397-42-3.*
35. *PCN's Manual for Foreign Pharmacy Graduate Orientation Programme (FPGOP). 2nd ed., 2009. ISBN: 978-2397-89-X.*
36. *PCN's Manual for Pharmaceutical Inspectors in Nigeria. 2nd ed., 2009. ISBN: 978-2397-41-5.*
37. *PCN's Training Manual for Pharmacy Technicians in Nigeria. 3rd ed., 2009. ISBN: 978-2397-26-1.*
38. *PCN.EdT.010. (2009). Proceedings of Nationwide Advocacy Visits and Interactive Sessions with Hospital Pharmacists. Pharmacists Council of Nigeria.*
39. *PCN.ReC.006. (2009). Meeting the Leadership Challenge of a Repositioned PCN: The Council's 2003–2007 Scorecard.*
40. *PCN.ReC.007. (2009). Administrative and Organizational Restructuring for an Effective PCN Registry (2003–2008).*
41. *Rosenberg, J. M. (2004). The Essential Dictionary of Management and Human Resources. Barnes & Noble.*
42. *Tayo, F. (2007). Management and Control of Pharmaceutical Products Distribution in Nigeria. Fred Adenika Memorial Lecture (Unpublished).*
43. *The Establishment of the Pharmacists Council of Nigeria Decree No. 91 of 1992. Reprinted 2009. ISBN: 708-2397-56-3.*

44. *The Four-Part Compendium of Minimum Standards for the Assurance of Pharmaceutical Care in Nigeria. 2nd ed., 2009. ISBN: 978-2397-66-0.*
45. *The PCN Sourcebook on Pharmacy Regulation in Nigeria. 2nd ed., 2009. ISBN: 978-2397-49-0.*
46. *Weir, D. (2011). Cultural Diversity and Change Management in Public Services. Liverpool Hope University.*
47. *World Health Organization. (2007). Quality Assurance of Pharmaceuticals: A Compendium of Guidelines and Related Materials (Vol. 2). Geneva.*

INDEX

1

1957 February.....	18
1963 Classmates	24
1970	
class group.....	40
class of.....	109, 220, 276, 277
january 17.....	25, 274
1974	
june.....	25
1983 October.....	27
1986	
january	32
1988. xviii, xxxiii, 12, 33, 66, 283, 295,	
394, 506, 508	
1989	
february 19	82
1992	
december 5	37
1995	
january 22.....	534

2

2001 Kuru Declaration.....	343
2003	
august.....	84, 86, 101
september 23	86
september 25	115
2004	
february 20	101
2009	
october 26	553
2010	
november 15.....	521, 522, 528, 529
2010 February.....	27
2011	
december 21	530
2011 Annual Scientific Conference	
.....	xx, 388, 389
2012	
february 10	515
january	56, 234, 236

2013

september 27	111, 112
--------------------	----------

2019

august 9	551
----------------	-----

2020

june 15 xxiv, 58, 159, 235, 537, 542,	
546, 550	
june 18.....	535
august 18	236
september 24	552

2020 February	48
---------------------	----

2023

june 19.....	173
--------------	-----

2024

august 19 ..	126, 368, 410, 439, 457
--------------	-------------------------

2025

september 20	40
--------------------	----

7

70th Birthday xvii, 182, 183, 186, 188,	
196, 198, 206, 207, 217, 219, 220,	
223, 226, 229, 254, 255, 256, 257,	
262, 266, 268, 269, 270, 283, 289,	
290, 291, 292	

A

Abba Dasuki (Pharm.)	112
Abba Habib (Alh.)	21
Abba Kyari (Brig.)	21
Abbass, Isa (Prof.)	37
Abdallah, Rabi (Hajiya)	xviii, 287
Abdul-Alimi Bello (Dr.)	xv, 37, 202
Abdulazeez Sani Rafindadi (Pharm.)	
.....	86, 90, 101
Abdulbasir Abubakar Jargab'a (Alh.)	
.....	xv, 184
Abdulkadir Ajuji	24
Abdullahi (Alh.).....	21
Abdullahi (Shehu)	xxix
Abdullahi A. Sule (Engr. Nasarawa	
State Gov.)	49
Abdullahi Bin Fodio (Sheikh).....	184

Abdullahi Gwandu	184
Abdullahi Lawal Iro (Alh.)	10
Abdullahi Mahadi (Prof.)	110
Abdullahi Makarfi (Alh.)	21
Abdullahi Muhammad Bashir (Prof.)	27
Abdullahi Mustapha (Prof.)	29, 107
Abdullahi Tafida (Dr.).....	505
Abdullahi Zubairu Abdullahi (Barr.)	29
Abdulumumini Hassan Rafindadi (Prof.)	20, 39, 276, 278, 503, 504
Abdulumumini Ibrahim, <i>dsc, mni</i> (Prof.)	xvii, 280
Abdulyakini Daromosu (Alh.)	27
Abdurahman, E. M. (Prof.)	58, 103
Abdurrahman Mahmood (Alh.).....	39
Abdurrahman Mahmud	72
Abdurrahman Mora College of Islamic Studies	17
Abdurrahman Mora Educational and Vocational Centre	17
Abdurrahman Mora Lgea Primary School.....	17
Abdurrahman Okene (Alh.)	21
Abdurrahman, E. M. (Prof.)	29, 67
Abdurrahman, Mora (His Excellency Amb. Dr.) ...	5-10, 14-17, 19, 25, 37, 41, 65, 73-74, 199, 215-216, 263, 278, 283, 501-502
Abdussalam Baba Ahmed (Alh.)	5, 10
ABU.....	45
chancellor of	46, 79
ABU Alumni Association ...	42-43, 45- 46, 55-56, 62, 79, 106-107, 157, 177-179, 198, 206, 208, 238-239, 250
Abu Musawa (Alh.)	21
ABU Pharmacy Alumni Association	42, 107, 250, 272, 544
ABU Pharmacy Alumni Association (ABUPAA)	511
Abubakar (Mallam).....	24
Abubakar Alhaji Kbe (Sir, Alh.).....	xviii
Abubakar Imam (Late Alh. Dr.)	20, 22, 73, 175
Abubakar Imam, Binta Atti (Hajiya)	18, 22, 112, 199, 215
Abubakar Kaiama (Alh.).....	21
Abubakar Ladan (Alh.)	34
Abubakar Mahmood Gummi (Sheikh)	20
Abubakar Tafawa Balewa University (ATBU)	46, 562
Abubakar Tunau Mafara, <i>mfr</i> (Alh.)	20, 76, 157, 201
Abubakar Umar Abdullahi	495
Abubakar, M. S. (Prof.)	29
ABUPAA	See Ahmadu Bello University Pharmacy Alumni Association, See ABU Pharmacy Alumni Association
ABUTH 9, 27, 32, 42, 446, 510, 512, See Ahmadu Bello University Teaching Hospital CMD of	39
Academia and Public Service Trace	226
Academic and Medical Career	278
Academic Leadership	196, 560
Access to Quality Healthcare Services	420
Accident	247, 265, 283
Accountability	161, 163, 164, 226, 310, 398, 412, 419, 427, 564
Mechanisms	416
Accreditation Criteria for Pharmacy Technicians	xxi, 413
Evaluations	436
Institutions.....	119
Pharmacy Programmes	353
Pharmacy Technician	353, 498
Accreditation Process..	126, 137, 416
Accreditation Tools	136
Achievements xxix, xxxi, 15, 38, 43, 47, 55, 61, 63, 67, 70, 99, 101, 133,	

- 153, 160, 161, 163, 164, 171, 180,
189, 190, 196, 197, 198, 203, 204,
205, 206, 209, 218, 228, 229, 233,
239, 243, 247, 248, 251, 255, 256,
257, 272, 279, 287, 289, 345, 349,
356, 400
- ACPN See Association of Community
Pharmacists of Nigeria
- Adamu Adamu (Malam)..... 211
- Adamu Mamman Kontagora
(Mallam)xv, 45, 179
- Abubakar Tafawa University (Atbu).239
- Adeniji, S.A. (Pharm.)32
- Administrative Staff College of
Nigeria (ASCON) ..xxv, 28, 206, 563
- Advancement of Healthcare and
Humanity 165
- Advocacy. 49, 56, 118, 137, 139, 171,
197, 347, 368, 378
- AfDB.. 219, See African Development
Bank
- AfDF. See African Development Fund
- African Development Bank (AfDB)
..... xxv, 34, 36, 83, 217
- African Development Fund (AfDF)
..... xxv, 34, 36
- Ahmad Nuhu Bamalli, *cfr* (His
Highness Amb. Dr) 48, 190, 210
- Ahmadu Bello (Sir) 19
- Ahmadu Bello University (ABU) ...xxv,
xxxii, 2, 12, 18, 26, 28, 37, 42, 48,
55, 58, 63-64, 66, 68, 116-117, 177,
181, 192, 197-198, 200, 203, 205-
208, 210, 212, 217, 223, 231, 238,
240, 245, 247, 260, 266-268, 272-
274, 278, 285, 294, 444, 447, 503-
505, 538, 554, 556-557, 559-560,
566
- Ahmadu Bello University Pharmacy
Alumni Association (Abupaa).... 12,
37, 55, 64, 117, 240
- Ahmadu Bello University Teaching
Hospital (ABUTH) xxv, 26, 278, 444,
504, 554, 559
- Ahmadu Ido..... 19
- Ahmadu Mohammad (Alh.) 8
- Ahmadu Mohammed (Alh.)5, 9
- Ahmadu Muhammad (Alh.)7, 8
- Ahmed Awaisu, *ph.d, mpharm* (Prof.)
..... xvii, 262
- Ahmed Babba Kaita (Sen.)..... 49
- Ahmed Gana Mohammed (Prof.) . 67
- Ahmed Ibrahim Umar, *b.pharm,*
fpcpharm xxiv, xxxv, 561
- Ahmed Mohammed (Alh.) 19, 240
- Ahmed Nuhu Bamalli (Alh.)112
- Ahmed Sambo . 38, 40, 109, 177, 276
- Ahmed Talib (Alh.) 19
- Ahonaruogho, Richard Oma, *san*
(Chief)..... 46
- Aisha Abubakar (Hon. Hajiya)292
- Aisha Abubakar Tunau 76
- Aisha Uwa (Hajiya)..... 13, 63
- Aishatu (Hajiya) 18
- Aishatu Jibrin..... 24
- Akawu, Yakubu (Pharm.) xvii, 257
- Akunyili, Dora, *fpsn* (Prof.).... 69, 148,
213, 241, 538
- Alex Uzo Onyido, *fpsn* (HRM Igwe
Pharm.)..... 108, 166
- Alhaji Abubakar Alhaji .. xviii, 291, 292
- Aliko Mohammed (Alh.)..... 21
- Aliyu Jibrin, *fpsn* (Pharm.) .iii, xvi, xxiv,
xxxiii, xxxv, 66, 67, 79, 231, 242,
554, 555
- Aliyu Maiborno (Alh.)..... 20
- Aliyu Musa Mohammed (Prof.)..... 29
- Amina Ammani..... 24
- Amina J. (Hajiya)294
- Amina of Zazzau 1
- Aminatu Dikko (Hajiya) 14, 17, 18, 22,
30, 112, 215
- Aminatu Dikko (Hajiyya)14, 17, 22,
30, 112, 215
- Aminu Diyo Sheidu (Prof.) 27

Aminu Kano Teaching Hospital
(AKTH) 67, 561, 562
Aminu Salihu Mika'ilu (Prof.) . xvii, 24,
263, 265
Aminu Umar Bambale 25
Amos Bangudu (Dr.) 29
Ango Abdullahi (Prof.) 21
Annual Scientific Conferences 50,
59, 93, 204, 280, 282
Anthony Akhimien 532
Anthony Emeribe (Prof.) 363
Anti-Counterfeit Medicines
Campaign 69
ANUA See Association of Nigerian
Universities Alumni
APIM See Association of
Pharmaceutical Importers of
Nigeria
ASCON See Administrative Staff
College of Nigeria
Asiya (Hajiya) 292
Asiya Ladi (Hajiya) 13, 75
Asiya Suleiman (Hajiya) 63
Association of Community
Pharmacists of Nigeria (ACPN) 555
Association of Nigerian Universities
Alumni (ANUA) 12, 45, 64, 239
Association of Pharmaceutical
Importers of Nigeria (APIM) 425
Association of Vice-Chancellors of
Nigerian Universities (AVCNU) ... 49
ATBU .. See Abubakar Tafawa Balewa
University
Atueyi, I. (Pharm.) 102
Audu Dauda (Dr.) 31
Australia 244, 249
Auxiliary Nursing 437
Auxiliary Nursing Staff 366, 367, 403,
405, 429, 431, 432, 437, 449, 461,
496
Auxiliary Pharmacy Staff 32
AVCNU See Association of Vice-
Chancellors of Nigerian

Universities
Annual Conference of 49
Awards ..13, 41, 53-63, 69, 129, 131,
189, 200, 214, 238, 241, 250-251,
283, 430, 513, 548, 555, 558, 560,
563
Awwalu Aliyu 24
Awwalu M. Makarfi (Alh.) xv, 186
Azubike Okwor 532

B

Baba Ahmed (Alh.) ... 3-10, 20, 24, 62,
263, 264
Baba Ahmed Primary School.. 24, 62,
264
Baba Ahmed University 9
Baba Sha'aban 4, 7, 9, 10, 21
Babagana Umara Zulum (Prof.) 31
Babalola, Chinedum Peace (Prof.) 69
Babangida Musa 72
Babangida, I. U., *mfr, mni* (Brig. Gen.)
..... 75
Badawiyya Pharmaceutical Products
Manufacturing Company 183
Bah (Mallam) 32
Bakaries and Metal Nails 9
Baker, S. V. 25
Bala (Alh.) 21
Bala Bello Dambazau (Alh.) 21
Balarabe B. Ibrahim 25
Balewa, Abubakar Tafawa (Sir) 15, 19
Bamalli, Abdullahi Nuhu (Engr.) 37
Barau Dikko Hospital, Kaduna 555
Barau Dikko Teaching Hospital
(BDTH) xxi, 444, 449, 453, 458
Barewa College Management Board
..... 40
Barewa College, Zaria ... xxxiii, 17, 19,
25, 38, 40, 41, 72, 77, 177, 220,
221, 222, 224, 225, 227, 228, 238,
245, 266, 274, 278, 288, 289, 293
Bashir Ahmed Banadeen (Alh.) 21
Bashir Umar (Mallam) 363
Bawa, Salisu Bashir (Pharm.) xvii, 255

Bayero University, Kano.... 48, 50, 66, 68, 219, 240, 504, 556, 557
 Baze University 9, 48
 BDTH.....See Barau Dikko Teaching Hospital
 Bello Kano 15
 Bello Shaban 7
 Ben A. Chindo (Prof.) 30
 Benchmark Minimum Academic Standards (BMAS) ... 122, 137, 408, 430, 466, 473, 474
 Benin Kingdom..... 31
 Benjamin Dangiwa 510
 Bichi, Abdullahi Baffa (Dr.)..... 47
 Bingham University, Karu 171
 Bishop (Dr.) 30
 BMAS See Benchmark Minimum Academic Standards
 Board of Trustees (BOT) 6, 13, 37, 41, 42, 57, 64, 65, 77, 178, 202, 208, 238, 240, 272, 511
 Boba Class Group of 1970 276
 Boba Newsletter 238
 Bok, D. (Dr.) 33
 Bola Ahmed Tinubu, Gcfr (Nigeria President, Asiwaju) 44, 200, 227
 Bola Alaka 32
 Bola Haruna (Alh.) 363
 BOT..... See Board of Trustees
 British Colonial 2, 304, 456, 496
 Brown, A. A. (Pharm.) 84, 88, 90
 Bruno Nwankwo (Pharm.)..... 539
 Bulus Kyauta Tanyigna (Dr.) 28
 Burkina Faso..... xxi, 340, 413

C

CAANU See Conference of the Alumni Associations of Nigerian Universities
 Chairman 210, 211
 Patrons of..... 48, 210, 239
 Capacity Building . 44, 101, 135, 161, 171, 238, 258, 400, 460, 477-479, 480

Capital City University (CCU)503
 Career Employment..... 31
 CASSee College of Advanced Studies
 CCEPT .. See Compulsory Continuing Education Programme For Pharmacy Technicians
 CCU See Capital City University
 CFA See Chartered Financial Analyst
 Challenges of Health Regulation in Nigeriaxx, xxi, 402, 410, 428
 Charles Okechukwu Esimone, *fas, fpsn, fnapharm* (Prof.) xvii, 282
 Chartered Financial Analyst (CFA) 63
 Chief Nursing Officer 32, 33
 Chief Pharmacist... 11, 13, 27, 31, 33, 52, 81, 82, 84, 116, 186, 219, 232, 233, 248, 277, 506, 508, 538, 547, 562
 Childhood Friends190
 Chinedom Peace Babalola, *fpsn* (Prof.)241
 Chinyere Mercellina Chukwuani (Dr.)102
 CHPRBN See Community Health Practitioners Registration Board of Nigeria
 Chukwuemeka Odumegwu Ojukwu University.....171
 Ciroman Zazzau 37
 Clash of Mandates147
 Class Group xxiii, 12, 38, 39, 78, 109, 276, 503
 Classmates..... 24, 26, 221, 222, 227, 245, 246, 264, 274, 278, 293, 505
 Clinical Pharmacy xxix, 12, 13, 28-30, 69, 97, 137, 159, 184, 192, 200, 205, 234, 249, 262, 267, 394, 399, 424, 447, 450, 484, 506, 507, 548, 561, 563
 Clinical Pharmacy Services.....399
 Collaboration.. 35, 41, 44, 45, 50, 51, 60, 134, 138, 141-143, 145, 152,

159, 160, 162-164, 172, 206, 214,
247, 256, 284, 290, 312-342, 368,
370, 382, 397, 399, 400, 408-410,
426, 427, 435, 446, 453, 460, 464,
468, 478, 481, 485, 498, 535, 547
College of Advanced Studies (CAS) 6,
245
Committee of Registrars of Health
Professions (CRHP) .xx, xxxii, 56,
230, 235, 363, 366, 410, 439
Committee of Registrars of the
Health Professions (CRHP). 12, 35,
137, 138, 363, 368, 457
Community Health Practitioners
Registration Board of Nigeria
(CHPRBN).363, 366, 403, 404, 406,
429, 430, 432, 448, 460, 464
Compulsory Continuing Education
Programme for Pharmacy
Technicians (CCEPT)..... 129
Computerized Monitoring Systems
..... 427
Conference of Alumni Associations
of Nigerian Universities See
Association of Nigerian
Universities Alumni
Conference of Alumni Associations
of Nigerian Universities (CAANU)
.....45, 207, 209, 212, 250
Conference of the Alumni
Associations of Nigerian
Universities (CAANU) xxv, 12, 45
Conflict
Nafdac and the PCN 146, 147, 149
Operational 149
Pharm Mora and the PSN..... 146
Confrontation 147, 148, 173
Corporate Affairs Commission (CAC)
..... 240
Court of Appeal..... xxiii, 151, 515
Courts and Authorities517, 529
CRHPSee Committee of Registrars of
the Health Professions

objectives of.....229

D

Dabo Mohammed Lere (Late Alh.)
.....219
Dada Sare (Hajiya) 20
Dahiru Ibrahim (Engr.)..... 40
Dahiru Sulaiman Wali, *fpsn* (Pharm.)
....54, 59, 65, 66, 68, 240, 242, 279,
288, 289, 558
Dalhatu Sarki Tafida (Dr.)..... 21
Dalhatu Yusuf 24, 264
Dambazau, Abdurrahman Bello, *cfr*
(Rtd. Lt. Gen.) .xvi, 26, 39, 109, 225,
227, 293, 503
Danburan Zazzau 10
Danjuma, N. M. (Prof.) 29
Dankaden Gumelxvi, 245, 246
Danlami Attah (Mallam) 22
DDIC..... See Drug Distribution
Inspection Committee
Dental Surgeon 404, 430
Dental Technologists Registration
Board of Nigeria (DTRBN)230,
366, 403, 429, 448, 456, 460, 497
Dental Therapists Registration Board
of Nigeria (DTRBN)... 363, 366, 367,
403, 429, 432, 448, 460, 497
Descendants of Bayajidda 1
Dewu, Mohammed Abdullahi (Arc.)
..... 45, 79
DIFCSee Drug Information and
Formulary Centre
Disciplinary Committee .85, 407, 433
Disciplinary Procedures..... 164, 170
Disciplinary Tribunal.....85, 118, 170,
306, 320, 407, 410, 433, 434, 440
Discipline xxxv, 4, 83, 95, 96, 99, 131,
160, 165, 169, 181, 187, 191, 196,
217, 225, 226, 244, 254, 264, 267,
275, 288, 405, 424, 431, 479, 541
administrative 104, 164
financial.....101
managerial..... 96

Disposable Syringes Manufacturing
Plant Project 34

District Head in Borno State 24

Dora Akunyili College of Pharmacy,
Igbinedion 538

DRF See Drug Revolving Fund

Drug Abuse 67, 536, 540

Drug Administration .. 14, 28, 61, 147,
181, 213, 284, 304, 307, 323, 370,
421, 424, 451, 567

Drug Distribution .. xix, 69, 108, 110,
122, 123, 134, 143, 149, 237, 258,
266, 267, 277, 279, 315, 316, 326,
328, 329, 333, 342, 344, 347, 375,
379, 397, 422, 514

Drug Distribution and Inspection
Committee (DDIC) xix, 326, 333,
347, 375, 379

Drug Information and Formulary
Centre (DIFC) 508

Drug Laws 303, 328, 380

Drug Quality Control 557

Drug Reactions 140

Drug Registration Committee..... 308,
324

Drug Revolving Fund (DRF) 140

Drug Supply Chain 381

Drugs .. 81, 95, 122, 127, 134, 144,
147, 148, 150, 152, 304, 306, 307,
308, 309, 313, 314, 315, 316, 318,
320, 322, 323, 324, 325, 328, 334,
351, 356, 373, 374, 375, 379, 393,
397, 422, 424, 425, 450, 451, 455,
484, 508, 509, 514, 533, 561, 562
sources of 425

Drugs Act..... 306, 308, 323, 324, 393

DTRBN..... See Dental Technologists
Registration Board of Nigeria, See
Dental Therapists Registration
Board of Nigeria

Duke Daniels 58

E

Ebonyi 47, 48, 56, 131, 172

Echeazu Ogu, *ph.d* 214, 378

Echeazu Ugwu (Dr.) 214, 396

Economic Community of West
African States (ECOWAS) 340

ECOWAS.xxi, 399, 400, 413, 420, See
Economic Community of West
African States

Eghosa Osaghae (Prof.) 30

Elizabeth Bob-Manual (Pharm. Mrs.)
..... 88

Eme Ekaette (Pharm. Lady) 103

Eme Ekaite 532

Emir of Bauchi 48, 210

Emir of Lafia 48, 210

Emir of Zazzau 20, 37, 44, 48, 54, 112,
190, 200, 210, 220, 277, 285

Emirs of Zazzau xxx, 2

Enosa Omoghobo 530

Enugu State University of Science
and Technology 171

Eradiri, G. L. 84

Ereko Dispensary Rules 304, 351

Erne Ufot Ekaette (Lady) 88

Ernest Shonekan (Chief Head Interim
National Government) 98

Erosion of Professional Boundaries
..... 137

Essien, E. E. (Prof.) 103

Esua, E. E. 16

Ethical Regulation 104

Eyitayo Lambo (Prof.) . 153, 338, 368,
377, 410, 439, 458, 539

Ezemonye, Lawrence Ikechukwu
(Prof.) 31, 176

F

Faculty of Pharmacy Alumni
Association 178

Faith O. Ayosin (Pharm.) 59

Fajemirokun (Mrs.) 84

Fake Drugs 28, 134, 144, 181, 284,
309, 314, 325, 399, 437

Fatima Nana Bello Kagara (Hajiya)
..... 199

FCT See Federal Capital Territory
 Federal Capital Territory (FCT) 13, 81,
 150, 290, 352, 370, 395, 437, 534,
 547
 Federal Ministry of Health (FMOH) 81,
 85, 95, 97, 98, 130, 150, 162, 172,
 236, 279, 296, 297, 314, 315, 316,
 323, 366, 393, 399, 400, 403, 405,
 425, 428, 431, 442, 456, 461, 484,
 497, 532, 533, 546
 Femi-Oyewo, M.N., *mfr* (Prof.) 68,
 102
 Festus Adio Ogunbona (Emeritus
 Prof.) 88, 101, 235, 539
 Festus Adio Ogunbona, *fpsn*
 (Emeritus Prof.) 86, 87, 89, 102,
 396, 539
 Financial Autonomy 96, 123
 Financial Misappropriation .. xxiii, 530
 Financial Prudence 99
 First Director of Pharmaceutical
 Services 33, 115, 248
 First Governing Council (1992–1993)
 96
 FMOH.... 429, See Federal Ministry of
 Health
 Fola Tayo (Prof.) 84, 102, 380, 396
 Foreign Diplomatic Service 74
 Foreign Pharmacy Graduate
 Orientation Programme (FPGOP)
 310
 Four Wonders 24
 FPGOP See Foreign Pharmacy
 Graduate Orientation Programme
 Fraud xxiii, 530
 Fred B. Adenika (Dr.) xviii, xix, 89, 99,
 101, 103, 124, 279, 338, 357, 380,
 395, 396, 538, 539
 Fulani Jihad 2

G

Gabriel E. Osuide (Prof.) ... 38, 72, 89,
 107, 512
 Gadzama. Ahmed 32
 580

Gamani, K. S. (Prof.) 103
 Ganuwar Amina 1
 Garba Abubakar Daya (Alh.) 31
 Garba Ahmed (Col.) 5, 7
 Garba, M. (Prof.) 29
 Germane Kemba Ya'u 276
 Gheelt School 501
 Gmp See Good Manufacturing
 Practice
 Gombe State University (GSU) 48,
 110
 Good Manufacturing Practice (GMP)
 ... xix, 134, 339, 347, 348, 376, 377,
 378, 385, 425, 562
 Governing Council xxiii, xxiv, 6, 12,
 14, 31, 36, 52, 64, 65, 84, 86, 115,
 118, 150, 160, 169, 176, 234, 249,
 253, 259, 277, 298, 300, 301, 313,
 350, 352, 367, 395, 396, 413, 414,
 415, 416, 417, 419, 436, 437, 479,
 486, 488-490, 492, 493, 498, 504,
 507, 535, 537-540, 542, 543, 546,
 547, 549, 550
 chairman ... xxxi, xxxii, xxxiii, 36, 65,
 85, 159, 163, 169, 176, 180, 181,
 190, 194, 205, 223, 232, 235-
 237, 253, 259, 272, 293, 507,
 532, 547, 550
 chairman of the pcn ... 62, 267, 279,
 396
 first 96
 pcn 533
 prof. mora's leadership of the .. 164
 second 99
 seventh 169
 third 101
 Grünenthal GmbH 11, 32, 115
 GSU See Gombe State University
 Guidelines on Establishing Faculties
 of Pharmacy 311

H

Hadiza Ahmed (Prof.) 8, 9
 Hadiza Bala Usman (Hajiya) 43

Haematology	450
Hajj Rites	53
Hakeem Baba Ahmed (Dr.) 5, 7, 9, 10	
Halidu Tanko Soba (Justice)	24
Haliru Binji (His Eminence Alh.)	20
Hameed Ibrahim Ali (Col.)	294
Hamidu Ladan Zuru (Alh.)	21
Hamidu Shafi'i (Alh.)	21
Hamidu Yahaya (Alh.)	21
Hannatu Yako Shehu (Dr.)	33
Harisu Ibrahim	24
Haroun Umar, <i>mni</i> (Amb.)	39
Haruna Aliyu (Late Pharm.)	xxxvi
Haruna Muhammed Jumare (Dr.) ..xv,	112, 196, 198
Haruna Soba (Alh.)	21
Hassan Usman Katsina (Gen.)	78
Hassan, Rahmatu (Dr. Mrs.) . xvi, 205	
Hausa Bakwai	1
Hauwa Aliyu Jafiya	245
Health Broadcast Journalist	xviii, 286, 287
Health Care System	535
Health Management Board (HMB)	
.... xxvi, xxxii, 11, 33, 217, 233, 277,	506, 508
Health Records Officers Registration	
Board of Nigeria (HRORBN)	403, 429, 448, 496
Health Regulatory Agencies . xx, 229,	363, 366, 367, 369, 371, 390, 403,
411, 428, 429, 436, 440, 441, 443,	456, 478, 496, 520
Primary Function Of	402
Role of.. xxi, 56, 402, 407, 434, 456,	457, 459
Health Service Delivery	428
Health System	243, 344, 533
Healthcare Administration ..	561, 563
Healthcare Delivery ... xix, xxi, 53, 56,	60, 134, 192, 197, 226, 296, 334,
338, 342-344, 346, 349, 364, 368-	372, 407, 410, 413, 420, 434, 438-
440, 452, 456-459, 480, 496, 500,	564
Healthcare Discipline	541
Healthcare Facilities ..83, 217, 305,	330, 334, 402, 408-410, 428, 436,
438, 440, 452, 460, 463, 478, 479,	485
Healthcare Information	454, 455
Healthcare Products	336
Healthcare Professionals	52, 122,
287, 334, 339, 358, 372, 407-410,	436, 438, 440, 442, 445, 447, 448,
452-454, 457, 485, 496	
Healthcare Provider	xxi, 444, 446,
452-454, 456, 459	
Healthcare Reform	137, 267
Healthcare Services .305, 428, 444-	446, 452, 453, 456, 485
Healthcare Systems .139, 408, 435,	481, 564
Hospital and Administrative	
Pharmacists	60, 533
Hospital Pharmacy . 31, 33, 123, 139,	140, 161, 171, 248, 301, 329, 345,
350, 353, 374, 376, 447, 499, 541	
Hospital Pharmacy Practice .31, 33,	139, 140, 350, 376
Housemanship .408, 411, 436, 440,	441
HRH	See Human Resources for Health
HRORBN	See Health Records Officers Registration Board of Nigeria
Human Resources	133, 358, 395,
403, 413, 416, 420, 428, 441, 444,	488
Human Resources for Health (HRH)	
.....	403, 413, 428, 441
Humiliating	226
Humiliation	235, 541
Humility xxxv, 99, 104, 182, 187, 197,	205, 211, 216, 225, 243, 251, 255,

261, 265, 276, 277, 283, 286
Hussaini Mashī (Alh.)..... 20

I

IBB University..... 25
Ibraheem Mu’azzam Tunau, *mbbs*,
frcsi, *dip. ls* (Dr.)..... xv, 201
Ibrahim Adamu Yakasai (Prof.)..... iii, xxiv,
xxxv, 66, 67, 79, 240, 556
Ibrahim Ahmed..... iii
Ibrahim Amiduiii, 93
Ibrahim Baba Shehu Ahmed, Fpsn,
Fpcpharm, Fnapharm (Pharm.)
..... 231, 232, 237
Ibrahim Babashehu Ahmed (Pharm.)
..... 62, 92, 93, 170, 247, 253
Ibrahim Baba-Shehu Ahmed
(Pharm.) xvii
Ibrahim Balarabe 24
Ibrahim Dasuki (Sultan) 21
Ibrahim H. Musa (Pharm. Alh.) 20
Ibrahim Hassan Dankwambo (H.E.
Gov. of Gombe State) 44
Ibrahim Iliya (Prof.) 68
Ibrahim Jatau Abubakar, *ph.d*
(Pharm.) xvi, 243, 244, 249
Ibrahim Mohammed Amidu,
b.pharm, *msc*, *gppc*, *fpcpharm*
(Pharm.) 566, 567
Ibrahim Mu’azzam Tunau, *mbbs*,
frcsi, *dip. ls* (Dr.)..... 199
Ibrahim Mohammed Amidu (Pharm.)
..... xxiv, xxxvi
Ibrahim Namaska Kagara (Alh.) 21
Ibrahim Sambo 24, 26, 264
Ibrahim Sarki Lukman (Mallam) 24
Ibrahim Tunau (Late) 76, 201
Ibrahim, Hajara (Prof. Mrs.)..... 68
ICCON..... See Institute of Chartered
Chemists of Nigeria
Icon of Pharmacy..... 59, 62, 101
ICPCxxiii, 152, 235, 542, See
Independent Corrupt Practices
and Other Related Offences

Commission
Investigation151
Verdict.....151
Idon Ganin Alheri Ltd..... 8
Idris A. Abdulkadir (Prof.)111
Idris Abdulkadir (Prof.) 37
Idris Hayatudeen (Arc.) 24
Idris Morrow (Alh.) 21
Ifeanyi Atueyi (Sir)101
Igbinedion University Teaching
Hospital (IUTH).....31, 65, 259
Igbinedion University, Okada.....xxvi,
xxxii, 10, 12, 30, 31, 48, 62, 65, 69,
176, 181, 190, 192, 205, 223, 249,
255, 258, 259, 267, 272, 277, 279,
287, 504, 507, 538, 548
Igbinedion, Gabriel O., *cfr* (Sir) 31
Illegal Operators.....143
Independent Corrupt Practices and
Other Related Offences
Commission (ICPC)..... 151, 530
Influence of Family and Guardians 22
Innovation and Vision218
Innovations and Patents262
Institute of Chartered Chemists of
Nigeria (ICCON)366, 367, 403, 406,
407, 429, 432, 433, 449, 460, 468,
497
Institute of Corporate Administration
of Nigeria 53
Institute of Healthcare Management
..... xxi, 53, 402, 428
Institute of Human Virology... 65, 279
Institute of Public Analysts of Nigeria
(IPAN)363, 366, 367, 403, 406, 429,
432, 433, 449, 460, 497
Institutional Autonomy 98
Integrity .xviii, 84, 100, 104, 118, 145,
160, 162, 164, 165, 174, 179, 180,
187, 188, 189, 191, 193, 195-197,
199, 204, 211, 216, 217, 218, 222,
223, 243, 244, 248, 257, 266, 288,
289, 291, 374, 376, 386, 404, 410,

416, 418, 440, 441, 534, 539, 549, 567
 Institutional 229
 Interference. 411, 441, 442, 461, 463, 472, 477, 478, 479
 Interference and Duplication of Regulatory 463, 472
 International Society of Pharmacovigilance (ISOP) 567
 Internship . 26, 31, 42, 64, 120, 267, 281, 310, 318, 335, 353, 358, 393, 408, 411, 436, 440, 441, 489, 498, 559
 mandatory 26
 Internship Placements 267, 411
 Internship Training..... 26, 42, 64, 318, 335, 353, 358, 393, 489
 Inter-Professional . 35, 122, 123, 137, 138, 141, 145, 459
 Inter-Professional Collaboration. 145
 Inter-Professional Friction 123
 Inter-Professional Negotiation 138
 Inter-Professional Rivalry 459
 Inuwa Mai Gidan Sauro (Alh.)..... 20
 Iowa State University..... 501
 IPAN. See Institute of Public Analysts of Nigeria
 Irimiya Aboi (Dr.) 505
 Irimiya Yawuk 25
 Iro K'aya (Alh.)..... 10
 Isa Koto (Alh.) 20
 Isa Marte Hussain (Prof.) 241
 Isa Marte Hussaini (Prof.) 31, 70
 Isa Odidi (Prof.) xvii, 266, 268
 Isah Baba (Prof. Pharm.) .. 509, 510
 Ishaku Suleiman (Alh.)..... 21
 Islamic Theology 23
 Ismaila Datti (Rtd. Capt.) 24, 264
 Isobighe (Dr.) 30
 ISOP..... See International Society of Pharmacovigilance
 IUTH See Igbinedion University Teaching Hospital

J

Ja'afaru Dogo (Alh.) 21
 Jacob Kwaga (Prof.)..... 25
 Jaiz Bank 64
 James H. Yaro 39
 Jega, M. N. (Rtd. Maj. Gen.) 112
 Jekada, E. K., *ph.d, mni* (Rtd. Air Commodore)..... xv, 180, 182
 Jidda (Hajiya)..... 24
 Jidda Umaru Abdulmutallab (Hajiya) 24
 Jide Idris (Dr.) 347
 Jimada Pategi (Alh.) 20
 John C. Ejezie (Dr.)..... 59
 John Garba (Amb.) 21
 Jonathan Obiagwu 50, 303
 Jonathan Phraguwa Zamuna, JP (HRH)..... 54, 224
 Jonathan Phraguwa Zamuna, JP (HRM chief) xvi, 39, 166, 224
 Joseph Njoagbethe (Pharm.) 75
 Judgment 180, 222, 385, 515, 517-519, 525-529
 Julius Adelusi-Adeluyi (Prince) ... 103, 368, 394
 Jumare, Ibrahim Muhammad (Prof.) xvii, 27, 285

K

Kabiru Abubakar Gulma, *ph.d* (Prof.) iii, xxiv, xxxvi, 564, 565
 Kaduna Health Rehabilitation Project (KHRP) xxvi, 7, 11, 34, 83, 116, 219, 267, 509, 547
 Kaduna State Distribution Agency (KSDA) . xxvi, xxxi, xxxii, 11, 32, 506, 508
 Kaduna State University (KASU) . xxi, xxvi, xxxi, xxxii, 12, 29, 48, 54, 55, 57, 58, 63, 64, 68, 69, 103, 181, 192, 234, 249, 254, 255, 258, 261, 267, 272, 277, 282, 402, 412, 421, 445, 452, 458, 503, 505, 538, 548

Kaduna State Water Board	206
Kaita, H.A. (Prof.)	29, 67
KAJIPSA	See Kano/Jigawa Pharmaceutical Students Association
Kano/Jigawa Pharmaceutical Students Association (KAJIPSA)	261
KASU .	65, 176, 236, 444, 447, 450, 453, 454, 504, 506, 513, 514, 542, See Kaduna State University Vice-Chancellor of
K'ayayen Sardauna...	20, 76, 157, 201
Kayit, H. D. (Ms)	97
Kazir, A. J., <i>cfr</i> (Maj. Gen.)	40
Key Innovations As Secretary General (2012–2021)	40
KHRP	See Kaduna Health Rehabilitation Project
Kolo Ibrahim (Prof.)	25, 511
Kontagora, M.T. (Rtd. Maj. Gen.) ...	38
Kumai Bayang Akaahs, <i>jca</i> (Justice)	515

L

L.E.A. Primary School Tudun Wada	24, See Baba Ahmed Primary School
Ladan Na Mazadu (Mallam)	20
Ladan, A.C (Alh.)	157
Ladi (Khadijat, Hajiya)	157
Laraba (Mallama)	32
Lawal Ahmed	24
Lawal Balah	245
Lawal Ladan (Dr.)	xvi, 203, 204
Leaders .	xv, xxxv, 2, 36, 37, 60, 117, 164, 182, 189, 196-198, 203, 207, 209, 211, 219, 220, 222, 225, 229, 233-237, 241, 243, 260, 261, 267, 268, 272, 285, 288, 407, 483, 542 disciplined.....
	182
	dynamic and authoritative
	117
	visionary.....
	xv, 196, 209, 260, 261
Leadership	iv, xvii, xxiv, xxxii, xxxv, xxxvi, 17, 32, 35, 36, 38, 40, 46, 50,

53-62, 68, 100-102, 104-116, 138, 139, 141-145, 159-165, 170, 172-174, 177-181, 186, 190-192, 194, 196-198, 200, 205, 206, 209-211, 214, 218, 222, 225, 226, 229, 235-237, 241, 245, 247-252, 254, 256, 260, 264-268, 272, 273, 276, 277, 283-285, 288, 289, 320, 330, 356, 360, 389, 400, 433, 456, 483, 503, 512, 541, 542, 546, 553, 555, 561, 563, 566	
exemplary	165, 220, 279, 563
professional.....	83
transformational	209
transformative.....	279
Leadership Newspaper	553
Leadership Roles in Boba.....	40
Legacy xxxiii, xxxvi, 15, 17, 24, 45, 63, 66, 68, 103, 159, 163, 164, 165, 173, 174, 178, 179, 187, 189, 190, 192-194, 197, 198, 204, 205, 211, 215, 218-220, 226, 233, 243, 248, 251, 253-255, 257, 261, 262, 267, 270, 273, 286, 287, 288, 290, 292	
Legacy and Impact.....	163
Legacy of Excellence	286
Lessons Learned	251
Licensing and Practice.....	35
Life and Career	xvii, xxxi, 22, 243, 253, 271
Life and Legacy	205, 220
Life Patron	26, 63, 65, 231
Liman Muhammad Sani Gummi ...	20
Litigation	131, 132, 151, 319
Liverpool Hope University	28, 365, 369
Luka Panya Baba (HRH, Pharm.) .	102

M

Mabel John (Mrs.).....	363
Mahmud Sani Sha'aban (Hon.) ...	4, 9, 10
Maimuna Mohammed.....	24
Mallama Mairamu	24

- Mandatory Continuing Professional Development (MCPD).....xviii, 123, 126, 139, 154, 281, 295, 297, 302, 311, 317, 335, 345, 349, 351, 353, 358, 360, 385, 391, 397, 400, 409, 439, 489, 491, 498, 534
- Mandatory Continuing Professional Education (MCPE) ... 295, 297, 390, 409, 439
- Manufacturing Companies
 Local..... 425
 Pharmaceutical214, 326
- Martins Emeje, *ph.d, faas, fnapharm, fpsn* (Prof.).....xvii, 270
- Maryam Isa Dutse 245
- MCPD ..129, 298, 299, 337, 355, 382, 387, 492
 curriculum and credit units 297
 guidelines..... 493
 objectives of 296
- MCPE See Mandatory Continuing Professional Education
- MDCN ..410, 437, 458, 459, 471, 478, 480, See Medical and Dental Council of Nigeria
 act 430
 regulatory work 478
- MDGs. See Millennium Development Goals
- Medical and Dental Council of Nigeria (MDCN) 280, 366, 367, 403, 404, 406, 429, 432, 445, 448, 456, 461, 468, 469, 496
- Medical Laboratory Science Council of Nigeria (MLSCN).. 363, 403, 405, 429, 431, 435, 445, 448, 461, 468, 496
- Medical Laboratory Scientists
 Council of Nigeria (MLSCN) 366, 406, 432, 485
- Medical Rehabilitation Therapist
 Board Nigeria (MRTBN) 461
- Medical Rehabilitation Therapists
 Registration Board of Nigeria (MRTBN) . 366, 403, 406, 429, 449
- Medication Errors . xix, 329, 330, 331, 332, 334, 336, 338, 361, 438
- Medicines xxix, xxx, 8, 35, 41, 65, 67, 68, 100, 103, 122, 125, 137, 139, 150, 173, 194, 199, 200, 270, 275, 278, 285, 286, 287, 303, 304, 310, 311, 313, 315, 319, 322, 330, 331, 334, 336, 344-346, 349, 351, 357, 358, 373, 376, 378, 379, 380, 381, 388, 397, 421, 424, 426, 427, 437, 444, 447, 450, 452, 493, 496, 503, 536, 566, 567
 counterfeit 69, 143
 dispensing 120, 338
 patent .88, 119, 120, 147, 148, 150, 310, 317, 354, 355, 356, 357, 425, 494
 proprietary 100, 154, 169, 315, 316, 353, 380, 437, 488, 489, 493, 498
 substandard..... 144, 400
- Medicines Regulation ..381, 566, 567
- Mentorship.... xvi, 164, 178, 196, 198, 203, 205, 223, 233, 243, 245, 248-250, 254, 258, 260, 261, 267, 268, 281, 285, 400, 563, 567
- Michael Okpo274
- Millennium Development Goals (Mdgs).....420
- MLSCN See Medical Laboratory Science Council of Nigeria
- Mohammed A. Danjuma, *jca* (Justice)515
- Mohammed Abdullahi Yahaya (Air Commodore)..... 26
- Mohammed Dalhatu Zurmi 24
- Mohammed Garba Magaji (Prof.) xvii, 29, 67, 271, 273
- Mohammed, Amina 24, 44

Moqhtari Mohammed Danmallam	
Malumfashi	xvi, 228
Mora, Abbas A. (Alh.)	8, 112, 276
Mora, Ahmed Tijjani, <i>fpc pharm, phd, fpsn, ficen, fihmn, fnim, oon, mni</i> (Prof.)	i, iii, lv, xv, xvi, xvii, xviii, xix, xx, xxi, xxii, xxiii, xxiv, xxix, xxx, xxxi, xxxii, xxxiii, xxxv, xxxvi, 1, 6, 8-11, 13, 15, 22, 24, 61-63, 66, 68, 70, 72, 73, 75-79, 88, 93, 95, 96, 104, 106-113, 145, 153, 156, 159-166, 169, 174, 176, 177, 179-199, 201-205, 208, 209, 211-214, 216, 217, 219, 221, 224, 225, 227, 228, 230-232, 242-247, 253-268, 270-276, 278-283, 285-293, 295, 300, 303, 321, 326, 333, 338, 339, 342, 343, 347, 348, 365, 373, 382, 384, 396, 421, 444, 483, 486, 487, 495, 503, 504, 506, 508, 530, 532, 537, 542, 543, 545, 546, 550, 567
100 day in office at pcn.....	552
achievements	15, 38, 43, 47, 70, 133, 161, 190
ahmed's father	37
appointment as project coordinator.....	34
as a football goalkeeper	24
awards and honours	250, 558, 563
board and council appointments	64
career milestones	250
challenges.....	133, 146
clash with psn.....	150
father's legacy	14
historic achievements	209
honors, awards and fellowships	53
key positions held	11
leadership roles in the profession	555
learned societies & alumni associations	12
legacy and impact.....	163
lessons for success	252
letter from ICPC clearing pharm.	
Mora	152
literary works	12
married life.....	13
personal character and values .	197
petition	xxiii, 530
professional achievements	180, 196, 205, 243
qualifications, fellowships & national honour	11
step-mother	18, 199
stewardship at the PCN	553
students at the undergraduate level	29
tenure and re-election	46
traditional titles .	12, 189, 207, 246, 268, 272, 285, 291
Mora, Asiya (Pharm.....	64
Mora, Faizah (Barr.)	63
Mora, Fatima.....	64
Mora, Hafsat A. (Haj.).....	9
Mora, Halima Inuwa (Dr.)	xvi, 216, 293
Mora, Hamza Ahmed (Arc.) ...	63, 293
Mora, Hauwa.....	64
Mora, Imran Ahmed, <i>cfa</i>	63, 293
Mora, M. S. (Dr.)	112
Mora, Maryam	64
Mora, Muhammad Aminu (Alh.) ...	18, 23, 25, 64, 112
Mora, Muhammad Saifuddeen, <i>mbbs</i> (Dr.)...8, 9, 18, 19, 23, 25, 73, 185, 275	
Mora, Saratu (Dr.)	8
Mora, Umar Faruk (Alh.)	112
Mora, Yasmin	64
Mora, Zubaida (Barr.)	63, 188
MPAN	See Muslim Pharmacists Association of Nigeria
MRTBN....	See Medical Rehabilitation Therapist Board Nigeria
MRTBN .	See Medical Rehabilitation

Therapists Registration Board of Nigeria	
Muhammad Abali Ibn Muhammadu Idrissa (HRH Alh. Dr.)	39
Muhammad Ali Hussein, <i>mni</i> (Dr.)	39
Muhammad Anka (Alh.)	157
Muhammad Bello (Sultan) ..	xxix, 184
Muhammad Kaika (Alh. Shehu)	78
Muhammad Munri Ja'afaru, <i>mni</i> (Alh.)	106
Muhammad Sa'ad Abubakar Assembly Hall.....	77, 276
Muhammad Sa'ad Abubakar, <i>cfr</i> , <i>mni</i> (His Eminence Alh.) xxx, xxxii, xxxvi, 39, 55, 77-79, 109, 184, 227, 276, 504	
Muhammad Tanko (Prof.)	58
Muhammadu Aminu (His Highness Alh.)	20
Muhammadu Bello, <i>gcon</i> (Rtd. CJN)	19, 40
Muhammadu Buhari, Aisha (H.E. Former Nigeria First Lady).....	44
Muhammadu Buhari, <i>gcfr</i> (Former Nigeria President)	xxxi, 36, 159, 160, 169, 180, 235, 236, 256, 294, 532, 533, 537, 539, 542, 546, 547, 550
Muhammadu Inuwa Jibo (Alh.).....	20
Muhammadu Inuwa Mora (Late Alh.)	6, 18, 22, 175, 216, 293
Muhammadu Inuwa Musa (Late Alh.)	20, 22, 175
Muhammadu Jibo, <i>mfr</i> (Alh.).....	79
Muhammadu Lawal Sambo, <i>mni</i> (Amb.)	37
Muhammadu Lawal Tudun Wada (Alh.).....	21
Muhammadu Lawan (Alh.)	20
Muhammadu Sani (Alh.)	20
Muntak'a Kumasi (Sheikh)	21
Musa Jam'iyya (Alh.)	20
Musa S. Abubakar (Alh.).....	20

Musa Tugga Karam (Mallam)	39
Musa Yarima (Mallam)	33
Musa, B. D. (Prof.).....	40
Musibahu Mustapha, <i>ph.d</i> (Dr.) ...	195
Musibau Ade Mustapha, <i>ph.d</i> (Alh. Pharm.)	195
Muslim Pharmacists Association of Nigeria (MPAN)	61
Mustapha Abubakar Imam (Pharm.)	175
Mustapha M. Jumare (Lt. Col.)	283, 285

N

NAE Mohammed (Pharm.)	90, 214
NAF	See Nigerian Air Force
NAFDAC 86, 122, 143, 146, 150, 214, 308, 315, 324, 377, 381, 400, 424, 425, 426, 427, See National Agency for Food and Drug Administration and Control aggressive anti-counterfeiting campaign.....	148
chairman	103
clash of mandates	147
decree	307, 323
role	149
NAFDAC Act.....	147
Nafi'u B. Ahmed (Barr.)	8
NAGPPE... See Nigeria Association of General Practice Pharmacists Employers	
NAKAPS .See National Association of Kaduna State Pharmacy Students	
Nana (Fatima, Hajiya)	157
NANM See Nigerian Association of Nurses and Midwives	
Nanomedicines.....	421, 426, 427
NAPPSA . See Nigerian Association of Pharmacists and Pharmaceutical Scientists in the Americas	
Nasiru (Alh.)	21
National Agency for Food and Drug Administration and Control	

(NAFDAC)14, 28, 61, 147, 213, 284, 304, 307, 421, 424, 567

National Association of Kaduna State Pharmacy Students (NAKAPS).....54, 59

National Council on Health (NCH)409, 438

National Drug Law Enforcement Agency (NDLEA)304, 307

National Drug Policy (NDP) .310, 345, 346, 373, 379

National Electronic Pharmacy Policy 567

National Health Insurance Scheme (NHIS) 451

National Health Policy (NHP) 447

National Immunisation Days (NIDs)..... 510

National Initiatives in Pharmacy Regulation and Policy..... 567

National Institute For Pharmaceutical Research and Development (Niprd).. 82, 172, 498

National Institute for Policy and Strategic Studies (NIPSS) ..27, 180, 182, 266, 280, 283

National Orthopedic Hospital..... 447

National Programme Immunisation (NPI) 510

National Regulatory Authorities (NRAs) 381

National Teachers' Institute (NTI) 289

National Universities Commission (NUC) 29, 47, 136, 394, 484, 488

National Youth Service Corps (NYSC) xxvi, 31, 554

NAU ..See Nnamdi Azikiwe University

NDA See Nigerian Defence Academy

NDLEA..... See national drug law enforcement agency decree.....307, 323

NDPSee National Drug Policy

NHPSee National Health Policy

NIDs See National Immunisation Days

Nigeria Association of General Practice Pharmacists Employers (NAGPPE).....305

Nigeria Natural Medicine Development Agency (NNMDA) 270

Nigerian Air Force (NAF) 7

Nigerian Army 7, 181

Nigerian Association of Nurses and Midwives (NANM)485

Nigerian Association of Pharmacists and Pharmaceutical Scientists in the Americas (NAPPSA) xviii, xix, 50, 65, 203, 282, 303, 334

Nigerian Defence Academy (NDA) 26, 227, 503

Nigerian Health Regulation440

Nigerian Institute of International Affairs (NIIA) 30

Nigerian Institute of Management (NIM)..... 53, 55, 58, 250, 555

Nigerian Labour Congress (NLC) .. 53

Nigerian Law School..... 63

Nigerian Medical Association (NMA)485

Nigerian Police Force (NPF) 7

NIIA See Nigerian Institute of International Affairs

NIM..... See Nigerian Institute of Management

NIPRD See National Institute For Pharmaceutical Research and Development

NIPSS 75, 154, 181, 284, 285, See National Institute For Policy and Strategic Studies

NMCN..... See Nursing and Midwifery Council of Nigeria

Nnaemeka Alfred Ugochukwu Achebe, *cfr, mni* (HRM. Igwe Dr.) 46, 49

Nnaemeka Ugochukwu Alfred

Achebe, *cfr, mni* (HRM Igwe Dr.) 166
 Nnamani (Dr.) 30
 Nnamdi Azikiwe University, Awka (Nau) 48, 281, 282
 NNMDA See Nigeria Natural Medicine Development Agency
 Nnodum Iheme (Pharm.) 50, 389
 Northern Teachers Association (Nta) 16
 NPF See Nigerian Police Force
 NRAs See National Regulatory Authorities
 NTA See Northern Teachers Association
 NTI .. See National Teachers' Institute
 Nuhu Bamalli (Alh.) 6, 20
 Nuhu Bayero (Alh.) 20
 Nuhu Isah Abdullahi, *fpsn* (Pharm.) xvi, 245, 246
 Nuhu Mohammed (Amb.) 21
 Nuhu Namallam Ibrahim (Pharm.) 32, 245
 Nuhu Tahir Tajuddeen (Sheikh). 17, 20
 Nuhu, S. Z. (Pharm.) 102
 NYSC See National Youth Service Corps
O
 Obiaga, G. O. (Chief Pharm.) . 27, 232
 Obudu Cattle Ranch and Resort 363, 364, 369
 Odediran, Folake R. 58
 ODORBN See Optometrists and Dispensing Opticians Registration Board of Nigeria
 Ogunsanya (Pharm. Mrs) 90
 Okoli, Stella, Mon, *oon* (Chief Mrs.) 102, 108
 Olabisi Onabanjo University 102
 Olorukoola, Saratu (Late Dr. Mrs.) . 78
 Olufunmilayo Braide (Prof. Mrs.) 106, 107
 Olujimi Oyetomi 535

Oluleye, David Segun (Prof.) 363
 Olumide Akintayo 532
 Olurinola, P. F. (Prof.) 29
 Olusola Orimoloye (Mrs.) xvi, 230
 Oluwatoyin Adepeju Odeku (Prof. Mrs.) 93
 Onaolapo, J. A. (Prof.) 29
 Optometrists and Dispensing Opticians Board of Nigeria 364
 Optometrists and Dispensing Opticians Registration Board of Nigeria (Odorbn) 366, 367, 403, 429, 432, 449, 461
 Osagie Ehanire . xxiii, xxiv, 160, 483, 535, 537, 546
 Oseji (Dr.) 30
 Oviasu, P. (Pharm. Mrs.) ... 89, 96, 97, 539

P

Pama Nagaba 25
 PANS See Pharmaceutical Association of Nigerian Students
 Patent and Proprietary Medicine Vendors (PPMV)s 100, 154, 169, 315, 316, 353, 380, 488, 489, 493
 Patents and Proprietary Medicines Vendors Licenses (PPMVL) 125
 Patient Care and Healthcare Practice 408, 435
 PCN 93, 95, 97, 103, 305, 347, 459, See Pharmacist Council of Nigeria, See Pharmacy Council of Nigeria inspection and enforcement framework 145
 inspectors 100, 347, 375, 377, 382, 387
 law 50, 386
 responsibilities .. 488, 489, 492, 493
 Persecution 235, 541
 Perseverance xxxii, 181, 252
 Peter Adamu 25
 Peter Ewers 32
 Petition xxiii, 152, 159, 256, 530

Petroleum Trust Fund (PTF)	294	296, 300, 303-306, 309, 311, 312,
Pharco (Nig.) Ltd.	32	314, 316, 321, 322, 326, 334, 339,
Pharmaceutical Association of		340, 342, 343, 347-350, 352, 357,
Nigerian Students (PANS) ..	xxii, 26,	361, 366, 367, 373, 382, 388, 391,
60, 65, 231, 255, 260, 271, 495,		394, 395, 397, 400, 403, 404, 406,
566		413, 421, 425, 429, 430, 432, 436,
Pharmaceutical Care Services	533	445, 449, 456, 461, 465, 483, 486,
Pharmaceutical Inspection		487, 497, 507, 513, 515, 518, 530,
Committees (PICs) ..	133, 395, 409,	537, 542, 543, 546, 550, 557, 560,
437		563
Pharmaceutical Manufacturing		former pharmaceutical inspector
Companies	214, 326	563
Pharmaceutical Plants	377	Pharmacists of Council of Nigeria
Pharmaceutical Service Delivery.	161	(PCN)
Pharmaceutical Society of Nigeria		6
(PSN)	xxiii, 12, 38, 53, 56, 60, 61,	Pharmacovigilance 140, 398, 399,
69, 81, 82, 84, 143, 144, 150, 151,		566
162, 218, 246, 250, 259, 297, 298,		Pharmacy Act
347, 353, 392, 485, 515, 535, 538,		305, 308, 314, 318,
540, 543-545, 548, 555, 557, 560,		322, 323, 324, 351, 352
563		Pharmacy Council of Nigeria (PCN)
former chairman	563	xxvii, xxxi, 12, 14, 36, 62, 69, 92,
former, national financial secretary		169, 170, 173, 174, 205, 213, 223,
.....	563	234, 243, 247, 249, 253, 258, 259,
president	81, 347	267, 271, 277, 290, 496, 497, 500,
Pharmacists Council of Nigeria (PCN)		567
.. 115, 124, 132, 134, 135, 356, 535		Pharmacy Practice ..
Pharmacists		xviii, xix, xx, xxxi,
FG Shuns	xxiii, 535	12, 13, 28, 30, 31, 33, 36, 55-57,
Pharmacists Board of Nigeria (PBN)		62, 66, 67, 68, 82, 84, 85, 97, 103,
...xxvii, xxxi, 13, 81, 82, 95, 96, 234,		117, 121, 122, 125, 126, 136, 139,
337, 388, 391, 393, 397, 484, 497,		140, 159, 160, 165, 171, 172, 184,
507, 538, 541, 547		189, 191, 192, 194, 196, 200, 204,
Pharmacists Council of Nigeria		205, 225, 233, 234, 240-242, 256,
(PCN)	xviii, xix, xx, xxii, xxiii, xxiv,	258, 261, 267, 279, 287-289, 296-
xxvii, xxix, xxxi, xxxii, 11, 13, 14, 35,		298, 301, 303-306, 309, 311-314,
36, 50, 52, 54, 55, 57, 59, 60, 68,		316, 326, 327, 330, 334, 337-342,
82, 84, 95, 99, 104-116, 118, 120,		349-352, 356, 360-362, 376, 386,
121, 133, 135, 142, 144, 145, 147,		388-397, 399, 484, 491, 498, 499,
150, 151, 159, 160, 163, 164, 165,		506, 507, 541, 548, 554, 558, 567
169, 180, 191, 194, 213, 218, 234,		expanding scope of
249, 256, 260, 261, 267, 272, 277,		399
279, 280, 281, 286, 287, 290, 295,		Pharmacy Practice in Nigeriaxviii,
		xix, xx, 36, 66, 139, 225, 234, 240,
		242, 258, 261, 279, 287-289, 303-
		305, 311-313, 337, 349-352, 356,
		361, 362, 376, 388-390, 392, 397,

558, 567
 laws governing 305
 Pharmacy Profession .. v, xxxi, 55, 56,
 58, 60-62, 81, 84, 97, 105, 115,
 118, 121, 137, 138, 139, 149, 159,
 162, 164, 165, 169, 195, 197, 205,
 223, 226, 227, 233, 235, 236, 246,
 248, 260, 289, 355, 384, 391, 486,
 487, 492, 532, 537, 541- 543, 546,
 547, 549, 550, 555, 558
 Philip Emafo (Dr.) 89, 103, 532
 PICs .See Pharmaceutical Inspection
 Committees
 PIFSS 37, 56, 202, See Prominent
 Indigenes and Friends Scholarship
 Scheme
 Pillar of Pharmacy Regulation ...i, iii, iv, xv,
 xxxi, xxxv, 189, 203
 PIU .See Project Implementation Unit
 Pius S. Abanum (Pharm.) 543
 Policy Development 28, 69, 566
 Policy Implementation 441
 Political Battles..... 151
 Political Challenges 121, 122
 Political Support 381
 PPMVL 125, See Patents and
 Proprietary Medicines Vendors
 Licenses
 PPMVs.....See Patent and Proprietary
 Medicine Vendors
 PPP ..See Private-Public- Partnership
 PRBN..... See Radiographers
 Registration Board of Nigeria
 Private-Public- Partnership (PPP) 140
 Pro-Chancellor and Chairman .. xxxii,
 65, 176, 181, 223, 234, 259, 277,
 279, 504, 507
 Professional Development xxii, xxxiii,
 120, 122, 135, 136, 139, 141, 142,
 152, 160, 162, 164, 196, 197, 211,
 256, 302, 311, 317, 339, 364, 365,
 368, 369, 390, 398, 404, 408, 409,
 420, 429, 434, 438, 447, 452, 454,

457, 458, 466, 474, 475, 487, 488,
 493, 560, 567
 Professional Journey iv, xxxii, 196,
 229, 277
 Project Implementation Unit (PIU)
 ... xxvii, 7, 11, 34, 83, 217, 219, 509,
 547
 Prominent Indigenes and Friends
 Scholarship Scheme (PIFSS) 37
 Prosecutions.....314
 Prosecutions in Court314
 Prosecutorial Powers..... 314, 316
 PSN95, 98, 104, 533, 541, 542
 clash with150
 past presidents of.....532
 PTF See Petroleum Trust Fund

Q

Quality Assurance Mechanism490
 Quality of Healthcare... 259, 443, 446
 Queen Amina 1

R

Radiographers Registration Board of
 Nigeria (PRBN)..366, 403, 429, 438,
 449, 461, 496
 Rahina (Hajiya, popularly known as
 “Matan Mallam”) 291, 292
 Ramatuh Yarkano245
 Re-Accreditation Exercises413
 Real- Time Data Collection398
 Reciprocity Agreements.....317
 Reconciliation 210, 236, 542
 Reforms xxxiii, 35, 133, 141, 143-145,
 159, 161, 163, 164, 171, 243, 272,
 342, 365, 495, 564
 pharm Mora's141
 policy.....xxii, 495, 499
 regulatory160
 Registration and Licensing. 350, 403,
 429, 459, 460, 466, 470, 474, 475,
 477, 478, 479, 480
 Registration of Hospital Pharmacies
 125, 312, 319

Regulatory Challenges ...xx, xxii, 349, 358, 359, 495
 Regulatory Dichotomy..... 150
 Regulatory Enforcement Strategies 389
 Regulatory Frameworks..... 163, 196, 204, 493
 Regulatory Mechanisms349, 434
 Regulatory Roles. 381, 398, 442, 463, 472
 Rehabilitation ..xxvi, 7, 11, 34, 36, 83, 116, 217, 218, 219, 267, 366, 367, 403, 406, 407, 429, 432, 433, 449, 461, 464, 496, 509, 511, 547
 Rilwanu Suleiman Adamu, *cfr* (Hrh Alh. Dr.) 48
 Rima Pharmaceuticals Ltd, Ireland33
 Rita N. Pemu, *jca* (Justice) 515
 Role of Regulatory Agencies.....xxi, 421, 435
 Rowland Eyime (Dr.).... 363, 364, 369

S

Salahudeen Na'ibi Sulaiman Wali, *msna, ba, msc, mba*.....xviii, 289
 Salamatu Hussaini (Dr.) xvi, 207
 Salisu Ahmed (Alh.) 21
 Sam Nda-Isaiah (Late Pharm.) 194
 Sambo Kusallo (Mallam) 24
 Sambo, Muhammad Namadi (Arc.) 4, 8, 9, 10, 75, 513
 Sambo, Sabo (Capt.) 7
 Sani Abacha Children Hospital ... 449
 Sani Ahmed Aliyu 24
 Sani Baba 245
 Sani Ibrahim Mohammed (Alh.).....xv, 188
 Sani Isah Hadeja (Late)..... 25
 Sanjoy Hore 274
 Santuraki Babba xv, 193
 Sarah Jamima Ladi Hussan (Mrs.) . 33

Sarat J.L. Hassan (Mrs.) 33
 Sardauna of Sokoto xviii, 291
 Sardaunan Sokoto . xviii, 19, 291, 292
 SBS..... See School of Basic Studies
 School of Basic Studies (SBS)....xxvii, 26, 42, 245, 278, 554
 Sha'aban, Abubakar (Dr.) 21, 249
 Shatu Jibrin 24
 Shehu Garba (Alh.) xv, 190
 Shehu Idris (His Highness Alh.) . 20, 37, 44, 54, 111, 190, 192, 200
 Shehu Idris, *cfr* .. 37, 44, 54, 111, 190, 192
 Shehu Sambo (Alh.) 166
 Shehu Usman Ya'u Kauru (Hrh Alh.) 39
 Shekari 34
 Shettiman Sokoto 20
 Shoge 32
 Sidi Bage Muhammad I, *cfr* (HRH Justice (Rtd)) 48, 210
 SMS See State Medical Stores
 Sokomba, E.N. (Prof.) 90, 108
 Sokoto Caliphate..... xxxiii, 2, 64
 Sources of Drugs.....425
 Sowemimo, B. (Dr.)..... 84
 Staff Development.....163
 Staff Motivation461, 465, 477, 478
 Staff Training35, 154, 161, 499
 Staffing and Educational Quality .137
 State Medical Stores (SMS)508
 Steve Ozioko..... xviii, 290
 Successor.....31, 214, 307, 323
 Sule Dan Abdu (Wing Commander Dr.)505
 Sule Shaban 7
 Sule, M. I. (Prof.) 29
 Suleiman Dangaladiman Kaiama (Alh.) 20
 Suntai, Danbaba Danfulani (Fmr. Governor, Late) 38, 107, 240
 Surajuddeen Abubakar Imam..... 73
 Surajuddeen B. Yakubu, *fbr, fciod*

(Alh.) 193
 Suspension .. 306, 395, 519, 520, 521
 Suzanne Allagh (Prof. Mrs.) 29

T

Tajudeen Olusegun Uthman (Dr.) xvi, 212
 TAMEF... See the Abdurrahman Mora Education Foundation
 Tanimu Kubau (Alh.) 20
 Tanko Ayuba (Pharm.) 79
 Tenurexxxii, 30, 35, 36, 46, 47, 49, 50, 83, 84, 86, 98, 101, 104, 105, 121, 133, 136, 138, 142, 144, 152, 159, 160, 163, 164, 171, 173, 174, 185, 186, 190, 191, 197, 198, 207, 209, 211, 214, 217, 225, 229, 238, 239, 244, 249, 253-255, 257, 261, 279-282, 286, 287, 290
 Tenure and Re-Election 46
 The Abdurrahman Mora Education Foundation (TAMEF) 215
 Thisday Newspapers 534
 Three Musketeers 24
 Tijjani Manumfashi (Alh.) 21
 Tijjani Mohammed 24
 Tijjani Musa Isa (Alh.) 39, 504
 Transparency. 84, 160, 161, 162, 163, 164, 165, 218, 223, 385, 398, 427, 564
 Trans-Saharan Trade 1
 Trauma 236, 453, 520, 542
 Trial Court 516
 Tudun Wada Elders Association.... 56
 Tukura (Mallam) 24
 Turbaning Ceremony 111, 112

U

Umar Usman Pateh (Prof.)... iii, xxiv, xxxv, 29, 66, 67, 110, 240, 559
 Umar, Sunday Samuel (Late Dr.) ... 33
 Umaru Abdulmutallab, *con* (Alh. Dr.) 21, 24, 40, 77
 Umaru Idris (Alh.) 21

Umaru Musa Yar'adua, *gcfr* (Late Former Nigeria President) 343
 Umaru Shehu (Prof.).... 407, 434, 457
 Umaru Tanko Al-Makura (H.E. Fmr. Gov. Nasarawa State) 43
 Unauthorised Alteration 536
 UNI .. See University of Northern Iowa
 United Arewa Chemists, Zaria 22
 University of Iowa 501
 University of London Institute of Education Certificate Course 15
 University of Northern Iowa 16, 74
 University of Sokoto 9
 Usman Bugaje (Dr.) 25
 Usman Dan Fodio (Shehu) xxix, 2
 Usman Khalifa Baba Ahmed (Alh.) . 4, 8
 Usmanu Danfodiyo University, Sokotoxxvii, 27, 263, 265, 266, 284, 285

V

Vice-Chancellor 24, 30, 39, 265, 278, 282, 503
 First Female Pharmacist 69
 Visionary Leadership 35, 70, 83, 133, 163, 215, 243, 262, 272, 281, 288

W

WAHO See West African Health Organization
 Wakilin Magani of Zazzau ... xxix, 203, 223
 Wakilin Maganin Zazzau xxxiii, 12, 54, 55, 61, 63, 71, 111-113, 185, 189, 191, 192, 200, 207, 220, 231, 243, 246, 247, 250, 261, 268, 269, 272, 274, 277, 281, 294, 537, 542
 WAPCP 485, See West African Postgraduate College of Pharmacists fellowship of 27
 WAPMA See West African Pharmaceutical Manufacturers

Association
 WAUAA . See West African University
 Alumni Association
 Wayo, Clara (Mrs.) 24
 Waziri Umaru 503
 WDF See World Dental Federation
 West African Health Organization
 (WAHO) 339, 399
 West African Pharmaceutical
 Manufacturers Association
 (WAPMA) 41, 42, 68, 117, 341
 West African Postgraduate College
 of Pharmacists (WAPCP) .xxvii, 13,
 27, 71, 171, 341, 483, 555, 557,
 562
 West African University Alumni
 Association (WAUAA)49, 239
 WHPA See World Health Professional
 Alliance
 World Dental Federation (WDF) 408,
 435
 World Health Organization (WHO)
 . 162, 317, 331, 344, 359, 374, 378,
 403, 408, 428, 435, 481
 World Health Professional Alliance
 (WHPA) 481

Y

Ya'u Shehu Usman (HRH) xvi, 221
 Yahaya Barau (Mallam)..... 24
 Yahaya Gusau.....15, 20
 Yahaya Hassan 72
 Yahaya Ibrahim 166
 Yakubu Aboki Ochefu (Prof.) 50
 Yakubu Gowon, *gcfr* (Rtd. Gen. Fmr.
 Nigeria Head of State) ... 19, 20, 41,
 77
 Yemi-Esan, Folashade (Dr.) .236, 483
 Yohana Gaiya (Pharm.) 29
 Yohana Madaki (Dr.) 33
 Yomi Osewa (Pharm.) 377
 Yoosu Tersoo Jonathan, *fpcpharm*
xvii, 259

Yuguda Usman (Late Alh.) 33
 Yusuf Adamu..... 23
 Yusuf Datti Baba Ahmed (Sen. Dr.) 4,
 8, 9, 10
 Yusuf Hamisu Abubakar (Hon. Alh.)
 xv, 83, 219
 Yusufu Ladan (Alh.)..... 21

Z

Zakari Mohammed Sada (Alh.)..... 39
 Zakari Sada276
 Zakari, Yahaya (Dr.) 27
 Zaria Education Development
 Association (ZEDA) 6, 36, 56, 61,
 65, 202, 237
 Zaria Pharmaceutical Company Ltd.
 (ZPC) xxvii, xxxi, xxxii, 7, 11, 35, 65,
 83, 111, 191, 218, 249, 258, 260,
 267, 294, 509
 Zazzau Emirate xvi, xxxiii, 12, 36, 37,
 54, 185, 189, 192, 202, 203, 223,
 250, 275
 Zazzau Emirate Council
 Advancement Forum (ZECAF) ..192
 Zazzau Emirate Development
 Association (ZEDA)192
 Zazzau Pharmaceutical Industries
 Ltd (ZPIL)..xxvii, xxxi, 9, 11, 35, 116,
 249, 267
 ZECAF... See Zazzau Emirate Council
 Advancement Forum
 ZEDA.....37, See Zaria Education
 Development Association, See
 Zazzau Emirate Development
 Association
 ZPCSee Zaria Pharmaceutical
 Company Ltd.
 ZPILSee Zazzau Pharmaceutical
 Industries Ltd

